THE NCE OWNERSHIP METHOD

A Complete Study Guide for the National Counselor Examination

Every wrong answer belongs to someone. Learn who.

PassReady Prep · GA Integrated Therapeutic Perspectives LLC

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A note on abuse. No amount of abuse is acceptable — not physical, not emotional, not sexual, not financial, not verbal. Abuse is not a communication problem. If you or someone you know needs help, call the National Domestic Violence Hotline at 1-800-799-7233 (thehotline.org) or text HOME to 741741 (Crisis Text Line).

[KE — add cover designer and interior layout credits at layout.]

A note on safety. No amount of abuse is acceptable — not physical, not emotional, not sexual, not financial, not verbal. Abuse is not a communication problem. If you or someone you know is being hurt, contact the National Domestic Violence Hotline at 1-800-799-7233 (thehotline.org) or the Crisis Text Line (text HOME to 741741).

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About the Author

KekeTheLPC (MA, LPC, NCC, BC-TMH, CPCS) is a Licensed Professional Counselor, National Certified Counselor, Board Certified–TeleMental Health Provider, and Certified Professional Counselor Supervisor in Georgia. She is the founder of GA Integrated Therapeutic Perspectives LLC and of PassReady Prep, an exam-preparation platform for counselors.

She passed the NCE on her first attempt in 2014 using the two habits this book is built on: pairing every term with the theory that owns it, and listening to review audio on repeat until the pairs were automatic. As a supervisor she now watches pre-licensed counselors prepare for the same exam, and she wrote this book to give them the method she wishes someone had handed her on day one.

She is also the author of *The Complete NCMHCE Study Guide*.

[KE — add years in practice / clinical focus / city if you want them public.]

How I Passed

I have always been a nervous tester. So it surprised no one, least of all me, that I put off the NCE after graduation. I put it off for two years.

The job market at the time meant I had to work outside the field to keep the bills paid on time. Working outside the field cost me my counseling tribe, the people who would have talked shop with me and kept the material alive. Losing them made the idea of testing scarier, not easier, and I got very good at bellyaching my excuses.

Then I caught one of my best friends in a bad mood on the right day. He said, “I’m sick of you.” A few days later we went to lunch, and he secretly left me the money for the testing fee. I did not want to disappoint someone who believed in me that much, so I registered for the exam and set the date six months out.

I bought every prep resource I could find: the ones written by real practitioners, and one from an overpriced testing mill. Then I studied. Two hours a day or more, five days a week. The more I studied, the dumber I felt. Remember, I had been out of the field for over two years. So I quit, and I started trying to figure out how to pay my friend back.

Then, out of nowhere, the text came: *How’s the studying going?*

I hadn’t found the money. I believe in karma. So I picked that purple bible back up and started again. But this time, the Lord — or the universe, whichever you can accept — said that memorizing wasn’t going to get me what I needed. That is how this process was formed.

I stopped trying to memorize everything and started pairing each term with the theory that owns it. Empty chair: Gestalt. Miracle question: solution-focused. Differenti-

ation of self: Bowen. On the exam, the wrong answers are rarely nonsense. They are real counseling terms that belong to a different theory. If you know who owns what, you can see the trap before you step in it. I put the pairs on audio and played them on repeat during every drive, until I didn't have to think about them anymore. They were just there.

I took the NCE in 2014 and passed on my first attempt.

That's the method this book is built on. Learn who owns each term. Hear it until it's automatic. Then practice spotting the owner of every wrong answer.

This book should also keep you from buying everything you can get your hands on, or dusting off all the textbooks you have in storage. I tried to make it as comprehensive and affordable as possible, and still liftable. If you are a nervous tester who has been putting this off, I wrote this for you.

— KekeTheLPC

How to Use This Book

The idea in one sentence

NCE distractors — the wrong answer options — are almost never made-up words. They are real terms that belong to a different theory, theorist, test, ethical standard, or diagnosis. **If you can name the owner of every option, the right answer is the only one left standing.** This book calls that the Ownership Method.

The four-part loop

Chapters 2–22 are built for the same loop. Run it once per chapter, in order.

Step	What you do	Where it lives
1. Read	Learn the owners and the teaching. Notice the "What the exam rewards / punishes" box.	Chapter body
2. Drill	Cover the right column of the swap-pair table and say each owner out loud.	Swap-Pair Drill, end of each chapter
3. Listen	Play the chapter's audio track during any drive, walk, or chore. Answer during the pauses.	Audio Companion (PassReady Prep)
4. Test	Answer the quick checks. For every option, right or wrong, name its owner before you read the rationale.	Quick Checks, end of each chapter

Then log every miss in the error log (Workbook): *"Who did I think owned it? Who actually owns it?"* The error log is your personal study guide for the final two weeks.

How each chapter is laid out

1. **Domain map**: which NCE content-outline statements the chapter covers, so you know why it's here.
2. **Ownership tables**: term → owner, with the look-alike it gets confused with.
3. **Teaching**: the concepts, written for single-answer recall and application.
4. **What the exam rewards / punishes**: the answer patterns that score and the traps that don't.
5. **Swap-pair drill**: the pairs most likely to appear as each other's distractors.
6. **Quick checks**: 10 or more items (Chapter 1 has two). Every rationale names the owner of every wrong option, not just why the key is right.
7. **Audio**: the track number(s) and target length for that chapter.

Where to start

- **Eight weeks or more:** follow the 8-week plan in the Workbook. It is weighted the way the exam is weighted and schedules the three practice exams in weeks 5, 7, and 8.
- **Four weeks or less:** read Chapters 1 and 2, then Chapters 4–6 and 17. Take Practice Exam 1 at the end of week 1, and let its domain scores choose your next chapters.
- **Retaking:** start with your NBCC score report. Go straight to the chapters for your two weakest domains, then take a practice exam.

Chapter 2 is the spine. If you read nothing else before a practice test, read Chapter 2 and drill its swap-pair table until you can do it without looking.

How to use the practice exams

Take each one in a single 3-hour-45-minute sitting, at the pacing in Chapter 1. Then grade it with the **Ownership**

Map answer key: for each item, it gives the correct answer and its owner, each wrong option and its owner, and the pair to learn. The score-by-domain table at the end of each key tells you where your next hours should go.

A practice-exam score is a direction, not a prediction. NBCC sets a different cut score for each form, so no percentage here maps onto a pass or fail.

How to use the audio

The audio companion is call-and-response: the narrator gives a term, pauses, and you say the owner before the answer plays. Say it out loud. Hearing yourself answer is what moves a pair from “I’ve seen that” to “I know that.” Replay tracks on repeat; repetition is the point, not a sign you’re behind.

Track numbers match chapter numbers (Track 10.1 goes with Chapter 10). Chapter 1 has no track.

The NCE at a Glance

Sponsor	National Board for Certified Counselors (NBCC)
Used for	National Certified Counselor (NCC) credential and licensure in many states
Items	200 multiple-choice: 160 scored, 40 unscored and unmarked
Time	3 hours, 45 minutes (about 67 seconds per item)
Item format	Standalone single items; one best answer from four options
Passing score	Set per form by modified Angoff; no fixed percentage
Wrong-answer penalty	None. Never leave an item blank.

Domain	Scored items	Share	Chapters
Counseling Skills & Interventions	48	30%	4–9
Areas of Clinical Focus	47	29%	10–16
Professional Practice & Ethics	19	12%	17–18
Intake, Assessment & Diagnosis	19	12%	19–20
Treatment Planning	14	9%	21
Core Counseling Attributes	13	8%	22
Total	**160**	**100%**	

Two domains account for 95 of the 160 questions that count toward your score: 59% of everything that's scored. Those 160 scored items are mixed in among 200 total questions, and the 40 unscored practice items look exactly like the rest. You can't tell which ones count, so answer every question as if it does, and never skip one because you suspect it's a practice item.

Source: NBCC (2023), *Content Outline: The National Counselor Examination (NCE)*. Confirm current details at nbcc.org.

Testing on or after July 1, 2027? NBCC releases a new NCE on that date: 170 items with three answer options each (140 scored, 30 unscored field-test items), the same 3 hours 45 minutes with an optional 15-minute break at the halfway point, a scaled score (100–500, passing at 360), and six new task-based domains — Legal & Ethical Compliance and Provision of Counseling Interventions at 20% each, and a new Indirect Client Care domain at 12% (NBCC, 2026). If you test before July 1, 2027, this book matches your exam. If you test after, the owners, theories, ethics, and assessment content still apply, but the format, domains, and weights differ: use the second edition of this book, and confirm details at nbcc.org.

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PART ONE

The Exam and the Method

Chapter 1 — How the NCE Works and How to Take It

What the NCE Is

The National Counselor Examination (NCE) measures whether an entry-level counselor knows the theoretical and skill-based foundations needed to practice safely. It is used for two purposes: state licensure in many states, and the National Certified Counselor (NCC) credential (NBCC, 2023).

The exam was built from a national job analysis of more than 16,000 credentialed counselors. That analysis produced six **work behaviors**, called domains. The exam is also aligned with the eight CACREP core content areas you studied in graduate school. In practice, this means two things:

1. The **score** is built from the six domains.
2. The **content** inside those domains still draws on everything CACREP covers: theories, development, career, groups, assessment, research, ethics, and culture.

The Format

Feature	NCE
Total items	200 multiple-choice
Scored items	160
Unscored (field-test) items	40, mixed in randomly
Time	3 hours, 45 minutes
Item type	Standalone single items (no case sequences)

Source: NBCC (2023), *NCE Content Outline.*

Testing on or after July 1, 2027? NBCC releases a new NCE on that date: 170 items with three answer options each (140 scored, 30 unscored field-test items), the same 3 hours 45 minutes with an optional 15-minute break at the halfway point, a scaled score (100–500, passing at 360), and six new task-based domains — Legal & Ethical Compliance and Provision of Counseling Interventions at 20% each, and a new Indirect Client Care domain at 12% (NBCC, 2026). If you test before July 1, 2027, this book matches your exam. If you test after, the owners, theories, ethics, and assessment content still apply, but the format, domains, and weights differ: use the second edition of this book, and confirm details at nbcc.org.

The 40 unscored items look exactly like the scored ones. You will never know which is which. This matters psychologically: when you hit an item that seems strange, poorly worded, or about something you've never heard of, there is a real chance it doesn't count. Answer it, flag it if you want, and move on. Do not let one odd item cost you three good ones.

The Six Domains

NBCC #	Domain	% of items	Scored items
5	Counseling Skills & Interventions	30%	48
3	Areas of Clinical Focus	29%	47
1	Professional Practice & Ethics	12%	19
2	Intake, Assessment & Diagnosis	12%	19
4	Treatment Planning	9%	14
6	Core Counseling Attributes	8%	13
	Total	**100%**	**160**

Source: NBCC (2023).

Read that table twice. **Two domains account for 95 of the 160 questions that count toward your score: 59%**

of everything that's scored. Those 160 scored items are mixed in among 200 total questions, and the 40 unscored practice items look exactly like the rest. You can't tell which ones count, so answer every question as if it does, and never skip one because you suspect it's a practice item.

If your study plan spends as much time on Treatment Planning (14 items) as on Counseling Skills (48 items), it is misallocated by a factor of three.

How It's Scored

There is no fixed passing percentage. NBCC sets a passing score for each exam form using a **modified Angoff** standard-setting method: subject-matter experts estimate how a minimally qualified candidate would perform on each item, and those estimates produce the cut score for that form. Because forms differ slightly in difficulty, the passing score differs slightly too.

What this means for you:

- **Ignore any website that tells you "you need 70%" or "you need 84%."** Those numbers are not published by NBCC.
- **Every item you answer correctly counts the same.** There is no penalty for a wrong answer beyond not earning the point. **Never leave an item blank.**
- Your score report breaks performance down by domain. Use that if you need to retake.

Pacing

You have 225 minutes for 200 items: **about 67 seconds per item.** Aim for 60, which banks about 25 minutes for review.

Checkpoint	Target time elapsed	Hard limit
Item 50	0:50	0:56
Item 100	1:40	1:52

Checkpoint	Target time elapsed	Hard limit
Item 150	2:30	2:49
Item 200	3:20	3:45

Most NCE items are short. Recall items ("Which theorist…") should take 20–30 seconds. Applied items ("What should the counselor do next…") take longer. The time you save on recall pays for the applied items.

Flagging rule: flag an item only if you can name what you'd check on a second look. "I'm torn between B and D because I'm not sure whether this is Bowen or Minuchin" is a good flag. "I don't feel good about this" is not. Pick your best answer before you flag, every time.

The Four-Step Item Routine

Use this on every item until it is automatic.

1. Read the last line first. The final sentence tells you what's being asked: a theorist, a technique, a next step, an ethical action, a definition. Knowing the question type before you read the details tells you which details matter.

2. Name the owner. Ask: *which theory, domain, or framework is this item living in?* A stem (the question part of the item) that mentions "irrational beliefs" lives in REBT. A stem about "the group's second session, members testing the leader" lives in group stage theory. Naming the owner narrows the right answer before you see the options.

3. Predict, then look. Form your answer before reading the choices. If your prediction is among the options, it's probably right. If it isn't, read the options carefully. You may have named the wrong owner.

4. Eliminate by owner. For each wrong-looking option, ask who owns it. On the NCE, distractors are usually real terms from a *different* theory, stage model, or framework. When you can name the other owner, you can cross the option out with confidence instead of guessing.

Example (original item)

> A counselor asks a client, “Suppose you went to sleep tonight and, while you slept, the problem was solved. What would be different tomorrow?” This intervention is most associated with which approach?
> A. Narrative therapy B. Solution-focused brief therapy C. Reality therapy D. Gestalt therapy

Last line: which approach owns the technique. **Owner:** the miracle question belongs to solution-focused brief therapy (de Shazer & Berg). **Eliminate:** narrative owns externalizing and re-authoring; reality therapy owns WDEP and choice theory; Gestalt owns the empty chair and here-and-now awareness. **Answer: B.**

Notice that every wrong answer is a real, well-known approach a counselor might plausibly use. That’s how NCE distractors work: they are plausible because they are real.

Rules for Applied Items

When the item asks what a counselor *should do*, these priorities decide most of them:

1. **Safety first.** Any sign of risk to self or others comes before every other goal.
2. **Relationship before technique.** Early in counseling, the alliance-building response (empathy, reflection) beats the clever intervention.
3. **Assess before you intervene.** Don’t treat what you haven’t assessed.
4. **Follow the code, not your instincts.** When ethics are involved, the ACA Code of Ethics decides, and consultation is almost always part of the right answer.
5. **Stay in scope.** When a need falls outside your competence, refer or consult rather than improvise.
6. **Client autonomy.** Between two otherwise equal options, choose the one that involves the client in the decision.

Allocating Your Study Time

Study hours should follow scored items. For an 8-week plan of about 10 hours per week:

Domain	Scored items	Hours (of 80)
Counseling Skills & Interventions	48	24
Areas of Clinical Focus	47	23
Professional Practice & Ethics	19	10
Intake, Assessment & Diagnosis	19	10
Treatment Planning	14	7
Core Counseling Attributes	13	6

The Theory Ownership work in Chapters 2 and 3 is not a separate block. It runs through Counseling Skills (theory-based interventions), Areas of Clinical Focus (developmental and career theorists), and Professional Practice & Ethics (research and statistics owners). Do 15 minutes of ownership drills every day, and put the audio companion on during any drive, walk, or chore. That's where the repetition comes from.

Test Day

- Arrive early. Bring the ID your registration requires.
- Use the scratch materials the testing center provides to write the pacing checkpoints before you start.
- If you blank on an item, use step 4 anyway. Eliminating two options by owner turns a 25% guess into a 50% guess.
- At the end, check that no item is blank.

Quick Checks

1. Approximately how many NCE items contribute to your score? A. 100 B. 160 C. 175 D. 200

2. You encounter an item on a topic you've never studied. Best response? A. Skip it and leave it blank to save time B. Spend extra time, since it is probably scored C. Choose

your best answer, flag if useful, and move on D. Answer randomly without reading

Answers: 1-B. Of 200 items, 160 are scored; 40 are field-test items. 2-C. The item may be unscored, a blank earns nothing, and time spent here costs you items you know.

References

National Board for Certified Counselors. (2023). *Content outline: The National Counselor Examination (NCE).* https://www.nbcc.org/exams/nce

National Board for Certified Counselors. (2026). *National Counselor Examination (NCE): Examination specifications, effective July 1, 2027.* https://nbcc.org/assets/exam/NCE_exam_spec_2027.pdf

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Chapter 2 — The Theory Ownership Matrix

Domain map: 5.F Apply theory-based counseling interventions · 5.R Educate client about transference and defense mechanisms · 5.A–B Align intervention with developmental level and modality · CACREP: Counseling and Helping Relationships; Human Growth and Development (family systems content)

Why This Chapter Comes First

Most NCE theory items are not hard because the content is obscure. They are hard because **every option is real.** A question about Gestalt will offer you three other genuine techniques, each owned by a different approach. If you know a technique only vaguely ("that's one of the experiential ones"), all four options look possible. If you know exactly who owns each one, three of them disappear.

So this chapter teaches theory the way the exam tests it: as **ownership.** For every major approach you'll learn six things:

Field	The question it answers
Owner	Who founded or is most associated with it?
Key terms	What vocabulary signals this theory in a stem?
Signature techniques	What does the counselor actually do?
View of the problem	Where does distress come from?
Goal	What counts as success?
Counselor role	Expert, collaborator, coach, observer, director?

You don’t need to memorize the tables in one pass. Read through once, then use the three-pass drill at the end of the chapter.

Family 1 — Psychodynamic

The psychodynamic family shares one idea: **what you can’t see is driving what you do.** Early experience and unconscious material shape present behavior, and insight is the route to change. Stems signal this family with words like *unconscious, early childhood, insight, defense, transference,* and *interpretation.*

Theory	Owner	Key terms	Signature techniques	View of problem
Psychoanalysis	Sigmund Freud	Id, ego, superego; psychosexual stages (oral, anal, phallic, latency, genital); unconscious; anxiety (reality, neurotic, moral); defense mechanisms	Free association; dream analysis (manifest vs. latent content); interpretation; analysis of transference and resistance	Unresolve unconscio conflict, c from earl developm
Individual Psychology	Alfred Adler	Social interest (*Gemeinschaftsgefühl*); inferiority feelings and striving for superiority; lifestyle; birth order; family constellation; fictional finalism; basic mistakes	Early recollections; lifestyle assessment; encouragement; “acting as if”; “spitting in the client’s soup”; push-button technique; catching oneself; the Question (“What would be different if you were well?”)	Discourag and mista beliefs ab how to be

Theory	Owner	Key terms	Signature techniques	View of problem
Analytical Psychology	Carl Jung	Collective unconscious; archetypes; persona; shadow; anima/animus; self; introversion/extraversion; individuation	Dream and symbol work; active imagination	One-sided of person unintegra parts of t psyche
Object Relations / Attachment	Melanie Klein, Donald Winnicott, Margaret Mahler, John Bowlby	Internal objects; splitting; "good-enough mother" and holding environment (Winnicott); separation–individuation (Mahler); secure base, internal working models (Bowlby)	Exploring relational patterns as they appear in the therapeutic relationship	Early rela patterns internaliz repeated

Defense mechanisms belong to Freud (later systematized by Anna Freud). If a stem asks the counselor to help a client recognize repression, projection, or displacement, you are in the psychodynamic family, and the answer involves insight, not skills training.

Defense	What it looks like
Repression	Pushing threatening material out of awareness
Denial	Refusing to acknowledge an external reality
Projection	Attributing one's own unacceptable feelings to someone else
Displacement	Redirecting feelings to a safer target
Reaction formation	Expressing the opposite of what one feels
Rationalization	Creating a logical-sounding excuse
Regression	Returning to an earlier developmental level

Defense	What it looks like
Sublimation	Channeling impulses into acceptable outlets (considered mature)
Intellectualization	Retreating into abstract thinking to avoid feeling

Family 2 — Humanistic and Existential

This family shares a belief that **people have an inherent drive toward growth and meaning,** and that distress comes from being blocked from it. The relationship and present awareness do the healing. Signal words: *here and now, self-actualization, authenticity, meaning, freedom, responsibility, awareness.*

Theory	Owner	Key terms	Signature techniques	View of problem	Goal
Person-Centered	Carl Rogers	Actualizing tendency; conditions of worth; incongruence between self-concept and experience; core conditions (congruence/genuineness, unconditional positive regard, accurate empathic understanding)	Reflection of feeling; empathic responding; the core conditions themselves *are* the technique; nondirective stance	Incongruence created by conditions of worth	Cong beco fully func pers

Theory	Owner	Key terms	Signature techniques	View of problem	Goal
Gestalt	Fritz Perls (with Laura Perls, Paul Goodman)	Here and now; awareness; figure/ground; unfinished business; top dog/underdog; contact-boundary disturbances (introjection, projection, retroflection, confluence, deflection); "what" and "how" over "why"	Empty chair; two-chair dialogue; exaggeration; staying with the feeling; "I take responsibility for…"; making the rounds (group); dream work as parts of self	Blocked awareness; unfinished business; fragmented self	Awar and integ owni expe
Existential	Viktor Frankl; Rollo May; Irvin Yalom	Meaning; freedom and responsibility; existential anxiety (normal) vs. neurotic anxiety; four givens: death, freedom, isolation, meaninglessness (Yalom); logotherapy (Frankl)	Paradoxical intention and dereflection (Frankl); Socratic dialogue about meaning; confronting the givens	Avoiding or being overwhelmed by the givens of existence	Auth livin findi mean acce resp
Self-actualization (needs theory)	Abraham Maslow	Hierarchy of needs: physiological → safety → love/belonging → esteem → self-actualization; peak experiences	(Theory, not a therapy)	Unmet lower needs block growth	Self-actua

Watch the trap: Rogers' *actualizing tendency* and Maslow's *self-actualization* have different owners. Rogers wrote a therapy; Maslow wrote a motivational theory.

Family 3 — Behavioral and Cognitive-Behavioral

This family focuses on **what can be observed, measured, and changed.** Behavior is learned; thoughts drive feelings; change happens through practice, homework, and testing beliefs. Signal words: *reinforcement, homework, irrational, automatic thoughts, distortions, measurable, skills.*

Behavioral

Theory	Owner	Key terms	Signature techniques	View of problem
Classical conditioning	Ivan Pavlov; John B. Watson ("Little Albert")	Unconditioned/ conditioned stimulus and response; generalization; extinction	Exposure; counterconditioning	Learned associations
Systematic desensitization	Joseph Wolpe	Reciprocal inhibition; anxiety hierarchy; SUDS (subjective units of distress)	Relaxation training + graded imagined exposure up the hierarchy	Conditioned anxiety
Operant conditioning	B. F. Skinner	Positive/ negative reinforcement; positive/ negative punishment; extinction; shaping; schedules of reinforcement	Token economy; shaping by successive approximations; contingency management	Behavior maintained by its consequences
Social learning / social cognitive	Albert Bandura	Modeling; observational learning; self-efficacy; reciprocal determinism	Modeling; behavioral rehearsal; mastery experiences	Learned by observation; low self-efficacy

Other behavioral terms with owners: **Premack principle** (David Premack: a preferred activity reinforces a less-preferred one), **implosive therapy** (Thomas Stampfl: imagined flooding), **token economy** (Ayllon & Azrin).

Negative reinforcement is not punishment. Negative reinforcement *removes* something unpleasant to *increase* a behavior (seatbelt chime stops when you buckle). Punishment *decreases* a behavior. This distinction is easy to test and easy to miss.

Cognitive and Cognitive-Behavioral

Theory	Owner	Key terms	Signature techniques	View of problem
Rational Emotive Behavior Therapy (REBT)	Albert Ellis	A-B-C(-D-E): Activating event, Belief, Consequence, Disputing, Effect; irrational beliefs; "musts," "shoulds"; "musturbation"; awfulizing; low frustration tolerance; unconditional self-acceptance	Disputing irrational beliefs (often forcefully); shame-attacking exercises; rational-emotive imagery; homework	Irrational beliefs, not events, create disturbance
Cognitive Therapy (CT)	Aaron Beck	Automatic thoughts; cognitive distortions; schemas/core beliefs; cognitive triad of depression (negative view of self, world, future); collaborative empiricism	Socratic questioning; thought records; behavioral experiments; testing evidence; activity scheduling	Distorted information processing

Theory	Owner	Key terms	Signature techniques	View of problem
Cognitive Behavior Modification	Donald Meichenbaum	Self-talk; self-instructional training; stress inoculation training (conceptualization → skill acquisition → application)	Self-instructional training; stress inoculation	Maladaptive internal dialogue
Reality Therapy / Choice Theory	William Glasser (WDEP: Robert Wubbolding)	Choice theory; five basic needs (survival, love/belonging, power, freedom, fun); quality world; total behavior (acting, thinking, feeling, physiology); no excuses; no punishment; responsibility	WDEP: Wants, Doing, Evaluation, Planning; "Is what you're doing getting you what you want?"	Unsatisfying relationships and ineffective choices to meet needs
Dialectical Behavior Therapy (DBT)	Marsha Linehan	Dialectics (acceptance *and* change); biosocial theory; validation; wise mind	Skills modules: mindfulness, distress tolerance, emotion regulation, interpersonal effectiveness; diary cards; chain analysis	Emotional vulnerability + invalidating environment
Acceptance and Commitment Therapy (ACT)	Steven Hayes	Psychological flexibility; cognitive fusion/defusion; experiential avoidance; values; committed action; self-as-context	Defusion exercises; values clarification; mindfulness; willingness	Fusion with thoughts and avoidance of experience

Theory	Owner	Key terms	Signature techniques	View of problem
Multimodal Therapy	Arnold Lazarus	BASIC I.D. (Behavior, Affect, Sensation, Imagery, Cognition, Interpersonal, Drugs/biology); technical eclecticism	Assessment across all seven modalities; bridging; tracking	Problems across multiple modalities

Change-Process Models (often confused with theories)

Model	Owner	Key terms	What it's for
Motivational Interviewing (MI)	William Miller & Stephen Rollnick	Spirit of MI (partnership, acceptance, compassion, evocation); OARS (Open questions, Affirmations, Reflections, Summaries); change talk; sustain talk; developing discrepancy; rolling with resistance (called "responding to sustain talk and discord" in the 2013 edition)	A *method* for resolving ambivalence
Transtheoretical Model (Stages of Change)	James Prochaska & Carlo DiClemente	Precontemplation, contemplation, preparation, action, maintenance (and relapse/ termination)	A *description* of readiness, not a therapy

Model	Owner	Key terms	What it's for
Transactional Analysis (TA)	Eric Berne	Ego states (Parent, Adult, Child); transactions (complementary, crossed, ulterior); strokes; games; life scripts; life positions ("I'm OK, you're OK")	Analyzing communication and life scripts

Watch the trap: "Contemplation" is a *stage of change* (Prochaska & DiClemente). It is never a *group stage*. It makes a tempting distractor in group-stage items.

Family 4 — Postmodern and Constructivist

This family assumes **reality is constructed through language and stories,** and that clients are the experts on their own lives. Focus is on strengths, solutions, and alternative narratives, not on pathology or history. Signal words: *exceptions, solutions, strengths, story, externalize, preferred future, client as expert.*

Theory	Owner	Key terms	Signature techniques	View of problem	Goal	Coun role
Solution-Focused Brief Therapy (SFBT)	Steve de Shazer & Insoo Kim Berg	Exceptions; preferred future; small changes; client as expert; "if it works, do more of it"	Miracle question; exception questions; scaling questions (0–10); coping questions; compliments; first-session formula task	Clients get stuck doing more of what doesn't work	Construct solutions; do more of what works	Collab curiou future focuse

Theory	Owner	Key terms	Signature techniques	View of problem	Goal	Coun role
Narrative Therapy	Michael White & David Epston	Problem-saturated story; dominant discourse; unique outcomes; thin vs. thick descriptions; "the person is not the problem; the problem is the problem"	Externalizing the problem; mapping the problem's influence; identifying unique outcomes; re-authoring; therapeutic letters; definitional ceremonies/ outsider witnesses	Oppressive dominant stories	Re-author a preferred story	Co-au curiou expert

Watch the trap: Both use questions, and both are future/strength-oriented. Tell them apart by the signature move. *Exceptions and miracles* belong to SFBT. *Externalizing and unique outcomes* belong to narrative. "Unique outcome" and "exception" mean similar things; the word in the stem decides the owner.

Family 5 — Family Systems

In this family, **the client is the system,** not the individual. Symptoms serve a function in the family, and change in one member changes the whole. Signal words: *system, homeostasis, identified patient (IP), boundaries, triangles, hierarchy, circular causality, family of origin.*

Theory	Owner	Key terms	Signature techniques	View of problem
Bowenian / Multigenerational	Murray Bowen	Differentiation of self; triangles/ triangulation; emotional cutoff; multigenerational transmission process; nuclear family emotional process; family projection process; sibling position; societal regression	Genogram; detriangulation; coaching the client to differentiate in the family of origin; calm, process questions	Low differenti anxiety p across generatio
Structural	Salvador Minuchin	Structure; subsystems (spousal, parental, sibling); boundaries (rigid, clear, diffuse); enmeshment; disengagement; hierarchy; coalitions	Joining; accommodating; enactment; family mapping; boundary making; unbalancing; reframing	Dysfuncti structure unclear boundari inverted hierarchy
Strategic	Jay Haley; Cloé Madanes	Hierarchy; power; symptom as communication	Directives (straightforward and paradoxical); prescribing the symptom; ordeal therapy (Haley); pretend techniques (Madanes)	Symptom maintain dysfuncti hierarchy

Theory	Owner	Key terms	Signature techniques	View of problem
MRI / Brief Strategic	Mental Research Institute: Paul Watzlawick, John Weakland, Don Jackson, Richard Fisch (from Gregory Bateson's work)	Double bind (Bateson); homeostasis; first-order vs. second-order change; attempted solutions become the problem; "you cannot not communicate"	Reframing; paradoxical interventions; interrupting attempted solutions	The famil attempte solution maintains problem
Milan Systemic	Mara Selvini Palazzoli, Luigi Boscolo, Gianfranco Cecchin, Giuliana Prata	Hypothesizing, circularity, neutrality; family games	Circular questioning; positive connotation; invariant prescription; rituals; team behind mirror	Family "g maintains symptom
Experiential / Human Validation	Virginia Satir	Self-esteem; communication stances: placater, blamer, super-reasonable (computer), irrelevant (distracter), congruent; family rules	Family sculpting; family reconstruction; role-play; "parts party"	Low self- esteem a incongru communi
Symbolic-Experiential	Carl Whitaker	Absurdity; spontaneity; "the battle for structure, the battle for initiative"	Co-therapy; playfulness; provoking with absurdity	Emotiona deadness suppresse impulses

Theory	Owner	Key terms	Signature techniques	View of problem
Emotionally Focused Therapy (EFT)	Sue Johnson (with Les Greenberg)	Attachment; negative interaction cycle (pursue–withdraw); primary vs. secondary emotions; softening	De-escalating the cycle; accessing primary emotion; enactments; restructuring bonds	Insecure attachme expressed through a negative
Gottman Method	John & Julie Gottman	Four Horsemen (criticism, contempt, defensiveness, stonewalling); Sound Relationship House; repair attempts; flooding; 5:1 ratio	Assessment; softened startup; repair; love maps; building fondness	Negative interactio patterns the friend

Watch the traps: - **Bowen vs. Minuchin:** *Triangles, differentiation, genogram, cutoff* → Bowen. *Boundaries, enmeshment, enactment, joining, subsystems* → Minuchin. - **Minuchin vs. Haley:** both care about hierarchy. *Boundaries and structure* → Minuchin. *Power, directives, ordeals* → Haley. - **Haley vs. Frankl:** both use paradox. *Prescribing the symptom in a family* → strategic. *Paradoxical intention with an individual* → Frankl (existential/logotherapy). - **Satir vs. Bowen:** both look at family of origin. *Communication stances, sculpting, self-esteem* → Satir. *Differentiation and multigenerational transmission* → Bowen.

Family 6 — Feminist and Multicultural

Theory	Owner	Key terms	Signature techniques	Goal	C ro
Feminist Therapy	Jean Baker Miller; Carolyn Zerbe Enns; Laura Brown; others (collective founding)	"The personal is political"; egalitarian relationship; gender-role socialization; power; social change; relational-cultural theory (Jean Baker Miller)	Gender-role analysis; power analysis; assertiveness training; reframing symptoms as responses to oppression; bibliotherapy; social action	Empowerment; social as well as individual change	Eg pa de co
Multicultural Counseling Competencies	Derald Wing Sue and colleagues; Multicultural and Social Justice Counseling Competencies (MSJCC; Ratts et al.)	Counselor self-awareness, knowledge of client worldview, culturally appropriate skills; action; cultural humility; privilege and oppression	Broaching; culturally adapted interventions; advocacy	Culturally responsive, just practice	Av hu ac

The Swap-Pair Table

These are the pairs most likely to appear as each other's distractors. Learn them as pairs.

If you see…	It belongs to…	Not to… (because)
Disputing irrational beliefs; "musts"	Ellis (REBT)	Beck: Beck *tests* automatic thoughts collaboratively; Ellis *disputes* beliefs directly

If you see…	It belongs to…	Not to… (because)
Cognitive triad; collaborative empiricism; Socratic questioning	Beck (CT)	Ellis
Social interest; birth order; lifestyle	Adler	Freud: Freud is drives and psychosexual stages
Archetypes; collective unconscious; persona	Jung	Adler or Freud
Empty chair; unfinished business; "what" and "how"	Perls (Gestalt)	Rogers: Rogers is nondirective and reflective
Unconditional positive regard; nondirective	Rogers	Gestalt
Meaning; logotherapy; paradoxical intention	Frankl	Haley's paradoxical *directives*
Four givens (death, freedom, isolation, meaninglessness)	Yalom	Frankl, who focuses on meaning
Systematic desensitization; reciprocal inhibition; SUDS	Wolpe	Skinner: Skinner is operant (consequences)
Shaping; token economy; schedules of reinforcement	Skinner / operant	Wolpe
Modeling; self-efficacy	Bandura	Skinner
WDEP; quality world; five needs	Glasser / Wubbolding	Ellis
Stress inoculation; self-instructional training	Meichenbaum	Beck
BASIC I.D.	Lazarus	Beck
Ego states (Parent, Adult, Child); strokes; games	Berne (TA)	Freud's id/ego/superego

If you see…	It belongs to…	Not to… (because)
OARS; change talk; rolling with resistance	Miller & Rollnick (MI)	Prochaska & DiClemente
Precontemplation → maintenance	Prochaska & DiClemente	MI or group stages
Miracle question; exceptions; scaling	de Shazer & Berg (SFBT)	Narrative
Externalizing; unique outcomes; re-authoring	White & Epston (narrative)	SFBT
Differentiation; triangles; genogram; cutoff	Bowen	Minuchin
Boundaries; enmeshment; enactment; joining	Minuchin	Bowen or Haley
Directives; ordeal; prescribing the symptom	Haley (strategic)	Minuchin
Circular questioning; positive connotation; invariant prescription	Milan	MRI
Double bind	Bateson / MRI	Milan
Placater, blamer, super-reasonable, irrelevant; sculpting	Satir	Bowen
Four Horsemen	Gottman	Sue Johnson (EFT)
Pursue–withdraw cycle; softening; attachment in couples	Sue Johnson (EFT)	Gottman
Good-enough mother; holding environment	Winnicott	Bowlby
Separation–individuation	Mahler	Erikson
Hierarchy of needs	Maslow	Rogers

The Three-Pass Drill

Repeat these passes until each takes under five minutes per family.

Pass 1 — Term → Owner. Cover the right column of the swap-pair table. Read each term and say the owner out loud. Mark every miss.

Pass 2 — Owner → Terms. Name a theorist. List three key terms and one signature technique without looking. Example: "Minuchin — boundaries, enmeshment, subsystems; technique: enactment."

Pass 3 — Distractor autopsy. After every practice item, right or wrong, name the owner of each wrong option. If you can't, that's your next study target. Write it in the error log: *"Option C — I thought it was Bowen. Actually Satir."*

Pass 3 is the habit that pays off on test day. It trains you to see four owners in every item, not one right answer.

Daily dose: 15 minutes of drills at a desk, plus the matching audio track during any drive or walk.

What the Exam Rewards / What It Punishes

Rewards: - Matching a technique to its theory precisely - Matching the counselor's *role* to the theory (directive vs. nondirective is a frequent discriminator) - Knowing that a theory's goal shapes the intervention (insight vs. behavior change vs. new story)

Punishes: - Picking the option that "sounds therapeutic" rather than the one that fits the named theory - Confusing stage models (Prochaska's stages, Tuckman's group stages, Erikson's stages) - Confusing paired theorists (Ellis/Beck, Bowen/Minuchin, SFBT/narrative)

Quick Checks

Every rationale names the owner of every option. That is the point.

1. A counselor asks a client to rate her anxiety from 0 to 10 and then asks what would move her one point lower. This counselor is most likely practicing: A. Cognitive therapy B. Solution-focused brief therapy C. Systematic desensitization D. Reality therapy

2. Which concept is central to Bowen family systems theory? A. Enmeshment B. Differentiation of self C. Ordeal D. Family sculpting

3. A client says, "I absolutely must get this promotion, or I'm a failure." A counselor using REBT would most likely: A. Reflect the client's feelings without directing B. Dispute the belief that the client *must* get the promotion C. Ask what the client is doing to get what he wants D. Explore early recollections

4. A counselor helps a family see that the parents' subsystem has a diffuse boundary with the oldest child. This counselor's orientation is: A. Structural B. Bowenian C. Strategic D. Experiential

5. "Is what you're doing getting you what you want?" belongs to: A. Ellis B. Glasser/Wubbolding C. Adler D. Meichenbaum

6. Removing a chore when a child completes homework, in order to increase homework completion, is an example of: A. Positive punishment B. Negative punishment C. Negative reinforcement D. Extinction

7. A counselor says to a client describing depression, "How has Depression been trying to convince you to stay home?" This is: A. Externalizing (narrative) B. An exception question (SFBT) C. Reframing (MRI) D. Disputing (REBT)

8. Which counselor stance is most associated with person-centered therapy? A. Directive and didactic B. Neutral

and strategic C. Nondirective, genuine, and accepting D. Confrontive and experimental

9. Sue Johnson's emotionally focused couples therapy primarily targets: A. The Four Horsemen B. The negative interaction cycle and attachment needs C. Communication stances D. Differentiation of self

10. A counselor tells a client with insomnia to try as hard as possible to stay awake. This is most consistent with: A. Frankl's paradoxical intention B. Wolpe's reciprocal inhibition C. Beck's behavioral experiment D. Minuchin's enactment

Answers and Owner Rationales

1 — B. Scaling questions belong to SFBT (de Shazer & Berg). A: Beck uses thought records and Socratic questioning. C: Wolpe uses SUDS ratings, but to build a fear hierarchy, not to ask what would make things better. D: Glasser/Wubbolding use WDEP.

2 — B. Differentiation of self is Bowen's central concept. A: enmeshment is Minuchin (structural). C: ordeal therapy is Haley (strategic). D: family sculpting is Satir (experiential).

3 — B. Disputing "musts" is Ellis's REBT. A: nondirective reflection is Rogers. C: WDEP evaluation is Glasser/Wubbolding. D: early recollections are Adler.

4 — A. Subsystems and boundaries are Minuchin's structural vocabulary. B: Bowen would speak of triangles and differentiation. C: strategic (Haley) speaks of hierarchy and power and uses directives. D: experiential (Satir) speaks of communication stances and self-esteem.

5 — B. The evaluation step of WDEP (Wubbolding's operationalization of Glasser's reality therapy). A: Ellis disputes beliefs. C: Adler's "the Question" asks what would be different if the client were well. D: Meichenbaum works on self-talk.

6 — C. Removing something unpleasant (a chore) to increase a behavior (homework) is negative reinforcement (Skinner/operant). A: positive punishment *adds* something unpleasant to *decrease* behavior. B: negative punishment *removes* something pleasant to *decrease* behavior. D: extinction withholds reinforcement so a behavior fades.

7 — A. Speaking of Depression as a separate entity acting on the client is externalizing (White & Epston). B: an exception question (SFBT) would ask about times depression was absent or weaker. C: reframing (MRI/strategic) gives behavior a new meaning. D: disputing (Ellis) challenges an irrational belief.

8 — C. Rogers' core conditions: genuineness, unconditional positive regard, empathy, delivered nondirectively. A: directive and didactic is Ellis. B: neutral and strategic fits Milan or Haley. D: confrontive and experimental is Perls.

9 — B. EFT (Johnson) de-escalates the negative cycle and restructures attachment bonds. A: the Four Horsemen are Gottman. C: communication stances are Satir. D: differentiation is Bowen.

10 — A. Asking an individual to intend the feared symptom is Frankl's paradoxical intention (logotherapy). B: reciprocal inhibition (Wolpe) pairs relaxation with anxiety stimuli. C: a behavioral experiment (Beck) tests a specific belief with evidence. D: enactment (Minuchin) has family members interact in session.

Audio Companion

This chapter's audio is split into four tracks, grouped by theory family.

Track	Covers	Target length
2.1	Psychodynamic + Humanistic/Existential	~12 min
2.2	Behavioral + Cognitive-Behavioral + change models	~10 min

Track	Covers	Target length
2.3	Postmodern + Family Systems	~10 min
2.4	Swap Pairs Rapid Fire (all families)	~8 min

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Chapter 3 — Ownership Beyond Theory

Domain map: 3.S Developmental processes/tasks/issues · 3.U Occupation and career development · 3.J Gender identity development · 3.F Cultural adjustments · 5.AS Explain phases in the group process · 5.AU Create intervention based on the stage of group development · 6.I Foster the emergence of group therapeutic factors · 2.M Select, use, and interpret appropriate assessment instruments · 1.B Understand statistical concepts and methods in research · CACREP: Human Growth and Development, Career Development, Group Counseling, Assessment and Testing, Research and Program Evaluation

The Same Trap, Six More Times

Chapter 2 showed that NCE theory items are built from real terms owned by different theorists. The same construction runs through the rest of the CACREP recall content. A development item offers four real stage names from four different models. A career item offers four real theorists who all wrote about choosing work. A statistics item offers four real tests that all compare something.

This chapter gives you the ownership tables for six areas:

1. Human growth and development
2. Identity development models
3. Career development
4. Group work
5. Assessment and testing
6. Research and statistics

Each area ends with its swap pairs. The deeper clinical teaching for these topics comes later (Chapters 8, 12, 14, 15, 18, and 20). This chapter is the recall layer: the owners, the terms, and the order of the stages.

One rule for every stage model: learn the stages *in order*, with one anchor word each. NCE items frequently ask which stage comes next, or which stage a described person is in.

Area 1 — Human Growth and Development

The Stage Theorists

Owner	Theory	Stages / key terms (in order)	Anchor idea
Erik Erikson	Psychosocial development	1. Trust vs. mistrust (infancy) · 2. Autonomy vs. shame and doubt (toddler) · 3. Initiative vs. guilt (preschool) · 4. Industry vs. inferiority (school age) · 5. Identity vs. role confusion (adolescence) · 6. Intimacy vs. isolation (young adult) · 7. Generativity vs. stagnation (middle adult) · 8. Integrity vs. despair (late adult)	A psychosocial crisis at each stage across the *whole life span*
Sigmund Freud	Psychosexual development	Oral · anal · phallic (Oedipus/Electra) · latency · genital	Libido focused on body zones; fixation

Owner	Theory	Stages / key terms (in order)	Anchor idea
Jean Piaget	Cognitive development	1. Sensorimotor, 0–2 (object permanence) · 2. Preoperational, 2–7 (egocentrism, animism, centration, no conservation) · 3. Concrete operational, 7–11 (conservation, reversibility, classification) · 4. Formal operational, 11+ (abstract, hypothetical reasoning) · Processes: schema, assimilation, accommodation, equilibration	Children actively construct knowledge in stages
Lev Vygotsky	Sociocultural development	Zone of proximal development (ZPD); scaffolding; more knowledgeable other; private speech	Learning happens socially, just beyond current ability
Lawrence Kohlberg	Moral development	**Preconventional:** 1. Obedience and punishment · 2. Self-interest/ instrumental exchange · **Conventional:** 3. Interpersonal conformity (“good boy/good girl”) · 4. Law and order · **Postconventional:** 5. Social contract · 6. Universal ethical principles · Heinz dilemma	Moral *reasoning*, judged by justice

Owner	Theory	Stages / key terms (in order)	Anchor idea
Carol Gilligan	Moral development (ethic of care)	Orientation to individual survival → goodness as self-sacrifice → morality of nonviolence (care for self and others); *In a Different Voice*	Critique of Kohlberg; care and relationships, not only justice
Margaret Mahler	Separation–individuation	Normal autism → symbiosis → separation–individuation subphases: differentiation, practicing, rapprochement, object constancy	Infant psychologically "hatches" from caregiver
Daniel Levinson	Adult development (*The Seasons of a Man's Life*)	Early adult transition; age-30 transition; midlife transition; "the Dream"; mentor	Alternating stable and transitional periods
Robert Havighurst	Developmental tasks	Tasks at each life stage that must be mastered for later success	Mastery → happiness and success with later tasks
Arnold Gesell	Maturational theory	Development unfolds by biological timetable; developmental norms	Nature over nurture
David Elkind	Adolescent egocentrism	Imaginary audience; personal fable	Adolescents feel watched and invulnerable/ unique

Owner	Theory	Stages / key terms (in order)	Anchor idea
James Marcia	Identity statuses (built on Erikson)	Diffusion (no crisis, no commitment) · Foreclosure (commitment, no crisis) · Moratorium (crisis, no commitment) · Achievement (crisis, then commitment)	Crisis (exploration) × commitment

Attachment, Parenting, and Temperament

Owner	Key terms
John Bowlby	Attachment theory; secure base; internal working models; separation distress
Mary Ainsworth	Strange Situation; secure, anxious-avoidant, anxious-resistant (ambivalent)
Mary Main & Judith Solomon	Added the **disorganized** attachment classification
Harry Harlow	Rhesus monkeys; contact comfort beats food (wire vs. cloth mother)
Diana Baumrind	Parenting styles: authoritative (warm + firm, best outcomes), authoritarian (firm, not warm), permissive (warm, not firm)
Maccoby & Martin	Added the fourth style: neglectful/uninvolved
Thomas & Chess	Temperament: easy, difficult, slow-to-warm-up
Konrad Lorenz	Imprinting; critical period (geese)

Grief, Stress, and Loss

Owner	Key terms
Elisabeth Kübler-Ross	Five stages of dying: denial, anger, bargaining, depression, acceptance (described for terminally ill patients; not a required sequence)
J. William Worden	Four tasks of mourning: accept the reality of the loss · process the pain · adjust to a world without the deceased · find an enduring connection while embarking on a new life
Hans Selye	General Adaptation Syndrome: alarm → resistance → exhaustion; eustress vs. distress
Holmes & Rahe	Social Readjustment Rating Scale (life change units)
Nancy Schlossberg	Transition theory: the 4 S's (situation, self, support, strategies); anticipated, unanticipated, and nonevent transitions

Swap Pairs — Development

If you see…	Owner	Not…
Industry vs. inferiority	Erikson	Piaget (stages named by cognition, not crisis)
Conservation; object permanence	Piaget	Erikson or Vygotsky
Zone of proximal development; scaffolding	Vygotsky	Piaget
Law and order; social contract	Kohlberg	Gilligan
Ethic of care	Gilligan	Kohlberg
Rapprochement	Mahler	Bowlby
Strange Situation	Ainsworth	Bowlby (theory) or Harlow (monkeys)

If you see…	Owner	Not…
Foreclosure; moratorium	Marcia	Erikson (Marcia built on Erikson's identity stage)
Imaginary audience; personal fable	Elkind	Piaget
Authoritative vs. authoritarian	Baumrind	Thomas & Chess (temperament, not parenting)
Alarm, resistance, exhaustion	Selye	Kübler-Ross
Tasks of mourning	Worden	Kübler-Ross (stages of dying)
4 S's of transition	Schlossberg	Super (career stages)

Watch the age trap. Erikson's stage 4 (industry vs. inferiority) and Piaget's concrete operational stage cover about the same school-age years. If the stem describes a child comparing her work to classmates' and feeling incompetent, that's Erikson. If it describes a child who now understands that a tall glass and a wide glass hold the same water, that's Piaget.

Area 2 — Identity Development Models

These models describe how people come to understand a cultural, racial, or sexual identity. They appear in Areas of Clinical Focus (3.F, 3.J, 3.Z) and Core Attributes (6.D, 6.E).

Owner	Model	Stages (in order)
William Cross	Nigrescence (Black racial identity)	Pre-encounter → encounter → immersion–emersion → internalization → internalization–commitment

Owner	Model	Stages (in order)
Janet Helms	White racial identity	Contact → disintegration → reintegration → pseudo-independence → immersion/ emersion → autonomy
Atkinson, Morten, & Sue	Racial/Cultural Identity Development (R/CID)	Conformity → dissonance → resistance and immersion → introspection → integrative awareness
Vivienne Cass	Homosexual identity formation	Identity confusion → comparison → tolerance → acceptance → pride → synthesis
John Berry	Acculturation strategies	Integration (keep heritage + join host) · assimilation (drop heritage, join host) · separation (keep heritage, avoid host) · marginalization (neither)

Swap Pairs — Identity

If you see…	Owner	Not…
Encounter; immersion–emersion	Cross	Helms
Contact; disintegration; reintegration	Helms	Cross
Conformity → integrative awareness	Atkinson, Morten, & Sue	Cross
Identity confusion → synthesis	Cass	Marcia (identity statuses, not sexual identity)
Integration vs. assimilation	Berry	Piaget’s assimilation (a cognitive process)

Watch the word trap: “assimilation” belongs to *Piaget* when it means fitting new information into an existing schema, and to *Berry* when it means adopting the host culture. The stem’s context decides.

Area 3 — Career Development

Owner	Theory	Key terms	Anchor idea
Frank Parsons	Trait-factor	*Choosing a Vocation* (1909); know yourself, know the world of work, use “true reasoning” to match them	Father of vocational guidance
E. G. Williamson	Trait-factor (Minnesota point of view)	Directive counseling; analysis, synthesis, diagnosis, prognosis, counseling, follow-up	Systematized Parsons’ matching
John Holland	Theory of vocational personalities and work environments	RIASEC: Realistic, Investigative, Artistic, Social, Enterprising, Conventional; hexagon; congruence, consistency, differentiation, identity; Self-Directed Search (SDS)	Match personality type to environment type

Owner	Theory	Key terms	Anchor idea
Donald Super	Life-span, life-space	Vocational self-concept; Life-Career Rainbow; life roles; stages: growth (0–14), exploration (15–24), establishment (25–44), maintenance (45–64), disengagement (65+); career maturity; recycling	Career is implementing self-concept across a lifetime
Ginzberg, Ginsburg, Axelrad, & Herma	Developmental	Fantasy (to ~11) → tentative (11–17) → realistic (17+: exploration, crystallization, specification); compromise	Career choice is a developmental process
Anne Roe	Personality/ needs-based	Early parent–child relationships shape orientation toward or away from people; fields and levels of occupations; influenced by Maslow	Childhood needs → occupational choice
John Krumboltz	Social learning theory of career decision making; happenstance learning theory	Genetic endowment, environmental conditions, learning experiences, task-approach skills; planned happenstance	Career choice is learned; unplanned events are opportunities

Owner	Theory	Key terms	Anchor idea
Linda Gottfredson	Circumscription and compromise	Stages: orientation to size and power → sex roles → social valuation → internal, unique self; zone of acceptable alternatives	Children rule out options early by gender and prestige
Tiedeman & O'Hara	Decision-making	Anticipation (exploration, crystallization, choice, clarification) and implementation (induction, reformation, integration)	Career identity forms through repeated decisions
Lent, Brown, & Hackett	Social Cognitive Career Theory (SCCT)	Self-efficacy, outcome expectations, personal goals (built on Bandura)	Beliefs about ability shape interests and choices
Mark Savickas	Career construction theory; life design	Career adaptability: concern, control, curiosity, confidence; career stories	Clients construct careers through narrative
Dawis & Lofquist	Theory of Work Adjustment	Satisfaction (worker) and satisfactoriness (employer); correspondence	Fit between worker and work environment over time
Peterson, Sampson, & Reardon	Cognitive Information Processing (CIP)	Pyramid of information processing; CASVE cycle (Communication, Analysis, Synthesis, Valuing, Execution)	Career decisions as problem-solving

Owner	Theory	Key terms	Anchor idea
Robert Hoppock	Job satisfaction	Needs-based theory of occupational choice and satisfaction	Job satisfaction reflects how well needs are met

Career instruments and resources: Self-Directed Search (Holland), Strong Interest Inventory (Holland codes), Kuder Occupational Interest Survey, Career Maturity Inventory (Crites), Myers-Briggs Type Indicator (based on Jung's types), O*NET (Department of Labor database that replaced the *Dictionary of Occupational Titles*), *Occupational Outlook Handbook*.

Swap Pairs — Career

If you see…	Owner	Not…
RIASEC; hexagon; congruence	Holland	Super
Life-Career Rainbow; self-concept; maintenance stage	Super	Holland or Ginzberg
Fantasy, tentative, realistic	Ginzberg et al.	Super
Planned happenstance; learning experiences	Krumboltz	Savickas
Circumscription and compromise	Gottfredson	Ginzberg (who also used "compromise")
Parent–child relationship shapes career	Roe	Super
Self-efficacy + outcome expectations in career	Lent, Brown, & Hackett (SCCT)	Krumboltz
Career adaptability; career stories	Savickas	Super (who used "career maturity")
CASVE cycle	Peterson, Sampson, & Reardon	Tiedeman & O'Hara

If you see…	Owner	Not…
Know yourself + know the work world + match	Parsons	Williamson (who systematized it)

Area 4 — Group Work

Stage Models

Owner	Stages (in order)	Anchor
Bruce Tuckman (with Mary Ann Jensen, 1977)	Forming → storming → norming → performing → adjourning	Rhyming stages; adjourning added in 1977
Gerald Corey	Pregroup (formation) → initial (orientation, exploration, trust) → transition (anxiety, resistance, conflict, struggle for control) → working (cohesion, productivity, genuine self-disclosure) → final (consolidation, termination) → postgroup (follow-up)	The stage model most counseling texts use

Where the NCE tests this: behaviors matched to stages. Members testing the leader, conflict, and defensiveness: *transition* (Corey) or *storming* (Tuckman). Members expressing genuine thoughts and feelings, taking risks, giving direct feedback: *working* (Corey) or *norming/performing* (Tuckman). Members tentative, polite, and unsure of norms: *initial* or *forming*.

Other Group Owners

Owner	Key terms
Irvin Yalom	11 therapeutic factors: instillation of hope, universality, imparting information, altruism, corrective recapitulation of the primary family group, development of socializing techniques, imitative behavior, interpersonal learning, group cohesiveness, catharsis, existential factors; here-and-now focus; the group as a social microcosm
Kurt Lewin	Field theory; group dynamics; T-groups (National Training Laboratories); leadership styles with Lippitt & White: autocratic, democratic, laissez-faire
J. L. Moreno	Psychodrama (protagonist, director, auxiliary egos, audience, stage); doubling; role reversal; mirror technique; sociometry; credited with popularizing the term “group psychotherapy”
Joseph Pratt	First recognized therapeutic group (1905, tuberculosis patients; “class method”)
Wilfred Bion	Work group vs. basic assumption group: dependency, fight–flight, pairing
S. R. Slavson	Activity group therapy (children)
Luft & Ingham	Johari Window: open, blind, hidden, unknown
ASGW (Association for Specialists in Group Work)	Four group types: task/work, psychoeducational, counseling, psychotherapy; Best Practice Guidelines

Swap Pairs — Group

If you see…	Owner	Not…
Storming; norming	Tuckman	Corey

If you see…	Owner	Not…
Transition stage; working stage	Corey	Tuckman
Contemplation	Prochaska & DiClemente (stages of change)	Corey's group stages (contemplation is never a group stage)
Universality; instillation of hope	Yalom	Lewin
Autocratic, democratic, laissez-faire	Lewin, Lippitt, & White	Yalom
Protagonist; auxiliary ego; role reversal	Moreno (psychodrama)	Perls (Gestalt also uses dramatic techniques, but these terms are Moreno's)
Fight–flight; pairing	Bion	Tuckman
Blind and hidden areas	Luft & Ingham (Johari Window)	Yalom

Area 5 — Assessment and Testing

Intelligence Owners

Owner	Contribution
Francis Galton	Individual differences; early mental measurement; often called the father of psychometrics
Alfred Binet (with Théodore Simon)	First practical intelligence test (1905); mental age
Lewis Terman	Stanford-Binet; popularized the ratio IQ (mental age ÷ chronological age × 100)
David Wechsler	WAIS, WISC, WPPSI; deviation IQ (mean 100, SD 15)
Charles Spearman	*g* (general intelligence); two-factor theory; Spearman–Brown formula; Spearman rho
Louis Thurstone	Primary mental abilities; Thurstone attitude scale

Owner	Contribution
Raymond Cattell	Fluid vs. crystallized intelligence (with John Horn); 16PF personality inventory
Howard Gardner	Multiple intelligences
Robert Sternberg	Triarchic theory: analytical, creative, practical
J. P. Guilford	Structure of intellect; convergent vs. divergent thinking

Personality and Projective Owners

Instrument	Owner
MMPI	Hathaway & McKinley
Myers-Briggs Type Indicator	Isabel Briggs Myers & Katharine Cook Briggs (based on Jung)
16PF	Raymond Cattell
Rorschach Inkblot Test	Hermann Rorschach
Thematic Apperception Test (TAT)	Henry Murray (with Christiana Morgan); needs and press
Beck Depression Inventory	Aaron Beck
Likert scale	Rensis Likert
Semantic differential	Charles Osgood
Guttman scale	Louis Guttman
Social distance scale	Emory Bogardus

Reliability and Validity (concepts, not owners, but swapped constantly)

Term	Asks	Method / example
Test-retest reliability	Is it stable over time?	Same test, same people, two times
Alternate (parallel) forms	Are two versions equivalent?	Two forms, same people
Split-half reliability	Are the two halves consistent?	Correlate halves; correct with Spearman–Brown

Term	Asks	Method / example
Internal consistency	Do the items hang together?	Cronbach's alpha; KR-20 for right/ wrong items
Interrater reliability	Do scorers agree?	Two raters, same responses
Content validity	Does it sample the whole domain?	Expert review against the blueprint
Criterion validity: concurrent	Does it match a current criterion?	Correlate with an established measure now
Criterion validity: predictive	Does it predict a future criterion?	Correlate with later performance
Construct validity	Does it measure the theoretical trait?	Convergent (correlates with similar) and discriminant (doesn't correlate with unrelated); multitrait–multimethod matrix (Campbell & Fiske)
Face validity	Does it *look* like it measures the trait?	Not a technical form of validity

Rule: a test can be reliable without being valid, but it cannot be valid without being reliable. Reliability is necessary but not sufficient.

Scores

Score	Mean	Standard deviation
z-score	0	1
T-score	50	10
Deviation IQ (Wechsler)	100	15
Stanine	5	about 2 (range 1–9)

Normal curve: about 68% of scores fall within ±1 SD, 95% within ±2 SD, 99.7% within ±3 SD. **Standard error of measurement (SEM)** builds a confidence band around one person's score; higher reliability means a smaller SEM.

Norm-referenced compares a person to others. **Criterion-referenced** compares a person to a fixed standard (pass the driving test). **Ipsative** compares a person to themselves (strengths relative to the person's other traits).

Swap Pairs — Assessment

If you see…	Owner / answer	Not…
Deviation IQ	Wechsler	Terman (ratio IQ)
Ratio IQ (MA/CA × 100)	Terman / Stanford-Binet	Wechsler
g factor	Spearman	Thurstone (primary mental abilities)
Fluid vs. crystallized	Cattell (and Horn)	Gardner
Needs and press	Murray (TAT)	Maslow
Items hang together	Internal consistency	Test-retest
Predicts later performance	Predictive validity	Concurrent validity
Correlates with similar measures	Convergent validity	Discriminant validity
Mean 50, SD 10	T-score	Stanine or z

Area 6 — Research and Statistics

Scales of Measurement (S. S. Stevens) — learn as NOIR

Scale	Property	Example
Nominal	Categories only	Diagnosis, gender, jersey numbers
Ordinal	Rank order	Class rank, Likert ratings (usually treated as ordinal)
Interval	Equal intervals, no true zero	Temperature in °F, IQ scores
Ratio	Equal intervals and a true zero	Weight, age, number of sessions

Which Test Owns Which Question

Question	Test
Do two group means differ?	t-test (William Gosset, “Student”)
Do three or more group means differ?	ANOVA (Ronald Fisher; F statistic)
Do group means differ after controlling for a covariate?	ANCOVA
Do groups differ on two or more dependent variables at once?	MANOVA
Do observed frequencies differ from expected (nominal data)?	Chi-square (Karl Pearson)
How strongly do two interval/ratio variables relate?	Pearson r
How strongly do two ranked variables relate?	Spearman rho
Nonparametric version of the two-group t-test	Mann–Whitney U
Nonparametric version of one-way ANOVA	Kruskal–Wallis

Errors and Significance

Term	Meaning
Type I error (alpha)	Rejecting a true null hypothesis: a false positive. “Finding” an effect that isn’t there
Type II error (beta)	Failing to reject a false null hypothesis: a false negative. Missing a real effect
Power	$1 - \beta$: the probability of detecting a real effect. Increased by larger samples
Effect size	How large the difference is (e.g., Cohen’s d), independent of significance
Skew	Positive skew: tail to the right; mean > median > mode. Negative skew: tail to the left; mean < median < mode

Design Owners

Owner	Contribution
Campbell & Stanley	Threats to internal validity: history, maturation, testing, instrumentation, statistical regression, selection, experimental mortality (attrition), selection–maturation interaction; experimental vs. quasi-experimental designs
Richard Solomon	Solomon four-group design (controls for pretest sensitization)
Elton Mayo (Western Electric studies)	Hawthorne effect: behavior changes because participants know they're being studied
Robert Rosenthal	Experimenter expectancy effect (Pygmalion/Rosenthal effect)
Glaser & Strauss	Grounded theory (qualitative)
Lincoln & Guba	Qualitative trustworthiness: credibility, transferability, dependability, confirmability

Design anchors: a *true experiment* requires random assignment. A *quasi-experiment* lacks it. *Single-subject* designs use baselines (A) and interventions (B): AB, ABA, ABAB. *Formative* evaluation improves a program while it runs; *summative* evaluation judges it at the end.

Swap Pairs — Research

If you see…	Answer	Not…
Two groups, compare means	t-test	ANOVA
Nominal frequencies	Chi-square	Pearson r
Rejecting a true null	Type I	Type II
Random assignment present	True experiment	Quasi-experiment
Participants improve because they're observed	Hawthorne effect	Placebo effect

If you see…	Answer	Not…
Extreme scorers drift toward the mean on retest	Statistical regression	Maturation
Participants drop out unevenly	Mortality/attrition	Selection
Improve a program in progress	Formative evaluation	Summative evaluation

The Master "Who Owns It?" Drill

At the end of each week, run this mixed list from Chapters 2 and 3. Cover the right column and answer out loud.

Term	Owner
Social interest	Adler
Zone of proximal development	Vygotsky
RIASEC	Holland
Storming	Tuckman
Universality	Yalom
Differentiation of self	Bowen
Deviation IQ	Wechsler
Ethic of care	Gilligan
Planned happenstance	Krumboltz
Psychodrama	Moreno
Generativity vs. stagnation	Erikson
Life-Career Rainbow	Super
Pre-encounter	Cross
Disorganized attachment	Main & Solomon
Circumscription and compromise	Gottfredson
Threats to internal validity	Campbell & Stanley
Tasks of mourning	Worden
Fight–flight (group)	Bion
Fluid intelligence	Cattell

Term	Owner
Integration (acculturation)	Berry

What the Exam Rewards / What It Punishes

Rewards: - Stage order and the anchor behavior for each stage - Picking the precise owner when two theorists wrote about the same topic - Choosing the correct statistical test from the type of data and number of groups

Punishes: - Mixing stage models (stages of change, group stages, Erikson, Super) - Treating a near-synonym as a match (“exception” vs. “unique outcome”; “assimilation” in Piaget vs. Berry) - Confusing reliability with validity

Quick Checks

1. A 9-year-old becomes discouraged and says he’s “bad at everything” after comparing his schoolwork to classmates’. This reflects which Eriksonian stage? A. Initiative vs. guilt B. Industry vs. inferiority C. Identity vs. role confusion D. Concrete operational

2. A client who adopts a career identity chosen by her parents without exploring alternatives is in which identity status? A. Diffusion B. Moratorium C. Foreclosure D. Achievement

3. A counselor helps a client see that an unexpected layoff could open new career possibilities. This most reflects: A. Holland’s congruence B. Super’s maintenance stage C. Krumboltz’s planned happenstance D. Gottfredson’s circumscription

4. Group members begin challenging the leader and each other, and some express doubt about the group’s value. According to Corey, the group is most likely in the: A. Initial stage B. Transition stage C. Working stage D. Contemplation stage

5. A new member says, “I thought I was the only one who felt this way.” Yalom would call this: A. Catharsis B. Altruism C. Universality D. Imitative behavior

6. A researcher wants to compare depression scores across four treatment groups. The most appropriate test is: A. Independent t-test B. Chi-square C. Pearson r D. One-way ANOVA

7. A test produces nearly identical scores when the same clients take it two weeks apart. This demonstrates: A. Test-retest reliability B. Predictive validity C. Internal consistency D. Content validity

8. A T-score of 60 is how many standard deviations above the mean? A. 0.5 B. 1 C. 1.5 D. 2

9. An immigrant client maintains strong ties to her heritage culture while also participating fully in her new community. Berry would describe this as: A. Assimilation B. Separation C. Integration D. Marginalization

10. A researcher concludes a treatment works when, in reality, it does not. This is a: A. Type I error B. Type II error C. Lack of power D. Regression effect

11. Which theorist proposed the Life-Career Rainbow? A. Holland B. Super C. Ginzberg D. Roe

12. “Self-efficacy, outcome expectations, and personal goals” are core constructs of: A. Social Cognitive Career Theory B. Trait-factor theory C. Theory of Work Adjustment D. Career construction theory

Answers and Owner Rationales

1 — B. Erikson’s stage 4, industry vs. inferiority, covers school-age competence. A: initiative vs. guilt is Erikson’s preschool stage. C: identity vs. role confusion is Erikson’s adolescent stage. D: concrete operational is Piaget’s cognitive stage, not a psychosocial crisis.

2 — C. Commitment without exploration is Marcia’s foreclosure. A: diffusion has neither exploration nor commitment. B: moratorium is active exploration without com-

mitment. D: achievement is commitment after exploration.

3 — C. Turning unplanned events into opportunities is Krumboltz's planned happenstance. A: congruence is Holland's person–environment fit. B: maintenance is Super's stage of holding one's position in midcareer. D: circumscription is Gottfredson's process of ruling out options in childhood.

4 — B. Conflict, resistance, and challenges to the leader define Corey's transition stage (Tuckman's storming). A: the initial stage is tentative and polite. C: the working stage is cohesive and productive. D: contemplation is a stage of change (Prochaska & DiClemente), not a group stage.

5 — C. Realizing one is not alone is Yalom's universality. A: catharsis is emotional release. B: altruism is benefiting from helping others. D: imitative behavior is learning by watching members or the leader.

6 — D. Three or more group means: one-way ANOVA (Fisher). A: a t-test compares two means. B: chi-square compares frequencies of nominal data. C: Pearson r measures the relationship between two continuous variables.

7 — A. Stability over time is test-retest reliability. B: predictive validity is about forecasting a future criterion. C: internal consistency is about items agreeing with one another in one administration. D: content validity is about covering the domain.

8 — B. T-scores have a mean of 50 and an SD of 10; 60 is one SD above the mean. A, C, and D would be T-scores of 55, 65, and 70.

9 — C. Keeping heritage while joining the host culture is Berry's integration. A: assimilation gives up heritage. B: separation keeps heritage and avoids the host culture. D: marginalization connects with neither.

10 — A. A false positive, rejecting a true null, is a Type I error. B: a Type II error misses a real effect. C: low power

raises the risk of Type II, not Type I, error. D: regression is a threat to internal validity (Campbell & Stanley), not an error type.

11 — B. The Life-Career Rainbow is Super's. A: Holland owns RIASEC. C: Ginzberg owns fantasy–tentative–realistic. D: Roe owns parent–child relationships as career influences.

12 — A. SCCT (Lent, Brown, & Hackett) is built on Bandura's self-efficacy. B: trait-factor (Parsons, Williamson) matches traits to jobs. C: the Theory of Work Adjustment (Dawis & Lofquist) is about satisfaction and satisfactoriness. D: career construction (Savickas) is about adaptability and life stories.

Audio Companion

Track	Covers	Target length
3.1	Development + identity models	~5 min
3.2	Career theorists	~3 min
3.3	Group stages and group owners	~2 min
3.4	Assessment and research rapid fire	~4 min

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PART TWO

The Six Domains

Chapter 4 — The Alliance and the Microskills

Domain map: 5.E Establish therapeutic alliance · 5.S Facilitate trust and safety · 5.AA Summarize · 5.AB Reframe/redirect · 5.AC Facilitate empathic responses · 5.AD Use self-disclosure · 5.AE Use constructive confrontation · 5.AF Facilitate awareness of here-and-now interactions · 5.AL Contain and manage intense feelings · 6.M Use foundational listening, attending, and reflecting skills · 1.P Monitor the therapeutic relationship and build trust · CACREP: Counseling and Helping Relationships

Why This Chapter Matters

Counseling Skills and Interventions is the largest domain on the NCE: 48 of the 160 scored questions. A large share of those questions ask one thing in different clothes: **what should the counselor say or do next?**

The answer choices are usually all real skills. One is a reflection, one is a question, one is advice, one is a confrontation. The skill you need is the same one Chapter 2 taught for theories: **name what each option is.** Once you can label every option ("that's a paraphrase," "that's premature confrontation," "that's advice-giving"), the right answer is the one that fits the moment in the session.

This chapter teaches you to label options, and then to pick the one the moment calls for.

Who Owns the Skills Vocabulary

The skills have owners too. NCE items may name these owners outright.

Owner	Contribution	Key terms
Carl Rogers	Core conditions	Congruence (genuineness), unconditional positive regard, accurate empathic understanding
Edward Bordin	Working alliance	Three parts: agreement on **goals**, agreement on **tasks**, and the **bond**
Allen Ivey (with Mary Bradford Ivey & Carlos Zalaquett)	Microskills hierarchy; intentional interviewing	Attending behavior → basic listening sequence → influencing skills; the five-stage interview
Gerard Egan	*The Skilled Helper*	**SOLER** attending posture; three-stage model (current picture → preferred picture → the way forward)
Robert Carkhuff	Empathy rating scale	Five levels: 1–2 subtract from the client's message, **3 is interchangeable** (minimally facilitative), 4–5 add to it
Clara Hill	Helping skills model	Three stages: **exploration → insight → action**
Safran & Muran	Alliance ruptures and repair	Withdrawal ruptures and confrontation ruptures; repair by addressing the rupture openly

SOLER (Egan)

Letter	Behavior
S	Face the client **Squarely**
O	**Open** posture (arms and legs uncrossed)
L	**Lean** slightly toward the client

Letter	Behavior
E	Appropriate **Eye** contact
R	Stay **Relaxed**

Cultural caution: SOLER describes a Western norm. Sustained eye contact, direct facing, and close seating can read as disrespectful or aggressive in some cultures. When an item mentions a client's cultural background and nonverbal behavior, the correct answer usually adapts attending to the client rather than applying SOLER rigidly.

The Microskills Hierarchy: Attend → Listen → Influence

Ivey's hierarchy is a sequence, not a menu. You earn the right to influence by attending and listening first.

Level	Skills	What they do
Attending	Eye contact, body language, vocal tone, verbal tracking (staying on the client's topic), comfortable silence	Shows the client you are present
Listening (the basic listening sequence)	Open and closed questions, minimal encouragers, paraphrasing, reflection of feeling, summarizing	Shows the client you understand
Influencing	Reflection of meaning, confrontation, interpretation/ reframe, self-disclosure, immediacy, feedback, information-giving, directives	Helps the client see or do something new

The NCE's cardinal rule: validate before you challenge. When a client expresses distress, the best first re-

sponse reflects the feeling. When a client resists, the best first response acknowledges the resistance rather than arguing with it. When a client disagrees with the counselor, the best first response explores the client's view rather than defending the counselor's.

Labeling the Options: A Skill-by-Skill Guide

Use one client statement to see every skill side by side.

> **Client:** "My sister keeps calling to tell me how to raise my kids. I just let it go to voicemail now. I don't even want to deal with her anymore."

Skill	What it does	Example response
Minimal encourager	Signals attention, keeps the client talking	"Mm-hmm… go on."
Open question	Invites elaboration; usually begins with *what* or *how*	"What happens for you when you see her name on your phone?"
Closed question	Gathers a specific fact; answerable in a word	"How often does she call?"
Paraphrase	Restates the *content* in fresh words	"Your sister's been calling with parenting advice, and you've started letting it go to voicemail."
Reflection of feeling	Names the *emotion* under the content	"You sound worn out and pretty frustrated with her."
Reflection of meaning	Names the value or belief underneath	"Being trusted to raise your own kids matters a lot to you."
Summary	Ties together several statements or themes	"So far you've talked about your sister's calls, feeling second-guessed as a parent, and wanting some distance."

Skill	What it does	Example response
Clarification	Checks an unclear message	"When you say you don't want to deal with her, do you mean right now, or for good?"
Reframe	Offers a new meaning for the same facts	"It sounds like she's worried about the kids, even if the way she shows it feels like criticism."
Confrontation	Points out a discrepancy, gently	"You say you don't care what she thinks, and you've also told me her calls ruin your whole day."
Interpretation	Proposes a connection outside current awareness	"I wonder if her advice echoes how your mom used to correct you."
Self-disclosure	Shares something about the counselor	"I've had family members weigh in on my choices too, and it can sting."
Immediacy	Addresses what is happening *between counselor and client* right now	"I notice you looked away when I asked about your sister. What's happening for you right now, here with me?"
Advice	Tells the client what to do	"You should tell her to stop calling."

Advice is almost never the right answer on the NCE. It shifts responsibility from client to counselor and skips exploration. The exception is psychoeducation or information the client needs for safety or an informed decision. That is information-giving, not advice.

The Most-Confused Pairs

Pair	Tell them apart by...
Paraphrase vs. reflection of feeling	Paraphrase restates **facts**. Reflection names an **emotion**.

Pair	Tell them apart by…
Reflection of feeling vs. reflection of meaning	Feeling names **what they feel**. Meaning names **why it matters** to them.
Paraphrase vs. summary	Paraphrase covers **one statement**. Summary ties together **several**.
Reframe vs. interpretation	A reframe offers a **new angle on the same facts**. An interpretation proposes a **connection the client hasn't made** (often to the past).
Confrontation vs. immediacy	Confrontation points out a **discrepancy**. Immediacy talks about the **counselor–client relationship right now**.
Immediacy vs. here-and-now (Gestalt)	Immediacy is a **skill** about the relationship. Here-and-now is a **theoretical focus** (Perls) on present experience generally.
Self-disclosure vs. immediacy	Self-disclosure shares the **counselor's life or experience**. Immediacy shares the counselor's **reaction to this client, in this moment**.
Clarification vs. paraphrase	Clarification **asks** because the meaning is unclear. Paraphrase **states** a meaning the counselor already understood.

Empathy, Measured: Carkhuff's Five Levels

NCE items sometimes ask which response is "most empathic" or "interchangeable." Carkhuff's scale gives you the yardstick.

> **Client:** "I got passed over for the promotion again. I've worked there eleven years."

Level	Description	Example
1	Ignores or hurts; irrelevant	"Have you updated your résumé lately?"

Level	Description	Example
2	Subtracts; misses the feeling or gets it partly wrong	"Promotions can be political."
3	**Interchangeable**: captures the same content and feeling the client expressed	"After eleven years, being passed over again feels really discouraging."
4	Adds: reaches a feeling or meaning just below the surface	"After eleven years, being passed over again sounds discouraging, and maybe like you're starting to wonder whether your work is seen at all."
5	Significantly adds, and points toward action or deeper understanding	Level 4 plus an invitation to explore what the client wants next

Level 3 is the minimum facilitative response. On the NCE, a level-3 or level-4 reflection usually beats a question, advice, or reassurance, especially early in counseling or right after an emotional disclosure.

When Influencing Skills Are Right

Skill	Use it when…	Don't use it when…
Confrontation	The alliance is established; a real discrepancy exists (words vs. words, words vs. actions, words vs. nonverbals); the discrepancy is keeping the client stuck	It's the first session; the client is in acute distress; you'd be confronting to win a point

Skill	Use it when…	Don't use it when…
Interpretation	You've explored enough to have evidence; the client seems ready; you offer it tentatively ("I wonder if…")	Early in counseling; as a pronouncement
Self-disclosure	It's brief, relevant, for the client's benefit, and you return focus to the client right away	It shifts attention to the counselor, is lengthy, or meets the counselor's needs
Immediacy	Something in the counseling relationship matters more right now than the topic: the client is repeating a relational pattern with you, withdrawing, or there's tension between you	Nothing is happening in the relationship worth naming
Reframe	The client is stuck in one meaning that keeps the problem going, and an alternative meaning is believable to them	The reframe would minimize real harm (never reframe abuse as care)

Self-disclosure test: after the disclosure, whose experience is the session about? If the answer is the counselor's, it failed.

Questions: Open, Closed, and the "Why" Problem

Open questions are the default. They invite the client to lead.

Closed questions are the right tool for specific facts, and they are *required* in risk assessment. "Are you thinking about killing yourself?" is a closed question, and asking it

directly is correct practice. Asking about suicide does not plant the idea.

Avoid "why" questions. "Why did you do that?" asks the client to justify, which invites defensiveness. Convert it to *what* or *how*: "What was going on for you right before that?"

Avoid question strings. Asking three questions in a row without letting the client answer turns a session into an interrogation.

The Alliance: Building, Monitoring, Repairing

Bordin's working alliance has three parts, and an NCE item about a struggling alliance is usually asking which part broke:

Part	Breakdown looks like…	Repair
Goals	Client and counselor want different outcomes ("I'm here because my wife made me come")	Explore the client's own goals; find shared ground
Tasks	Client doesn't see the point of the work ("These worksheets are useless")	Explain the rationale, collaborate on methods, adjust
Bond	Distrust, distance, conflict with the counselor	Empathy, reliability, naming and repairing ruptures

Ruptures (Safran & Muran)

- **Withdrawal rupture:** the client goes quiet, agrees too easily, changes the subject, or misses sessions.
- **Confrontation rupture:** the client openly complains about the counselor or counseling.

The NCE answer for a rupture is almost always to name it non-defensively and explore it ("I noticed you

got quiet after I suggested homework. I'd like to hear how that landed for you."). Wrong answers defend the counselor, ignore the rupture, refer the client out, or press on with the plan.

A repaired rupture often *strengthens* the alliance.

Resistance and Reluctance

Treat resistance as information, not opposition. The scored response acknowledges it and explores it ("It sounds like part of you isn't sure counseling will help"). This mirrors motivational interviewing's "rolling with resistance" (Miller & Rollnick): don't argue, don't push harder.

Mandated clients: acknowledge the circumstances openly, clarify what will and won't be shared with the referral source (informed consent), and look for a goal the client actually wants.

Containing Intense Feelings (5.AL)

When a client becomes overwhelmed in session (sobbing, panic, rage, dissociation):

1. **Stay present and calm.** Your regulation helps theirs.
2. **Slow down.** Lower your voice, slow your pace, allow silence.
3. **Ground if needed.** Feet on the floor, breathing, naming things in the room, orienting to the present.
4. **Reflect without amplifying.** Name the feeling simply; don't push for more detail in the moment.
5. **Assess safety** if the intensity includes any sign of risk.
6. **Close the session with the client stabilized.** Don't open new painful material in the last minutes; leave time to contain and plan.

What the exam punishes: reassurance that dismisses the feeling ("It's going to be fine"), changing the subject, ending the session abruptly, or pushing for trauma detail while the client is flooded.

Silence

Silence after a significant disclosure gives the client room. Filling it with reassurance or a new question often reflects the counselor's discomfort. Comfort with silence varies across cultures, so calibrate it rather than applying a rule.

The Stages of the Counseling Relationship

Stage	Main task	Skills the exam expects
Beginning	Build rapport and trust; informed consent; explore the concern	Attending, open questions, reflections; no confrontation
Middle (working)	Deepen understanding; work toward goals	Listening plus influencing skills; immediacy; confrontation once the alliance is strong
Ending	Consolidate gains; prepare for termination	Summarizing progress; relapse prevention; processing feelings about ending; referral if needed

Termination is introduced *before* the final session and discussed openly, including the client's feelings about ending.

What the Exam Rewards / What It Punishes

Rewards: - Reflecting feeling after an emotional disclosure - Open, *what*/*how* questions over *why* - Naming and exploring ruptures and resistance - Brief, client-focused self-disclosure, if any - Adapting attending behavior to culture - Direct, closed questions about suicide when risk is present

Punishes: - Advice, reassurance, and premature problem-solving - Confrontation or interpretation before the

alliance is established - Defending the counselor when the client is upset with counseling - Self-disclosure that shifts the focus to the counselor - Pushing for detail when a client is flooded

Swap-Pair Drill

Cover the right column and label each response.

Counselor response	Skill
“You’ve been juggling work, your dad’s appointments, and the kids’ schedules, and you said you haven’t slept well in weeks.”	Summary
“You sound exhausted.”	Reflection of feeling
“Help me understand what you mean by ‘done.’”	Clarification
“You said the move doesn’t bother you, and you’ve cried twice talking about it today.”	Confrontation
“Right now, as you tell me this, I notice you’re apologizing to me a lot.”	Immediacy
“Maybe her checking in so often is her way of showing she’s scared of losing you.”	Reframe
“I wonder if being ignored by your boss stirs up how invisible you felt as a kid.”	Interpretation
“So your manager moved the deadline up without asking you.”	Paraphrase
“Being treated fairly is really important to you.”	Reflection of meaning
“You should just tell him how you feel.”	Advice

Quick Checks

Rationales label every option.

1. In a first session, a client says, “I don’t even know why I’m here. Nothing’s going to change.” The best response is: A. “Counseling has a high success rate, so try to stay positive.” B. “It sounds like you’re feeling pretty hopeless that anything can be different.” C. “Why did you make the appointment, then?” D. “You’re saying nothing will change, but you came today anyway.”

2. Which response best represents a level-3 (interchangeable) empathic response to: “My dog died last week, and the house is so quiet now”? A. “Have you thought about getting another dog?” B. “Losing a pet is hard for everyone.” C. “The house feels empty without him, and you’re really missing him.” D. “Grief has five stages, and you’re probably in denial.”

3. A client who has been engaged for six sessions arrives late, answers in single words, and says, “Whatever you think is fine” to every suggestion. The best counselor response is: A. Continue with the planned intervention B. Refer the client to a different counselor C. Note the change and explore it: “Today feels different between us. I’m wondering what’s going on for you.” D. Remind the client that being late violates the agreement

4. A counselor says, “Right now, I’m noticing that every time I ask about your marriage, you make a joke.” This is: A. Interpretation B. Immediacy C. Reframe D. Reflection of meaning

5. Which self-disclosure is most appropriate? A. A detailed account of the counselor’s own divorce B. “I’ve been through something similar, and I remember how lonely it felt. What’s it been like for you?” C. Sharing the counselor’s current stress about work to show the client they aren’t alone D. Telling the client about another client with the same problem

6. According to Bordin, a client who says, “I don’t see how talking about my childhood will help my anxiety” is signaling a problem with which part of the alliance? A. Bond B. Goals C. Tasks D. Transference

7. A client becomes tearful and begins breathing rapidly while describing a car accident. The counselor's best first action is to: A. Ask the client to describe the accident in more detail B. Help the client slow down and ground in the present C. End the session early D. Interpret the reaction as unresolved trauma

8. Which question is least likely to produce defensiveness? A. "Why didn't you take your medication?" B. "Why do you keep going back to him?" C. "What gets in the way of taking your medication?" D. "Don't you think you should leave him?"

9. "SOLER" is associated with: A. Ivey B. Egan C. Carkhuff D. Hill

10. A counselor tells a client, "It sounds like your son's arguing might also be his way of practicing independence." This is: A. Confrontation B. Reframe C. Paraphrase D. Advice

11. Hill's helping skills model proceeds in which order? A. Insight → exploration → action B. Exploration → action → insight C. Exploration → insight → action D. Action → insight → exploration

12. A mandated client says, "The judge sent me. I don't have a problem." The best response is: A. "If you don't participate, I'll have to report that you're noncompliant." B. "Everybody who comes here says that at first." C. "Being required to come here when you don't think you need it sounds frustrating. What would make this time worth something to you?" D. "Let's start with your drinking history."

Answers and Rationales

1 — B. A reflection of feeling meets hopelessness with understanding, which is the right first move in a first session. A: reassurance that dismisses the feeling. C: a "why" question that invites defensiveness. D: a confrontation, premature in a first session.

2 — C. It captures both the content (the empty house) and the feeling (missing him) without adding or subtracting: interchangeable. A: problem-solving advice (level 1). B: a generalization that loses the client's specific loss (level 2). D: a misused stage model and an interpretation, which is hurtful (level 1). Kübler-Ross described dying patients, not a required sequence for grief.

3 — C. A withdrawal rupture (Safran & Muran) is best named and explored non-defensively, using immediacy. A: ignores the rupture. B: referral abandons the alliance instead of repairing it. D: defends the counselor's rules and escalates the rupture.

4 — B. Commenting on what is happening between counselor and client in the moment is immediacy. A: interpretation would propose *why* (e.g., a link to the client's past). C: a reframe would offer a new meaning. D: reflection of meaning would name an underlying value.

5 — B. Brief, relevant, and immediately returns focus to the client. A: lengthy and counselor-centered. C: meets the counselor's needs and burdens the client. D: a confidentiality violation, not self-disclosure.

6 — C. Disagreement about the *methods* of counseling is a tasks problem. A: bond is the emotional connection and trust. B: goals are the desired outcomes (the client still wants less anxiety). D: transference is a psychodynamic concept, not part of Bordin's model.

7 — B. Containment and grounding come first when a client is flooded. A: pushing for detail increases dysregulation. C: ending abruptly leaves the client unstabilized. D: interpreting in the moment skips containment.

8 — C. A "what" question invites exploration. A and B: "why" questions invite justification. D: a leading question that is really advice.

9 — B. SOLER is Egan's (*The Skilled Helper*). A: Ivey owns the microskills hierarchy. C: Carkhuff owns the five-

level empathy scale. D: Hill owns the exploration–insight–action model.

10 — B. A new, plausible meaning for the same behavior is a reframe. A: confrontation would point out a discrepancy. C: a paraphrase would restate the content without new meaning. D: advice would tell the client what to do.

11 — C. Hill's model moves from exploration to insight to action. The other orders scramble the sequence.

12 — C. Acknowledges the mandate and the feeling, then looks for a goal the client owns. A: a threat that damages the alliance (and the reporting terms belong in informed consent, not a threat). B: dismisses the client's experience. D: jumps to assessment content before engagement.

Audio Companion

Track	Covers	Target length
4.1	Name That Skill: label the counselor's response	~10 min

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Chapter 5 — Theory into Intervention

Domain map: 5.A Align intervention with client's developmental level · 5.B Align intervention with counseling modality · 5.D Implement individual counseling in relation to a plan of treatment · 5.F Apply theory-based counseling interventions · 5.M Guide clients in developing skills or strategies · 5.O Help facilitate clients' motivation to make desired changes · 5.R Educate client about transference and defense mechanisms · 5.T Build communication skills · 5.U Develop conflict resolution strategies · 5.Y Provide education resources · 5.Z Provide psychoeducation · CACREP: Counseling and Helping Relationships; Human Growth and Development

From "Who Owns It" to "When Do I Use It"

Chapter 2 taught you to name the theory behind a technique. This chapter asks the next question the NCE asks: **given this client, which intervention fits?**

Most applied intervention items turn on one of four matches:

1. **Problem → approach.** Which intervention has the best support for this presenting concern?
2. **Development → approach.** Is the intervention right for this client's age and cognitive level?
3. **Modality → approach.** Is this individual, couple, family, or group work, and does the intervention fit that format?
4. **Readiness → approach.** Is the client ready for action, or still deciding whether to change?

When two options are both real interventions, the one that matches all four usually wins.

Match 1 — Problem → Approach

The NCE rewards knowing which approaches have the strongest research support for common concerns. Each one has an owner, so the ownership habit still applies.

Presenting concern	Strongly supported approaches (owner)	Key mechanism
Specific phobia	Exposure therapy; systematic desensitization (Wolpe)	Graded contact with the feared stimulus until anxiety drops
Panic	CBT for panic with interoceptive exposure	Exposure to feared body sensations; correcting catastrophic misinterpretation
Obsessive-compulsive symptoms	Exposure and response prevention (ERP)	Face the obsession *without* doing the compulsion
Generalized worry	CBT; worry exposure; applied relaxation	Test worry beliefs; tolerate uncertainty
Trauma (adults)	Prolonged exposure (Edna Foa); cognitive processing therapy (Patricia Resick); EMDR (Francine Shapiro)	Process the memory; change trauma-related beliefs
Trauma (children/ adolescents)	Trauma-focused CBT (Cohen, Mannarino, & Deblinger)	Includes caregiver; graduated trauma narrative; the PRACTICE components
Depression	CBT (Beck); behavioral activation (Lewinsohn; Neil Jacobson); interpersonal therapy (Klerman & Weissman)	Change thoughts, increase rewarding activity, or address role disputes, transitions, grief, interpersonal deficits
Chronic self-harm, emotion dysregulation	Dialectical behavior therapy (Linehan)	Skills: mindfulness, distress tolerance, emotion regulation, interpersonal effectiveness

Presenting concern	Strongly supported approaches (owner)	Key mechanism
Substance use	Motivational interviewing (Miller & Rollnick); CBT/ relapse prevention (Marlatt); contingency management; 12-step facilitation	Resolve ambivalence; cope with triggers; reinforce abstinence
Insomnia	CBT for insomnia (CBT-I)	Stimulus control, sleep restriction, sleep hygiene, cognitive work
Adolescent anorexia	Family-based treatment ("Maudsley approach"; Lock & Le Grange)	Parents temporarily take charge of refeeding
Young child behavior problems	Parent management training; Parent–Child Interaction Therapy (PCIT; Sheila Eyberg)	Coach parents in real time; consistent consequences
Couple distress	Emotionally focused therapy (Sue Johnson); Gottman Method	Attachment and the negative cycle; friendship and conflict management
Grief (uncomplicated)	Supportive counseling; meaning-making; Worden's tasks	Normalize, don't pathologize

The exam's trap: an option that is a *real* therapy but the *wrong match*. Psychoanalysis for a specific phobia. Insight work alone for OCD. Talk therapy without exposure for panic. When a strongly supported approach is among the options for one of the concerns above, it usually wins.

Exposure, Compared

Technique	Owner / family	How it works
Systematic desensitization	Wolpe	Relaxation + *imagined*, gradual steps up a hierarchy
Graded (in vivo) exposure	Behavioral	*Real-life* contact, step by step
Flooding	Behavioral	Immediate, intense, prolonged exposure (real life)
Implosive therapy	Stampfl	Intense *imagined* exposure, often exaggerated
ERP	Behavioral (OCD)	Exposure *plus* blocking the compulsion
Interoceptive exposure	CBT for panic	Bringing on feared body sensations (spinning, breathing through a straw)

Match 2 — Development → Approach (5.A)

The same goal needs a different intervention at different ages.

Children

Children communicate through play and concrete experience more than through talk.

Approach	Owner	Key idea
Child-centered play therapy	Virginia Axline; Garry Landreth	Rogers' core conditions applied to play; "toys are the child's words and play is the child's language" (Landreth's principle)
Filial therapy	Bernard & Louise Guerney	Parents are trained to conduct child-centered play sessions at home

Approach	Owner	Key idea
Sand tray / sandplay	Margaret Lowenfeld (world technique); Dora Kalff (Jungian sandplay)	Child builds scenes in sand with miniatures
Bibliotherapy	(general)	Stories that mirror the child's situation
Art and expressive therapies	(general)	Drawing, puppets, and play as communication
Behavioral charts, token systems	Skinner/operant	Concrete, immediate reinforcement

Rules for children: use concrete language, short sessions, and activity-based methods. Involve parents or caregivers. Remember Piaget: a preoperational child cannot do abstract cognitive restructuring, and a concrete-operational child does better with concrete examples than hypotheticals.

Adolescents

- Respect the drive for autonomy (Erikson: identity vs. role confusion).
- Clarify confidentiality and its limits with both teen and parent at the start.
- Collaborative, less directive approaches tend to engage better than lectures.
- Peer influence matters; group work is often effective.

Older Adults

Approach	Owner	Key idea
Life review	Robert Butler	Structured review of one's life to resolve conflicts and find meaning
Reminiscence therapy	(general)	Recalling positive memories to support mood and connection
Integrity vs. despair	Erikson	Developmental task framing late-life work

Rules for older adults: screen for cognitive changes and medical contributors; adjust pace, print size, and hearing accommodations; don't assume decline or that change isn't possible.

Intellectual Disability and Cognitive Limits

Use simpler language, concrete and visual materials, repetition, behavioral approaches, and involvement of support people. Assess capacity for informed consent (1.O).

Match 3 — Modality → Approach (5.B)

Modality	Who is the client?	What changes
Individual	One person	Intrapersonal focus; confidentiality with one person
Couple	The relationship	Both partners present; secrets policy must be set at the start; neutrality between partners
Family	The family system	Symptoms seen as serving a function; circular rather than linear causality; the identified patient (IP) is not "the problem"
Group	Each member, *and* the group as a whole	Confidentiality can't be guaranteed; group dynamics and therapeutic factors become tools (Chapter 8)

Key modality rules: - **Couples work is contraindicated when there is active intimate partner violence** with coercive control. Assess safety individually first. - In family work, the counselor treats the system, so blaming one member (the IP) is a wrong answer. - A **no-secrets policy** in couple and family work should be explained during informed consent, before it's needed.

Match 4 — Readiness → Approach (5.O)

Motivation is the match with the most moving parts. Two owners run this area. Keep them separate:

- **Prochaska & DiClemente** (Transtheoretical Model) describe *where the client is*.
- **Miller & Rollnick** (motivational interviewing) describe *how to talk with the client* about change.

Stage-Matched Interventions

Stage	Client sounds like…	Best counselor move	Wrong move
Precontemplation	"I don't have a problem."	Build rapport; raise awareness gently; provide information; explore what others are concerned about	Action plans; confrontation; arguing
Contemplation	"Maybe I drink too much, but it helps me relax."	Explore ambivalence; decisional balance (pros and cons of changing and not changing); develop discrepancy	Pushing for a quit date
Preparation	"I'm going to cut back starting next month."	Build a specific plan; set goals; strengthen commitment	Re-exploring whether to change
Action	"I haven't had a drink in three weeks."	Support new behaviors; skills; reinforcement; coping with triggers	Ignoring relapse risk

Stage	Client sounds like…	Best counselor move	Wrong move
Maintenance	"It's been eight months."	Relapse prevention; lifestyle change; identify high-risk situations	Ending support abruptly

Relapse is a normal part of change, not a failure. Treat it as information and help the client re-enter the cycle.

Motivational Interviewing in Brief

Element	Content
Spirit	Partnership, acceptance, compassion, evocation
Four processes	Engaging → focusing → evoking → planning
Core skills (OARS)	Open questions, Affirmations, Reflections, Summaries
Change talk (DARN-CAT)	Preparatory: **D**esire, **A**bility, **R**easons, **N**eed. Mobilizing: **C**ommitment, **A**ctivation, **T**aking steps
Sustain talk	The client's arguments *for* staying the same, met with reflection, not argument
Developing discrepancy	Highlighting the gap between current behavior and the client's own values or goals
Righting reflex	The counselor's urge to fix or persuade, which MI teaches you to resist
Rulers	"On a scale of 0 to 10, how important is this change to you? How confident are you that you could do it?"

The MI move the NCE rewards: when a client is ambivalent, reflect both sides. "Part of you wants to keep drinking because it helps you relax, and part of you is worried about what it's doing to your marriage."

Relapse Prevention (Marlatt)

- Identify **high-risk situations** (people, places, emotions).
- Build **coping responses** for each.
- Watch for **seemingly irrelevant decisions** that set up a lapse.
- Address the **abstinence violation effect**: after one slip, the belief "I've blown it, so I might as well keep going." Reframe a lapse as a learning event, not a collapse.

Skills Training and Psychoeducation (5.M, 5.T, 5.U, 5.Y, 5.Z)

Psychoeducation is teaching: giving clients information about their condition, treatment, or coping strategies. Skills training goes further: the client practices.

Skill area	Core content	Owner (where there is one)
Assertiveness	Passive vs. aggressive vs. assertive vs. passive-aggressive; "I" statements; broken record; saying no; behavior rehearsal	Assertiveness training grew from behavioral work (Wolpe, Salter, Lazarus)
Communication	Active listening; "I" messages instead of "you" messages; speaker–listener technique	Thomas Gordon (Parent Effectiveness Training: I-messages, active listening)
Conflict resolution	Separate people from problems; focus on interests, not positions; generate options; agree on criteria; time-outs when flooded	Interest-based negotiation (Fisher & Ury); Gottman (softened startup, repair attempts)

Skill area	Core content	Owner (where there is one)
Stress management	Progressive muscle relaxation; diaphragmatic breathing; mindfulness; time management; cognitive reappraisal	Progressive relaxation (Edmund Jacobson); relaxation response (Herbert Benson); Mindfulness-Based Stress Reduction (Jon Kabat-Zinn)
Problem-solving	Define the problem → generate alternatives → decide → implement → evaluate	D'Zurilla & Goldfried
Anger management	Identify cues; time-outs; cognitive restructuring; relaxation; assertive alternatives	(CBT tradition; Novaco)
Divorce adjustment	Grief for the relationship; co-parenting; children's adjustment; practical changes	(psychoeducational groups)

Assertive, Passive, Aggressive

Style	Example ("A coworker keeps taking credit for your work")
Passive	Says nothing; resents it privately
Aggressive	"You're a liar and a thief."
Passive-aggressive	"Must be nice to have someone else do your job."
Assertive	"When my work is presented as yours, I feel frustrated. I'd like us to credit each person's part going forward."

The "I" statement formula: *When* [specific behavior], *I feel* [feeling], *because* [impact]. *I'd like* [request].

Homework

Between-session tasks are a core part of CBT, REBT, reality therapy, SFBT (the first-session formula task), and skills training. The exam rewards homework that is **collaboratively set, specific, and reviewed at the next session.** Assigning homework and never asking about it teaches the client that it doesn't matter.

Educating Clients About Transference and Defenses (5.R)

One outline item asks you to *teach* these psychodynamic concepts in plain language, not just recognize them.

Term	Meaning
Transference	The client redirects feelings and expectations from important past relationships onto the counselor
Countertransference	The counselor's own emotional reactions to the client, often from the counselor's history
Defense mechanism	An unconscious strategy that protects against anxiety (see Chapter 2)

How to educate: use plain language, connect it to something the client has noticed, and offer it tentatively. ("Sometimes, when someone reminds us of an important person from our past, we react to them the way we reacted back then. I wonder if some of what you feel toward me might fit that.")

Countertransference is the counselor's responsibility. When a counselor notices strong personal reactions to a client, the correct answer is self-reflection and **supervision or consultation**, not disclosing the reaction to the client, ignoring it, or immediately referring the client out.

Implementing the Plan (5.D)

Every intervention should trace back to a treatment goal. If an item describes a counselor introducing a technique unrelated to the client's stated goals, that's the wrong answer even if the technique is valid. When progress stalls, the next step is to **review the plan with the client**, not to keep repeating the same intervention or to switch approaches without discussing it (Chapter 21).

What the Exam Rewards / What It Punishes

Rewards: - Matching strongly supported approaches to the presenting concern - Play-based and concrete methods for young children; involving caregivers - Stage-matched responses to motivation - Reflecting ambivalence instead of arguing with it - Collaborative, specific homework that gets reviewed - Supervision for countertransference

Punishes: - Action plans with a precontemplative client - Abstract cognitive work with young children - Conjoint couples work when violence is present - Advice disguised as psychoeducation - Treating a relapse as treatment failure

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Swap-Pair Drill

If you see…	It's…	Not…
Face the obsession, block the compulsion	ERP	Systematic desensitization
Relaxation + imagined hierarchy	Systematic desensitization (Wolpe)	Flooding
Intense imagined exposure	Implosive therapy (Stampfl)	Flooding (real life)
Parents run play sessions at home	Filial therapy (Guerney)	Child-centered play therapy (Axline/ Landreth)

If you see…	It's…	Not…
Structured review of one's whole life	Life review (Butler)	Reminiscence therapy
Parents take charge of eating	Family-based treatment (Maudsley)	Structural family therapy
Coaching parents live through an earpiece	PCIT (Eyberg)	Filial therapy
Desire, Ability, Reasons, Need	Change talk (MI)	Stages of change
"I've blown it, so I might as well keep going"	Abstinence violation effect (Marlatt)	Contemplation
I-messages	Thomas Gordon	Ellis
Relaxation response	Herbert Benson	Edmund Jacobson (progressive relaxation)

Quick Checks

1. A client with contamination obsessions washes her hands 40 times a day. The most strongly supported intervention is: A. Psychoanalytic exploration of early toilet training B. Exposure and response prevention C. Person-centered reflection of feelings D. Life review

2. A 5-year-old has become withdrawn since his parents' separation. The most developmentally appropriate approach is: A. Cognitive restructuring of his automatic thoughts B. Child-centered play therapy C. Decisional balance exercise D. Assertiveness training

3. A client says, "My wife thinks I drink too much, but I don't see a problem." He is in which stage of change? A. Precontemplation B. Contemplation C. Preparation D. Action

4. For the client in question 3, the best counselor response is: A. "Let's set a quit date." B. "You're an alcoholic, and denial is part of the disease." C. "Your wife's concerns don't seem to match how you see it. What do you

think she's noticing?" D. "Let's talk about relapse prevention."

5. A client says, "Part of me wants to stop gambling, but it's the only time I feel excited." In MI, the best response is: A. "Gambling is ruining your finances; you need to stop." B. "On one hand, gambling gives you a rush you don't get elsewhere, and on the other, part of you wants to stop." C. "Why do you keep gambling if you want to stop?" D. "Let's make a list of Gamblers Anonymous meetings."

6. After 90 days sober, a client drinks at a wedding and says, "I've ruined everything, so there's no point stopping now." This reflects: A. Precontemplation B. The abstinence violation effect C. Transference D. Sustain talk

7. A counselor finds herself feeling unusually irritated by a client who reminds her of her ex-husband. The best action is: A. Tell the client about the resemblance B. Refer the client immediately C. Seek supervision or consultation about the countertransference D. Ignore the reaction and continue

8. A counselor teaches parents to hold weekly child-centered play sessions at home. This is: A. PCIT B. Filial therapy C. Sandplay D. Structural family therapy

9. Which is an assertive response to a roommate who leaves dishes in the sink? A. Washing them silently and feeling resentful B. "You're such a slob." C. "When dishes stay in the sink overnight, I feel frustrated because the kitchen gets crowded. Could we agree to wash them by bedtime?" D. "Wow, nice of you to leave these for me again."

10. A 78-year-old client is struggling to find meaning after retirement. Which approach is best matched to her developmental stage? A. Life review B. PCIT C. Exposure therapy D. Filial therapy

11. A couple seeks counseling. During intake, one partner discloses being afraid of the other, who controls the fin-

ances and monitors her phone. The best next step is: A. Begin conjoint sessions focused on communication B. Assess safety individually before deciding whether couples counseling is appropriate C. Teach both partners “I” statements D. Use enactment to observe the couple’s conflict

12. Which intervention for panic disorder deliberately produces the physical sensations the client fears? A. Systematic desensitization B. Interoceptive exposure C. Flooding D. Implosive therapy

Answers and Rationales

1 — B. ERP has the strongest support for OCD symptoms. A: psychoanalysis (Freud) is a real therapy but a poor match. C: person-centered work (Rogers) doesn’t address the compulsion cycle. D: life review (Butler) is a late-life approach.

2 — B. Play is a young child’s language (Axline, Landreth). A: abstract restructuring is beyond a preoperational child (Piaget). C: decisional balance is an MI/TTM tool for adults weighing change. D: assertiveness training is a skills intervention for adolescents and adults.

3 — A. No acknowledgment of a problem is precontemplation (Prochaska & DiClemente). B: contemplation includes some acknowledgment and ambivalence. C: preparation includes intent and a plan. D: action involves changed behavior.

4 — C. Raising awareness gently, without arguing, fits precontemplation. A: action-stage move. B: labeling and confrontation increase resistance. D: maintenance-stage move.

5 — B. A double-sided reflection of ambivalence is a core MI skill (Miller & Rollnick). A: the righting reflex. C: a “why” question invites defensiveness. D: an action-stage referral before the client has resolved ambivalence.

6 — B. The all-or-nothing reaction to a lapse is Marlatt’s abstinence violation effect. A: precontemplation is not re-

cognizing a problem. C: transference involves feelings toward the counselor. D: sustain talk is MI's term for any argument against changing; this item describes a specific reaction to a lapse.

7 — C. Countertransference is managed through supervision or consultation. A: shifts the burden to the client. B: premature; the counselor's reaction can usually be managed. D: unmanaged countertransference can harm the client.

8 — B. Training parents to run play sessions is filial therapy (Guerney). A: PCIT (Eyberg) coaches parents live during interaction, with a behavioral focus. C: sandplay (Kalff) is a therapist-run sand tray method. D: structural therapy (Minuchin) works on boundaries and hierarchy.

9 — C. Specific behavior, feeling, impact, request: an assertive "I" statement. A: passive. B: aggressive. D: passive-aggressive.

10 — A. Life review (Butler) fits Erikson's integrity vs. despair. B and D: child-focused parent interventions. C: exposure targets fear and avoidance.

11 — B. Signs of coercive control make conjoint work potentially dangerous; assess safety individually first. A, C, D: all proceed with couples work before safety is assessed. D is also Minuchin's technique, which could escalate risk.

12 — B. Interoceptive exposure targets feared body sensations. A: systematic desensitization (Wolpe) uses imagined scenes with relaxation. C: flooding is intense real-life exposure to external situations. D: implosive therapy (Stampfl) is intense imagined exposure.

Audio Companion

Track	Covers	Target length
5.1	Match It: problem, age, and stage → intervention	~10 min

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Chapter 6 — Crisis, Safety, and Risk

Domain map: 5.Q Provide crisis intervention · 5.V Develop safety plans · 2.S Conduct ongoing assessment for at-risk behaviors (suicide, homicide, self/other injury, relationship violence) · 3.AJ Suicidal thoughts/behaviors · 3.AP Child abuse–related concerns · 3.AT Family abuse/violence · 3.AU Interpersonal partner violence concerns · 1.F Discuss limits of confidentiality · 1.S Provide information to third parties · 4.E Refer to different levels of treatment · CACREP: Counseling and Helping Relationships; Professional Counseling Orientation and Ethical Practice

Safety Outranks Everything

Chapter 1 gave you six priorities for applied items. The first one was safety. This chapter is where that rule does most of its work.

When any answer option addresses a risk to life, it usually beats every option that doesn't, no matter how good the others are clinically. Building rapport, exploring feelings, and staying on the treatment plan all wait until safety is addressed.

The NCE tests risk in five forms:

1. **Crisis** — a person overwhelmed and unable to cope
2. **Suicide risk** — assessment, level of care, and safety planning
3. **Harm to others** — threats, homicide risk, and the duty to protect
4. **Abuse** — children, older adults, and dependent adults; mandated reporting
5. **Intimate partner violence** — assessment and safety planning with the person at risk

Part 1 — Crisis Theory and Intervention

Who Owns Crisis Theory

Owner	Contribution
Erich Lindemann	Studied survivors' grief after the 1942 Cocoanut Grove nightclub fire; described acute grief reactions; founded the field of crisis intervention
Gerald Caplan	Preventive psychiatry; crisis as a time-limited state (often described as about 4–6 weeks) when usual coping fails; primary, secondary, and tertiary prevention
Albert Roberts	Seven-stage crisis intervention model
James & Gilliland	Six-step crisis intervention model; a person in crisis experiences both danger and opportunity
Kenneth Doka	Disenfranchised grief: loss that isn't socially recognized (e.g., death of an ex-partner, pregnancy loss, a pet)

What a Crisis Is

A crisis is not the event. It is a person's response when an event overwhelms their usual ways of coping. The same event can be a crisis for one person and a manageable stressor for another.

Key features: - **Time-limited.** Crises resolve, for better or worse, within weeks. - **Danger and opportunity.** A crisis can lead to lower functioning or to growth. - **Heightened openness.** People in crisis are often more receptive to help than at other times.

Type	Example
Developmental	Predictable life transitions: birth of a child, retirement, adolescence

Type	Example
Situational	Sudden, unexpected events: accident, job loss, assault, sudden death
Existential	Inner conflicts about meaning, purpose, or identity
Ecosystemic	Disasters affecting whole communities: hurricane, pandemic, mass violence

Roberts' Seven-Stage Model

1. Plan and conduct a thorough **biopsychosocial and lethality/imminent danger assessment**
2. **Make psychological contact** and rapidly establish the relationship
3. Identify the **major problems**, including crisis precipitants
4. Encourage exploration of **feelings and emotions** (active listening, validation)
5. Generate and explore **alternatives and new coping strategies**
6. Restore functioning through an **action plan**
7. Plan **follow-up** and booster sessions

James & Gilliland's Six-Step Model

Listening steps: 1. Define the problem 2. Ensure client safety 3. Provide support

Acting steps: 4. Examine alternatives 5. Make plans 6. Obtain commitment

Safety assessment runs *throughout* all six steps, not just at step 2.

How Crisis Counseling Differs

Ongoing counseling	Crisis intervention
Explores history and patterns	Focuses on the here and now and the immediate problem
Counselor is often less directive	Counselor is **more directive** as needed

Ongoing counseling	Crisis intervention
Long-term goals	Goal: **restore pre-crisis functioning** and safety
Weekly sessions	Brief, immediate, often a single contact
Insight is a goal	Stabilization comes first; insight can come later

The NCE's crisis rule: in an acute crisis, a more directive response is correct. An option that explores childhood patterns or begins long-term goal setting during an active crisis is a wrong answer.

Psychological First Aid (PFA)

PFA is an evidence-informed approach for the hours and days after a disaster or traumatic event (National Child Traumatic Stress Network and National Center for PTSD). Core actions:

- Contact and engagement
- Safety and comfort
- Stabilization (if needed)
- Information gathering on current needs
- Practical assistance
- Connection with social supports
- Information on coping
- Linkage with collaborative services

PFA is not debriefing. Forcing survivors to describe the event in detail soon after it happens (single-session psychological debriefing) is not recommended and may increase distress for some people. PFA does not push people to retell the trauma.

Part 2 — Suicide Risk

Myths the Exam Tests

Myth	Fact
Asking about suicide puts the idea in someone's head	Asking directly does not increase risk and is the standard of care
People who talk about suicide won't do it	Many people who die by suicide told someone first
A sudden improvement in mood means the danger has passed	A sudden calm can follow a decision to die; reassess
A no-suicide contract keeps a client safe	No-suicide contracts have no evidence of preventing suicide and can give the counselor false reassurance; use a safety plan
Suicidal people want to die	Most are ambivalent; part of them wants to live

Who Owns Suicide Theory

Owner	Contribution
Émile Durkheim	Sociological types of suicide: **egoistic** (too little social integration), **altruistic** (too much integration; self-sacrifice for the group), **anomic** (sudden disruption of social norms or regulation, e.g., economic collapse), **fatalistic** (excessive regulation and oppression)
Edwin Shneidman	Founder of suicidology; **psychache** (intolerable psychological pain); psychological autopsy
Thomas Joiner	Interpersonal theory of suicide: **thwarted belongingness + perceived burdensomeness** produce the desire for death; **acquired capability** (reduced fear of death, increased pain tolerance) makes an attempt possible

Owner	Contribution
Aaron Beck	Hopelessness as a stronger predictor of suicide than depression itself; Beck Hopelessness Scale; Beck Scale for Suicide Ideation
Kelly Posner and colleagues	Columbia–Suicide Severity Rating Scale (C-SSRS)
Barbara Stanley & Gregory Brown	Safety Planning Intervention
David Jobes	CAMS (Collaborative Assessment and Management of Suicidality)
Patterson, Dohn, Bird, & Patterson	SAD PERSONS mnemonic (a teaching tool; not validated for predicting risk on its own)

Assessing Risk

Ask directly, calmly, and in plain language: **"Are you having thoughts of killing yourself?"** Avoid euphemisms like "hurting yourself," which a client may not interpret as suicide.

Then assess, starting with ideation and escalating through plan, intent, and means:

Element	Questions
Ideation	Passive ("I wish I wouldn't wake up") or active ("I want to kill myself")? How often, how long, how intense?
Plan	Is there a plan? How specific (method, time, place)?
Intent	Does the client intend to act?
Means	Does the client have access to the method? Firearms in the home?
Preparatory behavior	Rehearsing, giving away possessions, writing a note, saying goodbyes, acquiring means

Element	Questions
History	Prior attempts (the strongest single predictor of a future attempt); family history of suicide
Current state	Hopelessness, intoxication, agitation, insomnia, recent loss, isolation
Protective factors	Reasons for living, children, faith, social support, engagement in treatment, future plans

Warning signs (the American Association of Suicidology's IS PATH WARM mnemonic): Ideation, **S**ubstance use, **P**urposelessness, **A**nxiety, **T**rapped, **H**opelessness, **W**ithdrawal, **A**nger, **R**ecklessness, **M**ood change.

Demographic patterns the NCE may test: men die by suicide at higher rates; women make more nonfatal attempts. Firearms account for a large share of suicide deaths in the United States, which is why means restriction matters. Check current CDC data before quoting specific rates; they change.

Matching Level of Care to Risk

Risk	Typical picture	Typical response
Low	Passive ideation; no plan or intent; protective factors present	Collaborative safety plan; monitor at every session; address contributing problems
Moderate	Active ideation; vague plan; no clear intent; some risk factors	Safety plan; means restriction; increase session frequency; involve supports (with consent); consider a higher level of care
High	Specific plan, intent, access to means, recent attempt or preparation, intoxication	Do not leave the client alone; arrange immediate evaluation for hospitalization; voluntary admission if possible

Risk	Typical picture	Typical response
Imminent	States intent to act now and has means	Emergency services; stay with the client; involuntary hold if the client won't accept voluntary care

Least restrictive appropriate care: the NCE doesn't reward hospitalizing every client who mentions suicide. It rewards matching the response to the risk. Under-responding to high risk is wrong. Over-responding to low risk (hospitalization for passive ideation with strong protective factors) is also wrong.

The Safety Plan (Stanley & Brown)

A safety plan is written **with** the client, in the client's words, and kept where the client can reach it. The six steps:

1. **Warning signs** — thoughts, moods, situations, or behaviors that signal a crisis is starting
2. **Internal coping strategies** — things the client can do alone to take their mind off the crisis
3. **People and social settings that provide distraction** — contacts and places that help, without needing to discuss the crisis
4. **People the client can ask for help** — family or friends who can be told about the crisis
5. **Professionals and agencies to contact** — counselor, after-hours line, local emergency services, and the **988 Suicide & Crisis Lifeline** (call or text 988 in the United States)
6. **Making the environment safe** — reducing access to lethal means

Many counselors also add the client's **reasons for living** to the plan. Review and update the plan at later sessions.

Means restriction counseling (also called lethal means counseling) is a strongly supported prevention strategy: helping the client and family limit access to firearms,

stockpiled medications, and other lethal means during periods of risk. Many suicidal crises are brief, and putting time and distance between a person and a lethal method saves lives.

Self-Injury Without Suicidal Intent

Non-suicidal self-injury (NSSI) is deliberate self-harm *without* the intent to die, often used to regulate overwhelming emotion. It still requires a suicide risk assessment: NSSI is associated with higher later suicide risk, and the two can coexist. Respond calmly and without judgment, assess medical needs and suicide risk, and work on alternative ways of coping (DBT skills have strong support). Don't demand that the client stop immediately or make continued counseling conditional on stopping.

Part 3 — Harm to Others and the Duty to Protect

Tarasoff

Case	Holding
***Tarasoff v. Regents of the University of California* (1974)** — "Tarasoff I"	Therapists have a **duty to warn** an identifiable victim of a client's serious threat
***Tarasoff v. Regents of the University of California* (1976)** — "Tarasoff II"	Broadened to a **duty to protect**: take reasonable steps to protect the intended victim, which may include warning, notifying police, or arranging hospitalization
***Jablonski v. United States* (1983)**	Duty can extend to a foreseeable victim based on the client's history of violence, even without an explicit threat
***Ewing v. Goldstein* (2004)**	A credible threat reported to the therapist by a client's family member can trigger the duty (California)

State law varies. Some states *require* counselors to act (mandatory duty), some *permit* disclosure without requiring it, and a few have no statute. The NCE tests the principle, not your state's law.

The general triggers: - A **serious threat of physical violence** - Against a **reasonably identifiable victim** - That the counselor believes, or should believe, is credible

What the counselor does: assess the threat (plan, means, history of violence, intent); consult with a supervisor or colleague and document; take protective action required or permitted by law; continue to treat the client if safe to do so. Disclose only the information needed to protect the person.

Assessing Risk of Violence

Ask about: intent, plan, access to weapons, identifiable target, history of violence (the strongest predictor of future violence), substance use, impulsivity, recent losses or humiliations, and protective factors.

Part 4 — Abuse and Mandated Reporting

Child Abuse and Neglect

- Counselors are **mandated reporters** in every U.S. state.
- The standard is **reasonable suspicion**, not proof. **Don't investigate.** That's the job of child protective services and law enforcement.
- Report **promptly**, within the time your state requires.
- Mandated reporters who report in **good faith** are protected from liability.
- Reporting is an **exception to confidentiality**. Explain this limit during informed consent, before it comes up.
- **Neglect** is the most commonly reported form of child maltreatment.

Type	Possible signs
Physical abuse	Unexplained or patterned injuries; explanations that don't fit the injury; fear of going home
Sexual abuse	Sexual knowledge or behavior beyond developmental level; genital pain; sudden behavior change
Emotional abuse	Extreme withdrawal or aggression; persistent belittling by caregiver
Neglect	Chronic hunger; poor hygiene; untreated medical needs; frequent absences; lack of supervision

When a child discloses: stay calm, listen, believe them, don't ask leading questions, don't promise to keep it secret, tell the child what happens next in age-appropriate words, document the child's words, and report.

Older Adults and Dependent Adults

Most states require or permit reporting suspected abuse, neglect, or exploitation of older or dependent adults to adult protective services. Types include physical, sexual, emotional, financial exploitation, neglect by a caregiver, and **self-neglect**. Requirements vary by state more than child abuse laws do; know your state's.

What Is *Not* Usually Reportable

- **Past abuse disclosed by an adult client about their own childhood**, when no child is currently at risk, is generally not a mandated report (check state law; in some states the answer changes if the abuser has current access to children).
- **Intimate partner violence between competent adults** is generally not a mandated report to protective services (some states require health care providers to report certain injuries). The counselor's role is assessment and safety planning with the client.

Part 5 — Intimate Partner Violence (IPV)

Owner	Contribution
Lenore Walker	Cycle of violence: tension-building → acute battering incident → honeymoon (reconciliation/calm)
Domestic Abuse Intervention Programs (Duluth, MN)	Power and Control Wheel
Jacquelyn Campbell	Danger Assessment instrument for lethality risk in IPV
Evan Stark	Coercive control

The Exam's IPV Rules

1. **Screen privately.** Ask about safety only when the partner is not in the room.
2. **Couples counseling is contraindicated** while there is ongoing violence or coercive control. It can increase danger and prevent the victim from speaking freely.
3. **Respect the client's autonomy.** Leaving is often the most dangerous time. The counselor helps the client assess risk and plan, without pressuring the client to leave on the counselor's timeline.
4. **Assess lethality.** High-risk indicators include prior strangulation, threats to kill, access to a firearm, escalating frequency or severity, extreme jealousy, and the victim's own sense that the partner could kill them.
5. **Safety planning with the victim:** emergency contacts, a safe place to go, important documents and money ready, a code word with trusted people, the National Domestic Violence Hotline, and local shelter resources.
6. **Batterer intervention programs**, not anger management alone, are the standard referral for those who use violence.

Part 6 — Documentation and Consultation

In every risk situation: - **Consult** with a supervisor or colleague when possible. - **Document** the assessment, the reasoning behind your decision, the actions taken, and the consultation. - **Follow up.** Check the safety plan, confirm the client reached the referral, and reassess at the next contact.

“If it isn’t documented, it didn’t happen” is a guiding principle in risk management.

What the Exam Rewards / What It Punishes

Rewards: - Asking about suicide directly - Safety plans and means restriction over no-suicide contracts - Matching the level of care to the level of risk - A more directive stance during an acute crisis - Reporting on reasonable suspicion, promptly, without investigating - Assessing IPV privately and declining conjoint work when violence is present - Consultation and documentation

Punishes: - Avoiding the word “suicide” or relying on euphemisms - Hospitalizing every client who mentions suicide, or none of them - Exploring childhood history during an active crisis - Promising a child to keep abuse secret - Waiting for proof before reporting abuse - Pressuring an IPV victim to leave immediately - Forcing trauma retelling right after a disaster

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Swap-Pair Drill

If you see…	It belongs to…	Not…
Cocoanut Grove fire; acute grief	Lindemann	Caplan
Preventive psychiatry; crisis lasts about 4–6 weeks	Caplan	Lindemann
Seven-stage crisis intervention model	Roberts	James & Gilliland

If you see…	It belongs to…	Not…
Six-step crisis model: listening, then acting	James & Gilliland	Roberts
Anomic suicide	Durkheim	Shneidman
Psychache	Shneidman	Joiner
Thwarted belongingness; perceived burdensomeness	Joiner	Durkheim
Hopelessness predicts suicide	Beck	Joiner
Safety Planning Intervention	Stanley & Brown	Jobes (CAMS)
Duty to warn (1974)	Tarasoff I	Tarasoff II (duty to protect, 1976)
Tension → battering → honeymoon	Walker	Duluth (Power and Control Wheel)
Danger Assessment	Campbell	Posner (C-SSRS, suicide)
Coercive control	Stark	Walker
Grief not socially recognized	Doka (disenfranchised grief)	Worden

Quick Checks

1. A client says, “Sometimes I think everyone would be better off without me.” The counselor’s best next response is: A. “You have so much to live for.” B. “Are you having thoughts of killing yourself?” C. “Let’s talk about your childhood.” D. “Will you promise not to hurt yourself before our next session?”

2. Which is the strongest single predictor of a future suicide attempt? A. Female gender B. A previous suicide attempt C. Being married D. Anxiety

3. A client with passive ideation, no plan, no intent, strong family support, and no prior attempts is best served by: A. Involuntary hospitalization B. A collaborat-

ive safety plan and continued outpatient counseling with ongoing monitoring C. A no-suicide contract D. Referral to a psychiatrist and ending counseling

4. A depressed client who has been withdrawn for weeks arrives calm and cheerful, and mentions he gave his guitar to his brother. The counselor should: A. Celebrate the improvement and reduce session frequency B. Reassess suicide risk directly C. Begin termination planning D. Explore his relationship with his brother

5. Which theorist proposed that thwarted belongingness and perceived burdensomeness create the desire for suicide? A. Durkheim B. Shneidman C. Joiner D. Beck

6. A client says he's going to "make sure" his ex-girlfriend's new partner "pays," and that he knows where the man works and owns a gun. The counselor's best action is to: A. Keep the information confidential because the client hasn't acted B. Assess the threat, consult, document, and take protective action consistent with state law C. Terminate counseling immediately D. Confront the client about his jealousy

7. A 9-year-old tells her school counselor that her stepfather hits her with a belt and asks her not to tell anyone. The counselor should: A. Promise to keep it secret to protect the relationship B. Interview the stepfather to confirm the story C. Report to child protective services and explain to the child, in simple terms, what will happen D. Wait until there is physical evidence

8. A woman in individual counseling discloses that her husband has choked her twice and recently threatened to kill her. He has a gun. The counselor's priority is to: A. Recommend couples counseling B. Assess lethality and develop a safety plan with her C. Insist she leave the house tonight D. Report the husband to child protective services

9. A client calls in crisis after a house fire. Using James and Gilliland's model, which step comes first? A. Examine

alternatives B. Obtain commitment C. Define the problem D. Make plans

10. Which is the most appropriate approach in the first days after a community disaster? A. Mandatory group debriefing in which every survivor describes the event in detail B. Psychological first aid C. Long-term psychodynamic therapy D. Exposure therapy

11. A client cuts her arms to "feel something" when overwhelmed but says she doesn't want to die. The counselor should: A. Tell her counseling will stop if she cuts again B. Accept her statement and skip suicide assessment C. Assess suicide risk and medical needs, and work on alternative coping skills D. Hospitalize her immediately

12. The 1976 Tarasoff decision established that therapists have a duty to: A. Keep all threats confidential B. Protect an intended victim of a client's serious threat C. Report all clients with violent histories to police D. Warn the client's family about any threat

Answers and Rationales

1 — B. Indirect statements of being a burden call for a direct question about suicide. A: reassurance that dismisses the feeling. C: exploring history before assessing safety. D: a no-suicide contract instead of an assessment.

2 — B. A prior attempt is the strongest single predictor. A: women attempt more often, but gender alone isn't the strongest predictor, and men die by suicide at higher rates. C: being married is generally protective. D: anxiety can raise risk but is not the strongest predictor.

3 — B. Low risk matched to a low-intensity, collaborative response. A: overly restrictive for low risk. C: no-suicide contracts lack evidence and can create false reassurance. D: abandons the client.

4 — B. Sudden calm plus giving away a possession are warning signs; reassess directly. A and C: treat a warning sign as improvement. D: explores before assessing safety.

5 — C. Joiner's interpersonal theory. A: Durkheim's types are sociological (egoistic, altruistic, anomic, fatalistic). B: Shneidman owns psychache. D: Beck owns hopelessness as a predictor.

6 — B. A serious threat, an identifiable victim, and access to a weapon can trigger the duty to protect; assess, consult, document, and act per state law. A: confidentiality has limits. C: abandonment doesn't protect anyone. D: misses the safety issue.

7 — C. Reasonable suspicion triggers a mandated report; tell the child honestly what comes next. A: counselors can't promise secrecy about abuse. B: investigation is CPS's role and could put the child at greater risk. D: proof isn't required.

8 — B. Strangulation, threats to kill, and a firearm are high lethality indicators (Campbell); assess and plan for safety. A: conjoint work is contraindicated. C: pressuring her to leave on the counselor's timeline ignores her autonomy and the danger of leaving without a plan. D: CPS is for children; nothing here indicates a child at risk.

9 — C. Defining the problem is step 1 of James & Gilliland's model. A: step 4; D: step 5; B: step 6 (the acting steps).

10 — B. PFA is the recommended early response. A: forced detailed retelling soon after trauma isn't recommended and may increase distress for some. C and D: not first-response approaches in the immediate aftermath.

11 — C. NSSI still requires suicide and medical assessment, plus skill-building (DBT). A: an ultimatum harms the alliance and doesn't build skills. B: NSSI and suicide risk can coexist. D: over-response without assessment.

12 — B. Tarasoff II (1976) established the duty to protect. A: the opposite. C: a history of violence alone doesn't create a duty to report. D: the duty runs to the intended victim, not the client's family.

Audio Companion

Track	Covers	Target length
6.1	Safety First: crisis models, suicide assessment, duty to protect, reporting	~8 min

Note to readers: If you or someone you know is in crisis, call or text 988 (Suicide & Crisis Lifeline, United States) or call 911.

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Chapter 7 — Couples and Family Systems in Session

Domain map: 5.I Address family composition and cultural considerations · 5.J Evaluate and explain systemic patterns of interaction · 5.K Explore family member interaction · 5.P Improve interactional patterns · 5.W Facilitate systemic change · 5.AG Facilitate resolution of interpersonal conflict · 5.AK Address impact of extended families · 5.AM Explore the influence of family of origin patterns and themes · 5.AN Address the impact of social support network · 2.R Evaluate interactional dynamics · 5.B Align intervention with counseling modality · CACREP: Counseling and Helping Relationships; Human Growth and Development

A Different Way of Seeing

Chapter 2 gave you the owners of the family therapy models: Bowen, Minuchin, Haley, MRI, Milan, Satir, Whitaker, Johnson, and Gottman. This chapter teaches what they share and how the NCE tests it in session.

The shift is this: **in systems work, the client is the relationship or the family, not one person.** A teenager's behavior problem is not seen as a defect inside the teenager. It is seen as part of a pattern that the whole family keeps going, often without meaning to. The NCE rewards answers that look at the pattern and punishes answers that blame one member.

Systems Concepts and Their Owners

Concept	Owner	Meaning
General systems theory	Ludwig von Bertalanffy	A system is more than the sum of its parts; parts are understood through their relationships
Cybernetics	Norbert Wiener	The study of feedback and self-regulation in systems
Double bind	Gregory Bateson (with Jackson, Haley, Weakland)	Contradictory messages at different levels, with no way to comment on or leave the contradiction
Homeostasis	Don Jackson (applying Cannon's term)	Families resist change and work to restore their usual balance
Second-order cybernetics	Heinz von Foerster	The observer (counselor) is part of the system being observed

Core Vocabulary

Term	Meaning	Exam example
Wholeness	The family is more than the sum of its members	Change one member, and the whole family is affected
Circular causality	Behaviors cause and are caused by each other in loops	"She nags because he withdraws; he withdraws because she nags"
Linear causality	A causes B (the view systems thinking replaces)	"He withdraws because she nags"
Negative feedback	Reduces deviation; keeps the system the same (morphostasis)	Parents tighten rules when a teen pushes limits
Positive feedback	Amplifies deviation; pushes the system toward change (morphogenesis)	An argument that escalates with each reply

Term	Meaning	Exam example
Morphostasis vs. morphogenesis	Stability vs. change	Healthy families need both
Equifinality	Different starting points can lead to the same outcome	Two very different families can end up with the same problem
Multifinality	The same starting point can lead to different outcomes	Two siblings respond to the same divorce differently
Identified patient (IP)	The member whose symptoms bring the family to counseling	The symptom often serves a function for the family
First-order change	Change within the system's rules; the pattern stays	Grounding a teen for a week instead of a day
Second-order change	Change to the system's rules themselves	The parents start agreeing on rules together
Open vs. closed systems	Exchange with the outside world vs. isolation	Closed families resist outside help and new information

"Negative" and "positive" feedback describe direction, not value. Negative feedback is not bad; it keeps things stable. Positive feedback is not good; it can escalate a conflict. This is the same trap as negative reinforcement in Chapter 2.

Who Is the Client? Ethics in Couple and Family Work

Systems work changes several ethical defaults (ACA Code of Ethics, 2014: A.8, Multiple Clients, and B.4.b, Couples and Family Counseling).

Issue	Rule
Identifying the client	At the start, clarify who the clients are and the counselor's relationship with each person

Issue	Rule
Informed consent	Every participating member consents; minors assent, and parents or guardians consent
Secrets policy (B.4.b)	Decide and explain at the start: a **no-secrets** policy (individually shared information may be brought into joint sessions) or a **limited-secrets** policy. Explain it *before* anyone shares a secret
Confidentiality	Each person's right to confidentiality is discussed; the counselor can't promise what other members will keep private
Conflicting roles	If the counselor may be asked to serve in conflicting roles (e.g., testifying in a custody dispute), clarify, adjust, or withdraw
Neutrality	The counselor doesn't take sides or form alliances with one member (Bowen: stay out of the triangle; Milan: neutrality)
Safety	Couples work is contraindicated when there is ongoing violence or coercive control (Chapter 6)

When a partner discloses an affair in an individual session and the counselor has a no-secrets policy, the scored answer is usually to encourage and support that partner in disclosing it in a joint session, and to follow the policy already explained. The wrong answers: promising to keep the secret despite a no-secrets policy, revealing it in a joint session without warning the partner, or ending counseling.

Assessing the Family

Tool	Owner	What it shows
Genogram	Bowen (concept); standardized by Monica McGoldrick & Randy Gerson	Three or more generations: relationships, patterns, cutoffs, illnesses, substance use, conflict
Ecomap	Ann Hartman	The family's connections to outside systems: school, work, church, extended family, agencies, friends
Family sculpting	Satir (also Duhl, Kantor, & Duhl)	Members physically arrange each other to show closeness, distance, and power
Family mapping	Minuchin	Diagrams of boundaries, subsystems, coalitions
Circumplex Model; FACES	David Olson	Two dimensions: **cohesion** (disengaged → enmeshed) and **flexibility** (rigid → chaotic); balanced families are in the middle range of both
McMaster Model of Family Functioning	Nathan Epstein and colleagues	Six dimensions: problem solving, communication, roles, affective responsiveness, affective involvement, behavior control
Beavers Systems Model	W. Robert Beavers	Family competence and style

Genogram vs. ecomap: the genogram looks **across generations** inside the family. The ecomap looks **outward** at the family's connections to the community and support network (5.AN).

Common Genogram Conventions

Symbol	Meaning
Square	Male
Circle	Female
X inside a symbol	Death
Horizontal line between partners	Marriage
One slash across the line	Separation
Two slashes across the line	Divorce
Dashed line	Cohabiting or committed relationship without marriage
Double or triple lines between people	Close or fused relationship
Zigzag line	Conflict
Broken line with gap	Cutoff
Double outline around a symbol	The identified patient (index person)

The Family Life Cycle

Families move through predictable stages, and symptoms often appear when a family struggles to make a transition (Carter & McGoldrick; McGoldrick, Garcia Preto, & Carter).

Stage	Key emotional process	Second-order changes required
1. Leaving home: single young adults	Accepting emotional and financial responsibility for self	Differentiation from family of origin; intimate peer relationships; work and financial independence
2. Joining of families through marriage/ union	Commitment to a new system	Forming the couple system; realigning relationships with extended families and friends

Stage	Key emotional process	Second-order changes required
3. Families with young children	Accepting new members into the system	Making space for children; taking on parenting roles; realigning with grandparents
4. Families with adolescents	Increasing flexibility of boundaries to allow adolescents' independence	Shifting parent–child relationships; refocus on midlife couple and career issues; beginning care for older generation
5. Launching children and moving on	Accepting many exits from and entries into the family	Renegotiating the couple as a dyad; adult-to-adult relationships with grown children; in-laws and grandchildren; coping with parents' disability and death
6. Families in later life	Accepting the shifting of generational roles	Maintaining functioning in the face of decline; supporting a more central role for the middle generation; dealing with loss of spouse, siblings, peers

This is the classic six-stage version of the model (Carter & McGoldrick). Newer editions (McGoldrick, Garcia Preto, & Carter, 2016) divide the later stages more finely, but the exam-level point is the same: symptoms cluster at transitions. Divorce and remarriage add their own stages and tasks (Chapter 13).

Exam use: when a family's symptoms begin right around a transition (a first child, a teen's push for independence, the last child leaving), the systemic answer frames the problem as difficulty making the transition, not as an individual disorder.

Running a Family or Couple Session

Haley's Structured First Interview

Haley described stages for a first family session:

1. **Social stage** — greet each member; help everyone feel comfortable; observe seating and interactions
2. **Problem stage** — ask *each* member for their view of the problem
3. **Interaction stage** — have family members talk to each other about the problem while the counselor observes
4. **Goal-setting stage** — define a clear, solvable goal
5. **Task-setting stage** — give a directive (homework) to move toward the goal

Joining Comes First

Every family model starts by **joining**: connecting with each member, accommodating the family's style, and earning trust. Minuchin named it, but all systemic approaches require it.

- Greet and engage **every** member, including young children and quiet members.
- Don't side with the person who called for the appointment.
- Respect the family's hierarchy and culture at first, even if you will later help them change it.

Systemic Interventions by Purpose

Purpose	Intervention	Owner
See the pattern live	**Enactment** — family members interact in session	Minuchin
Follow the sequence	**Tracking** — ask who does what, then what happens next	Structural/strategic
Show relationships and positions	**Sculpting**	Satir

Purpose	Intervention	Owner
Reveal multigenerational patterns	**Genogram**, process questions	Bowen
Draw out how members see each other	**Circular questions** ("When Dad gets quiet, what does Mom do?")	Milan
Change meaning	**Reframing** ("Her nagging may be her way of trying to stay close to you")	MRI, structural, strategic
Change structure	**Boundary making**, **unbalancing**	Minuchin
Interrupt a stuck pattern	**Directives**, **prescribing the symptom**	Haley; MRI
Lower reactivity	**Detriangulation**; calm "I" positions	Bowen
Change the problem story	**Externalizing** the problem	White & Epston
De-escalate a couple's cycle	Identify the pursue–withdraw cycle; access primary emotions	Johnson (EFT)
Strengthen friendship and repair	Softened startup; repair attempts; love maps	Gottman

Circular Questions vs. Linear Questions

Linear (individual focus)	Circular (systemic focus)
"Why is Jamal acting out?"	"When Jamal acts out, who gets most upset? What does Dad do then?"
"How do you feel about your husband's drinking?"	"When your husband drinks, who in the family notices first? Who tries to fix it?"
"What's wrong with your daughter?"	"If your daughter's problem disappeared tomorrow, how would things change between the two of you?"

Circular questions put the pattern, not the person, in the center.

Family of Origin and Extended Family (5.AK, 5.AM)

- **Family of origin work** explores how patterns from the families partners grew up in shape the current relationship (Bowen's multigenerational transmission; Satir's family reconstruction).
- **Extended family** may be a source of support or stress: in-law conflict, grandparent involvement, triangulation of a child into a grandparent's conflict.
- **Culture shapes what "healthy" looks like.** In many cultures, close involvement of extended family, shared living, and strong parental authority are normal and protective. Labeling them "enmeshment" without considering culture is a scored error.

Triangles in Practice

When two people in conflict draw in a third to reduce tension, that's a triangle (Bowen). Examples: a parent who confides in a child about the other parent; a couple who only talk about the child's problems; a mother-in-law pulled into every argument.

The counselor's job: notice the triangle, stay out of it, and help the original pair deal directly with each other.

Couples Work

Communication and Conflict Patterns

Pattern	Owner	Description
Four Horsemen	Gottman	Criticism, contempt, defensiveness, stonewalling. **Contempt** is identified as the strongest predictor of divorce
Antidotes	Gottman	Gentle startup (for criticism); building a culture of appreciation (for contempt); taking responsibility (for defensiveness); physiological self-soothing (for stonewalling)
Pursue–withdraw (demand–withdraw)	Johnson (EFT); also Christensen	One partner pursues connection or change; the other withdraws; each reaction intensifies the other
Communication stances	Satir	Placater, blamer, super-reasonable, irrelevant; the goal is congruent communication
Speaker–listener technique	PREP (Markman, Stanley, & Blumberg)	One speaks briefly; the other paraphrases before replying

Integrative Behavioral Couple Therapy (IBCT)

Andrew Christensen and Neil Jacobson. Combines change strategies (communication and problem-solving training) with **acceptance** strategies (empathic joining around the problem; unified detachment; tolerance building). Some differences between partners won't change; acceptance of them is a goal.

The Exam's Couples Rules

1. **Assess safety privately** before conjoint work (Chapter 6).
2. **Stay neutral.** Don't side with the partner who seems more reasonable.
3. **Focus on the pattern**, not on who's right.
4. **Set the secrets policy** at the start.
5. **Have partners talk to each other**, not just to the counselor (enactment).

Culture and Family Composition (5.I)

- Families come in many forms: single-parent, blended, multigenerational, same-sex parents, foster and adoptive, grandparents raising grandchildren, chosen family. Don't assume a two-parent nuclear model.
- **Acculturation gaps** (Chapter 3, Berry) between immigrant parents and children raised in the host culture are a common source of conflict. Frame them as a difference in acculturation, not as defiance or rigidity.
- Ask about the family's own beliefs about roles, hierarchy, decision-making, and help-seeking before applying a model built on one culture's norms.
- **Broaching**: openly acknowledging cultural differences between counselor and family can strengthen trust.

What the Exam Rewards / What It Punishes

Rewards: - Seeing the problem as a pattern; using circular questions - Joining with every member before attempting change - Clarifying who the client is and the secrets policy at the start - Staying neutral and out of triangles - Using the genogram for generational patterns and the ecomap for outside supports - Framing symptoms around life cycle transitions - Considering culture before labeling closeness as enmeshment

Punishes: - Treating the identified patient as “the problem” - Siding with one partner or parent - Keeping secrets that contradict the stated policy - Starting conjoint work without screening for violence - Linear “why” questions about one member

Swap-Pair Drill

If you see…	It’s…	Not…
General systems theory	von Bertalanffy	Wiener (cybernetics)
Observer is part of the system	Second-order cybernetics (von Foerster)	First-order
“She nags because he withdraws because she nags”	Circular causality	Linear causality
Feedback that keeps things the same	Negative feedback (morphostasis)	Positive feedback
Same start, different outcomes	Multifinality	Equifinality
Change in the rules themselves	Second-order change	First-order change
Family’s links to school, church, agencies	Ecomap (Hartman)	Genogram
Cohesion and flexibility	Olson’s Circumplex Model	McMaster Model
Six dimensions incl. roles and behavior control	McMaster Model (Epstein)	Circumplex
Social, problem, interaction, goal-setting, task-setting	Haley’s first interview	Roberts’ crisis model
Contempt predicts divorce	Gottman	Sue Johnson (EFT)
Acceptance plus change in couples	IBCT (Christensen & Jacobson)	EFT
Speaker–listener technique	PREP (Markman et al.)	Gottman

Quick Checks

1. A family brings in their 14-year-old for skipping school. A systems-oriented counselor would most likely: A. Assess the teen individually for a conduct disorder B. Explore how the teen's behavior fits within patterns in the whole family C. Refer the teen for individual counseling D. Advise the parents to set stricter consequences

2. "When your mother gets upset, what does your father do?" is an example of: A. A linear question B. A circular question C. Enactment D. Reframing

3. At the start of couples counseling, the counselor explains that information shared in individual sessions may be brought into joint sessions. This is: A. A breach of confidentiality B. A no-secrets policy C. Triangulation D. Paradoxical intervention

4. Which tool best shows a family's connections to outside resources such as school, church, and social services? A. Genogram B. Ecomap C. FACES D. Family sculpting

5. A couple's conflict escalates each time one partner raises his voice and the other raises hers in response. This is an example of: A. Negative feedback B. Positive feedback C. Homeostasis D. Multifinality

6. Olson's Circumplex Model assesses families on: A. Problem solving and roles B. Cohesion and flexibility C. Differentiation and triangles D. Boundaries and hierarchy

7. In Haley's first interview, which stage asks each member for their view of the problem? A. Social stage B. Problem stage C. Interaction stage D. Task-setting stage

8. A mother regularly complains to her 12-year-old son about his father's spending. A Bowenian counselor would describe this as: A. Enmeshment B. Triangulation C. Disengagement D. Coalition building

9. A family from a culture where three generations share a home and grandparents help make parenting decisions

comes to counseling. The counselor's best first approach is to: A. Identify the grandparents' involvement as enmeshment B. Coach the parents to set firmer boundaries with the grandparents C. Explore the family's own views of roles and decision-making before applying a model D. See only the parents, since they are the legal guardians

10. According to Gottman, the strongest predictor of divorce among the Four Horsemen is: A. Criticism B. Contempt C. Defensiveness D. Stonewalling

11. Symptoms appear in a family soon after their oldest child begins high school and pushes for more independence. The systemic view is that the family is: A. Dysfunctional and needs restructuring B. Struggling with a life cycle transition that requires more flexible boundaries C. Showing signs of the adolescent's oppositional defiant disorder D. In homeostasis and needs no change

12. After another curfew violation, the parents extend their teen's grounding from one week to two, while each parent keeps enforcing rules separately. This is an example of: A. Second-order change B. First-order change C. Morphogenesis D. Equifinality

Answers and Rationales

1 — B. Systems thinking looks at the pattern, not one member. A: an individual, linear focus on the IP. C: removes the problem from the family context. D: advice that assumes a linear cause.

2 — B. Asking about one member's response to another's behavior is a circular question (Milan). A: a linear question would ask why one person does something. C: enactment (Minuchin) has members interact in session. D: a reframe (MRI) offers a new meaning.

3 — B. A no-secrets policy explained during informed consent (ACA A.8; B.4.b). A: it isn't a breach because clients agreed in advance. C: triangulation (Bowen) pulls a third person into a conflict. D: paradox (Haley, MRI) prescribes the symptom.

4 — B. The ecomap (Hartman) maps outside connections. A: the genogram (McGoldrick & Gerson) maps generations within the family. C: FACES (Olson) measures cohesion and flexibility. D: sculpting (Satir) shows relationships through physical positions.

5 — B. Escalation that amplifies change is positive feedback. A: negative feedback dampens change. C: homeostasis is a return to balance. D: multifinality is about different outcomes from the same start.

6 — B. Cohesion and flexibility are Olson's dimensions. A: problem solving and roles are McMaster dimensions (Epstein). C: differentiation and triangles are Bowen's concepts. D: boundaries and hierarchy are Minuchin's.

7 — B. The problem stage gathers each person's view. A: the social stage is greeting and engagement. C: the interaction stage has members talk to each other. D: the task-setting stage gives a directive.

8 — B. Pulling a child into the parents' conflict is triangulation (Bowen). A: enmeshment (Minuchin) is diffuse boundaries generally. C: disengagement (Minuchin) is rigid, distant boundaries. D: coalition is a structural term for an alliance against a third member, but this item asks for the Bowenian term.

9 — C. Cultural humility before applying a model. A: labeling culturally normal closeness as pathology. B: imposes one culture's norms. D: excludes members who share family decision-making.

10 — B. Gottman identified contempt as the strongest predictor. A, C, D: the other three Horsemen.

11 — B. The adolescent stage of the family life cycle (Carter & McGoldrick) requires more flexible boundaries. A: pathologizes a normal transition. C: individual diagnosis of the IP. D: homeostasis is the family's resistance to the needed change, not a sign that no change is needed.

12 — B. Doing more of the same within the existing rules is first-order change (MRI). A: second-order change alters

the rules. C: morphogenesis is change in the system's structure. D: equifinality is about different paths to the same outcome.

Audio Companion

Track	Covers	Target length
7.1	Think in Systems: concepts, tools, life cycle, couples	~7 min

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Chapter 8 — Group Work

Domain map: 5.AH Use linking and blocking in a group context · 5.AI Management of leader–member dynamics · 5.AJ Model giving and receiving of feedback · 5.AO Use "structured" activities · 5.AP Promote and encourage interactions among group members · 5.AQ Promote and encourage interactions with the group leader · 5.AR Use psychoeducation as a part of the group process · 5.AS Explain phases in the group process · 5.AT Identify and discuss group themes and patterns · 5.AU Create intervention based on the stage of group development · 5.AV Challenge harmful group member behaviors · 5.AW Address the potential interaction of members outside of the group · 1.N Establish group rules, expectations, and termination criteria · 4.I Discuss transitions in group membership · 6.I Foster the emergence of group therapeutic factors · CACREP: Group Counseling and Group Work

Why Group Gets Its Own Chapter

The content outline lists 12 separate group tasks in the Counseling Skills domain alone (5.AH–5.AJ and 5.AO–5.AW), plus group items in Ethics, Treatment Planning, and Core Attributes. Group work is one of the most heavily specified topics on the NCE.

Chapter 3 gave you the group owners: Tuckman, Corey, Yalom, Lewin, Moreno, Bion, and Luft & Ingham (the Johari Window). This chapter teaches what a group leader actually **does**, and how the NCE tests it.

The central shift: **in group, the group itself is a tool.** A good leader doesn't do individual counseling with an audience. The leader gets members talking to each other, because members learn from each other in ways they can't learn from the leader alone.

Types of Groups (ASGW)

The Association for Specialists in Group Work (ASGW) describes four types.

Type	Purpose	Example	Leader role
Task/work group	Accomplish a goal or product	Committee, team, planning group	Facilitator of process and productivity
Psychoeducational	Teach information and skills; prevention	Anger management, parenting skills, divorce adjustment for children	Teacher; structured
Counseling	Address normal life problems and growth through interpersonal support	Grief support, career transition, adjustment	Facilitator; less structured
Psychotherapy	Treat more serious or longer-standing psychological problems	Group for depression, personality patterns, trauma	Clinician; deeper work

Self-help and support groups (e.g., 12-step groups) are usually peer-led and not run by a counselor.

Structure Choices

Choice	Options	Trade-offs
Open vs. closed	Open: members join and leave over time. Closed: same members from start to end	Open groups have more turnover and lower cohesion but are available when needed. Closed groups build more cohesion and trust

Choice	Options	Trade-offs
Homogeneous vs. heterogeneous	Members share a key characteristic, or differ	Homogeneous groups bond faster and suit focused, short-term goals. Heterogeneous groups offer more perspectives and suit interpersonal learning
Size	Varies by age and type	Smaller for children and intensive work; psychoeducational groups can be larger. Interpersonal process groups often run about 7–8 members (Yalom)
Duration and frequency	Time-limited or ongoing	Children need shorter sessions; adults commonly meet 60–120 minutes
Co-leadership	One leader or two	Co-leaders share the load, model healthy interaction, and offer two perspectives. Co-leaders must meet before and after sessions to plan and process; open disagreement in front of members without repair can harm the group

Before the Group: Screening and Preparation

Screening (ACA A.9.a)

Counselors screen prospective members to select people whose needs and goals fit the group, who won't impede the group process, and whose well-being won't be jeopardized by it.

Members who are often **not** a good fit for a general interpersonal group: - Acutely suicidal clients - Clients in active psychosis or severe crisis - Clients who are actively violent or extremely hostile - Clients whose needs require individual attention first

The pregroup interview is also the member's chance to decide whether the group is right for them.

Protecting Members (ACA A.9.b)

Leaders take reasonable precautions to protect members from physical, emotional, and psychological harm.

Informed Consent and Group Rules (1.N)

Explain at the start: - Purpose, format, and goals of the group - **Confidentiality and its limits**, including that the leader **cannot guarantee** members will keep what's shared private (ACA B.4.a) - Attendance expectations and how to leave the group - Rules about contact between members outside group - Fees, records, and the leader's qualifications - Termination criteria

Confidentiality in group: the leader keeps confidentiality and asks members to do the same, but must tell members plainly that it can't be guaranteed. A wrong NCE answer promises members that everything said in group will stay private.

Stages and Stage-Matched Interventions (5.AS, 5.AU)

This table is Corey's model, with Tuckman's equivalent names. Learn what members do *and* what the leader should do at each stage.

Corey stage (Tuckman)	What members do	What the leader does
Pregroup	(Not yet a group)	Screen, select, prepare members; set goals and structure

Corey stage (Tuckman)	What members do	What the leader does
Initial (forming)	Polite, tentative, testing safety; look to the leader; superficial talk; identify themes	Provide structure; explain process and rules; model openness; build trust; help members set personal goals; use **linking** to show common ground; teach members to give feedback
Transition (storming)	Anxiety, defensiveness, resistance, conflict, challenges to the leader, struggle for control	Normalize conflict; respond to challenges **non-defensively**; encourage members to express reactions directly; work with resistance rather than against it; model direct, respectful conflict
Working (norming/ performing)	Cohesion, trust, genuine self-disclosure, risk-taking, direct feedback, focus on here-and-now, change	Step back and let members lead more; reinforce risk-taking; encourage translating insight into action; process themes; use **here-and-now** work
Final (adjourning)	Sadness and anxiety about ending; consolidating learning; may pull back	Address feelings about ending; review and consolidate learning; plan how to apply changes outside; deal with unfinished business; give and receive final feedback; discuss referrals
Postgroup	—	Follow-up session or contact; evaluate the group

Exam pattern: match the described behavior to the stage, then pick the leader response that fits that stage. A

leader who pushes deep disclosure in session one, or who responds defensively when challenged in the transition stage, is a wrong answer.

Transition-stage challenge example: > A member says in session four, “I don’t see the point of this. You just sit there and don’t tell us anything.”

The best response explores the reaction non-defensively (“It sounds like you’re frustrated with how I’m leading. I’d like to hear more, and I’m curious whether others feel something similar.”). Wrong answers defend the leader’s approach, ignore the challenge, or ask the member to leave.

Leader Skills (5.AH, 5.AI, 5.AJ, 5.AP, 5.AQ)

Skill	What it does	Example
Linking	Connects one member’s concern to another’s; builds universality and member-to-member interaction	“Maria, what you said sounds a lot like what Devon shared last week. Devon, does that fit?”
Blocking	Stops harmful or unproductive behavior **while protecting the member**	“Let me stop you there, Jim. I’d like to hear how you’re feeling, rather than more about what happened.”
Cutting off	Interrupts a member who goes on too long or off topic	“I want to pause here so others have room to respond.”
Drawing out	Invites quieter members in	“Tasha, you’ve been nodding. What’s coming up for you?”
Modeling	Demonstrates the behavior the leader wants members to learn	Giving feedback that’s specific, honest, and kind
Facilitating	Encourages members to talk to each other, not just to the leader	“Instead of telling me, could you tell Ray directly?”

Skill	What it does	Example
Process observation	Comments on *how* the group is interacting, not just *what* it's discussing	"I notice that every time conflict comes up, someone makes a joke."
Universalizing	Highlights shared experience	"Several of you have mentioned feeling invisible at home."
Protecting	Keeps members safe from attacks or pressure	Stepping in when a member is being ganged up on
Supporting	Encourages a member who's taking a risk	"That took courage to say."
Summarizing	Ties together themes at the end of a segment or session	"Today's theme seemed to be trust."
Goal setting	Helps members define specific, workable goals	"What would you like to be different by the end of the group?"
Evaluating	Assesses the group's progress	A check-in on whether the group is meeting its goals
Terminating	Prepares the group and members for ending	Reviewing growth; planning for after the group

Content vs. Process

- **Content** is *what* members talk about (a member's divorce).
- **Process** is *how* members interact while talking about it (who speaks, who's silent, who rescues, how the group reacts).

The NCE rewards attention to process, especially in the working stage. A leader who only discusses content is running individual counseling in a group setting.

Here-and-Now Focus (Yalom)

Yalom describes two tiers: the group first **experiences** the here-and-now (members react to each other in the room), then **illuminates** it (reflects on what just

happened). The group is a **social microcosm**: members eventually show the same interpersonal patterns in group that cause problems outside it, which makes those patterns available to work on directly.

Feedback (5.AJ)

Effective feedback is: - **Specific** and about behavior, not character - **Timely** - **Owned** by the giver ("I felt…" rather than "Everyone thinks…") - **Balanced**, with strengths as well as concerns - Offered in a way the receiver can **use**

The leader models both giving and receiving feedback, including receiving criticism without defensiveness.

Structured Activities (5.AO)

Structured exercises (icebreakers, rounds, role-plays, sentence completions, art activities, psychodrama techniques) are useful especially in the initial stage, in psychoeducational groups, and with children. They lower anxiety and focus the group.

The caution: exercises should serve a purpose and be processed afterward. Overusing them can make the group leader-dependent and keep members from the less structured interaction that builds cohesion. Members can decline to participate in an exercise.

Psychoeducation in Group (5.AR)

In psychoeducational groups, the leader teaches content, but process still matters. Leave time for members to react to and personalize the material, and link it to their experiences.

Yalom's Therapeutic Factors in Action (6.I)

Factor	What it looks like in group
Instillation of hope	Seeing other members improve
Universality	"I'm not the only one"
Imparting information	Psychoeducation from the leader or advice from members

Factor	What it looks like in group
Altruism	Helping another member raises one's own sense of worth
Corrective recapitulation of the primary family group	Re-experiencing family-like dynamics in the group and responding differently
Development of socializing techniques	Learning social skills through feedback
Imitative behavior	Learning by watching the leader or other members
Interpersonal learning	Learning how one affects others and how others affect oneself (input and output)
Group cohesiveness	Belonging and acceptance; the group equivalent of the therapeutic alliance
Catharsis	Emotional release, most helpful when paired with understanding
Existential factors	Accepting responsibility, mortality, and the limits of what others can provide

Cohesiveness is often described as the group equivalent of the alliance in individual counseling, and a precondition for other factors to work.

Challenging Harmful Member Behaviors (5.AV)

Behavior	What it looks like	Leader response
Monopolizing	One member dominates the time	Cut off gently; invite others; explore what monopolizing does for the member and how others feel
Silence/withdrawal	A member rarely speaks	Draw out gently without forcing; respect cultural differences in participation; explore what silence means for the member

Behavior	What it looks like	Leader response
Storytelling	Long narratives about past or outside events	Redirect to feelings and the here-and-now: “How are you feeling as you tell us this?”
Questioning/ interrogating	Firing questions at another member	Ask the questioner to share the statement behind the question
Advice-giving	Rushing to tell others what to do	Encourage sharing personal experience or feelings instead; explore the impulse to fix
Rescuing (“band-aiding”)	Smoothing over another member’s pain too quickly	Block gently; let the member stay with the feeling; explore the rescuer’s discomfort
Hostility/attacking	Sarcasm, put-downs, aggression	**Block** to protect the target; ask for direct, owned feedback; explore the anger
Scapegoating	The group targets one member	**Block and protect**; redirect members to their own feelings; explore what the group is avoiding
Intellectualizing	Talking about feelings in abstract terms	Invite the member to speak personally and in the present
Dependency	Constantly seeking the leader’s approval or answers	Redirect to members; encourage self-reliance
Socializing outside group	Subgroups, relationships, or secrets formed outside sessions	See below

The leader’s priority when a member is attacked: protect. Blocking, not ignoring, is the scored answer.

Contact Outside the Group (5.AW)

Contact between members outside sessions can form **subgroups** that pull energy away from the group, create secrets, and threaten cohesion and safety.

- Set expectations about outside contact **during informed consent**.
- Expectations vary by group type. Outside contact is often discouraged in psychotherapy groups; it may be acceptable or even encouraged in some support and task groups.
- When outside contact happens, the expected norm is that members **bring it back into the group** to discuss, rather than keeping it secret.

Transitions in Membership (4.I)

Situation	Leader response
A new member joins an open group	Prepare the new member beforehand; prepare existing members; introduce and orient the new member; expect a temporary dip in cohesion
A member wants to leave	Encourage the member to tell the group and say goodbye rather than disappear; explore the decision, without coercion (members have the right to leave)
A member stops attending	Follow up; address the absence with the group
The group ends	Allow time to process endings; review progress; plan maintenance; offer referrals; follow up

Multicultural Group Work

ASGW's *Multicultural and Social Justice Competence Principles for Group Workers* (2012) call for leaders to: - Be aware of their own cultural values and biases - Recog-

nize that norms about self-disclosure, eye contact, conflict, and emotional expression vary across cultures - Avoid treating one culture's style of participation as the only "healthy" one - Address microaggressions and power dynamics within the group - Adapt structure, language, and goals to members' cultural contexts

A quiet member from a culture that values restraint in front of strangers is not automatically "resistant."

Group Ethics Summary

Topic	Rule
Screening	Required (ACA A.9.a)
Protecting members	Required (ACA A.9.b)
Confidentiality	Leader explains it **can't be guaranteed** (ACA B.4.a)
Voluntary participation	Members can decline exercises and can leave the group
Leader competence	Leaders need group-specific training and supervision
Dual relationships	Same standards as individual counseling
Minors in groups	Parent/guardian consent; age-appropriate explanation of confidentiality limits
Records	Individual records for each member, with care not to include other members' identifying information

What the Exam Rewards / What It Punishes

Rewards: - Screening before the group begins - Telling members confidentiality can't be guaranteed - Stage-matched leader responses - Responding to challenges in the transition stage without defensiveness - Linking members and promoting member-to-member interaction - Blocking to protect a member who is attacked or scape-

goated - Attending to process, not just content - Bringing outside contact back into the group

Punishes: - Doing individual counseling in front of the group - Promising confidentiality - Pushing deep disclosure in the initial stage - Defending yourself when members challenge the leader - Ignoring scapegoating or attacks - Forcing silent members to speak - Using “contemplation” as a group stage (it’s a stage of change)

Swap-Pair Drill

If you see…	It’s…	Not…
Connecting two members’ concerns	Linking	Universalizing (the leader names a shared theme for the whole group)
Stopping harmful behavior to protect a member	Blocking	Cutting off (managing time or focus)
Inviting a quiet member in	Drawing out	Linking
Smoothing over pain too fast	Rescuing / band-aiding	Supporting
Commenting on how the group interacts	Process observation	Summarizing content
Experiencing, then reflecting on, what just happened in the room	Here-and-now (Yalom)	Catharsis
Group equivalent of the alliance	Cohesiveness	Universality
Learning how one affects others	Interpersonal learning	Imitative behavior
Members can join anytime	Open group	Closed group
Teaching skills, prevention focus	Psychoeducational group	Counseling group
Members test the leader and conflict appears	Transition (Corey) / storming (Tuckman)	Initial / forming

Quick Checks

1. During the first session of a counseling group, the leader explains the group's purpose and rules, including that confidentiality can't be guaranteed. Which stage is this? A. Transition B. Working C. Initial D. Final

2. A member says, "Everything said here stays here, right?" The best leader response is: A. "Yes, everything is completely confidential." B. "I'll keep what you share confidential except where the law requires otherwise, and I'll ask everyone to do the same, but I can't guarantee what other members will do." C. "Confidentiality doesn't apply in groups." D. "That's up to each member to decide."

3. In the fourth session, a member says, "This group is pointless, and you don't know what you're doing." The best leader response is: A. Explain the leader's training and qualifications B. Ask the member to leave if he's not committed C. Acknowledge the frustration non-defensively and invite the group to share reactions D. Move on to the planned activity

4. A member begins crying after describing her father's death. Another member immediately says, "Don't cry, he's in a better place now." The leader should: A. Agree, to comfort the grieving member B. Gently block the rescue and give the grieving member room to stay with her feelings C. Change the topic to reduce distress D. Ask the grieving member to leave until she's composed

5. Several members repeatedly criticize one member for "always complaining." The leader's priority is to: A. Let the group handle it on its own B. Agree that the member complains too much C. Block the scapegoating, protect the member, and explore what the group is reacting to D. Ask the targeted member to change

6. "James, when Lena talked about feeling unseen at work, you nodded a lot. Does that connect to anything for you?" This is: A. Blocking B. Linking C. Cutting off D. Summarizing

7. A leader comments, “I notice that each time someone gets close to talking about anger, someone else changes the subject.” This is: A. Process observation B. Content focus C. Drawing out D. Imparting information

8. Two members admit they’ve been meeting for coffee after group and talking about other members. The best leader response is to: A. Ignore it because it happened outside the group B. Remove both members immediately C. Bring it into the group to discuss its impact, consistent with the rules set at the start D. Meet privately with each member and keep it confidential

9. Which of Yalom’s factors is the group equivalent of the therapeutic alliance? A. Universality B. Catharsis C. Group cohesiveness D. Altruism

10. A prospective member is acutely suicidal and was discharged from the hospital yesterday. Based on screening guidelines, the best decision is to: A. Add her to the group immediately B. Defer group placement and provide individual services until she’s more stable C. Add her but tell no one about her history D. Refuse services entirely

11. Which group type is most appropriate for teaching coping skills to middle school students whose parents are divorcing? A. Psychotherapy group B. Task group C. Psychoeducational group D. Self-help group

12. A new member is joining an ongoing open group. The best leader action is to: A. Add the member without telling the group, so things stay normal B. Prepare the new member and the existing members, then help integrate the new member C. Ask the new member to stay silent for the first few sessions D. Restart the group from the initial stage

Answers and Rationales

1 — C. Orientation, rules, and structure belong to Corey’s initial stage (Tuckman’s forming). A: transition involves conflict and resistance. B: working involves cohesion and risk-taking. D: final involves consolidation and ending.

2 — B. Accurate informed consent: the leader's commitment, its legal limits, and the fact that members' confidentiality can't be guaranteed (ACA B.4.a). A: a promise the leader can't keep. C: confidentiality does apply to the leader. D: abdicates the leader's role in setting norms.

3 — C. Transition-stage challenges are met non-defensively and opened to the group. A: defensive. B: punitive and threatening to the group's safety. D: ignores what's happening in the group.

4 — B. Blocking the band-aid lets the member experience her grief and lets the group learn. A: joins the rescue. C: avoids the feeling. D: shames emotion and harms safety.

5 — C. Protecting a scapegoated member is the leader's priority (ACA A.9.b). A: leaves the member unprotected. B: joins the attack. D: blames the target.

6 — B. Connecting one member's experience to another's is linking. A: blocking stops harmful behavior. C: cutting off manages time or focus. D: summarizing ties together themes.

7 — A. Commenting on how the group interacts is process observation. B: content would be the subject itself (anger at someone). C: drawing out invites a quiet member. D: imparting information is one of Yalom's factors, teaching content.

8 — C. Outside contact is brought back into the group, following norms set at the start. A: ignores a threat to cohesion and trust. B: punitive without exploration. D: creates more secrets.

9 — C. Cohesiveness is the group's version of the alliance. A: universality is "I'm not alone." B: catharsis is emotional release. D: altruism is benefiting from helping others.

10 — B. Screening excludes members whose well-being could be jeopardized or who need individual attention first (ACA A.9.a). A: unsafe. C: deceptive and unsafe. D: abandons her; she still needs services.

11 — C. Teaching coping skills with a prevention focus is a psychoeducational group. A: psychotherapy groups treat more serious problems. B: task groups accomplish a product. D: self-help groups are peer-led.

12 — B. Preparing both the new member and the group supports integration (4.I). A: blindsides the group. C: excludes the new member. D: unnecessary; the group can integrate the member while expecting a temporary dip in cohesion.

Audio Companion

Track	Covers	Target length
8.1	Lead the Group: stages, skills, harmful behaviors, ethics	~7 min

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Chapter 9 — Delivery Formats and Special Populations

Domain map: 5.C Align intervention with client population (e.g., veterans, minorities, disenfranchised, disabled) · 5.G Address addiction issues · 5.H Address cultural considerations · 5.L Explore religious and spiritual values · 5.X Provide distance counseling or telemental health · 1.J Explain uses and limits of social media · 1.M Discuss confidentiality as it applies to electronic communication · 1.R Provide adequate accommodations for clients with disabilities · 6.E Demonstrate knowledge of and sensitivity to multicultural issues · 6.L Respect and acceptance for diversity · CACREP: Social and Cultural Diversity; Counseling and Helping Relationships

One Question Runs Through This Chapter

Every section of this chapter asks the same question: **does this intervention fit this client?** Not just the client's problem, which Chapter 5 covered, but the client's culture, faith, identity, life circumstances, and the format through which counseling is delivered.

The NCE's wrong answers in this area tend to share one flaw: they apply a single standard to everyone. The right answers adapt.

Part 1 — Multicultural Counseling

Owners and Frameworks

Owner	Contribution
C. Gilbert Wrenn (1962)	**Cultural encapsulation**: counselors who substitute their own cultural assumptions for reality and ignore cultural variation

Owner	Contribution
Sue, Arredondo, & McDavis (1992)	Multicultural Counseling Competencies: counselor **awareness** of own values and biases, **knowledge** of the client's worldview, and culturally appropriate intervention **skills**
Ratts, Singh, Nassar-McMillan, Butler, & McCullough (2016)	Multicultural and Social Justice Counseling Competencies (MSJCC): four quadrants based on privilege and marginalization of counselor and client; developmental domains include self-awareness, client worldview, the counseling relationship, and counseling and advocacy interventions
Pamela Hays	**ADDRESSING** framework for cultural influences
Derald Wing Sue	Racial microaggressions; worldview model based on locus of control and locus of responsibility
Norma Day-Vines	**Broaching**: openly raising race, ethnicity, and culture in counseling
Joshua Hook and colleagues	**Cultural humility**: an other-oriented, curious stance that recognizes the limits of one's cultural knowledge
Edward T. Hall	High-context vs. low-context communication
Geert Hofstede	Cultural dimensions, including individualism vs. collectivism and power distance
Lewis, Arnold, House, & Toporek (2002); updated by Toporek & Daniels (2018)	ACA Advocacy Competencies

ADDRESSING (Hays)

Letter	Influence
A	Age and generational influences
D	Developmental disabilities

Letter	Influence
D	Disabilities acquired later in life
R	Religion and spiritual orientation
E	Ethnic and racial identity
S	Socioeconomic status
S	Sexual orientation
I	Indigenous heritage
N	National origin
G	Gender

Key Concepts

Term	Meaning
Etic	A universal approach: the same concepts apply across cultures
Emic	A culture-specific approach: understand the client from within their own culture
Individualism vs. collectivism	Priority on the individual's goals and independence vs. the family or group's goals and interdependence
High-context vs. low-context	Meaning carried in context, relationship, and nonverbals vs. meaning carried in explicit words
Worldview (Sue)	Two dimensions: **locus of control** (internal vs. external: who controls what happens to me?) and **locus of responsibility** (internal vs. external: who is responsible for my situation, me or the system?)
Acculturation	Psychological and cultural change after contact with another culture (Berry's four strategies, Chapter 3)
Acculturative stress	Stress from the process of acculturation

Term	Meaning
Privilege	Unearned advantages based on group membership
Intersectionality (Kimberlé Crenshaw)	Overlapping identities create unique experiences of privilege and oppression
Cultural humility	Ongoing self-reflection and openness, rather than claiming "competence" in a culture

Microaggressions (Sue)

Type	Description	Example
Microassault	Explicit, often conscious, derogatory action	A slur; deliberately avoiding someone
Microinsult	Subtle rudeness or insensitivity that demeans identity	"You're so articulate" to a Black client, implying surprise
Microinvalidation	Negates the thoughts, feelings, or experiences of a person	"I don't see color"; "Where are you *really* from?"

When a counselor commits a microaggression and the client reacts, the scored response is to acknowledge it, take responsibility, explore its impact non-defensively, and repair. Wrong answers deny it, explain the counselor's intent, or move on.

Broaching

Broaching means the counselor raises cultural factors directly rather than waiting for the client to bring them up: "I'm a White counselor, and you've mentioned experiences of racism at work. I'm wondering how it feels to talk about that with me." Done well, broaching builds trust and signals that culture is welcome in the room. The NCE generally rewards respectful broaching over avoidance.

Interpreters

- Use a **trained professional interpreter**.

- **Don't use family members, especially children**, as interpreters. It compromises accuracy, confidentiality, and family roles.
- Speak directly to the client, not to the interpreter.
- Explain the interpreter's confidentiality obligations to the client.

The Cultural Formulation Interview (CFI)

The DSM-5-TR includes a 16-question Cultural Formulation Interview covering four areas: the cultural definition of the problem; cultural perceptions of cause, context, and support; cultural factors affecting self-coping and past help-seeking; and cultural factors affecting current help-seeking. Use it at intake to understand the problem in the client's own terms (Chapter 19).

Advocacy (ACA Advocacy Competencies)

The competencies describe advocacy at three levels, each with acting **with** the client (collaboration, empowerment) or acting **on behalf of** the client:

Level	Acting *with*	Acting *on behalf of*
Client/student	Empowerment: help clients advocate for themselves	Client advocacy: negotiate services for the client
School/community	Community collaboration	Systems advocacy
Public arena	Collective action	Social/political advocacy

Exam principle: where possible, empower clients to advocate for themselves before acting on their behalf.

Part 2 — Religious and Spiritual Values (5.L)

ASERVIC Competencies

The Association for Spiritual, Ethical, and Religious Values in Counseling (ASERVIC) competencies (2009) call on

counselors to: - Explore their own religious and spiritual beliefs and how they might affect counseling - Understand diverse religious and spiritual beliefs, including the absence of belief - Respond to clients' spiritual expressions with acceptance and sensitivity - Recognize when spiritual beliefs are a resource and when they are part of the client's distress - Use spiritual practices in counseling only when they fit the client's beliefs and goals, and with the client's consent

Spiritual Assessment

Ask open questions about whether faith or spirituality matters to the client, how it helps, whether it creates conflict, and whether the client wants it included in counseling. Spiritual and religious resources (community, prayer, meaning) can be strong protective factors.

Fowler's Stages of Faith

James Fowler described six stages of faith development (plus a pre-stage of "primal faith" in infancy): intuitive–projective, mythic–literal, synthetic–conventional, individuative–reflective, conjunctive, and universalizing. This is another owner to know for development items.

Values Imposition and Referral

Standard	Rule
ACA A.4.b	Counselors are aware of their own values and **avoid imposing** them on clients, especially when the counselor's values are inconsistent with the client's goals or discriminatory in nature
ACA A.11.b	Counselors **refrain from referring** clients **based solely on the counselor's personally held values**, attitudes, beliefs, and behaviors

Standard	Rule
ACA C.5	Counselors don't discriminate based on age, culture, disability, ethnicity, race, religion/ spirituality, gender, gender identity, sexual orientation, marital/partnership status, language, socioeconomic status, immigration status, or any basis proscribed by law

The exam pattern: a counselor's personal or religious values conflict with a client's identity or choice (e.g., a gay client seeking relationship help; a client considering divorce). The correct answer is to **seek supervision or consultation, examine one's own values, and continue serving the client competently.** Referring the client away because of the counselor's beliefs is a values-based referral and is the wrong answer. Two well-known court cases involved counseling students who sought to refer LGBTQ clients for values reasons: *Ward v. Wilbanks* and *Keeton v. Anderson-Wiley*.

Referral is appropriate when the issue is competence, not values: a client needs a specialized treatment the counselor isn't trained to provide, and supervision or training can't bridge the gap in time.

Part 3 — Addiction (5.G)

Models of Addiction

Model	Core idea	Owner / association
Moral model	Addiction is a personal failing or choice	Historical; now rejected by the field
Disease model	Addiction is a chronic, progressive disease	E. M. Jellinek (phases of alcoholism); AA
Biopsychosocial model	Biological, psychological, and social factors interact	Current mainstream view

Model	Core idea	Owner / association
Learning/ behavioral model	Use is learned and maintained by reinforcement	Skinner's principles applied
Harm reduction	Reduce the harms of use, whether or not the person is ready to stop	Public health approach (e.g., naloxone access, safer use)

Key Terms

Term	Meaning
Tolerance	Needing more of a substance to get the same effect, or getting less effect from the same amount
Withdrawal	A substance-specific syndrome when use stops or decreases
Cross-tolerance	Tolerance to one substance produces tolerance to another in the same class
Synergism (potentiation)	Two substances together produce a stronger effect than either alone, as with alcohol and benzodiazepines; this can be deadly
Enabling	Others' behavior that shields the person from consequences of use
Codependency	A family member's over-focus on the user, often at their own expense

Alcohol and benzodiazepine withdrawal can be medically dangerous (seizures; delirium tremens with alcohol). A client in significant withdrawal needs medical evaluation; medically managed withdrawal comes before outpatient counseling.

Screening and Assessment

Tool	Use
CAGE	Four alcohol questions: Cut down, Annoyed, Guilty, Eye-opener

Tool	Use
AUDIT (World Health Organization)	10-item alcohol screen
CRAFFT	Adolescent substance use screen (Car, Relax, Alone, Forget, Friends, Trouble)
DAST	Drug Abuse Screening Test
SASSI	Substance Abuse Subtle Screening Inventory
ASI	Addiction Severity Index
SBIRT	Screening, Brief Intervention, and Referral to Treatment: a public health process for identifying and responding to risky use

ASAM Criteria

The American Society of Addiction Medicine (ASAM) Criteria use six dimensions to determine level of care (third-edition names shown; most exam prep materials still use them):

1. Acute intoxication and/or withdrawal potential
2. Biomedical conditions and complications
3. Emotional, behavioral, or cognitive conditions and complications
4. Readiness to change
5. Relapse, continued use, or continued problem potential
6. Recovery/living environment

Levels range from early intervention and outpatient services through intensive outpatient, partial hospitalization, residential, and medically managed intensive inpatient care. The fourth edition (2023) revised some dimension names and level numbers, so check the current edition before quoting exact levels. **The exam principle: match the level of care to the dimensions, and use the least restrictive level that's safe.**

Treatment Approaches

Approach	Notes
Motivational interviewing	Resolve ambivalence (Chapter 5)
CBT and relapse prevention	Coping skills, high-risk situations, abstinence violation effect (Marlatt)
Contingency management	Tangible rewards for verified abstinence
Community Reinforcement Approach (CRA)	Make a sober lifestyle more rewarding than use
CRAFT (Robert Meyers)	Community Reinforcement and Family Training: teaches family members to encourage a loved one into treatment; generally more effective at engaging loved ones than confrontational "Johnson-style" interventions
12-step facilitation	Encourages active involvement in AA/NA; sponsor, meetings, steps
Medications for addiction treatment	For opioid use: methadone, buprenorphine, naltrexone. For alcohol use: naltrexone, acamprosate, disulfiram. Counselors **don't prescribe**; they support, coordinate, and refer
Integrated treatment	For co-occurring mental health and substance use disorders, treat both at the same time, ideally by the same team

Mutual Support

Group	For
Alcoholics Anonymous (AA), Narcotics Anonymous (NA)	People with addiction; 12-step
SMART Recovery	People with addiction; CBT-based, not 12-step
Al-Anon	Family and friends of people with alcohol problems
Alateen	Teens affected by someone's drinking

Group	For
Adult Children of Alcoholics (ACoA)	Adults who grew up with alcoholic or dysfunctional caregivers

Family Roles (Wegscheider-Cruse)

Sharon Wegscheider-Cruse described roles in families affected by addiction: the **dependent** (the user), the **enabler** (chief enabler), the **hero**, the **scapegoat**, the **lost child**, and the **mascot**.

Confidentiality: 42 CFR Part 2

Federal regulations (42 CFR Part 2) give extra confidentiality protection to records from federally assisted substance use disorder programs. A 2024 final rule aligned Part 2 more closely with HIPAA, but its consent requirements remain stricter than general health records in several respects. On the exam: **substance use treatment records generally require specific written consent to disclose.**

Process Addictions

Gambling disorder is the only behavioral addiction included as a diagnosis in the DSM-5-TR's Substance-Related and Addictive Disorders chapter. **Internet gaming disorder** is listed in Section III as a condition for further study. Other compulsive behaviors (sex, pornography, shopping, internet use) are addressed clinically but aren't separate DSM diagnoses.

Part 4 — Distance Counseling and Technology (5.X, 1.J, 1.M)

The ACA Code of Ethics (2014) **Section H** covers distance counseling, technology, and social media.

Standard	Rule
H.1.a — Knowledge and competency	Counselors who use technology develop knowledge and skills in its technical, ethical, and legal aspects
H.1.b — Laws and statutes	Counselors follow the laws of both the counselor's location and the **client's place of residence**
H.2.a — Informed consent	Covers distance-specific issues: credentials and physical location, risks and benefits, possibility of technology failure and backup plans, response time, **emergency procedures when the counselor isn't available**, time zones, cultural and language differences, and possible denial of insurance
H.2.b–d — Confidentiality and security	Inform clients of the limits of confidentiality with technology; use encryption and security measures that meet legal requirements
H.3 — Client verification	Verify the client's identity at the start and throughout (e.g., code words, numbers, graphics)
H.4.c–d — Technology-assisted services; effectiveness	Determine whether distance counseling is appropriate for this client (H.4.c); when it's ineffective, consider face-to-face services (H.4.d)
H.4.e–f	Access to services and accessibility (H.4.e); effects of lacking visual and audio cues (H. 4.f)
H.5 — Records	Maintain electronic records according to law; inform clients about how records are stored
H.6.a — Virtual professional presence	Keep **separate professional and personal** social media accounts
H.6.c — Client privacy on social media	Counselors respect clients' privacy on social media and **don't view clients' social media without consent**

Standard	Rule
H.6.d	Counselors take precautions not to disclose confidential information through social media

Practical Telehealth Rules

- **Licensure follows the client.** Counselors generally must be licensed (or otherwise authorized) in the state where the client is physically located during the session. The interstate **Counseling Compact** is creating a privilege-to-practice system across member states; check its current implementation status before relying on it.
- At the start of each session, **confirm the client's physical location** (for emergencies) and that they're in a private space.
- Have an **emergency plan** in place: the client's local emergency resources and an emergency contact.
- Use **HIPAA-compliant platforms** and a business associate agreement (BAA) with the vendor.
- **Not every client is a good fit.** Acute risk, severe dissociation, or lack of privacy at home may call for in-person care.

Social Media and Electronic Communication

- Don't "friend" or follow current clients on personal accounts (a boundary issue).
- Don't search clients online without consent and a clinical reason.
- Explain in informed consent how email and text will (and won't) be used, response times, and that they're not for emergencies.
- Avoid discussing clinical details by unencrypted email or text.

Artificial Intelligence in Counseling

AI tools (note-writing assistants, chatbots, screening apps, scheduling and billing systems) are spreading

quickly in counseling practice. The 2014 ACA Code of Ethics doesn't mention AI by name, and the NBCC's 2023 NCE content outline doesn't list it, so expect few, if any, direct exam items. But the existing standards for technology, confidentiality, competence, and informed consent apply, and the American Counseling Association's AI Work Group has published recommendations for counselors:

Principle	What it means in practice
Informed consent and transparency	Tell clients when and how AI tools are used (for example, an AI note-taker in session), explain the purpose, benefits, and risks, and get their consent. Clients can decline
Clinical judgment stays human	Don't use AI as the **sole** basis for diagnosis, assessment, or risk decisions. The counselor reviews and is responsible for any AI-generated output, including notes
Privacy and security	Use tools that meet HIPAA and other legal requirements, with a business associate agreement where required. Don't enter identifiable client information into consumer AI tools that aren't built for protected health information
Bias	AI can carry bias from the data it was trained on, with greater risk of harm to marginalized groups. Check output critically
Competence	Understand a tool's capabilities and limits before using it; practice within the boundaries of your competence (ACA C.2.a)
The relationship comes first	AI can support counseling but shouldn't replace the human therapeutic relationship

Principle	What it means in practice
Clients' own AI use	Ask about it. Some clients use AI chatbots for emotional support; explore the benefits and risks with them, including privacy, accuracy, and the absence of crisis response

The exam-style principle: when a new technology appears in a question, apply the standards you already know: informed consent, confidentiality and security, competence, the counselor's responsibility for clinical decisions, and the client's welfare.

Part 5 — Specific Populations (5.C)

Population	What the exam expects
Veterans and military	Understand military culture (mission, unit cohesion, chain of command, stigma about help-seeking); screen for PTSD, traumatic brain injury (TBI), military sexual trauma (MST), substance use, and suicide risk; **moral injury** (distress after acts that violate one's moral code); know VA and Vet Center resources; involve families, who experience deployment too
LGBTQ+ clients	**Affirmative** counseling; use the client's name and pronouns; don't assume sexual orientation is the presenting problem; understand minority stress (Ilan Meyer); conversion/reparative therapy is harmful and contrary to professional standards; ALGBTIC (now SAIGE) competencies

Population	What the exam expects
People with disabilities	Ask the client's preference for person-first ("person with a disability") or identity-first ("Deaf person," "autistic person") language; know the **medical model** (disability as an individual deficit) vs. the **social model** (disability created by environmental and social barriers); provide reasonable accommodations under the **Americans with Disabilities Act (ADA)**; don't assume the disability is the focus of counseling
Deaf clients	Deaf culture (capital D) as a cultural and linguistic identity; use a qualified **ASL interpreter**; ASL is a full language, not signed English
Older adults	See Chapters 5 and 12; screen for cognitive and medical contributors; ageism; elder abuse reporting (Chapter 6)
Immigrants and refugees	Acculturative stress; pre-migration trauma; language access; immigration-related fears; collectivist family structures; practical needs first
People living in poverty	Maslow: basic needs before insight work; reduce barriers (sliding scale, scheduling, transportation, telehealth); case management and advocacy
Rural clients	Fewer resources; stigma; unavoidable overlapping relationships (manage boundaries carefully rather than refusing all contact); telehealth expands access
Justice-involved clients	Mandated treatment (Chapter 4); clarify limits of confidentiality with the court or probation officer during informed consent

Population	What the exam expects
Racial and ethnic minority clients	Racial trauma and race-based stress; mistrust grounded in historical and current experience; broaching; avoid cultural encapsulation

What the Exam Rewards / What It Punishes

Rewards: - Self-awareness and cultural humility - Broaching culture respectfully - Acknowledging and repairing a microaggression - Professional interpreters, not family members - Exploring spiritual values with the client's consent - Supervision when the counselor's values conflict with a client's - Matching addiction treatment level to the ASAM dimensions; medical evaluation for dangerous withdrawal - Telehealth informed consent with emergency plans and location checks - Following the law where the client is located - Informed consent for AI tools; keeping clinical judgment with the counselor

Punishes: - Cultural encapsulation: one standard for everyone - Values-based referrals - Imposing the counselor's beliefs - Using a child as an interpreter - Confrontational interventions and moral judgment in addiction work - "Friending" clients or viewing their social media without consent - Telehealth across state lines without authorization

Swap-Pair Drill

If you see…	It's…	Not…
Counselor imposes own culture as reality	Cultural encapsulation (Wrenn)	Microaggression
Awareness, knowledge, skills	Sue, Arredondo, & McDavis	MSJCC (Ratts et al.)

If you see…	It's…	Not…
Privileged/ marginalized counselor and client quadrants	MSJCC (Ratts et al.)	Sue et al. (1992)
"I don't see color"	Microinvalidation	Microassault
Universal (culture-general) approach	Etic	Emic
Locus of control and locus of responsibility	Sue's worldview model	Rotter's locus of control alone
Raising culture directly in session	Broaching (Day-Vines)	Cultural humility
Faith develops through six stages	Fowler	Kohlberg
Referring a client because of the counselor's beliefs	Values-based referral (prohibited, ACA A. 11.b)	Competence-based referral
Training family to encourage a loved one into treatment	CRAFT (Meyers)	Johnson intervention
CBT-based, non-12-step mutual support	SMART Recovery	AA
Hero, scapegoat, lost child, mascot	Wegscheider-Cruse	Satir's stances
Disability from environmental barriers	Social model	Medical model
Licensure for telehealth	Where the **client** is located	Where the counselor is located only

Quick Checks

1. A counselor says to a Latina client, "Your English is so good!" The client looks uncomfortable. This is an example of: A. A microassault B. A microinsult C. Broaching D. Cultural humility

2. In the situation above, the counselor's best next step is to: A. Explain that it was meant as a compliment B. Move on so the client doesn't feel awkward C. Acknowledge the

comment, take responsibility, and invite the client to share how it landed D. Refer the client to a Latina counselor

3. A counselor with strong religious beliefs is uncomfortable working with a client in a same-sex relationship who wants help with communication. The counselor should: A. Refer the client to another counselor B. Seek supervision, examine personal values, and continue providing competent services C. Tell the client about the counselor's beliefs so the client can decide D. Focus counseling on the client's sexual orientation

4. A family arrives for an intake; the parents speak limited English. The best choice for interpretation is: A. Their 11-year-old son, who is fluent B. A trained professional interpreter C. An English-speaking neighbor D. Gestures and simplified English

5. Which DSM-5-TR tool helps counselors understand a problem in the client's cultural terms? A. MMPI-3 B. Cultural Formulation Interview C. CAGE D. ASI

6. A client reports drinking heavily every day for years and says he plans to stop "cold turkey" tomorrow. The counselor's best action is to: A. Praise the decision and schedule a session next week B. Recommend he wait until he feels more motivated C. Arrange a medical evaluation because alcohol withdrawal can be dangerous D. Refer him to AA only

7. A mother wants help getting her adult son, who uses opioids, into treatment. The approach with the strongest support for engaging him is: A. A surprise confrontational intervention B. CRAFT C. Cutting off all contact until he agrees D. Waiting until he hits rock bottom

8. Before beginning telehealth with a new client, the counselor's informed consent should include: A. A promise that technology is completely secure B. Emergency procedures, risks of technology failure, and backup plans C. The counselor's personal social media handles D. A statement that location doesn't matter

9. A counselor licensed only in Georgia wants to continue video sessions with a client who has permanently moved to Oregon. The counselor should first: A. Continue, since the counselor is licensed in Georgia B. Determine whether Oregon law permits the counselor to practice with the client there, and arrange a transition if not C. Switch to phone sessions, which aren't regulated D. Ask the client to give a Georgia address

10. A counselor looks up a new client's Instagram account to "understand her better." Under the ACA Code, this is: A. Recommended to build rapport B. Acceptable if the account is public C. Inconsistent with respecting client privacy on social media without consent D. Required as part of intake

11. Which framework lists Age, Developmental and acquired Disabilities, Religion, Ethnicity, Socioeconomic status, Sexual orientation, Indigenous heritage, National origin, and Gender? A. MSJCC B. ADDRESSING C. ASERVIC D. CFI

12. A veteran describes intense guilt over actions he took during combat that violated his own moral beliefs. This is best described as: A. Military sexual trauma B. Moral injury C. Traumatic brain injury D. Survivor stress

13. A client mentions she's been talking to an AI chatbot "every night" about her anxiety and finds it helpful. The counselor's best response is to: A. Tell her to stop, since chatbots are unethical B. Ignore it, since it happens outside of counseling C. Ask about her experience with it and explore the benefits and risks, including privacy and the lack of crisis response D. Recommend she replace counseling with the chatbot

Answers and Rationales

1 — B. A comment that conveys surprise about a person's ability based on ethnicity is a microinsult (Sue). A: a microassault is explicit and deliberate. C: broaching is raising culture respectfully on purpose. D: cultural humility is a stance of openness, not a remark.

2 — C. Acknowledge, take responsibility, and explore impact. A: defends intent over impact. B: avoids repair. D: unnecessary; a rupture can be repaired.

3 — B. ACA A.4.b and A.11.b: avoid imposing values and don't refer based solely on personal values; seek supervision. A: a values-based referral. C: burdens the client with the counselor's beliefs. D: the client's orientation isn't the presenting concern.

4 — B. A professional interpreter protects accuracy, confidentiality, and family roles. A: using a child reverses family roles and exposes the child to adult material. C: confidentiality and accuracy concerns. D: inadequate communication.

5 — B. The CFI is the DSM-5-TR's cultural assessment. A: the MMPI-3 is a personality inventory. C: CAGE is an alcohol screen. D: the ASI measures addiction severity.

6 — C. Abrupt cessation after heavy, long-term drinking can cause seizures or delirium tremens; medical evaluation comes first (ASAM Dimension 1). A: ignores medical risk. B: discourages change. D: AA can be part of recovery but doesn't address withdrawal risk.

7 — B. CRAFT (Meyers) has strong support for engaging treatment-refusing loved ones. A: confrontational interventions are generally less effective at engagement. C and D: withdraw support without evidence of benefit.

8 — B. ACA H.2.a requires distance-specific informed consent, including emergency and technology-failure plans. A: no system is completely secure. C: separate professional and personal presence (H.6.a). D: location matters for law and emergencies.

9 — B. Counselors must be authorized where the client is located, and H.1.b makes them subject to the laws of both their own location and the client's place of residence. A: Georgia licensure alone doesn't authorize practice in Oregon. C: phone sessions are still counseling. D: dishonest and unethical.

10 — C. ACA H.6.c: respect clients' privacy on social media unless given consent. A, B, D: not supported by the Code.

11 — B. ADDRESSING (Hays). A: MSJCC (Ratts et al.) is built on privilege and marginalization quadrants. C: ASERVIC covers spiritual competencies. D: the CFI is the DSM-5-TR interview.

12 — B. Distress after acts that violate one's own moral code is moral injury. A: MST is sexual assault or harassment during military service. C: TBI is a physical brain injury. D: not a standard term for this experience.

13 — C. The ACA AI Work Group recommends asking about clients' AI use and exploring benefits and risks; this also respects autonomy. A: imposes the counselor's view without exploring. B: misses information relevant to her care and safety. D: AI shouldn't replace the human counseling relationship.

Audio Companion

Track	Covers	Target length
9.1	Fit the Client: culture, faith, addiction, telehealth, populations	~7 min

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Chapter 10 — Mood, Anxiety, Obsessive, and Trauma Presentations

Domain map: 3.H Fear and panic · 3.L Hopelessness/depression · 3.T Obsessive thoughts/behaviors · 3.V Physical issues related to anxiety · 3.W Physical issues related to depression · 3.X Physical/emotional issues related to trauma · 3.AC Ruminating and/or intrusive thoughts · 3.AM Worry and anxiety · 3.AX Emotional dysregulation · 2.E Determine diagnosis · 2.G Consider co-occurring diagnoses · 2.O Assess for trauma · CACREP: Counseling and Helping Relationships; Assessment and Testing; Human Growth and Development

How the NCE Tests Clinical Focus

Areas of Clinical Focus is the second-largest domain: 47 of the 160 scored items. Unlike the NCMHCE, the NCE doesn't hand you a diagnosis and walk you through a case. It asks single questions about **presenting concerns**: what a symptom pattern suggests, how long symptoms must last, which screening tool fits, which theory explains the problem, and which treatment approach has the best support.

This chapter covers the presentations that bring the most clients to counseling: depression, mood swings, worry, panic, fear, obsessions, trauma, and emotional dysregulation. For each one you'll learn four things:

1. **What it looks like** and the key time frames
2. **Who owns the theory** that explains it
3. **How to screen** for it
4. **What helps**, and where the counselor's scope ends

Time frames are high-yield. Many NCE diagnostic items turn on duration alone. They're collected in one table at the end of the chapter.

Part 1 — Depression and Hopelessness (3.L, 3.W, 3.AC)

Presentations

Presentation	Key features	Time frame
Major depressive episode	At least **5 of 9** symptoms, including **depressed mood or loss of interest/ pleasure (anhedonia)**; clinically significant distress or impairment	At least **2 weeks**
Persistent depressive disorder (dysthymia)	Depressed mood most of the day, more days than not, plus at least 2 other symptoms	At least **2 years** in adults (**1 year** in children and adolescents, and mood can be irritable)
Premenstrual dysphoric disorder	Mood symptoms in the final week before menses that improve after onset	Most cycles in the past year
Disruptive mood dysregulation disorder (DMDD)	Severe recurrent temper outbursts (3 or more per week) with persistently irritable mood between them	At least **12 months**; not diagnosed before age 6 or after age 18; onset before age 10
Adjustment disorder with depressed mood	Emotional or behavioral symptoms in response to an identifiable stressor, not meeting criteria for another disorder	Begins within **3 months** of the stressor; resolves within **6 months** after the stressor ends

The nine symptoms of a major depressive episode (a common memory aid is **SIG E CAPS** plus depressed

mood): **S**leep change, loss of **I**nterest, **G**uilt or worthlessness, low **E**nergy, poor **C**oncentration, **A**ppetite or weight change, **P**sychomotor agitation or slowing, **S**uicidal thoughts, and depressed mood.

Useful specifiers: - **With peripartum onset**: onset during pregnancy or within 4 weeks after delivery - **With seasonal pattern**: a regular relationship between episodes and a particular time of year - **With anxious distress**, **with melancholic features**, **with psychotic features**

Grief and depression: the DSM-5 removed the "bereavement exclusion," so major depression can be diagnosed during bereavement if criteria are met. Normal grief tends to come in waves tied to reminders of the loss, with self-esteem usually preserved; depression is more pervasive, with worthlessness and self-loathing. Prolonged grief disorder is covered in Chapter 12.

Physical Signs of Depression (3.W)

Sleep changes, appetite and weight changes, fatigue, psychomotor slowing or agitation, unexplained aches and pain, low libido, and slowed thinking. **Medical conditions can cause or mimic depression**, especially hypothyroidism, anemia, vitamin deficiencies, and some medications. A counselor who sees new depressive symptoms should make sure the client has had a recent medical evaluation.

Who Owns the Theories of Depression

Owner	Theory	Core idea
Aaron Beck	Cognitive theory	Cognitive triad (negative views of self, world, future); distortions; schemas
Martin Seligman	**Learned helplessness**	Repeated uncontrollable bad events teach that action is useless

Owner	Theory	Core idea
Abramson, Seligman, & Teasdale	Reformulated helplessness (attributional) theory	Depression follows attributing bad events to **internal, stable, global** causes
Abramson, Metalsky, & Alloy	**Hopelessness theory**	Expecting bad outcomes with no way to change them leads to a hopelessness subtype of depression
Peter Lewinsohn	Behavioral theory	Too little **response-contingent positive reinforcement**; basis for behavioral activation
Susan Nolen-Hoeksema	**Response styles theory**	**Rumination** (repetitively focusing on distress and its causes) prolongs and deepens depression; distraction and problem-solving shorten it
Klerman & Weissman	Interpersonal therapy (IPT)	Depression linked to four problem areas: grief, role disputes, role transitions, interpersonal deficits
John Bowlby	Attachment	Early loss and insecure attachment increase vulnerability

Rumination (3.AC) is repetitive, passive dwelling on one's distress. It's the core target in rumination-focused CBT and mindfulness-based cognitive therapy (MBCT; Segal, Williams, & Teasdale), which was developed to prevent depressive relapse.

Screening Tools

Tool	Notes
PHQ-9 (Kroenke, Spitzer, & Williams)	9 items matching the DSM symptoms. **Item 9 asks about thoughts of death or self-harm; a positive answer requires a suicide risk assessment**
PHQ-2	First two PHQ-9 items as a quick screen
BDI-II (Beck)	21-item self-report
Beck Hopelessness Scale	Hopelessness as a suicide risk indicator
Hamilton Depression Rating Scale (HAM-D)	Clinician-rated
Edinburgh Postnatal Depression Scale (EPDS)	Perinatal depression
Geriatric Depression Scale (GDS)	Older adults
Children's Depression Inventory (CDI)	Children and adolescents

What Helps

- **CBT**, **behavioral activation**, and **IPT** have strong support.
- **MBCT** for preventing relapse after recovery.
- **Combined psychotherapy and medication** often works better than either alone for moderate to severe depression.
- **Always assess suicide risk** (Chapter 6).
- Counselors don't prescribe. Refer for a medication evaluation when symptoms are severe, don't respond to counseling, or include psychotic features. Know that antidepressants carry an FDA boxed warning about increased suicidal thoughts and behaviors in children, adolescents, and young adults under 25, so monitoring early in treatment matters.

Part 2 — Mood Swings: Bipolar Spectrum

Presentation	Key features	Time frame
Manic episode	Abnormally elevated, expansive, or irritable mood **and** increased energy or activity, plus at least 3 symptoms (4 if mood is only irritable); marked impairment, hospitalization, or psychotic features	At least **1 week** (any duration if hospitalized)
Hypomanic episode	Same symptom pattern, but **not** severe enough for marked impairment or hospitalization, and **no psychotic features**	At least **4 consecutive days**
Bipolar I	At least one manic episode (depression common but not required)	—
Bipolar II	At least one hypomanic episode **and** at least one major depressive episode; **never** a full manic episode	—
Cyclothymic disorder	Many periods of hypomanic and depressive symptoms that don't meet full episode criteria	At least **2 years** in adults (**1 year** in children and adolescents)

Manic symptoms (a common memory aid is **DIG FAST**): **D**istractibility, **I**mpulsivity or risky activities, **G**randiosity, **F**light of ideas or racing thoughts, increased goal-directed **A**ctivity or agitation, decreased need for **S**leep, **T**alkativeness or pressured speech.

Why it matters for counselors: - Ask about past manic or hypomanic periods before treating depression. Antidepressants alone can trigger mania in people with bipolar

disorder. - Bipolar disorder needs a medication evaluation. Mood stabilizers (such as lithium) are a mainstay; counseling supports adherence, sleep regularity, early warning signs, and family psychoeducation. - **Interpersonal and social rhythm therapy** (IPSRT; Ellen Frank) stabilizes daily routines and sleep. - Screening: **Mood Disorder Questionnaire (MDQ)**.

Part 3 — Worry, Fear, and Panic (3.H, 3.V, 3.AM)

Fear vs. Anxiety

- **Fear** is the response to a real or perceived **immediate** threat.
- **Anxiety** is anticipation of a **future** threat.
- **Worry** is the thinking part of anxiety: repetitive "what if" thoughts about possible bad outcomes.

Presentations

Presentation	Key features	Time frame
Generalized anxiety disorder	Excessive, hard-to-control worry about many things, plus at least 3 of 6 symptoms (restlessness, fatigue, poor concentration, irritability, muscle tension, sleep problems); 1 symptom needed in children	At least **6 months**
Panic disorder	**Recurrent unexpected** panic attacks, followed by persistent worry about more attacks or a change in behavior to avoid them	At least **1 month** of worry or behavior change after an attack

Presentation	Key features	Time frame
Panic attack (specifier, not a disorder)	Abrupt surge of intense fear peaking within minutes, with at least 4 of 13 symptoms	Peaks within minutes
Agoraphobia	Fear or avoidance of at least **2 of 5** situations (public transportation, open spaces, enclosed spaces, lines or crowds, being outside home alone) because escape or help might be hard	Typically **6 months** or more
Specific phobia	Marked fear of a specific object or situation (animal, natural environment, blood-injection-injury, situational, other)	Typically **6 months** or more
Social anxiety disorder	Fear of social situations where one might be scrutinized or judged negatively; **performance only** specifier	Typically **6 months** or more
Separation anxiety disorder	Excessive fear about separation from attachment figures	At least **4 weeks** in children and adolescents; typically **6 months** or more in adults
Selective mutism	Consistently not speaking in specific situations (such as school) despite speaking in others	At least **1 month** (not just the first month of school)

Blood-injection-injury phobia is the one phobia that often involves a **vasovagal response**: heart rate and blood pressure rise briefly, then drop, and the person may faint. Treatment can include **applied tension** (Öst), tensing muscles to keep blood pressure up during exposure.

Physical Signs of Anxiety (3.V)

Racing or pounding heart, shortness of breath, chest tightness, dizziness, trembling, sweating, nausea and GI upset, muscle tension, headaches, and sleep problems. **Medical rule-outs matter**: hyperthyroidism, cardiac conditions, asthma, low blood sugar, caffeine, stimulants, and substance withdrawal can all cause anxiety symptoms. A first panic attack, especially with chest pain, warrants medical evaluation.

Who Owns the Theories of Anxiety

Owner	Theory	Core idea
O. Hobart Mowrer	**Two-factor theory**	Fear is **acquired** through classical conditioning and **maintained** through operant conditioning (avoidance is negatively reinforced because it reduces anxiety)
David Clark	Cognitive model of panic	**Catastrophic misinterpretation** of normal body sensations ("My heart is racing, so I'm having a heart attack")
David Barlow	Panic control treatment; Unified Protocol	Interoceptive exposure; transdiagnostic treatment for emotional disorders
Thomas Borkovec	Avoidance theory of worry	Worry is a mostly verbal activity that dampens emotional and physical arousal, so it's negatively reinforced and kept going
Michel Dugas	Intolerance of uncertainty model	People with GAD find uncertainty especially hard to tolerate

Owner	Theory	Core idea
Adrian Wells	Metacognitive model	Beliefs *about* worry ("Worrying keeps me prepared"; "My worry is uncontrollable") drive GAD
Clark & Wells	Cognitive model of social anxiety	Self-focused attention and safety behaviors maintain social anxiety
Martin Seligman	**Preparedness**	People are biologically prepared to fear some things (snakes, heights) more easily than others
Yerkes & Dodson	**Yerkes–Dodson law**	Performance improves with arousal up to a point, then declines

Screening Tools

Tool	Notes
GAD-7 (Spitzer et al.)	7-item anxiety screen
BAI (Beck)	21-item self-report focused on physical symptoms
Hamilton Anxiety Rating Scale (HAM-A)	Clinician-rated
State-Trait Anxiety Inventory (STAI) (Spielberger)	Separates **state** anxiety (right now) from **trait** anxiety (general tendency)
Liebowitz Social Anxiety Scale	Social anxiety
SCARED	Child anxiety

What Helps

- **Exposure** is central for phobias, panic, and social anxiety (Chapter 5): graded exposure, interoceptive exposure for panic, and in vivo exposure for agoraphobia.

- **CBT** for GAD and panic: cognitive restructuring, worry exposure, problem-solving, relaxation, and tolerance of uncertainty.
- **Reduce safety behaviors and avoidance**, which maintain the anxiety cycle.
- **Benzodiazepines** relieve anxiety quickly but carry risks of dependence and withdrawal and can interfere with exposure-based learning. This is a medication-prescriber decision; the counselor's role is to coordinate care and educate.

Part 4 — Obsessions and Intrusive Thoughts (3.T, 3.AC)

Presentations

Presentation	Key features
Obsessive-compulsive disorder	**Obsessions** (intrusive, unwanted thoughts, urges, or images that cause anxiety) and/or **compulsions** (repetitive behaviors or mental acts done to reduce anxiety or prevent a feared outcome); time-consuming (often over 1 hour a day) or impairing. Insight specifiers: **good or fair**, **poor**, **absent/ delusional**. **Tic-related** specifier
Body dysmorphic disorder	Preoccupation with perceived flaws in appearance that others don't see or see as slight; repetitive checking, grooming, or comparing
Hoarding disorder	Persistent difficulty discarding possessions, regardless of value, leading to cluttered living areas
Trichotillomania	Recurrent hair pulling with attempts to stop
Excoriation disorder	Recurrent skin picking with attempts to stop

Common obsession themes: contamination, harm (fear of hurting others), symmetry and exactness, unacceptable sexual thoughts, religious or moral obsessions (scrupulosity), and doubt.

Obsessions vs. Similar Experiences

Experience	How it differs from an obsession
GAD worry	Worry is about real-life concerns (money, health, family) and feels reasonable to the person; obsessions are often intrusive, unwanted, and feel alien (**ego-dystonic**)
Depressive rumination	Rumination dwells on past losses and one's own distress; it isn't usually experienced as intrusive and doesn't drive compulsions
Obsessive-compulsive personality disorder (OCPD)	A pervasive pattern of perfectionism, orderliness, and control that the person usually sees as right (**ego-syntonic**); no true obsessions or compulsions
Delusions	Fixed false beliefs held with full conviction; OCD obsessions are usually recognized as excessive (except with absent insight)
Intrusive trauma memories	Tied to a specific traumatic event (PTSD)

Who Owns the Theories of OCD

Owner	Contribution
Stanley Rachman	Cognitive theory: everyone has intrusive thoughts; OCD develops when the person interprets them as meaningful or dangerous
Paul Salkovskis	**Inflated responsibility**: belief that one is responsible for preventing harm drives neutralizing and compulsions

Owner	Contribution
Mowrer	Two-factor theory applies: compulsions are negatively reinforced because they reduce anxiety
Victor Meyer	Early development of exposure and response prevention
Edna Foa	Developed and studied ERP extensively

Screening and Treatment

- **Y-BOCS** (Yale-Brown Obsessive Compulsive Scale; Goodman et al.); **CY-BOCS** for children.
- **ERP** has the strongest support (Chapter 5).
- **SSRIs** (often at higher doses) are a common medication approach; this is a prescriber decision.
- **Reassurance is a compulsion.** Repeatedly reassuring a client with OCD ("No, you didn't hit anyone with your car") reduces anxiety short-term but strengthens the cycle. The scored answer helps the client tolerate uncertainty instead.

Part 5 — Trauma (3.X, 2.O)

Presentations

Presentation	Key features	Time frame
Acute stress disorder	Trauma symptoms right after exposure	**3 days to 1 month** after the trauma
Posttraumatic stress disorder	Exposure to actual or threatened death, serious injury, or sexual violence, followed by symptoms in **four clusters**	More than **1 month**
Adjustment disorder	Distress after a stressor that need not be traumatic	Begins within **3 months**; resolves within **6 months** after the stressor ends

Presentation	Key features	Time frame
Reactive attachment disorder	Inhibited, emotionally withdrawn behavior toward caregivers after severe neglect	Evident before age 5; developmental age of at least 9 months
Disinhibited social engagement disorder	Overly familiar behavior with unfamiliar adults after severe neglect	Developmental age of at least 9 months

PTSD's Four Symptom Clusters

Cluster	Examples
Intrusion	Intrusive memories, nightmares, flashbacks, distress at reminders, physical reactions to reminders
Avoidance	Avoiding memories, thoughts, feelings, people, places, or situations linked to the trauma
Negative alterations in cognition and mood	Amnesia for parts of the event, persistent negative beliefs, distorted blame, persistent negative emotions, loss of interest, detachment, inability to feel positive emotions
Alterations in arousal and reactivity	Irritability or anger, reckless behavior, hypervigilance, exaggerated startle, poor concentration, sleep problems

Specifiers: **with dissociative symptoms** (depersonalization or derealization) and **with delayed expression** (full criteria not met until at least 6 months after the event). There are separate criteria for children 6 and younger.

Exposure can be direct or indirect: experiencing the event, witnessing it, learning it happened to a close family member or friend (if violent or accidental), or repeated or extreme exposure to aversive details (as for first responders). Exposure through media doesn't count unless it's work-related.

Who Owns Trauma Theory

Owner	Contribution
Judith Herman	*Trauma and Recovery*; **complex trauma** from prolonged, repeated trauma; three stages of recovery: **(1) safety and stabilization, (2) remembrance and mourning, (3) reconnection**
Bessel van der Kolk	Trauma's effects on the body and brain; *The Body Keeps the Score*
Daniel Siegel	**Window of tolerance**: the zone of arousal where a person can function; trauma narrows it, causing swings into **hyperarousal** (anxiety, panic, anger) or **hypoarousal** (numbness, shutdown, dissociation)
Felitti & Anda	The **Adverse Childhood Experiences (ACE) Study** (1998): more childhood adversity is linked to higher risk of later physical and mental health problems
Ronnie Janoff-Bulman	**Shattered assumptions**: trauma breaks basic beliefs that the world is benevolent and meaningful and that the self is worthy
Tedeschi & Calhoun	**Posttraumatic growth**: positive change that can follow struggle with trauma
George Bonanno	**Resilience** is the most common response to potential trauma
Stephen Porges	Polyvagal theory

Physical and Emotional Effects of Trauma (3.X)

Hyperarousal, sleep disturbance, chronic pain, headaches, GI problems, fatigue, startle, numbing, shame, guilt, anger, difficulty trusting, and dissociation. The ACE research links childhood adversity to later heart disease, substance use, depression, and other health problems.

Trauma-Informed Care (SAMHSA)

SAMHSA describes six principles: **safety**; **trustworthiness and transparency**; **peer support**; **collaboration and mutuality**; **empowerment, voice, and choice**; and **cultural, historical, and gender issues**. Its four "Rs": a trauma-informed program **realizes** the impact of trauma, **recognizes** its signs, **responds** by building that knowledge into practice, and **resists re-traumatization**.

Screening and Treatment

Tool	Notes
PCL-5	PTSD Checklist for DSM-5; 20-item self-report
CAPS-5	Clinician-Administered PTSD Scale; structured interview, the standard for diagnosis in research
PC-PTSD-5	Brief primary care screen
Life Events Checklist (LEC-5)	Identifies trauma exposure
ACE questionnaire	Childhood adversity history

Treatments with strong support: prolonged exposure (Foa), cognitive processing therapy (Resick), EMDR (Shapiro), and trauma-focused CBT for children and adolescents (Cohen, Mannarino, & Deblinger). See Chapter 5.

The phase-oriented rule: stabilize before processing. A client who is unsafe, using substances heavily, or frequently dissociating needs safety and coping skills first (Herman's stage 1). Pushing for a detailed trauma narrative before the client can stay within the window of tolerance is a scored error.

Part 6 — Emotional Dysregulation (3.AX)

Emotional dysregulation is difficulty managing the intensity and duration of emotions: quick, intense reactions; slow return to baseline; and impulsive behavior to escape the feeling. It appears across many presentations, includ-

ing borderline personality disorder, PTSD and complex trauma, bipolar disorder, ADHD, and DMDD in children.

Owner	Contribution
Marsha Linehan	**Biosocial theory**: emotional vulnerability (biological sensitivity) plus an **invalidating environment** produces pervasive emotion dysregulation; DBT
James Gross	**Process model of emotion regulation**: situation selection, situation modification, attentional deployment, cognitive change (reappraisal), response modulation (e.g., suppression)
Daniel Siegel	Window of tolerance; "name it to tame it" (labeling emotions helps regulate them)

What helps: DBT skills (mindfulness, distress tolerance, emotion regulation, interpersonal effectiveness), validation, grounding, the window of tolerance as a shared language, and attention to safety when dysregulation includes self-harm or suicidal behavior (Chapter 6).

Time Frames at a Glance

Presentation	Time frame
Major depressive episode	2 weeks
Persistent depressive disorder	2 years adults; 1 year children/adolescents
Manic episode	1 week (any duration if hospitalized)
Hypomanic episode	4 days
Cyclothymic disorder	2 years adults; 1 year children/adolescents
DMDD	12 months; diagnosed ages 6–18; onset before 10
GAD	6 months

Presentation	Time frame
Panic disorder	1 month of worry/behavior change after attacks
Specific phobia, social anxiety, agoraphobia	Typically 6 months
Separation anxiety	4 weeks (children/adolescents); typically 6 months (adults)
Selective mutism	1 month (not the first month of school)
Acute stress disorder	3 days to 1 month after trauma
PTSD	More than 1 month; delayed expression if criteria met 6+ months later
Adjustment disorder	Onset within 3 months; resolves within 6 months after stressor ends

What the Exam Rewards / What It Punishes

Rewards: - Knowing the time frames - Asking about past mania before treating depression - Medical rule-outs for new depression or anxiety symptoms - Following up any positive PHQ-9 item 9 with a suicide risk assessment - Exposure for phobias, panic, and OCD; reducing avoidance and safety behaviors - Stabilizing before trauma processing - Validation and skills for emotional dysregulation

Punishes: - Reassuring a client with OCD repeatedly - Pushing trauma detail before safety and stabilization - Assuming grief rules out depression - Treating anxiety only with relaxation when exposure is indicated - Recommending or adjusting medications (outside scope)

Swap-Pair Drill

If you see...	It belongs to...	Not...
Learned helplessness	Seligman	Beck
Internal, stable, global attributions	Abramson, Seligman, & Teasdale	Lewinsohn

If you see…	It belongs to…	Not…
Too little response-contingent reinforcement	Lewinsohn	Seligman
Rumination prolongs depression	Nolen-Hoeksema	Beck
Fear acquired by classical, maintained by operant	Mowrer's two-factor theory	Wolpe
Catastrophic misinterpretation of body sensations	Clark	Borkovec
Worry as avoidance of emotional arousal	Borkovec	Clark
Intolerance of uncertainty	Dugas	Wells
Beliefs about worry	Wells (metacognitive)	Dugas
Inflated responsibility in OCD	Salkovskis	Rachman
Safety, remembrance and mourning, reconnection	Herman	Kübler-Ross
Window of tolerance	Siegel	Porges
ACE Study	Felitti & Anda	van der Kolk
Shattered assumptions	Janoff-Bulman	Tedeschi & Calhoun
Posttraumatic growth	Tedeschi & Calhoun	Bonanno
Biosocial theory of dysregulation	Linehan	Gross
Process model of emotion regulation	Gross	Linehan
State vs. trait anxiety	Spielberger (STAI)	Beck (BAI)
Ego-dystonic obsessions	OCD	OCPD (ego-syntonic)

Quick Checks

1. A client reports 3 weeks of depressed mood, insomnia, poor appetite, fatigue, trouble concentrating, and feelings

of worthlessness. This best fits: A. Persistent depressive disorder B. A major depressive episode C. Cyclothymic disorder D. Adjustment disorder

2. A client seeking help for depression says that last spring she slept 3 hours a night for 5 days, felt “on top of the world,” and started three businesses, but kept working normally. The counselor should first: A. Begin CBT for depression B. Explore this history further because it suggests possible hypomania C. Recommend an antidepressant D. Diagnose persistent depressive disorder

3. The theory that depression results from attributing failures to internal, stable, and global causes belongs to: A. Lewinsohn B. Nolen-Hoeksema C. Abramson, Seligman, and Teasdale D. Klerman and Weissman

4. A client has had sudden episodes of racing heart, shortness of breath, and fear of dying, “out of the blue,” and now avoids driving in case one happens. The most likely presentation is: A. GAD B. Panic disorder C. Specific phobia D. Social anxiety disorder

5. According to Mowrer’s two-factor theory, avoidance maintains fear because it is: A. Positively reinforced B. Negatively reinforced C. Punished D. Extinguished

6. A client with OCD asks the counselor several times per session whether she might have accidentally poisoned her family. The best counselor response is to: A. Reassure her each time that she didn’t B. Help her notice the reassurance-seeking and tolerate the uncertainty C. Refer her for a psychotic disorder evaluation D. Explore the childhood origins of her fear

7. Symptoms began 10 days after a car accident and include nightmares, avoidance of driving, and hypervigilance. The most fitting presentation is: A. PTSD B. Acute stress disorder C. Adjustment disorder D. Specific phobia

8. A client with complex trauma, frequent dissociation, and recent self-harm asks to “talk through everything that happened” in the first session. The best approach is to: A.

Begin a detailed trauma narrative right away B. Focus first on safety, stabilization, and coping skills C. Refer the client to inpatient care D. Tell the client she isn't ready for counseling

9. A positive response on PHQ-9 item 9 requires the counselor to: A. Score the remaining items first B. Conduct a suicide risk assessment C. Refer immediately for hospitalization D. Repeat the PHQ-9 next week

10. Daniel Siegel's window of tolerance describes: A. The range of arousal in which a person can function and process experience B. Stages of grief C. The period after trauma before PTSD can be diagnosed D. Tolerance to a substance

11. A client believes, "If I stop worrying, something bad will happen to my kids." This belief is central to which model? A. Clark's cognitive model of panic B. Wells' metacognitive model C. Seligman's preparedness D. Linehan's biosocial theory

12. A new client describes sudden anxiety, weight loss, heat intolerance, and a racing heart. The counselor's most important step is to: A. Teach diaphragmatic breathing B. Ensure she gets a medical evaluation C. Start exposure therapy D. Diagnose GAD

Answers and Rationales

1 — B. Five or more symptoms including depressed mood for at least 2 weeks. A: PDD requires 2 years. C: cyclothymia requires 2 years of fluctuating hypomanic and depressive symptoms. D: adjustment disorder requires an identifiable stressor and not meeting criteria for another disorder.

2 — B. Four or more days of decreased need for sleep, elevated mood, and increased goal-directed activity without marked impairment suggests hypomania; exploring this is essential before treating depression. A: risks missing bipolar II. C: outside scope, and antidepressants alone can trigger mania. D: ignores the hypomanic history.

3 — C. The reformulated helplessness (attributional) theory. A: Lewinsohn emphasizes low reinforcement. B: Nolen-Hoeksema emphasizes rumination. D: Klerman and Weissman developed IPT.

4 — B. Recurrent unexpected attacks followed by behavior change. A: GAD involves ongoing worry, not discrete unexpected attacks. C: a phobia is triggered by a specific object or situation. D: social anxiety centers on fear of judgment.

5 — B. Avoidance removes anxiety, which negatively reinforces it. A: nothing pleasant is added. C: punishment would decrease avoidance. D: extinction would weaken fear, which avoidance prevents.

6 — B. Reassurance is a compulsion; tolerating uncertainty is the therapeutic goal. A: strengthens the cycle. C: she recognizes the fear as excessive, which doesn't suggest psychosis. D: misses the maintaining cycle.

7 — B. Symptoms between 3 days and 1 month after trauma. A: PTSD requires more than 1 month. C: adjustment disorder applies when criteria for a trauma disorder aren't met. D: a phobia doesn't explain the nightmares and hypervigilance.

8 — B. Herman's stage 1: safety and stabilization first. A: risks overwhelming the client and pushing her outside her window of tolerance. C: not indicated without imminent risk. D: abandons the client.

9 — B. Item 9 screens for thoughts of death or self-harm and requires follow-up assessment. A: delays assessing safety. C: over-response before assessment. D: ignores a possible risk.

10 — A. The optimal arousal zone (Siegel). B: stage models of loss belong to Kübler-Ross (dying) or Bowlby (mourning), not Siegel. C: the 1-month PTSD threshold is a DSM time frame. D: tolerance is a substance term.

11 — B. Positive beliefs about worry are central to Wells' metacognitive model. A: Clark's model concerns cata-

strophic misreading of body sensations. C: preparedness concerns biologically easy-to-learn fears. D: Linehan's theory concerns emotion dysregulation.

12 — B. These symptoms suggest a possible medical cause (such as hyperthyroidism); rule it out first. A and C: treat before ruling out a medical cause. D: diagnoses without ruling out medical conditions and without 6 months of worry.

Audio Companion

Track	Covers	Target length
10.1	Time Frames and Owners: mood, anxiety, OCD, trauma	~8 min

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Chapter 11 — Substance Use and Process Addictions

Domain map: 3.AI Substance use/addiction issues · 3.Y Process addictions (pornography, gambling) · 2.P Assess substance use · 2.G Consider co-occurring diagnoses · 2.H Determine level of care needed · CACREP: Counseling and Helping Relationships; Assessment and Testing; Human Growth and Development

Where This Chapter Fits

Chapter 9 covered how counselors **treat** addiction: models, screening tools, the ASAM dimensions, motivational and behavioral approaches, mutual support, and confidentiality under 42 CFR Part 2. This chapter covers how addiction **presents**: the diagnostic criteria, severity levels, what intoxication and withdrawal look like for each substance class, and behavioral addictions such as gambling.

NCE items here usually ask one of four things: 1. How many criteria, and what severity? 2. What do intoxication or withdrawal from this substance look like? 3. Is this withdrawal dangerous? 4. Is this a substance-induced problem or an independent disorder?

Part 1 — Substance Use Disorder Criteria

The DSM-5-TR uses one set of **11 criteria** for substance use disorders, grouped into four clusters. Symptoms must cause clinically significant impairment or distress within a **12-month period**.

Cluster	Criteria
Impaired control	1. Using more, or for longer, than intended · 2. Persistent desire or unsuccessful efforts to cut down · 3. A lot of time spent getting, using, or recovering · 4. **Craving**
Social impairment	5. Failure to fulfill major role obligations · 6. Continued use despite social or interpersonal problems · 7. Giving up important activities
Risky use	8. Recurrent use in physically hazardous situations · 9. Continued use despite knowing it causes or worsens a physical or psychological problem
Pharmacological	10. **Tolerance** · 11. **Withdrawal**

Severity

Severity	Criteria met
Mild	2–3
Moderate	4–5
Severe	6 or more

Important exceptions: tolerance and withdrawal **don't count** toward the diagnosis when they occur only with medications taken as prescribed under medical supervision (opioid pain medications, sedatives, stimulants). A patient taking prescribed opioids who develops tolerance doesn't have an opioid use disorder on that basis alone.

Remission Specifiers

Specifier	Meaning
Early remission	At least 3 months but less than 12 months with no criteria met, **except craving**
Sustained remission	12 months or more with no criteria met, **except craving**
In a controlled environment	Access to substances is restricted (e.g., a locked facility)

Specifier	Meaning
On maintenance therapy	For opioid and tobacco use: taking an agonist or antagonist medication (e.g., methadone, buprenorphine, nicotine replacement)

DSM History the NCE May Test

The DSM-IV had two separate diagnoses, **substance abuse** and **substance dependence**. The DSM-5 combined them into a single **substance use disorder** on a severity continuum, dropped the "legal problems" criterion, and added **craving**.

Part 2 — Substance Classes: Intoxication and Withdrawal

The DSM-5-TR recognizes ten classes: alcohol; caffeine; cannabis; hallucinogens (phencyclidine and other hallucinogens); inhalants; opioids; sedatives, hypnotics, and anxiolytics; stimulants; tobacco; and other or unknown substances.

Class	Intoxication	Withdrawal	Withdrawal danger
Alcohol	Slurred speech, poor coordination, unsteady gait, nystagmus, impaired attention or memory, stupor or coma	Tremor, sweating, rapid pulse, insomnia, nausea, anxiety, agitation, transient hallucinations, **seizures**; **delirium tremens** (confusion, hallucinations, autonomic instability) typically 2–4 days after stopping	**Potentially life-threatening**

Class	Intoxication	Withdrawal	Withdrawal danger
Sedatives, hypnotics, anxiolytics (benzodiazepines, barbiturates)	Like alcohol: slurred speech, poor coordination, memory problems	Like alcohol: anxiety, tremor, insomnia, **seizures**, delirium	**Potentially life-threatening**; taper under medical care
Opioids (heroin, fentanyl, oxycodone)	**Pinpoint (constricted) pupils**, drowsiness, slurred speech, **slowed breathing**; overdose can be fatal	**Flu-like**: dilated pupils, runny nose, tearing, yawning, goosebumps, sweating, muscle aches, nausea, vomiting, diarrhea, insomnia	Very uncomfortable but **rarely life-threatening** in otherwise healthy adults; the big danger is **overdose after tolerance drops**
Stimulants (cocaine, methamphetamine, amphetamines)	Euphoria, **dilated pupils**, rapid heart rate, high blood pressure, sweating, agitation, paranoia, possible psychosis	**"Crash"**: depressed mood, fatigue, vivid unpleasant dreams, increased sleep and appetite	Not usually medically dangerous, but **depression and suicide risk** can be high
Cannabis	Euphoria or anxiety, red eyes, increased appetite, dry mouth, rapid heart rate, impaired coordination and time sense	Irritability, anxiety, sleep difficulty, decreased appetite, restlessness, depressed mood	Not medically dangerous

Class	Intoxication	Withdrawal	Withdrawal danger
Tobacco/nicotine	(No intoxication diagnosis)	Irritability, anxiety, difficulty concentrating, increased appetite, restlessness, depressed mood, insomnia	Not medically dangerous
Caffeine	Restlessness, nervousness, insomnia, flushed face, GI upset, rapid heart rate	Headache, fatigue, low mood, difficulty concentrating	Not medically dangerous
Hallucinogens (LSD, psilocybin, MDMA)	Perceptual changes, synesthesia, dilated pupils, anxiety or paranoia	**No withdrawal diagnosis**	—
Phencyclidine (PCP)	Agitation, violence, numbness, nystagmus, high blood pressure	**No withdrawal diagnosis**	—
Inhalants (glue, solvents, aerosols)	Dizziness, slurred speech, incoordination, euphoria; most common among younger adolescents; risk of sudden death	**No withdrawal diagnosis**	—

High-yield distinctions: - **Pinpoint pupils → opioids. Dilated pupils → stimulants** (and hallucinogens; opioid *withdrawal*). - **Alcohol and sedative withdrawal can kill.** Refer for medical evaluation and medically managed withdrawal. - **Opioid overdose → naloxone** (Narcan) can

reverse it. Fentanyl's potency has made accidental overdose much more common. - **Mixing respiratory depressants** (alcohol + benzodiazepines, or opioids + benzodiazepines) multiplies the risk of fatal respiratory depression. - After a period of abstinence (detox, incarceration, treatment), **tolerance drops**. Returning to a previous dose can be fatal. This is a key relapse-prevention teaching point. - **Hallucinogen persisting perception disorder**: flashbacks of perceptual changes after stopping hallucinogens.

Part 3 — Substance-Induced vs. Independent Disorders (2.G)

Many people who use substances also have depression, anxiety, psychosis, or sleep problems. The question is whether the substance is causing the symptoms.

Points toward substance-induced	Points toward an independent disorder
Symptoms began during or soon after intoxication or withdrawal	Symptoms **began before** substance use
Symptoms improve with sustained abstinence	Symptoms **persist for a substantial period** (about **1 month**) after acute withdrawal or severe intoxication ends
The substance is known to cause these symptoms	There's a history of the disorder during periods without substance use

Co-occurring disorders are common. Integrated treatment that addresses both at once, ideally by the same team, is the standard (Chapter 9). Don't wait for sobriety before treating a co-occurring mental health problem, and don't ignore the substance use while treating the mental health problem.

Part 4 — Who Owns Addiction Theory

Owner	Contribution
E. M. Jellinek	Disease concept of alcoholism; described **phases**: prealcoholic, prodromal, crucial, chronic
Alan Marlatt	Relapse prevention; abstinence violation effect; harm reduction advocate
Miller & Rollnick	Motivational interviewing
Prochaska & DiClemente	Stages of change
Sharon Wegscheider-Cruse	Family roles in addiction (hero, scapegoat, lost child, mascot, enabler)
Claudia Black	"Don't talk, don't trust, don't feel" rules in alcoholic families
Hunt & Azrin	Community reinforcement approach (CRA)
Robert Meyers	CRAFT (Community Reinforcement and Family Training), developed from CRA
Bill W. and Dr. Bob	Founded Alcoholics Anonymous (1935)
José Szapocznik	Brief Strategic Family Therapy (BSFT) for adolescent substance use
Howard Liddle	Multidimensional Family Therapy (MDFT) for adolescent substance use

Risk Factors

Family history and genetics, early age of first use, childhood adversity and trauma (ACEs; Chapter 10), co-occurring mental health conditions, peer use, easy access, and stressful life circumstances. Protective factors include strong family bonds, parental monitoring, school connection, and coping skills.

The Brain

Substances act on the brain's **reward pathway** (the mesolimbic dopamine system). Repeated use changes the

brain in ways that strengthen craving and habit and weaken self-control. This is the basis for viewing addiction as a chronic, relapsing condition rather than a moral failing.

Part 5 — Special Populations

Population	What the exam expects
Adolescents	Screen with **CRAFFT**; involve family; family-based approaches (BSFT, MDFT, adolescent CRA) have strong support; confidentiality rules for minors vary by state
Older adults	Alcohol and prescription misuse are often missed; lower tolerance; interactions with medications; falls and cognitive effects
Pregnant clients	Prioritize engagement and prenatal care; medications for opioid use disorder (methadone, buprenorphine) are recommended during pregnancy rather than medically supervised withdrawal
People with co-occurring disorders	Integrated treatment
Family members	Al-Anon, Alateen, CRAFT; address enabling and codependency

Part 6 — Process (Behavioral) Addictions (3.Y)

Gambling Disorder

The only behavioral addiction in the DSM-5-TR's Substance-Related and Addictive Disorders chapter.

- **At least 4 of 9 criteria in a 12-month period**: needing to gamble with increasing amounts to get

excited (tolerance); restless or irritable when cutting down (withdrawal); repeated unsuccessful efforts to stop; preoccupation; gambling when distressed; **"chasing" losses** (returning to win back money); lying to hide gambling; jeopardizing a relationship, job, or opportunity; relying on others for money to relieve a desperate financial situation.

- **Severity:** mild 4–5, moderate 6–7, severe 8–9.
- **Specifiers:** episodic or persistent; early or sustained remission.
- **Not better explained by a manic episode.**

"Chasing" losses is the classic gambling criterion.

Suicide risk is elevated in gambling disorder, especially with mounting debt. Always assess.

Treatment and support: CBT (addressing cognitive distortions like the "gambler's fallacy" and illusion of control), motivational interviewing, **Gamblers Anonymous**, **Gam-Anon** for families, financial counseling, and self-exclusion programs at casinos and online platforms.

Other Behavioral Problems

Problem	DSM-5-TR status
Internet gaming disorder	Section III: a **condition for further study**, not an official diagnosis
Compulsive sexual behavior / pornography use	Not a DSM-5-TR diagnosis; **ICD-11** includes **compulsive sexual behaviour disorder** as an impulse control disorder
Gaming disorder	**ICD-11** includes it as a disorder due to addictive behaviors
Compulsive buying, internet use, exercise	Not DSM diagnoses; addressed clinically when they cause distress or impairment

Exam principle: a counselor can address a behavior that is causing distress or impairment even when it isn't a formal diagnosis. Assess the function the behavior serves, its consequences, the client's own values, co-occurring

problems (depression, anxiety, ADHD, trauma), and readiness to change. Avoid imposing the counselor's moral views (Chapter 9, ACA A.4.b).

What the Exam Rewards / What It Punishes

Rewards: - Counting criteria and assigning severity correctly - Recognizing dangerous withdrawal (alcohol and sedatives) and arranging medical care - Knowing pupil signs (pinpoint = opioids; dilated = stimulants) - Teaching overdose risk after lost tolerance; naloxone - Deciding whether symptoms are substance-induced or independent - Integrated treatment for co-occurring disorders - Assessing suicide risk in gambling disorder and stimulant withdrawal

Punishes: - Counting prescribed-medication tolerance or withdrawal toward a diagnosis - Treating alcohol withdrawal as a counseling-only issue - Requiring sobriety before addressing a co-occurring disorder - Calling internet gaming disorder an official DSM diagnosis - Moralizing about behavioral addictions

Swap-Pair Drill

If you see…	It's…	Not…
SUD severity: 2–3 criteria	Mild	Moderate
SUD severity: 4–5 criteria	Moderate	Severe
SUD severity: 6 or more criteria	Severe	Moderate
SUD remission: 3–12 months without criteria (except craving)	Early remission	Sustained remission
SUD remission: 12+ months without criteria (except craving)	Sustained remission	Early remission

If you see…	It's…	Not…
Pinpoint pupils, slow breathing	Opioid intoxication	Stimulant intoxication
Runny nose, yawning, goosebumps, diarrhea	Opioid withdrawal	Alcohol withdrawal
Tremor, sweating, seizures, hallucinations	Alcohol or sedative withdrawal	Opioid withdrawal
Crash: fatigue, hypersomnia, hunger, low mood	Stimulant withdrawal	Cannabis withdrawal
Substances with no DSM withdrawal diagnosis	Hallucinogens, PCP, inhalants	Cannabis (which has one)
Chasing losses	Gambling disorder	Substance use disorder
Gambling disorder severity: 6–7 criteria	Moderate	Mild
Gaming addiction: DSM condition for further study	Internet gaming disorder	Gambling disorder
Prealcoholic, prodromal, crucial, chronic	Jellinek	Prochaska
Adolescent family therapy for substance use	BSFT (Szapocznik), MDFT (Liddle)	Filial therapy

Quick Checks

1. A client meets 5 substance use disorder criteria for alcohol in the past year. The severity is: A. Mild B. Moderate C. Severe D. Cannot be determined

2. A patient takes prescribed opioid pain medication as directed and has developed tolerance and mild withdrawal when doses are late. She meets no other criteria. She: A. Has mild opioid use disorder B. Doesn't meet criteria, because tolerance and withdrawal from prescribed use

don’t count C. Has moderate opioid use disorder D. Has opioid-induced depressive disorder

3. Which withdrawal syndrome is most likely to be medically life-threatening? A. Opioid B. Cannabis C. Alcohol D. Caffeine

4. A client arrives with pinpoint pupils, slurred speech, and very slow breathing. The most likely cause is: A. Cocaine intoxication B. Opioid intoxication C. Cannabis withdrawal D. Hallucinogen intoxication

5. After 6 weeks in jail, a client with opioid use disorder is released and plans to “celebrate just once.” The most important teaching point is: A. Opioid withdrawal is life-threatening B. His tolerance has likely dropped, so his previous dose could be fatal; carry naloxone and avoid using alone C. He should switch to stimulants D. He’s in sustained remission

6. Four months after stopping heavy drinking, a client has no use disorder symptoms except occasional cravings. The specifier is: A. Sustained remission B. Early remission C. In a controlled environment D. On maintenance therapy

7. A client’s depressive symptoms began during heavy cocaine use and resolved two weeks after he stopped. This best suggests: A. An independent major depressive disorder B. A substance-induced depressive disorder C. Bipolar disorder D. Persistent depressive disorder

8. “Chasing losses” is a criterion of: A. Stimulant use disorder B. Gambling disorder C. Internet gaming disorder D. Compulsive buying

9. In the DSM-5-TR, internet gaming disorder is: A. An official diagnosis in the addictions chapter B. A condition for further study in Section III C. A type of impulse control disorder D. Listed under gambling disorder

10. The DSM-5 changed substance diagnoses by: A. Adding a “legal problems” criterion B. Combining abuse and dependence into one use disorder and adding craving

C. Removing tolerance as a criterion D. Creating separate diagnoses for each severity level

11. Which substance class has no withdrawal diagnosis in the DSM-5-TR? A. Cannabis B. Tobacco C. Inhalants D. Caffeine

12. A client with severe gambling disorder has lost his savings and says his family "would be better off" with his life insurance. The counselor's first priority is to: A. Refer him to Gamblers Anonymous B. Conduct a suicide risk assessment C. Develop a budget D. Explore cognitive distortions about gambling

Answers and Rationales

1 — B. 4–5 criteria is moderate. A: mild is 2–3. C: severe is 6 or more. D: the count determines severity.

2 — B. Tolerance and withdrawal from medically supervised, prescribed use aren't counted. A and C: incorrectly count those criteria. D: no depressive symptoms are described.

3 — C. Alcohol (and sedative) withdrawal can cause seizures and delirium tremens. A: opioid withdrawal is miserable but rarely life-threatening in healthy adults. B and D: not medically dangerous.

4 — B. Pinpoint pupils and respiratory depression are classic opioid signs. A: cocaine dilates pupils and speeds the heart. C: cannabis withdrawal involves irritability and sleep problems. D: hallucinogens dilate pupils.

5 — B. Lost tolerance after abstinence makes overdose a serious risk; naloxone and not using alone are harm reduction steps. A: the danger is overdose, not withdrawal. C: harmful and not counseling. D: 6 weeks is too short for any remission specifier; early remission requires at least 3 months.

6 — B. At least 3 but less than 12 months with no criteria except craving. A: sustained requires 12 months or more.

C: he isn't in a restricted environment. D: no maintenance medication is mentioned.

7 — B. Onset during use and resolution with abstinence point to a substance-induced disorder. A: independent depression would typically predate use or persist a month or more after abstinence. C and D: not supported by the history.

8 — B. Chasing losses is a gambling disorder criterion. A: stimulant use disorder uses the 11 substance criteria. C: internet gaming disorder is a condition for further study. D: compulsive buying isn't a DSM diagnosis.

9 — B. It's in Section III as a condition for further study. A: only gambling disorder is in the chapter. C and D: incorrect placement.

10 — B. DSM-5 merged abuse and dependence into one use disorder and added craving. A: the legal problems criterion was removed. C: tolerance remains. D: severity is a specifier of one diagnosis.

11 — C. Inhalants (along with hallucinogens and PCP) have no withdrawal diagnosis. A, B, D: cannabis, tobacco, and caffeine all have withdrawal diagnoses.

12 — B. A statement suggesting he'd be better off dead, plus financial devastation, requires a suicide risk assessment first (Chapter 6). A, C, D: useful later, but safety comes first.

Audio Companion

Track	Covers	Target length
11.1	Count It and Spot It: criteria, severity, intoxication, withdrawal, gambling	~5 min

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Chapter 12 — Life Span and Development

Domain map: 3.B Aging/geriatric concerns · 3.G End-of-life issues · 3.K Grief/loss · 3.O Intellectual functioning issues · 3.S Developmental processes/tasks/issues · 3.AB Retirement concerns · 3.AD Separation from primary caregivers · 3.AK Terminal illness issues · 3.AQ Child development issues · 5.A Align intervention with client's developmental level · CACREP: Human Growth and Development

From Owners to Clients

Chapter 3 gave you the developmental theorists and their stages: Erikson, Piaget, Vygotsky, Kohlberg, Gilligan, Mahler, Marcia, Bowlby, Ainsworth, Baumrind, Kübler-Ross, Worden, Selye, and Schlossberg. This chapter puts those owners to work on the concerns clients actually bring:

1. Children's development and neurodevelopmental differences
2. Separation from caregivers
3. Adolescence and adulthood
4. Aging and retirement
5. Grief and loss
6. Terminal illness and the end of life

The theme that runs through the chapter: **normal developmental struggle is not pathology.** The NCE rewards answers that frame a problem as a developmental task under strain, while still screening for the disorders that can come with it.

Part 1 — Child Development and Neurodevelopmental Presentations (3.AQ, 3.O)

Developmental Domains

Child development is usually tracked across five domains: **physical/motor**, **cognitive**, **language**, **social-emotional**, and **adaptive** (self-care). When a child is behind in one or more domains, the counselor's role is to notice, screen, and refer for a **developmental or educational evaluation**, not to diagnose by observation alone.

Neurodevelopmental Presentations

Presentation	Key features	Key details
Intellectual developmental disorder (intellectual disability)	Deficits in **intellectual functioning** (reasoning, problem-solving, learning) **and** in **adaptive functioning** (conceptual, social, practical), with onset in the developmental period	**Severity (mild, moderate, severe, profound) is based on adaptive functioning, not IQ score alone.** Intellectual functioning roughly two standard deviations below the mean (an IQ around 70, allowing for measurement error)
Global developmental delay	Delays in several areas in children under 5 who can't yet be reliably assessed	Reassess later
Autism spectrum disorder	Persistent deficits in **social communication and interaction**, plus **restricted, repetitive behaviors, interests, or activities** (including sensory differences); present from early development	Severity described by **level of support needed**: Level 1 (requiring support), Level 2 (substantial support), Level 3 (very substantial support)

Presentation	Key features	Key details
ADHD	Persistent inattention and/or hyperactivity-impulsivity	**6 or more** symptoms (5 or more for age 17+) for at least **6 months**; several symptoms present **before age 12**; symptoms in **two or more settings** (home, school, work). Presentations: combined, predominantly inattentive, predominantly hyperactive-impulsive
Specific learning disorder	Persistent difficulty in reading, written expression, or math well below age expectations, despite targeted help	At least **6 months** despite intervention
Tourette's disorder	**Multiple motor tics and at least one vocal tic**	Tics for more than **1 year**; onset **before age 18**
Communication disorders	Language, speech sound, childhood-onset fluency (stuttering), social (pragmatic) communication	Speech-language evaluation

Borderline intellectual functioning is not a disorder; it's a condition that may be a focus of clinical attention (coded R41.83).

Who Owns Child Development Assessment

Instrument	Use
Wechsler Intelligence Scale for Children (WISC)	Intellectual functioning, ages 6–16
Wechsler Preschool and Primary Scale of Intelligence (WPPSI)	Young children

Instrument	Use
Vineland Adaptive Behavior Scales	**Adaptive functioning** (required for intellectual disability diagnosis)
ADOS-2 (Autism Diagnostic Observation Schedule)	Autism assessment
M-CHAT	Autism screening for toddlers
Conners Rating Scales; **Vanderbilt**	ADHD rating scales (parent and teacher forms)
Ages and Stages Questionnaires (ASQ)	Developmental screening

School Supports

Law or plan	What it provides
IDEA (Individuals with Disabilities Education Act)	Special education for eligible students; an **Individualized Education Program (IEP)** with goals and services; ages 3–21; early intervention (Part C) for birth to 3
Section 504 (Rehabilitation Act of 1973)	A **504 plan** with accommodations for students with a disability that substantially limits a major life activity, even if they don't need special education
FERPA	Privacy of student education records; parents' rights to access them (these rights transfer to the student at 18 or in postsecondary school)

Counseling Approaches

- Use **concrete, developmentally matched** methods (play, art, activities; Chapter 5).
- Involve parents and teachers; **parent training** has strong support for ADHD and behavior problems.
- For autism, **applied behavior analysis (ABA)** has broad research support for skill-building; social skills groups help many children and adolescents.

- Address the whole family: siblings and parents of children with disabilities have their own stresses.

Part 2 — Separation from Primary Caregivers (3.AD)

Separation happens through divorce, military deployment, incarceration of a parent, foster care placement, immigration and family separation, hospitalization, and death.

Owners

Owner	Contribution
John Bowlby	Attachment theory; children's responses to separation: **protest → despair → detachment** (with James Robertson's observations of hospitalized children)
Mary Ainsworth	Strange Situation; secure, avoidant, and resistant/ambivalent attachment
Main & Solomon	Disorganized attachment
Margaret Mahler	Separation–individuation; rapprochement
Pauline Boss	**Ambiguous loss**: a loss without clarity or closure, either physically absent but psychologically present (a missing or deployed parent) or physically present but psychologically absent (a parent with dementia or addiction)

Related Presentations

Presentation	Key features
Separation anxiety disorder	Excessive fear about separation from attachment figures; at least 4 weeks in children (Chapter 10)

Presentation	Key features
Reactive attachment disorder	Emotionally withdrawn, rarely seeks or responds to comfort, after **severe neglect**; evident before age 5
Disinhibited social engagement disorder	Overly familiar with unfamiliar adults, little hesitation to go off with strangers, after severe neglect
Adjustment disorder	Distress after a stressor such as divorce or a move

What Helps

- **Predictability and honest, age-appropriate information** about the separation.
- **Maintaining connection** with the absent caregiver when safe (letters, calls, visits, video).
- **Stable routines** and a consistent caregiver.
- **Caregiver involvement**: interventions that strengthen the caregiver–child relationship (e.g., Parent–Child Interaction Therapy, Child–Parent Psychotherapy, Attachment and Biobehavioral Catch-up for children in foster care).
- **Children of deployed service members**: emotional cycle of deployment; school counselors and military family life resources.

Part 3 — Adolescence and Adulthood (3.S)

Adolescence

Concept	Owner
Identity vs. role confusion	Erikson
Identity statuses (diffusion, foreclosure, moratorium, achievement)	Marcia
Imaginary audience and personal fable	Elkind
Formal operational thinking	Piaget

Concept	Owner
Emerging adulthood (roughly 18–29): identity exploration, instability, self-focus, feeling in-between, possibilities	**Jeffrey Arnett**

Counseling adolescents: respect growing autonomy, clarify confidentiality and its limits with both teen and parents at the start, and expect some risk-taking as normal while screening for serious risk (substances, self-harm, unsafe relationships).

Adulthood

Concept	Owner
Intimacy vs. isolation; generativity vs. stagnation	Erikson
Life structure; transitions (e.g., age-30 transition, midlife transition); the Dream; mentor	Levinson
Developmental tasks of each life stage	Havighurst
Transitions: situation, self, support, strategies (the 4 S's)	Schlossberg
Sandwich generation: midlife adults caring for both children and aging parents	(general term; Dorothy Miller is often credited with coining it)

Midlife: "midlife crisis" is a popular idea, but research suggests most adults don't experience a dramatic crisis. Midlife is better understood as a period of reassessment.

Part 4 — Aging and Retirement (3.B, 3.AB)

Theories of Aging

Theory	Owner	Core idea
Disengagement theory	Cumming & Henry (1961)	Older adults and society mutually withdraw; seen as natural (largely criticized now)

Theory	Owner	Core idea
Activity theory	Havighurst and colleagues	Staying active and involved leads to greater satisfaction
Continuity theory	Robert Atchley	People adapt by keeping patterns, roles, and identity consistent with earlier life
Successful aging	Rowe & Kahn (1997)	Low risk of disease and disability, high mental and physical functioning, and active engagement with life
Selective optimization with compensation (SOC)	Paul & Margret Baltes	Choose what matters most (selection), work to maintain it (optimization), and use strategies or aids to make up for losses (compensation)
Socioemotional selectivity theory	Laura Carstensen	As time horizons shorten, people focus on emotionally meaningful goals and relationships and prune peripheral ones
Integrity vs. despair	Erikson	Accepting one's life as it was
Gerotranscendence	Lars Tornstam	A shift toward a more cosmic, less materialistic perspective in late life
Ageism	Coined by **Robert Butler** (1969)	Stereotyping and discrimination based on age

Cognitive Changes: Delirium, Dementia, and Depression

Feature	Delirium	Major neurocognitive disorder (dementia)	Depression
Onset	Sudden (hours to days)	Gradual (months to years), though some types (e.g., vascular) can begin more abruptly	Weeks to months
Course	Fluctuates over the day	Usually progressive	Often worse in the morning; tied to mood
Attention	**Markedly impaired**	Usually intact early	May be poor
Cause	Medical: infection (e.g., urinary tract infection), medications, dehydration, surgery	Alzheimer's, vascular, Lewy body, frontotemporal, others	Mood disorder
Response	**Medical emergency**; usually reversible if treated	Support, planning, caregiver help	Treatable; cognition often improves with treatment

Mild vs. major neurocognitive disorder: both involve decline in **one or more** cognitive domains. In **major**, the decline interferes with **independence** in everyday activities; in **mild**, independence is preserved (perhaps with more effort or compensation).

Screening tools: Montreal Cognitive Assessment (MoCA), **Mini-Mental State Examination (MMSE)**, **Mini-Cog**, **Geriatric Depression Scale (GDS)**.

Exam rule: sudden confusion in an older adult is a **medical** problem until proven otherwise. Refer for immediate medical evaluation.

Other Late-Life Concerns

- **Elder abuse**: physical, sexual, emotional, financial exploitation, neglect, and self-neglect; report per state law (Chapter 6).
- **Suicide risk**: older men have among the highest suicide rates of any group; screen even when the client doesn't raise it.
- **Loneliness and isolation** (Chapter 16).
- **Caregiver strain** for spouses and adult children (Chapter 16).
- **Medication and alcohol interactions**.

Retirement (3.AB)

Phase (Atchley)	What happens
Pre-retirement	Planning and anticipating
Honeymoon	Enjoying freedom; travel and new activities
Disenchantment	Letdown when retirement doesn't match expectations
Reorientation	Developing a more realistic, satisfying routine
Stability	A settled, predictable lifestyle
Termination	Retirement role ends, often due to illness or disability

Not everyone goes through every phase, or in order.

Counseling retirement: identity beyond work (Super's disengagement stage, Chapters 3 and 14), structure and meaning, relationships (the couple now home together), finances, health, and **bridge employment** (part-time or new work after retiring). Involuntary retirement is harder than chosen retirement.

Part 5 — Grief and Loss (3.K)

Owners

Owner	Contribution
Erich Lindemann	Acute grief reactions; also described **anticipatory grief**
Elisabeth Kübler-Ross	Five stages of **dying**: denial, anger, bargaining, depression, acceptance (from work with terminally ill patients; **not a required sequence**)
John Bowlby	Phases of mourning: **numbness → yearning and searching → disorganization and despair → reorganization**
J. William Worden	Four **tasks** of mourning: accept the reality of the loss; process the pain of grief; adjust to a world without the deceased; find an enduring connection with the deceased while embarking on a new life
Therese Rando	Six "R" processes: **R**ecognize the loss, **R**eact to the separation, **R**ecollect and reexperience, **R**elinquish old attachments, **R**eadjust, **R**einvest
Stroebe & Schut	**Dual process model**: healthy grieving **oscillates** between **loss-oriented** coping (facing the grief) and **restoration-oriented** coping (attending to life changes, taking breaks from grief)
Klass, Silverman, & Nickman	**Continuing bonds**: maintaining a connection to the deceased can be healthy, not a failure to "let go"
Kenneth Doka	**Disenfranchised grief**: loss not socially recognized (a pet, an ex-partner, a miscarriage, a death by overdose, a loss in a stigmatized relationship)
Pauline Boss	**Ambiguous loss**

Owner	Contribution
Robert Neimeyer	**Meaning reconstruction** after loss

Stages vs. tasks: Kübler-Ross (writing about dying patients) and Bowlby describe what people **go through**. Worden and Rando describe what grievers **do**. Worden's task language makes grief active, which is why many counselors prefer it.

Prolonged Grief Disorder (DSM-5-TR)

Feature	Requirement
Time since death	At least **12 months** for adults; at least **6 months** for children and adolescents
Core symptom	Intense **yearning** for, or **preoccupation** with, the deceased, nearly every day for at least the last month
Additional symptoms	At least **3 of 8**: identity disruption; marked disbelief about the death; avoidance of reminders; intense emotional pain; difficulty reintegrating into life; emotional numbness; feeling life is meaningless; intense loneliness
Exceeds norms	Duration and severity clearly exceed social, cultural, or religious norms

Grief is not a disorder. Most people grieve without needing a diagnosis. Prolonged grief disorder applies only when grief remains intense and impairing well beyond expected norms. Uncomplicated bereavement is a Z code, not a mental disorder.

Children and Grief

Children's understanding of death develops with cognitive growth (Piaget). Understanding of the core concepts (**irreversibility**: death is permanent; **universality**: all living things die; **nonfunctionality**: the body stops work-

ing; **causality**: why death happens) usually develops between about **ages 5 and 7**.

- Use **direct words**: “died,” not “went to sleep” or “we lost him.” Euphemisms confuse young children and can cause fear (of sleep, of getting lost).
- Answer questions honestly at the child’s level.
- Children often grieve in **bursts**, playing one minute and sad the next. This is normal.
- Let children choose whether to take part in rituals such as funerals, after preparing them for what they’ll see.
- Expect grief to be revisited at new developmental stages.

What Helps

- **Normalize** grief; avoid pathologizing.
- **Don’t rush** or impose a sequence.
- Support both facing the loss and taking breaks from it (dual process).
- Honor **continuing bonds** and cultural rituals.
- **Screen** for depression, prolonged grief, substance use, and suicide risk, especially after sudden or violent deaths and death by suicide.
- For prolonged grief disorder, **complicated grief treatment** (Katherine Shear) has strong support.

Part 6 — Terminal Illness and End of Life (3.G, 3.AK)

Care Models

Term	Meaning
Palliative care	Care focused on comfort and quality of life for anyone with serious illness, at any stage, alongside curative treatment

Term	Meaning
Hospice	Comfort care for people expected to live about **6 months or less** who are no longer seeking curative treatment; modern hospice movement founded by **Dame Cicely Saunders**
Advance directive	A legal document stating a person's wishes for medical care if they can't speak for themselves
Living will	A type of advance directive specifying treatments the person wants or refuses
Health care power of attorney (health care proxy)	Names a person to make medical decisions if the patient can't
Medical aid in dying	Legal in some U.S. states for terminally ill, mentally capable adults; laws vary

Counseling the Dying

- **Follow the client's lead.** Some want to talk about death; some want to talk about living.
- Address **anticipatory grief** for both the client and family.
- Support **life review** and legacy work (Butler; Chapter 5).
- Help with practical and relational unfinished business.
- Respect cultural and religious beliefs about death, dying, and rituals.
- Coordinate with the hospice or palliative team.
- **Kübler-Ross's stages** describe common reactions, not a required order.

Ethics: ACA B.2.b

Counselors who provide services to **terminally ill clients who are considering hastening their own deaths** have the **option to maintain confidentiality**, depending on applicable laws and the specific circumstances, **after seeking consultation or supervision** from appropriate professional and legal parties.

This is different from the usual duty to protect a suicidal client (Chapter 6). The exam pattern: consultation first, knowledge of state law, and attention to the client's capacity, depression, and coercion. A terminally ill client's wish to die still deserves careful assessment for treatable depression and unmanaged pain.

What the Exam Rewards / What It Punishes

Rewards: - Framing struggles as developmental tasks while still screening for disorders - Basing intellectual disability severity on adaptive functioning - Referring for developmental, educational, or medical evaluation - Treating sudden confusion in an older adult as a medical issue - Using direct language about death with children - Normalizing grief and honoring continuing bonds - Consultation in end-of-life confidentiality decisions

Punishes: - Treating Kübler-Ross's stages as a required sequence - Pathologizing normal grief - Using euphemisms for death with young children - Assuming older adults can't change, or that confusion is "just aging" - Diagnosing autism or ADHD from one observation in one setting

Swap-Pair Drill

If you see…	It's…	Not…
Severity based on adaptive functioning	Intellectual developmental disorder	ADHD
Severity rated by levels of support needed	Autism spectrum disorder	Intellectual disability
Symptoms before age 12, two or more settings	ADHD	Specific learning disorder
Motor and vocal tics, over 1 year, onset before 18	Tourette's disorder	Persistent tic disorder

If you see…	It's…	Not…
Special education with an IEP	IDEA	Section 504
Accommodations without special education	Section 504	IDEA
Protest → despair → detachment	Bowlby (with Robertson)	Kübler-Ross
Physically present, psychologically absent	Ambiguous loss (Boss)	Disenfranchised grief (Doka)
Emerging adulthood	Arnett	Levinson
Mutual withdrawal of elders and society	Disengagement theory (Cumming & Henry)	Activity theory
Selection, optimization, compensation	Baltes & Baltes	Carstensen
Focus on meaningful relationships as time shortens	Socioemotional selectivity (Carstensen)	Continuity theory
Coined "ageism"	Robert Butler	Rowe & Kahn
Sudden, fluctuating confusion with poor attention	Delirium	Dementia
Honeymoon, disenchantment, reorientation	Atchley's retirement phases	Kübler-Ross
Oscillating between loss and restoration	Dual process model (Stroebe & Schut)	Worden's tasks
Ongoing connection to the deceased is healthy	Continuing bonds (Klass et al.)	Disenfranchised grief
Six R processes	Rando	Worden
Modern hospice movement	Cicely Saunders	Kübler-Ross
Prolonged grief disorder in adults	12 months	6 months (children and adolescents)

Quick Checks

1. A child's IQ is measured at 68, but she manages self-care, friendships, and schoolwork with typical independence. The best conclusion is: A. She has severe intellectual disability B. She meets criteria for intellectual disability based on IQ alone C. Intellectual disability requires deficits in adaptive functioning as well, which aren't present here D. She has autism spectrum disorder

2. A student with ADHD needs extended test time but doesn't need special education services. The most appropriate support is: A. An IEP under IDEA B. A Section 504 plan C. FERPA protection D. Early intervention under Part C

3. A 4-year-old whose father has deployed first cries and clings, then seems withdrawn and sad, then appears indifferent when the father calls. This sequence matches: A. Kübler-Ross's stages B. Bowlby's protest, despair, and detachment C. Marcia's identity statuses D. Worden's tasks

4. A woman's husband has advanced Alzheimer's disease. She says, "He's here, but he's gone." This is best described as: A. Disenfranchised grief B. Ambiguous loss C. Anticipatory grief only D. Prolonged grief disorder

5. An 82-year-old client becomes suddenly confused over two days, with attention that comes and goes. The counselor should first: A. Begin reminiscence therapy B. Screen with the MoCA and diagnose dementia C. Arrange an immediate medical evaluation for possible delirium D. Treat for depression

6. Which theory says older adults narrow their social circles to focus on emotionally meaningful relationships as their time horizons shorten? A. Disengagement theory B. Socioemotional selectivity theory C. Activity theory D. Continuity theory

7. A recently retired client says retirement has been "a big letdown" after an exciting first few months of travel. According to Atchley, he's most likely in which phase? A.

Honeymoon B. Disenchantment C. Stability D. Termination

8. A widow keeps her husband's photos up, talks to him at his grave, and says it comforts her, while she's also reengaging with friends. The best interpretation is: A. Prolonged grief disorder; she needs to let go B. Healthy continuing bonds C. Denial, the first stage of grief D. Disenfranchised grief

9. Seven months after his mother's death, an adult client has intense yearning, loneliness, and difficulty reengaging in life. Can prolonged grief disorder be diagnosed? A. Yes, because symptoms are present for more than 6 months B. No, because adults require at least 12 months since the death C. Yes, because grief is always a disorder after 6 months D. No, because prolonged grief disorder doesn't exist in the DSM-5-TR

10. A 5-year-old asks where her grandfather is. The most helpful response is: A. "Grandpa went to sleep and won't wake up." B. "We lost Grandpa." C. "Grandpa died. His body stopped working, and he can't come back. It's OK to feel sad." D. "Grandpa went on a long trip."

11. Which model of grief describes healthy coping as moving back and forth between facing the loss and attending to life changes? A. Kübler-Ross's stages B. Worden's tasks C. Stroebe and Schut's dual process model D. Rando's six Rs

12. A terminally ill client in a state where medical aid in dying is legal tells her counselor she plans to pursue it. Under ACA B.2.b, the counselor should: A. Immediately break confidentiality to prevent a suicide B. Consult with appropriate professional and legal parties, then decide whether to maintain confidentiality, considering applicable law and circumstances C. Refuse to discuss the topic D. Encourage the client to proceed

Answers and Rationales

1 — C. The diagnosis requires deficits in both intellectual and adaptive functioning, and severity is based on adaptive functioning. A: severity isn't set by IQ alone, and her adaptive functioning is typical. B: IQ alone isn't sufficient. D: nothing suggests social communication deficits or restricted behaviors.

2 — B. Section 504 provides accommodations without special education. A: an IEP is for students needing special education. C: FERPA protects records. D: Part C serves infants and toddlers.

3 — B. Bowlby (with Robertson) described protest, despair, and detachment in separated young children. A: Kübler-Ross described reactions to dying. C: Marcia's statuses are adolescent identity. D: Worden's tasks are for mourning a death.

4 — B. Physically present but psychologically absent is ambiguous loss (Boss). A: disenfranchised grief is unrecognized loss. C: anticipatory grief may also be present, but "here but gone" is the hallmark of ambiguous loss. D: he hasn't died.

5 — C. Sudden onset with fluctuating attention suggests delirium, a medical emergency. A: delays medical care. B: dementia typically develops gradually. D: misses a likely medical cause.

6 — B. Carstensen's socioemotional selectivity theory. A: disengagement theory describes mutual withdrawal from society. C: activity theory says staying active brings satisfaction. D: continuity theory emphasizes keeping consistent patterns.

7 — B. A letdown after the honeymoon phase is disenchantment. A: honeymoon is the early excitement. C: stability is a settled routine. D: termination is the end of the retirement role.

8 — B. Maintaining a comforting connection while reengaging with life is healthy continuing bonds (Klass et al.).

A: she's reengaging and functioning, and the time frame isn't given. C: Kübler-Ross's stages described dying patients and aren't a required sequence. D: her loss is socially recognized.

9 — B. Adults require at least 12 months since the death. A and C: 6 months applies to children and adolescents. D: prolonged grief disorder was added in the DSM-5-TR.

10 — C. Direct, honest, age-appropriate language that names death and permits feelings. A: can create fear of sleep. B and D: euphemisms suggest the person can be found or will return.

11 — C. Oscillation between loss and restoration is the dual process model. A: Kübler-Ross describes stages of dying. B: Worden describes tasks. D: Rando describes six processes.

12 — B. B.2.b allows counselors to maintain confidentiality for terminally ill clients considering hastening death, depending on law and circumstances, after consultation. A: ignores B.2.b and the legal context. C: abandons the client. D: imposes the counselor's view rather than supporting the client's own decision-making.

Audio Companion

Track	Covers	Target length
12.1	Across the Life Span: neurodevelopment, separation, aging, grief, end of life	~6 min

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Chapter 13 — Relationships and Family Life

Domain map: 3.R Remarriage/recommitment · 3.AN Adoption issues · 3.AO Blended family issues · 3.AP Child abuse-related concerns · 3.AR Dating/relationship problems · 3.AS Divorce · 3.AT Family abuse/violence (e.g., physical, sexual, emotional) · 3.AU Interpersonal partner violence concerns · 3.AV Marital/partner communication problems · 3.AW Parenting/co-parenting conflicts · CACREP: Human Growth and Development; Counseling and Helping Relationships

Where This Chapter Fits

Chapter 7 taught **how** to work with couples and families: systems concepts, assessment tools, joining, and systemic interventions. Chapter 6 covered the **safety** side: mandated reporting and intimate partner violence safety planning. This chapter covers **what** couples and families bring to counseling: love and dating, communication, divorce, co-parenting, remarriage and stepfamilies, adoption, parenting, and abuse.

As in earlier chapters, the NCE tends to test three things here: the owners, the research findings that overturn common assumptions, and the counselor's ethical role.

Part 1 — Love, Dating, and Relationship Problems (3.AR, 3.AV)

Who Owns Love

Owner	Theory	Key terms
Robert Sternberg	**Triangular theory of love**	Three components: **intimacy**, **passion**, **commitment**. All three together = **consummate love**. Intimacy + passion = romantic love; intimacy + commitment = companionate love; passion + commitment = fatuous love; commitment alone = empty love; passion alone = infatuation; intimacy alone = liking
John Alan Lee	**Love styles**	Eros (passionate), ludus (game-playing), storge (friendship), pragma (practical), mania (possessive), agape (selfless)
Hazan & Shaver	**Adult attachment**	Romantic love as an attachment process; secure, anxious, and avoidant adult attachment styles
Caryl Rusbult	**Investment model**	Commitment depends on **satisfaction**, the **quality of alternatives**, and the size of **investments**

Owner	Theory	Key terms
John Gottman	Research on couple interaction	Four Horsemen; 5:1 positive-to-negative ratio in stable couples; repair attempts; most couple conflicts are **perpetual** (about lasting differences) rather than solvable
Gary Chapman	Five love languages (popular, not research-based)	Words of affirmation, quality time, gifts, acts of service, physical touch

Exam note on Rusbult: people stay in unsatisfying (even harmful) relationships when alternatives seem poor and investments are high. This helps explain why "Why doesn't she just leave?" is the wrong question in IPV (Chapter 6).

Communication Problems (3.AV)

Chapter 7 covered Gottman, Satir's stances, EFT's pursue–withdraw cycle, and PREP's speaker–listener technique. The core counseling moves:

- Slow the conversation down and **have partners talk to each other** in session.
- Teach **"I" statements** and **active listening** (reflect before responding).
- Replace criticism with a **softened startup** (Gottman).
- Help partners recognize **flooding** and take breaks (at least 20 minutes) before returning to the conversation.
- Distinguish **solvable** problems from **perpetual** ones; for perpetual problems, the goal is dialogue and acceptance, not a final solution (IBCT, Chapter 7).

Infidelity and Recommitment (3.R)

Recovery from an affair is commonly described in three phases: **(1) dealing with the impact** (crisis, disclosure, safety, stabilizing), **(2) understanding the context** (how the relationship and each partner contributed, without ex-

cusing the affair), and **(3) moving forward** (forgiveness, recommitment, or separation) (Baucom, Snyder, & Gordon).

The counselor stays neutral, attends to both partners' pain, and follows the secrets policy set at intake (Chapter 7).

Part 2 — Divorce (3.AS)

Bohannan's Six Stations of Divorce

Paul Bohannan described six overlapping experiences ("stations") of divorce:

Station	What it involves
Emotional	The relationship deteriorates
Legal	The court process
Economic	Dividing property and finances
Coparental	Custody, visitation, and co-parenting arrangements
Community	Changes in friends and social networks
Psychic	Regaining individual identity and autonomy

Children and Divorce: What the Research Shows

Researcher	Finding
E. Mavis Hetherington	**Most children of divorce adjust well** within about two years; roughly 75–80% show no lasting serious problems. Children do best with competent parenting and low conflict
Judith Wallerstein	A more pessimistic view from a smaller clinical sample; described a **"sleeper effect"** in which some effects appear in adulthood, especially around intimate relationships

Researcher	Finding
Paul Amato	**Parental conflict** is a stronger predictor of children's problems than divorce itself; children in high-conflict marriages may do better after divorce, while children whose low-conflict parents divorce may struggle more

The exam principle: conflict, not divorce itself, is the bigger risk to children. Interventions that reduce children's exposure to conflict (not using children as messengers, not criticizing the other parent in front of them, not making them choose) are the priority.

Children's Reactions by Age

Age	Common reactions
Preschool	Regression, fear of abandonment, self-blame ("Daddy left because I was bad"), confusion
School age	Sadness, loyalty conflicts, reconciliation fantasies, anger, school problems
Adolescents	Anger, worry about their own future relationships, taking on adult roles, distancing, acting out

What helps children: tell them together if possible, in age-appropriate words; reassure them the divorce isn't their fault and both parents still love them; keep routines; let them love both parents; protect them from conflict and adult details.

Co-Parenting Styles

Style	Description
Cooperative	Parents communicate, coordinate, and support each other's parenting

Style	Description
Parallel	Parents disengage from each other, each parenting independently with minimal contact; often recommended for **high-conflict** families to reduce children's exposure to conflict
Conflicted	Ongoing hostility; children are caught in the middle

Parenting coordinators and **co-parenting apps** can help high-conflict parents communicate.

Ethics in Divorce-Related Work

Issue	Rule
Consent to treat a minor	Check **legal custody** and any court orders. In general, a parent with legal custody can consent, but orders vary. Get a copy of the custody documents
The other parent's rights	A noncustodial parent may still have rights to records, depending on state law and the order
Role conflict	Counselors **don't serve as forensic evaluators** (custody evaluators) for **their own current or former clients** (ACA E.13.c). A child's or parent's counselor who is asked to recommend custody should explain the role conflict
Court testimony	A treating counselor may be called as a fact witness; that role is different from being a custody evaluator
Neutrality	Avoid being pulled into one parent's side (triangulation)

Part 3 — Remarriage and Stepfamilies (3.R, 3.AO)

Owners

Owner	Contribution
Emily & John Visher	Early stepfamily research and support; identified stepfamily **myths**: **instant love** (stepparent and stepchildren will love each other quickly), the **wicked stepmother**, and that a stepfamily is just like a first-marriage family
Patricia Papernow	**Stepfamily Cycle**, seven stages in three phases: **Early** (fantasy, immersion, awareness), **Middle** (mobilization, action), **Later** (contact, resolution). Integration often takes **several years** (commonly cited as about 4–7 or more)
McGoldrick & Carter	Remarriage stages added to the family life cycle (Chapter 7)

Stepfamily Challenges

- **Insider/outsider positions**: the biological parent and children share history; the stepparent starts as an outsider.
- **Loyalty binds**: children may feel that liking a stepparent betrays the other biological parent.
- **Discipline**: early on, the **biological parent** should handle discipline while the **stepparent builds a relationship** (more like a mentor, coach, or friendly adult). Stepparents who take on discipline too early often face resentment.
- **Different family cultures** merging: routines, rules, holidays, money.
- **Grief**: children (and adults) may still be grieving the previous family.
- **Ex-partners** remain part of the system.

Exam principle: slow integration does not mean a step-family is "failing." Normalize the time it takes, and correct unrealistic expectations (the instant-love myth).

Part 4 — Adoption (3.AN)

Key Terms

Term	Meaning
Adoption triad (or **adoption constellation**)	The **birth parents**, the **adoptive parents**, and the **adoptee**. "Constellation" includes extended family and others affected
Closed adoption	No contact or identifying information shared
Open adoption	Some ongoing contact or information sharing between birth and adoptive families
Transracial adoption	Adoptive parents and child are of different races
International (intercountry) adoption	Child adopted from another country
Kinship adoption	Adoption by relatives

Seven Core Issues in Adoption (Silverstein & Kaplan)

Deborah Silverstein and Sharon Kaplan identified issues that can affect **all members** of the triad across the life span: **loss, rejection, guilt and shame, grief, identity, intimacy, and mastery/control.**

Counseling Principles

- **Tell children about their adoption early** and in age-appropriate ways, as an ongoing conversation rather than a single disclosure. Secrecy tends to harm trust when the truth comes out later.
- **Adolescence** often brings identity questions and interest in birth family; this is normal, not a sign of rejecting the adoptive family.

- **Transracial and international adoptees** benefit from connection to their racial, ethnic, and cultural heritage and from adoptive parents who prepare them to face racism.
- **Children adopted after early adversity** (neglect, institutional care, foster care) may have attachment, trauma, or developmental needs (Chapter 12).
- Don't assume every problem an adopted person has is about adoption, or that adoption is never relevant. Explore its meaning with the client.
- **Birth parents** experience grief and loss, sometimes disenfranchised (Chapter 12).

Part 5 — Parenting and Co-Parenting (3.AW)

Who Owns Parenting

Owner	Contribution
Diana Baumrind	Parenting styles: **authoritative** (warm and firm; best outcomes), **authoritarian** (firm, not warm), **permissive** (warm, not firm); Maccoby & Martin added **neglectful/uninvolved**
Rudolf Dreikurs	Adlerian parenting; four **goals of misbehavior**: **attention, power, revenge, and display of inadequacy**; **natural and logical consequences**; family meetings; encouragement
Dinkmeyer & McKay	**STEP** (Systematic Training for Effective Parenting), based on Adlerian principles
Thomas Gordon	**PET** (Parent Effectiveness Training): active listening, I-messages, "who owns the problem," no-lose conflict resolution
Haim Ginott	Communicating with children: acknowledge feelings, set limits on behavior

Owner	Contribution
Gerald Patterson	**Coercion theory**: parents and children reinforce each other's escalating behavior (the child escalates until the parent gives in, which rewards the child's escalation; the parent's giving in is itself rewarded when the child's behavior stops); basis for **parent management training**
Carolyn Webster-Stratton	**Incredible Years** parenting program
Matthew Sanders	**Triple P** (Positive Parenting Program)
Sheila Eyberg	**PCIT** (Chapter 5)
Jane Nelsen	**Positive Discipline** (Adlerian)

Natural vs. Logical Consequences (Dreikurs)

Type	Meaning	Example
Natural consequence	Happens on its own, without adult action	A child who refuses a coat feels cold
Logical consequence	Set by the adult and **related** to the behavior	A child who rides a bike without a helmet loses bike time

A logical consequence should be **related, respectful, and reasonable**. A punishment unrelated to the behavior is not a logical consequence.

Dreikurs' Goals of Misbehavior

Goal	Child's mistaken belief	Adult's typical feeling
Attention	"I count only when I'm noticed"	Annoyed
Power	"I count only when I'm in control"	Angry, challenged
Revenge	"I count only when I hurt others as I feel hurt"	Hurt

Goal	Child's mistaken belief	Adult's typical feeling
Display of inadequacy	"I can't do anything right, so leave me alone"	Hopeless, helpless

The adult's own feeling is a clue to the child's goal.

Co-Parenting Conflict

Whether parents are together or apart, children do better when parents present a reasonably united front, don't undermine each other, and keep children out of adult conflict. In systems terms (Chapter 7), a strong **parental subsystem** with clear boundaries is the target (Minuchin).

Part 6 — Abuse and Violence in Families (3.AP, 3.AT, 3.AU)

Chapter 6 covered mandated reporting, how to respond to a child's disclosure, IPV lethality assessment, and safety planning. This section adds concepts the NCE may test about the effects and patterns of family violence.

Effects of Child Sexual Abuse: Traumagenic Dynamics (Finkelhor & Browne)

David Finkelhor and Angela Browne described four **traumagenic dynamics** that explain much of the harm of child sexual abuse:

Dynamic	Effect
Traumatic sexualization	Sexuality is shaped in developmentally inappropriate and harmful ways
Betrayal	Trust is broken by someone the child depended on, or by caregivers who didn't protect
Powerlessness	The child's will and sense of control are repeatedly overridden

Dynamic	Effect
Stigmatization	Shame, guilt, and a sense of being "damaged" become part of the child's self-image

Types of Intimate Partner Violence (Michael P. Johnson)

Type	Description
Intimate terrorism (coercive controlling violence)	Violence used as part of a pattern of **coercive control**; usually more frequent, severe, and escalating
Situational couple violence	Violence arising from specific conflicts that escalate, **without** a broader pattern of control; the most common type in community samples
Violent resistance	Violence used in response to a partner's intimate terrorism, often in self-defense
Mutual violent control	Both partners are controlling and violent (rare)

Why it matters: the type of violence affects safety and treatment decisions. Conjoint counseling is contraindicated while there is ongoing violence or coercive control (Chapter 6). Distinguishing types requires careful, **separate** assessment, never in front of the partner.

Children Exposed to IPV

Witnessing violence between caregivers is itself harmful to children and is associated with anxiety, depression, trauma symptoms, behavior problems, and later relationship difficulties. Some states treat exposure to domestic violence as a form of child maltreatment; know your state's reporting rules (Chapter 6).

Other Family Abuse

- **Sibling abuse** is often overlooked and dismissed as "normal fighting." Look for a **power imbalance**, repeated victimization of the same child, and fear.

- **Elder abuse** (Chapter 6, Chapter 12).
- **Emotional abuse**: persistent belittling, threats, rejection, isolation. It does lasting harm even without physical injury.
- **Teen dating violence**: screen adolescents; healthy relationship education is a prevention strategy.

What the Exam Rewards / What It Punishes

Rewards: - Knowing conflict, not divorce itself, is the bigger risk to children - Keeping children out of adult conflict; parallel parenting in high conflict - Checking custody documents before treating a minor of divorced parents - Declining to act as custody evaluator for one's own client - Normalizing slow stepfamily integration; biological parent disciplines early - Early, ongoing, age-appropriate adoption disclosure - Related, respectful, reasonable logical consequences - Assessing IPV type separately before any conjoint work

Punishes: - Assuming children of divorce will be damaged - Expecting "instant love" in stepfamilies - Stepparent taking over discipline early - Keeping adoption secret - Recommending custody for one's own client - Treating all IPV as the same - Dismissing sibling abuse as normal rivalry

Swap-Pair Drill

If you see...	It belongs to...	Not...
Intimacy, passion, commitment	Sternberg	Lee
Eros, ludus, storge, pragma, mania, agape	Lee	Sternberg
Satisfaction, alternatives, investments	Rusbult	Gottman

If you see…	It belongs to…	Not…
Adult romantic attachment styles	Hazan & Shaver	Ainsworth (infants)
Six stations of divorce	Bohannan	Kübler-Ross
Most children of divorce adjust well	Hetherington	Wallerstein
Sleeper effect	Wallerstein	Hetherington
Conflict matters more than divorce	Amato	Wallerstein
Fantasy, immersion, awareness…	Papernow	Visher & Visher
Instant love myth; wicked stepmother	Visher & Visher	Papernow
Loss, rejection, guilt, grief, identity, intimacy, mastery	Silverstein & Kaplan	Worden
Attention, power, revenge, inadequacy	Dreikurs	Baumrind
STEP	Dinkmeyer & McKay	Gordon (PET)
Who owns the problem; I-messages	Gordon (PET)	Dreikurs
Coercion theory	Patterson	Baumrind
Incredible Years	Webster-Stratton	Sanders (Triple P)
Traumagenic dynamics	Finkelhor & Browne	Walker
Intimate terrorism vs. situational couple violence	Michael P. Johnson	Walker’s cycle
Counselor can’t evaluate own client for court	ACA E.13.c	ACA A.8

Quick Checks

1. According to Sternberg, a relationship with intimacy and commitment but little passion is: A. Consummate love B. Companionate love C. Fatuous love D. Infatuation

2. A client stays in an unhappy relationship, saying, “We have a house, kids, and fifteen years together, and I don’t think I’d find anyone better.” This is best explained by: A. Sternberg’s triangular theory B. Rusbult’s investment model C. Lee’s love styles D. Bohannan’s stations

3. A parent asks whether divorce will “ruin” her children. Research most strongly supports telling her that: A. Most children of divorce have serious lasting problems B. Children’s adjustment depends heavily on how much conflict they’re exposed to and the quality of parenting C. Staying together is always better for children D. Children are unaffected by divorce

4. Two divorced parents can’t speak without arguing in front of their children. Which co-parenting arrangement is most likely to help the children? A. Cooperative co-parenting with joint family outings B. Parallel parenting with minimal direct contact between parents C. Having the children carry messages between homes D. Requiring weekly joint parent meetings with the children present

5. A divorced father brings his 10-year-old for counseling and says the mother doesn’t need to know. The counselor’s first step is to: A. Begin counseling, since the father brought the child B. Review the custody order to determine who has legal authority to consent C. Contact the mother to get her permission before treating the child D. Refuse to see the child

6. The father’s attorney later asks the child’s counselor to recommend which parent should have primary custody. The counselor should: A. Provide a recommendation, since the counselor knows the child best B. Explain that serving as a custody evaluator for one’s own client is a conflict of roles C. Recommend the parent who pays for counseling D. Ask the child which parent to recommend

7. In a new stepfamily, the stepfather starts enforcing strict rules on his stepchildren, who become angry and resentful. The best guidance is: A. Support the stepfather’s authority so the children learn respect B. Have the

biological parent take the lead on discipline while the stepfather builds a relationship with the children C. Have the children spend less time at home D. Recommend the couple separate

8. A newly remarried couple worries that, after a year, their blended family "still doesn't feel like a family." The counselor should: A. Assess for a family disorder B. Normalize that stepfamily integration often takes several years and explore expectations C. Recommend individual counseling for each child D. Suggest the couple was too hasty in marrying

9. Adoptive parents ask when to tell their 3-year-old she's adopted. The best guidance is: A. Wait until she's 18 B. Wait until she asks C. Begin now with simple, age-appropriate language, and keep it an ongoing conversation D. Never tell her, to protect her

10. A child who refuses to do homework, argues constantly, and leaves the parent feeling angry and challenged is most likely pursuing which goal of misbehavior (Dreikurs)? A. Attention B. Power C. Revenge D. Display of inadequacy

11. A teen who stays out past curfew loses use of the family car the next weekend. This is an example of: A. A natural consequence B. A logical consequence C. Negative reinforcement D. Extinction

12. A wife reports that her husband occasionally pushes her during heated arguments but doesn't otherwise control her money, contacts, or movements. According to Johnson's typology, this pattern is most consistent with: A. Intimate terrorism B. Situational couple violence C. Violent resistance D. Mutual violent control

Answers and Rationales

1 — B. Intimacy plus commitment is companionate love. A: consummate love has all three components. C: fatuous love is passion plus commitment. D: infatuation is passion alone.

2 — B. Investments and poor perceived alternatives keep people committed (Rusbult). A: Sternberg describes love's components. C: Lee describes love styles. D: Bohannan describes divorce.

3 — B. Research (Hetherington, Amato) emphasizes conflict exposure and parenting quality. A: most children adjust well. C: high-conflict marriages can be worse for children than divorce. D: overstates; children are affected, but most adjust.

4 — B. Parallel parenting reduces children's exposure to conflict. A and D: put the children in the conflict. C: using children as messengers is harmful.

5 — B. Legal authority to consent depends on the custody order. A: may proceed without proper consent. C: misstates the issue. D: premature; the child may be eligible for services with proper consent.

6 — B. Treating counselors don't serve as forensic evaluators for their own clients (ACA E.13.c). A: role conflict. C: payment doesn't determine recommendations. D: inappropriate and burdens the child.

7 — B. Early on, the biological parent handles discipline while the stepparent builds a relationship (Visher & Visher; Papernow). A: escalates resentment. C and D: unwarranted.

8 — B. Integration often takes several years; the "instant family" expectation is a myth. A: pathologizes a normal process. C: misses the family-level issue. D: unsupportive and unsupported.

9 — C. Early, age-appropriate, ongoing disclosure supports trust. A, B, D: secrecy and delay tend to harm trust and identity.

10 — B. Power struggles leave adults feeling angry and challenged. A: attention-seeking leaves adults annoyed. C: revenge leaves adults hurt. D: inadequacy leaves adults feeling hopeless.

11 — B. An adult-imposed consequence related to the behavior is logical. A: a natural consequence would happen without adult action. C: negative reinforcement increases behavior by removing something unpleasant. D: extinction withholds reinforcement.

12 — B. Violence arising in specific conflicts without a broader pattern of control fits situational couple violence. A: intimate terrorism involves coercive control. C: violent resistance responds to a partner's intimate terrorism. D: mutual violent control requires both partners to be controlling and violent. **Assessment must still be done separately and carefully; situational violence is still violence and requires safety assessment.**

Audio Companion

Track	Covers	Target length
13.1	Families in Transition: love, divorce, stepfamilies, adoption, parenting, violence	~6 min

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Chapter 14 — Career and Occupation

Domain map: 3.U Occupation and career development · 4.K Use assessment instrument results to facilitate client decision making · 2.M Select, use, and interpret appropriate assessment instruments · 3.AH Stress management (work-related) · CACREP: Career Development

Career Is Counseling

The NCE treats career development as one of the eight CACREP core areas, and career concerns show up across the domains: as an Area of Clinical Focus (3.U), in assessment (2.M, 4.K), and in life-span development. Chapter 3 gave you the career theorists. This chapter covers the work itself: the career counseling process, the instruments and information sources, work-related distress, and career concerns for specific groups.

One principle anchors this chapter: **career and personal concerns are intertwined.** Job loss can trigger depression; anxiety can freeze decision-making; family expectations shape choices. The NCE rewards counselors who address the whole person rather than treating career counseling as test-and-tell.

Part 1 — Theory Review in One Table

Owner	Theory	Use it when the stem mentions…
Parsons	Trait-factor	Know yourself, know the world of work, match them (“true reasoning”)

Owner	Theory	Use it when the stem mentions…
Williamson	Trait-factor (Minnesota)	Directive, six-step counseling: analysis, synthesis, diagnosis, prognosis, counseling, follow-up
Holland	Personality types and environments	RIASEC, hexagon, congruence, Self-Directed Search
Super	Life-span, life-space	Self-concept, life roles, Life-Career Rainbow, stages, career maturity
Ginzberg et al.	Developmental	Fantasy, tentative, realistic periods
Roe	Needs/personality	Early parent–child relationships; 8 occupational groups and 6 levels
Gottfredson	Circumscription and compromise	Children rule out options by gender and prestige
Krumboltz	Social learning; happenstance	Learning experiences, task-approach skills, planned happenstance
Lent, Brown, & Hackett	SCCT	Self-efficacy, outcome expectations, goals; barriers and supports
Savickas	Career construction	Career adaptability (concern, control, curiosity, confidence); life stories
Dawis & Lofquist	Theory of Work Adjustment	Satisfaction (worker) and satisfactoriness (employer)
Peterson, Sampson, & Reardon	Cognitive Information Processing	Pyramid; CASVE cycle; negative career thoughts

Owner	Theory	Use it when the stem mentions…
Tiedeman & O'Hara	Decision-making	Anticipation and implementation phases
Hoppock	Needs-based satisfaction	Job satisfaction depends on how well needs are met

Holland's Hexagon in Practice

Realistic — **I**nvestigative — **A**rtistic — **S**ocial — **E**nterprising — **C**onventional, arranged in a hexagon in that order.

Concept	Meaning	Example
Congruence	Fit between a person's type and the work environment's type	A Social person working as a teacher
Consistency	How close a person's top types are on the hexagon; **adjacent** types (e.g., RI, SE) are consistent; **opposite** types (e.g., RS, AC, IE) are inconsistent	Code "SEA" is fairly consistent
Differentiation	How clearly one type stands out from the others; a flat profile is **undifferentiated**	A profile with one high score and five low scores is well differentiated
Identity	Clarity and stability of one's goals, interests, and talents	Measured by *My Vocational Situation*

Opposite pairs on the hexagon: R–S, I–E, A–C.

Super's Stages and Tasks

Stage	Ages (approx.)	Key tasks
Growth	0–14	Developing self-concept, attitudes, and needs; learning about work

Stage	Ages (approx.)	Key tasks
Exploration	15–24	**Crystallizing** (forming a preference), **specifying** (narrowing to a choice), **implementing** (acting on it)
Establishment	25–44	**Stabilizing**, **consolidating**, and **advancing**
Maintenance	45–64	**Holding**, **updating**, and **innovating**
Disengagement	65+	**Decelerating**, **retirement planning**, **retirement living**

Recycling: people can re-enter earlier stages (for example, re-exploring after a layoff in midlife). For adults, Super later proposed **career adaptability** in place of **career maturity**; Savickas built career construction theory around adaptability.

Life-Career Rainbow: life **roles** (child, student, leisurite, citizen, worker, spouse/partner, homemaker, parent, pensioner) played in **theaters** (home, community, school, workplace) across the life span.

Part 2 — The Career Counseling Process

Most models share these steps:

1. **Establish the relationship** and clarify the client's goals and expectations. Many clients expect "a test that tells me what to do."
2. **Assess** interests, values, skills, personality, and barriers, using both formal instruments and conversation.
3. **Explore** occupational information and options.

4. **Decide**, weighing options against values and constraints.
5. **Act**: plan, implement (training, job search), and follow up.

The exam's career rules: - **Assessments inform decisions; they don't make them.** The counselor interprets results **with** the client and connects them to the client's goals (4.K). - **Interest isn't ability.** A high Artistic score says the client likes artistic activities, not that the client is talented at them. - Explore **barriers** (financial, family, discrimination, disability, low self-efficacy) and **supports** (SCCT). - Address **mental health** issues that affect decisions: anxiety, depression, perfectionism.

Indecision vs. Indecisiveness

Term	Meaning	Approach
Career indecision	A developmental, often temporary lack of a career decision, usually due to limited information or experience	Information, exploration, experience
Indecisiveness	A **chronic**, generalized difficulty making decisions across life areas, often linked to anxiety	Address the underlying anxiety and decision-making patterns

Decision-Making Styles (Harren)

Style	Description
Rational	Systematic, logical, considers future consequences
Intuitive	Relies on feelings and fantasy; accepts responsibility
Dependent	Relies on others; denies personal responsibility

Part 3 — Career Assessment Instruments

Instrument	Measures	Owner / notes
Strong Interest Inventory (SII)	Interests; General Occupational Themes (Holland's RIASEC), Basic Interest Scales, Occupational Scales, Personal Style Scales	E. K. Strong; compares the client's interests with those of people satisfied in their occupations
Self-Directed Search (SDS)	RIASEC code; **self-administered, self-scored**	Holland
Kuder (Career Search, Occupational Interest Survey)	Interests	Frederic Kuder
O*NET Interest Profiler	RIASEC interests; free	U.S. Department of Labor
Myers-Briggs Type Indicator (MBTI)	Personality type (E/I, S/N, T/F, J/P)	Myers & Briggs, based on Jung; used for self-understanding, not for selecting a career
Career Maturity Inventory (CMI)	Career readiness attitudes and competencies	John Crites
Career Development Inventory (CDI)	Career maturity	Super and colleagues
Career Decision Scale	Career indecision	Samuel Osipow
My Vocational Situation	Vocational identity, need for information, barriers	Holland
Career Beliefs Inventory	Beliefs that block career progress	Krumboltz
Career Thoughts Inventory	Negative career thoughts	Sampson et al. (CIP)
Minnesota Importance Questionnaire (MIQ)	Work **values** and needs	Dawis & Lofquist (TWA)

Instrument	Measures	Owner / notes
ASVAB	Aptitudes; used for military enlistment and career exploration	Department of Defense
GATB	General aptitudes	U.S. Employment Service
Card sorts	Interests or values, qualitatively	Tyler (Tyler Vocational Card Sort)

Interest vs. aptitude vs. achievement vs. values: - **Interest**: what you like - **Aptitude**: what you could learn to do (potential) - **Achievement**: what you've already learned - **Values**: what matters to you in work

Occupational Information Sources

Source	What it is
O*NET (Occupational Information Network)	U.S. Department of Labor database of occupations; **replaced the Dictionary of Occupational Titles (DOT)**
Occupational Outlook Handbook (OOH)	Bureau of Labor Statistics guide to job duties, education, pay, and outlook; updated regularly
Standard Occupational Classification (SOC)	Federal system for classifying occupations
Informational interviewing, job shadowing, internships	First-hand information

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Part 4 — Work, Unemployment, and Distress

Job Loss (Jahoda's Latent Deprivation Model)

Marie Jahoda argued that work provides **manifest** benefits (income) and **latent** benefits: **time structure, social contact, collective purpose, status and identity, and regular activity.** Unemployment harms well-being partly because people lose these latent benefits, not just income.

Counseling after job loss: acknowledge the grief and identity loss; screen for depression, substance use, and suicide risk; rebuild structure and activity; address practical needs (finances, benefits, job search); and reframe where possible (Krumboltz's planned happenstance).

Burnout (Christina Maslach)

Three dimensions, measured by the **Maslach Burnout Inventory (MBI)**: 1. **Emotional exhaustion** 2. **Depersonalization** (cynicism, detachment from clients or work) 3. **Reduced personal accomplishment** (inefficacy)

Burnout vs. compassion fatigue vs. vicarious trauma:

Term	Meaning
Burnout	Exhaustion and cynicism from chronic workplace stress
Compassion fatigue (Charles Figley)	Emotional and physical exhaustion from caring for people who are suffering; includes secondary traumatic stress
Vicarious trauma (McCann & Pearlman)	Lasting changes in the helper's beliefs and worldview from repeated exposure to clients' trauma

These apply to counselors too: self-care and impairment are ethical issues (ACA C.2.g; Chapter 17).

Other Workplace Concerns

- **Work–family conflict**: competing demands between roles (Super's Life-Career Rainbow is a useful frame).
- **Underemployment**: working below one's skills or desired hours.
- **Workplace discrimination and harassment**: validate, help the client document incidents, and explore options (HR, legal resources, advocacy).
- **Glass ceiling**: invisible barriers to advancement for women and people of color.

- **Job satisfaction**: Hoppock; Dawis & Lofquist's satisfaction and satisfactoriness.
- **Employee Assistance Programs (EAPs)**: short-term, employer-sponsored counseling and referral.

Part 5 — Career Concerns for Specific Groups

Group	Key issues
Women	Gender-role socialization narrows options early (Gottfredson); lower self-efficacy in some fields (Betz & Hackett); work–family role conflict; pay gaps
Racial and ethnic minority clients	Discrimination; occupational stereotyping; family and community expectations; collectivist decision-making (involve family when the client wants that)
LGBTQ+ clients	Disclosure decisions at work; workplace climate; discrimination
People with disabilities	Accommodations under the ADA; **vocational rehabilitation** services; matching abilities to work; avoiding assumptions about limits
Veterans	Translating military skills to civilian jobs; identity shift; VA vocational services
Older workers	Age discrimination; bridge employment; retirement planning (Chapter 12)
Clients with criminal records	Barriers to hiring; "ban the box" policies; reentry programs
First-generation college students	Less family knowledge of career paths; financial pressure

Career self-efficacy (Betz & Hackett, building on Bandura) helps explain why capable clients avoid fields: low

confidence, not low ability. Building self-efficacy through mastery experiences, modeling, encouragement, and managing anxiety opens options.

Part 6 — Ethics in Career Counseling

- The **National Career Development Association (NCDA)** has its own Code of Ethics for career professionals.
- Use instruments you're **trained** to administer and interpret (ACA E.2).
- Choose instruments **normed** on groups similar to the client, and interpret cautiously when they aren't (ACA E.8).
- Online and computer-based assessments still require the counselor to make sure the instrument is valid and that the client receives a sound interpretation of the results (ACA E.7, H).
- Don't let a single test score determine a client's choice.

What the Exam Rewards / What It Punishes

Rewards: - Integrating career and personal concerns - Interpreting assessments **with** the client and linking results to goals - Distinguishing interest from aptitude - Exploring barriers, supports, and self-efficacy - Treating job loss as a loss: screening mood and suicide risk - Using O*NET and the OOH for occupational information

Punishes: - "Test-and-tell": letting a score choose the career - Treating a high interest score as proof of ability - Ignoring cultural and family context in decisions - Using the MBTI to select a career - Ignoring depression or anxiety in a career client

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Swap-Pair Drill

If you see…	It's…	Not…
Opposite on the hexagon	R–S, I–E, A–C	Adjacent types
Adjacent top codes	Consistency	Differentiation
One type clearly highest	Differentiation	Congruence
Person–environment fit	Congruence	Consistency
Self-administered, self-scored RIASEC	SDS (Holland)	Strong Interest Inventory
Compares interests with satisfied workers	Strong Interest Inventory	SDS
Work values	MIQ (Dawis & Lofquist)	Strong
Career indecision	Career Decision Scale (Osipow)	Career Maturity Inventory (Crites)
Blocking career beliefs	Career Beliefs Inventory (Krumboltz)	Career Thoughts Inventory
Replaced the DOT	O*NET	OOH
Crystallizing, specifying, implementing	Super's exploration stage	Ginzberg
Time structure, social contact, purpose, status, activity	Jahoda	Maslach
Exhaustion, depersonalization, reduced accomplishment	Maslach (burnout)	Figley (compassion fatigue)
Career decision-making styles: rational, intuitive, dependent	Harren	Tiedeman
Chronic difficulty deciding anything	Indecisiveness	Career indecision

Quick Checks

1. A client's Holland code is "RSE." The R and S types are: A. Adjacent and consistent B. Opposite and inconsistent C. Undifferentiated D. Congruent

2. A client's SDS shows high scores on all six RIASEC types. This profile is: A. Highly differentiated B. Undifferentiated C. Highly consistent D. Congruent

3. A client asks, "Which career does the test say I should choose?" The best response is to: A. Give the client the top-matched occupation B. Explain that results are one source of information and explore them together in light of the client's goals and values C. Recommend the client take more tests D. Refuse to discuss the results

4. A 52-year-old client laid off after 25 years says he feels "useless" and hasn't left the house in weeks. The counselor's first priority is to: A. Administer the Strong Interest Inventory B. Assess mood and suicide risk C. Update his résumé D. Explain Super's recycling concept

5. According to Jahoda, unemployment harms well-being partly because people lose: A. Only income B. Latent benefits such as time structure, social contact, and purpose C. Career maturity D. Congruence

6. A client with strong math ability avoids engineering because she "isn't a math person." Which theory best explains this and suggests building her confidence? A. Holland's congruence B. SCCT / career self-efficacy C. Roe's needs theory D. Ginzberg's developmental theory

7. Which source replaced the Dictionary of Occupational Titles? A. Occupational Outlook Handbook B. O*NET C. SOC D. ASVAB

8. A college junior has narrowed her interests to two related fields and is choosing between them. In Super's model, she is: A. Crystallizing B. Specifying C. Stabilizing D. Holding

9. A nurse feels emotionally drained, has become cynical about patients, and feels she's no longer effective. This best matches: A. Vicarious trauma B. Burnout C. Indecisiveness D. Underemployment

10. A client struggles to make decisions about everything, from careers to meals, and feels anxious about any choice. This is best described as: A. Career indecision B. Indecisiveness C. Low differentiation D. Low congruence

11. Which instrument measures work values rather than interests? A. Strong Interest Inventory B. Self-Directed Search C. Minnesota Importance Questionnaire D. Kuder

12. A counselor uses the MBTI to tell a client that, as an "INFP," she should become a writer. This practice is: A. Appropriate, because MBTI types predict career success B. Inappropriate, because the MBTI is for self-understanding and shouldn't be used to choose a career C. Required by NCDA ethics D. Appropriate if the client agrees

Answers and Rationales

1 — B. R and S are opposite on the hexagon, so the code is inconsistent. A: adjacent types would be, for example, R and I. C: differentiation concerns how much one type stands out. D: congruence concerns person–environment fit.

2 — B. A flat profile is undifferentiated. A: differentiation means one or two types stand out. C: consistency concerns hexagon position of top types. D: congruence is environment fit.

3 — B. Assessments inform; the client decides, with the counselor helping interpret (4.K). A: test-and-tell. C: more tests don't address the misunderstanding. D: unhelpful.

4 — B. Withdrawal and feeling useless after job loss call for screening mood and suicide risk first. A, C, D: useful later.

5 — B. Jahoda's latent deprivation model. A: income is the manifest benefit. C and D: Super and Holland concepts.

6 — B. Low self-efficacy despite ability is central to SCCT (Lent, Brown, & Hackett; Betz & Hackett). A: Holland's fit. C: Roe's childhood needs. D: Ginzberg's developmental periods.

7 — B. O*NET replaced the DOT. A: the OOH is a separate BLS publication. C: SOC is a classification system. D: the ASVAB is an aptitude test.

8 — B. Narrowing to a specific choice is specifying. A: crystallizing forms a general preference. C: stabilizing is in establishment. D: holding is in maintenance.

9 — B. Maslach's three burnout dimensions. A: vicarious trauma involves changes in beliefs from exposure to trauma. C: indecisiveness is chronic difficulty deciding. D: underemployment is working below one's capacity.

10 — B. Chronic, generalized difficulty deciding is indecisiveness. A: career indecision is specific and often developmental. C and D: Holland concepts.

11 — C. The MIQ measures work values and needs (TWA). A, B, D: interest inventories.

12 — B. The MBTI supports self-understanding; it isn't designed to dictate career choice. A: overstates its predictive power. C: not an NCDA requirement. D: client agreement doesn't fix a misuse of the instrument.

Audio Companion

Track	Covers	Target length
14.1	Work It: Holland, Super, instruments, burnout, job loss	~4 min

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Chapter 15 — Culture, Identity, and Meaning

Domain map: 3.F Cultural adjustments · 3.J Gender identity development · 3.Z Racism/discrimination/oppression · 3.AA Religious values conflict · 3.AG Spiritual/existential concerns · 6.D Demonstrate knowledge of and sensitivity to gender orientation and gender issues · 6.E Demonstrate knowledge of and sensitivity to multicultural issues · 6.L Respect and acceptance for diversity · CACREP: Social and Cultural Diversity; Human Growth and Development

Where This Chapter Fits

Chapter 9 covered the **counselor's** side of culture: multicultural competencies, ADDRESSING, microaggressions, broaching, interpreters, ASERVIC, and the ban on values-based referral. Chapter 3 gave you the identity development models (Cross, Helms, Atkinson/Morten/Sue, Cass, Berry). This chapter covers the **client's** concerns: adjusting to a new culture, developing a gender identity, living with racism and oppression, conflicts over religious values, and questions of meaning.

The through-line here: **the client defines their identity and the meaning of their experience.** The counselor's job is to understand, validate, and support, not to diagnose identity or decide what a client's faith or culture should mean to them.

Part 1 — Cultural Adjustment (3.F)

Culture Shock

Kalervo Oberg described **culture shock** as the anxiety that comes from losing familiar cultural cues. A commonly taught sequence:

Phase	Experience
Honeymoon	Excitement and fascination with the new culture
Crisis (frustration)	Irritation, homesickness, confusion, anxiety as differences become stressful
Recovery (adjustment)	Learning the new culture's rules; growing competence
Adaptation	Functioning comfortably in the new culture

The **U-curve** (Lysgaard) describes adjustment that starts high, dips, then recovers. The **W-curve** (Gullahorn & Gullahorn) adds a second dip on returning home: **reverse culture shock**.

Acculturation

Review Berry's four strategies from Chapter 3: **integration**, **assimilation**, **separation**, **marginalization**. Integration is generally associated with the best adjustment, and marginalization with the most distress, though fit depends on context.

Acculturative stress is the stress that comes from adapting to a new culture: language barriers, discrimination, loss of status, separation from family, and changing roles.

Acculturation gaps within families: children often acculturate faster than parents, which can lead to conflict and to role reversal (children acting as interpreters and brokers; Chapter 9 explains why counselors shouldn't use them that way).

Immigrants and Refugees

Phase	Possible stressors
Pre-migration	War, persecution, violence, poverty, loss
Migration	Dangerous journeys, detention, family separation, camps
Post-migration	Acculturative stress, discrimination, legal uncertainty, unemployment or underemployment, isolation, grief

Counseling principles: - Address **safety and basic needs** first (Maslow). - Use **professional interpreters** (Chapter 9). - Assess trauma with care and **cultural humility**; distress may be expressed through physical complaints or culturally specific idioms. - Use the **Cultural Formulation Interview** (Chapter 9). - Build on **cultural strengths**: community, faith, family, and resilience. - Be aware that fears about legal status may affect disclosure and trust; clarify confidentiality.

Cultural Concepts of Distress (DSM-5-TR)

The DSM-5-TR describes **cultural concepts of distress**: ways cultures experience, understand, and communicate suffering. Examples include ***ataque de nervios*** (Latin American and Caribbean; episodes of intense emotional upset, shouting, crying, trembling), ***susto*** (“fright”; illness attributed to a frightening event that causes the soul to leave the body), ***taijin kyofusho*** (Japan; fear that one’s body or behavior offends others), ***khyâl cap*** (Cambodian “wind attacks” resembling panic), and ***kufungisisa*** (“thinking too much,” Zimbabwe).

Exam principle: understand a client’s distress in the client’s own cultural terms before applying a Western diagnosis.

Part 2 — Gender Identity Development (3.J, 6.D)

Terms

Term	Meaning
Sex assigned at birth	The sex recorded at birth, usually based on external anatomy
Gender identity	One's internal sense of one's own gender
Gender expression	How one outwardly presents gender (clothing, voice, behavior)
Sexual orientation	Whom one is romantically or sexually attracted to; **distinct from gender identity**
Transgender	Gender identity differs from sex assigned at birth
Cisgender	Gender identity matches sex assigned at birth
Nonbinary	Gender identity outside the woman/man binary
Intersex (differences of sex development)	Variations in sex characteristics

Being transgender is not a mental disorder. The DSM-5-TR diagnosis of **gender dysphoria** describes clinically significant **distress** related to incongruence between experienced gender and assigned gender. Criteria require a duration of at least **6 months**, with separate criteria for children and for adolescents and adults.

Who Owns Identity Development Models

Owner	Model
Arlene Istar Lev	**Transgender emergence**: awareness → seeking information and reaching out → disclosure to significant others → exploring identity and self-labeling → exploring transition issues and possible body modification → integration (acceptance and post-transition issues)
Vivienne Cass	Homosexual identity formation (Chapter 3): confusion → comparison → tolerance → acceptance → pride → synthesis
Richard Troiden	Sensitization → identity confusion → identity assumption → commitment
Anthony D'Augelli	Life-span model of lesbian, gay, and bisexual identity development; identity unfolds through six interrelated processes rather than fixed stages
Ilan Meyer	**Minority stress model** (below)
Caitlin Ryan	**Family Acceptance Project**: family rejection of LGBTQ youth is linked to much higher risk of depression, suicide attempts, and substance use; family acceptance is protective

Minority Stress (Meyer)

Type	Examples
Distal stressors (external)	Discrimination, harassment, violence, rejection
Proximal stressors (internal)	**Expectations of rejection**, **concealment** of identity, **internalized stigma**

Minority stress helps explain higher rates of depression, anxiety, and suicidality among sexual and gender minority people. The problem is the stress, not the identity.

Counseling Principles

- Use the client's **name and pronouns**.
- The client controls **coming out**: when, how, and to whom. Never disclose a client's identity without consent.
- Don't assume identity is the presenting concern, or that it's irrelevant.
- **Affirming** care; **conversion therapy is harmful** and contrary to professional standards (Chapter 9).
- Support families toward acceptance (Ryan).
- Know that the **World Professional Association for Transgender Health (WPATH)** publishes Standards of Care (Version 8, 2022) for transgender and gender diverse people. You may be asked to write letters supporting gender-affirming medical care; do so only within your competence and current standards.

Part 3 — Racism, Discrimination, and Oppression (3.Z)

Who Owns the Concepts

Owner	Concept
Robert T. Carter	**Race-based traumatic stress**: racism can produce trauma-like reactions
Sherman James	**John Henryism**: high-effort coping with chronic stressors (including discrimination), linked to health problems in Black Americans with limited resources
Claude Steele & Joshua Aronson	**Stereotype threat**: concern about confirming a negative stereotype about one's group lowers performance
Pauline Clance & Suzanne Imes	**Impostor phenomenon**: high achievers feeling like frauds; often heightened for people in groups underrepresented in their fields

Owner	Concept
Greenwald, McGhee, & Schwartz	**Implicit Association Test (IAT)**: measures implicit bias
Derald Wing Sue	Microaggressions (Chapter 9)
Iris Marion Young	**Five faces of oppression**: exploitation, marginalization, powerlessness, cultural imperialism, violence
Paulo Freire	**Critical consciousness** (conscientização): recognizing and acting against oppressive conditions
Ignacio Martín-Baró	**Liberation psychology**
Kimberlé Crenshaw	**Intersectionality** (Chapter 9)
Peggy McIntosh	"White Privilege: Unpacking the Invisible Knapsack"

Levels of Racism

Level	Description
Individual (interpersonal)	Prejudice and discrimination between people
Institutional	Policies and practices within organizations that produce unequal outcomes
Structural (systemic)	The combined effects of institutions and history that sustain inequality across society
Internalized	A member of a targeted group comes to accept negative messages about their own group

Colorism: discrimination based on skin tone, often within as well as across racial groups.

Counseling Principles

- **Validate** experiences of discrimination. Questioning whether racism "really" happened is a microinvalidation and damages the alliance.

- **Assess the impact**: race-based stress can look like anxiety, depression, hypervigilance, anger, or trauma symptoms.
- Recognize **cultural mistrust** as a reasonable response to historical and current experiences, not paranoia.
- Explore **coping and strengths**: racial and ethnic identity, community, faith, activism.
- Consider **advocacy** at the client, community, or public level (ACA Advocacy Competencies, Chapter 9).
- Counselors examine their **own** biases and privilege (Chapter 9).

Part 4 — Religious Values Conflict (3.AA)

Concepts and Owners

Owner	Concept
Gordon Allport	**Intrinsic religiosity** (faith as an end in itself, lived as the center of life) vs. **extrinsic religiosity** (faith used as a means, e.g., for social standing or comfort)
Kenneth Pargament	**Religious coping**: **positive** (seeking spiritual support, benevolent religious reappraisal) and **negative** (feeling punished or abandoned by God; spiritual struggle). Negative religious coping is linked to worse outcomes
James Fowler	Stages of faith (Chapter 9)
ASERVIC	Spiritual competencies (Chapter 9)

Common Religious Values Conflicts

- **Faith and identity**: a client whose sexual orientation or gender identity conflicts with their religious community's teachings
- **Faith transitions**: questioning, leaving, changing, or deepening faith

- **Interfaith relationships and family conflict** over religion
- **Religious scrupulosity**: obsessive fear of sin or offending God (an OCD presentation, Chapter 10)
- **Spiritual struggle** after loss or trauma ("Why did God let this happen?")
- **Religious trauma or harm** from coercive religious environments

Counseling Principles

- **The client leads.** Explore what the client values and wants, not what the counselor believes is right.
- **Don't impose values** (ACA A.4.b) in either direction: neither pushing a client to leave a faith nor to stay.
- Help clients **hold tension** and explore options; resolution may be integration, change, or acceptance of ambiguity.
- With the client's consent, faith leaders or spiritual practices can be resources.
- **Moral, Religious, or Spiritual Problem (Z65.8)** is a DSM-5-TR condition that may be a focus of clinical attention, not a mental disorder. It was called "Religious or Spiritual Problem" until the American Psychiatric Association's September 2025 update, which kept the code and added **moral problems**: experiences that disrupt one's understanding of right and wrong, or one's sense of the goodness of oneself, others, or institutions (this includes moral injury; Chapter 9). Religious or spiritual problems include loss or questioning of faith, problems with conversion to a new faith, and questioning spiritual values, whether or not they involve an organized religion.

Part 5 — Spiritual and Existential Concerns (3.AG)

Owners

Owner	Concept
Irvin Yalom	Four ultimate concerns: **death, freedom, isolation, meaninglessness** (Chapter 2)
Viktor Frankl	**Will to meaning**; meaning through creative work, love and experience, and the attitude one takes toward unavoidable suffering
Rollo May	Normal vs. neurotic anxiety
Ernest Becker	*The Denial of Death*
Greenberg, Solomon, & Pyszczynski	**Terror management theory**: awareness of death motivates people to hold onto cultural worldviews and self-esteem as buffers
John Welwood	**Spiritual bypassing**: using spiritual ideas or practices to avoid dealing with emotional pain or unresolved issues
Abraham Maslow	Peak experiences; self-transcendence (later added above self-actualization)
Stanislav & Christina Grof	**Spiritual emergency**: intense spiritual experiences that can resemble psychosis

Existential Concerns in Counseling

Clients may present with questions like “What’s the point?”, fear of death, loneliness that persists even in the company of others (**existential isolation**), anxiety about choices and responsibility, or a loss of meaning after retirement, illness, or loss.

Counseling principles: - Treat these as real human questions, not only as symptoms. - Screen for **depression and suicide risk** when meaninglessness is prominent

(Chapter 6, Chapter 10). - Support **meaning-making** (Frankl, Neimeyer) and **authentic choice**. - Distinguish **spiritual experiences** from psychosis with care: consider the client's culture, whether the experience is shared within the community, its effect on functioning, and the presence of other psychotic symptoms (Chapter 16).

What the Exam Rewards / What It Punishes

Rewards: - Understanding distress in the client's cultural terms - Normalizing culture shock and acculturation stress - Using the client's name and pronouns; client-controlled disclosure - Framing minority stress, not identity, as the source of distress - Validating experiences of discrimination - Client-led exploration of religious conflicts without imposing values - Taking existential questions seriously while screening for risk

Punishes: - Pathologizing gender identity or sexual orientation - Outing a client - Questioning whether discrimination "really" happened - Labeling cultural mistrust as paranoia - Pushing a client toward or away from a faith - Treating every spiritual experience as psychosis, or none of them

Swap-Pair Drill

If you see…	It's…	Not…
Honeymoon, crisis, recovery, adaptation	Oberg's culture shock	Kübler-Ross
Second dip on returning home	W-curve; reverse culture shock	U-curve
Ataque de nervios, susto	Cultural concepts of distress	Panic disorder
Gender identity vs. sexual orientation	Distinct constructs	The same thing
Distress about gender incongruence	Gender dysphoria	Being transgender

If you see…	It's…	Not…
Transgender emergence	Lev	Cass
Expected rejection, concealment, internalized stigma	Proximal minority stressors (Meyer)	Distal stressors
Family acceptance protects LGBTQ youth	Caitlin Ryan	Meyer
Race-based traumatic stress	Robert Carter	Sue
High-effort coping and health	John Henryism (Sherman James)	Stereotype threat
Fear of confirming a stereotype lowers performance	Stereotype threat (Steele & Aronson)	Impostor phenomenon
Feeling like a fraud	Impostor phenomenon (Clance & Imes)	Stereotype threat
Five faces of oppression	Iris Marion Young	Freire
Critical consciousness	Freire	Martín-Baró
Intrinsic vs. extrinsic religiosity	Allport	Pargament
Positive vs. negative religious coping	Pargament	Allport
Fear of death drives worldview defense	Terror management theory	Yalom
Using spirituality to avoid pain	Spiritual bypassing (Welwood)	Spiritual emergency (Grof)
Moral, religious, or spiritual problem	Z65.8 (focus of attention, not a disorder)	A mental disorder

Quick Checks

1. Four months after moving to the U.S. for graduate school, an international student who was initially excited now feels irritable, homesick, and confused. She is most

likely in which phase of culture shock? A. Honeymoon B. Crisis C. Recovery D. Adaptation

2. A client from El Salvador describes episodes of uncontrollable shouting, crying, and trembling after family conflicts, which she calls *ataque de nervios*. The counselor's best first step is to: A. Diagnose panic disorder B. Explore what the experience means to her and her family, using the Cultural Formulation Interview C. Refer her for a psychotic disorder evaluation D. Tell her it's a cultural myth

3. Which statement is accurate? A. Being transgender is a mental disorder in the DSM-5-TR B. Gender dysphoria refers to clinically significant distress related to gender incongruence C. Gender identity and sexual orientation are the same thing D. Gender dysphoria requires a duration of 2 years

4. A gay client hasn't told his coworkers about his partner and constantly monitors what he says. According to Meyer, this is an example of: A. A distal stressor B. A proximal stressor (concealment) C. Internalized racism D. Stereotype threat

5. A 16-year-old client comes out as transgender to her counselor but hasn't told her parents. The parents call to ask what their daughter discusses in counseling. The counselor should: A. Tell the parents about her gender identity B. Protect the client's disclosure within the limits of confidentiality and state law, and explore with the client whether, when, and how she wants to tell them C. Refer the client to a specialist without explanation D. Encourage her to keep it from her parents permanently

6. A Black client describes repeated discrimination at work and asks, "Am I being too sensitive?" The best response is: A. "Maybe they didn't mean it that way." B. Validate the experience and explore its impact on him C. Tell him to report it to HR immediately D. Change the subject to his treatment goals

7. A high-performing Latina engineer says, “Any day now, they’ll realize I don’t belong here.” This best reflects: A. Stereotype threat B. Impostor phenomenon C. John Henryism D. Acculturative stress

8. A client says that after his child’s death, he feels God has abandoned and is punishing him. Pargament would call this: A. Positive religious coping B. Negative religious coping C. Extrinsic religiosity D. Spiritual bypassing

9. A client who is a devout member of a conservative faith is struggling to reconcile her faith with her bisexual identity. The counselor should: A. Encourage her to leave her faith community B. Encourage her to change her orientation to fit her faith C. Help her explore her own values and options without imposing the counselor’s views D. Refer her to her pastor

10. A client meditates for hours daily and insists “everything happens for a reason” whenever she talks about her abusive childhood, refusing to discuss her feelings. This may reflect: A. Spiritual emergency B. Spiritual bypassing C. Positive religious coping D. Terror management

11. “Moral, religious, or spiritual problem” (Z65.8) in the DSM-5-TR is: A. A mental disorder B. A condition that may be a focus of clinical attention C. A cultural concept of distress D. A personality disorder

12. Yalom’s four ultimate concerns are: A. Death, freedom, isolation, meaninglessness B. Exploitation, marginalization, powerlessness, violence C. Loss, rejection, guilt, grief D. Safety, remembrance, mourning, reconnection

Answers and Rationales

1 — B. The shift from excitement to frustration and homesickness is the crisis phase (Oberg). A: honeymoon is the excited early period. C and D: recovery and adaptation bring growing comfort.

2 — B. Understand cultural concepts of distress in the client's terms first. A and C: premature Western diagnosis. D: dismissive.

3 — B. Gender dysphoria is distress related to incongruence. A: being transgender isn't a disorder. C: they're distinct. D: the duration is 6 months.

4 — B. Concealment is a proximal minority stressor. A: distal stressors are external events like discrimination. C: internalized racism concerns race. D: stereotype threat concerns performance.

5 — B. Disclosure belongs to the client; protect it within confidentiality law and support her decision-making. A: outing her could harm her. C: unexplained referral is abandonment. D: imposes the counselor's view.

6 — B. Validation and exploring impact. A: a microinvalidation. C: premature direction before understanding. D: avoids the client's concern.

7 — B. Feeling like a fraud despite success is the impostor phenomenon (Clance & Imes). A: stereotype threat is performance anxiety about confirming a stereotype in a specific situation. C: John Henryism is effortful coping linked to health. D: acculturative stress relates to adapting to a new culture.

8 — B. Feeling punished or abandoned by God is negative religious coping (Pargament). A: positive coping seeks spiritual support. C: extrinsic religiosity is Allport's concept of faith used as a means. D: bypassing uses spirituality to avoid feelings.

9 — C. Client-led exploration without imposing values (ACA A.4.b). A and B: impose a direction. D: referral away from counseling based on the topic isn't warranted; involving clergy would happen only at her request and with her consent.

10 — B. Using spiritual ideas to avoid emotional pain is spiritual bypassing (Welwood). A: spiritual emergency is an intense, destabilizing spiritual experience. C: positive

coping uses faith for support while still facing distress. D: terror management concerns death awareness.

11 — B. Z65.8 is a condition that may be a focus of clinical attention. A: not a disorder. C: cultural concepts are culture-specific idioms of distress. D: not a personality disorder.

12 — A. Yalom's ultimate concerns. B: Young's faces of oppression (minus cultural imperialism). C: part of Silverstein and Kaplan's core adoption issues. D: Herman's stages of trauma recovery.

Audio Companion

Track	Covers	Target length
15.1	Identity and Meaning: culture shock, gender identity, minority stress, racism, faith, existential concerns	~4 min

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Chapter 16 — Body and Mind

Domain map: 3.A Adjustment related to physical loss/injury/medical condition · 3.C Behavioral problems · 3.D Bullying · 3.E Caregiving concerns · 3.I Financial issues · 3.M Loneliness/attachment · 3.N Hyper/hypo mental focus · 3.P Insomnia/sleep issues · 3.Q Maladaptive eating behaviors · 3.AE Sexual functioning concerns · 3.AF Sleeping habits · 3.AH Stress management · 3.AL Visual/auditory hallucinations · CACREP: Counseling and Helping Relationships; Human Growth and Development

The Last Clinical Focus Chapter

This chapter gathers the remaining Areas of Clinical Focus. They're grouped loosely around the connection between body and mind: illness, sleep, eating, sex, stress, and attention, plus behavior problems, bullying, caregiving, money, loneliness, and hallucinations.

Two rules organize this chapter: 1. **Rule out the medical first.** Many of these concerns have medical causes or medical risks. The counselor's job is to make sure a medical evaluation happens, then to work within scope. 2. **Function over label.** Ask what the problem does in the client's life and what it costs, not just what to call it.

Part 1 — Adjustment to Illness, Injury, and Disability (3.A)

Owners

Owner	Contribution
George Engel (1977)	**Biopsychosocial model**: illness involves biological, psychological, and social factors

Owner	Contribution
Irwin Rosenstock and colleagues	**Health belief model**: health behavior depends on perceived **susceptibility**, perceived **severity**, perceived **benefits**, perceived **barriers**, **cues to action**, and (later) **self-efficacy**
Moos & Holahan	Adaptive tasks of coping with illness: dealing with symptoms, treatment, and providers; preserving emotional balance, self-image, and relationships; preparing for an uncertain future
Beatrice Wright	Disability and rehabilitation psychology; acceptance of disability; the "spread" effect (assuming one limitation means global limitation)

What Adjustment Looks Like

Reactions to illness, injury, amputation, chronic pain, or acquired disability vary widely: shock, anxiety, grief, anger, denial, depression, and gradual adaptation. **Adjustment isn't linear and doesn't follow a fixed sequence.** Grief over lost abilities is normal; persistent hopelessness, withdrawal, or suicidal thinking needs assessment.

Related Presentations (DSM-5-TR)

Presentation	Key features
Somatic symptom disorder	One or more distressing physical symptoms plus **excessive thoughts, feelings, or behaviors** about them; at least **6 months** of being symptomatic. The symptoms may or may not have a medical explanation
Illness anxiety disorder	Preoccupation with having or getting a serious illness; **few or no actual somatic symptoms**; at least 6 months

Presentation	Key features
Functional neurological symptom disorder (conversion disorder)	Neurological symptoms (weakness, seizures, numbness) inconsistent with recognized neurological conditions
Psychological factors affecting other medical conditions	Psychological or behavioral factors that worsen a medical condition (e.g., anxiety worsening asthma)
Adjustment disorder	Distress in response to a medical diagnosis or other stressor

Exam principle: never tell a client their symptoms are "all in your head." Validate the suffering, coordinate with medical providers, and focus on coping and functioning.

What Helps

- **CBT** and **ACT** for chronic pain and illness-related distress
- Coping skills, pacing, and problem-solving
- **Integrated care** with medical providers (with a release of information)
- Support groups
- Attention to **meaning**, identity, and relationships
- Accommodations and advocacy for clients with disabilities (ADA; Chapter 9)

Part 2 — Behavioral Problems (3.C)

Presentations

Presentation	Key features	Time frame
Oppositional defiant disorder	Angry/irritable mood, argumentative/ defiant behavior, or vindictiveness; at least **4 symptoms**, shown with at least one person who isn't a sibling	At least **6 months**

Presentation	Key features	Time frame
Conduct disorder	Violating others' rights or major norms: aggression to people and animals, destruction of property, deceitfulness or theft, serious rule violations; at least **3 of 15** criteria	In the past **12 months**, with at least one in the past 6 months. **Childhood-onset** (at least one symptom before age 10) vs. **adolescent-onset**. Specifier: **with limited prosocial emotions**
Intermittent explosive disorder	Recurrent outbursts of verbal or physical aggression out of proportion to the trigger	Age at least 6
Antisocial personality disorder	Pervasive disregard for others' rights	Age at least 18, with evidence of conduct disorder before age 15

DMDD vs. ODD: if criteria for both disruptive mood dysregulation disorder (DMDD) and ODD are met, diagnose only DMDD (Chapter 10).

Owners and Treatments

Owner	Approach
Gerald Patterson	Coercion theory; **parent management training** (Chapter 13)
Scott Henggeler	**Multisystemic therapy (MST)**: intensive home- and community-based treatment for serious antisocial behavior, targeting family, peers, school, and neighborhood
James Alexander & Bruce Parsons	**Functional family therapy (FFT)**
Alan Kazdin	**Problem-solving skills training (PSST)**
Sheila Eyberg	PCIT for young children (Chapter 5)

Functional Behavior Assessment

A **functional behavior assessment (FBA)** identifies what a behavior **does** for the person using the **ABC** model: - **A**ntecedent: what happens right before - **B**ehavior: what the person does - **C**onsequence: what happens right after, which may be reinforcing the behavior

Common functions: getting **attention**, **escaping** a demand, getting something **tangible**, or **sensory** stimulation. Interventions then teach a replacement behavior that serves the same function.

Part 3 — Bullying (3.D)

Dan Olweus defined bullying by three features: 1. **Intentional** harm 2. **Repeated** over time 3. A **power imbalance** between the person bullying and the target

Type	Examples
Physical	Hitting, pushing, damaging belongings
Verbal	Name-calling, threats, teasing
Relational (social)	Exclusion, rumors, damaging relationships
Cyberbullying	Harassment through texts, social media, or online platforms; can be constant and public

Roles: the person bullying, the target, **bully-victims** (who both bully and are bullied, and often have the most problems), and **bystanders**, who can either reinforce bullying or interrupt it.

Counseling Principles

- **Screen targets** for depression, anxiety, school avoidance, and **suicide risk**.
- **Peer mediation is not recommended for bullying.** Mediation assumes two equal parties in a conflict; bullying involves a power imbalance, and mediation can re-victimize the target.

- **Whole-school approaches** (such as the Olweus Bullying Prevention Program) work better than isolated interventions.
- Teach **bystanders** to intervene safely and to report.
- Work with the person who bullies as well: empathy, consequences, and underlying problems.
- Know school policies and state anti-bullying laws.

Part 4 — Caregiving and Financial Stress (3.E, 3.I)

Caregiving

Caring for a family member with illness, disability, or dementia can bring meaning and closeness, along with **caregiver burden**: physical exhaustion, isolation, financial strain, role changes, grief (including anticipatory and ambiguous loss; Chapter 12), and higher rates of depression and anxiety.

- Assess burden (e.g., the **Zarit Burden Interview**).
- Encourage **respite care**, support groups, and help from others.
- Address guilt about limits and about decisions such as placement in residential care.
- Watch for caregiver depression and for signs of elder abuse or neglect (Chapter 6).
- The **sandwich generation** cares for children and aging parents at once (Chapter 12).

Financial Issues

Financial stress is linked to anxiety, depression, relationship conflict, and suicide risk (especially with sudden losses or unmanageable debt).

- Address basic needs and stress first (Maslow).
- Explore **beliefs and emotions about money** (often shaped by family of origin).

- Couples often fight about money; it can reflect deeper issues of power, trust, and values.
- **Stay within scope:** counselors address the emotional and relational side and **refer** to financial professionals (credit counselors, financial planners, legal aid) for specific financial advice.
- Screen for **gambling** and compulsive buying (Chapter 11).

Part 5 — Loneliness and Attachment (3.M)

Term	Meaning
Social isolation	**Objective**: few social contacts
Loneliness	**Subjective**: the painful gap between desired and actual connection. A person can be lonely in a crowd or content alone
Emotional loneliness (Robert Weiss)	Missing a close, intimate attachment figure
Social loneliness (Robert Weiss)	Missing a broader social network

Research highlights: - **John Cacioppo** showed that loneliness changes social thinking: lonely people become more alert to social threat, which can make reaching out harder. - **Julianne Holt-Lunstad's** research links social isolation and loneliness to higher mortality risk. - The **U.S. Surgeon General's 2023 advisory** described an "epidemic of loneliness and isolation." - A meta-analysis by **Masi and colleagues (2011)** found that interventions targeting **maladaptive social cognition** (negative expectations and interpretations about others) were more effective than those that only increased social contact.

Counseling principles: assess attachment history (Chapter 12), social skills, and negative social expectations; use CBT to address those expectations; encourage graded social engagement; consider groups. Screen for

depression and suicide risk, especially in isolated older adults.

Part 6 — Hyper/Hypo Mental Focus (3.N)

Hypo-focus (difficulty sustaining attention) and **hyper-focus** (intense absorption that's hard to shift) both show up in counseling.

Possible source	Pattern
ADHD	Inattention, distractibility, and often **hyper-focus** on highly interesting tasks (Chapter 12)
Depression	Poor concentration, slowed thinking
Anxiety	Attention captured by worry or threat
Mania/hypomania	Distractibility, racing thoughts (Chapter 10)
Trauma	Hypervigilance; dissociation
Substances or sleep deprivation	Impaired attention
Neurocognitive disorder or delirium	Attention and memory problems (Chapter 12)
Obsessive preoccupation	Attention locked on obsessions (Chapter 10)

Dissociative symptoms include **depersonalization** (feeling detached from oneself, as if watching from outside) and **derealization** (the world feels unreal or dream-like). Dissociation often follows trauma and can disrupt focus and memory.

Exam principle: attention problems have many causes. Assess before assuming ADHD.

Part 7 — Sleep (3.P, 3.AF)

Sleep Basics

- Sleep cycles through **NREM** stages (N1, N2, N3; N3 is deep, slow-wave sleep) and **REM** sleep in cycles of roughly **90 minutes**.
- **REM** sleep is when most vivid dreaming occurs; the body's major muscles are normally paralyzed.
- The **circadian rhythm** is the roughly 24-hour internal clock regulated by light.

Presentations

Presentation	Key features
Insomnia disorder	Difficulty falling asleep, staying asleep, or waking too early, causing distress or impairment; at least **3 nights per week** for at least **3 months**, despite adequate opportunity to sleep
Hypersomnolence disorder	Excessive sleepiness despite at least 7 hours of sleep
Narcolepsy	Recurrent irresistible sleep episodes; often **cataplexy** (sudden loss of muscle tone, often triggered by emotion)
Obstructive sleep apnea (breathing-related sleep disorder)	Snoring, breathing pauses, daytime sleepiness; requires **medical evaluation** (sleep study)
Circadian rhythm sleep-wake disorders	Mismatch between internal clock and schedule (delayed sleep phase common in adolescents; shift work type)
NREM sleep arousal disorders	**Sleepwalking** and **sleep terrors**; occur in deep NREM sleep, usually early in the night; little or no memory of the episode

Presentation	Key features
Nightmare disorder	Repeated, well-remembered disturbing dreams; occurs in **REM** sleep, usually later in the night; person becomes alert on waking
REM sleep behavior disorder	Acting out dreams because muscle paralysis during REM is absent
Restless legs syndrome	Urge to move the legs, especially in the evening, relieved by movement

Sleep terrors vs. nightmares: sleep terrors happen in **NREM**, early in the night, with **little memory** and hard-to-wake confusion. Nightmares happen in **REM**, later in the night, are **remembered**, and the person wakes fully alert.

What Helps

CBT for insomnia (CBT-I) is the first-line treatment for chronic insomnia. Components:

Component	Owner / idea
Stimulus control	**Richard Bootzin**: use the bed only for sleep (and sex); go to bed only when sleepy; get up if not asleep in about 20 minutes; same wake time daily
Sleep restriction	**Arthur Spielman**: limit time in bed to actual sleep time to build sleep drive, then gradually extend
Sleep hygiene	Consistent schedule; limit caffeine, alcohol, nicotine, and screens near bedtime; comfortable environment (sleep hygiene alone is usually not enough for chronic insomnia)
Cognitive restructuring	Address worries about sleep (“If I don’t sleep 8 hours, tomorrow will be a disaster”)
Relaxation	Reduce physiological arousal

Imagery rehearsal therapy (rewriting a nightmare's script while awake and rehearsing it) helps nightmare disorder, including trauma-related nightmares.

Refer for **medical evaluation** when sleep apnea, narcolepsy, or another medical cause is suspected.

Part 8 — Maladaptive Eating (3.Q)

Presentations

Presentation	Key features
Anorexia nervosa	Restriction leading to **significantly low body weight**; intense fear of weight gain or behavior that interferes with weight gain; disturbed experience of body weight or shape. Types: **restricting** or **binge-eating/purging**. Severity for adults based on BMI (mild, moderate, severe, extreme)
Bulimia nervosa	Recurrent **binge eating** and **compensatory behaviors** (vomiting, laxatives, fasting, excessive exercise), both at least **once a week for 3 months**; self-worth overly tied to shape and weight; weight is often in the normal range
Binge-eating disorder	Recurrent binge eating with loss of control and marked distress, at least **once a week for 3 months**, **without** regular compensatory behaviors
ARFID (avoidant/restrictive food intake disorder)	Restricted eating due to sensory aversion, lack of interest, or fear of consequences (e.g., choking), **without** body-image concerns
Pica	Eating nonfood substances for at least **1 month**, inappropriate for developmental level
Rumination disorder	Repeated regurgitation of food for at least 1 month

Medical risk is serious. Anorexia has one of the highest mortality rates of any mental disorder. Purging can cause dangerous electrolyte imbalances and heart problems. **Refeeding syndrome** (dangerous shifts in electrolytes when nutrition is restored too quickly) requires medical management.

Screening and Treatment

Tool	Use
SCOFF	Five-question screen
EDE-Q (Fairburn & Beglin)	Eating Disorder Examination Questionnaire
EDE (Fairburn)	Clinician interview

Treatment	Notes
Family-based treatment (Maudsley; Lock & Le Grange)	First-line for **adolescent anorexia** (Chapter 5)
Enhanced CBT (CBT-E) (Christopher Fairburn)	Strong support for **bulimia** and **binge-eating disorder**, and used for anorexia in adults
DBT	When emotion dysregulation drives binge eating
Multidisciplinary team	Medical provider, dietitian, therapist, and psychiatric care as needed

Exam principles: coordinate with medical providers from the start; level of care depends on medical stability; avoid commenting on clients' weight or appearance; screen for suicide risk and co-occurring conditions.

Part 9 — Sexual Functioning (3.AE)

Models of Sexual Response

Owner	Model
Masters & Johnson	Four phases: **excitement → plateau → orgasm → resolution**; developed **sensate focus**

Owner	Model
Helen Singer Kaplan	Triphasic model: **desire → excitement → orgasm** (added desire as a phase)
Rosemary Basson	**Circular model** of women's sexual response: desire can be **responsive** (arising after arousal begins) rather than spontaneous; emotional intimacy is central
Jack Annon	**PLISSIT** model of sex counseling

PLISSIT (Annon)

Level	Counselor action
Permission	Give permission to talk about sex and normalize concerns
Limited **I**nformation	Provide basic, accurate information
Specific **S**uggestions	Offer specific strategies (e.g., sensate focus)
Intensive **T**herapy	Refer for or provide specialized sex therapy

Most clients' needs are met at the first two or three levels. **Intensive therapy requires specialized training.**

Sexual Dysfunctions (DSM-5-TR)

Include erectile disorder, delayed ejaculation, premature (early) ejaculation, female orgasmic disorder, female sexual interest/arousal disorder, male hypoactive sexual desire disorder, and genito-pelvic pain/penetration disorder. Most require symptoms for about **6 months** and clinically significant distress.

Exam principles: - **Rule out medical causes and medication effects first** (e.g., vascular disease, diabetes, hormonal changes; **SSRIs** commonly affect desire and orgasm). - Address relationship factors, anxiety (including performance anxiety), trauma history, and cultural and religious beliefs. - **Sensate focus** (Masters & Johnson) reduces performance pressure through structured, non-de-

mand touch. - Stay within competence; refer for specialized sex therapy when needed.

Part 10 — Stress Management (3.AH)

Owners

Owner	Contribution
Hans Selye	General Adaptation Syndrome: alarm → resistance → exhaustion; eustress vs. distress (Chapter 3)
Holmes & Rahe	Social Readjustment Rating Scale: life changes, positive or negative, add stress
Richard Lazarus & Susan Folkman	**Transactional model**: stress depends on **appraisal**. **Primary appraisal**: Is this a threat, harm, or challenge? **Secondary appraisal**: Do I have the resources to cope? Coping is **problem-focused** (change the situation) or **emotion-focused** (manage the feelings)
Suzanne Kobasa	**Hardiness**: the “three Cs” of **commitment**, **control**, and **challenge** buffer stress
Friedman & Rosenman	**Type A** behavior (competitive, impatient, hostile) vs. Type B; later research pointed to **hostility** as the harmful component for heart disease
Herbert Benson	Relaxation response
Edmund Jacobson	Progressive muscle relaxation
Jon Kabat-Zinn	Mindfulness-Based Stress Reduction (MBSR)
Donald Meichenbaum	Stress inoculation training (Chapter 2)

Matching coping to the stressor: problem-focused coping works best when the situation **can** be changed; emotion-focused coping works best when it **can’t** (e.g., a loved one’s terminal diagnosis).

Part 11 — Hallucinations and Psychotic Experiences (3.AL)

Presentations

Presentation	Key features	Time frame
Schizophrenia	At least **2 of 5** symptoms (delusions, hallucinations, disorganized speech, grossly disorganized or catatonic behavior, negative symptoms), at least one of the first three; decline in functioning	Active symptoms for at least **1 month**; continuous signs for at least **6 months**
Schizophreniform disorder	Same symptoms as schizophrenia	At least **1 month** but less than **6 months**
Brief psychotic disorder	Sudden psychotic symptoms with full return to functioning	At least **1 day** but less than **1 month**
Schizoaffective disorder	A major mood episode concurrent with schizophrenia symptoms, **plus** delusions or hallucinations for at least **2 weeks** without a mood episode	—
Delusional disorder	One or more delusions without other prominent psychotic symptoms; functioning often relatively intact	At least **1 month**
Mood disorder with psychotic features	Psychosis occurs **only** during mood episodes	—

Positive symptoms add something (hallucinations, delusions, disorganization). **Negative symptoms** take something away: **avolition** (low motivation), **alogia** (reduced

speech), **anhedonia**, **asociality**, and **flat or blunted affect**.

Hallucinations Without a Psychotic Disorder

Not every hallucination means psychosis: - **Hypnagogic** (falling asleep) and **hypnopompic** (waking) hallucinations are common - **Bereavement**: sensing, seeing, or hearing a deceased loved one is common and often comforting - **Substances**, withdrawal, and **delirium** (Chapter 11, Chapter 12) - **Trauma**: flashbacks and dissociative experiences - **Cultural and religious experiences** shared and accepted within the client's community - **Medical and neurological conditions**

Owners

Owner	Contribution
Emil Kraepelin	Described "dementia praecox"
Eugen Bleuler	Coined "schizophrenia"; the **four As**: loosened **associations**, blunted **affect**, **ambivalence**, **autism** (withdrawal into one's inner world)
George Brown; **Vaughn & Leff**	**Expressed emotion (EE)**: families high in criticism, hostility, and emotional over-involvement are linked to higher **relapse** risk; family psychoeducation lowers it

Counseling Principles

- **Don't argue with or agree with** delusions or hallucinations. Acknowledge the client's experience and feelings ("That sounds frightening") without confirming the content.
- **Assess risk**, especially **command hallucinations** telling the client to harm self or others (risk assessment, Chapter 6).
- **Refer for psychiatric evaluation**; antipsychotic medication is a mainstay of treatment for psychotic disorders.

- **Coordinated specialty care** for **first-episode psychosis** (team-based care including medication, therapy, family education, and supported employment and education) improves outcomes.
- **CBT for psychosis (CBTp)**, **family psychoeducation**, **social skills training**, and **supported employment** have research support.
- **Consider culture** before labeling an experience psychotic.

What the Exam Rewards / What It Punishes

Rewards: - Medical evaluation first for physical symptoms, sleep apnea, eating disorders, sexual dysfunction, and sudden changes - Validating physical suffering without saying “it’s all in your head” - Functional behavior assessment (ABC) - Rejecting peer mediation for bullying - Referring for specialized financial or sex therapy help when beyond scope - CBT-I for chronic insomnia - Family-based treatment for adolescent anorexia; CBT-E for bulimia and binge eating - Matching coping style to whether the stressor can be changed - Neither arguing with nor confirming hallucinations; assessing command hallucinations

Punishes: - Treating eating disorders without medical coordination - Assuming every attention problem is ADHD - Peer mediation between a bully and a target - Treating a bereaved client’s sense of the deceased’s presence as psychosis - Giving specific financial or medical advice outside scope

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Swap-Pair Drill

If you see…	It’s…	Not…
Biopsychosocial model	Engel	Rosenstock

If you see…	It's…	Not…
Susceptibility, severity, benefits, barriers, cues	Health belief model	Biopsychosocial model
Excessive thoughts about real physical symptoms	Somatic symptom disorder	Illness anxiety disorder
Fear of illness with few or no symptoms	Illness anxiety disorder	Somatic symptom disorder
At least 4 symptoms, 6 months, angry and defiant	ODD	Conduct disorder
3 of 15 criteria, 12 months, violating rights	Conduct disorder	ODD
Home- and community-based treatment for antisocial youth	MST (Henggeler)	FFT
Antecedent, behavior, consequence	Functional behavior assessment	Conflict resolution
Intentional, repeated, power imbalance	Olweus' definition of bullying	Conflict
Missing an intimate attachment	Emotional loneliness (Weiss)	Social loneliness
Insomnia: 3 nights a week for 3 months	Insomnia disorder	Hypersomnolence
NREM, early night, little memory	Sleep terrors	Nightmares
REM, later night, remembered	Nightmares	Sleep terrors
Bed only for sleep; get up if awake	Stimulus control (Bootzin)	Sleep restriction (Spielman)
Binge + compensatory behavior	Bulimia nervosa	Binge-eating disorder
Restriction without body-image concern	ARFID	Anorexia
Excitement, plateau, orgasm, resolution	Masters & Johnson	Kaplan

If you see…	It's…	Not…
Desire, excitement, orgasm	Kaplan	Masters & Johnson
Responsive desire	Basson	Kaplan
Permission, limited information, specific suggestions, intensive therapy	PLISSIT (Annon)	Sensate focus
Primary and secondary appraisal	Lazarus & Folkman	Selye
Commitment, control, challenge	Kobasa (hardiness)	Lazarus
Psychotic symptoms 1 day to under 1 month, full recovery	Brief psychotic disorder	Schizophreniform
Schizophrenia symptoms 1 to under 6 months	Schizophreniform	Schizophrenia
Associations, affect, ambivalence, autism	Bleuler	Kraepelin
Family criticism and over-involvement predict relapse	Expressed emotion (Brown; Vaughn & Leff)	Double bind

Quick Checks

1. A client with chronic back pain spends hours a day researching her symptoms, is convinced every twinge signals a serious disease, and has limited her life significantly for 8 months. This best fits: A. Illness anxiety disorder B. Somatic symptom disorder C. Functional neurological symptom disorder D. Malingering

2. A 12-year-old has, for the past year, bullied other students, stolen from stores, and repeatedly stayed out all night. This pattern best fits: A. ODD B. Conduct disorder C. ADHD D. Intermittent explosive disorder

3. A school counselor learns that a student is being repeatedly targeted by a larger, more popular peer. The counselor should NOT: A. Assess the target for depression

and suicide risk B. Bring the two students together for peer mediation C. Involve school administrators per policy D. Work with bystanders

4. A client reports trouble falling asleep 4–5 nights a week for the past 5 months and has started napping and spending long hours in bed. The first-line treatment is: A. Sleep hygiene education alone B. CBT for insomnia C. Long-term sleeping medication D. Imagery rehearsal therapy

5. A parent describes her 5-year-old sitting up screaming about an hour after falling asleep, unresponsive to comfort, and remembering nothing the next morning. This best fits: A. Nightmare disorder B. Sleep terrors C. REM sleep behavior disorder D. Narcolepsy

6. A client binges several times a week, then fasts and exercises excessively to compensate; her weight is in the normal range. This best fits: A. Anorexia nervosa, binge/purge type B. Bulimia nervosa C. Binge-eating disorder D. ARFID

7. A client reports low sexual desire since starting an SSRI two months ago. The counselor's first step is to: A. Begin sensate focus exercises B. Encourage the client to talk with the prescriber about possible medication side effects C. Diagnose female sexual interest/arousal disorder D. Explore childhood sexual history

8. In the PLISSIT model, explaining that it's common for desire to fluctuate in long-term relationships is an example of: A. Permission B. Limited information C. Specific suggestions D. Intensive therapy

9. A client's spouse has a terminal diagnosis. According to Lazarus and Folkman, which type of coping is likely to help most with the diagnosis itself? A. Problem-focused coping B. Emotion-focused coping C. Avoidance D. Hardiness

10. Three weeks after her husband's death, a widow reports she sometimes hears his voice saying her name and

finds it comforting. She has no other unusual symptoms. The counselor should: A. Refer her urgently for a psychotic disorder evaluation B. Normalize this as a common bereavement experience while continuing to monitor C. Diagnose brief psychotic disorder D. Tell her the voice isn't real

11. A client with schizophrenia says the TV is sending him messages. The best counselor response is: A. "That's impossible; TVs can't send messages." B. "Yes, the messages are real." C. "That sounds really unsettling. What are the messages telling you?" D. Ignore the comment and change the subject

12. Family members of a client with schizophrenia are highly critical and emotionally over-involved. Research on expressed emotion suggests this is linked to: A. Lower relapse risk B. Higher relapse risk C. No change in outcomes D. Faster recovery

Answers and Rationales

1 — B. Distressing physical symptoms plus excessive thoughts and behaviors for over 6 months. A: illness anxiety involves few or no somatic symptoms. C: functional neurological symptoms are neurological (weakness, seizures). D: malingering is intentional faking for external gain.

2 — B. Violating others' rights and major rules across 12 months meets conduct disorder patterns. A: ODD is anger, defiance, and vindictiveness without serious rule violations. C: ADHD involves inattention and hyperactivity. D: IED involves explosive outbursts.

3 — B. Peer mediation isn't appropriate because of the power imbalance. A, C, D: all appropriate.

4 — B. CBT-I is first-line for chronic insomnia. A: sleep hygiene alone is usually not enough. C: medication is a prescriber decision and not first-line long-term. D: imagery rehearsal targets nightmares.

5 — B. Early-night, unresponsive, no memory: NREM sleep terrors. A: nightmares are remembered and occur in REM later in the night. C: REM sleep behavior disorder involves acting out dreams during REM, usually in older adults. D: narcolepsy involves irresistible daytime sleep.

6 — B. Bingeing plus compensatory behavior at normal weight. A: anorexia requires significantly low weight. C: binge-eating disorder lacks regular compensation. D: ARFID lacks body-image concerns and bingeing.

7 — B. Rule out medication effects first; SSRIs commonly affect desire. A: treats before ruling out a medical cause. C: premature diagnosis. D: not the first step given the clear timing.

8 — B. Basic, accurate information is the LI level. A: permission normalizes talking about sex. C: specific suggestions are concrete strategies. D: intensive therapy is specialized treatment.

9 — B. When the stressor can't be changed, emotion-focused coping fits. A: problem-focused coping suits changeable situations. C: avoidance tends to worsen outcomes. D: hardiness is Kobasa's trait, not a coping type in this model.

10 — B. Sensing a deceased loved one is common in bereavement. A and C: pathologize a normal experience. D: dismissive.

11 — C. Acknowledge the feeling without confirming or arguing, and assess the content (which can reveal risk). A: arguing damages trust. B: colludes with the delusion. D: misses important information.

12 — B. High expressed emotion is linked to higher relapse risk. A, C, D: contradict the research.

Audio Companion

Track	Covers	Target length
16.1	Body and Mind: illness, behavior, bullying, sleep, eating, sex, stress, psychosis	~7 min

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Chapter 17 — Ethics and Law

Domain map: 1.A Assess your competency to work with a specific client · 1.C Practice legal and ethical counseling · 1.D Clarify counselor–client roles · 1.E Discuss client's rights and responsibilities · 1.F Discuss limits of confidentiality · 1.G Explain counselor agency policies · 1.H Review payment, fees, and insurance benefits · 1.I Explain counseling processes, procedures, risks, and benefits · 1.J Explain uses and limits of social media · 1.K Inform clients about the legal aspects of counseling · 1.L Obtain informed consent · 1.M Discuss confidentiality as it applies to electronic communication · 1.O Assess competency to provide informed consent · 1.P Monitor the therapeutic relationship · 1.Q Review client records · 1.R Provide adequate accommodations for clients with disabilities · 1.S Provide information to third parties · 1.T Provide referral sources · 1.U Advocate for professional and client issues · 1.V Seek supervision/consultation · 1.W Create and maintain documentation · 1.X Awareness and practice of self-care · CACREP: Professional Counseling Orientation and Ethical Practice

How the NCE Tests Ethics

Professional Practice and Ethics accounts for 19 of the 160 scored items. Ethics also shows up inside other domains: informed consent in group work, confidentiality in couples work, the duty to protect in crisis items. Every ethics item has the same structure: a situation, four plausible actions, and one that best follows the codes and the law.

Three habits answer most of them:

1. **Know the standard.** The ACA Code of Ethics (2014) is the main source. The NBCC Code of Ethics (2023)

applies to National Certified Counselors and largely agrees with ACA.

2. **Consult.** When an option includes consultation or supervision, it is very often part of the right answer.
3. **Least harm, most client welfare.** Between two options, choose the one that protects the client and preserves autonomy, and document your reasoning.

Part 1 — Foundations

Ethical Principles

The ACA Code's preamble names six fundamental principles. Karen Kitchener (1984) described the first five as the foundation of ethical decision-making.

Principle	Meaning
Autonomy	Fostering the client's right to control the direction of their own life
Nonmaleficence	Avoiding actions that cause harm
Beneficence	Working for the good of the individual and society by promoting mental health and well-being
Justice	Treating individuals equitably and fostering fairness and equality
Fidelity	Honoring commitments and keeping promises, including fulfilling one's responsibilities of trust
Veracity	Dealing truthfully with individuals

Exam patterns: keeping a promised appointment time is **fidelity**. Being honest about a diagnosis is **veracity**. Letting a client make a choice you disagree with is **autonomy**. Sliding-scale fees and equal access are **justice**.

Ethics vs. Law

Term	Meaning
Ethics	Standards set by the profession (ACA, NBCC). Violations can lead to loss of membership or certification
Law	Rules set by government (statutes, regulations, court decisions). Violations can lead to loss of license, lawsuits, or criminal penalties
Mandatory ethics	Following the minimum rules
Aspirational ethics	Striving for the highest standards of practice

When ethics and law conflict (ACA I.1.c): counselors make known their commitment to the ACA Code and take steps to resolve the conflict. If it can't be resolved that way, counselors may follow the law, regulations, or other governing legal authority.

Confidentiality, Privilege, and Privacy

Term	What it is	Who owns it
Privacy	A constitutional and legal right to decide what personal information is shared	The person
Confidentiality	An **ethical** (and often legal) duty of the counselor not to disclose client information	The counselor's duty to the client
Privileged communication	A **legal** right that protects client communications from being disclosed **in court**	**The client** holds the privilege and can waive it

***Jaffee v. Redmond* (1996):** the U.S. Supreme Court recognized psychotherapist–patient privilege in federal courts, including for licensed clinical social workers. States set their own privilege laws for counselors.

Credentials

Term	Meaning
Licensure	State permission to practice (e.g., LPC, LMHC); the strongest form of credential; protects title and practice
Certification	A credential from a professional body (e.g., NCC from NBCC); voluntary at the national level
Accreditation	Approval of **programs** (e.g., CACREP accredits counseling programs), not individuals
Registry	A listing of practitioners who meet basic criteria

Part 2 — Informed Consent (1.E, 1.G, 1.H, 1.I, 1.K, 1.L, 1.O)

Informed consent is an **ongoing process**, not a one-time form (ACA A.2.a). It's provided in writing and verbally, in language the client can understand (A.2.c), and revisited as counseling changes.

What Clients Need to Know (ACA A.2.b)

- Purposes, goals, techniques, procedures, limitations, potential risks, and benefits of services
- The counselor's qualifications, credentials, and relevant experience
- Continuation of services if the counselor becomes incapacitated or dies
- The role of technology (Chapter 9)
- Implications of diagnosis and the intended use of tests and reports
- Fees and billing arrangements, including procedures for nonpayment
- **Confidentiality and its limits**
- Clients' rights to obtain information about their records and to participate in ongoing counseling plans

- The right to **refuse** any services or modality change and to be advised of the consequences of refusal

Special Situations

Situation	Rule
Minors	Parents or guardians generally consent; the minor gives **assent**. State laws vary on when minors can consent on their own (e.g., for substance use or some mental health care). Counselors respect parents' legal rights while protecting the minor's confidentiality as much as possible (ACA B.5.b)
Clients unable to give consent (A.2.d)	Seek the client's assent; include those legally able to consent on the client's behalf, such as guardians
Mandated clients (A.2.e)	Explain what information will be shared with the referral source (court, probation, employer) **before** counseling begins; clients can refuse services, with the consequences explained
Assessing capacity (1.O)	Consider age, cognitive ability, intoxication, psychosis, and acute distress
Disabilities (1.R)	Provide reasonable accommodations (ADA), such as interpreters, accessible materials, and accessible space
Language	Use qualified interpreters (Chapter 9)

Part 3 — Confidentiality and Its Limits (1.F, 1.M, 1.S)

Exceptions (ACA B.2)

Exception	Standard
Serious and foreseeable harm (B.2.a)	Disclosure is permitted when needed to protect clients from serious and foreseeable harm, or when legal requirements demand it (duty to protect, Chapter 6)
Abuse reporting	Mandated by state law (Chapter 6)
End-of-life decisions (B.2.b)	Option to maintain confidentiality for terminally ill clients considering hastening death, after consultation (Chapter 12)
Contagious, life-threatening diseases (B.2.c)	Counselors **may** be justified in disclosing to an identifiable third party at serious and foreseeable risk of contracting the disease. **Before** disclosing, they assess whether the client intends to inform the third party or to engage in behavior that may harm that person, and they follow state law, which varies. (The 2005 Code also required confirming the diagnosis; the 2014 Code dropped that step)
Court-ordered disclosure (B.2.d)	Seek the client's written consent or take steps to limit disclosure; disclose as little as possible
Minimal disclosure (B.2.e)	When disclosure is required, share only essential information
Client consent	The client can authorize disclosure in writing
Client's lawsuit	A client who sues the counselor generally waives confidentiality for the issues in the suit

Subpoenas and Court Orders

Document	What to do
Subpoena (often issued by an attorney)	**Don't ignore it, and don't simply hand over records.** Contact the client and/or the client's attorney, consult your own attorney or licensing board guidance, and **assert privilege** on the client's behalf if the client hasn't waived it
Court order (signed by a judge)	Comply, but disclose only what is required, and ask the court to limit disclosure if needed

Sharing Information with Others (ACA B.3)

- **Subordinates** (assistants, students, billing staff): counselors make sure they protect confidentiality.
- **Interdisciplinary teams**: inform the client of the team's existence and composition and what information is shared.
- **Third-party payers**: disclose only with client authorization.
- **Deceased clients**: protect confidentiality consistent with law and agency policy.
- **Consultation** (B.7): don't reveal identifying information unless the client consents or disclosure is unavoidable; share only what's needed.

Groups and Families (ACA B.4)

Counselors explain that confidentiality **can't be guaranteed** in groups (Chapter 8). In couple and family work, counselors clarify who the client is and the secrets policy (Chapter 7).

Federal Privacy Laws

Law	What it covers
HIPAA (Health Insurance Portability and Accountability Act, 1996)	Privacy and security of protected health information (PHI) for covered entities. Clients have a right to access and request amendments to their records (with limited exceptions). **Psychotherapy notes** (a counselor's separate process notes) receive extra protection and generally require specific authorization to disclose. **Minimum necessary** rule for most uses and disclosures (it doesn't apply to disclosures for treatment or those the client authorizes)
HITECH Act (2009)	Strengthened HIPAA enforcement; breach notification requirements for electronic records
42 CFR Part 2	Extra protection for federally assisted substance use disorder program records (Chapter 9)
FERPA (Family Educational Rights and Privacy Act)	Privacy of student education records; parents' access rights (Chapter 12)

Psychotherapy notes vs. the clinical record: the clinical record (dates, diagnosis, treatment plan, progress summaries, medications, test results) is part of the designated record set. Psychotherapy notes are kept **separate** and include the counselor's analysis of session content.

Part 4 — Boundaries and Relationships (1.D, 1.P)

Sexual and Romantic Relationships (ACA A.5)

Relationship	Rule
Current clients, their romantic partners, or their family members (A.5.a)	**Prohibited**, in person and electronically
Previous romantic or sexual partners (A.5.b)	Counselors don't take them on as clients
Former clients, their partners, or family members (A.5.c)	**Prohibited for 5 years** after the last professional contact. After 5 years, counselors must still show forethought and document that the relationship isn't exploitative; if exploitation is possible, they avoid it
Friends or family members (A. 5.d)	Counselors don't counsel them when they can't be objective
Personal virtual relationships with current clients (A.5.e)	**Prohibited** (e.g., social media friendships)
Supervisees and students (F. 3.b, F.10.a)	Sexual or romantic relationships are prohibited

Boundary Extensions (ACA A.6)

Not every nonprofessional contact is harmful. Attending a client's graduation, a hospital visit during illness, or purchasing a service from a client's business might be **boundary extensions**. The counselor:

- Considers the risks and benefits
- Documents the rationale **before** the interaction when feasible (A.6.b)
- Gets client consent where appropriate
- Takes steps to repair harm if it occurs

Boundary crossing vs. boundary violation: a **crossing** departs from the usual role and may be harmless or even helpful (a brief condolence card). A **violation** harms or exploits the client (a business partnership, a sexual relationship). The NCE rewards careful, documented judg-

ment, not rigid avoidance of all contact. In small or rural communities, some overlap is unavoidable; it's managed, not prohibited.

Gifts and Bartering

Topic	Rule
Bartering (A.10.e)	Acceptable **only** if the relationship isn't exploitative or harmful, doesn't give the counselor an unfair advantage, **the client requests it**, and it's an accepted practice in the community. The counselor considers cultural implications and documents the agreement in a written contract
Gifts (A.10.f)	Counselors consider the therapeutic relationship, the **monetary value**, the client's **motivation**, and the counselor's motivation. Small gifts may be accepted, especially where culturally meaningful

Fees and Business Practices (ACA A.10)

- **Self-referral** (A.10.a): counselors in agencies don't refer clients to their own private practices unless the agency allows it and clients are informed of other options.
- **No kickbacks** (A.10.b): no fees for referrals.
- **Establishing fees** (A.10.c): consider clients' financial status and locality; help find comparable, affordable services if fees are a hardship.
- **Nonpayment** (A.10.d): inform clients in advance if collection agencies or legal measures will be used, and give clients a chance to pay first.
- **Contributing to the public good** (C.6.e): counselors make a reasonable effort to provide services for little or no financial return (pro bono).

Part 5 — Competence, Supervision, and Self-Care (1.A, 1.V, 1.X)

Competence (ACA C.2)

- Practice only within the boundaries of competence, based on education, training, supervised experience, credentials, and experience (C.2.a).
- New specialty areas require education, training, and supervised experience; take steps to ensure competence and protect clients while developing skills (C.2.b).
- Pursue continuing education (C.2.f).
- **Assess your competency for a specific client (1.A)**: when a client's needs exceed your competence and supervision or training can't close the gap in time, **refer** (A.11.a). Don't refer for **values** reasons (A.11.b; Chapter 9).

Impairment (ACA C.2.g)

Counselors monitor themselves for signs of impairment from their own physical, mental, or emotional problems and **refrain from offering services** when impaired. They seek help, and if necessary limit, suspend, or terminate their professional responsibilities until it's safe to resume. Colleagues who notice impairment in another counselor consult with and assist that colleague (C.2.g, I.2).

Self-Care (1.X)

Burnout, compassion fatigue, and vicarious trauma (Chapter 14) threaten competence. Self-care is an ethical responsibility: supervision, consultation, personal counseling, boundaries on workload, and attention to physical health and relationships.

Supervision and Consultation (1.V)

Term	Meaning
Supervision	An ongoing, **evaluative**, hierarchical relationship; the supervisor shares responsibility for client care
Consultation	A **voluntary**, nonhierarchical request for advice; the consulting counselor keeps full responsibility
Vicarious liability (*respondeat superior*)	Supervisors can be held legally responsible for supervisees' actions
Direct liability	Supervisors are responsible for their own negligent supervision

Supervision model owners: Bernard's **discrimination model** (supervisor roles: teacher, counselor, consultant; foci: intervention, conceptualization, personalization); Stoltenberg and Delworth's **Integrated Developmental Model** (IDM; supervisees develop through levels of self- and other-awareness, motivation, and autonomy).

Part 6 — Records and Documentation (1.Q, 1.W)

- Counselors create, safeguard, and maintain **records** needed to provide services (A.1.b), consistent with law and agency policy.
- Records are **accurate, timely, and sufficiently detailed**; they document services, risk assessments, consultations, and the rationale for clinical decisions. "If it isn't documented, it didn't happen."
- **Client access** (B.6.e): counselors provide reasonable access to records unless there is compelling evidence that access would cause harm; they help clients understand the records and document any limits on access.

- **Retention and disposal** (B.6.h): keep records for the period required by law (state laws vary, often around 7 years for adults, longer for minors), then dispose of them in a way that protects confidentiality.
- **Plan for incapacity or death** (C.2.h, B.6.i): counselors have a plan for transferring records, often through a designated records custodian or professional will.
- **Electronic records**: secure, encrypted, backed up (Chapter 9).
- **Correcting errors**: add a dated correction; never alter or destroy an original entry.

Part 7 — Termination, Referral, and Abandonment (1.T)

Concept	Rule
Appropriate termination (A.11.c)	When services are no longer needed, the client isn't benefiting, the client is being harmed, or the counselor is in danger from the client or someone in the client's life. Counselors provide **pretermination counseling** and recommend other providers when needed
Termination for nonpayment	Permitted when the fee arrangement was clearly explained, but with pretermination counseling and referrals; never in the middle of a crisis
Abandonment (A.12)	Counselors **don't abandon or neglect clients**. They arrange for continuation of care during absences, illness, and after termination

Concept	Rule
Referral (A.11.a)	When lacking competence, refer to appropriate resources. If the client declines the referral, the counselor discontinues the relationship
Appropriate transfer (A.11.d)	Ensure appropriate clinical and administrative processes and open communication with the client and new provider

Part 8 — Advocacy and Professional Identity (1.U)

- Counselors **advocate** at individual, group, institutional, and societal levels to address barriers clients face (A. 7.a), getting client consent before advocating on a specific client's behalf (A.7.b).
- **Advertising** (C.3): accurate, not misleading; counselors who use testimonials don't **solicit** them from current clients, former clients, or anyone else vulnerable to undue influence (C.3.b).
- **Credentials** (C.4): claim only credentials actually earned; use "Dr." only with a doctorate in counseling or a closely related field (C.4.d).
- **Nondiscrimination** (C.5) and **sexual harassment** prohibition (C.6.a).

Part 9 — Ethical Decision-Making

Forester-Miller and Davis (1996) — Seven Steps

1. Identify the problem
2. Apply the ACA Code of Ethics
3. Determine the nature and dimensions of the dilemma (principles involved, relevant literature, consultation)
4. Generate potential courses of action
5. Consider the potential consequences of all options and choose a course of action

6. Evaluate the selected course of action
7. Implement the course of action

Three Tests for the Chosen Action (Stadler)

Test	Question
Justice	Would I treat others in this situation the same way?
Publicity	Would I want my behavior reported in the press?
Universality	Would I recommend this course of action to another counselor in the same situation?

Resolving Ethical Violations by Colleagues (ACA I. 2)

1. If an informal resolution is appropriate, **first try to resolve it informally with the colleague** (I.2.a), as long as that doesn't violate confidentiality.
2. If informal resolution isn't appropriate or doesn't work, or the violation has caused or is likely to cause **substantial harm**, take further action: report to the state licensing board, ACA, or NBCC as appropriate (I. 2.b).
3. **Don't file unwarranted complaints** (I.2.e) intended to harm a counselor rather than protect clients.

Part 10 — Malpractice and Risk Management

A **malpractice** claim generally requires four elements, often remembered as the **four Ds**:

1. **Duty**: a professional relationship existed
2. **Dereliction** (breach): the counselor failed to meet the standard of care
3. **Direct causation**: the breach caused the harm
4. **Damages**: the client suffered harm

Common risk areas: sexual misconduct, failure to assess or respond to suicide risk, breaches of confidentiality, practicing outside competence, abandonment, and poor documentation.

Risk management: informed consent, documentation, consultation, practicing within competence, professional liability insurance, and following the code and law.

Part 11 — Artificial Intelligence and Emerging Technology

The 2014 ACA Code predates widespread AI, but its standards on technology (Section H), confidentiality (Section B), competence (C.2), and informed consent (A.2) apply directly. The ACA AI Work Group's recommendations (see Chapter 9) translate them into practice:

- **Informed consent:** tell clients when AI is used (e.g., an AI note-taker or scheduling assistant), what it does, and the risks; respect a client's refusal.
- **Responsibility:** the counselor is responsible for every clinical decision and every note, including AI-drafted ones. AI shouldn't be the sole basis for diagnosis, assessment, or risk decisions.
- **Confidentiality:** use only tools that meet HIPAA requirements (with a business associate agreement where required); never enter identifiable client information into consumer AI tools.
- **Bias and competence:** understand the tool's limits; watch for biased output.
- **Clients' AI use:** ask about it, and explore benefits and risks, including the lack of crisis response from chatbots.

On the exam, treat AI like any new technology: informed consent, confidentiality, competence, and the counselor's responsibility for clinical judgment.

What the Exam Rewards / What It Punishes

Rewards: - Informed consent as an ongoing process, including limits of confidentiality - Consulting and documenting before difficult decisions - Responding to a subpoena by contacting the client and asserting privilege - Minimal necessary disclosure - Referral based on competence, not values - Pretermination counseling and referrals; never abandonment - Trying informal resolution with a colleague first when appropriate - Self-monitoring for impairment

Punishes: - Releasing records in response to a subpoena without client consent or a court order - Any sexual or romantic relationship with a current client, or with a former client within 5 years - Bartering initiated by the counselor - Ignoring a colleague's harmful violation - Practicing while impaired - Altering records - Terminating abruptly during a crisis

Swap-Pair Drill

If you see...	It's...	Not...
Keeping promises	Fidelity	Veracity
Telling the truth	Veracity	Fidelity
Client's right to choose	Autonomy	Beneficence
Doing good	Beneficence	Nonmaleficence
Avoiding harm	Nonmaleficence	Beneficence
Fairness and equal access	Justice	Autonomy
Legal right in court; client holds it	Privileged communication	Confidentiality
Ethical duty not to disclose	Confidentiality	Privilege
Approves programs	Accreditation (CACREP)	Certification
State permission to practice	Licensure	Certification

If you see…	It's…	Not…
Attorney-issued document requesting records	Subpoena	Court order
Judge-signed order	Court order	Subpoena
Former client romantic relationship	Prohibited for 5 years (A.5.c)	2 years
Evaluative, ongoing, shared responsibility	Supervision	Consultation
Supervisor liable for supervisee	Vicarious liability	Direct liability
Duty, dereliction, direct causation, damages	Malpractice	Ethical complaint
Would I want this in the news?	Publicity test	Universality test
Seven-step ethical decision-making model	Forester-Miller & Davis	Kitchener
Extra protection for process notes	HIPAA psychotherapy notes	42 CFR Part 2

Quick Checks

1. A counselor promises to call a client back by Friday and does. Which principle does this reflect? A. Veracity B. Fidelity C. Justice D. Beneficence

2. A counselor receives a subpoena from an attorney requesting a client's records. The counselor should first: A. Send the records immediately B. Ignore the subpoena C. Contact the client (and/or the client's attorney) and assert privilege on the client's behalf unless the client consents D. Destroy the records

3. Who holds privilege in a counseling relationship? A. The counselor B. The client C. The court D. The agency

4. A counselor wants to date a former client whom she last saw 3 years ago. Under the ACA Code, this is: A. Permitted after 2 years B. Prohibited until at least 5 years

after the last professional contact C. Permitted with the former client's consent D. Permitted if they meet outside the office

5. A client who can't afford fees offers to paint the counselor's office in exchange for sessions. Bartering may be acceptable if: A. The counselor proposed it B. The client requests it, it isn't exploitative, it's accepted in the community, and it's documented in a contract C. It's kept confidential D. The work is worth more than the sessions

6. A counselor notices she's been distracted, tearful, and missing details in sessions since her divorce. The best action is to: A. Keep working; clients will understand B. Seek help and limit or suspend practice until she can practice safely C. Tell clients about her divorce D. Refer all clients immediately and close her practice permanently

7. A counselor learns that a colleague has been seeing clients while intoxicated. No client has yet been harmed, but the risk is substantial. The best action is to: A. Ignore it to protect the colleague B. Report to the licensing board or appropriate body, since informal resolution isn't sufficient for a risk of substantial harm C. Tell the colleague's clients D. Post about it on social media

8. Which pair correctly matches the concept? A. Supervision: voluntary, nonevaluative B. Consultation: evaluative, shared responsibility C. Supervision: evaluative, ongoing, shared responsibility D. Consultation: supervisor is vicariously liable

9. A client with a terminal illness is considering hastening his death. The ACA Code allows the counselor to: A. Always break confidentiality B. Choose to maintain confidentiality, depending on law and circumstances, after consultation C. Encourage the decision D. End counseling

10. A client stops paying and owes several sessions. Before sending the account to collections, the counselor must: A. Do nothing; collections can proceed B. Have informed the client in advance and give the client an oppor-

tunity to pay C. Disclose the client's diagnosis to the agency D. Terminate without notice

11. A counselor wants to use an AI tool to draft session notes. The most appropriate first step is to: A. Paste session details into a free online chatbot B. Verify the tool meets HIPAA requirements, explain its use to clients, and obtain consent C. Use it without telling clients, since notes are the counselor's property D. Let the AI finalize notes without review

12. Which question applies the universality test? A. Would I want this reported in the news? B. Would I treat others the same way? C. Would I recommend this action to another counselor in the same situation? D. Is this legal in my state?

Answers and Rationales

1 — B. Keeping commitments is fidelity. A: veracity is truthfulness. C: justice is fairness. D: beneficence is doing good.

2 — C. A subpoena isn't a court order; contact the client and assert privilege (and consult legal guidance). A: releases records without authorization. B: ignoring a subpoena can bring legal consequences. D: destroying records is illegal and unethical.

3 — B. The client holds privilege and can waive it. A, C, D: incorrect holders.

4 — B. ACA A.5.c prohibits such relationships for 5 years, and even then requires forethought and documentation that it isn't exploitative. A: wrong time frame. C and D: consent and setting don't change the rule.

5 — B. ACA A.10.e conditions. A: counselor-initiated bartering is inappropriate. C: confidentiality isn't the issue. D: unequal exchange risks exploitation.

6 — B. ACA C.2.g: seek help and limit or suspend services when impaired. A: puts clients at risk. C: burdens

clients. D: an overreaction; she can arrange coverage and return when ready.

7 — B. Substantial risk of harm calls for reporting (ACA I. 2.b). A: fails to protect clients. C: violates confidentiality and proper process. D: inappropriate.

8 — C. Supervision is evaluative, ongoing, and involves shared responsibility. A and B: reversed. D: vicarious liability applies to supervisors, not consultants.

9 — B. ACA B.2.b. A: ignores the option the Code provides. C: imposes the counselor's view. D: abandonment.

10 — B. ACA A.10.d. A: skips required notice. C: discloses unnecessary information. D: abandonment.

11 — B. Confidentiality, informed consent, and counselor responsibility apply to AI (ACA B, A.2, H; ACA AI Work Group recommendations). A: breaches confidentiality. C: omits informed consent. D: the counselor remains responsible for records.

12 — C. Universality asks whether you'd recommend the action to others. A: publicity. B: justice. D: law, not one of Stadler's tests.

Audio Companion

Track	Covers	Target length
17.1	Code and Law: principles, consent, confidentiality, boundaries, records, decision-making	~6 min

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Chapter 18 — Research, Statistics, and Program Evaluation

Domain map: 1.B Understand statistical concepts and methods in research · 2.T Use pre-test and post-test measures to assess outcomes · 2.U Evaluate counseling effectiveness · CACREP: Research and Program Evaluation

Why Research Matters on a Counseling Exam

Research items are a small share of the NCE, but they're among the most predictable. They test definitions and matching: which design fits, which statistic fits, which threat explains a result. Chapter 3 gave you the owners and a quick "which test" table. This chapter fills in the rest and connects research to practice: measuring whether counseling works.

Strategy: for statistics items, identify (1) how many groups, (2) what kind of data, and (3) whether the question is about **difference** or **relationship**. That narrows almost every option.

Part 1 — Research Basics

Variables

Term	Meaning	Example
Independent variable (IV)	What the researcher manipulates or compares	Type of treatment (CBT vs. waitlist)
Dependent variable (DV)	What's measured as the outcome	Depression score

Term	Meaning	Example
Confounding variable	An uncontrolled variable that offers an alternative explanation	Some participants also started medication
Moderator	Changes the **strength or direction** of a relationship (for whom, or when)	CBT works better for adults than adolescents
Mediator	**Explains how** or why the IV affects the DV	CBT reduces depression **by** reducing negative thoughts
Operational definition	How a variable is measured in a study	"Anxiety" = score on the GAD-7

Hypotheses

- **Null hypothesis (H_0):** no difference or no relationship.
- **Alternative (research) hypothesis (H_1):** there is a difference or relationship.
- **Directional** (one-tailed): predicts the direction ("CBT will **lower** scores").
- **Nondirectional** (two-tailed): predicts a difference without direction.

Researchers **reject** or **fail to reject** the null hypothesis. They never "prove" it.

Sampling

Method	Description
Simple random	Every member of the population has an equal chance of selection
Stratified random	Population divided into subgroups (strata), then randomly sampled from each, often in proportion
Cluster	Randomly select whole groups (e.g., schools), then study everyone in them

Method	Description
Systematic	Every *n*th person from a list
Convenience	Whoever is available; weakest generalizability
Purposive	Chosen for specific characteristics (common in qualitative research)
Snowball	Participants recruit others; useful for hard-to-reach populations

Random sampling supports **external validity** (generalizing to the population). **Random assignment** to groups supports **internal validity** (cause and effect). They're different things.

Part 2 — Research Designs

Quantitative Designs

Design	Key feature	Can it show cause and effect?
True experimental	**Random assignment** to groups; manipulation of the IV; control group	**Yes** (strongest)
Quasi-experimental	Manipulation of the IV **without random assignment** (intact groups, such as two classrooms)	Limited
Correlational	Measures relationships between variables without manipulation	**No**: correlation doesn't prove causation
Descriptive / survey	Describes characteristics of a group	No

Design	Key feature	Can it show cause and effect?
Ex post facto (causal-comparative)	Compares groups that already differ on a variable (e.g., people who were or weren't abused)	No
Single-subject (single-case)	One person or small group measured repeatedly across phases	Can suggest causation through replication
Longitudinal	Same participants measured over time	Shows change over time
Cross-sectional	Different age groups measured at one time	Faster, but cohort effects can confound
Meta-analysis	Statistically combines results of many studies, usually reporting an overall effect size	Summarizes evidence

Single-Subject Designs

Design	Phases
AB	Baseline (A), then intervention (B)
ABA (reversal/withdrawal)	Baseline, intervention, withdrawal of intervention
ABAB	Baseline, intervention, withdrawal, reintroduction; stronger evidence the intervention caused the change
Multiple baseline	Intervention introduced at **staggered times** across behaviors, settings, or people; used when withdrawing treatment would be unethical or impossible

Qualitative Traditions

Qualitative research explores meaning and experience, usually through interviews, observation, or documents.

Tradition	Focus	Owner / association
Phenomenology	The lived experience of a phenomenon; its essence	Husserl; Moustakas
Grounded theory	Developing a **theory** from data	**Glaser & Strauss**
Ethnography	A **culture** or group, through immersion and observation	Anthropology tradition
Case study	In-depth study of a bounded case	Stake; Yin
Narrative	Individuals' stories	—
Consensual qualitative research (CQR)	A team reaches consensus on themes	**Clara Hill**
Action / participatory action research	Researchers and participants collaborate to solve a practical problem and create change	Kurt Lewin (action research)

Trustworthiness in qualitative research (Lincoln & Guba): **credibility** (parallels internal validity), **transferability** (parallels external validity), **dependability** (parallels reliability), and **confirmability** (parallels objectivity). Strategies include **member checking**, **triangulation**, **peer debriefing**, **thick description**, and **audit trails**.

Mixed methods combine quantitative and qualitative data.

Part 3 — Validity in Research

Type	Question
Internal validity	Did the IV really cause the change in the DV?
External validity	Do the results generalize to other people, settings, and times?

Threats to Internal Validity (Campbell & Stanley)

Threat	What happens
History	An outside event during the study affects results (e.g., a natural disaster during a stress study)
Maturation	Natural changes in participants over time (growing older, tired, recovering on their own)
Testing	Taking a pretest affects posttest scores (practice effects)
Instrumentation	Changes in the measure or the raters between testings
Statistical regression	Participants selected for **extreme** scores drift toward the mean on retest
Selection	Groups differ at the start
Experimental mortality (attrition)	Participants drop out unevenly across groups
Selection–maturation interaction	Groups change at different rates
Diffusion of treatment	Control group learns about or receives part of the treatment (added to the list later by Cook & Campbell, 1979; not one of Campbell & Stanley's original eight)

Threats to External Validity and Other Effects

Effect	Description
Hawthorne effect	Participants change because they know they're being studied (Mayo)
Experimenter expectancy (Rosenthal effect)	Researcher's expectations influence results
Placebo effect	Improvement from the belief that one is receiving treatment
Demand characteristics	Participants guess the hypothesis and act accordingly

Effect	Description
John Henry effect	Control group works harder to compete with the treatment group
Reactivity of pretesting	The pretest sensitizes participants to the treatment (addressed by the Solomon four-group design)

Controls: random assignment, control groups, **double-blind** procedures (neither participants nor researchers know group assignments), and placebo conditions.

Part 4 — Descriptive Statistics

Central Tendency

Measure	Definition	Best for
Mean	Arithmetic average	Interval/ratio data with a roughly symmetrical distribution; **most affected by outliers**
Median	Middle score	Skewed data; ordinal data
Mode	Most frequent score	Nominal data; can have more than one (bimodal)

Variability

Measure	Definition
Range	Highest minus lowest; affected by extremes
Variance	Average of squared deviations from the mean
Standard deviation (SD)	Square root of the variance; the typical distance of scores from the mean
Interquartile range	Range of the middle 50% of scores

Distributions

- **Normal curve:** symmetrical; mean = median = mode; about **68%** of scores within ±1 SD, **95%** within ±2 SD, **99.7%** within ±3 SD.
- **Positive (right) skew:** tail to the right; most scores are low; **mean > median > mode** (the pattern after a hard test).
- **Negative (left) skew:** tail to the left; most scores are high; **mean < median < mode** (the pattern after an easy test).
- **Kurtosis:** peakedness. **Leptokurtic** (tall, narrow), **mesokurtic** (normal), **platykurtic** (flat).

Memory aid: the tail tells the tale. The skew is named for the direction of the tail, and the mean gets pulled toward the tail.

Part 5 — Inferential Statistics

Correlation

- **Pearson r** ranges from **−1.00 to +1.00**.
- The **sign** shows direction (positive: both rise together; negative: one rises as the other falls).
- The **absolute value** shows strength. **−.80 is stronger than +.50.**
- **Coefficient of determination (r^2):** the proportion of variance shared. An r of .60 means $r^2 = .36$, so 36% of the variance is shared.
- **Correlation does not imply causation.**

Choosing a Test

Question	Test
Two group means	**t-test** (William Gosset; independent for different groups; dependent/paired for the same people measured twice)
Three or more group means, one IV	**One-way ANOVA** (Ronald Fisher)

Question	Test
Two or more IVs	**Factorial ANOVA** (e.g., 2 × 3); can show **interaction effects**
Means, controlling for a covariate	**ANCOVA**
Two or more DVs	**MANOVA**
Frequencies in categories (nominal data)	**Chi-square**
Relationship between two interval/ratio variables	**Pearson r**
Relationship between two ranked variables	**Spearman rho**
Predicting one variable from another	**Regression** (multiple regression with several predictors)
Nonparametric alternative to independent t-test	**Mann–Whitney U**
Nonparametric alternative to one-way ANOVA	**Kruskal–Wallis**
Nonparametric alternative to paired t-test	**Wilcoxon signed-rank**
Reducing many variables into underlying factors	**Factor analysis**

Parametric tests (t-test, ANOVA, Pearson r) assume interval/ratio data and roughly normal distributions. **Nonparametric** tests (chi-square, Mann–Whitney, Kruskal–Wallis, Spearman) work with nominal or ordinal data or when assumptions aren't met.

Post hoc tests (e.g., Tukey, Scheffé) follow a significant ANOVA to find **which** groups differ.

Significance, Errors, and Effect Size

Term	Meaning
Alpha (α)	The chosen risk of a Type I error, usually **.05**

Term	Meaning
p-value	The probability of obtaining results at least this extreme **if the null hypothesis were true**. If $p < \alpha$, the result is statistically significant
Type I error	Rejecting a **true** null: a **false positive**
Type II error (β)	Failing to reject a **false** null: a **false negative**
Power ($1 - \beta$)	Probability of detecting a real effect; increased by **larger samples**, larger effects, and a higher alpha
Effect size	How large the effect is. **Cohen's d:** about **.2 small, .5 medium, .8 large**
Degrees of freedom	Related to sample size; used to evaluate test statistics

Lowering alpha (e.g., from .05 to .01) **reduces Type I errors** but **increases Type II errors**.

Statistical vs. clinical significance: a large study can find a statistically significant but tiny difference that doesn't matter in clients' lives. **Clinical significance** asks whether change is meaningful, often whether a client moved from a dysfunctional range to a functional range. The **reliable change index** (Jacobson & Truax) tests whether an individual's change is larger than measurement error.

Part 6 — Evaluating Counseling (2.T, 2.U)

Measuring Outcomes

- **Pre-test and post-test measures** compare a client's scores before and after counseling (e.g., PHQ-9 at intake and at week 8).
- **Measurement-based care:** routinely administering brief measures (often every session) and using the results to guide treatment.

- **Feedback-informed treatment** tools, such as the **Outcome Rating Scale (ORS)** and **Session Rating Scale (SRS)** (Miller & Duncan), track progress and the alliance session by session.
- **Goal Attainment Scaling** (Kiresuk & Sherman): rates progress on individualized goals.

Common Factors

Research on what makes counseling work points to **common factors** shared across approaches, especially the **therapeutic alliance**, client factors, counselor empathy, and expectancy. **Saul Rosenzweig** (1936) introduced the idea, quoting the Dodo from *Alice in Wonderland* ("Everybody has won, and all must have prizes"); Luborsky and colleagues (1975) later named it the **"Dodo bird verdict"**: many bona fide therapies produce similar outcomes. **Bruce Wampold** extended this research.

Evidence-Based Practice

The American Psychological Association's definition of **evidence-based practice** integrates three components: 1. **Best available research** 2. **Clinical expertise** 3. **Client characteristics, culture, and preferences**

Program Evaluation

Term	Meaning
Needs assessment	Identifies needs and gaps **before** a program is designed
Formative evaluation	Ongoing evaluation **during** a program to improve it
Summative evaluation	Evaluation **at the end** to judge effectiveness
Process evaluation	Was the program delivered as planned (fidelity)?
Outcome evaluation	Did the program achieve its goals?
Cost-benefit analysis	Compares costs with benefits measured in dollars

Term	Meaning
Cost-effectiveness analysis	Compares costs with outcomes measured in nonmonetary units (e.g., cost per client improved)
Logic model	Maps inputs → activities → outputs → outcomes
Accountability	Demonstrating to stakeholders (funders, schools, agencies) that services work

Part 7 — Research Ethics (ACA Section G)

- **Institutional review board (IRB)** approval for research with human participants.
- **Informed consent** (G.2.a): purpose, procedures, risks, benefits, the right to withdraw **without penalty**, and confidentiality.
- **Deception** (G.2.b): used only when alternative procedures aren't feasible and the prospective value justifies it; participants are **debriefed** as soon as possible.
- **Voluntary participation**: declining to participate doesn't affect clients' services or students' grades.
- **Confidentiality** of data.
- **Accurate reporting** (G.4): report results accurately, including unfavorable results; no fabrication or plagiarism.
- **Vulnerable populations**: extra protections for minors and those unable to consent.

Historical cases that shaped research ethics: the **Tuskegee syphilis study**, Milgram's obedience studies, and the Stanford prison experiment. The **Belmont Report** (1979) set three principles: **respect for persons, beneficence, and justice**.

What the Exam Rewards / What It Punishes

Rewards: - Matching the test to the number of groups and type of data - Knowing random assignment is what allows causal conclusions - Naming the specific threat to validity - Interpreting correlation strength by absolute value - Distinguishing statistical from clinical significance - Using pre/post and session-by-session measures to evaluate counseling

Punishes: - Concluding causation from correlation - Confusing random sampling with random assignment - Mixing up Type I and Type II errors - Reading a negative correlation as “weak” - Treating a significant p-value as proof of a large effect

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Swap-Pair Drill

If you see…	It’s…	Not…
Explains **how** the IV affects the DV	Mediator	Moderator
Changes **when or for whom** an effect holds	Moderator	Mediator
Random assignment	True experiment	Quasi-experiment
Intact groups, no random assignment	Quasi-experiment	True experiment
Staggered intervention across behaviors	Multiple baseline	ABAB
Building theory from data	Grounded theory (Glaser & Strauss)	Phenomenology
Essence of lived experience	Phenomenology	Ethnography
Team consensus on themes	CQR (Hill)	Grounded theory
Outside event during the study	History	Maturation

If you see…	It's…	Not…
Extreme scorers move toward the mean	Statistical regression	Testing
Changes because they know they're observed	Hawthorne effect	Placebo effect
Control group tries harder	John Henry effect	Hawthorne effect
Tail to the right; mean > median	Positive skew	Negative skew
Compare two group means	t-test	ANOVA
Compare group means on two or more IVs	Factorial ANOVA	One-way ANOVA
Compare groups on two or more DVs	MANOVA	ANCOVA
Nominal frequencies	Chi-square	t-test
False positive	Type I	Type II
Improve a program while running	Formative	Summative
Identify gaps before designing	Needs assessment	Outcome evaluation
Program evaluation: outcomes in dollars	Cost-benefit	Cost-effectiveness
Respect for persons, beneficence, justice	Belmont Report	ACA principles

Quick Checks

1. A researcher randomly assigns 80 clients to either CBT or a waitlist and measures anxiety after 10 weeks. The design is: A. Correlational B. Quasi-experimental C. True experimental D. Ex post facto

2. A study finds that the alliance explains why a treatment reduces symptoms. The alliance is a: A. Moderator B. Mediator C. Confound D. Dependent variable

3. A researcher compares anxiety scores of clients in three different treatment groups. The appropriate test is: A. Independent t-test B. Chi-square C. One-way ANOVA D. Pearson r

4. Which correlation shows the strongest relationship? A. +.45 B. −.72 C. +.60 D. −.10

5. If r = .50, what proportion of variance is shared? A. 50% B. 25% C. 5% D. 75%

6. On a very difficult exam, most students score low and a few score high. The distribution is: A. Normal B. Positively skewed C. Negatively skewed D. Bimodal

7. Participants chosen because they scored extremely high on an anxiety scale score lower on retest, even without treatment. This is: A. History B. Maturation C. Statistical regression D. Attrition

8. A researcher concludes a program reduced dropout when, in fact, it had no effect. This is a: A. Type I error B. Type II error C. Lack of power D. Mediator effect

9. A counselor gives clients the PHQ-9 at intake and again at session 8 to see whether depression decreased. This is an example of: A. Pre-test/post-test outcome measurement B. A needs assessment C. A cost-benefit analysis D. Phenomenological research

10. A study with 10,000 participants finds a statistically significant difference of half a point on a 60-point scale. The best interpretation is: A. The treatment has a large effect B. The difference is statistically significant but may not be clinically meaningful C. The study has low power D. A Type II error occurred

11. A school counselor wants to know what supports students need before designing a new program. She should conduct a: A. Summative evaluation B. Needs assessment C. Outcome evaluation D. Meta-analysis

12. Which qualitative tradition aims to develop a theory grounded in the data? A. Phenomenology B. Ethnography C. Grounded theory D. Narrative

Answers and Rationales

1 — C. Random assignment plus manipulation is a true experiment. A: correlational designs don't manipulate. B: quasi-experiments lack random assignment. D: ex post facto compares preexisting groups.

2 — B. A variable that explains how an effect occurs is a mediator. A: a moderator changes when or for whom. C: a confound offers an alternative explanation. D: the DV is the outcome (symptoms).

3 — C. Three or more group means: one-way ANOVA. A: a t-test compares two means. B: chi-square is for frequencies. D: Pearson r measures relationships.

4 — B. Strength is the absolute value; $|-.72|$ is the largest. A, C, D: smaller absolute values.

5 — B. $r^2 = .25$, so 25%. A: confuses r with r^2. C and D: incorrect.

6 — B. Most scores low with a tail to the right is positive skew. A: normal is symmetrical. C: negative skew has a tail to the left (most scores high). D: bimodal has two peaks.

7 — C. Extreme scores drift toward the mean: statistical regression. A: history is an outside event. B: maturation is natural change over time. D: attrition is dropout.

8 — A. A false positive is a Type I error. B: a Type II error misses a real effect. C: low power increases Type II risk. D: not relevant.

9 — A. Measuring before and after counseling (2.T). B: needs assessment precedes a program. C: cost-benefit compares dollars. D: phenomenology is qualitative.

10 — B. Large samples can make trivial differences statistically significant. A: the effect is tiny. C: power is very

high with 10,000 participants. D: the null was rejected, so a Type II error didn't occur.

11 — B. Needs assessment identifies gaps before design. A and C: evaluate after or during implementation. D: meta-analysis combines existing studies.

12 — C. Grounded theory (Glaser & Strauss) builds theory from data. A: phenomenology describes lived experience. B: ethnography studies a culture. D: narrative focuses on stories.

Audio Companion

Track	Covers	Target length
18.1	Pick the Test: designs, validity threats, statistics, evaluation	~5 min

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Chapter 19 — Intake, Interview, Mental Status Exam, and Diagnosis

Domain map: 2.A Conduct a biopsychosocial interview · 2.B Conduct a diagnostic interview · 2.C Conduct cultural formulation interview · 2.D Conduct an initial interview · 2.E Determine diagnosis · 2.F Perform a Mental Status Exam (MSE) · 2.G Consider co-occurring diagnoses · 2.H Determine level of care needed · 2.I Determine the appropriate modality of treatment · 2.J Assess the presenting problem and level of distress · 2.K Evaluate an individual's level of mental health functioning · 2.L Screen clients for appropriate services · 2.N Use formal and informal observations · 2.O Assess for trauma · 2.P Assess substance use · 2.Q Obtain client self-reports · 2.R Evaluate interactional dynamics · 2.S Conduct ongoing assessment for at-risk behaviors · CACREP: Assessment and Testing; Counseling and Helping Relationships

The Intake Is Where Everything Starts

Intake, Assessment, and Diagnosis is 19 of the 160 scored items. The domain asks what a counselor gathers at the start of counseling, in what order, and what to do with it. Several other chapters supply the content (risk in Chapter 6, culture in Chapter 9, presentations in Chapters 10–16). This chapter covers the **process**: the interview, the mental status exam, diagnosis, and matching clients to the right level and kind of care.

The guiding sequence: safety → relationship → assessment → diagnosis → plan. Assessment never stops after intake; it continues throughout counseling (2.S).

Part 1 — The Initial Interview (2.D, 2.J, 2.L, 2.Q)

Purposes of the First Session

1. **Informed consent** and limits of confidentiality (Chapter 17)
2. **Build rapport** (Chapter 4)
3. **Screen for safety**: suicide, homicide, abuse, violence (Chapter 6)
4. **Understand the presenting problem**: what brings the client in now
5. **Screen for fit**: are these the right services, at the right level of care, with the right counselor (2.L)?
6. **Begin collaborative goal-setting**

Exploring the Presenting Problem

Ask about: - **Onset**: When did it start? What was happening then? - **Duration and frequency** - **Severity and distress** (2.J): a **SUDS rating** (subjective units of distress, 0–100) or a 0–10 scale - **Impact on functioning**: work, school, relationships, self-care - **Precipitants**: Why now? - **Previous attempts** to cope and previous treatment - **The client's own explanation** of the problem (2.C; the cultural formulation)

Types of Interviews

Type	Description	Strength	Limitation
Structured	Fixed questions in fixed order (e.g., **SCID-5**, Structured Clinical Interview for DSM-5)	High reliability; good for research and diagnosis	Less flexible; can feel impersonal
Semi-structured	Core questions with room for follow-up	Balance of reliability and flexibility	Requires training

Type	Description	Strength	Limitation
Unstructured	Conversational, follows the client	Builds rapport; flexible	Lower reliability; may miss information

Self-Report and Observation (2.N, 2.Q)

- **Self-report**: interviews, questionnaires, symptom checklists, journals. Efficient, but subject to bias, insight, and honesty.
- **Formal observation**: structured methods (e.g., behavior rating scales, timed observations).
- **Informal observation**: what the counselor notices during the session — appearance, behavior, interaction, affect.
- **Collateral information** (with consent): family, teachers, prior providers, records.

Use several sources. Agreement among them increases confidence.

Part 2 — The Biopsychosocial Interview (2.A)

The **biopsychosocial** assessment (based on Engel's model, Chapter 16) gathers a whole-person history:

Area	What to ask about
Biological	Medical conditions, medications, sleep, appetite, pain, substance use, family medical and psychiatric history, recent physical exam
Psychological	Presenting symptoms, mental health history, previous treatment and hospitalizations, trauma history, coping style, strengths

Area	What to ask about
Social	Family of origin, current relationships, support system, culture and identity, religion and spirituality, education, work, finances, housing, legal issues, military history
Developmental	Milestones, childhood experiences, school history
Risk	Suicide, homicide, self-harm, abuse, violence (Chapter 6)
Strengths	Resources, skills, values, supports (essential for treatment planning; Chapter 21)

The exam's sequencing rule: when a client arrives in crisis, a full history waits. Address safety first, then gather history.

The 4 Ps Case Formulation

A common way to organize intake information:

P	Question
Predisposing	What made the client vulnerable? (genetics, early experiences, temperament)
Precipitating	What triggered the current problem?
Perpetuating	What keeps it going? (avoidance, substance use, family patterns, negative thinking)
Protective	What strengths and supports help?

Part 3 — The Cultural Formulation Interview (2.C)

The DSM-5-TR **Cultural Formulation Interview (CFI)** has **16 questions** in four domains:

1. **Cultural definition of the problem** ("What brings you here today? How would you describe the problem to your family and friends?")
2. **Cultural perceptions of cause, context, and support** ("Why do you think this is happening?" "What makes it better or worse?")
3. **Cultural factors affecting self-coping and past help-seeking**
4. **Cultural factors affecting current help-seeking** (including concerns about the counselor–client relationship)

There's also an **informant version** for family members and **supplementary modules** for specific topics (e.g., spirituality, immigrants and refugees, older adults). The CFI can be used with any client, not only clients from minority cultures.

Part 4 — The Mental Status Exam (2.F)

The **MSE** is a snapshot of the client's current mental functioning, based mostly on **observation** during the interview. It's the psychological equivalent of a physical exam.

Component	What's assessed	Example terms
Appearance	Grooming, hygiene, dress, apparent age, notable features	Well-groomed; disheveled; appears older than stated age
Behavior / psychomotor activity	Eye contact, posture, movements, cooperation	Psychomotor **agitation** or **retardation**; tremor; guarded; cooperative

Component	What's assessed	Example terms
Speech	**Rate, volume, rhythm, tone**, amount	Pressured; slow; soft; monotone; poverty of speech
Mood	The client's **self-reported**, sustained emotional state, often in the client's own words	"I've been really down"
Affect	The counselor's **observation** of emotional expression in the moment	**Range**: full, restricted, **blunted**, **flat**. **Congruence**: congruent or incongruent with mood and content. **Lability**: rapid shifts
Thought process	**How** the client thinks: organization and flow	**Linear/goal-directed**; **circumstantial** (wanders but gets to the point); **tangential** (wanders and never returns); **flight of ideas** (rapid jumps between loosely connected topics); **loose associations** (unconnected ideas); **thought blocking**; **perseveration**; **clang associations**; **word salad**
Thought content	**What** the client thinks about	Suicidal or homicidal ideation; **delusions**; **obsessions**; preoccupations; phobias
Perception	Sensory experiences	**Hallucinations** (no stimulus present; auditory most common in psychotic disorders); **illusions** (misperception of a real stimulus); depersonalization and derealization

Component	What's assessed	Example terms
Cognition / sensorium	Orientation, attention, concentration, memory	**Oriented ×3** (person, place, time) or ×4 (plus situation); serial sevens; recall of three words
Insight	The client's understanding of their problem	Good, fair, poor, absent
Judgment	Ability to make sound decisions	Good, fair, poor

High-Yield MSE Distinctions

Pair	Difference
Mood vs. affect	Mood is **reported** by the client and sustained ("climate"); affect is **observed** by the counselor in the moment ("weather")
Circumstantial vs. tangential	Circumstantial speech **reaches the point** eventually; tangential speech **never does**
Flight of ideas vs. loose associations	Flight of ideas jumps quickly between **loosely connected** topics (common in mania); loose associations have **no logical connection** (common in psychosis)
Hallucination vs. illusion	Hallucination: **no stimulus**; illusion: **misperceived real stimulus** (a coat on a chair looks like a person)
Delusion vs. obsession	A delusion is a **fixed false belief** held despite evidence; an obsession is an intrusive thought the person usually recognizes as excessive
Thought process vs. thought content	Process is **how**; content is **what**
Blunted vs. flat affect	Blunted: significantly **reduced** expression; flat: virtually **no** expression

Screening tools for cognition: MoCA, MMSE, Mini-Cog (Chapter 12).

Part 5 — Diagnosis (2.B, 2.E, 2.G)

How the DSM-5-TR Works

- The DSM-5 (2013) **eliminated the multiaxial system** (Axes I–V). Diagnoses are listed together, with medical conditions and psychosocial factors noted alongside.
- The **GAF** (Global Assessment of Functioning) was dropped. The DSM-5-TR suggests the **WHO Disability Assessment Schedule 2.0 (WHODAS 2.0)** for measuring functioning (2.K).
- Diagnoses use **ICD-10-CM codes** for billing in the U.S.
- **Specifiers** add detail (e.g., severity; "with anxious distress"; "in early remission").
- **Provisional** diagnosis: used when there's a strong presumption the criteria will be met but not enough information yet.
- **Other specified** disorder: the clinician states why criteria aren't fully met (e.g., "other specified depressive disorder, depressive episode with insufficient symptoms").
- **Unspecified** disorder: the clinician chooses not to, or can't, specify why criteria aren't met (common in emergency settings).
- **Z codes**: other conditions that may be a focus of clinical attention, such as relationship distress, uncomplicated bereavement, problems with housing or employment, and moral, religious, or spiritual problems (Chapter 15). They aren't mental disorders.
- **Principal diagnosis** (inpatient): the condition chiefly responsible for the admission. **Reason for visit** (outpatient): the condition chiefly responsible for the outpatient services received. Either is listed first.

Differential Diagnosis

A widely taught sequence (adapted from Michael First's *DSM-5-TR Handbook of Differential Diagnosis*):

1. **Rule out malingering and factitious disorder**
2. **Rule out a substance or medication cause** (Chapter 11)
3. **Rule out a medical condition** (Chapter 10, Chapter 12, Chapter 16)
4. **Determine the specific primary disorder(s)**
5. **Differentiate adjustment disorder** from other specified/unspecified disorders
6. **Establish the boundary with no mental disorder**: is there clinically significant distress or impairment?

Term	Meaning
Malingering	**Intentionally** producing or exaggerating symptoms for an **external** reward (avoiding work or prosecution, getting money or drugs). A Z code, not a mental disorder
Factitious disorder	**Intentionally** falsifying symptoms **without obvious external reward**, to assume the sick role. **Imposed on self** or **imposed on another** (formerly "Munchausen by proxy")
Somatic symptom and functional neurological disorders	Symptoms are **not** intentionally produced (Chapter 16)

Co-Occurring Diagnoses (2.G)

- Co-occurrence is common: depression with anxiety, substance use with trauma, ADHD with learning disorders.
- Diagnose every disorder for which criteria are met, unless the criteria say otherwise (some diagnoses exclude others: DMDD, for example, rules out a concurrent ODD diagnosis).

- Treat co-occurring substance use and mental health disorders together (integrated treatment; Chapter 9, Chapter 11).

Ethics of Diagnosis

- Explain the diagnosis and its implications (including insurance and records) as part of informed consent (ACA A.2.b, E.5).
- **Cultural sensitivity** in diagnosis (E.5.b): culture affects how problems are defined and expressed.
- **Historical and social prejudices** in diagnosis (E.5.c): be aware of how some groups have been misdiagnosed and pathologized.
- **Refraining from diagnosis** (E.5.d): counselors may refrain from making or reporting a diagnosis if they believe it would cause harm.
- **Never assign a diagnosis just for insurance reimbursement** when criteria aren't met. That's fraud.

Part 6 — Specialized Assessment

Area	Key points	Where covered
At-risk behaviors (2.S)	Assess suicide, homicide, self-harm, and relationship violence at intake and **on an ongoing basis**	Chapter 6
Trauma (2.O)	Ask about trauma with care and permission; screen (PC-PTSD-5, LEC-5, PCL-5, ACEs); don't push for detail at intake	Chapter 10
Substance use (2.P)	Ask every client; screen (CAGE, AUDIT, CRAFFT, DAST); assess withdrawal risk	Chapters 9, 11

Area	Key points	Where covered
Functioning (2.K)	WHODAS 2.0; daily activities; work, school, relationships	This chapter
Interactional dynamics (2.R)	In couple and family sessions, observe who speaks, who's silent, alliances, conflict patterns, and nonverbal responses	Chapter 7

Part 7 — Level of Care and Modality (2.H, 2.I)

Level of Care

Match the intensity of services to the client's needs, using the **least restrictive** level that's safe and effective.

Level	Description	Typical indications
Outpatient	Regular sessions (e.g., weekly)	Mild to moderate symptoms; safe; functioning in daily life
Intensive outpatient (IOP)	Several hours a day, several days a week; client lives at home	More support needed; stepping down from a higher level
Partial hospitalization (PHP)	Day program, most of the day, most days; client returns home at night	Significant symptoms without need for 24-hour care
Residential	Living in a structured treatment setting	Need for a structured environment (often substance use or eating disorders)
Inpatient hospitalization	24-hour medically supervised care	**Imminent danger** to self or others; severe inability to care for self; acute psychosis or mania; medically unstable

Tools: the **ASAM Criteria** for substance use (six dimensions; Chapter 9); **LOCUS** (Level of Care Utilization System) for adults and **CALOCUS/CASII** for children and adolescents.

Involuntary hospitalization generally requires evidence of danger to self, danger to others, or grave disability (inability to meet basic needs), according to state law.

Choosing a Modality (2.I)

Modality	Often indicated when…
Individual	Privacy is essential; individualized focus; safety concerns that make joint sessions risky
Couple	The relationship is the focus, and **no ongoing violence or coercive control** (Chapter 6)
Family	The problem is maintained by family patterns; a child or adolescent is the identified patient (Chapter 7)
Group	Social learning, universality, skill-building, and support would help, and the client has passed screening (Chapter 8)
Combined	Many clients benefit from more than one modality

What the Exam Rewards / What It Punishes

Rewards: - Safety screening first, especially in crisis - Asking every client about substance use and trauma, with care - Using multiple sources of information - Knowing the MSE terms precisely (mood vs. affect; tangential vs. circumstantial) - Ruling out substances and medical causes before primary diagnoses - Least restrictive level of care that's safe - Ongoing assessment of risk

Punishes: - Taking a full history before assessing safety in a crisis - Diagnosing from a single symptom or a single source - Confusing malingering (external gain) with facti-

tious disorder (sick role) - Using the GAF or multiaxial system as current practice - Giving a diagnosis to obtain insurance payment when criteria aren't met - Hospitalizing when outpatient care is safe, or keeping outpatient care when danger is imminent

Swap-Pair Drill

If you see…	It's…	Not…
Client says "I feel down"	Mood	Affect
Counselor observes tearful, restricted expression	Affect	Mood
Speech wanders but gets to the point	Circumstantial	Tangential
Speech wanders and never gets to the point	Tangential	Circumstantial
Rapid jumps between related topics	Flight of ideas	Loose associations
No logical connection between ideas	Loose associations	Flight of ideas
Hearing voices with no source	Hallucination	Illusion
Shadow mistaken for a person	Illusion	Hallucination
Person, place, time	Orientation ×3	Insight
Fixed questions in fixed order	Structured interview (SCID-5)	Unstructured
Replaced the GAF	WHODAS 2.0	Multiaxial Axis V
Fakes symptoms for money or to avoid prosecution	Malingering	Factitious disorder
Fakes symptoms to be seen as sick	Factitious disorder	Malingering
Clinician explains why criteria aren't met	Other specified	Unspecified

If you see…	It's…	Not…
Predisposing, precipitating, perpetuating, protective	4 Ps case formulation	Biopsychosocial model
Day program, home at night	Partial hospitalization	Residential
Adults' level of care tool	LOCUS	ASAM

Quick Checks

1. A client says, "I'm fine, really," while crying and looking at the floor. Her affect is best described as: A. Congruent with her stated mood B. Incongruent with her stated mood C. Flat D. Labile

2. Asked about his job, a client begins describing his commute, his coworkers, and his lunch, and eventually answers the question. This is: A. Tangential B. Circumstantial C. Flight of ideas D. Loose associations

3. A client sees a coat hanging on a door and briefly thinks it's a person standing there. This is: A. A hallucination B. An illusion C. A delusion D. Depersonalization

4. A client arrives saying he plans to kill himself tonight. The counselor's first priority is to: A. Complete the biopsychosocial history B. Assess and respond to the immediate safety risk C. Administer the WHODAS 2.0 D. Explore his family of origin

5. In the DSM-5-TR, which tool is suggested for assessing functioning? A. GAF B. WHODAS 2.0 C. Axis V D. MMPI-3

6. A client reports pain symptoms that worsen whenever a disability hearing approaches and admits he needs "to look sick enough" to get benefits. This most suggests: A. Somatic symptom disorder B. Factitious disorder C. Malingering D. Functional neurological symptom disorder

7. A client's symptoms nearly meet criteria for a depressive disorder, and the counselor records why they don't

fully meet them. The appropriate category is: A. Unspecified depressive disorder B. Other specified depressive disorder C. Provisional major depressive disorder D. No diagnosis

8. In the 4 Ps formulation, a client's daily drinking that keeps her anxiety from improving is a: A. Predisposing factor B. Precipitating factor C. Perpetuating factor D. Protective factor

9. Which is the first step in the differential diagnosis sequence? A. Determine the primary disorder B. Rule out malingering and factitious disorder C. Assign an adjustment disorder D. Establish the boundary with no disorder

10. A client with severe depression has no suicidal ideation, lives with supportive family, but has stopped functioning at work and needs more support than weekly sessions provide. The most appropriate level of care is likely: A. Inpatient hospitalization B. Intensive outpatient or partial hospitalization C. No treatment D. Residential care for a year

11. A counselor is asked by a client to "put down something insurance will pay for" even though the client's symptoms don't meet criteria for any disorder. The counselor should: A. Assign a mild diagnosis to help the client B. Decline, explain that diagnoses must reflect actual criteria, and discuss other payment options C. Assign an "unspecified" diagnosis D. Bill under a family member's diagnosis

12. The Cultural Formulation Interview's first domain addresses: A. Cultural definition of the problem B. Medications and medical history C. Suicide risk D. Treatment goals

Answers and Rationales

1 — B. Her observed affect (crying) doesn't match her stated mood ("fine"). A: they don't match. C: flat affect shows virtually no expression. D: labile affect shifts rapidly.

2 — B. Wandering but eventually reaching the point is circumstantial. A: tangential never returns. C: flight of ideas involves rapid jumps. D: loose associations lack logical connection.

3 — B. Misperceiving a real stimulus is an illusion. A: a hallucination has no stimulus. C: a delusion is a fixed false belief. D: depersonalization is feeling detached from oneself.

4 — B. Safety comes first (Chapter 6). A, C, D: wait until safety is addressed.

5 — B. The DSM-5-TR suggests the WHODAS 2.0. A and C: the GAF and Axis V were dropped. D: the MMPI-3 is a personality inventory.

6 — C. Intentional symptom production for external gain (benefits) is malingering. A and D: symptoms aren't intentionally produced. B: factitious disorder involves no obvious external reward.

7 — B. "Other specified" is used when the clinician states why criteria aren't met. A: "unspecified" doesn't specify the reason. C: provisional is used when criteria are expected to be met with more information. D: a diagnosis is appropriate given clinically significant symptoms.

8 — C. Drinking that maintains the problem is perpetuating. A: predisposing factors are vulnerabilities. B: precipitating factors trigger the problem. D: protective factors help.

9 — B. The first step rules out malingering and factitious disorder (First). A, C, D: later steps.

10 — B. Needs exceed outpatient care but don't require 24-hour care; IOP or PHP is the least restrictive appropriate level. A: no imminent danger. C: untreated severe depression is harmful. D: excessive.

11 — B. Diagnosing to obtain payment when criteria aren't met is fraud and unethical. A, C, D: all misrepresent the client's condition.

12 — A. The CFI begins with the cultural definition of the problem. B, C, D: not CFI domains.

Audio Companion

Track	Covers	Target length
19.1	Intake and MSE: interview, MSE terms, diagnosis, level of care	~5 min

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Chapter 20 — Assessment and Testing

Domain map: 2.M Select, use, and interpret appropriate assessment instruments · 4.K Use assessment instrument results to facilitate client decision making · 2.T Use pre-test and post-test measures to assess outcomes · CACREP: Assessment and Testing

From Owners to Practice

Chapter 3 gave you the testing owners (Binet, Terman, Wechsler, Spearman, Cattell, Gardner, Sternberg), the types of reliability and validity, and the main score scales. This chapter covers what the NCE asks about using tests: choosing the right instrument, administering and scoring it, interpreting scores honestly, communicating results, and following the ethical and legal rules.

The principle that runs through the chapter: a test is one source of information. It informs decisions; it never makes them alone.

Part 1 — Types of Assessment

Distinction	Options
Standardized vs. nonstandardized	Standardized: uniform administration, scoring, and norms. Nonstandardized: informal checklists, observation, interviews
Norm-referenced vs. criterion-referenced vs. ipsative	Compared with **others** / with a **fixed standard** / with **oneself** (Chapter 3)

Distinction	Options
Maximal vs. typical performance	**Maximal**: best possible performance (intelligence, achievement, aptitude). **Typical**: usual behavior (personality, interests)
Speed vs. power	**Speed**: easy items, strict time limit. **Power**: items get harder; generous time
Objective vs. projective	**Objective**: structured responses, standard scoring (MMPI-3). **Projective**: ambiguous stimuli, open responses (Rorschach, TAT)
Individual vs. group	One examinee with one examiner, or many at once
Verbal vs. nonverbal	Language-dependent or language-reduced (e.g., Raven's Progressive Matrices)

What Each Category Measures

Category	Measures	Examples
Intelligence	General cognitive ability	WAIS, WISC, Stanford-Binet, KABC
Aptitude	Potential to learn or perform	ASVAB, GATB, SAT (often described as aptitude)
Achievement	What has been learned	WIAT, WRAT, Woodcock-Johnson Tests of Achievement
Personality	Traits, patterns, psychopathology	MMPI-3, MCMI, PAI, NEO-PI, 16PF
Interests	Preferences	Strong, SDS, Kuder (Chapter 14)
Values	What matters	MIQ (Chapter 14)
Symptoms and screening	Specific problems	PHQ-9, GAD-7, PCL-5, AUDIT
Behavior	Observed behavior, often by parents or teachers	CBCL, BASC, Conners

Category	Measures	Examples
Neuropsychological	Brain–behavior functioning	Halstead-Reitan, Bender-Gestalt II

Part 2 — Choosing an Instrument (2.M)

Ask:

1. **Purpose:** screening, diagnosis, treatment planning, career decision, or outcome measurement?
2. **Reliability:** is it consistent enough? For important individual decisions, coefficients around **.90** are often expected; **.80** is considered acceptable for many uses.
3. **Validity:** does it measure what you need, for **this purpose**?
4. **Norms:** was it normed on people **like this client** (age, culture, language, disability)?
5. **Fairness:** is there evidence of bias for this client's group?
6. **Practicality:** time, cost, reading level, format, accommodations.
7. **Counselor competence:** are you trained and qualified to administer and interpret it (ACA E.2.a)?

User Qualification Levels

Publishers restrict instruments by user qualifications, often described in three levels:

Level	Typical requirement	Examples
A	Little special training	Some achievement and screening tools
B	Graduate coursework in testing; often a master's-level counselor	Interest inventories, many personality inventories, symptom scales
C	Advanced training, often a doctorate or specialized supervised training	Individual intelligence tests, projective tests

Publishers now use their own qualification codes, and state laws on test use vary. The principle is the same: **use only instruments you're qualified to use.**

Legal Cases on Testing Fairness

Case	Holding
Griggs v. Duke Power Co. (1971)	Employment tests must be **job-related**; tests that screen out protected groups without job relevance are discriminatory
Diana v. State Board of Education (1970)	Children must be tested in their **primary language** (and nonverbal measures used) before special education placement
Larry P. v. Riles (1979)	IQ tests could not be used to place Black children in classes for "educable mentally retarded" students in California because of cultural bias
Hobson v. Hansen (1967)	Ability-grouping ("tracking") based on biased group tests was unconstitutional
Debra P. v. Turlington (1981)	A graduation test must cover what students were actually taught (**curricular/instructional validity**)

Part 3 — Administration and Scoring

- Follow **standardized procedures** (instructions, time limits, materials). Departures make norms less meaningful.
- Provide **accommodations** for disability when appropriate (ACA E.7.a; ADA), and note them when interpreting results.
- Establish **rapport**; note factors that could affect performance (fatigue, illness, anxiety, language, medication).
- **Computer-administered** tests must also be validated, and the counselor remains responsible for interpretation (ACA E.7.c; Chapter 9).

Scores

A **raw score** means little on its own. It must be converted using norms or a criterion.

Score	Mean	SD	Notes
z-score	0	1	Basis for all standard scores
T-score	50	10	Common for personality inventories (MMPI-3: a T-score of **65 or above** is generally considered clinically elevated)
Deviation IQ	100	15	Wechsler; current Stanford-Binet also uses 15
Stanine	5	~2	Range 1–9
Normal curve equivalent (NCE)	50	21.06	Equal-interval scale from 1 to 99; used in education
Percentile rank	—	—	Percentage of the norm group scoring **at or below** a score. **Ordinal**: percentiles are **not equal intervals** (the difference between the 50th and 55th percentile is much smaller in raw-score terms than between the 90th and 95th)

Score	Mean	SD	Notes
Grade equivalent	—	—	Easily misinterpreted. A 3rd grader with a grade equivalent of 6.2 in math did as well as a typical 6th grader would **on 3rd-grade material**; it doesn't mean the child can do 6th-grade math

Converting: z = (score − mean) ÷ SD. A WAIS score of 130 is (130 − 100) ÷ 15 = **z of +2**, which is about the **98th percentile**, and equals a **T-score of 70**.

z	T	Deviation IQ	Approx. percentile
−2	30	70	2nd
−1	40	85	16th
0	50	100	50th
+1	60	115	84th
+2	70	130	98th

Standard Error of Measurement (SEM)

No test is perfectly reliable, so every observed score has error.

- **SEM = SD × √(1 − reliability)**
- **Higher reliability → smaller SEM.**
- **Confidence bands:** the true score falls within **±1 SEM about 68%** of the time and within **±2 SEM about 95%** of the time.
- **Example:** an IQ score of 100 with an SEM of 3 gives a 95% band of about **94 to 106**.

Exam principle: report and interpret scores as **ranges**, not single points.

Test Construction Terms

Term	Meaning
Item difficulty (p)	Proportion of test-takers who answer correctly; **higher p = easier** item; .50 is often ideal for discrimination
Item discrimination	How well an item separates high and low scorers on the whole test
Floor effect	Test is too hard; many score at the bottom; can't distinguish among low scorers
Ceiling effect	Test is too easy; many score at the top
Norm group	The sample whose scores provide comparison; must be **representative**, **recent**, and **relevant**

Diagnostic Accuracy

Term	Meaning
Sensitivity	Ability to correctly identify people **who have** the condition (true positive rate)
Specificity	Ability to correctly identify people **who don't** have it (true negative rate)
False positive	Test says the condition is present when it isn't
False negative	Test says it's absent when it's present
Base rate	How common the condition is in the population; low base rates produce more false positives even with good tests
Incremental validity	Whether a test adds useful information beyond what's already known

Screening tools prioritize **sensitivity** (don't miss cases); positives are then confirmed with more thorough assessment.

Part 4 — Major Instruments

Intelligence and Achievement

Instrument	Notes
Wechsler Adult Intelligence Scale (WAIS)	Ages 16+; Full Scale IQ and index scores (e.g., verbal comprehension, working memory, processing speed); current edition is the WAIS-5
Wechsler Intelligence Scale for Children (WISC-V)	Ages 6–16
WPPSI-IV	Ages 2½–7
Stanford-Binet (SB5)	Ages 2–85+; descended from Binet's scale
Kaufman Assessment Battery for Children (KABC-II)	Designed to reduce cultural and language bias
Raven's Progressive Matrices	**Nonverbal** reasoning; useful when language is a barrier
Woodcock-Johnson	Separate cognitive and achievement batteries
WIAT-4, **WRAT5**	Achievement

Personality: Objective

Instrument	Owner	Notes
MMPI-3 (2020); **MMPI-2**; **MMPI-A-RF** (adolescents)	Hathaway & McKinley (original)	Most researched personality and psychopathology inventory; **validity scales** detect inconsistent, overreported, or underreported responding
Millon Clinical Multiaxial Inventory (MCMI-IV)	Theodore Millon	Personality disorders and clinical syndromes
Personality Assessment Inventory (PAI)	Leslie Morey	Clinical, treatment, and interpersonal scales

Instrument	Owner	Notes
NEO Personality Inventory (NEO-PI-3)	Costa & McCrae	**Big Five**: **O**penness, **C**onscientiousness, **E**xtraversion, **A**greeableness, **N**euroticism (**OCEAN**)
16PF	Raymond Cattell	16 personality factors
California Psychological Inventory (CPI)	Harrison Gough	Normal personality
MBTI	Myers & Briggs (based on Jung)	Type, not pathology (Chapter 14)

Personality: Projective

Instrument	Owner	Task
Rorschach Inkblot Test	Hermann Rorschach; scoring by **Exner's Comprehensive System** and now **R-PAS**	Describe what inkblots look like
Thematic Apperception Test (TAT)	Henry Murray & Christiana Morgan	Tell stories about ambiguous pictures; needs and press
Rotter Incomplete Sentences Blank	Julian Rotter	Complete sentence stems
House-Tree-Person (H-T-P)	John Buck	Drawings
Draw-A-Person	Goodenough; Machover	Drawings
Kinetic Family Drawing	Burns & Kaufman	Draw family members doing something

Projective tests have lower reliability and validity evidence than objective tests, and scoring requires advanced training.

Behavior Rating Scales

Instrument	Notes
Child Behavior Checklist (CBCL)	Thomas Achenbach (ASEBA system); parent, teacher, and youth forms
BASC-3	Behavior Assessment System for Children
Conners	ADHD-related behaviors

Symptom and Outcome Measures

Instrument	Notes
SCL-90-R / BSI	Leonard Derogatis; broad symptom checklists
OQ-45	Michael Lambert; Outcome Questionnaire for tracking progress
ORS / SRS	Miller & Duncan; brief session-by-session outcome and alliance measures (Chapter 18)
PHQ-9, GAD-7, BDI-II, BAI, PCL-5	Chapter 10

Neuropsychological Screening

Instrument	Notes
Bender Visual-Motor Gestalt Test (Bender-Gestalt II)	Lauretta Bender; copying designs; visual-motor integration
Halstead-Reitan and **Luria-Nebraska** batteries	Comprehensive neuropsychological assessment (specialist-administered)
MoCA, MMSE	Cognitive screening (Chapter 12)

Part 5 — Interpreting and Communicating Results (4.K)

Interpretation

- Interpret within the **context** of the client's history, culture, current state, and other data.

- Consider the **norm group's** fit and any **accommodations** given.
- Use **confidence bands**.
- **Integrate multiple sources**; no single score should drive a major decision.
- Watch for **validity scale** problems on instruments like the MMPI-3 before interpreting clinical scales.

Communicating Results (ACA E.1, E.3, E.6, E.9)

- Explain results in **language the client understands** (E.3.a).
- Explain the purpose, limits, and meaning of the results before and after testing.
- Invite questions and the client's reactions; check that the client understands.
- **Use results to support the client's own decisions** (4.K): "Your results show strong Social and Artistic interests. How does that fit with what you know about yourself?"
- Avoid labels that overstate certainty.
- Note any reservations about validity or reliability in reports (E.9.a).

Ethics and Security

ACA standard	Rule
E.2.a Limits of competence	Use only tests you're trained to use
E.4 Release of data to qualified personnel	Release raw data (responses, test protocols) only to persons qualified to interpret them, with client consent
E.5 Diagnosis	Cultural sensitivity; refraining from diagnosis if harmful (Chapter 19)
E.6.a Appropriateness of instruments	Consider validity, reliability, psychometric limits, and appropriateness

ACA standard	Rule
E.7 Conditions of administration	Standard conditions; note irregularities; technological administration validated
E.8 Multicultural issues	Use caution with instruments normed on populations other than the client's
E.10 Assessment security	Maintain integrity and security of tests; don't reproduce or modify without permission
E.11 Obsolete assessment and outdated results	Don't use outdated instruments or results for current decisions

Test security: never photocopy test booklets for clients or post items online. That damages the test's validity for everyone and violates copyright.

What the Exam Rewards / What It Punishes

Rewards: - Choosing instruments by purpose, reliability, validity, norms, and the counselor's competence - Testing in the client's primary language; using nonverbal measures when appropriate - Reporting scores as ranges using the SEM - Explaining results clearly and using them to support the client's own decisions - Checking validity scales before interpreting clinical scales

Punishes: - Using one score to make a major decision - Interpreting percentile differences as equal intervals - Taking grade equivalents literally - Using outdated tests or norms that don't fit the client - Giving raw test data to unqualified people - Using tests beyond one's training

Swap-Pair Drill

If you see…	It's…	Not…
Test of best possible performance	Maximal performance	Typical performance
Test of usual (typical) behavior	Typical performance	Maximal performance

If you see…	It's…	Not…
Test with easy items, strict time	Speed test	Power test
Test with harder items, generous time	Power test	Speed test
Test of what's been learned	Achievement	Aptitude
Test of potential to learn	Aptitude	Achievement
Percentage scoring at or below	Percentile rank	Percentage correct
Mean 50, SD 10	T-score	Normal curve equivalent (SD 21.06)
True score range around observed	SEM confidence band	Standard deviation
Test identifies those who **have** the condition	Sensitivity	Specificity
Test identifies those who **don't** have the condition	Specificity	Sensitivity
Test too hard; everyone scores low	Floor effect	Ceiling effect
OCEAN	NEO-PI (Costa & McCrae)	16PF
Stories about ambiguous pictures	TAT (Murray)	Rorschach
Parent/teacher behavior ratings	CBCL (Achenbach)	MMPI-A
Nonverbal reasoning	Raven's	WAIS verbal index
Employment tests must be job-related	*Griggs v. Duke Power*	*Larry P. v. Riles*
Test in the primary language	*Diana v. State Board of Education*	*Hobson v. Hansen*
Test must cover what was taught	*Debra P. v. Turlington*	*Griggs*

Quick Checks

1. A score of 115 on the WAIS is about how many standard deviations above the mean? A. 0.5 B. 1 C. 1.5 D. 2

2. A T-score of 70 corresponds to a z-score of: A. +1 B. +2 C. +3 D. −2

3. A test has an SEM of 4. A client scores 90. The 95% confidence band is about: A. 86–94 B. 82–98 C. 88–92 D. 70–110

4. Which change would reduce a test's SEM? A. Lower reliability B. Higher reliability C. A smaller norm group D. Shorter time limits

5. A 4th grader's reading grade equivalent is 7.0. The best interpretation is: A. She can read 7th-grade books B. She should skip to 7th grade C. She scored as a typical 7th grader would on 4th-grade material D. The test is invalid

6. A recent immigrant from Haiti who speaks primarily Haitian Creole needs a cognitive assessment. The counselor's best choice is to: A. Use the English WAIS as usual B. Use a nonverbal measure and/or assessment in his primary language, interpreting with caution about norms C. Skip assessment entirely D. Use his 10-year-old as an interpreter

7. A screening tool for suicide risk should prioritize: A. Specificity B. Sensitivity C. Ceiling effects D. Low reliability

8. On the MMPI-3, before interpreting clinical scales, the counselor should check: A. The T-score of the first clinical scale B. The validity scales C. The client's IQ D. The client's Holland code

9. A counselor with only a master's-level testing course wants to administer and interpret the Rorschach. The best guidance is: A. Proceed, since it's a common test B. Don't, unless the counselor has the advanced specialized training the instrument requires C. Proceed with the cli-

ent's consent D. Use an online scoring program to compensate

10. A client asks for a copy of the MMPI-3 items to study. The counselor should: A. Provide a copy B. Explain that test materials must be kept secure, while offering to discuss the results C. Post the items online for other clients D. Let the client take the test home

11. Which court case held that IQ tests could not be used to place Black children in classes for students with intellectual disabilities in California because of cultural bias? A. *Griggs v. Duke Power* B. *Larry P. v. Riles* C. *Diana v. State Board of Education* D. *Debra P. v. Turlington*

12. A career client's interest inventory shows strong Investigative interests. The best way to use the result is to: A. Tell the client to become a scientist B. Explore with the client how the results fit his experience and goals, and use them to guide further exploration C. Ignore the result D. Retest until a different result appears

Answers and Rationales

1 — B. (115 − 100) ÷ 15 = 1 SD. A, C, D: incorrect calculations.

2 — B. T = 50 + 10z, so 70 is z = +2. A: z = +1 is T = 60. C: z = +3 is T = 80. D: z = −2 is T = 30.

3 — B. ±2 SEM = ±8, so 82–98. A: ±1 SEM is the 68% band. C: too narrow. D: too wide.

4 — B. SEM = SD × √(1 − r); higher reliability shrinks it. A: lower reliability increases SEM. C and D: don't directly lower SEM.

5 — C. Grade equivalents compare performance on the test's own material. A and B: common misinterpretations. D: no evidence of invalidity.

6 — B. Primary-language and nonverbal assessment, with cautious interpretation (*Diana*; ACA E.8). A: English testing would measure language, not ability. C: assessment can still help. D: a child shouldn't interpret (Chapter 9).

7 — B. Screening prioritizes not missing cases (sensitivity). A: specificity matters for confirming. C and D: undesirable properties.

8 — B. Validity scales detect inconsistent, overreported, or underreported responding. A, C, D: not the first check.

9 — B. Use only instruments within your competence (ACA E.2.a); projective scoring requires advanced training. A and C: competence isn't granted by popularity or consent. D: software doesn't replace training.

10 — B. Test security (ACA E.10). A, C, D: compromise test integrity.

11 — B. *Larry P. v. Riles*. A: *Griggs* concerns employment testing. C: *Diana* concerns language of testing. D: *Debra P.* concerns curricular validity.

12 — B. Results support the client's own decision-making (4.K). A: test-and-tell. C: discards useful information. D: inappropriate.

Audio Companion

Track	Covers	Target length
20.1	Score It: test types, scores, SEM, instruments, ethics	~5 min

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Chapter 21 — Treatment Planning, Referral, and Termination

Domain map: 4.A Collaborate with client to establish treatment goals and objectives · 4.B Establish short- and long-term counseling goals consistent with clients' diagnoses · 4.C Identify barriers affecting client goal attainment · 4.D Identify strengths that improve the likelihood of goal attainment · 4.E Refer to different levels of treatment · 4.F Refer to others for concurrent treatment · 4.G Guide treatment planning · 4.H Discuss termination process and issues · 4.I Discuss transitions in group membership · 4.J Follow-up after discharge · 4.K Use assessment instrument results to facilitate client decision making · 4.L Review and revise the treatment plan · 4.M Engage clients in review of progress toward treatment goals · 4.N Collaborate with other providers and client support systems (documentation and report writing) · 4.O Discuss with clients the integration and maintenance of therapeutic progress · 4.P Educate client to the value of treatment plan compliance · CACREP: Counseling and Helping Relationships

The Plan Connects Everything

Treatment Planning is 14 of the 160 scored items, but the plan is where every other domain lands. Intake findings (Chapter 19) become problem statements. Theory and interventions (Chapters 2, 5) become methods. Assessment results (Chapter 20) become baselines and outcome measures. Ethics (Chapter 17) governs consent, referral, and termination.

The exam's core principle: the plan is **collaborative**, **measurable**, **sequenced by priority**, and **revised** as counseling unfolds.

Part 1 — Building the Plan (4.A, 4.B, 4.G)

Components of a Treatment Plan

Component	Description	Example
Problem statement	The concern, stated in **behavioral** terms, often linked to the diagnosis	"Client reports depressed mood most days, sleeping 4 hours a night, and missing work 2 days a week"
Long-term goal	The broad outcome	"Client will experience reduced depression and return to full functioning"
Short-term objectives	**Measurable** steps toward the goal	"Client will score below 10 on the PHQ-9 within 12 weeks"
Interventions	What the counselor will do, tied to theory and evidence	"Behavioral activation and cognitive restructuring in weekly sessions"
Responsible parties	Who does what (client, counselor, other providers)	"Client will complete activity log daily"
Target dates and review dates	When objectives should be met and when the plan will be reviewed	"Review at session 6 and session 12"
Strengths and resources	What the client brings	"Supportive sister; previous recovery from depression"
Barriers	What may get in the way	"Night-shift work schedule; transportation"

SMART Objectives

Letter	Meaning	Weak	Strong
S	Specific	"Feel better"	"Attend one social activity a week"
M	Measurable	"Be less anxious"	"Reduce GAD-7 score from 16 to below 10"
A	Achievable	"Never feel anxious again"	"Use a coping skill in 4 of 5 anxious moments, by self-report"
R	Relevant	Unrelated to the client's concern	Tied to the client's stated goal
T	Time-bound	"Eventually"	"Within 8 weeks"

Goals in the client's words: de Shazer's solution-focused criteria for **well-formed goals**: small, concrete, behavioral, stated as the **presence** of something (not the absence), important to the client, and within the client's control.

- Weak: "Stop fighting with my son."
- Strong: "Have three calm conversations with my son this week about his day."

Collaboration (4.A)

- The client helps choose goals. Goals the counselor sets alone are less likely to be met, and they conflict with **autonomy**.
- Explore the client's **readiness** (stages of change; Chapter 5). A precontemplative client's first goal may be exploring whether change is wanted.
- Respect **cultural** values about goals, family involvement, and help-seeking.
- With minors, involve parents while respecting the young client's voice.

Sequencing Priorities

When a client has several problems, order them:

1. **Safety** (suicide, violence, abuse, medical danger)
2. **Basic needs and stabilization** (housing, food, acute substance withdrawal, severe symptoms)
3. **Problems that block other work** (e.g., substance use that undermines therapy)
4. **The client's most pressing concerns**
5. **Longer-term growth and prevention**

This mirrors Maslow and the crisis principle from Chapter 6.

Part 2 — Strengths and Barriers (4.C, 4.D)

Strengths (4.D)	Barriers (4.C)
Support system	Lack of transportation, childcare, or money
Past successes and coping skills	Work schedules
Motivation and readiness	Low motivation; ambivalence
Faith and community	Stigma; cultural mistrust
Education, intelligence, humor	Limited literacy or language access
Health	Co-occurring medical or mental health conditions
Insight	Substance use
	Unsafe living situation; family resistance

Strengths-based practice builds the plan on what's already working (SFBT, Chapter 2; the strengths perspective in social work, Dennis Saleebey). Barriers are named early so the plan can address them, for example with telehealth for transportation problems or a sliding scale for cost.

Part 3 — Referral (4.E, 4.F)

Referring to a Different Level of Care (4.E)

Use the level-of-care options from Chapter 19 (outpatient, IOP, PHP, residential, inpatient), choosing the **least restrictive** level that's safe and effective. **Step up** when symptoms worsen or outpatient care isn't enough; **step down** as the client stabilizes.

Referring for Concurrent Treatment (4.F)

Clients often need more than one provider at the same time:

Need	Refer to
Medication evaluation	Psychiatrist, psychiatric nurse practitioner, or primary care provider
Medical evaluation	Primary care or specialist (Chapter 16)
Substance use treatment	Specialized program (Chapter 9)
Eating disorder	Medical provider and dietitian (Chapter 16)
Specialized trauma, sex, or couples therapy	Specialist with the needed training
Legal, financial, housing	Appropriate community resources
Case management	Social services

How to refer well: - Explain **why**, and involve the client in the decision. - Offer **several options** where possible. - Get a **signed release of information** before sharing information. - **Follow up** to confirm the client connected. - Avoid abandonment: when appropriate, continue care until the transition is complete (Chapter 17).

Part 4 — Collaboration and Documentation (4.N)

Working with Other Providers and Supports

- With a **release of information**, coordinate with physicians, psychiatrists, schools, probation officers, and family.
- Share only what the purpose requires (ACA B.2.e, minimal disclosure; Chapter 17). HIPAA's "minimum necessary" rule doesn't apply to disclosures for treatment or disclosures the client has authorized, but the ethical standard of sharing only what's needed still applies.
- **Integrated care** teams (behavioral health in primary care) improve outcomes for many clients.
- Involve **natural supports** (family, faith community, friends) when the client wants that.

Progress Note Formats

Format	Sections
SOAP	**S**ubjective (client's report), **O**bjective (counselor's observations, test scores), **A**ssessment (counselor's clinical impression and progress), **P**lan (next steps)
DAP	**D**ata (subjective and objective), **A**ssessment, **P**lan
BIRP	**B**ehavior (client's presentation), **I**ntervention (what the counselor did), **R**esponse (how the client responded), **P**lan
GIRP	**G**oal, **I**ntervention, **R**esponse, **P**lan

Exam favorite: in SOAP, the client's own words go in **S**; what the counselor observes or measures (affect, MSE findings, PHQ-9 score) goes in **O**.

Reports

Write reports that are clear, relevant to the reader's purpose, free of jargon, and limited to what's necessary. Note limitations of the data and any reservations about test validity (ACA E.9.a). Distinguish facts from clinical opinions.

Part 5 — Monitoring and Revising the Plan (4.K, 4.L, 4.M, 4.P)

Measuring Progress

- Use **baseline** and repeated measures (PHQ-9, GAD-7, OQ-45, ORS/SRS; Chapters 18, 20).
- **Review progress with the client** (4.M) at planned intervals: "Here's where your scores were when we started, and here's where they are now. What do you notice?"
- Use **assessment results** to help the client make decisions (4.K), such as whether to add group therapy or step down to biweekly sessions.

When Progress Stalls (4.L)

Don't keep doing the same thing, and don't switch approaches without discussion. Instead:

1. **Review the plan with the client.**
2. **Check the alliance** (goals, tasks, bond; Chapter 4). A stalled plan is often an alliance problem.
3. **Reassess**: missed diagnosis, substance use, medical issue, trauma, safety concern, new stressor?
4. **Check readiness**: has the client's stage of change shifted?
5. **Consider adjustments**: different interventions, frequency, modality, level of care, or referral for concurrent treatment.
6. **Consult** or seek supervision.

Adherence (4.P)

What the outline calls “treatment plan compliance” is increasingly termed **adherence**, which emphasizes collaboration over obedience.

- Explain the **rationale** for homework, attendance, and medication.
- Explore **barriers** to adherence without judgment.
- Use **motivational interviewing** when ambivalence is the barrier (Chapter 5).
- Respect the client’s right to refuse treatment, and explain the possible consequences (ACA A.2.b).

Part 6 — Termination (4.H, 4.O, 4.J)

Types of Termination

Type	Description
Planned / natural	Goals are met; termination is discussed in advance
Client-initiated	The client decides to end, sometimes abruptly
Counselor-initiated	The counselor ends services (e.g., client isn’t benefiting, counselor leaving, competence limits, safety threat)
Premature (dropout)	The client stops before goals are met; meta-analytic estimates put dropout around **one in five** clients (Swift & Greenberg, 2012)
Forced	External limits: insurance, agency policy, a move, end of a training placement

The Termination Process

Raise termination early. Introduce the idea in informed consent and revisit it as goals are approached, not in the last session.

Tasks in the final phase: 1. **Review progress** and credit the client's work. 2. **Process feelings** about ending: pride, sadness, anxiety, sometimes anger. Endings can stir up past losses. 3. **Consolidate and generalize** learning (4.O): what helped, and how to keep using it. 4. **Relapse prevention**: warning signs, coping plan, support system, what to do if problems return. 5. **Plan for the future**: referrals, support groups, booster sessions. 6. **Discuss the open door**: how the client can return if needed.

Tapering sessions (weekly to biweekly to monthly) can ease the transition.

Follow-Up After Discharge (4.J)

- Follow-up contacts or **booster sessions** support maintenance.
- After hospital or higher-level discharge, the **first days and weeks** are a period of elevated suicide risk. A prompt follow-up appointment, an updated safety plan (Chapter 6), and caring contacts are important.
- A **discharge summary** documents the reason for services, the treatment provided, progress, the status at discharge, and recommendations.

Group Membership Transitions (4.I)

See Chapter 8: prepare new members and the group, encourage departing members to say goodbye, and process endings with the whole group.

Ethical Reminders

- **Pretermination counseling** and referrals (ACA A.11.c).
- **No abandonment** (A.12).
- Termination for nonpayment only after the fee policy was explained and with referrals; never in the middle of a crisis.
- When a counselor is in danger from a client, termination is permitted (A.11.c), with appropriate safety steps.

What the Exam Rewards / What It Punishes

Rewards: - Collaborative goals in measurable, behavioral terms - Safety first in sequencing - Naming strengths and barriers early - Referring with releases, options, and follow-up - Reviewing progress with the client using data - Revisiting the alliance and reassessing when progress stalls - Introducing termination early; relapse prevention; open door - Prompt follow-up after discharge from a higher level of care

Punishes: - Counselor-written goals the client didn't help choose - Vague goals ("feel better") - Treating long-term growth before safety and stabilization - Continuing an ineffective plan unchanged - Switching approaches without discussing it with the client - Raising termination for the first time in the last session - Ending services without referrals

Swap-Pair Drill

If you see…	It's…	Not…
Treatment plan: broad outcome	Long-term goal	Short-term objective
Treatment plan: measurable step	Short-term objective	Long-term goal
Small, concrete, presence of something	Well-formed goal (de Shazer)	SMART
Client's own words in the note	S in SOAP	O
Counselor-observed affect; test score	O in SOAP	S
Behavior, intervention, response, plan	BIRP	DAP
Treatment progress stalls	Review plan, check alliance, reassess	Repeat the same intervention

If you see…	It's…	Not…
Client's collaborative follow-through with treatment	Adherence	Compliance (older term)
Termination: raise the ending early	Planned termination	Last-session announcement
Reducing frequency before ending	Tapering	Abrupt termination
High-risk period after hospital discharge	Prompt follow-up	Discharge without plan
Strengths perspective	Saleebey	Maslow

Quick Checks

1. Which is the best short-term objective? A. "Client will be happier." B. "Client will reduce panic attacks from 5 a week to 1 or fewer within 8 weeks, by self-report log." C. "Client will understand his anxiety." D. "Client will never panic again."

2. A client's goal is "stop yelling at my kids." Rewritten as a well-formed goal, it would be: A. "Never yell again." B. "Use a calm voice and take a 5-minute break when frustrated with the kids, at least 4 times this week." C. "Be a better parent." D. "Understand why I yell."

3. A client presents with suicidal ideation with a plan, alcohol use, marital conflict, and career dissatisfaction. The first treatment priority is: A. Career exploration B. Marital counseling C. Safety D. Alcohol education

4. In SOAP notes, the client's statement "I haven't slept more than four hours all week" belongs in: A. Subjective B. Objective C. Assessment D. Plan

5. After 10 sessions, a client's PHQ-9 scores haven't changed. The counselor's best next step is to: A. Continue the same plan B. Terminate counseling C. Review progress with the client, check the alliance, and reassess D. Switch to a new theory without telling the client

6. A counselor refers a client to a psychiatrist for a medication evaluation. Before sharing clinical information with the psychiatrist, the counselor needs: A. Nothing; providers can always share B. A signed release of information from the client C. Permission from the counselor's supervisor D. A court order

7. When should termination first be discussed? A. In the last session B. Only when the client brings it up C. Early, beginning with informed consent, and revisited as goals approach D. After the client stops attending

8. A client who just completed a 5-day inpatient stay for a suicide attempt is being discharged to outpatient counseling. The most important step is: A. Scheduling the first outpatient appointment within a month B. Arranging a prompt follow-up appointment and safety planning, since risk is elevated after discharge C. Waiting for the client to call D. Closing the inpatient record

9. The outline phrase "treatment plan compliance" is increasingly framed as adherence because it: A. Emphasizes obedience B. Emphasizes collaboration and the client's active role C. Requires a court order D. Applies only to medication

10. During termination, helping a client identify early warning signs and a plan for coping if symptoms return is called: A. Relapse prevention B. Tapering C. Pretermination counseling D. Step-down care

11. A client with severe depression isn't improving in weekly outpatient sessions and is struggling to function, but has no imminent risk. An appropriate option is: A. Inpatient hospitalization B. Stepping up to intensive outpatient or partial hospitalization, discussed with the client C. Terminating counseling D. Reducing sessions to monthly

12. Which is the best example of identifying a strength in a treatment plan? A. "Client has poor coping skills." B. "Client successfully stopped smoking 3 years ago and has

a supportive sister." C. "Client is resistant." D. "Client misses appointments."

Answers and Rationales

1 — B. Specific, measurable, achievable, relevant, and time-bound. A and C: vague. D: unachievable.

2 — B. Behavioral, concrete, stated as the presence of something, and within the client's control (de Shazer). A: absence-based and unrealistic. C: vague. D: insight, not behavior.

3 — C. Safety first. A, B, D: come after safety is addressed.

4 — A. The client's report goes in Subjective. B: Objective holds counselor observations and measurements. C: Assessment is the counselor's clinical impression. D: Plan is next steps.

5 — C. Review, check the alliance, reassess, then adjust (4.L). A: repeats what isn't working. B: abandons a client who still needs help. D: changes without collaboration.

6 — B. A signed release is required (ACA B.6.g). HIPAA itself permits disclosures for treatment, but the ethics code and many state laws require the client's written consent. A: incorrect. C and D: not what's required.

7 — C. Termination is part of informed consent and is revisited as goals approach. A, B, D: too late.

8 — B. Risk is elevated after discharge; prompt follow-up and safety planning matter (4.J; Chapter 6). A and C: dangerous delays. D: administrative, not clinical.

9 — B. Adherence emphasizes collaboration. A: the opposite. C and D: incorrect.

10 — A. Planning for warning signs and coping is relapse prevention (4.O). B: tapering reduces frequency. C: pre-termination counseling is the broader ethical process. D: step-down is a level-of-care change.

11 — B. Step up to the least restrictive appropriate higher level, collaboratively (4.E). A: no imminent risk. C and D: reduce care when more is needed.

12 — B. A specific, usable strength (4.D). A, C, D: deficits or labels.

Audio Companion

Track	Covers	Target length
21.1	Plan It: goals, priorities, referral, notes, revision, termination	~4 min

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Chapter 22 — The Counselor as Instrument: Core Counseling Attributes

Domain map: 6.A Awareness of self and impact on clients · 6.B Genuineness · 6.C Congruence · 6.D Demonstrate knowledge of and sensitivity to gender orientation and gender issues · 6.E Demonstrate knowledge of and sensitivity to multicultural issues · 6.F Demonstrate conflict tolerance and resolution · 6.G Empathic attunement · 6.H Empathic responding · 6.I Foster the emergence of group therapeutic factors · 6.J Non-judgmental stance · 6.K Positive regard · 6.L Respect and acceptance for diversity · 6.M Use foundational listening, attending, and reflecting skills · CACREP: Counseling and Helping Relationships; Professional Counseling Orientation

The Smallest Domain, the Biggest Influence

Core Counseling Attributes is the smallest domain on the NCE: 13 of the 160 scored items. But it describes the ingredient that runs through every other domain: **the counselor's own presence.** Research on what makes counseling work keeps pointing to the relationship and the counselor's qualities as strongly as to specific techniques (Chapter 18, common factors).

On the exam, these items often look like skills items (Chapter 4), but the right answer turns on an attribute: the response that is most **genuine**, most **empathic**, most **accepting**, or most **self-aware**.

Part 1 — Who Owns the Attributes

Owner	Contribution
Carl Rogers (1957)	**Six necessary and sufficient conditions** for therapeutic change (below)
Truax & Carkhuff	Operationalized Rogers' conditions as **accurate empathy, nonpossessive warmth, and genuineness**; Carkhuff's five-level empathy scale (Chapter 4)
Heinz Kohut	Self psychology; empathy as "**vicarious introspection**"
Norcross & Lambert	APA Task Forces on **evidence-based therapy relationships**: elements judged **demonstrably effective** include the **alliance**, **empathy**, **positive regard and affirmation**, and **collecting client feedback**; **congruence/genuineness**, **repairing alliance ruptures**, and **managing countertransference** were also found effective or probably effective
Jeffrey Hayes, Charles Gelso	Managing countertransference: **self-insight, self-integration, anxiety management, empathy, conceptualizing skills**
Derald Wing Sue; Hook et al.	Multicultural competence and cultural humility (Chapter 9)
Gerald Corey	Personal characteristics of effective counselors

Rogers' Six Necessary and Sufficient Conditions (1957)

1. Two people are in **psychological contact**.
2. The **client** is in a state of **incongruence** (vulnerable or anxious).
3. The **counselor** is **congruent** (integrated) in the relationship.

4. The counselor experiences **unconditional positive regard** for the client.
5. The counselor experiences **empathic understanding** of the client's internal frame of reference and tries to communicate it.
6. The client **perceives**, at least to a minimal degree, the counselor's empathy and unconditional positive regard.

Exam point: condition 6 means it isn't enough for the counselor to feel empathy and acceptance; **the client has to perceive them.**

Part 2 — The Attributes, One by One

Self-Awareness and Impact on Clients (6.A)

Counselors bring their own history, values, biases, emotions, and blind spots into every session.

- **Countertransference**: the counselor's emotional reactions to the client, often from the counselor's own history (Chapter 5). Managing it is the counselor's responsibility, through self-reflection, **supervision**, **consultation**, and **personal counseling**.
- **The Johari Window** (Luft & Ingham; Chapter 3) shows how feedback from supervisors and colleagues reveals the counselor's **blind** area.
- **Power and privilege**: the counselor's role and identity carry power in the relationship. Awareness of that power is part of ethical practice (MSJCC; Chapter 9).
- **Self-care**: an exhausted counselor's presence suffers (Chapter 14, Chapter 17).

Exam pattern: when a counselor notices a strong personal reaction to a client, the scored answer is **reflect and seek supervision**, not disclose it to the client, act on it, or immediately refer the client out.

Genuineness and Congruence (6.B, 6.C)

Term	Meaning
Genuineness	Being real and authentic, not hiding behind a professional role or façade
Congruence	The counselor's **inner experience matches** what the counselor outwardly communicates

The terms are close; Rogers often used them together. **Congruence doesn't mean saying everything you feel.** It means not pretending to feel something you don't, and sharing reactions when doing so serves the client (immediacy; Chapter 4).

Incongruent: a counselor who is bored says, "This is so interesting!" **Congruent:** a counselor notices drifting attention, refocuses, and, if useful, uses immediacy: "I notice we keep circling the same story. I wonder what we might be avoiding."

Empathic Attunement vs. Empathic Responding (6.G, 6.H)

Attribute	What it is
Empathic attunement	The **internal** process of sensing and resonating with the client's experience, moment to moment, including what's unspoken
Empathic responding	**Communicating** that understanding back to the client, through reflection, tone, and nonverbal behavior

Attunement without responding leaves the client unaware they've been understood (Rogers' condition 6). Responding without attunement sounds formulaic.

Empathy vs. sympathy:

Empathy	Sympathy
Understanding the client's experience **from the client's frame of reference**	Feeling sorry **for** the client from one's own frame of reference
"It sounds like you feel completely alone with this."	"Oh, you poor thing. I feel so bad for you."

Empathy is the scored answer. Sympathy, reassurance, and identification ("I know exactly how you feel") are distractors.

Positive Regard and the Nonjudgmental Stance (6.J, 6.K)

- **Unconditional positive regard (UPR):** valuing the client as a person, **without conditions**. It doesn't mean approving of everything the client does.
- **Nonjudgmental stance:** accepting the client's experience and choices without moral evaluation, while still being able to address harmful behavior.
- **Warmth** (Truax & Carkhuff's "nonpossessive warmth"): caring that doesn't create dependence.

Exam pattern: a client discloses something the counselor finds distasteful (an affair, a past crime, a choice that conflicts with the counselor's values). The scored response accepts the **person**, explores the client's experience, and, where relevant, addresses consequences or safety, without lecturing or showing disapproval.

Conflict Tolerance and Resolution (6.F)

Effective counselors can **stay present with conflict**: a client's anger at the counselor, disagreement, criticism, or tension in a couple or group.

- **Don't avoid, appease, or retaliate.**
- **Name the conflict** non-defensively and explore it.
- **Repair alliance ruptures** (Safran & Muran; Chapter 4): a well-handled rupture often strengthens the relationship.

- In couples, families, and groups, **model** respectful conflict (Chapter 7, Chapter 8).

Exam pattern: “You’re not helping me at all.” The scored response: “It sounds like you’re frustrated with how counseling is going. I’d really like to understand what’s not working for you.” Wrong answers defend credentials, offer reassurance, or refer the client away.

Gender, Multicultural Sensitivity, and Respect for Diversity (6.D, 6.E, 6.L)

Covered in depth in Chapters 9 and 15. The attribute side: - **Cultural humility**: curiosity, openness, and awareness of the limits of one’s knowledge. - **Awareness of one’s own gender-role and cultural assumptions**, and their effect on the client. - **Using the client’s name, pronouns, and language** for their identity. - **Broaching** culture respectfully. - **Not imposing values** (ACA A.4.b).

Fostering Group Therapeutic Factors (6.I)

The leader’s attributes shape the group: **modeling** genuineness and feedback, building **cohesiveness** (Yalom’s group equivalent of the alliance), **instilling hope**, and protecting members (Chapter 8).

Foundational Listening, Attending, and Reflecting (6.M)

The microskills of Chapter 4 express the attributes: **attending** shows presence, **reflecting feelings** shows empathy, **paraphrasing and summarizing** show understanding, and **comfortable silence** shows acceptance.

Part 3 — Personal Characteristics of Effective Counselors

Drawing on Corey and others, effective counselors typically: - Have an identity of their own and know what they value - Respect and appreciate themselves, and can give and receive help - Are open to change and to feedback -

Make choices that are life-oriented - Are authentic, sincere, and honest - Have a sense of humor - Make mistakes and are willing to admit them - Live in the present - Appreciate the influence of culture - Have a sincere interest in others' welfare - Become deeply involved in their work and derive meaning from it, while keeping healthy boundaries - Maintain healthy relationships and self-care

Personal counseling for counselors is widely encouraged (though not universally required) as a way to increase self-awareness and manage countertransference.

Part 4 — How Attribute Items Look on the Exam

Most items give four plausible responses. Ask: **which one best shows the attribute?**

If the response…	It shows…	Watch for distractors that…
Reflects the client's feeling from the client's view	Empathy	Sympathize, reassure, or say "I know how you feel"
Matches the counselor's real reaction, used for the client's benefit	Congruence	Fake enthusiasm, or vent the counselor's feelings
Accepts the person while exploring the behavior	Positive regard, nonjudgment	Lecture, moralize, or show disgust
Explores the client's anger non-defensively	Conflict tolerance	Defend, apologize excessively, or end the session
Seeks supervision about a strong reaction	Self-awareness	Disclose it to the client, or refer out immediately
Invites the client's cultural meaning	Cultural humility	Assume, stereotype, or ignore culture

What the Exam Rewards / What It Punishes

Rewards: - Empathy communicated so the client can perceive it - Congruence that serves the client - Acceptance of the person, even when addressing harmful behavior - Staying present and non-defensive in conflict; repairing ruptures - Supervision, consultation, and personal counseling for countertransference - Cultural humility

Punishes: - Sympathy or reassurance in place of empathy - “I know exactly how you feel” - Pretending to feel something (incongruence) - Judgment, lecturing, or visible disapproval - Defensiveness when the client is angry with the counselor - Acting on or disclosing countertransference without reflection

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Swap-Pair Drill

If you see…	It’s…	Not…
Inner experience matches outward expression	Congruence	Positive regard
Being real, no façade	Genuineness	Empathy
Sensing the client’s experience internally	Empathic attunement	Empathic responding
Communicating understanding	Empathic responding	Empathic attunement
Feeling with, from the client’s frame	Empathy	Sympathy
Feeling sorry for the client	Sympathy	Empathy
Valuing the person without conditions	Unconditional positive regard	Approval of all behavior
Six necessary and sufficient conditions	Rogers (1957)	Truax & Carkhuff
Accurate empathy, nonpossessive warmth, genuineness	Truax & Carkhuff	Rogers’ six conditions
Vicarious introspection	Kohut	Rogers

If you see…	It's…	Not…
Demonstrably effective relationship elements	Norcross & Lambert	Wampold
Self-insight, self-integration, anxiety management	Managing countertransference (Hayes & Gelso)	Burnout prevention
Client must perceive empathy and UPR	Rogers' sixth condition	Rogers' first condition

Quick Checks

1. A client says, "My mother died last month, and I can't stop crying at work." The most empathic response is: A. "I'm so sorry. That's terrible." B. "I know exactly how you feel; I lost my mother too." C. "Losing your mother so recently, and then having the grief come up at work, sounds overwhelming." D. "Time heals all wounds."

2. A counselor feels bored as a client repeats the same story but says, "This is fascinating." This counselor is being: A. Congruent B. Incongruent C. Empathic D. Nonjudgmental

3. According to Rogers, which condition requires that the client perceive the counselor's empathy and regard? A. The first B. The third C. The fifth D. The sixth

4. A client discloses that he cheated on his wife and feels terrible. The best response reflecting positive regard is: A. "Cheating is wrong; you need to tell her." B. "It sounds like you're carrying a lot of guilt about what happened. Tell me more about what you're feeling." C. "Everyone makes mistakes; don't worry about it." D. Saying nothing and changing the subject

5. A client yells, "You're useless! I've wasted months here!" The best response shows: A. Defending the counselor's credentials B. Conflict tolerance: acknowledging the frustration and exploring what isn't working C. Ending the session D. Agreeing that counseling is useless

6. A counselor notices strong irritation toward a client who reminds her of her father. The best action is to: A. Tell the client about the resemblance B. Seek supervision and reflect on the countertransference C. Refer the client immediately D. Ignore the feeling

7. "Empathic attunement" differs from "empathic responding" in that attunement is: A. Verbal B. The internal process of sensing the client's experience C. Always shared with the client D. A group therapeutic factor

8. Which set of conditions did Truax and Carkhuff identify as core to effective counseling? A. Interpretation and insight B. Accurate empathy, nonpossessive warmth, and genuineness C. Confrontation and directives D. Free association and transference

9. A counselor working with a client from a different cultural background asks, "How do you and your family understand what's happening?" This reflects: A. Cultural encapsulation B. Cultural humility C. Countertransference D. Sympathy

10. Which relationship element did the Norcross and Lambert task forces judge demonstrably effective? A. Counselor self-disclosure as a primary intervention B. Empathy C. Advice-giving D. Interpretation

11. Unconditional positive regard means the counselor: A. Approves of everything the client does B. Values the client as a person regardless of the client's behavior C. Avoids discussing harmful behavior D. Agrees with the client's views

12. A counselor says, "I feel so sorry for you. You poor thing." This is an example of: A. Empathy B. Sympathy C. Immediacy D. Congruence

Answers and Rationales

1 — C. Reflects the client's experience from the client's frame of reference (empathic responding). A: sympathy. B: identification that shifts focus to the counselor. D: a cliché that dismisses the feeling.

2 — B. Saying the opposite of what one feels is incongruent. A: congruence would match inner and outer. C: no empathy is shown. D: the issue is genuineness, not judgment.

3 — D. The sixth condition: the client perceives empathy and UPR. A: psychological contact. B: counselor congruence. C: the counselor's empathic understanding.

4 — B. Accepts the person and explores his experience. A: judgment and directive. C: minimizing reassurance. D: avoidance.

5 — B. Non-defensive exploration of the conflict (6.F; rupture repair). A: defensive. C: avoids. D: unhelpful and untrue.

6 — B. Countertransference is managed through reflection and supervision (6.A). A: burdens the client. C: premature. D: unmanaged countertransference can harm.

7 — B. Attunement is internal sensing; responding communicates it. A and C: describe responding. D: unrelated.

8 — B. Truax and Carkhuff's conditions. A: psychodynamic aims. C: directive techniques. D: psychoanalytic concepts.

9 — B. Curiosity about the client's cultural meaning reflects cultural humility. A: encapsulation imposes the counselor's assumptions. C: countertransference is the counselor's emotional reaction. D: sympathy is feeling sorry for.

10 — B. Empathy was judged demonstrably effective. A, C, D: not among the demonstrably effective relationship elements.

11 — B. UPR values the person, not every behavior. A, C, D: misunderstandings of UPR.

12 — B. Feeling sorry for someone from one's own frame is sympathy. A: empathy understands from the client's frame. C: immediacy addresses the present relationship. D: congruence is about genuineness.

Audio Companion

Track	Covers	Target length
22.1	Be the Instrument: Rogers' conditions, empathy, congruence, regard, conflict, self-awareness	~4 min

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PART THREE

Practice

NCE Practice Exam 1

200 items · 3 hours 45 minutes · Answer every item.

Take this exam under timed conditions before checking the key. Use the four-step routine on every item: read the last line first, name the owner, predict, then eliminate by owner.

Pacing checkpoints: item 50 by 0:50 · item 100 by 1:40 · item 150 by 2:30 · item 200 by 3:20.

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1. One partner says, “He shuts down because I criticize.” The other says, “She criticizes because I shut down.” Explaining that each behavior triggers the other illustrates:

A. Equifinality
B. Circular causality
C. Linear causality
D. A double bind

2. A woman repeatedly visits emergency rooms with symptoms she secretly causes by misusing medication. She gains no money, time off, or legal advantage and seems to want to be cared for as a patient. This most suggests:

A. Somatic symptom disorder
B. Factitious disorder imposed on self
C. Malingering
D. Factitious disorder imposed on another

3. A 9-year-old boy refuses to consider nursing because “that’s a girl job,” without thinking about whether he would enjoy it. Which concept best explains this?

A. The growth stage
B. Circumscription
C. Consistency
D. The fantasy period

4. A client with severe depression needs structured treatment most of each weekday. He has no suicidal intent, is medically stable, and has a supportive home to return to each evening. Which level of care fits best?

A. Residential treatment
B. Inpatient hospitalization
C. Weekly outpatient counseling
D. Partial hospitalization

5. A client who worked hard in counseling invites the counselor to her college graduation. The counselor's most appropriate approach is to:

A. Attend only if she agrees to end counseling first
B. Attend and bring a gift, with no need to document
C. Weigh risks and benefits and document the rationale
D. Decline, since any outside contact is a violation

6. A counselor sees that a colleague's website features a glowing testimonial from one of the colleague's current clients. No client appears to have been harmed. The counselor's best first step is to:

A. Contact the client to ask her to remove it
B. Ignore it, since advertising isn't a clinical issue
C. File a complaint with the state licensing board
D. Raise the concern with the colleague informally

7. A client in an intensive outpatient program now has fewer symptoms, is back at work, and is using her coping skills consistently. The most appropriate level-of-care decision is to:

A. Continue IOP until all symptoms resolve
B. Step down to standard outpatient
C. Step up to partial hospitalization
D. Discharge with no further services

8. A counselor notices that a client from a culture where direct eye contact with authority figures is disrespectful keeps her gaze lowered. The best response is to:

A. Encourage her to make more eye contact
B. Point out the gap between her words and her gaze
C. Interpret the lowered gaze as a sign of shame
D. Adjust eye contact and posture to her comfort

9. During outpatient counseling for depression, a client discloses daily purging and recent weight loss. Along with continuing counseling, the counselor should refer her to:

A. A medical provider and a dietitian
B. A case manager for housing resources
C. A substance use treatment program
D. A couples therapist for relationship stress

10. Which theorist attributed depression to too little response-contingent positive reinforcement?

A. Peter Lewinsohn
B. Martin Seligman
C. Aaron Beck
D. Susan Nolen-Hoeksema

11. A counselor with no eating disorder training learns that a client needs specialized eating disorder treatment soon, and supervision can’t close the gap in time. The counselor should:

A. End services, since treating her would be unethical
B. Refer the client to an appropriately trained provider
C. Continue treatment while reading about eating disorders
D. Keep her but avoid discussing eating behaviors

12. A member of an ongoing therapy group is moving out of state and will attend two more sessions. The leader’s best approach is to:

A. Add a new member the next week without preparation
B. Have the member end now so the group can move on
C. Encourage a goodbye and process it with the whole

group
D. Let the member leave quietly to avoid upsetting others

13. A career client says, “I’ll major in whatever my uncle thinks is best. It’s not really my decision to make.” Harren would describe this decision-making style as:

A. Rational
B. Dependent
C. Indecisiveness
D. Intuitive

14. A counselor asks two parents to discuss bedtime rules with each other right now, in session, while she observes how their son interrupts. This technique is:

A. Enactment
B. Prescribing the symptom
C. Family sculpting
D. Circular questioning

15. Genetic endowment, environmental conditions, learning experiences, and task-approach skills are core factors in the career theory of:

A. John Krumboltz
B. Anne Roe
C. John Holland
D. Lent, Brown, and Hackett

16. A client afraid of elevators learns muscle relaxation, ranks feared scenes from mild to severe, and imagines each scene while relaxed, one step at a time. This is:

A. Flooding
B. Exposure and response prevention
C. Systematic desensitization
D. Implosive therapy

17. In session eight, with a strong alliance, a client says finishing his degree is his top priority, yet he has skipped class for three weeks. The best response is:

A. “The degree matters most to you, and you’ve missed three weeks of class.”

B. “Why do you keep skipping if the degree is so important?”
C. “You should email your professors today and catch up.”
D. “I wonder if skipping is a way of rebelling against your father.”

18. A teacher pairs a student with a slightly more skilled peer and gradually removes hints as the student masters long division. This approach best reflects:

A. Development by biological timetable
B. Industry vs. inferiority
C. Scaffolding in the zone of proximal development
D. Accommodation of an existing schema

19. A counselor asks a teen, “Tell me about a time when Anger tried to take over and you didn’t let it.” The counselor is searching for:

A. A unique outcome
B. A disputation
C. An exception
D. A miracle-day sign

20. Midway through an intake, a client says quietly, “Something happened when I was deployed,” and looks away. The counselor’s best response is to:

A. Ask the client to describe exactly what happened, in full detail
B. Skip trauma questions for now so rapport is not disrupted
C. Give a projective test to uncover what the client is withholding
D. Acknowledge it, ask permission to return to it, and offer a brief screen

21. A client’s panic attacks began at age 14, three years before she started drinking, and continued during a 6-month period of sobriety. This history best supports:

A. Alcohol withdrawal
B. Hallucinogen persisting perception disorder

C. An independent anxiety disorder
D. Alcohol-induced anxiety disorder

22. A test has a standard error of measurement of 5. A client's observed score is 110. The band within which the true score falls about 68% of the time is:

A. 100 to 120
B. 110 exactly
C. 105 to 115
D. 95 to 125

23. A client says her partner has never hit her but monitors her phone, controls all the money, and has cut her off from friends. Which concept best fits?

A. Enmeshment
B. The Danger Assessment
C. Coercive control
D. The cycle of violence

24. A client says she chooses partners by checklist: compatible career, same faith, and wants children. In John Alan Lee's terms, her love style is:

A. Pragma
B. Storge
C. Quality of alternatives
D. Commitment

25. A client scores 85 on the WAIS. Expressed as a T-score, this result is:

A. 40
B. 30
C. 60
D. 35

26. A counselor and client treat the client's thought "Nobody at work respects me" as a hypothesis and plan a small experiment to gather evidence for and against it. This collaborative stance is most characteristic of:

A. Meichenbaum's cognitive behavior modification
B. Ellis's rational emotive behavior therapy

C. Glasser's reality therapy
D. Beck's cognitive therapy

27. After being laid off three times despite strong reviews, a client says, "Nothing I do changes anything, so why bother trying?" This statement best reflects:

A. Catastrophic misinterpretation
B. Learned helplessness
C. Shattered assumptions
D. Low response-contingent reinforcement

28. After an assault, a client says, "I used to believe the world was basically fair and that I was a good person. Now none of that makes sense." This best reflects:

A. Shattered assumptions
B. Posttraumatic growth
C. Remembrance and mourning
D. Resilience

29. Which term from Truax and Carkhuff describes caring for a client in a way that does not create dependence?

A. Cultural humility
B. Unconditional positive regard
C. Empathic attunement
D. Nonpossessive warmth

30. A client sent by his spouse says his drinking "isn't really a problem" and he isn't sure why he's here. The most appropriate first treatment goal is to:

A. Confront his denial with his spouse present
B. Explore whether he wants anything to change
C. Begin relapse prevention for future slips
D. Set a goal of complete abstinence in 30 days

31. In session three, members argue over whose problems deserve group time, and one says the leader is "playing favorites." The group is most likely in which stage?

A. Contemplation
B. Working (performing)

C. Transition (storming)
D. Initial (forming)

32. A client's primary care physician requests records from the federally assisted substance use disorder program where the client was treated. What is generally required before these records are released?

A. Only the minimum necessary, with no consent
B. A parent's signature under student records law
C. The client's specific written consent
D. Breach notification to the client afterward

33. An employer requires a general aptitude test for warehouse jobs. The test screens out applicants from a protected group and has no demonstrated connection to the work. Which case addresses this practice?

A. Debra P. v. Turlington
B. Diana v. State Board of Education
C. Larry P. v. Riles
D. Griggs v. Duke Power Co.

34. Viewing crisis as a time-limited state, often about four to six weeks, within a framework of primary, secondary, and tertiary prevention, is associated with:

A. Albert Roberts
B. Kenneth Doka
C. Erich Lindemann
D. Gerald Caplan

35. A client asks whether the diagnosis on her insurance claim will appear in her health record. The counselor explains plainly that it will and what the diagnosis means. Which ethical principle does this most directly reflect?

A. Fidelity
B. Veracity
C. Justice
D. Nonmaleficence

36. A partner who usually opens complaints with "You never help" is coached to say, "I'm overwhelmed tonight.

Could you do the dishes?" This Gottman intervention targets:

A. Stonewalling
B. Criticism
C. The pursue-withdraw cycle
D. Contempt

37. A client reports new episodes of racing heart and dread. She drinks several energy drinks a day and recently started a stimulant medication. Malingering and factitious disorder have been ruled out. In First's differential diagnosis sequence, the next step is to:

A. Determine the specific primary disorder
B. Rule out a general medical condition
C. Rule out a substance or medication cause
D. Differentiate an adjustment disorder

38. Teaching parents to use I-messages and active listening instead of "you" messages is most associated with:

A. Albert Ellis
B. Virginia Satir
C. Thomas Gordon
D. John Gottman

39. The hero, scapegoat, lost child, and mascot are roles in families affected by addiction described by:

A. Murray Bowen
B. Sharon Wegscheider-Cruse
C. Virginia Satir
D. E. M. Jellinek

40. A client recovering from alcohol use wants a mutual-support group but prefers a CBT-based approach to a 12-step program. The best referral is:

A. Alcoholics Anonymous
B. Al-Anon
C. SMART Recovery
D. Adult Children of Alcoholics

41. Which theorist proposed that worry is a mostly verbal activity that dampens emotional and physical arousal, so it is negatively reinforced?

A. Adrian Wells
B. Thomas Borkovec
C. Michel Dugas
D. David Clark

42. A client is grieving the overdose death of her ex-husband. Friends tell her, "You were divorced, so why are you this upset?" Her experience is best described as:

A. Anticipatory grief
B. Ambiguous loss
C. Prolonged grief disorder
D. Disenfranchised grief

43. The PTSD specifier "with delayed expression" applies when:

A. Full criteria aren't met until at least 6 months after the event
B. Symptoms begin within 3 months of a stressor
C. Symptoms occur from 3 days to 1 month after the event
D. Depersonalization or derealization is present

44. Yalom's idea that members eventually show in group the same interpersonal patterns that cause them problems outside it is called:

A. Corrective recapitulation of the family
B. The social microcosm
C. Universality
D. Catharsis

45. A counselor tells a Black client, "I don't really see race. I just see you as a person." This statement is best described as:

A. A microinsult
B. A microinvalidation

C. Broaching
D. A microassault

46. A client says, “When I was nineteen, I sold drugs. I’ve never told anyone, and I’m ashamed of it.” The response that best reflects a nonjudgmental stance is:

A. “Selling drugs harms whole communities, so that shame makes sense.”
B. “You’ve carried this alone a long time. What’s the shame like now?”
C. “That was ages ago. You really shouldn’t feel bad about it now.”
D. “Let’s set that aside and talk about how your week went instead.”

47. In the eighth session of a closed group, members give each other direct feedback and take real risks. The leader’s best stance is to:

A. Explain the group rules and provide more structure
B. Rescreen members to confirm fit
C. Step back and help members turn insight into action
D. Focus on consolidating learning and saying goodbye

48. A client with GAD says, “Waiting for my biopsy results is unbearable. I’d almost rather get bad news than not know.” Which model most directly targets this belief?

A. Intolerance of uncertainty
B. Salkovskis’s inflated responsibility
C. Clark and Wells’s safety behaviors
D. Wells’s metacognitive model

49. Which theorist described moral development as moving from a focus on individual survival, to goodness as self-sacrifice, to a morality of nonviolence?

A. Jean Piaget
B. Robert Havighurst
C. Carol Gilligan
D. Lawrence Kohlberg

50. The strengths perspective, which builds treatment plans on what is already working in a client's life, is most associated with:

A. Gerald Caplan
B. Abraham Maslow
C. Dennis Saleebey
D. Saul Rosenzweig

51. A member keeps turning to the leader and asking, "What do you think I should do?" The leader's best response is to:

A. Block the member to protect the group
B. Universalize the shared theme of loneliness
C. Open the question to other members
D. Give a clear answer to model expertise

52. A community agency wants a brief alcohol screen to give every adult client at intake. Which instrument fits that purpose?

A. AUDIT
B. GAD-7
C. PCL-5
D. CBCL

53. Situation selection, attentional deployment, cognitive reappraisal, and response modulation are points of intervention in which framework?

A. Linehan's biosocial theory
B. Porges's polyvagal theory
C. Gross's process model
D. Siegel's window of tolerance

54. A motivated client has missed four sessions because the bus route to the clinic was canceled. The counselor's best response is to:

A. Document the client as resistant to treatment
B. Address the barrier, such as offering telehealth
C. Increase session frequency to rebuild momentum
D. Terminate services for repeated no-shows

55. Which approach holds that distress grows from conditions of worth that create incongruence between self-concept and experience?

A. Hierarchy of needs
B. Individual psychology
C. Gestalt therapy
D. Person-centered therapy

56. After climbing stairs, a client notices his heart pounding, concludes he is having a heart attack, and his fear spirals into panic. Which model best explains the panic?

A. Seligman's preparedness
B. Clark's cognitive model of panic
C. Mowrer's two-factor theory
D. Borkovec's avoidance theory

57. Which term names the legal right that protects a client's communications from being disclosed in court, a right the client holds and can waive?

A. Privileged communication
B. Privacy
C. Minimal disclosure
D. Confidentiality

58. A counselor in private practice is relocating across the country in two months. The most ethical way to handle her current clients is to:

A. Provide pretermination counseling and arrange transfers
B. Give clients a phone directory of local counselors
C. Keep seeing clients by video from the new state
D. Notify clients by letter in the final week

59. A client keeps glancing at the clock and giving short answers. Which counselor statement uses immediacy?

A. "You've mentioned being tired, being busy, and wanting to leave early."
B. "Maybe you're bored because you resent being told what to do."

C. “Right now I notice you watching the clock as we talk. What’s that like?”
D. “When I was in school, I felt restless in long meetings too.”

60. A counselor whose faith strongly opposes divorce is working with a client who has decided to divorce and wants help telling her children. The counselor should:

A. Refer her because of the value conflict
B. Seek supervision and keep serving her competently
C. Share her faith views so the client can decide
D. Urge her to reconsider for the children’s sake

61. A counseling intern’s practicum placement ends in May, so her clients’ services with her must end even though some goals remain unmet. This type of termination is:

A. Premature
B. Planned
C. Forced
D. Client-initiated

62. A counselor feels deep understanding for a client and offers frequent reflections. After five sessions, the client says, “I can never tell whether you actually get me.” Which of Rogers’ conditions is most clearly unmet?

A. The counselor’s own congruence
B. The client perceiving empathy and regard
C. Psychological contact between the two
D. The counselor experiencing empathic understanding

63. A client sues a counselor for harm. Which set of elements must generally be shown for a malpractice claim to succeed?

A. Respect for persons, beneficence, justice
B. Duty, dereliction, direct causation, damages
C. Justice, publicity, and universality
D. Serious threat, identifiable victim, credibility

64. A current client sends her counselor a friend request on the counselor's personal social media account. The counselor should:

A. Accept after documenting it as a boundary extension
B. Refer her to another counselor to avoid conflict
C. Accept, but avoid commenting on her posts
D. Decline; personal virtual relationships are prohibited

65. A counselor whose father died recently notices she changes the subject whenever a client talks about his own dying father. Her best next step is to:

A. Reflect on her own grief and bring it to supervision
B. Tell the client about her father so he feels understood
C. Refer the client to a colleague right away
D. Keep steering sessions toward his work stress

66. Asked what brought him in, a client speaking at an ordinary pace moves from his car repairs to his mechanic's dog to a TV show. Each topic follows from the last, but he never answers the question. His thought process is best described as:

A. Circumstantial
B. Linear and goal-directed
C. Tangential
D. Loose associations

67. Maccoby and Martin expanded Baumrind's model of parenting by adding which style?

A. Permissive
B. Authoritative
C. Slow-to-warm-up
D. Neglectful/uninvolved

68. The same symptoms as schizophrenia, lasting at least 1 month but less than 6 months, define which presentation?

A. Schizophreniform disorder
B. Delusional disorder

C. Brief psychotic disorder
D. Schizophrenia

69. A client describes 3 days of euphoric mood, almost no sleep, racing thoughts, and grandiose spending, ending with a psychiatric hospitalization on day 3. This episode is best classified as:

A. Cyclothymic disorder
B. A hypomanic episode
C. Brief psychotic disorder
D. A manic episode

70. While building a safety plan, a client writes, “Go to my cousin’s to watch the game. I don’t have to talk about how I feel.” This belongs in which step?

A. People and places that provide distraction
B. Making the environment safe from lethal means
C. People I can ask for help with the crisis
D. Internal coping strategies I can use alone

71. A counselor realizes a progress note written last week lists the wrong medication dosage. The counselor should:

A. Add a dated correction to the record
B. Leave it, since the next note will be accurate
C. Move the correction into the psychotherapy notes
D. Rewrite the note so the error no longer appears

72. Prealcoholic, prodromal, crucial, and chronic are phases described by:

A. Alan Marlatt
B. Prochaska & DiClemente
C. E. M. Jellinek
D. Sharon Wegscheider-Cruse

73. For the past 14 months, a 15-year-old has been irritable most days, with low energy and poor sleep. He has never met criteria for a major depressive episode and has no temper outbursts. The best fit is:

A. Disruptive mood dysregulation disorder
B. Cyclothymic disorder

C. Adjustment disorder with depressed mood
D. Persistent depressive disorder

74. A client says he attends religious services mainly because it helps his business contacts and standing in town. Allport would describe this as:

A. Intrinsic religiosity
B. Spiritual bypassing
C. Negative religious coping
D. Extrinsic religiosity

75. A client links her strong preference for working with data rather than people to growing up in a cold, emotionally distant home. Which theorist's ideas best fit her explanation?

A. Linda Gottfredson
B. Anne Roe
C. John Krumboltz
D. Donald Super

76. When assessing a client's risk of harming someone else, which factor is the strongest single predictor of future violence?

A. Recent substance use
B. A previous suicide attempt
C. Hopelessness about the future
D. A history of violence

77. Perceived susceptibility, perceived severity, perceived benefits, perceived barriers, and cues to action are elements of:

A. The transactional model of stress
B. The adaptive tasks of illness
C. The biopsychosocial model
D. The health belief model

78. Loss, rejection, guilt and shame, grief, identity, intimacy, and mastery/control are:

A. Worden's tasks of mourning
B. Silverstein and Kaplan's core issues in adoption

C. Rando’s six R processes
D. Finkelhor and Browne’s traumagenic dynamics

79. In a family with an alcoholic parent, the oldest daughter earns top grades, cares for her siblings, and makes the family look successful to outsiders. This best fits:

A. The abstinence violation effect
B. “Don’t talk, don’t trust, don’t feel”
C. The family hero role
D. The crucial phase

80. An agency reviews its new peer-support program halfway through the year so staff can make improvements while it is running. This is a:

A. Cost-benefit analysis
B. Summative evaluation
C. Needs assessment
D. Formative evaluation

81. A client says her faith guides her decision to stay in a difficult but nonviolent marriage. The counselor personally believes she should leave. The counselor should:

A. Suggest she talk to her clergy rather than to a counselor
B. Explore how her faith shapes the choice, without imposing values
C. Share the counselor’s view that leaving would be healthier
D. Point out the research on outcomes of unhappy marriages

82. Which researcher described a “sleeper effect” in which some effects of parental divorce appear in adulthood, especially in intimate relationships?

A. Judith Wallerstein
B. Paul Amato
C. Paul Bohannan
D. E. Mavis Hetherington

83. At her final session, a client offers her counselor a small handmade bracelet, explaining that giving such gifts is an important custom in her family. Under the ACA Code, the counselor should consider:

A. Whether the agency allows self-referral
B. Whether a five-year waiting period has passed
C. Whether a written barter contract was signed
D. Its monetary value and the client's motivation

84. A couples approach that teaches communication and problem-solving skills while also helping partners accept differences that likely won't change is:

A. Emotionally focused therapy
B. The Gottman Method
C. Integrative behavioral couple therapy
D. The speaker-listener technique

85. Goals that are small, concrete, behavioral, stated as the presence of something, and within the client's control are the well-formed goals described by:

A. de Shazer (solution-focused)
B. Prochaska and DiClemente
C. Doran (SMART objectives)
D. Saleebey (strengths perspective)

86. Near the end of a session the counselor says, "Today you've talked about the move, missing your old friends, and feeling unsure at your new job." This skill is:

A. Reflecting meaning
B. Paraphrasing
C. Summarizing
D. Clarifying

87. A client is referred by his probation officer as a condition of release. Before counseling begins, the counselor should:

A. Obtain his assent while probation gives consent
B. Promise that sessions will stay fully confidential

C. Explain what information will be shared with probation
D. Wait to discuss reporting until probation requests it

88. An 80-year-old pianist now performs fewer pieces, practices those more often, and slows down before fast passages so they sound quicker by contrast. This best illustrates:

A. Selective optimization with compensation
B. Gerotranscendence
C. Continuity theory
D. Socioemotional selectivity theory

89. A counselor is updating an old agency intake form that still asks for a functioning rating on Axis V. To align the form with the DSM-5-TR, the functioning section should use:

A. The ASAM Criteria
B. The SCID-5 interview
C. A GAF rating on Axis V
D. The WHODAS 2.0

90. In a SOAP note, the counselor writes, “Client is making moderate progress toward Objective 1; depressive symptoms appear to be lessening.” This entry belongs in which section?

A. Objective
B. Plan
C. Subjective
D. Assessment

91. A client says, “I told my parents I’m leaving college, and my dad hasn’t spoken to me since.” Which response best shows empathic responding?

A. “I know exactly how you feel; my dad went quiet on me too.”
B. “Oh no, I feel so bad for you. That’s awful of him to do.”
C. “Give him some time. I’m sure he’ll come around soon enough.”

D. “His silence seems to leave you shut out and unsure where you stand.”

92. A Black man with limited resources works two jobs and pushes himself relentlessly to overcome workplace discrimination. He has developed hypertension. Which concept best fits?

A. Stereotype threat
B. John Henryism
C. Internalized racism
D. Impostor phenomenon

93. A 10-year-old eats only a handful of foods because other textures make him gag. He has lost weight but has no concern about body shape or size. The best fit is:

A. ARFID
B. Anorexia nervosa, restricting type
C. Rumination disorder
D. Pica

94. The father of a depressed teen asks what he can do at home to help keep his son safe. The counselor’s most important recommendation is to:

A. Avoid mentioning suicide so the idea isn’t planted
B. Treat a sudden calm as a sign danger has passed
C. Limit access to firearms and stockpiled medications
D. Have his son sign a no-suicide contract

95. A supervision model describes the supervisor shifting among the roles of teacher, counselor, and consultant, with foci on intervention, conceptualization, and personalization. This model belongs to:

A. Forester-Miller and Davis
B. Bernard (discrimination model)
C. Stoltenberg and Delworth (IDM)
D. Kitchener

96. In a study of a new anxiety technique, several waitlist participants hear about the technique from treatment-

group friends and begin practicing it. This threat to internal validity is:

A. John Henry effect
B. Hawthorne effect
C. Experimental mortality
D. Diffusion of treatment

97. A counselor licensed only in Georgia has a client who will spend three months working in Colorado and wants to continue video sessions from there. The key issue is that:

A. The client must be offered face-to-face sessions
B. The platform must verify identity with a code word
C. Licensure generally follows the client’s location
D. Her Georgia license covers sessions she provides

98. On a genogram, two slashes across the line connecting partners indicates:

A. Divorce
B. Separation
C. Conflict
D. Cutoff

99. A widower spends some days looking through photos and crying, and other days sorting insurance papers and taking a cooking class. His counselor sees this back-and-forth as healthy. Which model supports that view?

A. Kübler-Ross’s bargaining stage
B. Continuing bonds
C. Stroebe & Schut’s dual process model
D. Bowlby’s reorganization phase

100. A counselor wants to measure whether a client’s symptoms improve from the first session to the twelfth, checking progress along the way. Which instrument best serves this purpose?

A. OQ-45
B. Rorschach Inkblot Test

C. WAIS-5
D. Strong Interest Inventory

101. A counselor moves a client who has met her goals from weekly to biweekly and then monthly sessions before ending. This practice is called:

A. Booster sessions
B. Relapse prevention
C. Tapering
D. Step-down care

102. A counselor coaches an adult client to stay calm and take clear "I" positions with his parents instead of being pulled into their arguments. The goal is to increase:

A. Congruent communication
B. Differentiation of self
C. Second-order change
D. Clear subsystem boundaries

103. A client says, "I know smoking is hurting me, and I've thought about quitting, but it's how I handle stress." The best stage-matched intervention is to:

A. Confront the client's denial
B. Set a specific quit date this week
C. Explore a decisional balance
D. Begin relapse prevention planning

104. Each week a member spends long stretches describing her coworkers' arguments in detail. The leader's best response is to:

A. Interpret her stories as avoidance of her childhood
B. Let her finish, since any sharing builds cohesion
C. Invite members to advise her about her coworkers
D. Ask how she is feeling as she tells the group this

105. Which grief model describes what mourners actively do, ending with finding an enduring connection with the deceased while embarking on a new life?

A. Kübler-Ross's stages of dying
B. Worden's tasks of mourning

C. Bowlby's phases of mourning
D. The dual process model

106. After her divorce, a client spends hours each night replaying what went wrong and asking, "Why am I like this?" Her low mood deepens each time. Which theory best explains why her depression is lasting longer?

A. Rachman's cognitive theory of OCD
B. Seligman's preparedness
C. Nolen-Hoeksema's response styles theory
D. Klerman and Weissman's role disputes

107. A client laughs while describing being left out of his brother's wedding. The counselor feels a real heaviness. Which response best shows congruence?

A. "That sounds like such a funny family story. Tell me more about it!"
B. "I notice I feel heavy hearing this, even as you laugh. What's that like for you?"
C. "Honestly, my own brother left me out once, and I'm still angry about it."
D. "I'm sure he didn't mean to hurt you. Families are complicated."

108. A 19-year-old says, "I haven't really thought about what I believe or what I want to do, and I'm not worried about it." Marcia would classify her identity status as:

A. Identity diffusion
B. Moratorium
C. Identity vs. role confusion
D. Pre-encounter

109. A client says, "I need to cut back on drinking because my doctor is worried about my liver." In motivational interviewing, this statement is:

A. Contemplation
B. Sustain talk
C. The righting reflex
D. Change talk

110. Career adaptability, made up of concern, control, curiosity, and confidence, is central to which theory?

A. Cognitive Information Processing
B. Life-span, life-space theory
C. Theory of Work Adjustment
D. Career construction theory

111. In Roberts' seven-stage crisis intervention model, the first stage is to:

A. Assess lethality and imminent danger
B. Define the problem
C. Make psychological contact
D. Offer contact and engagement

112. A client still angry at her late mother speaks directly to an empty chair as though her mother were sitting in it. The concept this technique most directly targets is:

A. Basic mistakes
B. Unfinished business
C. Emotional cutoff
D. Transference

113. Who held that meaning can be found through creative work, through love and experience, and through the attitude one takes toward unavoidable suffering?

A. Irvin Yalom
B. Rollo May
C. Viktor Frankl
D. Ernest Becker

114. A client's attorney, who has no testing training, asks for the client's MMPI-3 item responses and answer sheet. The client has signed a release. The counselor should:

A. Refuse to share anything about the testing with anyone
B. Release raw data only to a qualified professional the client designates
C. Send the item responses to the attorney because consent was given

D. Photocopy the test booklet so the attorney can review the items

115. Which identity model moves from awareness, through disclosure to significant others and exploring transition issues, to integration and acceptance?

A. Cass's homosexual identity formation
B. Lev's transgender emergence
C. Troiden's model
D. D'Augelli's life-span model

116. A 15-year-old with serious antisocial behavior receives intensive home- and community-based treatment that targets his family, peers, school, and neighborhood. This approach is:

A. Parent-child interaction therapy
B. Problem-solving skills training
C. Multisystemic therapy
D. Functional family therapy

117. A counselor computes Cronbach's alpha for a 20-item anxiety scale given once to a sample of clients. What is she evaluating?

A. Internal consistency
B. Test-retest reliability
C. Alternate forms reliability
D. Interrater reliability

118. An immigrant father keeps his social life entirely within his heritage community and avoids contact with the host culture. Berry would call this strategy:

A. Marginalization
B. Immersion-emersion
C. Separation
D. Assimilation

119. Two days after a methamphetamine binge, a client is sleeping 14 hours a day, eating constantly, having vivid unpleasant dreams, and feeling deeply depressed. The counselor should recognize this as:

A. Stimulant withdrawal, with possible suicide risk
B. Alcohol withdrawal
C. Opioid withdrawal
D. Cannabis withdrawal

120. Which case held that a therapist's duty can extend to a foreseeable victim based on a client's history of violence, even without an explicit threat?

A. Tarasoff v. Regents, Tarasoff I (1974)
B. Ewing v. Goldstein (2004)
C. Tarasoff v. Regents, Tarasoff II (1976)
D. Jablonski v. United States (1983)

121. A 4-year-old insists the moon follows her car because it wants to play with her. Which concept best describes her thinking?

A. Rapprochement
B. Animism
C. Initiative vs. guilt
D. Personal fable

122. A counselor uses an AI note-taking tool that meets HIPAA requirements. A new client says she doesn't want the tool used in her sessions. The counselor should:

A. Use a free consumer chatbot without her name instead
B. Refer her to a counselor who doesn't use AI tools
C. Respect her refusal and document without the tool
D. Use it anyway, since notes are the counselor's responsibility

123. A counselor gives 25 clients the GAD-7 at intake and again at week 10 and wants to know whether the mean score changed. The appropriate test is:

A. One-way ANOVA
B. Dependent (paired) t-test
C. Chi-square
D. Independent t-test

124. A client says, “I took the lower-paying job so I could be home when my kids get off the bus.” Which response is a reflection of meaning?

A. “You chose a job with less pay and more time at home.”
B. “Why did you decide money was less important?”
C. “Being there for your kids matters more than the pay.”
D. “You sound relieved that you made the decision.”

125. A client snaps, “You’re judging me, just like my father always did,” though the counselor has said nothing critical. The client’s reaction is best understood as:

A. Reaction formation
B. Sublimation
C. Transference
D. Countertransference

126. Over the past year, a client has had cravings, drunk more than intended, tried and failed to cut down, and given up softball to drink. No other criteria apply. The alcohol use disorder severity is:

A. Mild
B. Early remission
C. Severe
D. Moderate

127. A parent notes that one child rose from the 50th to the 55th percentile and another from the 90th to the 95th, and concludes both gained the same amount. The counselor’s most accurate explanation is that:

A. Both children gained five points of the same ability
B. Percentile ranks are ordinal, not equal intervals
C. The difference is explained by the standard error of measurement
D. The higher scores reflect a ceiling effect on the test

128. In Olweus’s definition, which feature distinguishes bullying from ordinary peer conflict?

A. Physical rather than verbal harm
B. Vindictive behavior toward peers

C. A clear antecedent and consequence
D. A power imbalance between the students

129. A counselor asks a client to describe her earliest specific memories to understand her lifestyle and her mistaken beliefs about how to belong. The counselor is using:

A. Early recollections
B. Life review
C. Free association
D. Active imagination

130. A counselor receives an order signed by a judge requiring release of a client's treatment records. The counselor's best response is to:

A. Destroy any psychotherapy notes before responding
B. Comply, releasing only what the order requires
C. Refuse and assert privilege until the client consents
D. Send the complete record so nothing is withheld

131. Who described a Stepfamily Cycle that begins with fantasy, immersion, and awareness?

A. Patricia Papernow
B. Paul Bohannan
C. McGoldrick and Carter
D. Emily and John Visher

132. During a video session, a client suddenly says she just swallowed a handful of pills. Which step, taken at the start of the session, matters most now?

A. Explaining that email isn't for emergencies
B. Verifying her identity with a code word
C. Confirming her physical location
D. Confirming the platform has a BAA

133. Near the end of counseling, a client asks, "How do I keep this going once we stop meeting?" The counselor's best response is to:

A. Review which skills helped and how she'll keep using them
B. Refer her for a medication evaluation to protect gains

C. Step her up to intensive outpatient for support
D. Add a new long-term goal to extend counseling

134. To help a child who can't stay seated, a counselor reinforces sitting for 2 minutes, then 4, then 8, until the child sits through the whole lesson. This is:

A. Modeling
B. Systematic desensitization
C. Shaping
D. The Premack principle

135. At intake a client says, "I've felt empty and irritable for weeks." In the mental status exam, this statement is recorded under which component?

A. Mood
B. Thought content
C. Insight
D. Affect

136. Months after her divorce is final, a client says, "I'm finally learning who I am on my own, not as half of a couple." Bohannan would place this in which station?

A. Emotional
B. Coparental
C. Reorientation
D. Psychic

137. A client calls hours after losing her job and being evicted, crying that she doesn't know where she and her children will sleep tonight. The counselor's best response is to:

A. Take a directive role to secure shelter tonight
B. Explore how early losses shape her reaction
C. Reflect her feelings and let her lead
D. Set long-term career goals with her

138. A member says sarcastically, "Maybe if you weren't so needy, your husband wouldn't have left." The leader says, "Let me stop you there. Can you tell her what you're feeling instead?" This is:

A. Drawing out
B. Blocking
C. Linking
D. Cutting off

139. Which DSM-5-TR term applies when the clinician strongly presumes a client will meet full criteria but does not yet have enough information?

A. Unspecified disorder
B. Other specified disorder
C. Principal diagnosis
D. Provisional diagnosis

140. After being sent to his room, a boy deliberately breaks his sister's favorite toy. His father says he feels deeply hurt. According to Dreikurs, the boy's most likely goal is:

A. Revenge
B. Power
C. Coercive escalation
D. Attention

141. For the first time, a client says, "Some days I feel like a failure as a mother," then goes quiet and looks down. The counselor's best response is to:

A. Ask what her children would say about her parenting
B. Reassure her that she is clearly a good mother
C. Summarize everything she has shared so far in the session
D. Allow a comfortable silence

142. At a planned session 6 review, the best way for the counselor to engage a client in reviewing progress is to:

A. Wait for her to raise any concerns about progress
B. Show baseline and current scores and ask what she notices
C. Readminister the full intake assessment battery
D. Tell her she is improving and move on to new material

143. A worldview model built on two dimensions, locus of control and locus of responsibility, was proposed by:

A. Geert Hofstede
B. Derald Wing Sue
C. Pamela Hays
D. Edward T. Hall

144. Before a job interview, a client asks herself, “Do I have the skills and support to handle this?” In Lazarus and Folkman’s model, she is making a:

A. Shift to the resistance stage
B. Primary appraisal
C. Secondary appraisal
D. Hardiness judgment of control

145. A researcher wants to show that a social skills intervention caused improvement, but withdrawing the intervention would be unethical. She introduces it at staggered times across three settings. This design is:

A. Cross-sectional
B. Multiple baseline
C. Quasi-experimental
D. ABAB reversal

146. A client reports active thoughts of suicide and a vague plan, with no clear intent and a supportive partner. The best response matched to this level of risk is:

A. Immediate involuntary hospitalization until risk passes
B. A no-suicide contract with routine weekly sessions as before
C. Noting it and revisiting if it comes up again later
D. A safety plan, means restriction, and more frequent sessions

147. In a children’s divorce-adjustment group, the leader runs a sentence-completion activity. Right afterward, the leader should:

A. Move straight to the next activity
B. Process it so members can share reactions

C. Collect the sheets to review privately later
D. Require each child to read an answer aloud

148. A client meets 6 of the 9 gambling disorder criteria in the past 12 months. The severity is:

A. Mild
B. Severe
C. Severe, as with 6 or more criteria
D. Moderate

149. A counselor tells a client with insomnia to use the bed only for sleep, go to bed only when sleepy, and get up if not asleep after about 20 minutes. This component of CBT-I is:

A. Stimulus control
B. Imagery rehearsal therapy
C. Sleep restriction
D. Progressive muscle relaxation

150. A client is terrified of being judged. Her counselor assigns her to wear mismatched shoes to the grocery store on purpose and notice that being stared at is not awful. This homework is:

A. An exaggeration exercise
B. Acting as if
C. A shame-attacking exercise
D. Paradoxical intention

151. A White counselor says to a client describing racist jokes at work, “I wonder how it feels to talk about racism with someone who doesn’t share your background.” This is:

A. An etic approach
B. Cultural humility
C. Broaching
D. Cultural encapsulation

152. In session five a client says angrily, “These breathing exercises are a waste of time, and I’m not sure you get me.” The counselor’s best response is to:

A. Explain the research supporting breathing exercises
B. Thank the client and ask what hasn't fit
C. Offer a referral to a different counselor
D. Continue with the planned exercise

153. At intake three months ago, a client denied suicidal thoughts. Today she reports a breakup, nightly drinking, and pulling away from friends. The counselor should:

A. Ask her directly about suicidal thoughts today
B. Rely on the intake screen, which found no ideation
C. Arrange inpatient admission because of the new stressors
D. Have her sign a no-suicide contract before leaving

154. Directives, ordeals, and a focus on power within the family hierarchy are most associated with:

A. Carl Whitaker
B. Jay Haley
C. Salvador Minuchin
D. Murray Bowen

155. A supervisor points out that a new counselor often finishes clients' sentences, something the counselor had never noticed. In the Johari Window, this behavior was in the counselor's:

A. Blind area
B. Hidden area
C. Unknown area
D. Open area

156. Immigrant parents and their 16-year-old, raised mostly in the U.S., argue constantly about curfew and dating. The best systemic framing is:

A. The parents' overly rigid family boundaries
B. The teen's oppositional defiance toward parents
C. An emotional cutoff between the generations
D. A gap in acculturation between generations

157. Parents in a calm, low-conflict marriage divorce, and their teen struggles more than peers whose high-conflict parents divorced. Whose research best predicts this?

A. Judith Wallerstein
B. Paul Amato
C. E. Mavis Hetherington
D. Diana Baumrind

158. A client with OCD checks the stove 20 times before leaving because "if the house burns down, it will be entirely my fault." Which concept best explains the checking?

A. Learned helplessness
B. Avoidance theory of worry
C. Inflated responsibility
D. Intolerance of uncertainty

159. A client says, "I cried in front of my team when we lost the contract. I'm so weak." Which response is a reframe?

A. "Crying then may show how much you cared about the work."
B. "I wonder if crying stirs up how you were told not to cry as a boy."
C. "You felt embarrassed after crying in front of your team."
D. "You call yourself weak, yet you led the team to the final round."

160. Which theory explains fit between worker and job through the worker's satisfaction and the employer's view of the worker's satisfactoriness?

A. Theory of Work Adjustment
B. Holland's theory of vocational personalities
C. Social cognitive career theory
D. Hoppock's needs-based theory

161. A client of a different racial background than the counselor begins describing racism at work, then hesit-

ates. Which counselor response best reflects multicultural sensitivity?

A. “Let’s focus on what you can control, like your work habits.”
B. “Many people from your background deal with that at work.”
C. “I try to treat everyone the same, so race won’t matter here.”
D. “I wonder what it’s like to talk about this with someone of a different race.”

162. A researcher lowers alpha from .05 to .01. What is the result?

A. Fewer Type I errors, more Type II errors
B. A larger effect size
C. Fewer Type II errors, more Type I errors
D. Greater statistical power

163. A five-level empathy scale on which level 3 is the interchangeable, minimally facilitative response was developed by:

A. Robert Carkhuff
B. Gerard Egan
C. Clara Hill
D. Edward Bordin

164. Which progress note format organizes entries as Goal, Intervention, Response, and Plan?

A. DAP
B. BIRP
C. SOAP
D. GIRP

165. Which model of sexual response added desire as a phase, describing desire, excitement, and orgasm?

A. Annon’s PLISSIT model
B. Masters and Johnson’s four phases
C. Basson’s circular model
D. Kaplan’s triphasic model

166. During the Strange Situation, a toddler approaches his returning mother, then suddenly freezes and turns away with a dazed look. This pattern best fits:

A. Anxious-resistant attachment
B. Slow-to-warm-up temperament
C. Anxious-avoidant attachment
D. Disorganized attachment

167. A couple requests couples counseling. During a private screening, one partner discloses that the other controls the money, monitors the phone, and makes threats. The counselor's best decision is to:

A. Decline joint sessions and offer individual counseling with safety planning
B. Refer the controlling partner to an anger management class
C. Begin couples counseling that focuses on communication skills
D. Raise the disclosure with both partners in the next joint session

168. In Joiner's interpersonal theory of suicide, what makes an attempt possible, as distinct from what creates the desire for death?

A. Psychache
B. Anomie
C. Acquired capability
D. Perceived burdensomeness

169. Which theorist described empathy as "vicarious introspection"?

A. Carl Rogers
B. Edward Bordin
C. Robert Carkhuff
D. Heinz Kohut

170. A client who has barely slept in three days talks rapidly, jumping from her business plan to a song lyric to a trip to Paris. Each jump has a loose link to the one before. Her thought process is best described as:

A. Tangential thinking
B. Loose associations
C. Circumstantial thinking
D. Flight of ideas

171. With ten minutes left, a client begins describing a sexual assault for the first time, then goes quiet and stares blankly. The counselor's best action is to:

A. End the session on time and schedule the next one
B. Reassure her that she is completely safe now
C. Slow down and help her ground in the room
D. Encourage her to keep going while the memory is fresh

172. Which attribute describes a counselor being real and authentic rather than hiding behind a professional role or façade?

A. Genuineness
B. Unconditional positive regard
C. Empathic attunement
D. Nonpossessive warmth

173. Death, freedom, isolation, and meaninglessness are the four givens of existence described by:

A. Irvin Yalom
B. Viktor Frankl
C. Abraham Maslow
D. Fritz Perls

174. Two families with very different histories both come to counseling with a teen who refuses to attend school. This illustrates:

A. Homeostasis
B. Morphogenesis
C. Multifinality
D. Equifinality

175. A new depression screen correctly flags 95% of people who truly have depression. This property of the screen is its:

A. Sensitivity
B. Specificity
C. Base rate
D. Incremental validity

176. A 45-year-old client discloses that an uncle sexually abused her when she was a child. The uncle died years ago, and no child is currently at risk. The counselor should generally:

A. Support her in processing it; no mandated report applies
B. Notify police under the duty to protect
C. Report the abuse to adult protective services
D. Report the abuse to child protective services today

177. A member who has felt useless since retiring brightens after helping a newer member find a job lead. Which of Yalom's therapeutic factors is at work?

A. Imparting information
B. Instillation of hope
C. Universality
D. Altruism

178. A client has not completed his thought records for three weeks and says, "Part of me wants to, part of me doesn't see the point." The counselor's best response is to:

A. Explore his ambivalence using motivational interviewing
B. Increase the homework load to build discipline
C. Remind him that homework is required to continue
D. Drop homework from the plan to avoid any further conflict

179. A client with panic attacks mentions that his mother and grandmother both had panic disorder. In a 4 Ps case formulation, this family history is a:

A. Protective factor
B. Predisposing factor

C. Perpetuating factor
D. Precipitating factor

180. Scores on a college admissions test are correlated with the same students' grade point averages one year later. This study provides evidence of:

A. Content validity
B. Discriminant validity
C. Predictive validity
D. Concurrent validity

181. A client with a confirmed contagious, life-threatening disease says he has no plans to tell his partner and will continue high-risk behavior. Under the ACA Code, the counselor:

A. May keep it confidential as an end-of-life decision
B. Must keep this confidential as he is a competent adult
C. May be justified in disclosing to the identifiable partner
D. Must file a report with child protective services

182. A counselor asks, "Suppose tonight while you sleep the problem is solved. When you wake up, what is the first thing you'd notice?" This is:

A. An externalizing question
B. Adler's "the Question"
C. The miracle question
D. A circular question

183. A counselor coaches a mother in real time through an earpiece while she plays with her 4-year-old who has frequent tantrums. This approach is:

A. Sandplay
B. Filial therapy
C. Child-centered play therapy
D. Parent-Child Interaction Therapy

184. After eight sessions, a client's anxiety scores haven't improved. She mentions she has been drinking every night since starting a stressful new job. The counselor's best next step is to:

A. Refer her out, since drinking is outside the plan
B. Reassess and revise the plan with her
C. Switch to a new approach without discussing it
D. Continue the plan, since change takes time

185. In the second session, a client gently corrects the counselor: “I use they/them pronouns.” The counselor’s best response is to:

A. Apologize at length for the mistake
B. Explain that intake records list another gender
C. Ask why they decided to use those pronouns
D. Thank them briefly and use their pronouns consistently

186. A client denies any suicidal thoughts but faces eviction this week and has no food at home. She wants to explore how her childhood shaped her relationships. The first treatment priority is:

A. Exploring childhood relationship patterns
B. Basic needs and stabilization
C. Career counseling to raise her income
D. A full trauma history in the next session

187. After years of hearing clients’ trauma stories, a counselor notices that her beliefs about safety and her trust in people have lastingly changed. This best describes:

A. Latent deprivation
B. Burnout
C. Compassion fatigue
D. Vicarious trauma

188. Early in an intake, a counselor asks, “Why do you think this is happening?” and “What makes it better or worse?” These questions come from which DSM-5-TR tool?

A. Cultural Formulation Interview
B. Mental Status Exam
C. 4 Ps case formulation
D. Structured Clinical Interview (SCID-5)

189. A treatment plan objective reads: “Client will attend two support group meetings per week, as shown by attendance sheets.” Which SMART element is missing?

A. Relevant
B. Measurable
C. Specific
D. Time-bound

190. An adolescent survivor of sexual abuse says she feels “dirty and damaged” compared with other kids. Finkelhor and Browne would describe this as:

A. Stigmatization
B. Shattered assumptions
C. Betrayal
D. Powerlessness

191. Self-insight, self-integration, anxiety management, empathy, and conceptualizing skills are factors in which counselor task?

A. Building the working alliance
B. Offering the core conditions
C. Managing countertransference
D. Repairing alliance ruptures

192. The W-curve of cultural adjustment adds which experience to the U-curve?

A. An initial honeymoon period
B. Reverse culture shock on returning home
C. Marginalization from both cultures
D. Children acculturating faster than parents

193. In an individual session, one partner in couples counseling discloses an affair. The counselor explained a no-secrets policy at intake. The best response is to:

A. Agree to keep the affair private
B. End couples counseling and refer both partners
C. Reveal it at the next joint session without warning
D. Support the partner in disclosing, per the policy

194. A 48-year-old accountant takes courses on new tax software so she can stay competitive in her current position. In Super's model, she is:

A. Showing congruence
B. Stabilizing in the establishment stage
C. Using planned happenstance
D. Updating in the maintenance stage

195. In a treatment plan, "Client will experience reduced anxiety and return to full participation at work" is best classified as a:

A. Short-term objective
B. Long-term goal
C. Intervention
D. Problem statement

196. A client says, "My wife wants me to quit drinking, but I'm only here to get her off my back." In Bordin's model, the alliance is weakest in:

A. The bond
B. Agreement on goals
C. Collaborative empiricism
D. Agreement on tasks

197. Which statement from one group member to another best models effective feedback?

A. "A while back you seemed distracted at some point."
B. "You're just a really negative person to be around."
C. "When you checked your phone as I spoke, I felt brushed off."
D. "Everyone in here thinks you're kind of selfish, honestly."

198. Which of Yalom's therapeutic factors is considered the group equivalent of the therapeutic alliance?

A. Group cohesiveness
B. Universality
C. Altruism
D. Catharsis

199. In a group, a member snaps at the leader, “You always let Dana talk the most. You obviously like her better.” The leader’s best response is:

A. “I’m so sorry. I never meant to make anyone feel that way.”
B. “You’re feeling overlooked here. I’d like to hear more about that.”
C. “Let’s talk about this privately after group ends today.”
D. “I give every member equal time, so that isn’t accurate.”

200. After six weeks without gambling, a client places one bet and says, “I’ve already blown it.” Using Marlatt’s relapse prevention, the counselor’s best response is to:

A. Start treatment over at precontemplation
B. Explore the childhood roots of gambling
C. Confront him about breaking his commitment
D. Treat the lapse as a learning event

NCE Practice Exam 1 — Answer Key with Ownership Map

Every rationale names who or what **owns** each option. Read all four, not just the correct one: each item teaches four pairings.

How to review: for every item you missed *or guessed*, write in your error log (Workbook, Part 4) the owner you picked and the owner of the correct answer.

Quick Key

1. B	2. B	3. B	4. D	5. C	6. D	7. B	8. D	9. A	10. A
11. B	12. C	13. B	14. A	15. A	16. C	17. A	18. C	19. A	20. D
21. C	22. C	23. C	24. A	25. A	26. D	27. B	28. A	29. D	30. B
31. C	32. C	33. D	34. D	35. B	36. B	37. C	38. C	39. B	40. C
41. B	42. D	43. A	44. B	45. B	46. B	47. C	48. A	49. C	50. C
51. C	52. A	53. C	54. B	55. D	56. B	57. A	58. A	59. C	60. B
61. C	62. B	63. B	64. D	65. A	66. C	67. D	68. A	69. D	70. A
71. A	72. C	73. D	74. D	75. B	76. D	77. D	78. B	79. C	80. D
81. B	82. A	83. D	84. C	85. A	86. C	87. C	88. A	89. D	90. D
91. D	92. B	93. A	94. C	95. B	96. D	97. C	98. A	99. C	100. A
101. C	102. B	103. C	104. D	105. B	106. C	107. B	108. A	109. D	110. D
111. A	112. B	113. C	114. B	115. B	116. C	117. A	118. C	119. A	120. D

121. B	122. C	123. B	124. C	125. C	126. D	127. B	128. D	129. A	130. B
131. A	132. C	133. A	134. C	135. A	136. D	137. A	138. B	139. D	140. A
141. D	142. B	143. B	144. C	145. B	146. D	147. B	148. D	149. A	150. C
151. C	152. B	153. A	154. B	155. A	156. D	157. B	158. C	159. A	160. A
161. D	162. A	163. A	164. D	165. D	166. D	167. A	168. C	169. D	170. D
171. C	172. A	173. A	174. D	175. A	176. A	177. D	178. A	179. B	180. C
181. C	182. C	183. D	184. B	185. D	186. B	187. D	188. A	189. D	190. A
191. C	192. B	193. D	194. D	195. B	196. B	197. C	198. A	199. B	200. D

Rationales

1 — B

Counseling Skills & Interventions · Outline 5.J · Chapter 7

- **B. Circular causality — Owner: Systems theory.** Behaviors that cause and are caused by each other in a loop show circular causality.
- **A. Equifinality** — Owner: Systems theory. Equifinality means different starting points can lead to the same outcome.
- **C. Linear causality** — Owner: Individual (non-systemic) view. Linear causality says A causes B, the view systems thinking replaces.
- **D. A double bind** — Owner: Bateson / MRI. A double bind is contradictory messages at different levels with no way to comment or leave.
- **Pair to learn:** Behavior loop → circular causality

2 — B

Intake, Assessment & Diagnosis · Outline 2.E · Chapter 19

- **B. Factitious disorder imposed on self — Owner: Factitious disorder (sick role, no external reward).** Intentionally falsifying symptoms without obvious external reward, to assume the sick role, is factitious disorder; she is doing it to herself.
- **A. Somatic symptom disorder** — Owner: Somatic symptom disorder (not intentional). In somatic symptom disorder the symptoms are not intentionally produced, but she secretly causes hers.
- **C. Malingering** — Owner: Malingering (external gain). Malingering requires an external reward such as money or avoiding work, which the stem rules out.
- **D. Factitious disorder imposed on another** — Owner: Factitious disorder imposed on another (formerly Munchausen by proxy). Imposed on another means falsifying illness in someone else; she is producing symptoms in herself.
- **Pair to learn:** Fakes symptoms to be seen as sick → Factitious disorder (not malingering)

3 — B

Areas of Clinical Focus · Outline 3.U · Chapter 3

- **B. Circumscription — Owner: Gottfredson (circumscription and compromise).** Gottfredson held that children progressively rule out options, by sex-role fit (about ages 6-8) and then by prestige.
- **A. The growth stage** — Owner: Super (life-span, life-space). Super's growth stage is about developing self-concept, not narrowing options by gender.
- **C. Consistency** — Owner: Holland (RIASEC). Consistency concerns how close a person's top types sit on the hexagon.
- **D. The fantasy period** — Owner: Ginzberg et al. (developmental). Ginzberg's fantasy period involves

imagining careers freely, not ruling them out by gender.

- **Pair to learn:** Ruling out options by sex role → Gottfredson

4 — D

Intake, Assessment & Diagnosis · Outline 2.H · Chapter 19

- **D. Partial hospitalization — Owner: Partial hospitalization (PHP): day program, home at night.** PHP is a day program most of the day, most days, with the client returning home at night, for significant symptoms that don't need 24-hour care.
- **A. Residential treatment** — Owner: Residential treatment (living in a structured setting). Residential care means living in the treatment setting; this client can safely go home each night.
- **B. Inpatient hospitalization** — Owner: Inpatient hospitalization (imminent danger, 24-hour care). Inpatient care is for imminent danger, severe inability to care for self, or medical instability, none of which is present.
- **C. Weekly outpatient counseling** — Owner: Outpatient (mild to moderate symptoms). Weekly outpatient care fits mild to moderate symptoms, but this client needs structured treatment most of each day.
- **Pair to learn:** Day program, home at night → Partial hospitalization (not residential)

5 — C

Professional Practice & Ethics · Outline 1.P · Chapter 17

- **C. Weigh risks and benefits and document the rationale — Owner: ACA A.6 boundary extension (A.6.b).** Attending a graduation can be a boundary extension; the counselor considers risks and benefits and documents the reasoning beforehand when feasible.

- **A. Attend only if she agrees to end counseling first** — Owner: Premature termination (A.11.c). Graduation attendance is not a reason to terminate a client who may still benefit.
- **B. Attend and bring a gift, with no need to document** — Owner: Undocumented boundary crossing. Skipping the risk–benefit review and documentation ignores A.6.b.
- **D. Decline, since any outside contact is a violation** — Owner: Rigid avoidance (not the ACA standard). The Code distinguishes crossings from violations; not every outside contact is harmful.
- **Pair to learn:** Graduation invitation → Boundary extension, documented (ACA A.6)

6 — D

Professional Practice & Ethics · Outline 1.U · Chapter 17

- **D. Raise the concern with the colleague informally — Owner: ACA I.2.a informal resolution.** When appropriate and no substantial harm is involved, counselors first try to resolve the issue with the colleague directly.
- **A. Contact the client to ask her to remove it** — Owner: Contacting a colleague's client (confidentiality breach). Reaching out to the colleague's client bypasses the colleague and intrudes on that relationship.
- **B. Ignore it, since advertising isn't a clinical issue** — Owner: Ignoring a violation (ACA I.2). Using a current client's testimonial conflicts with C.3.b, which bars soliciting testimonials from current or former clients, so ignoring it fails the duty to address violations.
- **C. File a complaint with the state licensing board** — Owner: ACA I.2.b reporting ethical violations. Formal reporting fits when informal resolution fails or substantial harm is likely.

- **Pair to learn:** Minor colleague violation, no harm → Informal resolution first (ACA I.2.a)

7 — B

Treatment Planning · Outline 4.E · Chapter 21

- **B. Step down to standard outpatient — Owner: Step down (least restrictive level).** As the client stabilizes, care steps down to the least restrictive level that is safe and effective.
- **A. Continue IOP until all symptoms resolve** — Owner: Overly restrictive care. Staying at a higher level than needed ignores the least-restrictive principle.
- **C. Step up to partial hospitalization** — Owner: Step up (higher level of care). Stepping up is for worsening symptoms, not improvement.
- **D. Discharge with no further services** — Owner: Discharge without a plan. Ending all care abruptly skips a supported transition.
- **Pair to learn:** Client stabilizing → Step down to least restrictive level

8 — D

Counseling Skills & Interventions · Outline 5.S · Chapter 4

- **D. Adjust eye contact and posture to her comfort — Owner: Culturally adapted attending.** SOLER describes a Western norm; attending should adapt to the client's culture.
- **A. Encourage her to make more eye contact** — Owner: Rigid SOLER (Egan). Applying SOLER rigidly imposes one cultural norm on the client.
- **B. Point out the gap between her words and her gaze** — Owner: Premature confrontation. There is no real discrepancy to confront; the gaze reflects a cultural norm.
- **C. Interpret the lowered gaze as a sign of shame** — Owner: Premature interpretation. An interpretation

without evidence ignores the cultural meaning of the behavior.

- **Pair to learn:** SOLER is a Western norm → adapt attending (Egan)

9 — A

Treatment Planning · Outline 4.F · Chapter 21

- **A. A medical provider and a dietitian — Owner: Concurrent referral (eating disorder).** Eating disorders call for concurrent referral to a medical provider and dietitian while counseling continues.
- **B. A case manager for housing resources** — Owner: Concurrent referral (social services). Case management meets social needs, not eating disorder medical needs.
- **C. A substance use treatment program** — Owner: Concurrent referral (substance use). Nothing in the stem indicates substance use.
- **D. A couples therapist for relationship stress** — Owner: Concurrent referral (couples therapy). Couples work doesn't address the medical risk of purging.
- **Pair to learn:** Eating disorder in counseling → Concurrent medical and dietitian referral

10 — A

Areas of Clinical Focus · Outline 3.L · Chapter 10

- **A. Peter Lewinsohn — Owner: Lewinsohn (behavioral theory).** Lewinsohn's behavioral theory of depression is the basis for behavioral activation.
- **B. Martin Seligman** — Owner: Seligman (learned helplessness). Seligman explained depression through learning that action is useless after uncontrollable events.
- **C. Aaron Beck** — Owner: Beck (cognitive theory). Beck emphasized the cognitive triad and distorted thinking.

- **D. Susan Nolen-Hoeksema** — Owner: Nolen-Hoeksema (response styles). Nolen-Hoeksema emphasized rumination as prolonging depression.
- **Pair to learn:** Low response-contingent reinforcement → Lewinsohn

11 — B

Professional Practice & Ethics · Outline 1.A · Chapter 17

- **B. Refer the client to an appropriately trained provider — Owner: ACA A.11.a competence-based referral.** When needs exceed competence and the gap can't be closed in time, referral is the ethical response.
- **A. End services, since treating her would be unethical** — Owner: Abandonment (ACA A.12). Ending without a referral abandons the client.
- **C. Continue treatment while reading about eating disorders** — Owner: Practicing outside competence (ACA C.2.a). Self-study alone doesn't establish competence for a specialized, urgent need.
- **D. Keep her but avoid discussing eating behaviors** — Owner: Avoidance (fails client welfare). Ignoring the presenting need leaves the client untreated.
- **Pair to learn:** Needs exceed competence → Competence-based referral (ACA A.11.a)

12 — C

Treatment Planning · Outline 4.I · Chapter 21

- **C. Encourage a goodbye and process it with the whole group — Owner: Group membership transitions (4.I).** Departing members are encouraged to say goodbye, and the ending is processed with the group.
- **A. Add a new member the next week without preparation** — Owner: Unprepared membership change. New members and the group both need preparation before a transition.

- **B. Have the member end now so the group can move on** — Owner: Abrupt termination. Ending early removes the remaining sessions for closure.
- **D. Let the member leave quietly to avoid upsetting others** — Owner: Unprocessed ending. Skipping the goodbye denies the group a chance to process the loss.
- **Pair to learn:** Member leaving group → Goodbye processed with the whole group (4.I)

13 — B

Areas of Clinical Focus · Outline 3.U · Chapter 14

- **B. Dependent — Owner: Harren (decision-making styles).** The dependent style relies on others and denies personal responsibility.
- **A. Rational** — Owner: Harren (decision-making styles). The rational style is systematic and considers future consequences.
- **C. Indecisiveness** — Owner: Chronic indecisiveness (anxiety-linked). Indecisiveness is chronic difficulty deciding across life areas; he is deferring, not stuck.
- **D. Intuitive** — Owner: Harren (decision-making styles). The intuitive style relies on feelings but accepts personal responsibility.
- **Pair to learn:** Relies on others, denies responsibility → Harren's dependent style

14 — A

Counseling Skills & Interventions · Outline 5.K · Chapter 7

- **A. Enactment — Owner: Minuchin (structural).** Having family members interact in session so the pattern appears live is Minuchin's enactment.
- **B. Prescribing the symptom** — Owner: Haley / MRI (strategic). Prescribing the symptom is a paradoxical directive to interrupt a stuck pattern.

- **C. Family sculpting** — Owner: Satir (experiential). Sculpting has members physically arrange each other to show closeness and power.
- **D. Circular questioning** — Owner: Milan systemic. Circular questions ask members about each other's responses rather than having them interact.
- **Pair to learn:** Enactment → Minuchin (structural)

15 — A

Areas of Clinical Focus · Outline 3.U · Chapter 3

- **A. John Krumboltz — Owner: Krumboltz (social learning theory of career decision making).** Krumboltz held that career choice is learned through these four factors.
- **B. Anne Roe** — Owner: Roe (needs-based). Roe tied career orientation to early parent-child relationships.
- **C. John Holland** — Owner: Holland (RIASEC). Holland matched personality types to work environments.
- **D. Lent, Brown, and Hackett** — Owner: SCCT. SCCT centers on self-efficacy, outcome expectations, and personal goals.
- **Pair to learn:** Learning experiences; task-approach skills → Krumboltz

16 — C

Counseling Skills & Interventions · Outline 5.F · Chapter 5

- **C. Systematic desensitization — Owner: Wolpe.** Relaxation paired with gradual, imagined steps up a hierarchy is Wolpe's systematic desensitization.
- **A. Flooding** — Owner: Behavioral (flooding). Flooding is immediate, intense, prolonged real-life exposure without a graded hierarchy.
- **B. Exposure and response prevention** — Owner: Behavioral (ERP for OCD). ERP pairs exposure with

blocking a compulsion and is the match for obsessive-compulsive symptoms.

- **D. Implosive therapy** — Owner: Stampfl. Implosive therapy is intense, often exaggerated imagined exposure, not graded steps with relaxation.
- **Pair to learn:** Relaxation + imagined hierarchy → Wolpe (systematic desensitization)

17 — A

Counseling Skills & Interventions · Outline 5.AE · Chapter 4

- **A. "The degree matters most to you, and you've missed three weeks of class." — Owner: Constructive confrontation.** Once the alliance is established, gently naming a words-versus-actions discrepancy is appropriate confrontation.
- **B. "Why do you keep skipping if the degree is so important?"** — Owner: "Why" question (invites defensiveness). A why question invites justification instead of reflection on the discrepancy.
- **C. "You should email your professors today and catch up."** — Owner: Advice-giving (not a counseling skill). Advice shifts responsibility to the counselor and skips the client's own view of the gap.
- **D. "I wonder if skipping is a way of rebelling against your father."** — Owner: Premature interpretation. Nothing in the session supports a link to the father; interpretation needs evidence first.
- **Pair to learn:** Words vs. actions discrepancy → confrontation

18 — C

Areas of Clinical Focus · Outline 3.S · Chapter 3

- **C. Scaffolding in the zone of proximal development — Owner: Vygotsky (sociocultural development).** Support from a more knowledgeable other that is withdrawn as skill grows is Vygotsky's scaffolding within the ZPD.

- **A. Development by biological timetable** — Owner: Gesell (maturational theory). Gesell emphasized maturation unfolding on its own schedule, the opposite of socially guided learning.
- **B. Industry vs. inferiority** — Owner: Erikson (psychosocial stage 4). Industry vs. inferiority is the school-age crisis of competence, not a teaching method.
- **D. Accommodation of an existing schema** — Owner: Piaget (cognitive processes). Accommodation is Piaget's internal process of changing a schema; it doesn't describe social support that fades.
- **Pair to learn:** Scaffolding; ZPD → Vygotsky

19 — A

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **A. A unique outcome — Owner: White & Epston (narrative).** Speaking of Anger as a separate entity is externalizing, and a time the client resisted it is a narrative unique outcome.
- **B. A disputation** — Owner: Ellis (REBT). Disputing challenges an irrational belief; this question looks for evidence of the client's strength.
- **C. An exception** — Owner: de Shazer & Berg (SFBT). Exceptions are similar, but the externalized 'Anger' language marks this as narrative; the owner is decided by the word.
- **D. A miracle-day sign** — Owner: de Shazer & Berg (SFBT). The miracle question imagines a future without the problem rather than a past moment of resisting it.
- **Pair to learn:** Externalizing + unique outcomes → White & Epston (narrative)

20 — D

Intake, Assessment & Diagnosis · Outline 2.O · Chapter 19

- **D. Acknowledge it, ask permission to return to it, and offer a brief screen — Owner: Trauma assessment with care and permission (PC-PTSD-5 screening).** Trauma is assessed with care and permission, using a screen such as the PC-PTSD-5, without pushing for detail at intake.
- **A. Ask the client to describe exactly what happened, in full detail** — Owner: Pushing for trauma detail at intake. Pressing for a detailed account at intake is what the exam punishes and can flood the client before safety and rapport exist.
- **B. Skip trauma questions for now so rapport is not disrupted** — Owner: Avoiding trauma assessment. Every client should be asked about trauma, with care; skipping it misses information the client has just signaled.
- **C. Give a projective test to uncover what the client is withholding** — Owner: Projective testing (Rorschach/TAT; advanced training, weaker evidence). Projective tests have weaker reliability and validity evidence and are not a trauma screen.
- **Pair to learn:** Trauma at intake → Ask with care and permission, screen, don’t push for detail

21 — C

Areas of Clinical Focus · Outline 3.AI · Chapter 11

- **C. An independent anxiety disorder — Owner: Substance-induced vs. independent rule.** Symptoms that began before use and persisted during abstinence point to an independent disorder.
- **A. Alcohol withdrawal** — Owner: DSM-5-TR alcohol withdrawal. Withdrawal follows stopping heavy use; her attacks predate drinking entirely.

- **B. Hallucinogen persisting perception disorder** — Owner: DSM-5-TR hallucinogen-related. This involves perceptual flashbacks after hallucinogen use, not panic attacks.
- **D. Alcohol-induced anxiety disorder** — Owner: Substance-induced disorder. Substance-induced symptoms begin during or soon after use and improve with sustained abstinence.
- **Pair to learn:** Symptoms predate use and persist in abstinence → independent disorder

22 — C

Intake, Assessment & Diagnosis · Outline 2.M · Chapter 20

- **C. 105 to 115 — Owner: ±1 SEM (68% confidence band).** The true score falls within ±1 SEM about 68% of the time: 110 ± 5.
- **A. 100 to 120** — Owner: ±2 SEM (95% confidence band). ±2 SEM gives the 95% band, not the 68% band.
- **B. 110 exactly** — Owner: Single-point score (exam rewards ranges). Every observed score has error, so scores are reported as ranges, not single points.
- **D. 95 to 125** — Owner: ±3 SEM (wider than the 68% band). Three SEMs on each side is far wider than the 68% band the stem asks for.
- **Pair to learn:** True-score range around an observed score → SEM confidence band (±1 SEM = 68%)

23 — C

Counseling Skills & Interventions · Outline 5.Q · Chapter 6

- **C. Coercive control — Owner: Stark.** A pattern of monitoring, financial control, and isolation is coercive control, even without physical violence.
- **A. Enmeshment** — Owner: Minuchin (structural). Enmeshment describes diffuse family boundaries, not one partner dominating the other.

- **B. The Danger Assessment** — Owner: Campbell. The Danger Assessment is a lethality instrument, not a description of this pattern.
- **D. The cycle of violence** — Owner: Walker. Walker's cycle describes tension-building, an acute battering incident, and a honeymoon phase.
- **Pair to learn:** Coercive control → Stark

24 — A

Areas of Clinical Focus · Outline 3.AR · Chapter 13

- **A. Pragma — Owner: John Alan Lee (love styles).** Pragma is Lee's practical, logical love style.
- **B. Storge** — Owner: John Alan Lee (love styles). Storge is friendship-based love that grows slowly, not practical selection.
- **C. Quality of alternatives** — Owner: Rusbult (investment model). Rusbult's concept explains staying committed, not how a partner is chosen.
- **D. Commitment** — Owner: Sternberg (triangular theory). Commitment is a component of Sternberg's triangle, not one of Lee's styles.
- **Pair to learn:** Pragma (practical love) → Lee

25 — A

Intake, Assessment & Diagnosis · Outline 2.M · Chapter 20

- **A. 40 — Owner: T-score (mean 50, SD 10).** (85 − 100) ÷ 15 = z of −1, and a T-score one SD below 50 is 40.
- **B. 30** — Owner: T-score at z of −2 (Deviation IQ 70). A T-score of 30 matches a z of −2, which is a Wechsler score of 70, not 85.
- **C. 60** — Owner: Sign error (T of 60 is z of +1). A T-score of 60 is one SD above the mean; 85 is one SD below the Wechsler mean.

- **D. 35** — Owner: Raw-point subtraction (ignores the SD). Subtracting 15 points from 50 ignores that T-scores have an SD of 10, not 15.
- **Pair to learn:** Mean 50, SD 10 → T-score (Deviation IQ uses mean 100, SD 15)

26 — D

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **D. Beck's cognitive therapy — Owner: Beck (cognitive therapy).** Testing automatic thoughts together, like scientists, is Beck's collaborative empiricism.
- **A. Meichenbaum's cognitive behavior modification** — Owner: Meichenbaum (cognitive behavior modification). Meichenbaum trains new self-statements and coping skills, such as stress inoculation, rather than testing evidence collaboratively.
- **B. Ellis's rational emotive behavior therapy** — Owner: Ellis (REBT). Ellis disputes irrational beliefs directly and often forcefully rather than testing them as joint hypotheses.
- **C. Glasser's reality therapy** — Owner: Glasser/Wubbolding (reality therapy). Reality therapy evaluates whether current behavior gets the client what he wants (WDEP), not the evidence for a thought.
- **Pair to learn:** Collaborative empiricism → Beck (CT)

27 — B

Areas of Clinical Focus · Outline 3.L · Chapter 10

- **B. Learned helplessness — Owner: Martin Seligman.** Repeated uncontrollable bad events teaching that action is useless is Seligman's learned helplessness.
- **A. Catastrophic misinterpretation** — Owner: David Clark (panic). Catastrophic misinterpretation is misreading body sensations in panic.

- **C. Shattered assumptions** — Owner: Ronnie Janoff-Bulman. Shattered assumptions describes trauma breaking basic beliefs, not giving up after repeated uncontrollable setbacks.
- **D. Low response-contingent reinforcement** — Owner: Peter Lewinsohn. Lewinsohn's idea concerns too few rewards for activity, not a learned belief that effort can't change outcomes.
- **Pair to learn:** Learned helplessness → Seligman

28 — A

Areas of Clinical Focus · Outline 3.X · Chapter 10

- **A. Shattered assumptions — Owner: Ronnie Janoff-Bulman.** Trauma breaking basic beliefs that the world is benevolent and meaningful and that the self is worthy is Janoff-Bulman's concept.
- **B. Posttraumatic growth** — Owner: Tedeschi & Calhoun. Posttraumatic growth is positive change after struggle, which she isn't describing.
- **C. Remembrance and mourning** — Owner: Judith Herman (stage 2). This is Herman's second recovery stage, a treatment phase, not the experience of lost beliefs.
- **D. Resilience** — Owner: George Bonanno. Bonanno's resilience describes stable functioning after potential trauma, not broken worldviews.
- **Pair to learn:** Shattered assumptions → Janoff-Bulman

29 — D

Core Counseling Attributes · Outline 6.K · Chapter 22

- **D. Nonpossessive warmth — Owner: Truax & Carkhuff.** Truax and Carkhuff's nonpossessive warmth is caring that doesn't create dependence.
- **A. Cultural humility** — Owner: Hook et al. (cultural humility). Cultural humility is curiosity and openness about the limits of one's cultural knowledge.

- **B. Unconditional positive regard** — Owner: Rogers (fourth condition). UPR is Rogers' term for valuing the person without conditions; the stem asks for Truax and Carkhuff's term.
- **C. Empathic attunement** — Owner: Empathic attunement (internal sensing). Attunement is sensing the client's experience internally, not a form of caring about dependence.
- **Pair to learn:** Caring without fostering dependence → Nonpossessive warmth (Truax & Carkhuff)

30 — B

Treatment Planning · Outline 4.A · Chapter 21

- **B. Explore whether he wants anything to change — Owner: Readiness (stages of change: precontemplation).** A precontemplative client's first goal may be exploring whether change is wanted, respecting readiness and autonomy.
- **A. Confront his denial with his spouse present** — Owner: Premature confrontation. Confrontation doesn't build collaboration with a client who isn't yet considering change.
- **C. Begin relapse prevention for future slips** — Owner: Relapse prevention (termination phase). Relapse prevention fits later, after change has occurred.
- **D. Set a goal of complete abstinence in 30 days** — Owner: Counselor-set goal (conflicts with autonomy). A goal the client didn't help choose is less likely to be met.
- **Pair to learn:** Precontemplative client → Explore whether change is wanted (readiness)

31 — C

Counseling Skills & Interventions · Outline 5.AS · Chapter 8

- **C. Transition (storming) — Owner: Corey: transition / Tuckman: storming.** Conflict, struggles

for control, and challenges to the leader mark the transition stage.

- **A. Contemplation** — Owner: Prochaska & DiClemente (stages of change). Contemplation is a stage of change, never a group stage.
- **B. Working (performing)** — Owner: Corey: working / Tuckman: norming-performing. The working stage shows cohesion, trust, and direct feedback, not power struggles.
- **D. Initial (forming)** — Owner: Corey: initial / Tuckman: forming. The initial stage is polite and tentative, with members looking to the leader.
- **Pair to learn:** Challenges to the leader → Corey transition / Tuckman storming

32 — C

Professional Practice & Ethics · Outline 1.S · Chapter 17

- **C. The client's specific written consent — Owner: 42 CFR Part 2.** Federally assisted SUD program records carry extra protection and generally require specific written consent to disclose.
- **A. Only the minimum necessary, with no consent** — Owner: HIPAA minimum necessary rule. HIPAA's minimum necessary rule doesn't apply to treatment disclosures and never overrides Part 2's written-consent requirement.
- **B. A parent's signature under student records law** — Owner: FERPA. FERPA governs student education records, not SUD treatment records.
- **D. Breach notification to the client afterward** — Owner: HITECH Act. HITECH addresses breach notification for electronic records, not authorization to release.
- **Pair to learn:** SUD program records → 42 CFR Part 2 (specific written consent)

33 — D

Intake, Assessment & Diagnosis · Outline 2.M · Chapter 20

- **D. Griggs v. Duke Power Co. — Owner: Griggs v. Duke Power (1971).** Griggs held that employment tests must be job-related; tests that screen out protected groups without job relevance are discriminatory.
- **A. Debra P. v. Turlington** — Owner: Debra P. v. Turlington (1981). Debra P. held that a graduation test must cover what students were actually taught.
- **B. Diana v. State Board of Education** — Owner: Diana v. State Board of Education (1970). Diana required testing children in their primary language before special education placement.
- **C. Larry P. v. Riles** — Owner: Larry P. v. Riles (1979). Larry P. concerned biased IQ testing for special class placement of Black children in California schools, not employment.
- **Pair to learn:** Employment tests must be job-related → Griggs v. Duke Power (not Larry P.)

34 — D

Counseling Skills & Interventions · Outline 5.Q · Chapter 6

- **D. Gerald Caplan — Owner: Caplan (preventive psychiatry).** Caplan described crisis as time-limited and framed intervention as levels of prevention.
- **A. Albert Roberts** — Owner: Roberts (seven-stage model). Roberts owns the seven-stage crisis intervention model.
- **B. Kenneth Doka** — Owner: Doka (disenfranchised grief). Doka described disenfranchised grief, loss that isn’t socially recognized.
- **C. Erich Lindemann** — Owner: Lindemann (crisis intervention). Lindemann studied acute grief after the Cocoanut Grove fire and founded crisis intervention.

- **Pair to learn:** Crisis lasts about 4 to 6 weeks; prevention levels → Caplan

35 — B

Professional Practice & Ethics · Outline 1.C · Chapter 17

- **B. Veracity — Owner: Veracity (ACA principle).** Dealing truthfully with the client, even about unwelcome facts like a diagnosis on record, is veracity.
- **A. Fidelity** — Owner: Fidelity (ACA principle). Fidelity is keeping promises and commitments; no promise is being honored here.
- **C. Justice** — Owner: Justice (ACA principle). Justice concerns fairness and equitable access, not truthful disclosure.
- **D. Nonmaleficence** — Owner: Nonmaleficence (ACA principle). Nonmaleficence is avoiding harm; the defining act here is telling the truth.
- **Pair to learn:** Truthful disclosure → Veracity

36 — B

Counseling Skills & Interventions · Outline 5.P · Chapter 7

- **B. Criticism — Owner: Gottman: gentle startup.** Gentle startup is Gottman's antidote to criticism.
- **A. Stonewalling** — Owner: Gottman: physiological self-soothing. Stonewalling's antidote is physiological self-soothing.
- **C. The pursue-withdraw cycle** — Owner: Johnson (EFT). The pursue-withdraw cycle belongs to EFT, not Gottman's Four Horsemen.
- **D. Contempt** — Owner: Gottman: culture of appreciation. Contempt's antidote is building a culture of appreciation.
- **Pair to learn:** Gentle startup → antidote to criticism (Gottman)

37 — C

Intake, Assessment & Diagnosis · Outline 2.B · Chapter 19

- **C. Rule out a substance or medication cause — Owner: First's differential sequence, step 2.** After malingering and factitious disorder, the sequence rules out a substance or medication cause, which the energy drinks and stimulant make especially relevant.
- **A. Determine the specific primary disorder** — Owner: First's differential sequence, step 4. The primary disorder is determined only after substance and medical causes are ruled out.
- **B. Rule out a general medical condition** — Owner: First's differential sequence, step 3. Ruling out a medical condition comes after ruling out substances and medications.
- **D. Differentiate an adjustment disorder** — Owner: First's differential sequence, step 5. Adjustment disorder is considered late in the sequence, after the primary disorders.
- **Pair to learn:** Rule out substances before primary diagnosis → First's differential sequence, step 2

38 — C

Counseling Skills & Interventions · Outline 5.T · Chapter 5

- **C. Thomas Gordon — Owner: Gordon (Parent Effectiveness Training).** I-messages and active listening come from Gordon's Parent Effectiveness Training.
- **A. Albert Ellis** — Owner: Ellis (REBT). Ellis disputes irrational beliefs; I-messages are not his signature.
- **B. Virginia Satir** — Owner: Satir (experiential). Satir is known for communication stances and family sculpting, not I-messages.
- **D. John Gottman** — Owner: Gottman Method. Gottman is known for the Four Horsemen, softened startup, and repair attempts.

- **Pair to learn:** I-messages → Thomas Gordon

39 — B

Counseling Skills & Interventions · Outline 5.G · Chapter 9

- **B. Sharon Wegscheider-Cruse — Owner: Wegscheider-Cruse (family roles).** Wegscheider-Cruse described these roles, along with the dependent and the chief enabler.
- **A. Murray Bowen** — Owner: Bowen (multigenerational). Bowen described triangles, differentiation, and sibling position, not these roles.
- **C. Virginia Satir** — Owner: Satir (communication stances). Satir described communication stances: placater, blamer, super-reasonable, irrelevant.
- **D. E. M. Jellinek** — Owner: Jellinek (disease model). Jellinek described phases of alcoholism within the disease model.
- **Pair to learn:** Hero, scapegoat, lost child, mascot → Wegscheider-Cruse

40 — C

Counseling Skills & Interventions · Outline 5.G · Chapter 9

- **C. SMART Recovery — Owner: SMART Recovery (CBT-based mutual support).** SMART Recovery is a CBT-based, non-12-step mutual-support option.
- **A. Alcoholics Anonymous** — Owner: AA (12-step). AA is a 12-step program, which the client doesn't want.
- **B. Al-Anon** — Owner: Al-Anon (family support). Al-Anon serves family and friends of people with alcohol problems.
- **D. Adult Children of Alcoholics** — Owner: ACA/ACoA (adult children). This group serves adults who grew up with alcoholic or dysfunctional caregivers.
- **Pair to learn:** CBT-based, non-12-step → SMART Recovery

41 — B

Areas of Clinical Focus · Outline 3.AM · Chapter 10

- **B. Thomas Borkovec — Owner: Borkovec (avoidance theory of worry).** Borkovec's avoidance theory holds that worry is maintained because it reduces arousal.
- **A. Adrian Wells** — Owner: Wells (metacognitive model). Wells emphasized beliefs about worry, such as worry being useful or uncontrollable.
- **C. Michel Dugas** — Owner: Dugas (intolerance of uncertainty). Dugas emphasized difficulty tolerating uncertainty in GAD.
- **D. David Clark** — Owner: Clark (cognitive model of panic). Clark explained panic through catastrophic misinterpretation of body sensations.
- **Pair to learn:** Worry as avoidance of arousal → Borkovec

42 — D

Areas of Clinical Focus · Outline 3.K · Chapter 12

- **D. Disenfranchised grief — Owner: Kenneth Doka.** A loss that others don't recognize as legitimate, such as an ex-partner or a death by overdose, is Doka's disenfranchised grief.
- **A. Anticipatory grief** — Owner: Erich Lindemann. Anticipatory grief occurs before an expected death, not after a sudden one.
- **B. Ambiguous loss** — Owner: Pauline Boss. Ambiguous loss lacks clarity about whether the person is gone; this death is clear.
- **C. Prolonged grief disorder** — Owner: DSM-5-TR. Prolonged grief disorder requires at least 12 months of impairing grief in adults; nothing here suggests that.
- **Pair to learn:** Disenfranchised grief → Doka

43 — A

Areas of Clinical Focus · Outline 3.X · Chapter 10

- **A. Full criteria aren't met until at least 6 months after the event — Owner: DSM-5-TR PTSD specifier.** Delayed expression means full PTSD criteria are not met until at least 6 months after the trauma.
- **B. Symptoms begin within 3 months of a stressor** — Owner: DSM-5-TR adjustment disorder. Onset within 3 months is the adjustment disorder rule.
- **C. Symptoms occur from 3 days to 1 month after the event** — Owner: DSM-5-TR acute stress disorder. That window defines acute stress disorder.
- **D. Depersonalization or derealization is present** — Owner: DSM-5-TR dissociative specifier. That describes the "with dissociative symptoms" specifier.
- **Pair to learn:** Delayed expression → criteria met 6+ months after trauma

44 — B

Counseling Skills & Interventions · Outline 5.AT · Chapter 8

- **B. The social microcosm — Owner: Yalom: social microcosm.** The group becomes a social microcosm, making outside patterns available to work on directly.
- **A. Corrective recapitulation of the family** — Owner: Yalom: corrective recapitulation of the primary family group. This factor is re-experiencing family-like dynamics and responding differently, not interpersonal patterns in general.
- **C. Universality** — Owner: Yalom: universality. Universality is recognizing that others share one's struggles.
- **D. Catharsis** — Owner: Yalom: catharsis. Catharsis is emotional release, most useful when paired with understanding.
- **Pair to learn:** Outside patterns appear in group → social microcosm (Yalom)

45 — B

Counseling Skills & Interventions · Outline 5.H · Chapter 9

- **B. A microinvalidation — Owner: Sue (microaggressions).** Statements like 'I don't see color' negate the client's experience, which is a microinvalidation.
- **A. A microinsult** — Owner: Sue (microaggressions). A microinsult is subtle rudeness that demeans identity, such as surprise at a client's ability.
- **C. Broaching** — Owner: Day-Vines (broaching). Broaching raises culture directly; this statement avoids it.
- **D. A microassault** — Owner: Sue (microaggressions). A microassault is explicit, often conscious, derogatory action such as a slur.
- **Pair to learn:** "I don't see color" → microinvalidation (Sue)

46 — B

Core Counseling Attributes · Outline 6.J · Chapter 22

- **B. "You've carried this alone a long time. What's the shame like now?" — Owner: Nonjudgmental stance (accepts the person, explores experience).** It accepts the person and explores his experience without moral evaluation.
- **A. "Selling drugs harms whole communities, so that shame makes sense."** — Owner: Moralizing (judgment, not acceptance). Lecturing about the harm shows moral judgment, which the exam punishes.
- **C. "That was ages ago. You really shouldn't feel bad about it now."** — Owner: Minimizing reassurance. Minimizing dismisses the shame the client just disclosed instead of exploring it.
- **D. "Let's set that aside and talk about how your week went instead."** — Owner: Avoidance (changing

the subject). Changing the subject signals discomfort or disapproval and leaves the disclosure unexplored.

- **Pair to learn:** Accepts the person, explores the behavior → Nonjudgmental stance (not moralizing)

47 — C

Counseling Skills & Interventions · Outline 5.AU · Chapter 8

- **C. Step back and help members turn insight into action — Owner: Corey: working-stage leader role.** In the working stage the leader lets members lead more and reinforces translating insight into action.
- **A. Explain the group rules and provide more structure** — Owner: Corey: initial-stage leader role. Heavy structure and orientation fit the initial stage.
- **B. Rescreen members to confirm fit** — Owner: Pregroup stage (ACA A.9.a). Screening happens before the group begins.
- **D. Focus on consolidating learning and saying goodbye** — Owner: Corey: final-stage leader role. Consolidation and goodbyes belong to the final stage.
- **Pair to learn:** Cohesion and risk-taking → Corey working stage

48 — A

Areas of Clinical Focus · Outline 3.AM · Chapter 10

- **A. Intolerance of uncertainty — Owner: Michel Dugas.** Finding uncertainty itself especially hard to tolerate is the core of Dugas's model of GAD.
- **B. Salkovskis's inflated responsibility** — Owner: Paul Salkovskis (OCD). Inflated responsibility concerns believing one must prevent harm, a driver of OCD compulsions.
- **C. Clark and Wells's safety behaviors** — Owner: Clark & Wells (social anxiety). Safety behaviors and self-focused attention maintain social anxiety, not distress about not knowing.

- **D. Wells's metacognitive model** — Owner: Adrian Wells. Wells's model targets beliefs about worry itself, which she isn't expressing.
- **Pair to learn:** Intolerance of uncertainty → Dugas

49 — C

Areas of Clinical Focus · Outline 3.S · Chapter 3

- **C. Carol Gilligan — Owner: Gilligan (ethic of care).** Gilligan's ethic of care sequence, from In a Different Voice, ends in care for both self and others.
- **A. Jean Piaget** — Owner: Piaget (cognitive development). Piaget's stages describe cognitive development, not a care-based moral sequence.
- **B. Robert Havighurst** — Owner: Havighurst (developmental tasks). Havighurst described tasks to master at each life stage, not stages of moral orientation.
- **D. Lawrence Kohlberg** — Owner: Kohlberg (moral reasoning stages). Kohlberg's levels are preconventional, conventional, and postconventional, judged by justice rather than care.
- **Pair to learn:** Ethic of care → Gilligan

50 — C

Treatment Planning · Outline 4.D · Chapter 21

- **C. Dennis Saleebey — Owner: Saleebey (strengths perspective).** Saleebey is the owner of the strengths perspective in social work.
- **A. Gerald Caplan** — Owner: Caplan (crisis, preventive psychiatry). Caplan is associated with crisis theory and levels of prevention.
- **B. Abraham Maslow** — Owner: Maslow (hierarchy of needs). Maslow's hierarchy guides sequencing basic needs before growth.
- **D. Saul Rosenzweig** — Owner: Rosenzweig (common factors, Dodo bird verdict). Rosenzweig introduced the common factors idea, not the strengths perspective.

- **Pair to learn:** Strengths perspective → Saleebey

51 — C

Counseling Skills & Interventions · Outline 5.AI · Chapter 8

- **C. Open the question to other members — Owner: Managing dependency (leader-member dynamics).** Dependency is handled by redirecting to members and encouraging self-reliance.
- **A. Block the member to protect the group** — Owner: Misapplied blocking. Blocking is for harmful behavior such as attacks; seeking approval isn't harmful to others.
- **B. Universalize the shared theme of loneliness** — Owner: Misapplied universalizing. Naming a theme no one has raised doesn't address the member's reliance on the leader.
- **D. Give a clear answer to model expertise** — Owner: Advice-giving. Answering reinforces dependence on the leader and skips member interaction.
- **Pair to learn:** Dependency on leader → redirect to members

52 — A

Intake, Assessment & Diagnosis · Outline 2.P · Chapter 19

- **A. AUDIT — Owner: AUDIT (alcohol use screening).** The AUDIT is a substance use screen listed among the tools (with CAGE, CRAFFT, and DAST) for asking every client about substance use.
- **B. GAD-7** — Owner: GAD-7 (anxiety screen). The GAD-7 screens anxiety symptoms, not substance use.
- **C. PCL-5** — Owner: PCL-5 (trauma symptom measure). The PCL-5 screens trauma symptoms, not alcohol use.
- **D. CBCL** — Owner: CBCL (Achenbach; child behavior ratings). The CBCL is a parent, teacher, and youth behavior rating scale for children, not an adult alcohol screen.

- **Pair to learn:** Alcohol screen for every client → AUDIT (not PCL-5)

53 — C

Areas of Clinical Focus · Outline 3.AX · Chapter 10

- **C. Gross's process model — Owner: James Gross (emotion regulation).** Gross described emotion regulation as occurring at successive points in the emotion process, from situation selection to response modulation.
- **A. Linehan's biosocial theory** — Owner: Marsha Linehan (DBT). Linehan's theory explains how dysregulation develops from emotional vulnerability plus an invalidating environment.
- **B. Porges's polyvagal theory** — Owner: Stephen Porges. Polyvagal theory concerns the autonomic nervous system, not a sequence of regulation strategies.
- **D. Siegel's window of tolerance** — Owner: Daniel Siegel. The window of tolerance is the arousal zone in which a person can function.
- **Pair to learn:** Process model of emotion regulation → Gross

54 — B

Treatment Planning · Outline 4.C · Chapter 21

- **B. Address the barrier, such as offering telehealth — Owner: Barriers (4.C).** Transportation is a barrier; naming it and adapting the plan, for example with telehealth, supports goal attainment.
- **A. Document the client as resistant to treatment** — Owner: Labeling (deficit language). Calling a transportation problem resistance mislabels a barrier as a client trait.
- **C. Increase session frequency to rebuild momentum** — Owner: Intensifying the plan. More sessions add to the same transportation problem.

- **D. Terminate services for repeated no-shows** — Owner: Premature termination (abandonment risk). Ending services ignores a solvable barrier and leaves the client without care.
- **Pair to learn:** Lack of transportation → Barrier to address (4.C)

55 — D

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **D. Person-centered therapy — Owner: Rogers (person-centered).** Conditions of worth and incongruence are Rogers' explanation of distress; the core conditions restore congruence.
- **A. Hierarchy of needs** — Owner: Maslow (needs theory). Maslow explains blocked growth through unmet lower needs, and his is a motivational theory, not a therapy.
- **B. Individual psychology** — Owner: Adler (individual psychology). Adler locates the problem in discouragement and mistaken beliefs about how to belong.
- **C. Gestalt therapy** — Owner: Perls (Gestalt). Gestalt locates the problem in blocked awareness and unfinished business.
- **Pair to learn:** Conditions of worth → Rogers

56 — B

Areas of Clinical Focus · Outline 3.H · Chapter 10

- **B. Clark's cognitive model of panic — Owner: David Clark.** Catastrophically misinterpreting normal body sensations is the core of Clark's panic model.
- **A. Seligman's preparedness** — Owner: Martin Seligman. Preparedness explains why some objects, like snakes, are feared easily.
- **C. Mowrer's two-factor theory** — Owner: O. Hobart Mowrer. Two-factor theory explains how fears are

acquired and maintained by conditioning, not how a body sensation is misread.

- **D. Borkovec's avoidance theory** — Owner: Thomas Borkovec (worry). Borkovec's theory explains GAD worry, not sudden panic from body sensations.
- **Pair to learn:** Catastrophic misinterpretation → Clark

57 — A

Professional Practice & Ethics · Outline 1.K · Chapter 17

- **A. Privileged communication — Owner: Privileged communication (legal; client holds it).** Privilege is a legal protection from disclosure in court, and the client is its holder.
- **B. Privacy** — Owner: Privacy (constitutional and legal right of the person). Privacy is the broad right to decide what personal information is shared, not a court-specific protection.
- **C. Minimal disclosure** — Owner: ACA B.2.e minimal disclosure. Minimal disclosure limits what is shared once disclosure is required; it is not a right the client holds.
- **D. Confidentiality** — Owner: Confidentiality (ethical duty of the counselor). Confidentiality is the counselor's ethical duty not to disclose, not a courtroom right the client waives.
- **Pair to learn:** Client-held legal right in court → Privileged communication

58 — A

Professional Practice & Ethics · Outline 1.T · Chapter 17

- **A. Provide pretermination counseling and arrange transfers — Owner: ACA A.11.c and A.11.d appropriate termination and transfer.** Counselors provide pretermination counseling, recommend other providers, and coordinate transfers so care continues.
- **B. Give clients a phone directory of local counselors** — Owner: Referral without follow-through.

A list alone lacks the clinical and administrative transfer process A.11.d expects.

- **C. Keep seeing clients by video from the new state** — Owner: Unauthorized telehealth (licensure follows the client). Practicing across state lines requires authorization where clients are located.
- **D. Notify clients by letter in the final week** — Owner: Abrupt termination (abandonment, A.12). Last-minute notice leaves clients without time to process or connect with new care.
- **Pair to learn:** Counselor relocating → Pretermination counseling and transfer (A.11.c, A.11.d)

59 — C

Counseling Skills & Interventions · Outline 5.AF · Chapter 4

- **C. "Right now I notice you watching the clock as we talk. What's that like?" — Owner: Immediacy.** It addresses what is happening between counselor and client in the present moment.
- **A. "You've mentioned being tired, being busy, and wanting to leave early."** — Owner: Summary. It ties together several statements but doesn't name the dynamic between them.
- **B. "Maybe you're bored because you resent being told what to do."** — Owner: Interpretation. It proposes a hidden cause, offered without evidence or tentativeness.
- **D. "When I was in school, I felt restless in long meetings too."** — Owner: Self-disclosure. It shares the counselor's own past experience rather than the here-and-now relationship.
- **Pair to learn:** Counselor-client relationship right now → immediacy

60 — B

Counseling Skills & Interventions · Outline 5.L · Chapter 9

- **B. Seek supervision and keep serving her competently — Owner: ACA A.4.b and A.11.b.** Counselors avoid imposing values, don't refer solely over personal values, and use supervision to manage the conflict.
- **A. Refer her because of the value conflict** — Owner: Values-based referral (prohibited, ACA A.11.b). Referring based solely on the counselor's personal values is prohibited.
- **C. Share her faith views so the client can decide** — Owner: Values imposition (ACA A.4.b). Disclosing personal beliefs burdens the client and pressures her choice.
- **D. Urge her to reconsider for the children's sake** — Owner: Values imposition (ACA A.4.b). Steering the client toward the counselor's values is imposition.
- **Pair to learn:** Values conflict → supervision, not referral (ACA A.11.b)

61 — C

Treatment Planning · Outline 4.H · Chapter 21

- **C. Forced — Owner: Forced termination.** External limits, such as the end of a training placement, create forced termination.
- **A. Premature** — Owner: Premature termination (dropout). Premature termination is a client stopping before goals are met.
- **B. Planned** — Owner: Planned/natural termination. Planned termination happens when goals are met and is discussed in advance.
- **D. Client-initiated** — Owner: Client-initiated termination. Here the ending comes from the placement, not the client's decision.

- **Pair to learn:** End of training placement → Forced termination

62 — B

Core Counseling Attributes · Outline 6.H · Chapter 22

- **B. The client perceiving empathy and regard — Owner: Rogers' sixth condition.** The sixth condition requires that the client perceive the counselor's empathy and positive regard; feeling and expressing them isn't enough.
- **A. The counselor's own congruence** — Owner: Rogers' third condition. The third condition is counselor congruence; nothing suggests the counselor is pretending.
- **C. Psychological contact between the two** — Owner: Rogers' first condition. The first condition is psychological contact, which five ongoing sessions of dialogue show is present.
- **D. The counselor experiencing empathic understanding** — Owner: Rogers' fifth condition. The counselor does experience and try to communicate empathy; what is missing is the client's perception of it.
- **Pair to learn:** Client must perceive empathy and UPR → Rogers' sixth condition

63 — B

Professional Practice & Ethics · Outline 1.C · Chapter 17

- **B. Duty, dereliction, direct causation, damages — Owner: Malpractice (the four Ds).** Malpractice requires a professional duty, a breach of the standard of care, a causal link, and actual harm.
- **A. Respect for persons, beneficence, justice** — Owner: Belmont Report principles. These are research ethics principles from the Belmont Report.
- **C. Justice, publicity, and universality** — Owner: Stadler's three tests. These are tests for checking a

chosen ethical action, not legal elements of malpractice.

- **D. Serious threat, identifiable victim, credibility** — Owner: Duty to protect triggers (Tarasoff). These trigger the duty to protect; they aren't elements of a malpractice claim.
- **Pair to learn:** Duty, dereliction, direct causation, damages → Malpractice

64 — D

Professional Practice & Ethics · Outline 1.D · Chapter 17

- **D. Decline; personal virtual relationships are prohibited — Owner: ACA A.5.e personal virtual relationships.** The ACA Code prohibits personal virtual relationships, such as social media friendships, with current clients.
- **A. Accept after documenting it as a boundary extension** — Owner: ACA A.6 boundary extension. Boundary extensions cover events like a graduation; they don't permit a prohibited relationship.
- **B. Refer her to another counselor to avoid conflict** — Owner: Unnecessary referral (abandonment risk, A. 12). Declining the request resolves the issue; ending the relationship is not needed.
- **C. Accept, but avoid commenting on her posts** — Owner: Boundary violation (A.5.e). Accepting creates the very personal virtual relationship the Code prohibits.
- **Pair to learn:** Social media friendship with a current client → Prohibited (ACA A.5.e)

65 — A

Core Counseling Attributes · Outline 6.A · Chapter 22

- **A. Reflect on her own grief and bring it to supervision — Owner: Self-awareness: managing countertransference through supervision.** When a counselor notices a strong personal reaction, the

scored answer is to reflect and seek supervision so the reaction doesn't steer the session.

- **B. Tell the client about her father so he feels understood** — Owner: Self-disclosure that meets the counselor's needs. Disclosing her own loss here serves her needs and shifts the focus to the counselor.
- **C. Refer the client to a colleague right away** — Owner: Premature referral. Immediate referral is premature; countertransference is first managed through reflection and supervision.
- **D. Keep steering sessions toward his work stress** — Owner: Avoidance (unmanaged countertransference). Continuing to avoid the topic lets unmanaged countertransference harm the client's work.
- **Pair to learn:** Counselor's own reaction steering the session → Self-awareness: reflect and seek supervision

66 — C

Intake, Assessment & Diagnosis · Outline 2.F · Chapter 19

- **C. Tangential — Owner: Tangential thought process.** Tangential thinking wanders away from the question and never returns to the point.
- **A. Circumstantial** — Owner: Circumstantial thought process. Circumstantial thinking wanders but eventually reaches the point; this client never does.
- **B. Linear and goal-directed** — Owner: Linear/goal-directed thought process. Linear, goal-directed thinking moves in an organized way toward the answer, which is the opposite of what is described.
- **D. Loose associations** — Owner: Loose associations (no logical connection). Loose associations have no logical connection between ideas, but here each topic follows from the one before.
- **Pair to learn:** Never gets to the point → Tangential (not circumstantial)

67 — D

Areas of Clinical Focus · Outline 3.S · Chapter 3

- **D. Neglectful/uninvolved — Owner: Maccoby & Martin.** Baumrind described three styles; Maccoby and Martin added the low-warmth, low-firmness neglectful style.
- **A. Permissive** — Owner: Baumrind (parenting styles). Permissive is one of Baumrind's original three styles.
- **B. Authoritative** — Owner: Baumrind (parenting styles). Authoritative is one of Baumrind's original three styles.
- **C. Slow-to-warm-up** — Owner: Thomas & Chess (temperament). Slow-to-warm-up is a child temperament category, not a parenting style.
- **Pair to learn:** Neglectful/uninvolved style → Maccoby & Martin

68 — A

Areas of Clinical Focus · Outline 3.AL · Chapter 16

- **A. Schizophreniform disorder — Owner: DSM-5-TR (1 to under 6 months).** Schizophreniform disorder has the same symptoms as schizophrenia, lasting 1 month to under 6 months.
- **B. Delusional disorder** — Owner: DSM-5-TR (delusions, 1+ month). Delusional disorder involves delusions without other prominent psychotic symptoms.
- **C. Brief psychotic disorder** — Owner: DSM-5-TR (1 day to under 1 month). Brief psychotic disorder lasts at least 1 day but less than 1 month.
- **D. Schizophrenia** — Owner: DSM-5-TR (continuous signs 6+ months). Schizophrenia requires continuous signs for at least 6 months.
- **Pair to learn:** 1 to under 6 months → schizophreniform

69 — D

Areas of Clinical Focus · Outline 3.L · Chapter 10

- **D. A manic episode — Owner: DSM-5-TR (1 week, or any duration if hospitalized).** Mania normally requires 1 week, but any duration counts when hospitalization is needed.
- **A. Cyclothymic disorder** — Owner: DSM-5-TR (2 years of subthreshold symptoms). Cyclothymia involves 2 years of symptoms that don't meet full episode criteria.
- **B. A hypomanic episode** — Owner: DSM-5-TR (4 days, no hospitalization). Hypomania can't involve hospitalization or marked impairment, and it requires 4 days.
- **C. Brief psychotic disorder** — Owner: DSM-5-TR (psychotic disorders). Brief psychotic disorder involves psychotic symptoms, not a mood episode of elevated energy and grandiosity.
- **Pair to learn:** Hospitalization → manic episode regardless of duration

70 — A

Counseling Skills & Interventions · Outline 5.V · Chapter 6

- **A. People and places that provide distraction — Owner: Stanley & Brown: step 3.** Contacts and settings that help without discussing the crisis are the distraction step.
- **B. Making the environment safe from lethal means** — Owner: Stanley & Brown: step 6. That step reduces access to lethal means.
- **C. People I can ask for help with the crisis** — Owner: Stanley & Brown: step 4. That step lists people who can be told about the crisis, which the client isn't planning to do here.

- **D. Internal coping strategies I can use alone** — Owner: Stanley & Brown: step 2. Internal coping strategies are things the client does alone.
- **Pair to learn:** Distraction without disclosing → Stanley & Brown step 3

71 — A

Professional Practice & Ethics · Outline 1.W · Chapter 17

- **A. Add a dated correction to the record — Owner: Correcting records (never alter the original).** Errors are corrected with a dated addition; the original entry is never altered or destroyed.
- **B. Leave it, since the next note will be accurate** — Owner: Inaccurate documentation (ACA A.1.b). Records must be accurate; leaving a known error uncorrected falls short.
- **C. Move the correction into the psychotherapy notes** — Owner: HIPAA psychotherapy notes. Psychotherapy notes hold the counselor's process analysis, not corrections to the clinical record.
- **D. Rewrite the note so the error no longer appears** — Owner: Altering records (punished on the exam). Changing the original entry is altering a record, which is prohibited.
- **Pair to learn:** Error in a note → Dated correction, never alter the original

72 — C

Areas of Clinical Focus · Outline 3.AI · Chapter 11

- **C. E. M. Jellinek — Owner: Jellinek (disease concept of alcoholism).** Jellinek described these phases as part of the disease concept of alcoholism.
- **A. Alan Marlatt** — Owner: Marlatt (relapse prevention). Marlatt is known for relapse prevention and the abstinence violation effect.
- **B. Prochaska & DiClemente** — Owner: Stages of change. Their stages run from precontemplation to maintenance and apply to any behavior change.

- **D. Sharon Wegscheider-Cruse** — Owner: Wegscheider-Cruse (family roles). Wegscheider-Cruse described family roles such as hero and scapegoat.
- **Pair to learn:** Prealcoholic → chronic phases → Jellinek

73 — D

Areas of Clinical Focus · Outline 3.L · Chapter 10

- **D. Persistent depressive disorder — Owner: DSM-5-TR (1 year in youth).** In children and adolescents, persistent depressive disorder requires 1 year of depressed or irritable mood plus at least 2 other symptoms.
- **A. Disruptive mood dysregulation disorder** — Owner: DSM-5-TR (DMDD). DMDD requires severe temper outbursts 3 or more times a week, which he doesn't have.
- **B. Cyclothymic disorder** — Owner: DSM-5-TR. Cyclothymia requires periods of hypomanic symptoms as well as depressive ones.
- **C. Adjustment disorder with depressed mood** — Owner: DSM-5-TR. Adjustment disorder requires an identifiable stressor and doesn't apply when another disorder's criteria are met.
- **Pair to learn:** Persistent depressive disorder in youth → 1 year (2 years adults)

74 — D

Areas of Clinical Focus · Outline 3.AA · Chapter 15

- **D. Extrinsic religiosity — Owner: Gordon Allport.** Using faith as a means, such as for social standing, is Allport's extrinsic religiosity.
- **A. Intrinsic religiosity** — Owner: Gordon Allport. Intrinsic religiosity treats faith as an end in itself, lived as the center of life.
- **B. Spiritual bypassing** — Owner: John Welwood. Spiritual bypassing uses spiritual ideas to avoid emotional pain.

- **C. Negative religious coping** — Owner: Kenneth Pargament. Negative religious coping involves feeling punished or abandoned by God.
- **Pair to learn:** Faith as a means → extrinsic religiosity (Allport)

75 — B

Areas of Clinical Focus · Outline 3.U · Chapter 3

- **B. Anne Roe — Owner: Roe (personality/needs-based).** Roe held that early parent-child relationships shape orientation toward or away from people.
- **A. Linda Gottfredson** — Owner: Gottfredson (circumscription). Gottfredson emphasized ruling out options by gender and prestige.
- **C. John Krumboltz** — Owner: Krumboltz (social learning). Krumboltz emphasized learning experiences and happenstance, not parental warmth.
- **D. Donald Super** — Owner: Super (vocational self-concept). Super emphasized implementing self-concept across life roles, not childhood warmth.
- **Pair to learn:** Parent-child relationships shape career → Roe

76 — D

Intake, Assessment & Diagnosis · Outline 2.S · Chapter 6

- **D. A history of violence — Owner: History of violence (strongest violence predictor).** A history of violence is the strongest predictor of future violence.
- **A. Recent substance use** — Owner: Substance use (a violence risk factor). Substance use is one factor to ask about, but history of violence is the strongest single predictor.
- **B. A previous suicide attempt** — Owner: Prior attempt (strongest suicide-attempt predictor). A prior attempt is the strongest single predictor of a future suicide attempt, not of violence toward others.

- **C. Hopelessness about the future** — Owner: Beck: hopelessness predicts suicide. Beck identified hopelessness as a strong predictor of suicide, not of violence toward others.
- **Pair to learn:** Strongest predictor of future violence → History of violence (not prior suicide attempt)

77 — D

Areas of Clinical Focus · Outline 3.A · Chapter 16

- **D. The health belief model — Owner: Rosenstock and colleagues.** The health belief model explains health behavior through these perceptions, with self-efficacy added later.
- **A. The transactional model of stress** — Owner: Lazarus & Folkman. The transactional model centers on primary and secondary appraisal.
- **B. The adaptive tasks of illness** — Owner: Moos & Holahan. Moos and Holahan described tasks such as managing symptoms and preserving relationships.
- **C. The biopsychosocial model** — Owner: George Engel. Engel's model says illness involves biological, psychological, and social factors.
- **Pair to learn:** Susceptibility, severity, benefits, barriers, cues → health belief model (Rosenstock)

78 — B

Areas of Clinical Focus · Outline 3.AN · Chapter 13

- **B. Silverstein and Kaplan's core issues in adoption — Owner: Silverstein & Kaplan.** These seven issues can affect all members of the adoption triad across the life span.
- **A. Worden's tasks of mourning** — Owner: J. William Worden. Worden's four tasks concern mourning a death.
- **C. Rando's six R processes** — Owner: Therese Rando. Rando's processes begin with recognizing the loss and end with reinvesting.

- **D. Finkelhor and Browne's traumagenic dynamics** — Owner: Finkelhor & Browne. Their four dynamics explain the harm of child sexual abuse.
- **Pair to learn:** Seven core issues in adoption → Silverstein & Kaplan

79 — C

Areas of Clinical Focus · Outline 3.AI · Chapter 11

- **C. The family hero role — Owner: Wegscheider-Cruse (family roles).** The hero overachieves and brings outside validation to the family in Wegscheider-Cruse's roles.
- **A. The abstinence violation effect** — Owner: Alan Marlatt. This is a relapse concept about guilt after a lapse, not a family role.
- **B. "Don't talk, don't trust, don't feel"** — Owner: Claudia Black (family rules). Black's rules describe the family's unspoken norms, not one child's role.
- **D. The crucial phase** — Owner: E. M. Jellinek. Jellinek's crucial phase describes the drinker's progression, not a child's role.
- **Pair to learn:** Hero role → Wegscheider-Cruse

80 — D

Professional Practice & Ethics · Outline 1.B · Chapter 18

- **D. Formative evaluation — Owner: Formative evaluation.** Ongoing evaluation during a program, done to improve it, is formative.
- **A. Cost-benefit analysis** — Owner: Cost-benefit analysis. Cost-benefit analysis compares costs with benefits measured in dollars.
- **B. Summative evaluation** — Owner: Summative evaluation. Summative evaluation judges effectiveness at the end.
- **C. Needs assessment** — Owner: Needs assessment. A needs assessment identifies gaps before a program is designed.

- **Pair to learn:** Evaluate during a program to improve it → Formative evaluation

81 — B

Core Counseling Attributes · Outline 6.L · Chapter 22

- **B. Explore how her faith shapes the choice, without imposing values — Owner: Not imposing values (ACA A.4.b).** Respect for diversity means exploring the client's own values and meaning without imposing the counselor's.
- **A. Suggest she talk to her clergy rather than to a counselor** — Owner: Deflecting the client's concern. Sending her elsewhere avoids the counselor's own task of respecting and working with her values.
- **C. Share the counselor's view that leaving would be healthier** — Owner: Imposing counselor values. Steering the client toward the counselor's preference imposes values, which ACA A.4.b prohibits.
- **D. Point out the research on outcomes of unhappy marriages** — Owner: Persuasion disguised as information-giving. Using information to push the counselor's preferred outcome still imposes values.
- **Pair to learn:** Client's values differ from the counselor's → Don't impose values (ACA A.4.b)

82 — A

Areas of Clinical Focus · Outline 3.AS · Chapter 13

- **A. Judith Wallerstein — Owner: Wallerstein (clinical sample).** Wallerstein's more pessimistic view, from a smaller clinical sample, includes the sleeper effect.
- **B. Paul Amato** — Owner: Amato. Amato emphasized parental conflict as a stronger predictor than divorce itself.
- **C. Paul Bohannan** — Owner: Bohannan (six stations). Bohannan described six stations of divorce, not long-term effects on children.

- **D. E. Mavis Hetherington** — Owner: Hetherington. Hetherington found that most children of divorce adjust well within about two years.
- **Pair to learn:** Sleeper effect → Wallerstein

83 — D

Professional Practice & Ethics · Outline 1.H · Chapter 17

- **D. Its monetary value and the client's motivation — Owner: ACA A.10.f gifts.** For gifts, counselors weigh the relationship, monetary value, and both parties' motivations; small, culturally meaningful gifts may be accepted.
- **A. Whether the agency allows self-referral** — Owner: ACA A.10.a self-referral. Self-referral concerns sending agency clients to a private practice.
- **B. Whether a five-year waiting period has passed** — Owner: ACA A.5.c former clients. The five-year rule governs romantic relationships with former clients, not gifts.
- **C. Whether a written barter contract was signed** — Owner: ACA A.10.e bartering. Bartering exchanges goods or services for counseling; a farewell gift is not payment.
- **Pair to learn:** Client gift → ACA A.10.f (value, motivation, relationship)

84 — C

Counseling Skills & Interventions · Outline 5.P · Chapter 7

- **C. Integrative behavioral couple therapy — Owner: Christensen & Jacobson (IBCT).** IBCT combines change strategies with acceptance strategies such as unified detachment and tolerance building.
- **A. Emotionally focused therapy** — Owner: Johnson (EFT). EFT de-escalates the pursue-withdraw cycle and accesses primary attachment emotions.

- **B. The Gottman Method** — Owner: Gottman. Gottman targets the Four Horsemen, repair, and the couple's friendship.
- **D. The speaker-listener technique** — Owner: PREP (Markman, Stanley, & Blumberg). This structured exercise has one partner paraphrase before replying; it isn't an acceptance model.
- **Pair to learn:** Acceptance plus change → IBCT (Christensen & Jacobson)

85 — A

Treatment Planning · Outline 4.A · Chapter 21

- **A. de Shazer (solution-focused) — Owner: de Shazer (well-formed goals, SFBT).** de Shazer's well-formed goal criteria match the stem exactly.
- **B. Prochaska and DiClemente** — Owner: Prochaska & DiClemente (stages of change). The stages of change describe readiness, not how to word a goal.
- **C. Doran (SMART objectives)** — Owner: SMART objectives (Doran). SMART stresses specific, measurable, achievable, relevant, time-bound, not presence over absence.
- **D. Saleebey (strengths perspective)** — Owner: Saleebey (strengths perspective). Saleebey's strengths perspective builds on client assets, not goal-wording criteria.
- **Pair to learn:** Small, concrete, presence of something → Well-formed goal (de Shazer)

86 — C

Counseling Skills & Interventions · Outline 5.AA · Chapter 4

- **C. Summarizing — Owner: Summary (Ivey: basic listening sequence).** Tying together several statements or themes is a summary.
- **A. Reflecting meaning** — Owner: Reflection of meaning. Reflection of meaning names the value underneath; no value is named here.

- **B. Paraphrasing** — Owner: Paraphrase. A paraphrase restates one statement, not several themes.
- **D. Clarifying** — Owner: Clarification. Clarification asks about an unclear message; this states an understanding.
- **Pair to learn:** Several statements tied together → summary

87 — C

Professional Practice & Ethics · Outline 1.L · Chapter 17

- **C. Explain what information will be shared with probation — Owner: ACA A.2.e mandated clients.** Mandated clients must know, before services begin, what will be shared with the referral source, and that they may refuse with consequences explained.
- **A. Obtain his assent while probation gives consent** — Owner: Assent (minors and clients unable to consent, A.2.d). Assent paired with another party's consent applies to minors or clients lacking capacity, not a competent adult.
- **B. Promise that sessions will stay fully confidential** — Owner: Overpromising confidentiality. Information will be shared with probation, so a promise of full confidentiality is false.
- **D. Wait to discuss reporting until probation requests it** — Owner: Delayed informed consent. Consent is required before counseling begins, not after a report is requested.
- **Pair to learn:** Court or probation referral → ACA A.2.e (explain what is shared first)

88 — A

Areas of Clinical Focus · Outline 3.B · Chapter 12

- **A. Selective optimization with compensation — Owner: Paul & Margret Baltes.** Choosing what matters (selection), practicing it (optimization), and using strategies to offset losses (compensation) is the SOC model.

- **B. Gerotranscendence** — Owner: Lars Tornstam. Gerotranscendence is a shift toward a cosmic, less materialistic outlook, not skill compensation.
- **C. Continuity theory** — Owner: Robert Atchley. Continuity theory emphasizes keeping consistent patterns and identity, not strategies that compensate for decline.
- **D. Socioemotional selectivity theory** — Owner: Laura Carstensen. Carstensen's theory concerns narrowing relationships toward emotionally meaningful ones, not maintaining skills.
- **Pair to learn:** Selection, optimization, compensation → Baltes & Baltes

89 — D

Intake, Assessment & Diagnosis · Outline 2.K · Chapter 19

- **D. The WHODAS 2.0 — Owner: WHODAS 2.0 (WHO Disability Assessment Schedule).** The DSM-5-TR dropped the GAF and the multiaxial system and suggests the WHODAS 2.0 for measuring functioning.
- **A. The ASAM Criteria** — Owner: ASAM Criteria (substance use level of care). The ASAM Criteria guide level-of-care decisions for substance use, not general functioning ratings.
- **B. The SCID-5 interview** — Owner: SCID-5 (structured diagnostic interview). The SCID-5 is a structured interview for making diagnoses, not the DSM-5-TR's suggested functioning measure.
- **C. A GAF rating on Axis V** — Owner: GAF / multiaxial Axis V (eliminated in DSM-5). The GAF and Axis V were dropped; using them treats an outdated system as current practice.
- **Pair to learn:** Replaced the GAF → WHODAS 2.0 (not multiaxial Axis V)

90 — D

Treatment Planning · Outline 4.N · Chapter 21

- **D. Assessment — Owner: SOAP: Assessment.** The counselor's clinical impression and judgment of progress belongs in Assessment.
- **A. Objective** — Owner: SOAP: Objective. Objective holds observations and measurements, such as affect or a PHQ-9 score.
- **B. Plan** — Owner: SOAP: Plan. Plan holds next steps, not an evaluation of progress.
- **C. Subjective** — Owner: SOAP: Subjective. Subjective holds the client's own report and words.
- **Pair to learn:** Counselor's clinical impression of progress → SOAP: Assessment

91 — D

Core Counseling Attributes · Outline 6.H · Chapter 22

- **D. "His silence seems to leave you shut out and unsure where you stand." — Owner: Empathic responding (from the client's frame).** It communicates the client's experience from the client's own frame of reference.
- **A. "I know exactly how you feel; my dad went quiet on me too."** — Owner: Identification ("I know exactly how you feel"). Identification shifts focus to the counselor and assumes rather than understands.
- **B. "Oh no, I feel so bad for you. That's awful of him to do."** — Owner: Sympathy (not empathy). Feeling sorry for the client from the counselor's frame is sympathy, which the exam treats as a distractor.
- **C. "Give him some time. I'm sure he'll come around soon enough."** — Owner: Reassurance (not empathy). Reassurance skips over the client's feeling and predicts an outcome the counselor can't know.
- **Pair to learn:** Client's feeling from the client's frame → Empathic responding (not sympathy)

92 — B

Areas of Clinical Focus · Outline 3.Z · Chapter 15

- **B. John Henryism — Owner: Sherman James.** High-effort coping with chronic stressors, linked to health problems in Black Americans with limited resources, is John Henryism.
- **A. Stereotype threat** — Owner: Steele & Aronson. Stereotype threat is lowered performance from fear of confirming a stereotype.
- **C. Internalized racism** — Owner: Levels of racism. Internalized racism is accepting negative messages about one's own group, which he isn't described as doing.
- **D. Impostor phenomenon** — Owner: Clance & Imes. The impostor phenomenon is feeling like a fraud despite success.
- **Pair to learn:** High-effort coping and health → John Henryism (Sherman James)

93 — A

Areas of Clinical Focus · Outline 3.Q · Chapter 16

- **A. ARFID — Owner: DSM-5-TR avoidant/restrictive food intake disorder.** Restricted eating due to sensory aversion without body-image concern is ARFID.
- **B. Anorexia nervosa, restricting type** — Owner: DSM-5-TR anorexia nervosa. Anorexia requires fear of weight gain or disturbed body image, which he doesn't have.
- **C. Rumination disorder** — Owner: DSM-5-TR rumination disorder. Rumination disorder is repeated regurgitation of food.
- **D. Pica** — Owner: DSM-5-TR pica. Pica is eating nonfood substances.
- **Pair to learn:** Restriction without body-image concern → ARFID

94 — C

Counseling Skills & Interventions · Outline 5.V · Chapter 6

- **C. Limit access to firearms and stockpiled medications — Owner: Lethal means counseling.** Means restriction is a strongly supported prevention strategy because many suicidal crises are brief.
- **A. Avoid mentioning suicide so the idea isn't planted** — Owner: Suicide myth. Asking about suicide directly does not increase risk; avoiding it is a myth.
- **B. Treat a sudden calm as a sign danger has passed** — Owner: Suicide myth. A sudden calm can follow a decision to die and calls for reassessment.
- **D. Have his son sign a no-suicide contract** — Owner: No-suicide contract (lacks evidence). No-suicide contracts haven't been shown to prevent suicide and can give false reassurance.
- **Pair to learn:** Means restriction → strongly supported prevention

95 — B

Professional Practice & Ethics · Outline 1.V · Chapter 17

- **B. Bernard (discrimination model) — Owner: Bernard's discrimination model.** Bernard's discrimination model defines the three supervisor roles and three foci named in the stem.
- **A. Forester-Miller and Davis** — Owner: Forester-Miller & Davis seven-step model. This is an ethical decision-making model, not a supervision model.
- **C. Stoltenberg and Delworth (IDM)** — Owner: Integrated Developmental Model (Stoltenberg & Delworth). The IDM describes supervisee development through levels of awareness, motivation, and autonomy.
- **D. Kitchener** — Owner: Kitchener's ethical principles. Kitchener described the foundational ethical principles, not supervisor roles.

- **Pair to learn:** Teacher, counselor, consultant roles → Bernard (discrimination model)

96 — D

Professional Practice & Ethics · Outline 1.B · Chapter 18

- **D. Diffusion of treatment — Owner: Diffusion of treatment (Cook & Campbell).** When the control group learns about or receives part of the treatment, it is diffusion of treatment.
- **A. John Henry effect** — Owner: John Henry effect. The John Henry effect is the control group working harder to compete, not adopting the treatment.
- **B. Hawthorne effect** — Owner: Hawthorne effect (Mayo). The Hawthorne effect is change from knowing one is being studied.
- **C. Experimental mortality** — Owner: Experimental mortality (attrition). Mortality is uneven dropout across groups.
- **Pair to learn:** Control group receives part of treatment → Diffusion of treatment

97 — C

Professional Practice & Ethics · Outline 1.M · Chapter 9

- **C. Licensure generally follows the client's location — Owner: ACA H.1.b laws of the client's location.** Counselors must follow the laws where the client is physically located and generally need authorization to practice there.
- **A. The client must be offered face-to-face sessions** — Owner: ACA H.4.c-d fit and effectiveness of distance counseling. Fit for distance counseling is a clinical question; the barrier here is legal authorization.
- **B. The platform must verify identity with a code word** — Owner: ACA H.3 client verification. Verifying identity is required, but it doesn't address authorization to practice across state lines.
- **D. Her Georgia license covers sessions she provides** — Owner: Counselor-location licensure

(misconception). Being licensed where the counselor sits isn't enough; the client's location governs.

- **Pair to learn:** Client in another state for telehealth → Licensure follows the client (H.1.b)

98 — A

Counseling Skills & Interventions · Outline 5.AM · Chapter 7

- **A. Divorce — Owner: Genogram conventions (McGoldrick & Gerson).** Two slashes mark divorce; one slash marks separation.
- **B. Separation** — Owner: Genogram conventions (McGoldrick & Gerson). Separation is shown with a single slash.
- **C. Conflict** — Owner: Genogram conventions (McGoldrick & Gerson). Conflict is shown with a zigzag line.
- **D. Cutoff** — Owner: Genogram conventions (McGoldrick & Gerson). A cutoff is shown with a broken line with a gap.
- **Pair to learn:** Two slashes → divorce (genogram)

99 — C

Areas of Clinical Focus · Outline 3.K · Chapter 12

- **C. Stroebe & Schut's dual process model — Owner: Stroebe & Schut.** Moving between loss-oriented coping and restoration-oriented coping is the core of the dual process model.
- **A. Kübler-Ross's bargaining stage** — Owner: Elisabeth Kübler-Ross. Bargaining is a reaction to dying, and nothing in his behavior reflects bargaining.
- **B. Continuing bonds** — Owner: Klass, Silverman, & Nickman. Continuing bonds concerns an ongoing connection to the deceased, not alternating between grief work and life tasks.

- **D. Bowlby's reorganization phase** — Owner: John Bowlby. Reorganization is the final phase of mourning, not a healthy alternation between grief and life tasks.
- **Pair to learn:** Oscillating loss and restoration coping → Stroebe & Schut

100 — A

Intake, Assessment & Diagnosis · Outline 2.T · Chapter 20

- **A. OQ-45 — Owner: OQ-45 (Lambert; Outcome Questionnaire).** The OQ-45 is an outcome questionnaire designed for tracking progress over treatment, which supports pre-test and post-test comparison.
- **B. Rorschach Inkblot Test** — Owner: Rorschach (projective; Exner/R-PAS). The Rorschach is a projective personality test with lower reliability evidence, not a repeated outcome measure.
- **C. WAIS-5** — Owner: WAIS (Wechsler intelligence test). The WAIS measures general cognitive ability, which is not what counseling outcomes track.
- **D. Strong Interest Inventory** — Owner: Strong (interest inventory). The Strong measures career interests, not symptom change.
- **Pair to learn:** Tracking progress across sessions → OQ-45 (Lambert)

101 — C

Treatment Planning · Outline 4.H · Chapter 21

- **C. Tapering — Owner: Tapering.** Gradually reducing session frequency before ending is tapering.
- **A. Booster sessions** — Owner: Booster sessions (follow-up). Boosters are follow-up contacts after discharge, not the wind-down schedule.
- **B. Relapse prevention** — Owner: Relapse prevention. Relapse prevention plans for warning signs and coping, not session spacing.

- **D. Step-down care** — Owner: Step-down (level-of-care change). Step-down moves a client to a less intensive program, like IOP to outpatient.
- **Pair to learn:** Weekly to biweekly to monthly → Tapering

102 — B

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **B. Differentiation of self — Owner: Bowen (multigenerational).** Calm I-positions and staying out of the parents' triangle build Bowen's differentiation of self.
- **A. Congruent communication** — Owner: Satir (experiential). Satir targets congruent communication in place of placating, blaming, and similar stances.
- **C. Second-order change** — Owner: MRI (brief strategic). The MRI goal is changing the system's rules by interrupting attempted solutions.
- **D. Clear subsystem boundaries** — Owner: Minuchin (structural). Minuchin restructures boundaries and hierarchy in session, often through enactment and joining.
- **Pair to learn:** Differentiation of self → Bowen

103 — C

Counseling Skills & Interventions · Outline 5.O · Chapter 5

- **C. Explore a decisional balance — Owner: Prochaska & DiClemente: contemplation-stage move.** Acknowledging the problem while still ambivalent is contemplation, where exploring pros and cons of changing fits best.
- **A. Confront the client's denial** — Owner: Confrontation (contrary to MI). The client isn't denying anything, and arguing increases resistance at any stage.

- **B. Set a specific quit date this week** — Owner: Preparation/action-stage move. Pushing for a quit date is the classic wrong move in contemplation, before ambivalence is resolved.
- **D. Begin relapse prevention planning** — Owner: Maintenance-stage move (Marlatt). Relapse prevention fits after change is underway, not before the client has decided.
- **Pair to learn:** Contemplation → decisional balance (Prochaska & DiClemente)

104 — D

Counseling Skills & Interventions · Outline 5.AV · Chapter 8

- **D. Ask how she is feeling as she tells the group this — Owner: Redirect to here-and-now (Yalom).** Storytelling is redirected to the member's present feelings and the here-and-now.
- **A. Interpret her stories as avoidance of her childhood** — Owner: Premature interpretation. An unsupported interpretation doesn't bring her into the present group.
- **B. Let her finish, since any sharing builds cohesion** — Owner: Ignoring unproductive behavior. Allowing the pattern to continue keeps the group on outside content.
- **C. Invite members to advise her about her coworkers** — Owner: Encouraging advice-giving. Advice-giving is itself a harmful member behavior and keeps the focus on content.
- **Pair to learn:** Storytelling → redirect to feelings and here-and-now

105 — B

Areas of Clinical Focus · Outline 3.K · Chapter 12

- **B. Worden's tasks of mourning — Owner: J. William Worden.** Worden framed grief as four active

tasks; the fourth is finding an enduring connection while moving forward.

- **A. Kübler-Ross's stages of dying** — Owner: Elisabeth Kübler-Ross. Kübler-Ross described reactions of terminally ill patients, not tasks for mourners.
- **C. Bowlby's phases of mourning** — Owner: John Bowlby. Bowlby's phases (numbness through reorganization) describe what grievers go through, not what they do.
- **D. The dual process model** — Owner: Stroebe & Schut. The dual process model describes oscillating between loss and restoration coping, not a list of tasks.
- **Pair to learn:** Tasks of mourning → Worden

106 — C

Areas of Clinical Focus · Outline 3.AC · Chapter 10

- **C. Nolen-Hoeksema's response styles theory** — **Owner: Susan Nolen-Hoeksema.** Repetitively focusing on distress and its causes (rumination) prolongs and deepens depression in response styles theory.
- **A. Rachman's cognitive theory of OCD** — Owner: Stanley Rachman. Rachman explains how intrusive thoughts become obsessions; her repetitive thinking isn't intrusive or ego-dystonic.
- **B. Seligman's preparedness** — Owner: Martin Seligman. Preparedness explains why some fears are learned easily, not why depression persists.
- **D. Klerman and Weissman's role disputes** — Owner: Klerman & Weissman (IPT). Role disputes is an IPT problem area about conflict with others, not repetitive self-focus.
- **Pair to learn:** Rumination prolongs depression → Nolen-Hoeksema

107 — B

Core Counseling Attributes · Outline 6.C · Chapter 22

- **B. "I notice I feel heavy hearing this, even as you laugh. What's that like for you?" — Owner: Congruence (Rogers) expressed through immediacy.** Congruence means the counselor's outward words match inner experience, shared because it serves the client.
- **A. "That sounds like such a funny family story. Tell me more about it!"** — Owner: Incongruence (pretending to feel something). Matching the client's laughter while feeling heaviness is pretending, the opposite of congruence.
- **C. "Honestly, my own brother left me out once, and I'm still angry about it."** — Owner: Venting the counselor's feelings (not congruence). Congruence doesn't mean saying everything you feel; this shifts focus to the counselor's needs.
- **D. "I'm sure he didn't mean to hurt you. Families are complicated."** — Owner: Reassurance (not empathy or congruence). Reassurance dismisses the client's experience and doesn't share the counselor's real reaction.
- **Pair to learn:** Inner experience matches outward expression → Congruence (Rogers)

108 — A

Areas of Clinical Focus · Outline 3.S · Chapter 3

- **A. Identity diffusion — Owner: Marcia (identity statuses).** No exploration (crisis) and no commitment defines Marcia's diffusion status.
- **B. Moratorium** — Owner: Marcia (identity statuses). Moratorium requires active exploration, which she isn't doing.
- **C. Identity vs. role confusion** — Owner: Erikson (psychosocial stage 5). This is Erikson's adolescent

crisis; Marcia's statuses, not Erikson's stage name, classify her current position.

- **D. Pre-encounter** — Owner: Cross (Nigrescence). Pre-encounter is the first stage of Black racial identity development, not a general identity status.
- **Pair to learn:** Diffusion (no crisis, no commitment) → Marcia

109 — D

Counseling Skills & Interventions · Outline 5.O · Chapter 5

- **D. Change talk — Owner: Miller & Rollnick (MI).** Statements of need and reasons for change are preparatory change talk (the N and R of DARN-CAT).
- **A. Contemplation** — Owner: Prochaska & DiClemente (stages of change). Contemplation is a stage of readiness, not a type of client statement.
- **B. Sustain talk** — Owner: Miller & Rollnick (MI). Sustain talk argues for staying the same; this client is arguing for change.
- **C. The righting reflex** — Owner: Miller & Rollnick (MI). The righting reflex is the counselor's urge to fix or persuade, not something the client says.
- **Pair to learn:** Desire, Ability, Reasons, Need → change talk (MI)

110 — D

Areas of Clinical Focus · Outline 3.U · Chapter 3

- **D. Career construction theory — Owner: Savickas (career construction; life design).** Savickas built career construction around adaptability and the client's career stories.
- **A. Cognitive Information Processing** — Owner: Peterson, Sampson, & Reardon. CIP uses the pyramid and the CASVE cycle.

- **B. Life-span, life-space theory** — Owner: Super. Super is linked to career maturity and the Life-Career Rainbow, not the four Cs of adaptability.
- **C. Theory of Work Adjustment** — Owner: Dawis & Lofquist. TWA concerns satisfaction and satisfactoriness between worker and environment.
- **Pair to learn:** Career adaptability (four Cs) → Savickas

111 — A

Counseling Skills & Interventions · Outline 5.Q · Chapter 6

- **A. Assess lethality and imminent danger — Owner: Roberts: stage 1.** Roberts begins with a biopsychosocial and lethality/imminent danger assessment.
- **B. Define the problem** — Owner: James & Gilliland: step 1. Defining the problem is the first step of James and Gilliland's six-step model.
- **C. Make psychological contact** — Owner: Roberts: stage 2. Establishing contact and the relationship is Roberts' second stage.
- **D. Offer contact and engagement** — Owner: Psychological First Aid (NCTSN/NCPTSD). Contact and engagement is the first PFA core action after a disaster.
- **Pair to learn:** Seven stages starting with lethality assessment → Roberts

112 — B

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **B. Unfinished business — Owner: Perls (Gestalt).** The empty chair is a Gestalt technique for bringing unfinished business into present awareness.
- **A. Basic mistakes** — Owner: Adler (individual psychology). Basic mistakes are Adlerian faulty beliefs uncovered through lifestyle assessment, not empty chair dialogue.

- **C. Emotional cutoff** — Owner: Bowen (multigenerational). Emotional cutoff is a Bowen family systems concept addressed through differentiation, not chair work.
- **D. Transference** — Owner: Freud (psychoanalysis). Transference is analyzed as it appears toward the counselor, not by addressing an empty chair.
- **Pair to learn:** Empty chair / unfinished business → Perls (Gestalt)

113 — C

Areas of Clinical Focus · Outline 3.AG · Chapter 15

- **C. Viktor Frankl — Owner: Frankl (will to meaning).** Frankl's will to meaning identifies these three sources of meaning.
- **A. Irvin Yalom** — Owner: Yalom (four ultimate concerns). Yalom named death, freedom, isolation, and meaninglessness as ultimate concerns.
- **B. Rollo May** — Owner: May (normal vs. neurotic anxiety). May is known for distinguishing normal from neurotic anxiety.
- **D. Ernest Becker** — Owner: Becker (The Denial of Death). Becker wrote about how people deny death, not sources of meaning.
- **Pair to learn:** Will to meaning; attitude toward suffering → Frankl

114 — B

Intake, Assessment & Diagnosis · Outline 2.M · Chapter 20

- **B. Release raw data only to a qualified professional the client designates — Owner: ACA E.4 (release of data to qualified personnel).** Raw test data and protocols go only to persons qualified to interpret them, with client consent.
- **A. Refuse to share anything about the testing with anyone** — Owner: Over-restriction (ignores client's right to results). Results can still be explained to the

client and raw data released to qualified personnel; a blanket refusal goes too far.

- **C. Send the item responses to the attorney because consent was given** — Owner: Consent alone (ignores ACA E.4 qualification rule). Client consent is required but not sufficient; the recipient must also be qualified to interpret the data.
- **D. Photocopy the test booklet so the attorney can review the items** — Owner: Breach of assessment security (ACA E.10). Reproducing test materials violates test security and copyright.
- **Pair to learn:** Raw test data → ACA E.4: qualified personnel only

115 — B

Areas of Clinical Focus · Outline 3.J · Chapter 15

- **B. Lev's transgender emergence — Owner: Arlene Istar Lev.** Lev's model describes the stages of transgender emergence ending in integration and acceptance.
- **A. Cass's homosexual identity formation** — Owner: Vivienne Cass. Cass's model runs from identity confusion to synthesis.
- **C. Troiden's model** — Owner: Richard Troiden. Troiden's stages are sensitization, identity confusion, identity assumption, and commitment.
- **D. D'Augelli's life-span model** — Owner: Anthony D'Augelli. D'Augelli described six interrelated processes rather than fixed stages.
- **Pair to learn:** Transgender emergence → Lev

116 — C

Areas of Clinical Focus · Outline 3.C · Chapter 16

- **C. Multisystemic therapy — Owner: Henggeler (MST).** MST is intensive home- and community-based treatment across the systems around the youth.

- **A. Parent-child interaction therapy** — Owner: Eyberg (PCIT). PCIT is designed for young children and their parents.
- **B. Problem-solving skills training** — Owner: Kazdin (PSST). PSST teaches the youth problem-solving skills rather than targeting multiple systems.
- **D. Functional family therapy** — Owner: Alexander & Parsons (FFT). FFT is a family-focused treatment, not intensive work across peers, school, and neighborhood.
- **Pair to learn:** Home- and community-based, multiple systems → MST (Henggeler)

117 — A

Intake, Assessment & Diagnosis · Outline 2.M · Chapter 3

- **A. Internal consistency — Owner: Internal consistency (Cronbach's alpha; KR-20).** Cronbach's alpha tests whether the items hang together in a single administration.
- **B. Test-retest reliability** — Owner: Test-retest reliability (stability over time). Test-retest requires giving the same test twice; this scale was given once.
- **C. Alternate forms reliability** — Owner: Alternate (parallel) forms reliability. Alternate forms compares two versions of a test; only one version is involved here.
- **D. Interrater reliability** — Owner: Interrater reliability (scorer agreement). Interrater reliability checks whether two scorers agree, not whether items agree with each other.
- **Pair to learn:** Items hang together → Internal consistency (not test-retest)

118 — C

Areas of Clinical Focus · Outline 3.F · Chapter 15

- **C. Separation — Owner: Berry (acculturation strategies).** Keeping heritage while avoiding the host culture is Berry's separation.

- **A. Marginalization** — Owner: Berry (acculturation strategies). Marginalization means connecting with neither culture; he is strongly connected to his heritage.
- **B. Immersion-emersion** — Owner: Cross (Nigrescence). Immersion-emersion is a stage of Black racial identity development, not an acculturation strategy.
- **D. Assimilation** — Owner: Berry (acculturation strategies). Assimilation drops the heritage culture to join the host culture.
- **Pair to learn:** Keep heritage, avoid host → separation (Berry)

119 — A

Areas of Clinical Focus · Outline 3.AI · Chapter 11

- **A. Stimulant withdrawal, with possible suicide risk — Owner: DSM-5-TR stimulant withdrawal (crash).** The stimulant crash brings fatigue, hypersomnia, increased appetite, and depressed mood; it's not usually medically dangerous, but suicide risk can be high.
- **B. Alcohol withdrawal** — Owner: DSM-5-TR alcohol withdrawal. Alcohol withdrawal involves tremor, sweating, insomnia, and possible seizures, not hypersomnia.
- **C. Opioid withdrawal** — Owner: DSM-5-TR opioid withdrawal. Opioid withdrawal is flu-like, with runny nose, yawning, goosebumps, and diarrhea.
- **D. Cannabis withdrawal** — Owner: DSM-5-TR cannabis withdrawal. Cannabis withdrawal brings insomnia and decreased appetite, not the hypersomnia and increased eating seen here.
- **Pair to learn:** Crash (hypersomnia, hunger, low mood) → stimulant withdrawal

120 — D

Professional Practice & Ethics · Outline 1.F · Chapter 6

- **D. Jablonski v. United States (1983) — Owner: Jablonski v. United States (1983).** Jablonski extended the duty to foreseeable victims based on violent history, without a specific stated threat.
- **A. Tarasoff v. Regents, Tarasoff I (1974)** — Owner: Tarasoff v. Regents (1974), duty to warn. Tarasoff I established a duty to warn an identifiable victim of a serious threat.
- **B. Ewing v. Goldstein (2004)** — Owner: Ewing v. Goldstein (2004). Ewing held that a threat reported by a client's family member can trigger the duty.
- **C. Tarasoff v. Regents, Tarasoff II (1976)** — Owner: Tarasoff v. Regents (1976), duty to protect. Tarasoff II broadened warning to protecting an intended victim of a client's threat.
- **Pair to learn:** Violent history, no explicit threat → Jablonski v. United States

121 — B

Areas of Clinical Focus · Outline 3.S · Chapter 3

- **B. Animism — Owner: Piaget: preoperational stage.** Attributing life and intentions to objects is animism, a hallmark of Piaget's preoperational stage (ages 2 to 7).
- **A. Rapprochement** — Owner: Mahler (separation-individuation). Rapprochement is a toddler subphase of returning to the caregiver after exploring, not a way of reasoning about objects.
- **C. Initiative vs. guilt** — Owner: Erikson (psychosocial stage 3). Initiative vs. guilt is Erikson's preschool psychosocial crisis; it names a crisis, not a cognitive feature.
- **D. Personal fable** — Owner: Elkind (adolescent egocentrism). The personal fable is an adolescent's

sense of being unique or invulnerable, not a preschooler's belief that objects are alive.

- **Pair to learn:** Animism → Piaget (preoperational)

122 — C

Professional Practice & Ethics · Outline 1.M · Chapter 17

- **C. Respect her refusal and document without the tool — Owner: Informed consent for AI (ACA A.2; ACA AI Work Group).** Clients must be told when AI is used and may decline; the counselor honors the refusal.
- **A. Use a free consumer chatbot without her name instead** — Owner: Confidentiality breach (consumer AI tools). Consumer AI tools aren't built for protected health information and still bypass her refusal.
- **B. Refer her to a counselor who doesn't use AI tools** — Owner: Unnecessary referral. Simply not using the tool meets her request; transferring her care is not needed.
- **D. Use it anyway, since notes are the counselor's responsibility** — Owner: Counselor responsibility for records (misapplied). The counselor is responsible for notes, but that doesn't override the client's right to refuse.
- **Pair to learn:** Client declines AI note-taker → Informed consent; respect refusal

123 — B

Professional Practice & Ethics · Outline 1.B · Chapter 18

- **B. Dependent (paired) t-test — Owner: Dependent/paired t-test.** Two means from the same people measured twice call for a dependent (paired) t-test.
- **A. One-way ANOVA** — Owner: One-way ANOVA. One-way ANOVA compares three or more group means.
- **C. Chi-square** — Owner: Chi-square. Chi-square analyzes frequencies in categories, not mean scores.

- **D. Independent t-test** — Owner: Independent t-test. The independent t-test compares two different groups, not the same clients twice.
- **Pair to learn:** Same people measured twice → Dependent (paired) t-test

124 — C

Counseling Skills & Interventions · Outline 5.AC · Chapter 4

- **C. "Being there for your kids matters more than the pay." — Owner: Reflection of meaning.** It names the value underneath the choice: why it matters to the client.
- **A. "You chose a job with less pay and more time at home."** — Owner: Paraphrase. It restates the facts in fresh words without naming why they matter.
- **B. "Why did you decide money was less important?"** — Owner: "Why" question (invites defensiveness). A why question asks the client to justify the choice rather than reflecting it.
- **D. "You sound relieved that you made the decision."** — Owner: Reflection of feeling. It names an emotion, not the underlying value.
- **Pair to learn:** Names the value underneath → reflection of meaning

125 — C

Counseling Skills & Interventions · Outline 5.R · Chapter 5

- **C. Transference — Owner: Freud (psychoanalysis).** Redirecting feelings and expectations from an important past relationship onto the counselor is transference, which the counselor can explore tentatively.
- **A. Reaction formation** — Owner: Freud / A. Freud (defense mechanisms). Reaction formation is expressing the opposite of what one feels; the client is expressing anger openly.

- **B. Sublimation** — Owner: Freud / A. Freud (defense mechanisms). Sublimation channels impulses into acceptable outlets and doesn't involve reacting to the counselor as to a parent.
- **D. Countertransference** — Owner: Freud (psychoanalysis). Countertransference is the counselor's own emotional reaction to the client, handled in supervision, not taught to the client here.
- **Pair to learn:** Feelings from past relationships onto counselor → transference (Freud)

126 — D

Areas of Clinical Focus · Outline 3.AI · Chapter 11

- **D. Moderate — Owner: DSM-5-TR severity (4-5 criteria).** Four criteria falls in the moderate range of 4 to 5.
- **A. Mild** — Owner: DSM-5-TR severity (2-3 criteria). Mild requires only 2 to 3 criteria; she meets 4.
- **B. Early remission** — Owner: DSM-5-TR remission specifier. Early remission requires 3 to 12 months with no criteria except craving; she is actively meeting criteria.
- **C. Severe** — Owner: DSM-5-TR severity (6+ criteria). Severe requires 6 or more criteria.
- **Pair to learn:** 4-5 criteria → moderate SUD

127 — B

Intake, Assessment & Diagnosis · Outline 2.M · Chapter 20

- **B. Percentile ranks are ordinal, not equal intervals — Owner: Percentile rank (ordinal scale).** Percentiles are not equal intervals; the gap from the 90th to 95th is much larger in raw-score terms than from the 50th to 55th.
- **A. Both children gained five points of the same ability** — Owner: Equal-interval assumption (interpreting percentiles as interval data). Treating

percentile differences as equal intervals is the exact error the exam punishes.

- **C. The difference is explained by the standard error of measurement** — Owner: Standard error of measurement (score error band). SEM describes error around one score; it does not explain why equal percentile gaps differ in size.
- **D. The higher scores reflect a ceiling effect on the test** — Owner: Ceiling effect (test too easy). A ceiling effect means many score at the top; nothing suggests the test is too easy.
- **Pair to learn:** Percentile differences are unequal → Percentile rank is ordinal (not interval)

128 — D

Areas of Clinical Focus · Outline 3.D · Chapter 16

- **D. A power imbalance between the students — Owner: Olweus (definition of bullying).** Olweus defined bullying as intentional, repeated harm with a power imbalance; that imbalance is why peer mediation is inappropriate.
- **A. Physical rather than verbal harm** — Owner: Types of bullying. Verbal, relational, and cyberbullying all count as bullying.
- **B. Vindictive behavior toward peers** — Owner: ODD symptom (DSM-5-TR). Vindictiveness is an ODD symptom, not a defining feature of bullying.
- **C. A clear antecedent and consequence** — Owner: Functional behavior assessment (ABC). The ABC model analyzes any behavior's function; it doesn't define bullying.
- **Pair to learn:** Intentional, repeated, power imbalance → Olweus

129 — A

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **A. Early recollections — Owner: Adler (individual psychology).** Early recollections are an Adlerian lifestyle assessment tool for uncovering basic mistakes.
- **B. Life review** — Owner: Butler (life review). Life review is a structured review of the whole life for older adults, not an assessment of lifestyle.
- **C. Free association** — Owner: Freud (psychoanalysis). Free association asks the client to say whatever comes to mind to reach unconscious material, not to recall specific early memories.
- **D. Active imagination** — Owner: Jung (analytical psychology). Active imagination is Jung's method for working with symbols and images from the unconscious.
- **Pair to learn:** Early recollections → Adler

130 — B

Professional Practice & Ethics · Outline 1.S · Chapter 17

- **B. Comply, releasing only what the order requires — Owner: Court order (ACA B.2.d).** A judge-signed order must be followed, while disclosing as little as possible and asking the court to limit disclosure if needed.
- **A. Destroy any psychotherapy notes before responding** — Owner: Record destruction (unethical and illegal). Destroying records in response to legal process is never acceptable.
- **C. Refuse and assert privilege until the client consents** — Owner: Subpoena response (assert privilege). Asserting privilege is the response to an attorney-issued subpoena, not to an order signed by a judge.
- **D. Send the complete record so nothing is withheld** — Owner: Overdisclosure (violates ACA B.

2.e). Sending everything ignores the minimal disclosure standard.

- **Pair to learn:** Judge-signed order → Comply with minimal disclosure (ACA B.2.d)

131 — A

Areas of Clinical Focus · Outline 3.AO · Chapter 13

- **A. Patricia Papernow — Owner: Papernow (Stepfamily Cycle).** Papernow's seven stages fall into early (fantasy, immersion, awareness), middle, and later phases.
- **B. Paul Bohannan** — Owner: Bohannan (six stations of divorce). Bohannan described the stations of divorce.
- **C. McGoldrick and Carter** — Owner: McGoldrick & Carter (family life cycle). They added remarriage stages to the family life cycle.
- **D. Emily and John Visher** — Owner: Visher & Visher. The Vishers identified stepfamily myths such as instant love, not the stage cycle.
- **Pair to learn:** Fantasy, immersion, awareness → Papernow

132 — C

Counseling Skills & Interventions · Outline 5.X · Chapter 9

- **C. Confirming her physical location — Owner: Telehealth practice (location check for emergencies).** Knowing where the client is lets the counselor send local emergency help right away.
- **A. Explaining that email isn't for emergencies** — Owner: Electronic communication consent. This is good informed consent but doesn't help reach her during a live emergency.
- **B. Verifying her identity with a code word** — Owner: ACA H.3 (client verification). Verification confirms who the client is but doesn't help emergency responders find her.

- **D. Confirming the platform has a BAA** — Owner: HIPAA (business associate agreement). A BAA protects privacy but does nothing for an emergency in progress.
- **Pair to learn:** Telehealth emergency → confirm client's location each session

133 — A

Treatment Planning · Outline 4.O · Chapter 21

- **A. Review which skills helped and how she'll keep using them — Owner: Consolidate and generalize learning (4.O).** Consolidating what helped and planning how to apply it independently supports maintenance of progress.
- **B. Refer her for a medication evaluation to protect gains** — Owner: Concurrent referral (medication). Nothing suggests a need for medication; the question is about maintaining gains.
- **C. Step her up to intensive outpatient for support** — Owner: Step up (higher level of care). She is improving, so a higher level of care isn't indicated.
- **D. Add a new long-term goal to extend counseling** — Owner: Long-term goal (new). Adding goals delays ending rather than preparing her to maintain gains.
- **Pair to learn:** Maintaining gains after ending → Consolidate and generalize (4.O)

134 — C

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **C. Shaping — Owner: Skinner (operant conditioning).** Reinforcing successive approximations of a target behavior is shaping.
- **A. Modeling** — Owner: Bandura (social learning). Modeling teaches through observation of another person's behavior.
- **B. Systematic desensitization** — Owner: Wolpe. Systematic desensitization pairs relaxation with an anxiety hierarchy; it targets fear, not seat time.

- **D. The Premack principle** — Owner: Premack. The Premack principle uses a preferred activity to reinforce a less-preferred one, not gradually rising criteria.
- **Pair to learn:** Shaping → Skinner (operant)

135 — A

Intake, Assessment & Diagnosis · Outline 2.F · Chapter 19

- **A. Mood — Owner: Mood (client-reported).** Mood is the client's self-reported, sustained emotional state, often recorded in the client's own words. The client is describing weeks of feeling, which is the sustained 'climate.'
- **B. Thought content** — Owner: Thought content (what the client thinks). Thought content covers what the client thinks about, such as delusions, obsessions, or suicidal ideation, not a reported emotional state.
- **C. Insight** — Owner: Insight (understanding of one's problem). Insight rates the client's understanding of the problem as good, fair, poor, or absent; it is not a feeling the client reports.
- **D. Affect** — Owner: Affect (counselor-observed). Affect is what the counselor observes of emotional expression in the moment, not what the client reports.
- **Pair to learn:** Client-reported emotional state → Mood (not affect)

136 — D

Areas of Clinical Focus · Outline 3.AS · Chapter 13

- **D. Psychic — Owner: Bohannan (six stations of divorce).** The psychic station involves regaining individual identity and autonomy.
- **A. Emotional** — Owner: Bohannan (six stations of divorce). The emotional station is the deterioration of the relationship itself.
- **B. Coparental** — Owner: Bohannan (six stations of divorce). The coparental station concerns custody and co-parenting arrangements.

- **C. Reorientation** — Owner: Atchley (retirement phases). Reorientation is a retirement phase, not a station of divorce.
- **Pair to learn:** Regaining individual identity → Bohannan's psychic station

137 — A

Counseling Skills & Interventions · Outline 5.Q · Chapter 6

- **A. Take a directive role to secure shelter tonight — Owner: Crisis intervention (directive stance).** In an acute crisis the counselor is more directive and focuses on immediate safety and restoring functioning.
- **B. Explore how early losses shape her reaction** — Owner: Psychodynamic insight (Freud). Exploring history during an active crisis is a wrong answer; stabilization comes first.
- **C. Reflect her feelings and let her lead** — Owner: Nondirective stance (Rogers). A fully nondirective stance doesn't address the urgent need for a safe place tonight.
- **D. Set long-term career goals with her** — Owner: Ongoing counseling (not crisis). Long-term goal setting waits until the immediate crisis is stabilized.
- **Pair to learn:** Acute crisis → more directive, here-and-now

138 — B

Counseling Skills & Interventions · Outline 5.AH · Chapter 8

- **B. Blocking — Owner: Group leader skill: blocking.** Stopping a harmful behavior while protecting the target and redirecting to owned feelings is blocking.
- **A. Drawing out** — Owner: Group leader skill: drawing out. Drawing out invites a quiet member into the conversation.

- **C. Linking** — Owner: Group leader skill: linking. Linking connects one member's concern to another's.
- **D. Cutting off** — Owner: Group leader skill: cutting off. Cutting off manages time or focus, as with a member who talks too long.
- **Pair to learn:** Stop harm, protect member → blocking

139 — D

Intake, Assessment & Diagnosis · Outline 2.E · Chapter 19

- **D. Provisional diagnosis — Owner: Provisional diagnosis (DSM-5-TR).** A provisional diagnosis is used when there is a strong presumption criteria will be met but information is still incomplete.
- **A. Unspecified disorder** — Owner: Unspecified (reason not specified). Unspecified is used when the clinician chooses not to, or cannot, say why criteria aren't met, often in emergency settings.
- **B. Other specified disorder** — Owner: Other specified (clinician states why criteria aren't met). Other specified is used when the clinician records the specific reason criteria are not fully met, not when information is pending.
- **C. Principal diagnosis** — Owner: Principal diagnosis (chief reason for the visit). The principal diagnosis (inpatient) or reason for visit (outpatient) is the condition chiefly responsible for the admission or services and is listed first; it says nothing about incomplete information.
- **Pair to learn:** Criteria expected but information incomplete → Provisional diagnosis (not other specified)

140 — A

Areas of Clinical Focus · Outline 3.AW · Chapter 13

- **A. Revenge — Owner: Dreikurs (goals of misbehavior).** In Dreikurs's model, an adult who feels hurt is a clue that the child's goal is revenge.

- **B. Power** — Owner: Dreikurs (goals of misbehavior). Power struggles leave adults feeling angry and challenged, not hurt.
- **C. Coercive escalation** — Owner: Patterson (coercion theory). Coercion theory describes escalation until a parent gives in, not a hurtful act after a consequence.
- **D. Attention** — Owner: Dreikurs (goals of misbehavior). Attention-seeking leaves adults feeling annoyed.
- **Pair to learn:** Adult feels hurt → revenge (Dreikurs)

141 — D

Core Counseling Attributes · Outline 6.M · Chapter 22

- **D. Allow a comfortable silence — Owner: Comfortable silence (attending; shows acceptance).** Silence after a significant disclosure gives the client room and shows acceptance; filling it usually reflects the counselor's discomfort.
- **A. Ask what her children would say about her parenting** — Owner: Premature question (fills the silence). Jumping to a new question fills the silence and redirects before she has finished.
- **B. Reassure her that she is clearly a good mother** — Owner: Reassurance (dismisses the feeling). Reassurance dismisses the feeling she just risked sharing.
- **C. Summarize everything she has shared so far in the session** — Owner: Summary (mistimed). A summary ties together several statements; here it would cut off the moment after a single disclosure.
- **Pair to learn:** Pause after a significant disclosure → Comfortable silence (attending, 6.M)

142 — B

Treatment Planning · Outline 4.M · Chapter 21

- **B. Show baseline and current scores and ask what she notices — Owner: Reviewing progress with the client (4.M).** Sharing data from baseline to now and

inviting the client's reflection makes the review collaborative.

- **A. Wait for her to raise any concerns about progress** — Owner: Passive monitoring. The counselor should initiate planned reviews rather than waiting.
- **C. Readminister the full intake assessment battery** — Owner: Reassessment (used when progress stalls). Full reassessment is a response to stalled progress, not a routine planned review.
- **D. Tell her she is improving and move on to new material** — Owner: Unilateral counselor judgment. Announcing progress without her input skips her active role in the review.
- **Pair to learn:** Planned progress review → Share data and invite client reflection (4.M)

143 — B

Counseling Skills & Interventions · Outline 5.H · Chapter 9

- **B. Derald Wing Sue — Owner: Sue (worldview model).** Sue's worldview model crosses locus of control with locus of responsibility.
- **A. Geert Hofstede** — Owner: Hofstede (cultural dimensions). Hofstede described dimensions such as individualism vs. collectivism and power distance.
- **C. Pamela Hays** — Owner: Hays (ADDRESSING). Hays created the ADDRESSING framework of cultural influences.
- **D. Edward T. Hall** — Owner: Hall (high- vs. low-context). Hall described high-context vs. low-context communication.
- **Pair to learn:** Locus of control + locus of responsibility → Sue

144 — C

Areas of Clinical Focus · Outline 3.AH · Chapter 16

- **C. Secondary appraisal — Owner: Lazarus & Folkman (transactional model).** Secondary appraisal asks whether one has the resources to cope.
- **A. Shift to the resistance stage** — Owner: Selye (General Adaptation Syndrome). Resistance is a physiological stage of the GAS, not a cognitive appraisal.
- **B. Primary appraisal** — Owner: Lazarus & Folkman (transactional model). Primary appraisal asks whether the situation is a threat, harm, or challenge.
- **D. Hardiness judgment of control** — Owner: Kobasa (hardiness). Control is one of Kobasa's three Cs, a personality trait rather than an appraisal step.
- **Pair to learn:** Do I have the resources? → secondary appraisal (Lazarus & Folkman)

145 — B

Professional Practice & Ethics · Outline 1.B · Chapter 18

- **B. Multiple baseline — Owner: Multiple baseline (single-subject design).** Staggering the intervention across behaviors, settings, or people is multiple baseline, used when withdrawal is unethical.
- **A. Cross-sectional** — Owner: Cross-sectional design. Cross-sectional studies measure different age groups at one time.
- **C. Quasi-experimental** — Owner: Quasi-experimental design. Quasi-experiments compare intact groups without random assignment, not staggered phases in one case.
- **D. ABAB reversal** — Owner: ABAB (single-subject design). ABAB requires withdrawing the intervention, which the stem rules out.
- **Pair to learn:** Staggered intervention, withdrawal unethical → Multiple baseline

146 — D

Counseling Skills & Interventions · Outline 5.V · Chapter 6

- **D. A safety plan, means restriction, and more frequent sessions — Owner: Moderate-risk response (level of care matching).** Active ideation with a vague plan and no clear intent is moderate risk, matched with a safety plan, means restriction, and closer contact.
- **A. Immediate involuntary hospitalization until risk passes** — Owner: Imminent-risk response. Involuntary holds are for imminent risk with stated intent and means; this over-responds.
- **B. A no-suicide contract with routine weekly sessions as before** — Owner: No-suicide contract (lacks evidence). Contracts lack evidence and routine scheduling under-responds to active ideation.
- **C. Noting it and revisiting if it comes up again later** — Owner: Under-response. Active ideation with a plan requires action now, not waiting.
- **Pair to learn:** Moderate risk → safety plan + means restriction (least restrictive appropriate care)

147 — B

Counseling Skills & Interventions · Outline 5.AO · Chapter 8

- **B. Process it so members can share reactions — Owner: Processing structured activities.** Exercises should serve a purpose and be processed afterward.
- **A. Move straight to the next activity** — Owner: Overuse of structured exercises. Stacking exercises without processing makes the group leader-dependent.
- **C. Collect the sheets to review privately later** — Owner: Content focus without process. Reviewing content alone skips the member interaction that builds cohesion.

- **D. Require each child to read an answer aloud** — Owner: Forced participation. Members can decline to participate in an exercise.
- **Pair to learn:** Structured activity → process it afterward

148 — D

Areas of Clinical Focus · Outline 3.Y · Chapter 11

- **D. Moderate — Owner: DSM-5-TR gambling severity (6-7).** Gambling disorder severity is mild at 4 to 5, moderate at 6 to 7, and severe at 8 to 9.
- **A. Mild** — Owner: DSM-5-TR gambling severity (4-5). Mild gambling disorder is 4 to 5 criteria.
- **B. Severe** — Owner: DSM-5-TR gambling severity (8-9). Severe gambling disorder requires 8 to 9 criteria.
- **C. Severe, as with 6 or more criteria** — Owner: DSM-5-TR substance use severity rule. Six or more is severe for substance use disorders, but gambling uses its own scale.
- **Pair to learn:** Gambling 6-7 criteria → moderate (not the SUD 6+ rule)

149 — A

Areas of Clinical Focus · Outline 3.P · Chapter 16

- **A. Stimulus control — Owner: Bootzin.** Bootzin's stimulus control strengthens the link between bed and sleep.
- **B. Imagery rehearsal therapy** — Owner: Krakow (imagery rehearsal therapy). Imagery rehearsal rewrites nightmare scripts; it isn't a CBT-I insomnia component.
- **C. Sleep restriction** — Owner: Spielman. Sleep restriction limits time in bed to actual sleep time to build sleep drive.

- **D. Progressive muscle relaxation** — Owner: Jacobson. Relaxation reduces arousal but doesn't involve rules about using the bed.
- **Pair to learn:** Bed only for sleep; get up if awake → stimulus control (Bootzin)

150 — C

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **C. A shame-attacking exercise — Owner: Ellis (REBT).** Deliberately doing something mildly embarrassing to attack the belief that disapproval is awful is Ellis's shame-attacking exercise.
- **A. An exaggeration exercise** — Owner: Perls (Gestalt). Gestalt exaggeration amplifies a gesture or statement in session to heighten awareness, not to test a belief out in public.
- **B. Acting as if** — Owner: Adler (individual psychology). Acting as if asks the client to behave like the person she wishes to be, not to provoke the judgment she fears.
- **D. Paradoxical intention** — Owner: Frankl (logotherapy). Paradoxical intention asks a client to intend the feared symptom itself, such as trying to stay awake, not to court disapproval.
- **Pair to learn:** Shame-attacking exercise → Ellis (REBT)

151 — C

Counseling Skills & Interventions · Outline 5.H · Chapter 9

- **C. Broaching — Owner: Day-Vines (broaching).** Openly raising race and culture in session, rather than waiting for the client, is broaching.
- **A. An etic approach** — Owner: Etic (universal approach). An etic approach applies universal concepts, while this statement centers the client's specific experience.

- **B. Cultural humility** — Owner: Hook and colleagues (cultural humility). Cultural humility is an ongoing stance of openness, not a specific in-session statement.
- **D. Cultural encapsulation** — Owner: Wrenn (cultural encapsulation). Encapsulation substitutes the counselor's assumptions for reality and ignores culture.
- **Pair to learn:** Raising race directly in session → broaching (Day-Vines)

152 — B

Counseling Skills & Interventions · Outline 5.E · Chapter 4

- **B. Thank the client and ask what hasn't fit — Owner: Safran & Muran (rupture repair).** A confrontation rupture is best named non-defensively and explored; repair often strengthens the alliance.
- **A. Explain the research supporting breathing exercises** — Owner: Defending the counselor. Defending the method dismisses the client's complaint and deepens the rupture.
- **C. Offer a referral to a different counselor** — Owner: Premature referral. Referral abandons the alliance instead of repairing it.
- **D. Continue with the planned exercise** — Owner: Ignoring the rupture. Pressing on with the plan ignores the client's openly stated complaint.
- **Pair to learn:** Confrontation rupture → name and explore (Safran & Muran)

153 — A

Intake, Assessment & Diagnosis · Outline 2.S · Chapter 6

- **A. Ask her directly about suicidal thoughts today — Owner: Ongoing risk assessment (2.S).** Risk assessment is ongoing, not a one-time intake task; a recent loss, intoxication, and isolation call for direct reassessment now.

- **B. Rely on the intake screen, which found no ideation** — Owner: One-time screening (ignores ongoing assessment). Risk changes; the intake result does not cover new risk factors that have appeared since.
- **C. Arrange inpatient admission because of the new stressors** — Owner: Over-response (ignores least restrictive care). Hospitalizing before assessing the level of risk skips the step of matching care to risk.
- **D. Have her sign a no-suicide contract before leaving** — Owner: No-suicide contract (no evidence of preventing suicide). No-suicide contracts lack evidence and can give false reassurance; a safety plan follows an actual assessment.
- **Pair to learn:** New risk factors after intake → Ongoing risk assessment (not one-time screening)

154 — B

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **B. Jay Haley — Owner: Haley (strategic).** Haley's strategic therapy uses directives, ordeal therapy, and attention to power in the hierarchy.
- **A. Carl Whitaker** — Owner: Whitaker (symbolic-experiential). Whitaker uses spontaneity, absurdity, and co-therapy.
- **C. Salvador Minuchin** — Owner: Minuchin (structural). Minuchin also cares about hierarchy, but his tools are boundaries, joining, and enactment, not ordeals.
- **D. Murray Bowen** — Owner: Bowen (multigenerational). Bowen works on differentiation and triangles as a calm coach, not through directives.
- **Pair to learn:** Directives / ordeal → Haley (strategic)

155 — A

Core Counseling Attributes · Outline 6.A · Chapter 22

- **A. Blind area — Owner: Luft & Ingham (Johari Window): blind area.** The blind area holds what others see but the person doesn't; feedback from supervisors reveals it.
- **B. Hidden area** — Owner: Luft & Ingham (Johari Window): hidden area. The hidden area holds what the person knows but keeps from others; the counselor didn't know about this habit.
- **C. Unknown area** — Owner: Luft & Ingham (Johari Window): unknown area. The unknown area is unseen by self and others; the supervisor clearly saw this behavior.
- **D. Open area** — Owner: Luft & Ingham (Johari Window): open area. The open area is known to both self and others; the counselor wasn't aware of the behavior.
- **Pair to learn:** Supervisor feedback reveals what the counselor can't see → Blind area (Johari Window)

156 — D

Counseling Skills & Interventions · Outline 5.I · Chapter 7

- **D. A gap in acculturation between generations — Owner: Berry (acculturation).** Framing the conflict as an acculturation difference avoids labeling either side as defiant or rigid.
- **A. The parents' overly rigid family boundaries** — Owner: Minuchin (structural). Labeling the parents' norms as rigid applies one culture's standard before exploring theirs.
- **B. The teen's oppositional defiance toward parents** — Owner: Individual diagnosis of the IP. Treating the teen as the problem ignores the family pattern and the cultural context.
- **C. An emotional cutoff between the generations** — Owner: Bowen (multigenerational). A cutoff is a

severed relationship; this family is highly engaged, just in conflict.

- **Pair to learn:** Immigrant parent-teen conflict → acculturation gap

157 — B

Areas of Clinical Focus · Outline 3.AS · Chapter 13

- **B. Paul Amato — Owner: Amato (conflict over divorce).** Amato found that children from high-conflict marriages may do better after divorce, while those from low-conflict marriages may struggle more.
- **A. Judith Wallerstein** — Owner: Wallerstein. Wallerstein's sleeper effect concerns problems surfacing in adulthood.
- **C. E. Mavis Hetherington** — Owner: Hetherington. Hetherington's main finding is that most children adjust well, not this conflict comparison.
- **D. Diana Baumrind** — Owner: Baumrind (parenting styles). Baumrind described parenting styles, not the effects of marital conflict on divorce outcomes.
- **Pair to learn:** Conflict matters more than divorce → Amato

158 — C

Areas of Clinical Focus · Outline 3.T · Chapter 10

- **C. Inflated responsibility — Owner: Paul Salkovskis.** Salkovskis held that believing one is responsible for preventing harm drives neutralizing and compulsions.
- **A. Learned helplessness** — Owner: Martin Seligman. Learned helplessness leads to giving up, not to repeated checking.
- **B. Avoidance theory of worry** — Owner: Thomas Borkovec. Borkovec's theory explains verbal worry in GAD, not compulsive checking.

- **D. Intolerance of uncertainty** — Owner: Michel Dugas (GAD). Dugas's model explains GAD worry; the stem emphasizes personal blame for harm.
- **Pair to learn:** Inflated responsibility → Salkovskis

159 — A

Counseling Skills & Interventions · Outline 5.AB · Chapter 4

- **A. "Crying then may show how much you cared about the work." — Owner: Reframe.** It offers a new, believable meaning for the same facts.
- **B. "I wonder if crying stirs up how you were told not to cry as a boy."** — Owner: Interpretation. It proposes a connection to the past rather than a new angle on the present facts.
- **C. "You felt embarrassed after crying in front of your team."** — Owner: Reflection of feeling. It names an emotion but leaves the client's meaning unchanged.
- **D. "You call yourself weak, yet you led the team to the final round."** — Owner: Confrontation. It points out a discrepancy instead of offering a new meaning.
- **Pair to learn:** New angle on the same facts → reframe

160 — A

Areas of Clinical Focus · Outline 3.U · Chapter 14

- **A. Theory of Work Adjustment — Owner: Dawis & Lofquist.** TWA describes correspondence between worker and environment through satisfaction and satisfactoriness.
- **B. Holland's theory of vocational personalities** — Owner: John Holland. Holland's congruence matches type to environment but doesn't use satisfactoriness.
- **C. Social cognitive career theory** — Owner: Lent, Brown, & Hackett. SCCT centers on self-efficacy and outcome expectations.

- **D. Hoppock's needs-based theory** — Owner: Robert Hoppock. Hoppock focused on job satisfaction as needs being met, without the employer's satisfactoriness.
- **Pair to learn:** Satisfaction and satisfactoriness → Dawis & Lofquist (TWA)

161 — D

Core Counseling Attributes · Outline 6.E · Chapter 22

- **D. "I wonder what it's like to talk about this with someone of a different race." — Owner: Broaching (cultural humility).** Broaching culture respectfully invites the client's experience and acknowledges the counselor's own position and power.
- **A. "Let's focus on what you can control, like your work habits."** — Owner: Avoiding the cultural topic. Redirecting away from racism avoids the topic and implies the problem is the client's behavior.
- **B. "Many people from your background deal with that at work."** — Owner: Stereotyping (cultural encapsulation). Generalizing about the client's group assumes rather than inviting the client's own meaning.
- **C. "I try to treat everyone the same, so race won't matter here."** — Owner: Ignoring culture (not cultural humility). Declaring race irrelevant ignores culture and the power the counselor's identity carries.
- **Pair to learn:** Naming cultural difference respectfully → Broaching / cultural humility (not ignoring culture)

162 — A

Professional Practice & Ethics · Outline 1.B · Chapter 18

- **A. Fewer Type I errors, more Type II errors — Owner: Alpha and error trade-off.** Lowering alpha reduces false positives but makes it harder to detect real effects, increasing Type II errors.
- **B. A larger effect size** — Owner: Effect size (Cohen's d). Effect size reflects the magnitude of the effect, which alpha doesn't change.

- **C. Fewer Type II errors, more Type I errors** — Owner: Reversed trade-off (Type I vs. Type II confusion). This reverses the relationship; a stricter alpha raises Type II risk.
- **D. Greater statistical power** — Owner: Power (1 − β). A lower alpha decreases power; a higher alpha increases it.
- **Pair to learn:** Lower alpha → Fewer Type I, more Type II errors

163 — A

Counseling Skills & Interventions · Outline 5.AC · Chapter 4

- **A. Robert Carkhuff — Owner: Carkhuff (empathy scale).** Carkhuff's scale runs from levels that subtract (1 and 2) to interchangeable (3) to additive (4 and 5).
- **B. Gerard Egan** — Owner: Egan (The Skilled Helper). Egan owns SOLER and a three-stage helping model.
- **C. Clara Hill** — Owner: Hill (helping skills model). Hill's model moves from exploration to insight to action.
- **D. Edward Bordin** — Owner: Bordin (working alliance). Bordin described the working alliance as goals, tasks, and bond.
- **Pair to learn:** Five-level empathy scale → Carkhuff

164 — D

Treatment Planning · Outline 4.N · Chapter 21

- **D. GIRP — Owner: GIRP note format.** GIRP stands for Goal, Intervention, Response, Plan.
- **A. DAP** — Owner: DAP note format. DAP stands for Data, Assessment, Plan.
- **B. BIRP** — Owner: BIRP note format. BIRP begins with Behavior, the client's presentation, not Goal.
- **C. SOAP** — Owner: SOAP note format. SOAP stands for Subjective, Objective, Assessment, Plan.

- **Pair to learn:** Goal, Intervention, Response, Plan → GIRP

165 — D

Areas of Clinical Focus · Outline 3.AE · Chapter 16

- **D. Kaplan's triphasic model — Owner: Helen Singer Kaplan.** Kaplan added desire to the model of sexual response.
- **A. Annon's PLISSIT model** — Owner: Jack Annon. PLISSIT is a model of sex counseling levels, not of sexual response.
- **B. Masters and Johnson's four phases** — Owner: Masters & Johnson. Their model is excitement, plateau, orgasm, and resolution, without a desire phase.
- **C. Basson's circular model** — Owner: Rosemary Basson. Basson described responsive desire arising after arousal, with intimacy central.
- **Pair to learn:** Desire, excitement, orgasm → Kaplan

166 — D

Areas of Clinical Focus · Outline 3.AD · Chapter 3

- **D. Disorganized attachment — Owner: Main & Solomon.** Contradictory, confused behavior on reunion is the disorganized classification that Main and Solomon added.
- **A. Anxious-resistant attachment** — Owner: Ainsworth (Strange Situation). Resistant infants seek contact and then angrily resist it; they don't freeze in a dazed, contradictory way.
- **B. Slow-to-warm-up temperament** — Owner: Thomas & Chess (temperament). Temperament describes inborn style, not an attachment pattern seen on reunion.
- **C. Anxious-avoidant attachment** — Owner: Ainsworth (Strange Situation). Avoidant infants consistently ignore the caregiver on reunion rather than approaching and freezing.

- **Pair to learn:** Disorganized attachment → Main & Solomon

167 — A

Intake, Assessment & Diagnosis · Outline 2.I · Chapter 19

- **A. Decline joint sessions and offer individual counseling with safety planning — Owner: Individual modality when coercive control is present.** Couple work is indicated only when there is no ongoing violence or coercive control; individual sessions allow safety assessment and planning.
- **B. Refer the controlling partner to an anger management class** — Owner: Anger management alone (not batterer intervention). Batterer intervention programs, not anger management alone, are the standard referral for those who use violence or control.
- **C. Begin couples counseling that focuses on communication skills** — Owner: Couple modality (contraindicated with coercive control). Conjoint work while coercive control is ongoing can increase danger and keep the at-risk partner from speaking freely.
- **D. Raise the disclosure with both partners in the next joint session** — Owner: Conjoint confrontation (breaks private IPV screening). IPV is screened privately; exposing the disclosure in a joint session can put the at-risk partner in danger.
- **Pair to learn:** Coercive control present → Individual modality (couples counseling contraindicated)

168 — C

Counseling Skills & Interventions · Outline 5.Q · Chapter 6

- **C. Acquired capability — Owner: Joiner (interpersonal theory).** Reduced fear of death and increased pain tolerance make an attempt possible.
- **A. Psychache** — Owner: Shneidman (suicidology). Psychache is Shneidman's term for intolerable psychological pain.

- **B. Anomie** — Owner: Durkheim (sociological types). Anomic suicide is Durkheim's sociological type tied to disrupted social norms.
- **D. Perceived burdensomeness** — Owner: Joiner (interpersonal theory). Burdensomeness, with thwarted belongingness, produces the desire for death, not the capability.
- **Pair to learn:** Acquired capability → Joiner

169 — D

Core Counseling Attributes · Outline 6.G · Chapter 22

- **D. Heinz Kohut — Owner: Kohut (self psychology).** Kohut, in self psychology, described empathy as vicarious introspection.
- **A. Carl Rogers** — Owner: Rogers (six necessary and sufficient conditions). Rogers defined empathic understanding as one of his six conditions but did not coin 'vicarious introspection.'
- **B. Edward Bordin** — Owner: Bordin (working alliance). Bordin defined the working alliance as goals, tasks, and bond.
- **C. Robert Carkhuff** — Owner: Carkhuff (five-level empathy scale). Carkhuff built a five-level scale for rating empathic responses.
- **Pair to learn:** Vicarious introspection → Kohut (not Rogers)

170 — D

Intake, Assessment & Diagnosis · Outline 2.F · Chapter 19

- **D. Flight of ideas — Owner: Flight of ideas (common in mania).** Flight of ideas is rapid jumping between loosely connected topics and is common in mania, which fits the reduced sleep and rapid talk.
- **A. Tangential thinking** — Owner: Tangential thought process. Tangential thinking drifts off the question without returning, but the rapid, loosely linked jumps in a manic picture define flight of ideas.

- **B. Loose associations** — Owner: Loose associations (common in psychosis). Loose associations have no logical connection at all; here each jump still has a loose link.
- **C. Circumstantial thinking** — Owner: Circumstantial thought process. Circumstantial thinking takes a long detour but reaches the point; it is not defined by rapid jumps.
- **Pair to learn:** Rapid jumps between loosely connected topics → Flight of ideas (not loose associations)

171 — C

Counseling Skills & Interventions · Outline 5.AL · Chapter 4

- **C. Slow down and help her ground in the room — Owner: Containment and grounding.** A flooded client needs grounding and stabilization before the session ends; detail can wait.
- **A. End the session on time and schedule the next one** — Owner: Abrupt ending. Ending without stabilizing leaves the client dysregulated as she leaves.
- **B. Reassure her that she is completely safe now** — Owner: Reassurance that dismisses the feeling. Reassurance skips the client's present experience and doesn't ground her.
- **D. Encourage her to keep going while the memory is fresh** — Owner: Pushing for trauma detail. Pushing for detail while a client is flooded increases dysregulation.
- **Pair to learn:** Flooded client → contain and ground before closing

172 — A

Core Counseling Attributes · Outline 6.B · Chapter 22

- **A. Genuineness — Owner: Genuineness (Rogers; Truax & Carkhuff).** Genuineness is being real and authentic, not hiding behind a professional role or façade.

- **B. Unconditional positive regard** — Owner: Unconditional positive regard (Rogers). UPR is valuing the client as a person without conditions, not the counselor's authenticity.
- **C. Empathic attunement** — Owner: Empathic attunement (internal sensing). Attunement is the internal process of sensing the client's experience, not being free of a façade.
- **D. Nonpossessive warmth** — Owner: Truax & Carkhuff: nonpossessive warmth. Nonpossessive warmth is caring that doesn't create dependence, not authenticity.
- **Pair to learn:** Being real, no façade → Genuineness (not empathy)

173 — A

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **A. Irvin Yalom — Owner: Yalom (existential).** Yalom named these four givens; existential counseling helps clients confront them.
- **B. Viktor Frankl** — Owner: Frankl (logotherapy). Frankl's focus is the search for meaning, with techniques such as paradoxical intention and dereflection.
- **C. Abraham Maslow** — Owner: Maslow (needs theory). Maslow described a hierarchy of needs ending in self-actualization.
- **D. Fritz Perls** — Owner: Perls (Gestalt). Perls focused on here-and-now awareness and unfinished business.
- **Pair to learn:** Four givens → Yalom

174 — D

Counseling Skills & Interventions · Outline 5.J · Chapter 7

- **D. Equifinality — Owner: Systems theory.** Different starting points leading to the same outcome is equifinality.

- **A. Homeostasis** — Owner: Don Jackson. Homeostasis is a family's tendency to resist change and restore balance.
- **B. Morphogenesis** — Owner: Systems theory. Morphogenesis is change in the system's structure, pushed by positive feedback.
- **C. Multifinality** — Owner: Systems theory. Multifinality is the reverse: the same starting point leading to different outcomes.
- **Pair to learn:** Different starts, same outcome → equifinality

175 — A

Intake, Assessment & Diagnosis · Outline 2.M · Chapter 20

- **A. Sensitivity — Owner: Sensitivity (true positive rate).** Sensitivity is the ability to correctly identify people who have the condition, and screening tools prioritize it.
- **B. Specificity** — Owner: Specificity (true negative rate). Specificity is the ability to correctly identify people who don't have the condition.
- **C. Base rate** — Owner: Base rate (how common the condition is). Base rate describes how common the condition is in a population, not how well a test detects it.
- **D. Incremental validity** — Owner: Incremental validity (adds information beyond what's known). Incremental validity asks whether a test adds useful information beyond what's already known.
- **Pair to learn:** Identifies those who have the condition → Sensitivity (not specificity)

176 — A

Professional Practice & Ethics · Outline 1.F · Chapter 6

- **A. Support her in processing it; no mandated report applies — Owner: Past adult-disclosed abuse (generally not reportable).** Past abuse

disclosed by an adult, with no child currently at risk, is generally not a mandated report; the work is clinical.

- **B. Notify police under the duty to protect** — Owner: Tarasoff duty to protect. The duty to protect requires a serious threat against an identifiable victim; there is no threat here.
- **C. Report the abuse to adult protective services** — Owner: Adult protective services reporting. APS addresses current abuse of older or dependent adults, not a competent adult's childhood history.
- **D. Report the abuse to child protective services today** — Owner: Mandated child abuse reporting. Child abuse reporting protects children currently at risk; none is identified here.
- **Pair to learn:** Adult's past childhood abuse, no child at risk → Generally not reportable

177 — D

Counseling Skills & Interventions · Outline 5.AT · Chapter 8

- **D. Altruism — Owner: Yalom: altruism.** Helping another member raises one's own sense of worth.
- **A. Imparting information** — Owner: Yalom: imparting information. This factor is about receiving information; the benefit here goes to the helper.
- **B. Instillation of hope** — Owner: Yalom: instillation of hope. Hope comes from seeing other members improve, not from helping them.
- **C. Universality** — Owner: Yalom: universality. Universality is the relief of learning 'I'm not the only one.'
- **Pair to learn:** Helping others raises self-worth → altruism (Yalom)

178 — A

Treatment Planning · Outline 4.P · Chapter 21

- **A. Explore his ambivalence using motivational interviewing — Owner: Adherence: motivational interviewing for ambivalence.** When ambivalence is the barrier to adherence, MI explores it without judgment.
- **B. Increase the homework load to build discipline** — Owner: Escalating demands. More homework doesn't address the ambivalence behind nonadherence.
- **C. Remind him that homework is required to continue** — Owner: Compliance (obedience framing). Demanding compliance replaces collaboration with obedience.
- **D. Drop homework from the plan to avoid any further conflict** — Owner: Abandoning the rationale. Removing the task skips exploring barriers and explaining its value.
- **Pair to learn:** Ambivalence about homework → MI to support adherence (4.P)

179 — B

Intake, Assessment & Diagnosis · Outline 2.A · Chapter 19

- **B. Predisposing factor — Owner: 4 Ps: predisposing (vulnerability).** Predisposing factors are what made the client vulnerable, such as genetics, early experiences, and temperament.
- **A. Protective factor** — Owner: 4 Ps: protective (strengths and supports). Protective factors are strengths and supports that help; a family history of panic disorder adds risk.
- **C. Perpetuating factor** — Owner: 4 Ps: perpetuating (maintains the problem). Perpetuating factors keep the problem going, such as avoidance or substance use, not inherited risk.
- **D. Precipitating factor** — Owner: 4 Ps: precipitating (trigger). A precipitating factor is what triggered the

current problem; family history is a background vulnerability, not a trigger.

- **Pair to learn:** Genetic vulnerability → Predisposing factor (4 Ps)

180 — C

Intake, Assessment & Diagnosis · Outline 2.M · Chapter 3

- **C. Predictive validity — Owner: Criterion validity: predictive.** Predictive validity asks whether a test forecasts a future criterion, here later grades.
- **A. Content validity** — Owner: Content validity (samples the domain; expert review). Content validity is judged by expert review of whether items sample the whole domain.
- **B. Discriminant validity** — Owner: Construct validity: discriminant. Discriminant validity shows a test does not correlate with unrelated measures; it is not about forecasting.
- **D. Concurrent validity** — Owner: Criterion validity: concurrent. Concurrent validity correlates a test with a criterion measured now, not a year later.
- **Pair to learn:** Predicts later performance → Predictive validity (not concurrent)

181 — C

Professional Practice & Ethics · Outline 1.F · Chapter 17

- **C. May be justified in disclosing to the identifiable partner — Owner: ACA B.2.c contagious, life-threatening diseases.** After assessing whether the client intends to inform the partner, disclosure to an identifiable third party at serious and foreseeable risk may be justified, subject to state law.
- **A. May keep it confidential as an end-of-life decision** — Owner: ACA B.2.b end-of-life decisions. B. 2.b covers terminally ill clients considering hastening death, not risk to a partner.
- **B. Must keep this confidential as he is a competent adult** — Owner: Absolute confidentiality

(not the ACA standard). Confidentiality has limits, and B.2.c names this situation as one of them.

- **D. Must file a report with child protective services** — Owner: Mandated child abuse reporting. CPS reporting applies to suspected child abuse or neglect, not to disease transmission between adults.
- **Pair to learn:** Identifiable partner at risk of contagious disease → ACA B.2.c

182 — C

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **C. The miracle question — Owner: de Shazer & Berg (SFBT).** Imagining a solved problem and its first small signs is the SFBT miracle question.
- **A. An externalizing question** — Owner: White & Epston (narrative). Externalizing questions separate the person from the problem by naming it as an outside entity.
- **B. Adler's "the Question"** — Owner: Adler (individual psychology). Adler asks what would be different if the client were well, to learn what the symptom helps the client avoid.
- **D. A circular question** — Owner: Milan systemic. Circular questions ask how family members respond to one another, not about a solved future.
- **Pair to learn:** Miracle question → de Shazer & Berg (SFBT)

183 — D

Counseling Skills & Interventions · Outline 5.A · Chapter 5

- **D. Parent-Child Interaction Therapy — Owner: Eyberg (PCIT).** Live coaching of parents during interaction with a young child is PCIT.
- **A. Sandplay** — Owner: Kalff (Jungian sandplay). Sandplay is a therapist-run method in which the child builds scenes with miniatures.

- **B. Filial therapy** — Owner: Guerney (filial therapy). Filial therapy trains parents to run child-centered play sessions at home, not with live coaching.
- **C. Child-centered play therapy** — Owner: Axline / Landreth. In child-centered play therapy, the counselor plays with the child, applying Rogers' core conditions.
- **Pair to learn:** Live earpiece coaching → PCIT (Eyberg)

184 — B

Treatment Planning · Outline 4.L · Chapter 21

- **B. Reassess and revise the plan with her — Owner: Reassess when progress stalls (4.L).** New substance use and a new stressor may be blocking progress; reassessing and revising the plan together is the scored response.
- **A. Refer her out, since drinking is outside the plan** — Owner: Premature referral. Drinking calls for reassessment and possibly concurrent referral, not ending the relationship.
- **C. Switch to a new approach without discussing it** — Owner: Unilateral change (lacks collaboration). Changing approaches without the client's input undermines collaboration.
- **D. Continue the plan, since change takes time** — Owner: Continuing an ineffective plan. Repeating what isn't working is what the exam punishes.
- **Pair to learn:** Stalled progress plus new drinking → Reassess and revise the plan (4.L)

185 — D

Core Counseling Attributes · Outline 6.D · Chapter 22

- **D. Thank them briefly and use their pronouns consistently — Owner: Respect for gender identity (using the client's pronouns).** Using the client's name, pronouns, and language for their identity is the attribute the exam rewards.
- **A. Apologize at length for the mistake** — Owner: Excessive apology (centers the counselor). A long

apology shifts attention to the counselor's discomfort instead of the client.

- **B. Explain that intake records list another gender** — Owner: Imposing counselor assumptions (not cultural humility). Pointing to records overrides the client's self-identification instead of respecting it.
- **C. Ask why they decided to use those pronouns** — Owner: "Why" question (invites justification). A 'why' question asks the client to justify their identity and invites defensiveness.
- **Pair to learn:** Client states pronouns → Use them (gender sensitivity, 6.D)

186 — B

Treatment Planning · Outline 4.G · Chapter 21

- **B. Basic needs and stabilization — Owner: Sequencing: basic needs and stabilization (Maslow).** With safety addressed, housing and food come before insight-oriented work.
- **A. Exploring childhood relationship patterns** — Owner: Sequencing: longer-term growth. Insight work is longer-term growth and comes after stabilization.
- **C. Career counseling to raise her income** — Owner: Sequencing: client's later concerns. Career work may help later but doesn't address this week's eviction and hunger.
- **D. A full trauma history in the next session** — Owner: Premature trauma exploration. A detailed history doesn't address urgent basic needs.
- **Pair to learn:** Eviction and no food → Basic needs before insight (sequencing)

187 — D

Areas of Clinical Focus · Outline 3.U · Chapter 14

- **D. Vicarious trauma — Owner: McCann & Pearlman.** Lasting changes in a helper's beliefs and worldview from repeated exposure to clients' trauma is vicarious trauma.

- **A. Latent deprivation** — Owner: Jahoda. Latent deprivation explains harm from unemployment, not effects of trauma exposure.
- **B. Burnout** — Owner: Maslach. Burnout is exhaustion, depersonalization, and reduced accomplishment from chronic work stress, not changed worldview.
- **C. Compassion fatigue** — Owner: Figley. Compassion fatigue emphasizes emotional and physical exhaustion from caring, rather than altered core beliefs.
- **Pair to learn:** Changed beliefs from clients' trauma → vicarious trauma (McCann & Pearlman)

188 — A

Intake, Assessment & Diagnosis · Outline 2.C · Chapter 19

- **A. Cultural Formulation Interview — Owner: Cultural Formulation Interview (DSM-5-TR, 16 questions).** These questions belong to the CFI domain on cultural perceptions of cause, context, and support.
- **B. Mental Status Exam** — Owner: Mental Status Exam (observation of current functioning). The MSE is a snapshot of current mental functioning, based mostly on observation, not the client's view of causes.
- **C. 4 Ps case formulation** — Owner: 4 Ps case formulation (predisposing, precipitating, perpetuating, protective). The 4 Ps is a way to organize intake information, not a DSM-5-TR set of interview questions about the client's own explanation.
- **D. Structured Clinical Interview (SCID-5)** — Owner: SCID-5 (structured diagnostic interview). The SCID-5 asks fixed diagnostic questions in fixed order; it does not center the client's cultural explanation of the problem.
- **Pair to learn:** Client's own explanation of cause and context → Cultural Formulation Interview (not MSE)

189 — D

Treatment Planning · Outline 4.B · Chapter 21

- **D. Time-bound — Owner: SMART: Time-bound.** The objective has no target date or time frame, such as within 8 weeks.
- **A. Relevant** — Owner: SMART: Relevant. Nothing suggests the objective is unrelated to the client's concern.
- **B. Measurable** — Owner: SMART: Measurable. Attendance sheets make the objective measurable.
- **C. Specific** — Owner: SMART: Specific. Two support group meetings per week is already specific.
- **Pair to learn:** No target date → SMART: Time-bound

190 — A

Areas of Clinical Focus · Outline 3.AP · Chapter 13

- **A. Stigmatization — Owner: Finkelhor & Browne (traumagenic dynamics).** Shame, guilt, and a sense of being damaged becoming part of the self-image is the stigmatization dynamic.
- **B. Shattered assumptions** — Owner: Janoff-Bulman. Shattered assumptions is a general trauma concept about broken worldviews, not one of the four traumagenic dynamics.
- **C. Betrayal** — Owner: Finkelhor & Browne (traumagenic dynamics). Betrayal is broken trust in someone the child depended on.
- **D. Powerlessness** — Owner: Finkelhor & Browne (traumagenic dynamics). Powerlessness is the repeated overriding of the child's will and control.
- **Pair to learn:** Feeling damaged → stigmatization (Finkelhor & Browne)

191 — C

Core Counseling Attributes · Outline 6.A · Chapter 22

- **C. Managing countertransference — Owner: Hayes & Gelso (countertransference management).**

Hayes and Gelso identified these five factors in managing countertransference.

- **A. Building the working alliance** — Owner: Bordin (goals, tasks, bond). Bordin's working alliance has three parts: goals, tasks, and bond.
- **B. Offering the core conditions** — Owner: Truax & Carkhuff (accurate empathy, nonpossessive warmth, genuineness). Truax and Carkhuff's core conditions are accurate empathy, nonpossessive warmth, and genuineness.
- **D. Repairing alliance ruptures** — Owner: Safran & Muran (rupture and repair). Safran and Muran describe withdrawal and confrontation ruptures repaired by naming them openly, not these five factors.
- **Pair to learn:** Self-insight, self-integration, anxiety management → Managing countertransference (Hayes & Gelso)

192 — B

Areas of Clinical Focus · Outline 3.F · Chapter 15

- **B. Reverse culture shock on returning home — Owner: Gullahorn & Gullahorn (W-curve).** The W-curve adds a second dip in adjustment when the person returns to the home culture.
- **A. An initial honeymoon period** — Owner: Oberg (culture shock phases). The honeymoon period is already part of Oberg's sequence and the U-curve.
- **C. Marginalization from both cultures** — Owner: Berry (acculturation strategies). Marginalization is one of Berry's acculturation strategies.
- **D. Children acculturating faster than parents** — Owner: Acculturation gap. An acculturation gap is a family pattern, not a phase of the W-curve.
- **Pair to learn:** Second dip on returning home → W-curve (Gullahorn & Gullahorn)

193 — D

Counseling Skills & Interventions · Outline 5.AG · Chapter 7

- **D. Support the partner in disclosing, per the policy — Owner: ACA A.8 and B.4.b (no-secrets policy).** Following the policy explained during informed consent, the counselor encourages and supports the partner in disclosing.
- **A. Agree to keep the affair private** — Owner: Violating the stated secrets policy. Keeping a secret contradicts the policy both partners agreed to.
- **B. End couples counseling and refer both partners** — Owner: Premature termination. Ending counseling abandons the couple instead of following the agreed policy.
- **C. Reveal it at the next joint session without warning** — Owner: Disclosure without preparation. Revealing it without first working with the partner blindsides the client.
- **Pair to learn:** No-secrets policy → support disclosure (ACA A.8, B.4.b)

194 — D

Areas of Clinical Focus · Outline 3.U · Chapter 14

- **D. Updating in the maintenance stage — Owner: Super (life-span, life-space).** Holding, updating, and innovating are tasks of Super's maintenance stage (45 to 64).
- **A. Showing congruence** — Owner: Holland (RIASEC). Congruence is person-environment fit, not a developmental task.
- **B. Stabilizing in the establishment stage** — Owner: Super (establishment, 25-44). Stabilizing belongs to establishment; at 48, keeping current is a maintenance task.
- **C. Using planned happenstance** — Owner: Krumboltz (happenstance learning). Planned

happenstance turns unplanned events into opportunities; her learning is deliberate.

- **Pair to learn:** Holding, updating, innovating → Super's maintenance stage

195 — B

Treatment Planning · Outline 4.B · Chapter 21

- **B. Long-term goal — Owner: Long-term goal.** A broad outcome statement is a long-term goal.
- **A. Short-term objective** — Owner: Short-term objective. Objectives are measurable steps, such as a target score by a date.
- **C. Intervention** — Owner: Intervention. Interventions describe what the counselor will do.
- **D. Problem statement** — Owner: Problem statement. A problem statement describes the current concern in behavioral terms.
- **Pair to learn:** Broad outcome → Long-term goal

196 — B

Counseling Skills & Interventions · Outline 5.E · Chapter 4

- **B. Agreement on goals — Owner: Bordin (working alliance): goals.** The client and the counselor's work aren't yet aimed at an outcome the client wants, which is a goals problem.
- **A. The bond** — Owner: Bordin (working alliance): bond. Nothing here shows distrust of or conflict with the counselor.
- **C. Collaborative empiricism** — Owner: Beck (cognitive therapy). Collaborative empiricism is Beck's way of testing thoughts, not part of Bordin's alliance model.
- **D. Agreement on tasks** — Owner: Bordin (working alliance): tasks. A tasks problem is disagreement about methods, such as doubting that worksheets help.

- **Pair to learn:** Client wants a different outcome → Bordin: goals

197 — C

Counseling Skills & Interventions · Outline 5.AJ · Chapter 8

- **C. "When you checked your phone as I spoke, I felt brushed off." — Owner: Effective feedback: specific, timely, owned.** It is specific to behavior, recent, and owned by the speaker.
- **A. "A while back you seemed distracted at some point."** — Owner: Vague, untimely feedback. It isn't specific or timely, so it's hard to act on.
- **B. "You're just a really negative person to be around."** — Owner: Character-based feedback. It targets character, which the receiver can't use.
- **D. "Everyone in here thinks you're kind of selfish, honestly."** — Owner: Unowned, character-based feedback. It speaks for others and labels character instead of behavior.
- **Pair to learn:** Specific, timely, owned → effective feedback

198 — A

Core Counseling Attributes · Outline 6.I · Chapter 22

- **A. Group cohesiveness — Owner: Yalom: group cohesiveness.** Cohesiveness is described as Yalom's group equivalent of the alliance, and leaders build it through their own attributes.
- **B. Universality** — Owner: Yalom: universality. Universality is realizing one is not alone in one's problems, not the group's version of the alliance.
- **C. Altruism** — Owner: Yalom: altruism. Altruism is benefiting from helping other members, not the group's version of the alliance.
- **D. Catharsis** — Owner: Yalom: catharsis. Catharsis is emotional release, not the bond among members.

- **Pair to learn:** Group equivalent of the alliance → Group cohesiveness (Yalom)

199 — B

Core Counseling Attributes · Outline 6.F · Chapter 22

- **B. "You're feeling overlooked here. I'd like to hear more about that." — Owner: Conflict tolerance: name and explore non-defensively.** Staying present, naming the conflict, and exploring it non-defensively models respectful conflict for the group.
- **A. "I'm so sorry. I never meant to make anyone feel that way."** — Owner: Excessive apology (appeasement). Appeasing with an apology closes the conflict instead of exploring it.
- **C. "Let's talk about this privately after group ends today."** — Owner: Avoiding the conflict. Moving the conflict out of the room avoids it and misses a chance to model respectful conflict for the group.
- **D. "I give every member equal time, so that isn't accurate."** — Owner: Defensiveness (not conflict tolerance). Defending the leader escalates the conflict and is what the exam punishes.
- **Pair to learn:** Member anger at the leader → Conflict tolerance: explore non-defensively

200 — D

Counseling Skills & Interventions · Outline 5.M · Chapter 5

- **D. Treat the lapse as a learning event — Owner: Marlatt (relapse prevention).** Reframing a slip as information counters the abstinence violation effect and identifies the high-risk situation.
- **A. Start treatment over at precontemplation** — Owner: Misapplied stages of change (Prochaska & DiClemente). Relapse is a normal part of change; the client hasn't stopped recognizing the problem.
- **B. Explore the childhood roots of gambling** — Owner: Psychodynamic insight (Freud). Insight work

skips the immediate task of managing the lapse and its trigger.

- **C. Confront him about breaking his commitment** — Owner: Confrontation (contrary to MI). Confrontation adds shame and feeds the all-or-nothing reaction to the lapse.
- **Pair to learn:** Abstinence violation effect → Marlatt (relapse prevention)

Score by Domain

Domain	Items	Your correct	%
Counseling Skills & Interventions	60		
Areas of Clinical Focus	58		
Professional Practice & Ethics	24		
Intake, Assessment & Diagnosis	24		
Treatment Planning	18		
Core Counseling Attributes	16		
Total	**200**		

There’s no official NCE passing percentage. Use this table to find your two weakest domains and give them extra study time next week.

Item numbers by domain

- **Counseling Skills & Interventions:** 1, 8, 14, 16, 17, 19, 23, 26, 31, 34, 36, 38, 39, 40, 44, 45, 47, 51, 55, 59, 60, 70, 84, 86, 94, 98, 102, 103, 104, 109, 111, 112, 124, 125, 129, 132, 134, 137, 138, 143, 146, 147, 150, 151, 152, 154, 156, 159, 163, 168, 171, 173, 174, 177, 182, 183, 193, 196, 197, 200
- **Areas of Clinical Focus:** 3, 10, 13, 15, 18, 21, 24, 27, 28, 41, 42, 43, 48, 49, 53, 56, 67, 68, 69, 72, 73, 74, 75, 77, 78, 79, 82, 88, 92, 93, 99, 105, 106, 108, 110, 113, 115, 116, 118, 119, 121, 126, 128, 131, 136, 140, 144, 148, 149, 157, 158, 160, 165, 166, 187, 190, 192, 194

- **Professional Practice & Ethics:** 5, 6, 11, 32, 35, 57, 58, 63, 64, 71, 80, 83, 87, 95, 96, 97, 120, 122, 123, 130, 145, 162, 176, 181
- **Intake, Assessment & Diagnosis:** 2, 4, 20, 22, 25, 33, 37, 52, 66, 76, 89, 100, 114, 117, 127, 135, 139, 153, 167, 170, 175, 179, 180, 188
- **Treatment Planning:** 7, 9, 12, 30, 50, 54, 61, 85, 90, 101, 133, 142, 164, 178, 184, 186, 189, 195
- **Core Counseling Attributes:** 29, 46, 62, 65, 81, 91, 107, 141, 155, 161, 169, 172, 185, 191, 198, 199

Pair Review

Cover the right column. Say the owner out loud. Add every pair you missed to your error log.

#	Pair
1	Behavior loop → circular causality
2	Fakes symptoms to be seen as sick → Factitious disorder (not malingering)
3	Ruling out options by sex role → Gottfredson
4	Day program, home at night → Partial hospitalization (not residential)
5	Graduation invitation → Boundary extension, documented (ACA A.6)
6	Minor colleague violation, no harm → Informal resolution first (ACA I.2.a)
7	Client stabilizing → Step down to least restrictive level
8	SOLER is a Western norm → adapt attending (Egan)
9	Eating disorder in counseling → Concurrent medical and dietitian referral

#	Pair
10	Low response-contingent reinforcement → Lewinsohn
11	Needs exceed competence → Competence-based referral (ACA A.11.a)
12	Member leaving group → Goodbye processed with the whole group (4.I)
13	Relies on others, denies responsibility → Harren's dependent style
14	Enactment → Minuchin (structural)
15	Learning experiences; task-approach skills → Krumboltz
16	Relaxation + imagined hierarchy → Wolpe (systematic desensitization)
17	Words vs. actions discrepancy → confrontation
18	Scaffolding; ZPD → Vygotsky
19	Externalizing + unique outcomes → White & Epston (narrative)
20	Trauma at intake → Ask with care and permission, screen, don't push for detail
21	Symptoms predate use and persist in abstinence → independent disorder
22	True-score range around an observed score → SEM confidence band (±1 SEM = 68%)
23	Coercive control → Stark
24	Pragma (practical love) → Lee
25	Mean 50, SD 10 → T-score (Deviation IQ uses mean 100, SD 15)
26	Collaborative empiricism → Beck (CT)

#	Pair
27	Learned helplessness → Seligman
28	Shattered assumptions → Janoff-Bulman
29	Caring without fostering dependence → Nonpossessive warmth (Truax & Carkhuff)
30	Precontemplative client → Explore whether change is wanted (readiness)
31	Challenges to the leader → Corey transition / Tuckman storming
32	SUD program records → 42 CFR Part 2 (specific written consent)
33	Employment tests must be job-related → Griggs v. Duke Power (not Larry P.)
34	Crisis lasts about 4 to 6 weeks; prevention levels → Caplan
35	Truthful disclosure → Veracity
36	Gentle startup → antidote to criticism (Gottman)
37	Rule out substances before primary diagnosis → First's differential sequence, step 2
38	I-messages → Thomas Gordon
39	Hero, scapegoat, lost child, mascot → Wegscheider-Cruse
40	CBT-based, non-12-step → SMART Recovery
41	Worry as avoidance of arousal → Borkovec
42	Disenfranchised grief → Doka
43	Delayed expression → criteria met 6+ months after trauma
44	Outside patterns appear in group → social microcosm (Yalom)
45	"I don't see color" → microinvalidation (Sue)

#	Pair
46	Accepts the person, explores the behavior → Nonjudgmental stance (not moralizing)
47	Cohesion and risk-taking → Corey working stage
48	Intolerance of uncertainty → Dugas
49	Ethic of care → Gilligan
50	Strengths perspective → Saleebey
51	Dependency on leader → redirect to members
52	Alcohol screen for every client → AUDIT (not PCL-5)
53	Process model of emotion regulation → Gross
54	Lack of transportation → Barrier to address (4.C)
55	Conditions of worth → Rogers
56	Catastrophic misinterpretation → Clark
57	Client-held legal right in court → Privileged communication
58	Counselor relocating → Pretermination counseling and transfer (A.11.c, A.11.d)
59	Counselor-client relationship right now → immediacy
60	Values conflict → supervision, not referral (ACA A.11.b)
61	End of training placement → Forced termination
62	Client must perceive empathy and UPR → Rogers' sixth condition
63	Duty, dereliction, direct causation, damages → Malpractice

#	Pair
64	Social media friendship with a current client → Prohibited (ACA A.5.e)
65	Counselor's own reaction steering the session → Self-awareness: reflect and seek supervision
66	Never gets to the point → Tangential (not circumstantial)
67	Neglectful/uninvolved style → Maccoby & Martin
68	1 to under 6 months → schizophreniform
69	Hospitalization → manic episode regardless of duration
70	Distraction without disclosing → Stanley & Brown step 3
71	Error in a note → Dated correction, never alter the original
72	Prealcoholic → chronic phases → Jellinek
73	Persistent depressive disorder in youth → 1 year (2 years adults)
74	Faith as a means → extrinsic religiosity (Allport)
75	Parent-child relationships shape career → Roe
76	Strongest predictor of future violence → History of violence (not prior suicide attempt)
77	Susceptibility, severity, benefits, barriers, cues → health belief model (Rosenstock)
78	Seven core issues in adoption → Silverstein & Kaplan
79	Hero role → Wegscheider-Cruse
80	Evaluate during a program to improve it → Formative evaluation

#	Pair
81	Client's values differ from the counselor's → Don't impose values (ACA A.4.b)
82	Sleeper effect → Wallerstein
83	Client gift → ACA A.10.f (value, motivation, relationship)
84	Acceptance plus change → IBCT (Christensen & Jacobson)
85	Small, concrete, presence of something → Well-formed goal (de Shazer)
86	Several statements tied together → summary
87	Court or probation referral → ACA A.2.e (explain what is shared first)
88	Selection, optimization, compensation → Baltes & Baltes
89	Replaced the GAF → WHODAS 2.0 (not multiaxial Axis V)
90	Counselor's clinical impression of progress → SOAP: Assessment
91	Client's feeling from the client's frame → Empathic responding (not sympathy)
92	High-effort coping and health → John Henryism (Sherman James)
93	Restriction without body-image concern → ARFID
94	Means restriction → strongly supported prevention
95	Teacher, counselor, consultant roles → Bernard (discrimination model)
96	Control group receives part of treatment → Diffusion of treatment
97	Client in another state for telehealth → Licensure follows the client (H.1.b)

#	Pair
98	Two slashes → divorce (genogram)
99	Oscillating loss and restoration coping → Stroebe & Schut
100	Tracking progress across sessions → OQ-45 (Lambert)
101	Weekly to biweekly to monthly → Tapering
102	Differentiation of self → Bowen
103	Contemplation → decisional balance (Prochaska & DiClemente)
104	Storytelling → redirect to feelings and here-and-now
105	Tasks of mourning → Worden
106	Rumination prolongs depression → Nolen-Hoeksema
107	Inner experience matches outward expression → Congruence (Rogers)
108	Diffusion (no crisis, no commitment) → Marcia
109	Desire, Ability, Reasons, Need → change talk (MI)
110	Career adaptability (four Cs) → Savickas
111	Seven stages starting with lethality assessment → Roberts
112	Empty chair / unfinished business → Perls (Gestalt)
113	Will to meaning; attitude toward suffering → Frankl
114	Raw test data → ACA E.4: qualified personnel only
115	Transgender emergence → Lev
116	Home- and community-based, multiple systems → MST (Henggeler)

#	Pair
117	Items hang together → Internal consistency (not test-retest)
118	Keep heritage, avoid host → separation (Berry)
119	Crash (hypersomnia, hunger, low mood) → stimulant withdrawal
120	Violent history, no explicit threat → Jablonski v. United States
121	Animism → Piaget (preoperational)
122	Client declines AI note-taker → Informed consent; respect refusal
123	Same people measured twice → Dependent (paired) t-test
124	Names the value underneath → reflection of meaning
125	Feelings from past relationships onto counselor → transference (Freud)
126	4-5 criteria → moderate SUD
127	Percentile differences are unequal → Percentile rank is ordinal (not interval)
128	Intentional, repeated, power imbalance → Olweus
129	Early recollections → Adler
130	Judge-signed order → Comply with minimal disclosure (ACA B. 2.d)
131	Fantasy, immersion, awareness → Papernow
132	Telehealth emergency → confirm client's location each session
133	Maintaining gains after ending → Consolidate and generalize (4.O)
134	Shaping → Skinner (operant)
135	Client-reported emotional state → Mood (not affect)

#	Pair
136	Regaining individual identity → Bohannan's psychic station
137	Acute crisis → more directive, here-and-now
138	Stop harm, protect member → blocking
139	Criteria expected but information incomplete → Provisional diagnosis (not other specified)
140	Adult feels hurt → revenge (Dreikurs)
141	Pause after a significant disclosure → Comfortable silence (attending, 6.M)
142	Planned progress review → Share data and invite client reflection (4.M)
143	Locus of control + locus of responsibility → Sue
144	Do I have the resources? → secondary appraisal (Lazarus & Folkman)
145	Staggered intervention, withdrawal unethical → Multiple baseline
146	Moderate risk → safety plan + means restriction (least restrictive appropriate care)
147	Structured activity → process it afterward
148	Gambling 6-7 criteria → moderate (not the SUD 6+ rule)
149	Bed only for sleep; get up if awake → stimulus control (Bootzin)
150	Shame-attacking exercise → Ellis (REBT)
151	Raising race directly in session → broaching (Day-Vines)

#	Pair
152	Confrontation rupture → name and explore (Safran & Muran)
153	New risk factors after intake → Ongoing risk assessment (not one-time screening)
154	Directives / ordeal → Haley (strategic)
155	Supervisor feedback reveals what the counselor can't see → Blind area (Johari Window)
156	Immigrant parent-teen conflict → acculturation gap
157	Conflict matters more than divorce → Amato
158	Inflated responsibility → Salkovskis
159	New angle on the same facts → reframe
160	Satisfaction and satisfactoriness → Dawis & Lofquist (TWA)
161	Naming cultural difference respectfully → Broaching / cultural humility (not ignoring culture)
162	Lower alpha → Fewer Type I, more Type II errors
163	Five-level empathy scale → Carkhuff
164	Goal, Intervention, Response, Plan → GIRP
165	Desire, excitement, orgasm → Kaplan
166	Disorganized attachment → Main & Solomon
167	Coercive control present → Individual modality (couples counseling contraindicated)
168	Acquired capability → Joiner

#	Pair
169	Vicarious introspection → Kohut (not Rogers)
170	Rapid jumps between loosely connected topics → Flight of ideas (not loose associations)
171	Flooded client → contain and ground before closing
172	Being real, no façade → Genuineness (not empathy)
173	Four givens → Yalom
174	Different starts, same outcome → equifinality
175	Identifies those who have the condition → Sensitivity (not specificity)
176	Adult's past childhood abuse, no child at risk → Generally not reportable
177	Helping others raises self-worth → altruism (Yalom)
178	Ambivalence about homework → MI to support adherence (4.P)
179	Genetic vulnerability → Predisposing factor (4 Ps)
180	Predicts later performance → Predictive validity (not concurrent)
181	Identifiable partner at risk of contagious disease → ACA B.2.c
182	Miracle question → de Shazer & Berg (SFBT)
183	Live earpiece coaching → PCIT (Eyberg)
184	Stalled progress plus new drinking → Reassess and revise the plan (4.L)
185	Client states pronouns → Use them (gender sensitivity, 6.D)

#	Pair
186	Eviction and no food → Basic needs before insight (sequencing)
187	Changed beliefs from clients' trauma → vicarious trauma (McCann & Pearlman)
188	Client's own explanation of cause and context → Cultural Formulation Interview (not MSE)
189	No target date → SMART: Time-bound
190	Feeling damaged → stigmatization (Finkelhor & Browne)
191	Self-insight, self-integration, anxiety management → Managing countertransference (Hayes & Gelso)
192	Second dip on returning home → W-curve (Gullahorn & Gullahorn)
193	No-secrets policy → support disclosure (ACA A.8, B.4.b)
194	Holding, updating, innovating → Super's maintenance stage
195	Broad outcome → Long-term goal
196	Client wants a different outcome → Bordin: goals
197	Specific, timely, owned → effective feedback
198	Group equivalent of the alliance → Group cohesiveness (Yalom)
199	Member anger at the leader → Conflict tolerance: explore non-defensively
200	Abstinence violation effect → Marlatt (relapse prevention)

NCE Practice Exam 2

200 items · 3 hours 45 minutes · Answer every item.

Take this exam under timed conditions before checking the key. Use the four-step routine on every item: read the last line first, name the owner, predict, then eliminate by owner.

Pacing checkpoints: item 50 by 0:50 · item 100 by 1:40 · item 150 by 2:30 · item 200 by 3:20.

Original items written for this book. Not affiliated with or endorsed by NBCC. No content from any live examination.

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1. Interpreting a client's scores relative to her own other scores, rather than to a norm group or a fixed standard, is called:

A. Norm-referenced interpretation
B. Projective interpretation
C. Criterion-referenced interpretation
D. Ipsative interpretation

2. Without using any checklist, a counselor notices that a client wears sunglasses indoors and glances at the door each time someone passes in the hall. This information comes from:

A. Collateral information
B. Formal observation
C. Informal observation
D. Client self-report

3. Three sessions before a planned ending, a client who has met her goals says, "I'm a little nervous about doing this on my own." The best response is to:

A. Reflect her nervousness and summarize her gains
B. Add new goals so counseling can continue

C. Reassure her that she will definitely be fine
D. End counseling at this session to build independence

4. Three days after stopping years of heavy daily drinking, a client is confused and disoriented, sees insects crawling on the wall, is sweating heavily, and has a racing pulse. This most likely reflects:

A. Hallucinogen persisting perception disorder
B. Stimulant withdrawal
C. Cannabis withdrawal
D. Delirium tremens

5. In session two, a client says, “I’ve seen counselors before. None of it helped, and I doubt this will either.” The best response is:

A. “After past letdowns, you’re not sure this will be any different.”
B. “Our approach has strong research support, so give it a chance.”
C. “You say it won’t help, yet you came back today.”
D. “Why do you think the other counselors failed?”

6. Ego states labeled Parent, Adult, and Child, along with strokes, games, and crossed transactions, belong to:

A. Satir’s communication stances
B. Gottman’s Four Horsemen
C. Berne’s transactional analysis
D. Freud’s structural model

7. A counselor asks each client to complete a brief rating at every session to learn how the work is landing. The APA task forces that judged collecting client feedback demonstrably effective were led by:

A. Norcross and Lambert
B. Safran and Muran
C. Truax and Carkhuff
D. Hayes and Gelso

8. A client meets full criteria for both generalized anxiety disorder and alcohol use disorder. Neither diagnosis's criteria exclude the other. The counselor should:

A. Diagnose both and plan integrated treatment
B. Record the drinking as a Z code
C. Diagnose only the principal disorder
D. Treat the drinking first, then the anxiety

9. A counselor brings a difficult case to her peer consultation group. The client has not consented to being identified. The counselor should:

A. Share the full chart so peers can help
B. Share only needed, nonidentifying details
C. Hand clinical responsibility to the group
D. Skip consulting to protect confidentiality

10. In Ivey's microskills hierarchy, staying with the topic the client raises instead of introducing a new one is called:

A. Minimal encouragers
B. Summarizing
C. Verbal tracking
D. Immediacy

11. A client stopped drinking 14 months ago. She lives at home, takes no medication for alcohol use, and reports no use disorder criteria except occasional cravings. The correct specifier is:

A. In early remission
B. In a controlled environment
C. In sustained remission
D. On maintenance therapy

12. Which statistic tests whether an individual client's change in scores is larger than what measurement error alone could produce?

A. Coefficient of determination
B. Goal Attainment Scaling

C. Cohen’s d
D. Reliable change index

13. A client asks to read her counseling records. There is no evidence that reading them would harm her. The counselor should:

A. Show her the psychotherapy notes only
B. Allow access and help her understand them
C. Release them only under a court order
D. Refuse, since records belong to the counselor

14. Which of Rogers’ six necessary and sufficient conditions describes the client as vulnerable or anxious?

A. The client perceives empathy and positive regard
B. The client is in a state of incongruence
C. Two people are in psychological contact
D. The counselor is congruent in the relationship

15. After scoring high on Artistic interests, a client wants to know whether she has the potential to learn graphic design work. Which type of measure addresses her question?

A. An aptitude test
B. An interest inventory
C. A work values inventory
D. An achievement test

16. A manager insists that every report follow her exact formatting rules and redoes coworkers’ work. She sees her standards as simply correct and is frustrated that others don’t follow them. She reports no intrusive thoughts or rituals. This pattern best fits:

A. Obsessive-compulsive personality disorder
B. Generalized anxiety disorder
C. Hoarding disorder
D. Obsessive-compulsive disorder

17. A client arrives for his first appointment smelling of alcohol and slurring his words, and he is eager to sign the consent forms. The counselor’s best action is to:

A. Delay consent until he is sober
B. Ask a relative to sign on his behalf
C. Have him sign now and review it later
D. Proceed, since he is an adult volunteer

18. The tendency of a control group to work harder in order to compete with the treatment group is called the:

A. Rosenthal effect
B. Placebo effect
C. John Henry effect
D. Hawthorne effect

19. A 23-year-old has changed jobs and apartments three times in two years. She says she is exploring who she is, focusing on herself, and feels “not a teenager, but not really an adult either.” Which concept fits best?

A. Emerging adulthood
B. Intimacy vs. isolation
C. Age-30 transition
D. Personal fable

20. Who identified the stepfamily myths of instant love, the wicked stepmother, and the belief that a stepfamily is just like a first-marriage family?

A. Patricia Papernow
B. McGoldrick and Carter
C. Silverstein and Kaplan
D. Emily and John Visher

21. Which treatment for nightmare disorder has the client rewrite a nightmare’s script while awake and rehearse the new version?

A. Sensate focus
B. Stimulus control
C. Sleep restriction
D. Imagery rehearsal therapy

22. To reduce a teen’s curfew violations, his parents take away his phone for the weekend each time he comes home late. In operant terms, this is:

A. Negative reinforcement
B. Extinction
C. Positive punishment
D. Negative punishment

23. A study finds that a mindfulness program lowers stress much more for first-year college students than for seniors. In this study, class year is best described as a:

A. Moderator
B. Mediator
C. Confounding variable
D. Dependent variable

24. A daughter caring for her mother with dementia reports exhaustion, isolation, and guilt about considering residential care. To assess her caregiver burden, the counselor could use the:

A. Social Readjustment Rating Scale
B. Maslach Burnout Inventory
C. Zarit Burden Interview
D. Geriatric Depression Scale

25. A 79-year-old client wants to look back over his whole life, from childhood on, to resolve old regrets and find meaning. The best-matched approach is:

A. Behavioral activation
B. Life review
C. Filial therapy
D. Reminiscence therapy

26. A counselor who finds prayer helpful in her own life wants to open every session with a brief prayer. According to the ASERVIC competencies, she should:

A. Pray with all clients, since faith is protective
B. Refer religious clients to clergy instead
C. Use prayer only when it fits and the client consents
D. Avoid all talk of spirituality to stay neutral

27. A counselor wants an instrument designed specifically to measure a college student's career indecision. Which one fits?

A. Career Beliefs Inventory
B. Career Maturity Inventory
C. Career Decision Scale
D. Minnesota Importance Questionnaire

28. Communication, Analysis, Synthesis, Valuing, and Execution make up the decision cycle in which career theory?

A. Social Cognitive Career Theory
B. Cognitive Information Processing
C. Williamson's trait-factor counseling
D. Tiedeman and O'Hara's decision theory

29. "Spitting in the client's soup," "acting as if," and the push-button technique are signature techniques of:

A. Adler's individual psychology
B. Frankl's logotherapy
C. Ellis's rational emotive behavior therapy
D. Perls's Gestalt therapy

30. Under IDEA, which provision serves infants and toddlers with developmental delays from birth to age 3?

A. FERPA records rights
B. A Section 504 plan
C. Part C early intervention
D. An Individualized Education Program

31. While building a plan with a client who has recurring depression, which counselor question best identifies strengths to include?

A. Which symptom bothers you most right now?
B. What do you think is wrong with you?
C. How did your childhood shape this pattern?
D. What helped you get through hard times before?

32. An adult child of an alcoholic father says, "Nobody ever mentioned Dad's drinking. We learned not to count

on anyone and to keep our feelings to ourselves.” Who described these family rules?

A. Gerald Patterson
B. Claudia Black
C. E. M. Jellinek
D. Sharon Wegscheider-Cruse

33. A client with advanced heart failure is still receiving active treatment for her disease, but she wants a team focused on her pain, breathlessness, and quality of life. Which care model fits?

A. A health care proxy
B. Hospice care
C. A living will
D. Palliative care

34. Since an assault, a client says that at times she feels detached from herself, as if she were watching her own body from across the room. This experience is best described as:

A. Hyperfocus
B. Depersonalization
C. Derealization
D. Hypnopompic hallucination

35. A client tells her counselor she has decided not to take the antidepressant her psychiatrist recommended. The counselor’s best response is to:

A. Avoid the topic to protect the alliance
B. Respect her choice and explore her reasons and the risks
C. Insist she take it as a condition of counseling
D. Suggest a different medication she might tolerate better

36. At a first session in a small outpatient practice, a counselor learns that a client with an eating disorder needs a structured treatment setting the practice can’t provide. She is not in imminent danger. The best response is to:

A. Begin weekly sessions and reassess in a month
B. Arrange inpatient hospitalization today
C. Discuss the fit and refer to a suitable setting
D. Offer a weekly support group as a substitute

37. A counselor recommends that a client add weekly group therapy. The client declines, saying she wants to stay in individual sessions only. The counselor should:

A. Make group attendance a condition of counseling
B. Respect her choice and explain any consequences
C. Enroll her in group and revisit consent later
D. End counseling since she rejected the plan

38. Which classification system includes compulsive sexual behaviour disorder, listed as an impulse control disorder?

A. DSM-5-TR Section III
B. ICD-11
C. The DSM-IV substance abuse category
D. The DSM-5-TR addictive disorders chapter

39. Which theory holds that parents and children reinforce each other's escalating behavior, and serves as the basis for parent management training?

A. Coercion theory
B. Systematic Training for Effective Parenting
C. Goals of misbehavior
D. Parenting styles

40. One member keeps firing questions at another: "Why don't you just leave him? What are you waiting for?" The leader's best response is to:

A. Tell the targeted member what the leader thinks she should do
B. Let it continue, since members are interacting
C. Ask the questioner to share the statement behind her questions
D. Ask the other member to answer each question fully

41. Early in an intake, a counselor asks a client, “On a scale from 0 to 100, how upsetting is this for you right now?” The counselor is using:

A. A SUDS rating
B. A Likert attitude scale
C. The WHODAS 2.0
D. The OQ-45

42. In a treatment plan, “Client works rotating night shifts and has no reliable childcare” is best listed under:

A. Strengths and resources
B. Problem statement
C. Barriers
D. Responsible parties

43. Eighteen months into retirement, a client says the early travel was fun and the letdown that followed was hard. Now he is building a weekly routine of volunteering and golf that feels more realistic and satisfying. In Atchley’s model, he is in which phase?

A. Recovery
B. Reorientation
C. Disenchantment
D. Reinvest

44. After a factory closes and thousands lose their jobs overnight, the town’s suicide rate rises. Durkheim would classify these deaths as:

A. Egoistic
B. Altruistic
C. Fatalistic
D. Anomic

45. Who framed romantic love as an attachment process and described secure, anxious, and avoidant styles in adult partners?

A. Caryl Rusbult
B. Robert Sternberg

C. Hazan and Shaver
D. Mary Ainsworth

46. A client asks, “You agree I was right to quit without notice, don’t you?” The counselor values the client but doesn’t share that view. Which response best reflects unconditional positive regard?

A. “Quitting without notice wasn’t very professional.”
B. “Yes, you did exactly the right thing by leaving.”
C. “Let’s set that aside and talk about your budget.”
D. “You needed out. How are you feeling about it now?”

47. As a client jokes about losing his job, the counselor quietly senses grief beneath the humor but has not yet said anything about it. This internal process is:

A. Countertransference
B. Empathic responding
C. Immediacy
D. Empathic attunement

48. A screen with good sensitivity and specificity is used in a population where the disorder is very rare. Most of its positive results turn out to be wrong. What best explains this?

A. A floor effect on the screen
B. The condition’s low base rate
C. Poor interrater reliability
D. Weak incremental validity

49. A counselor asks a new client, “What would be different if you were well?” This question is most closely associated with:

A. Solution-focused brief therapy
B. Narrative therapy
C. Adler’s individual psychology
D. Reality therapy

50. As planned termination nears, a client whose father left when she was young says, “Of course you’re done with me too.” The counselor’s best response is to:

A. Extend counseling indefinitely until she feels ready
B. Explore her feelings about ending and earlier losses
C. Shift the session to building her relapse prevention plan
D. Remind her the end date was set together at the start

51. A client says, “My son moved across the country, and he hasn’t called in a month.” Which response is additive, at about level 4 on Carkhuff’s scale?

A. “Have you thought about texting him instead of waiting?”
B. “You really miss him since he moved so far away.”
C. “Young adults often get busy and forget to call home.”
D. “You miss him and maybe wonder where you fit in his life.”

52. In the first session of a counseling group, members are polite, discuss safe topics, and look to the leader for direction. The leader’s best response is to:

A. Address members’ feelings about the group ending
B. Push members to share their deepest secrets now
C. Provide structure and help members set goals
D. Step back and let members run the session

53. In Lincoln and Guba’s criteria for trustworthiness in qualitative research, which criterion parallels external validity?

A. Credibility
B. Dependability
C. Transferability
D. Confirmability

54. After a cancer diagnosis, a client prays for strength, leans on her congregation, and comes to see the illness as a chance for spiritual growth, while still talking openly about her fear. Pargament would describe this as:

A. Positive religious coping
B. Negative religious coping
C. Terror management
D. Spiritual bypassing

55. A client says, “I’m the first in my family to go to college, and I feel like a fraud here.” The counselor was also a first-generation student. The most empathic response is:

A. “I was first-gen too, so I know exactly how you feel.”
B. “That must be so hard. I feel really bad for you.”
C. “You earned your place there, so don’t doubt yourself.”
D. “You got in, and still you wonder if you really belong.”

56. A repeating pattern of tension-building, an acute battering incident, and then a honeymoon of apology and calm describes:

A. The Power and Control Wheel
B. Coercive control
C. Walker’s cycle of violence
D. The Danger Assessment

57. A leader of a stress-management psychoeducational group spends all 90 minutes presenting slides. The best improvement is to:

A. Leave time for members to personalize the material
B. Convert the group into a psychotherapy group
C. Add more content so members get full value
D. Have members read the slides at home instead

58. A client with chronic insomnia spends 10 hours in bed but sleeps only about 5. The counselor limits his time in bed to about 5.5 hours to build sleep drive, then gradually extends it. This CBT-I component is:

A. Stimulus control
B. Sleep hygiene
C. Progressive muscle relaxation
D. Sleep restriction

59. A mother’s adult son refuses any help for his drinking. She wants to learn to reward his sober behavior and encourage him to enter treatment without confrontation. Which approach was designed for her situation?

A. Community Reinforcement and Family Training
B. Brief Strategic Family Therapy

C. Multisystemic therapy
D. Multidimensional Family Therapy

60. Months after their youngest child leaves for college, a couple fights constantly and says, “We don’t know how to be just the two of us.” The systemic framing is difficulty with:

A. Renegotiating the couple as a dyad
B. Forming a new couple system
C. Loosening boundaries for a teen’s independence
D. Accepting shifting generational roles

61. A counselor calls a client’s daughter by the wrong name, and the client’s face falls. The response that best reflects genuineness is to:

A. Continue as though nothing happened
B. Apologize at length until the client reassures her
C. Explain that a heavy caseload makes names hard
D. Briefly own the mistake and correct it

62. A 10-year-old has stopped trying in math. He says, “I’m just dumb, so leave me alone,” and his mother says she feels hopeless about helping him. According to Dreikurs, his goal is most likely:

A. Learned helplessness
B. Industry vs. inferiority
C. Attention
D. Display of inadequacy

63. Who described compassion fatigue, the emotional and physical exhaustion of caring for suffering people that includes secondary traumatic stress?

A. Charles Figley
B. Hans Selye
C. Christina Maslach
D. McCann and Pearlman

64. A man with terminal cancer tells his counselor, “If I can just make it to my grandson’s birth in May, I’ll make peace with my brother.” This reaction best fits:

A. Yearning and searching
B. Accepting the reality of the loss
C. Relinquishing old attachments
D. Bargaining

65. A shy member watches another member calmly voice disagreement, then tries the same behavior the next week. This illustrates which of Yalom's therapeutic factors?

A. Interpersonal learning
B. Universality
C. Catharsis
D. Imitative behavior

66. Informed consent tells clients how their care will continue if the counselor dies or becomes incapacitated. The plan that names a records custodian for this purpose is often called a:

A. Release of information
B. Business associate agreement
C. Professional will
D. Discharge summary

67. Which progress note format puts both the client's report and the counselor's observations into a single first section?

A. GIRP
B. BIRP
C. DAP
D. SOAP

68. A client in her second major depressive episode recalls 5 days last year when she felt on top of the world, slept 4 hours a night, talked rapidly, and started several projects. Coworkers noticed, but she kept working normally and was never hospitalized or psychotic. The best-supported diagnosis is:

A. Persistent depressive disorder
B. Bipolar II disorder

C. Cyclothymic disorder
D. Bipolar I disorder

69. A Deaf client who uses American Sign Language requests counseling at a small agency. The agency's best response is to:

A. Communicate through written notes only
B. Provide a qualified ASL interpreter
C. Refer her elsewhere to avoid the cost
D. Have her hearing daughter interpret

70. A supervisor suggests that a client at moderate suicide risk sign a no-suicide contract. The best evidence-based alternative is to:

A. Rely on the SAD PERSONS score alone
B. Arrange involuntary hospitalization today
C. Build a written safety plan with her
D. Have her sign the contract and file a copy

71. A telehealth client begins having severe dissociative episodes during video sessions, and family members often walk into her room. Under the ACA Code, the counselor should:

A. Reassess fit and consider in-person sessions
B. Move sessions to unencrypted text for privacy
C. Verify her identity with a code word each session
D. Continue unchanged, since she consented to telehealth

72. Which type of termination describes a client who stops attending before treatment goals are met?

A. Counselor-initiated termination
B. Forced termination
C. Planned termination
D. Premature termination

73. An anxiety scale has a mean of 40 and a standard deviation of 8. A client's raw score is 52. Expressed as a T-score, her result is:

A. 62
B. 70

C. 60
D. 65

74. During conflict, a wife's heart races, she goes silent, and she turns away from her husband. Using Gottman's antidotes, the counselor coaches her in:

A. Physiological self-soothing
B. Gentle startup
C. Building a culture of appreciation
D. Taking responsibility

75. Which case held that a graduation test must cover what students were actually taught?

A. Griggs v. Duke Power Co.
B. Hobson v. Hansen
C. Larry P. v. Riles
D. Debra P. v. Turlington

76. A client whose Holland code is clearly Social works alone entering data all day. She is capable and her employer is pleased, but she feels drained and unfulfilled. The core issue is low:

A. Satisfactoriness
B. Differentiation
C. Consistency
D. Congruence

77. Which interest inventory compares a client's interests with those of people who are satisfied in various occupations?

A. Career Thoughts Inventory
B. Self-Directed Search
C. Strong Interest Inventory
D. Myers-Briggs Type Indicator

78. Which category of test measures what a person has already learned, such as the WIAT-4 or WRAT5?

A. Neuropsychological
B. Achievement

C. Aptitude
D. Intelligence

79. Parents describe their 14-year-old as “rough” with his 8-year-old brother. He repeatedly hits him and takes his things, and the younger boy is now afraid to be home alone with him. The parents call it normal fighting. The counselor should recognize this as possible:

A. Situational couple violence
B. Power-seeking misbehavior
C. Sibling abuse
D. Sibling rivalry

80. Which cultural concept of distress refers to Cambodian “wind attacks” that can resemble panic attacks?

A. Susto
B. Kufungisisa
C. Khyâl cap
D. Taijin kyofusho

81. A husband snaps angrily whenever his wife works late. The counselor helps him find, beneath the anger, a fear of being unimportant to her. This move is central to:

A. The Gottman Method
B. Bowen family systems therapy
C. Emotionally focused therapy
D. Strategic family therapy

82. Who proposed that fear is acquired through classical conditioning and maintained through operant conditioning, because avoidance reduces anxiety?

A. David Clark
B. O. Hobart Mowrer
C. Yerkes and Dodson
D. Martin Seligman

83. Which case held that a credible threat reported to a therapist by a client’s family member can trigger the duty to protect?

A. Jablonski v. United States
B. Ewing v. Goldstein
C. Tarasoff v. Regents (1976)
D. Jaffee v. Redmond

84. A client who stopped using heroin two days ago has a runny nose, watery eyes, frequent yawning, goosebumps, dilated pupils, muscle aches, and diarrhea. This presentation is most consistent with:

A. Alcohol withdrawal
B. Opioid withdrawal
C. Cannabis withdrawal
D. Stimulant intoxication

85. For blood-injection-injury phobia, which technique has clients tense their muscles during exposure to keep blood pressure from dropping?

A. Relaxation response
B. Progressive muscle relaxation
C. Applied tension
D. Interoceptive exposure

86. When a 13-year-old's parents give legal consent for counseling, the young client's own agreement to take part is called:

A. Informed consent
B. Assent
C. Authorization
D. Privileged communication

87. Months after an affair was disclosed, a couple has stabilized. They are now exploring how the relationship and each partner contributed to their vulnerability, without excusing the affair. In Baucom, Snyder, and Gordon's model, they are in which phase?

A. Awareness
B. Understanding the context
C. Emotional station
D. Dealing with the impact

88. Which is the best problem statement for a treatment plan?

A. Client has a great deal of anxiety and simply wants to feel calmer overall
B. Client will reduce panic attacks to 1 or fewer a week within 8 weeks
C. Client reports 4 panic attacks a week and missed 3 workdays this month
D. Counselor will teach the panic cycle using CBT in weekly sessions

89. Understanding a client from within the client's own culture, rather than applying the same concepts to everyone, is called:

A. Low-context communication
B. An etic approach
C. Individualism
D. An emic approach

90. A 4-year-old adopted after two years of severe neglect in an institution climbs into strangers' laps, hugs unfamiliar adults, and would leave the park with anyone without checking back with his parents. This best fits:

A. Reactive attachment disorder
B. Anxious-resistant attachment
C. Disinhibited social engagement disorder
D. Separation anxiety disorder

91. For the third week, one member talks for most of the session while others glance at the clock. The leader says, "I'd like to pause you there so others have time to respond." This skill is:

A. Blocking
B. Cutting off
C. Linking
D. Process observation

92. A strategic counselor tells a family who argue every evening to hold the argument on purpose at 7:00 p.m. for exactly fifteen minutes. This intervention is:

A. Paradoxical intention
B. Prescribing the symptom
C. Enactment
D. Circular questioning

93. A person-centered counselor trusts that every client has an inborn actualizing tendency. This concept belongs to:

A. Viktor Frankl
B. Abraham Maslow
C. Alfred Adler
D. Carl Rogers

94. In Forester-Miller and Davis's seven-step ethical decision-making model, which step comes right after applying the ACA Code of Ethics?

A. Apply the publicity test to the choice
B. Evaluate the selected course of action
C. Implement the course of action
D. Determine the nature of the dilemma

95. For 15 years, a couple has argued about his wish for quiet weekends at home and her love of busy social plans. Neither wants to change who they are. Gottman would classify this conflict as a:

A. Pursue-withdraw cycle
B. Perpetual problem
C. Solvable problem
D. Fatuous love

96. In Haley's structured first interview, which stage comes right after each member gives his or her view of the problem?

A. The task-setting stage
B. The goal-setting stage
C. The interaction stage
D. Exploring feelings and emotions

97. In session 11 of a 12-session closed group, members are quieter and two say they'll miss the group. The leader's best intervention is to:

A. Invite feelings about ending and review learning
B. Encourage members to challenge the leader's style
C. Introduce an intense new disclosure exercise
D. Add sessions so no one has to say goodbye

98. Which theory holds that expecting bad outcomes, with no way to change them, produces a distinct subtype of depression?

A. Reformulated helplessness theory
B. Cognitive triad
C. Hopelessness theory
D. Response styles theory

99. Black students score lower on a test when told it measures intellectual ability than when told the same items are a problem-solving exercise. Which concept explains the difference?

A. John Henryism
B. Stereotype threat
C. Impostor phenomenon
D. Race-based traumatic stress

100. A client's panic attacks began two weeks after he was laid off from a job he had held for 15 years. In a 4 Ps case formulation, the layoff is a:

A. Predisposing factor
B. Precipitating factor
C. Protective factor
D. Perpetuating factor

101. With a recently immigrated family, the counselor greets every member, including a quiet 7-year-old, speaks to the grandfather first as the family expects, and matches the family's pace. This is:

A. Detriangulation
B. Enactment

C. Joining
D. Circular questioning

102. A woman reports that her husband checks her phone, controls all the money, keeps her from seeing friends, and has hit her several times, with the violence getting worse. Using Johnson's typology, this pattern is:

A. Coercive family process
B. Violent resistance
C. Intimate terrorism
D. Situational couple violence

103. An 8-year-old has had severe temper outbursts about four times a week for the past 14 months, starting at age 7, and is irritable and angry most of the day between outbursts. The best fit is:

A. Intermittent explosive disorder
B. Bipolar I disorder
C. Disruptive mood dysregulation disorder
D. Oppositional defiant disorder

104. A man who has repeatedly hit his partner asks his counselor for help changing. The standard referral is:

A. A batterer intervention program
B. Conjoint couples counseling
C. An anger management class alone
D. An Al-Anon meeting

105. A client fused with the thought "I'm a failure" is helped to notice it as just a thought, without arguing with it, and to act on his values anyway. This approach is:

A. Rational emotive behavior therapy
B. Reality therapy
C. Cognitive therapy
D. Acceptance and commitment therapy

106. After a tense meeting with his supervisor, a client goes home and yells at his son over a small mess. The counselor gently helps him see the link. She is educating him about:

A. Projection
B. Reaction formation
C. Sublimation
D. Displacement

107. A client says, “I’m just over it with my job.” Which response is a clarification?

A. “Help me understand what ‘over it’ means for you.”
B. “You’re done with your job.”
C. “You should update your résumé.”
D. “You sound burned out and fed up.”

108. A client who uses a wheelchair was denied a reasonable accommodation at work. Following the ACA Advocacy Competencies, the counselor’s first move should be to:

A. Help her prepare to advocate for herself
B. Call her employer and negotiate for her
C. Launch a public campaign about workplace access
D. Refer her to an attorney and end counseling

109. Three weeks into kindergarten, a 5-year-old hasn’t spoken at school, though she talks freely at home. Her parents ask whether she has selective mutism. The most accurate answer is that:

A. Not yet; the first month of school doesn’t count
B. This fits social anxiety disorder
C. This fits separation anxiety disorder
D. She meets criteria for selective mutism now

110. Persona, shadow, archetypes, and the collective unconscious are key terms of:

A. Freud’s psychoanalysis
B. Adler’s individual psychology
C. Berne’s transactional analysis
D. Jung’s analytical psychology

111. Which score has a mean of 50 and a standard deviation of 21.06, forming an equal-interval scale from 1 to 99 used in education?

A. Stanine
B. Percentile rank
C. Normal curve equivalent
D. T-score

112. A counselor learns her client was denied housing assistance and wants to call the housing office to advocate on the client's behalf. First, the counselor should:

A. Get the client's consent
B. Post about the office's practices online
C. Stay out of it, since housing isn't counseling
D. Call the housing office right away

113. Which tool is designed to guide level-of-care decisions for children and adolescents?

A. The ASAM Criteria
B. CALOCUS/CASII
C. The WHODAS 2.0
D. LOCUS

114. On a self-report checklist, a client denies any alcohol problems. With his consent, his wife and his previous counselor both describe nightly heavy drinking. The counselor's best approach is to:

A. Accept the checklist, since self-report is most direct
B. Confront him with his wife's account right away
C. Diagnose alcohol use disorder from the collateral reports
D. Weigh all sources and explore the discrepancy with him

115. During company layoffs, an executive stays deeply involved in her work, believes her actions can shape what happens, and treats the upheaval as a chance to grow. Her stress buffer best fits:

A. Hardiness
B. Career adaptability
C. Resistance stage
D. Type A behavior

116. A client mentions, almost in passing, that she drives home after a few drinks a couple of times a month. The response that best reflects a nonjudgmental stance is:

A. “I can’t keep seeing you unless you stop this now.”
B. “Let’s stay with what you came in for today.”
C. “That’s illegal and could kill someone. You need to stop.”
D. “Thanks for telling me. How safe are those drives home?”

117. A counselor believes that making or reporting a diagnosis would harm a particular client. Which ACA standard addresses this situation?

A. E.5.b, Cultural sensitivity
B. E.5.d, Refraining from diagnosis
C. E.4, Release of data to qualified personnel
D. E.5.c, Historical and social prejudices

118. After six sessions, a client’s ORS scores are flat. Her SRS ratings show she feels the session tasks don’t fit her goals. The counselor’s best next step is to:

A. Discuss the alliance and adjust tasks with her
B. Continue the plan, since six sessions is early
C. Refer her to a higher level of care
D. Switch to a new approach without discussing it

119. An agency adds a sliding-scale fee so that clients with low incomes can receive the same services as everyone else. Which ethical principle does this most directly reflect?

A. Beneficence
B. Fidelity
C. Justice
D. Autonomy

120. Two co-leaders openly disagree in front of members about whether to continue an activity, and the session ends tense. The best next step is for the co-leaders to:

A. Ask members to vote on which leader was right
B. Never mention it, so members aren't upset
C. Meet to process the conflict and plan ahead
D. Let the senior leader make all decisions from now on

121. A year after a fire destroyed her home, a client says she now appreciates each day more, feels closer to her sisters, and has found strengths she didn't know she had. This best illustrates:

A. Window of tolerance
B. Posttraumatic growth
C. Resilience
D. Shattered assumptions

122. In a first family session, the counselor notices that the father answers every question asked of his 13-year-old daughter, while the mother stays silent and looks away. The counselor is assessing:

A. The cultural definition of the problem
B. The daughter's thought process
C. The daughter's affect
D. Interactional dynamics

123. Whose circular model holds that desire can be responsive, arising after arousal begins, with emotional intimacy at the center?

A. Helen Singer Kaplan
B. Masters and Johnson
C. Rosemary Basson
D. Jack Annon

124. A counselor helping evaluate a child for possible intellectual developmental disorder needs a measure of adaptive functioning. Which instrument fits?

A. Wechsler Intelligence Scale for Children
B. Vineland Adaptive Behavior Scales
C. Autism Diagnostic Observation Schedule
D. Conners Rating Scales

125. In James and Gilliland's six-step crisis model, which step is the first of the acting steps?

A. Provide support
B. Obtain commitment
C. Examine alternatives
D. Make psychological contact

126. Someone the counselor dated briefly in college, many years ago, calls and asks to begin counseling with her. Under the ACA Code, the counselor should:

A. Decline and refer him to another counselor
B. Accept after documenting the risks first
C. Accept if he signs a written consent
D. Accept, since more than 5 years have passed

127. A 13-year-old says she might become a veterinarian because she loves animals and does well in science, but she knows she still has a lot to figure out. Ginzberg and colleagues would place her in which period?

A. Tentative
B. Social valuation
C. Fantasy
D. Growth

128. A counselor wants to post mental health tips online to reach potential clients. Right now she has only a personal account with family photos. The best approach is to:

A. Share brief client success stories
B. Create a separate professional account
C. Post the tips from her personal account
D. Accept clients' friend requests to grow reach

129. Introjection, retroflection, confluence, and deflection are described as contact-boundary disturbances in:

A. Gestalt therapy
B. Cognitive therapy
C. Structural family therapy
D. Psychoanalysis

130. A 74-year-old retired teacher tutors children twice a week, keeps the morning walking routine she has had for decades, and still hosts the book club she started at 40. She says, “I’m the same person I’ve always been.” Which theory of aging best fits?

A. Continuity theory
B. Activity theory
C. Disengagement theory
D. Socioemotional selectivity

131. A client’s GAD-7 score has fallen from 16 at intake to 6 over ten weeks. She asks whether she is ready to move to every-other-week sessions. The counselor’s best response is to:

A. Review the scores with her and decide together
B. Refer her for a medication evaluation
C. Tell her the scores mean she must stay weekly
D. End counseling now, since her score is under 10

132. During a couples session, the partners begin talking over each other with rising voices. The counselor’s best response is to:

A. End the session until both partners have calmed down
B. Side with the partner who seems more reasonable
C. Slow things down so each partner can speak and be heard
D. Let them argue freely so pent-up anger is released

133. Studying survivors’ grief after the 1942 Cocoanut Grove nightclub fire, this theorist described acute grief reactions and helped found crisis intervention:

A. Gerald Caplan
B. Erich Lindemann
C. Kenneth Doka
D. Albert Roberts

134. A leader who names an experience shared across the whole group, such as “Several of you have felt invisible at home,” is using:

A. Linking
B. Drawing out
C. Process observation
D. Universalizing

135. A counselor who treats his own cultural values as simply "how healthy people live" and ignores cultural variation shows what Wrenn called:

A. Cultural humility
B. Cultural encapsulation
C. Acculturative stress
D. Microinvalidation

136. Intense imagined exposure to a feared situation, often exaggerated, without a gradual hierarchy, is:

A. Implosive therapy
B. Systematic desensitization
C. Interoceptive exposure
D. Flooding

137. Survival, love and belonging, power, freedom, and fun are the five basic needs in:

A. Maslow's hierarchy of needs
B. Lazarus's BASIC I.D.
C. Glasser's choice theory
D. Yalom's four givens

138. Eight months after his father's death, a 12-year-old yearns intensely for him nearly every day, feels life is meaningless, is intensely lonely, and can't rejoin friends or activities. His grief clearly exceeds his family's cultural norms. Which conclusion is best supported?

A. This is uncomplicated bereavement
B. It is too early, since 12 months must pass
C. He may meet criteria for prolonged grief disorder
D. This is persistent depressive disorder

139. A client agrees to a psychiatric medication evaluation while continuing counseling. Which plan best reflects a well-made referral?

A. Fax her records to the nearest psychiatrist today
B. Give her one name and close her counseling case
C. Tell her to find a prescriber through her insurer
D. Offer options, get a release, and follow up

140. A counselor is about to base a current placement decision on results from an outdated test edition given several years ago. Which ACA standard most directly applies?

A. E.7, Conditions of administration
B. E.11, Obsolete assessment and outdated results
C. E.10, Assessment security
D. E.8, Multicultural issues in assessment

141. A counselor realizes she tends to steer female clients toward caregiving careers and male clients toward leadership roles, whatever their stated interests. Her best response is to:

A. Disclose the pattern to each client and apologize
B. Examine her gender-role assumptions in supervision
C. Continue, since these reflect most clients' interests
D. Refer her male clients to a male colleague

142. A client checks the stove dozens of times before leaving home. The counselor has her check once, leave, and resist going back while her anxiety drops. This intervention is:

A. A thought record
B. Systematic desensitization
C. Interoceptive exposure
D. Exposure and response prevention

143. A client wants help choosing a new career, but he arrives high for most sessions and can't recall what was discussed. No safety risks are present. The first treatment priority is:

A. Career exploration
B. His substance use
C. Family-of-origin exploration
D. Long-term personal growth

144. A test has a standard deviation of 15 and a reliability coefficient of .91. Its standard error of measurement is:

A. 1.35
B. 13.65
C. 4.5
D. 14.3

145. A three-stage helping model that moves from the current picture to the preferred picture to the way forward was developed by:

A. Gerard Egan
B. Allen Ivey
C. Albert Roberts
D. Clara Hill

146. A projective test asks a client to finish sentence stems such as “I wish…” and “My mother…” This instrument is the:

A. Thematic Apperception Test
B. Kinetic Family Drawing
C. House-Tree-Person
D. Rotter Incomplete Sentences Blank

147. A member has been quiet all session but leaned forward when others discussed caring for aging parents. Which response best draws her out?

A. “Ana, everyone else has shared, so now it’s your turn.”
B. “Ana, I noticed you leaned in. Want to add anything?”
C. “Ana, your silence feels like resistance to the group.”
D. “Ana’s situation sounds just like Ben’s, I’m guessing.”

148. Under HIPAA, a counselor’s process notes, kept apart from the clinical record and containing the counselor’s analysis of session content, are called:

A. The designated record set
B. 42 CFR Part 2 records
C. Psychotherapy notes
D. SOAP progress notes

149. A client insists that his neighbor implanted a tracking device in his tooth. He holds this belief firmly even after dental X-rays show nothing. In the MSE, this is best recorded as:

A. A delusion, under thought content
B. An obsession, under thought content
C. A hallucination, under perception
D. Loose associations, under thought process

150. Which counselor statement is self-disclosure rather than immediacy?

A. “You’ve said you’re fine, yet you’ve teared up twice today.”
B. “Right now I feel a bit shut out as you keep checking your phone.”
C. “I moved far from family once too, and it took time to feel at home.”
D. “It sounds like you feel lonely in this new city.”

151. A counselor at a community agency also has a private practice. A client whose agency sessions are ending asks to keep seeing her privately. Under the ACA Code, this is acceptable only if:

A. Five years pass after agency services end
B. Agency policy allows it and she knows other options
C. Her supervisor gives informal verbal approval
D. The client pays the same fee charged at the agency

152. Kiresuk and Sherman developed which method for rating a client’s progress on individualized goals?

A. Reliable change index
B. Measurement-based care
C. Outcome Rating Scale
D. Goal Attainment Scaling

153. Exploitation, marginalization, powerlessness, cultural imperialism, and violence are the five faces of oppression described by:

A. Iris Marion Young
B. Ignacio Martín-Baró
C. Kimberlé Crenshaw
D. Paulo Freire

154. A couples technique in which one partner speaks briefly and the other must paraphrase before replying comes from:

A. Satir's human validation model
B. PREP (Markman, Stanley, & Blumberg)
C. The Gottman Method
D. Thomas Gordon's Parent Effectiveness Training

155. A client with complex trauma has been safe and stable for months and has solid coping skills. She now begins telling the full story of her abuse and grieving what it cost her. In Herman's model, she has entered:

A. Safety and stabilization
B. Remembrance and mourning
C. Disorganization and despair
D. Processing the pain of grief

156. In Super's life-span, life-space theory, stabilizing, consolidating, and advancing are tasks of which stage?

A. Establishment
B. Implementation
C. Maintenance
D. Realistic period

157. Hypothesizing, circularity, and neutrality, with a team observing from behind a mirror, are hallmarks of:

A. Satir's human validation model
B. Bowen family systems therapy
C. MRI brief strategic therapy
D. Milan systemic therapy

158. A client says, "My daughter moved out last month. I keep walking past her room." Which response is a level-4 (additive) response on Carkhuff's scale?

A. “Have you thought about turning her room into an office?”
B. “The house feels empty, and you may wonder what your role is now.”
C. “The house feels empty since she left, and you really miss her.”
D. “Kids grow up so fast these days, don’t they?”

159. During an intensive meditation retreat, a client with no history of psychosis had overwhelming visions and a sense of merging with the universe. She is frightened and struggling to function. Which concept was developed for experiences like this?

A. Existential isolation
B. Spiritual emergency
C. Spiritual bypassing
D. Terror management

160. A trainee wants to confront a client about a discrepancy in their very first session. Based on Ivey’s microskills hierarchy, the trainee should first:

A. Share a similar personal story to bond
B. Give direct advice to seem helpful
C. Offer an interpretation to show expertise
D. Attend and listen before trying to influence

161. A client says, “My landlord raised the rent again, and I had to pick up weekend shifts to cover it.” Which response is a paraphrase?

A. “Having a stable home really matters to you.”
B. “So the rent went up, and now you’re working weekends.”
C. “You sound stretched thin and anxious about money.”
D. “How many weekend shifts are you working now?”

162. For 8 months, a client has avoided buses and crowded stores because she fears she couldn’t get out or get help if she suddenly felt faint. She is not worried about being judged by others. This best fits:

A. Specific phobia, situational type
B. Social anxiety disorder
C. Agoraphobia
D. Panic disorder

163. After months of 50-hour weeks, a counselor notices she is short with clients and drifts during sessions. Her best next step is to:

A. Refer all of her current clients to colleagues
B. Tell her clients she is burned out and on edge
C. Push through, since her skills are still adequate
D. Address her self-care and raise it in supervision

164. Since his divorce, a man chats with coworkers daily and plays in a weekend basketball league, yet he feels painfully lonely because he has no one close to share his life with. Weiss would call this:

A. Ambiguous loss
B. Social loneliness
C. Social isolation
D. Emotional loneliness

165. Which framework uses six dimensions, including withdrawal potential and recovery environment, to match clients with substance use problems to a level of care?

A. SBIRT
B. ASAM Criteria
C. CRAFFT
D. Addiction Severity Index

166. A counselor joins an integrated care team with a physician and a psychiatric nurse practitioner who will discuss shared clients. Before discussing a client with the team, the counselor should:

A. Tell the client about the team and what is shared
B. Wait until the client asks about the team
C. Share the full record so care is coordinated
D. Keep all client information from the team

167. Earlier, a client described waiting at the window for a father who often didn’t show up. Now she says she keeps dating people who don’t call back. The counselor says, “I wonder if that pattern connects to waiting for your dad.” This is:

A. Confrontation
B. Reframe
C. Reflection of meaning
D. Interpretation

168. A client mentions that she is observing a religious fast this month, one the counselor knows little about. The response that best reflects cultural humility is:

A. “I’d like to understand what this fast means for you.”
B. “Let’s leave religion aside and focus on your stress.”
C. “I’ve read about that fast. It’s mainly about discipline.”
D. “A counselor who shares your faith might suit you better.”

169. In the mental status exam, affect that shows virtually no emotional expression is described as:

A. Incongruent
B. Labile
C. Blunted
D. Flat

170. In a busy emergency department, a counselor sees clear, impairing depressive symptoms but has no time to sort out which specific disorder, if any, fully fits. The most fitting DSM-5-TR designation is:

A. Provisional major depressive disorder
B. Unspecified depressive disorder
C. A Z code for a problem in living
D. Other specified depressive disorder

171. A clinic director wants to know whether clients referred by courts, schools, and physicians differ in how many complete treatment versus drop out. The data are counts in each category. The appropriate test is:

A. Chi-square
B. Kruskal-Wallis
C. Pearson r
D. One-way ANOVA

172. Whose cognitive theory of OCD holds that nearly everyone has intrusive thoughts, and that OCD develops from catastrophic misinterpretation of what those thoughts signify about the person?

A. Paul Salkovskis
B. Adrian Wells
C. Edna Foa
D. Stanley Rachman

173. Two sisters live through the same parental divorce. One becomes anxious and withdrawn; the other becomes more confident and independent. This illustrates:

A. Circular causality
B. Homeostasis
C. Multifinality
D. Equifinality

174. A client arrives 20 minutes late for the third week in a row, and the counselor feels a flash of frustration. Which response is congruent and serves the client?

A. “Honestly, I’m really annoyed with you right now.”
B. “Starting late throws me a bit. What’s getting in the way?”
C. “No problem at all! I’m just so glad you made it.”
D. “Being late again breaks the agreement we made together.”

175. A group leader invites a member near the end of treatment to tell newer members how her depression has lifted. For the newer members, the leader is mainly fostering which of Yalom’s therapeutic factors?

A. Universality
B. Altruism
C. Catharsis
D. Instillation of hope

176. Which instrument was developed to assess lethality risk in intimate partner violence?

A. The Danger Assessment
B. The Power and Control Wheel
C. The Beck Hopelessness Scale
D. The C-SSRS

177. A document that records the reason for services, the treatment provided, progress, status at discharge, and recommendations is a:

A. Discharge summary
B. Psychotherapy notes
C. Treatment plan
D. BIRP note

178. A 15-year-old's parents want counseling to focus on his grades. He says he mostly wants help with constant fights at home. In setting goals, the counselor should:

A. Adopt the parents' goal, since they consented
B. Involve his parents while honoring his goals
C. Adopt only his goal and leave the parents out
D. Set the goals herself as the expert

179. Stress inoculation training, which moves from conceptualization to skill acquisition to application, was developed by:

A. Donald Meichenbaum
B. Marsha Linehan
C. Aaron Beck
D. Joseph Wolpe

180. A client describes himself as "Deaf, part of the Deaf community." His referral form says "hearing impaired." The counselor's best approach is to:

A. Use his own language for his identity
B. Avoid mentioning deafness to stay neutral
C. Treat his hearing loss as the presenting problem
D. Use the referral's clinical term for accuracy

181. A counselor demonstrates how to ask a landlord for repairs, then has the client rehearse the request until she feels sure she can do it. The concept the counselor most aims to build is:

A. Unconditional self-acceptance
B. Social interest
C. Reciprocal inhibition
D. Self-efficacy

182. A client who uses opioids says she isn't ready to stop. The counselor gives her information on naloxone access and safer use while continuing to engage her. This reflects:

A. Harm reduction
B. 12-step facilitation
C. The disease model
D. The moral model

183. Who described gerotranscendence, a late-life shift toward a more cosmic and less materialistic perspective?

A. Robert Atchley
B. Robert Butler
C. Lars Tornstam
D. Laura Carstensen

184. A widowed mother of three says nothing is going right. The counselor asks, "With so much on your plate, how have you managed to keep going each day?" This is:

A. A unique outcome question
B. A coping question
C. A scaling question
D. Developing discrepancy

185. A new client asks whether she can text her counselor between sessions. The informed consent discussion should explain:

A. That texting is fine for any clinical topic
B. Its uses, reply times, and no emergency use

C. That texts are fully secure and private
D. That she should text if she feels suicidal at night

186. A client overwhelmed by credit card debt asks her counselor which debt consolidation loan she should choose. The best response is to:

A. Compare the loan offers with her and recommend the best one
B. Teach only emotion-focused coping, since debt can't change
C. Set the money topic aside until her anxiety symptoms resolve
D. Explore her money stress, and refer her to a credit counselor

187. A new depression scale correlates strongly with similar depression measures and weakly with measures of unrelated traits. This pattern is evidence of:

A. Construct validity
B. Face validity
C. Content validity
D. Predictive validity

188. A client says, "I agree with what we're working toward, and the exercises make sense, but I'm not sure I can trust you with the hard stuff." Which part of Bordin's working alliance needs attention?

A. The bond
B. Agreement on tasks
C. Unconditional positive regard
D. Agreement on goals

189. A new member will join an ongoing counseling group next week. The leader's best preparation is to:

A. Prepare both the new member and the group
B. Let her join without telling the group
C. Address her arrival only if conflict arises
D. Postpone her entry until the group ends

190. A client says she plans to overdose this weekend on pills she has stockpiled at home and intends to do it. She is willing to accept help. The counselor's best action is to:

A. Increase sessions to twice weekly and involve her partner
B. Have her sign a no-suicide contract before she leaves
C. Write a safety plan together and see her again next week
D. Stay with her and arrange an immediate hospital evaluation

191. Which type of interview uses fixed questions in a fixed order and offers the highest reliability for research and diagnosis?

A. A semi-structured interview with open follow-up
B. An unstructured interview that follows the client
C. A structured interview such as the SCID-5
D. The Cultural Formulation Interview

192. A client says, "Lately I just want everything to stop." Which question best assesses suicide risk?

A. "You're not thinking of hurting yourself, are you?"
B. "Why would you want everything to stop?"
C. "Are you having thoughts of killing yourself?"
D. "What do you mean by 'everything'?"

193. A clinic's AI screening app rates a client as low suicide risk. In session, the client mentions she has written goodbye letters to her children. The counselor should:

A. Ask her to sign a no-suicide contract
B. Rely on the app's low-risk rating
C. Assess her suicide risk directly
D. Hospitalize her involuntarily at once

194. A new client reports four weeks of fatigue, low mood, weight gain, and slowed thinking. She hasn't seen a physician in three years. Along with assessing mood and safety, the counselor's priority is to:

A. Begin behavioral activation right away
B. Make sure she gets a medical evaluation
C. Suggest she ask about an antidepressant
D. Diagnose major depression from the PHQ-9

195. A client says she plans to overdose this weekend, has pills saved at home, and intends to act. She refuses to go to the hospital voluntarily. The best response is to:

A. Stay with her and arrange a hospital evaluation
B. Ask her to sign a no-suicide contract
C. Complete a safety plan and see her next week
D. Explore the childhood roots of her hopelessness

196. During an argument about money, a husband answers his wife's tears with calm statistics and detached logic, showing no feeling. Which concept best describes his communication?

A. Emotional cutoff
B. Stonewalling
C. Placater stance
D. Super-reasonable stance

197. A 15-year-old whose father drinks heavily wants a peer support group for himself. The best referral is:

A. Alateen
B. SMART Recovery
C. Adult Children of Alcoholics
D. Narcotics Anonymous

198. Research showing that family rejection of LGBTQ youth is linked to much higher risk of depression, suicide attempts, and substance use, while family acceptance is protective, comes from:

A. Ilan Meyer's minority stress model
B. Anthony D'Augelli's life-span model
C. Richard Troiden's identity model
D. Caitlin Ryan's Family Acceptance Project

199. With the parents' signed release, a school asks for a written summary of a student's counseling assessment. The counselor's report should:

A. Present clinical opinions as established fact
B. Include every test score and session detail
C. Use plain language and note the data's limits
D. Use technical terms to show expertise

200. Since her mother died two months ago, a colleague has seemed exhausted, has missed appointments, and once fell asleep in a team meeting. No client harm is known. The counselor's best first step is to:

A. Warn the colleague's clients about her
B. File a complaint with the licensing board
C. Say nothing, since grief is private
D. Consult with and support the colleague

NCE Practice Exam 2 — Answer Key with Ownership Map

Every rationale names who or what **owns** each option. Read all four, not just the correct one: each item teaches four pairings.

How to review: for every item you missed *or guessed*, write in your error log (Workbook, Part 4) the owner you picked and the owner of the correct answer.

Quick Key

1. D	2. C	3. A	4. D	5. A	6. C	7. A	8. A	9. B	10. C
11. C	12. D	13. B	14. B	15. A	16. A	17. A	18. C	19. A	20. D
21. D	22. D	23. A	24. C	25. B	26. C	27. C	28. B	29. A	30. C
31. D	32. B	33. D	34. B	35. B	36. C	37. B	38. B	39. A	40. C
41. A	42. C	43. B	44. D	45. C	46. D	47. D	48. B	49. C	50. B
51. D	52. C	53. C	54. A	55. D	56. C	57. A	58. D	59. A	60. A
61. D	62. D	63. A	64. D	65. D	66. C	67. C	68. B	69. B	70. C
71. A	72. D	73. D	74. A	75. D	76. D	77. C	78. B	79. C	80. C
81. C	82. B	83. B	84. B	85. C	86. B	87. B	88. C	89. D	90. C
91. B	92. B	93. D	94. D	95. B	96. C	97. A	98. C	99. B	100. B
101. C	102. C	103. C	104. A	105. D	106. D	107. A	108. A	109. A	110. D
111. C	112. A	113. B	114. D	115. A	116. D	117. B	118. A	119. C	120. C

121. B	122. D	123. C	124. B	125. C	126. A	127. A	128. B	129. A	130. A
131. A	132. C	133. B	134. D	135. B	136. A	137. C	138. C	139. D	140. B
141. B	142. D	143. B	144. C	145. A	146. D	147. B	148. C	149. A	150. C
151. B	152. D	153. A	154. B	155. B	156. A	157. D	158. B	159. B	160. D
161. B	162. C	163. D	164. D	165. B	166. A	167. D	168. A	169. D	170. B
171. A	172. D	173. C	174. B	175. D	176. A	177. A	178. B	179. A	180. A
181. D	182. A	183. C	184. B	185. B	186. D	187. A	188. A	189. A	190. D
191. C	192. C	193. C	194. B	195. A	196. D	197. A	198. D	199. C	200. D

Rationales

1 — D

Intake, Assessment & Diagnosis · Outline 2.M · Chapter 20

- **D. Ipsative interpretation — Owner: Ipsative (compared with oneself).** Ipsative interpretation compares a person with herself, such as her strongest interest area relative to her other areas.
- **A. Norm-referenced interpretation** — Owner: Norm-referenced (compared with others). Norm-referenced interpretation compares the client with a norm group of other people.
- **B. Projective interpretation** — Owner: Projective testing (ambiguous stimuli). Projective tests use ambiguous stimuli and open responses; the term doesn’t describe the comparison being made.
- **C. Criterion-referenced interpretation** — Owner: Criterion-referenced (compared with a fixed standard). Criterion-referenced interpretation compares

performance with a fixed standard, like passing a driving test.

- **Pair to learn:** Compared with oneself → Ipsative (not norm-referenced)

2 — C

Intake, Assessment & Diagnosis · Outline 2.N · Chapter 19

- **C. Informal observation — Owner: Informal observation (2.N).** Informal observation is what the counselor notices during the session, such as appearance, behavior, and interaction, without a structured method.
- **A. Collateral information** — Owner: Collateral sources (family, teachers, prior providers, records). Collateral information comes from other people or records, with consent, not from the counselor's own observation.
- **B. Formal observation** — Owner: Formal observation (rating scales, timed observations). Formal observation uses structured methods such as behavior rating scales or timed observations; none was used here.
- **D. Client self-report** — Owner: Self-report (2.Q). Self-report comes from what the client says or writes in interviews, questionnaires, or checklists, not from what the counselor sees.
- **Pair to learn:** Noticed in session without a structured method → Informal observation (not formal)

3 — A

Counseling Skills & Interventions · Outline 5.AA · Chapter 4

- **A. Reflect her nervousness and summarize her gains — Owner: Ending stage: process feelings, consolidate gains.** The ending stage calls for processing feelings about ending and summarizing progress.

- **B. Add new goals so counseling can continue** — Owner: Fostering dependence. Extending counseling to avoid ending doesn't serve a client who has met her goals.
- **C. Reassure her that she will definitely be fine** — Owner: Reassurance that dismisses the feeling. Reassurance skips her feeling instead of exploring it.
- **D. End counseling at this session to build independence** — Owner: Ending support abruptly. Ending abruptly skips the planned processing of termination.
- **Pair to learn:** Ending stage → summarize progress and process feelings about ending

4 — D

Areas of Clinical Focus · Outline 3.AI · Chapter 11

- **D. Delirium tremens — Owner: Alcohol withdrawal (potentially life-threatening).** Confusion, hallucinations, and autonomic instability about 2 to 4 days after stopping alcohol indicate delirium tremens, which requires emergency medical care.
- **A. Hallucinogen persisting perception disorder** — Owner: Hallucinogens (flashbacks after stopping). HPPD involves flashbacks of perceptual changes after hallucinogen use, not confusion and autonomic instability after stopping alcohol.
- **B. Stimulant withdrawal** — Owner: Stimulants (crash: fatigue, hypersomnia, hunger). The stimulant crash brings fatigue, increased sleep and appetite, and low mood, not delirium.
- **C. Cannabis withdrawal** — Owner: Cannabis (irritability, restlessness). Cannabis withdrawal is uncomfortable but not medically dangerous and doesn't cause delirium.
- **Pair to learn:** Confusion, hallucinations, autonomic instability 2 to 4 days after drinking stops → delirium tremens

5 — A

Counseling Skills & Interventions · Outline 5.S · Chapter 4

- **A. "After past letdowns, you're not sure this will be any different." — Owner: Rolling with resistance / reflection (Miller & Rollnick).** It acknowledges the doubt without arguing, treating resistance as information.
- **B. "Our approach has strong research support, so give it a chance."** — Owner: Defending the counselor / reassurance. It argues with the resistance instead of exploring it.
- **C. "You say it won't help, yet you came back today."** — Owner: Premature confrontation. Confronting a discrepancy this early, before the alliance is built, risks pushback.
- **D. "Why do you think the other counselors failed?"** — Owner: "Why" question. It invites justification and defensiveness.
- **Pair to learn:** Acknowledge doubt, don't argue → rolling with resistance (MI)

6 — C

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **C. Berne's transactional analysis — Owner: Berne (TA).** Berne analyzes communication between ego states and the life scripts behind it.
- **A. Satir's communication stances** — Owner: Satir (experiential). Satir's stances are placater, blamer, super-reasonable, and irrelevant.
- **B. Gottman's Four Horsemen** — Owner: Gottman. The Four Horsemen are criticism, contempt, defensiveness, and stonewalling.
- **D. Freud's structural model** — Owner: Freud (id, ego, superego). Freud's structure is id, ego, and superego, not Parent, Adult, and Child.

- **Pair to learn:** Ego states (Parent, Adult, Child) → Berne, not Freud

7 — A

Core Counseling Attributes · Outline 6.A · Chapter 22

- **A. Norcross and Lambert — Owner: Norcross & Lambert (evidence-based therapy relationships).** The Norcross and Lambert task forces judged the alliance, empathy, positive regard and affirmation, and collecting client feedback demonstrably effective.
- **B. Safran and Muran** — Owner: Safran & Muran (alliance ruptures and repair). Safran and Muran described withdrawal and confrontation ruptures and their repair.
- **C. Truax and Carkhuff** — Owner: Truax & Carkhuff (operationalized Rogers' conditions). Truax and Carkhuff defined accurate empathy, nonpossessive warmth, and genuineness.
- **D. Hayes and Gelso** — Owner: Hayes & Gelso (managing countertransference). Hayes and Gelso identified the factors in managing countertransference, not the task force findings.
- **Pair to learn:** Demonstrably effective relationship elements, including client feedback → Norcross & Lambert

8 — A

Intake, Assessment & Diagnosis · Outline 2.G · Chapter 19

- **A. Diagnose both and plan integrated treatment — Owner: Co-occurring diagnoses; integrated treatment (2.G).** Counselors diagnose every disorder whose criteria are met unless the criteria say otherwise, and co-occurring substance use and mental health disorders are treated together.
- **B. Record the drinking as a Z code** — Owner: Z codes (not mental disorders). Z codes are for

conditions that are not mental disorders; full criteria for alcohol use disorder are met.

- **C. Diagnose only the principal disorder** — Owner: Principal diagnosis (listed first, not alone). The principal diagnosis (inpatient) or reason for visit (outpatient) is the condition chiefly responsible for the admission or services and is listed first; it doesn't replace other diagnoses whose criteria are met.
- **D. Treat the drinking first, then the anxiety** — Owner: Sequential (non-integrated) treatment. Treating one disorder and postponing the other runs against integrated treatment of co-occurring disorders.
- **Pair to learn:** Criteria met for two disorders → Diagnose both, integrated treatment (2.G)

9 — B

Professional Practice & Ethics · Outline 1.V · Chapter 17

- **B. Share only needed, nonidentifying details** — **Owner: ACA B.7 (consultation).** In consultation, counselors don't reveal identifying information without consent and share only what's needed.
- **A. Share the full chart so peers can help** — Owner: Excess disclosure (minimal disclosure, B.2.e). The full chart reveals far more than the consultation needs.
- **C. Hand clinical responsibility to the group** — Owner: Supervision (evaluative, shared responsibility), not consultation. In consultation, the consulting counselor keeps full responsibility for the client.
- **D. Skip consulting to protect confidentiality** — Owner: Avoiding consultation (contrary to risk management). Consultation can be done without identifying information; skipping it gives up a key safeguard.
- **Pair to learn:** Peer consultation → Share only what's needed, no identifiers (ACA B.7)

10 — C

Core Counseling Attributes · Outline 6.M · Chapter 4

- **C. Verbal tracking — Owner: Ivey: attending behavior.** Verbal tracking, staying on the client's topic, is one of Ivey's attending behaviors, along with eye contact, body language, vocal tone, and comfortable silence.
- **A. Minimal encouragers** — Owner: Ivey: basic listening sequence. Minimal encouragers such as "Mm-hmm, go on" signal attention and keep the client talking.
- **B. Summarizing** — Owner: Ivey: basic listening sequence. Summarizing ties together several statements or themes; it is a listening skill, not staying on topic.
- **D. Immediacy** — Owner: Ivey: influencing skills. Immediacy is an influencing skill that addresses the counselor–client relationship right now.
- **Pair to learn:** Staying on the client's topic → Verbal tracking (Ivey attending)

11 — C

Areas of Clinical Focus · Outline 3.AI · Chapter 11

- **C. In sustained remission — Owner: DSM-5-TR (12+ months, craving allowed).** Twelve months or more with no criteria met, except craving, is sustained remission.
- **A. In early remission** — Owner: DSM-5-TR (3 to under 12 months). Early remission covers at least 3 but less than 12 months; she has passed 12 months.
- **B. In a controlled environment** — Owner: DSM-5-TR (restricted access, e.g., locked facility). She lives at home with normal access to alcohol.
- **D. On maintenance therapy** — Owner: DSM-5-TR (agonist or antagonist medication for opioids or tobacco). This specifier applies to opioid and tobacco

use with maintenance medication, which doesn't apply here.

- **Pair to learn:** 12+ months without criteria except craving → sustained remission

12 — D

Professional Practice & Ethics · Outline 1.B · Chapter 18

- **D. Reliable change index — Owner: Reliable change index (Jacobson & Truax).** The reliable change index tests whether one person's change exceeds measurement error, a key step in judging clinical significance.
- **A. Coefficient of determination** — Owner: Coefficient of determination (r^2, shared variance). r^2 gives the proportion of variance two variables share, not whether an individual changed reliably.
- **B. Goal Attainment Scaling** — Owner: Goal Attainment Scaling (Kiresuk & Sherman). Goal Attainment Scaling rates progress on individualized goals; it isn't a test against measurement error.
- **C. Cohen's d** — Owner: Cohen's d (effect size). Cohen's d describes how large an effect is, typically between groups; it doesn't test one client's change against measurement error.
- **Pair to learn:** Individual change beyond measurement error → Reliable change index (Jacobson & Truax)

13 — B

Professional Practice & Ethics · Outline 1.Q · Chapter 17

- **B. Allow access and help her understand them — Owner: ACA B.6.e (client access to records).** Counselors provide reasonable access to records unless there is compelling evidence of harm, and help clients understand what they read.
- **A. Show her the psychotherapy notes only** — Owner: HIPAA psychotherapy notes (kept separate, extra protection). Psychotherapy notes are a separately

protected category; her request is for reasonable access to her clinical record.

- **C. Release them only under a court order** — Owner: Court order (ACA B.2.d; governs legal disclosure). A court order governs disclosure to the court, not a client's own access to her records.
- **D. Refuse, since records belong to the counselor** — Owner: Counselor ownership of records (inconsistent with B.6.e and HIPAA access rights). Clients have a right to access their records, with limited exceptions; none applies here.
- **Pair to learn:** Client asks to see records → Reasonable access (ACA B.6.e)

14 — B

Core Counseling Attributes · Outline 6.C · Chapter 22

- **B. The client is in a state of incongruence — Owner: Rogers: condition 2.** Rogers' second condition states that the client is in a state of incongruence, vulnerable or anxious.
- **A. The client perceives empathy and positive regard** — Owner: Rogers: condition 6. Condition 6 concerns the client perceiving the counselor's empathy and regard, not the client's anxiety.
- **C. Two people are in psychological contact** — Owner: Rogers: condition 1. Condition 1 requires contact between two people; it says nothing about anxiety.
- **D. The counselor is congruent in the relationship** — Owner: Rogers: condition 3. Condition 3 describes the counselor's integration in the relationship, not the client's vulnerability.
- **Pair to learn:** Client vulnerable or anxious → Rogers' condition 2 (incongruence)

15 — A

Areas of Clinical Focus · Outline 3.U · Chapter 14

- **A. An aptitude test — Owner: Aptitude (potential), e.g., GATB.** Aptitude tests measure what a person could learn to do, which is her question.
- **B. An interest inventory** — Owner: Interest (what one likes), e.g., Strong. She already knows her interests, and interest isn't ability.
- **C. A work values inventory** — Owner: Values, e.g., MIQ (Dawis & Lofquist). Values inventories measure what matters to a person in work, not potential.
- **D. An achievement test** — Owner: Achievement (what one has already learned). Achievement tests measure skills already learned, not potential to learn new ones.
- **Pair to learn:** What one could learn to do → aptitude (not interest or achievement)

16 — A

Areas of Clinical Focus · Outline 3.T · Chapter 10

- **A. Obsessive-compulsive personality disorder — Owner: OCPD (ego-syntonic).** A pervasive pattern of perfectionism, orderliness, and control that the person sees as right, without true obsessions or compulsions, is OCPD.
- **B. Generalized anxiety disorder** — Owner: DSM-5-TR (6 months of excessive worry). GAD centers on hard-to-control worry about many things, not rigid standards she fully endorses.
- **C. Hoarding disorder** — Owner: DSM-5-TR obsessive-compulsive related disorder. Hoarding involves persistent difficulty discarding possessions, which isn't described.
- **D. Obsessive-compulsive disorder** — Owner: OCD (ego-dystonic obsessions and compulsions). OCD requires obsessions and/or compulsions, which usually feel unwanted and alien; she has neither.

- **Pair to learn:** Ego-syntonic perfectionism without obsessions → OCPD (not OCD)

17 — A

Professional Practice & Ethics · Outline 1.O · Chapter 17

- **A. Delay consent until he is sober — Owner: Assessing capacity to consent (1.O): intoxication.** Intoxication is one of the factors that can impair capacity, so consent should wait until he can understand what he's agreeing to.
- **B. Ask a relative to sign on his behalf** — Owner: Surrogate consent (A.2.d; for those legally unable to consent). Surrogate consent is for clients legally unable to consent; this adult's capacity is only temporarily affected.
- **C. Have him sign now and review it later** — Owner: Consent without capacity (not cured later; A.2.a). A signature given without capacity isn't meaningful informed consent.
- **D. Proceed, since he is an adult volunteer** — Owner: Assuming capacity (ignores 1.O factors). Being an adult who wants services doesn't establish capacity when he is visibly intoxicated.
- **Pair to learn:** Intoxicated at intake → Capacity to consent (1.O)

18 — C

Professional Practice & Ethics · Outline 1.B · Chapter 18

- **C. John Henry effect — Owner: John Henry effect (control group tries harder).** When control participants push themselves to outdo the treatment group, the John Henry effect can shrink the apparent difference between groups.
- **A. Rosenthal effect** — Owner: Experimenter expectancy (Rosenthal effect). The Rosenthal effect is the researcher's expectations shaping results, not participants' competitiveness.

- **B. Placebo effect** — Owner: Placebo effect (improvement from believing one is treated). The placebo effect comes from believing one is receiving treatment; control participants here know they aren't.
- **D. Hawthorne effect** — Owner: Hawthorne effect (Mayo; change because one is being observed). The Hawthorne effect is change caused by knowing one is being studied, not a control group competing.
- **Pair to learn:** Control group tries harder → John Henry effect

19 — A

Areas of Clinical Focus · Outline 3.S · Chapter 12

- **A. Emerging adulthood — Owner: Arnett.** Arnett's emerging adulthood (roughly 18 to 29) is marked by identity exploration, instability, self-focus, feeling in-between, and possibilities.
- **B. Intimacy vs. isolation** — Owner: Erikson (psychosocial stage 6). Erikson's young-adult crisis centers on forming close relationships, not on instability and feeling in-between.
- **C. Age-30 transition** — Owner: Levinson (Seasons of a Man's Life). Levinson's age-30 transition is a reappraisal within his life structure model, not the in-between period of the early twenties.
- **D. Personal fable** — Owner: Elkind (adolescent egocentrism). The personal fable is an adolescent's sense of being unique or invulnerable, not identity exploration in the twenties.
- **Pair to learn:** Feeling in-between, ages 18 to 29 → emerging adulthood (Arnett, not Levinson)

20 — D

Areas of Clinical Focus · Outline 3.AO · Chapter 13

- **D. Emily and John Visher — Owner: Visher & Visher (stepfamily myths).** The Vishers' early stepfamily work identified these myths.

- **A. Patricia Papernow** — Owner: Papernow (Stepfamily Cycle). Papernow described the seven-stage Stepfamily Cycle, not these myths.
- **B. McGoldrick and Carter** — Owner: McGoldrick & Carter (family life cycle). McGoldrick and Carter added remarriage stages to the family life cycle.
- **C. Silverstein and Kaplan** — Owner: Silverstein & Kaplan (seven core issues in adoption). Silverstein and Kaplan described core issues in adoption, not stepfamily myths.
- **Pair to learn:** Instant love myth → Visher & Visher (not Papernow)

21 — D

Areas of Clinical Focus · Outline 3.P · Chapter 16

- **D. Imagery rehearsal therapy — Owner: Nightmare treatment (including trauma-related nightmares).** Imagery rehearsal therapy changes the nightmare's script while awake and rehearses it.
- **A. Sensate focus** — Owner: Masters & Johnson. Sensate focus is structured, non-demand touch for sexual concerns.
- **B. Stimulus control** — Owner: Bootzin (CBT-I). Stimulus control strengthens the bed-sleep link for insomnia, not nightmare content.
- **C. Sleep restriction** — Owner: Spielman (CBT-I). Sleep restriction limits time in bed to build sleep drive for insomnia.
- **Pair to learn:** Rewriting and rehearsing nightmares → imagery rehearsal therapy

22 — D

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **D. Negative punishment — Owner: Skinner (operant).** Removing something pleasant to decrease a behavior is negative punishment.

- **A. Negative reinforcement** — Owner: Skinner (operant). Negative reinforcement removes something unpleasant to increase a behavior.
- **B. Extinction** — Owner: Skinner (operant). Extinction withholds the reinforcement that maintained a behavior so it fades.
- **C. Positive punishment** — Owner: Skinner (operant). Positive punishment adds something unpleasant to decrease a behavior.
- **Pair to learn:** Remove something pleasant to decrease behavior → negative punishment (Skinner)

23 — A

Professional Practice & Ethics · Outline 1.B · Chapter 18

- **A. Moderator — Owner: Moderator (changes the strength or direction of an effect: for whom or when).** Class year changes how strong the program's effect is depending on who receives it, which is what a moderator does.
- **B. Mediator** — Owner: Mediator (explains how or why the IV affects the DV). A mediator would explain the mechanism, such as the program lowering stress by reducing rumination; class year doesn't explain how the program works.
- **C. Confounding variable** — Owner: Confounding variable (uncontrolled alternative explanation). A confound is an uncontrolled rival explanation; class year is a measured factor that shapes for whom the effect holds.
- **D. Dependent variable** — Owner: Dependent variable (the measured outcome). Stress is the outcome being measured here, not class year.
- **Pair to learn:** Changes for whom an effect holds → Moderator

24 — C

Areas of Clinical Focus · Outline 3.E · Chapter 16

- **C. Zarit Burden Interview — Owner: Caregiver burden measure.** The Zarit Burden Interview assesses caregiver burden.
- **A. Social Readjustment Rating Scale** — Owner: Holmes & Rahe (life change units). The SRRS totals stress from life changes, not caregiver burden.
- **B. Maslach Burnout Inventory** — Owner: Maslach (workplace burnout). The MBI measures burnout from chronic workplace stress, not family caregiving burden.
- **D. Geriatric Depression Scale** — Owner: Depression screen for older adults. The GDS screens older adults for depression; the caregiver here is the client.
- **Pair to learn:** Caregiver burden → Zarit Burden Interview (not MBI)

25 — B

Counseling Skills & Interventions · Outline 5.A · Chapter 5

- **B. Life review — Owner: Butler (life review).** Life review is a structured review of one's whole life to resolve conflicts and find meaning, fitting Erikson's integrity vs. despair.
- **A. Behavioral activation** — Owner: Lewinsohn; N. Jacobson (behavioral activation). Behavioral activation increases rewarding activity for depression; it doesn't review the life story.
- **C. Filial therapy** — Owner: Guerney (filial therapy). Filial therapy trains parents to run play sessions with young children.
- **D. Reminiscence therapy** — Owner: Reminiscence therapy (general). Reminiscence recalls positive memories to support mood and connection; it isn't a structured whole-life review aimed at resolving conflict.

- **Pair to learn:** Structured review of one's whole life → life review (Butler), not reminiscence

26 — C

Counseling Skills & Interventions · Outline 5.L · Chapter 9

- **C. Use prayer only when it fits and the client consents — Owner: ASERVIC competencies (2009).** ASERVIC limits spiritual practices to those that fit the client's beliefs and goals, with consent.
- **A. Pray with all clients, since faith is protective** — Owner: Values imposition (ACA A.4.b). Applying her own practice to everyone imposes her values.
- **B. Refer religious clients to clergy instead** — Owner: Unneeded referral. Faith alone isn't a competence issue requiring referral.
- **D. Avoid all talk of spirituality to stay neutral** — Owner: Ignoring spiritual values. ASERVIC calls for exploring spirituality when it matters to the client.
- **Pair to learn:** Spiritual practices only when they fit the client and with consent → ASERVIC

27 — C

Areas of Clinical Focus · Outline 3.U · Chapter 14

- **C. Career Decision Scale — Owner: Osipow.** Osipow's Career Decision Scale measures career indecision.
- **A. Career Beliefs Inventory** — Owner: Krumboltz. The Career Beliefs Inventory identifies beliefs that block career progress.
- **B. Career Maturity Inventory** — Owner: Crites. The CMI measures career readiness attitudes and competencies, not indecision.
- **D. Minnesota Importance Questionnaire** — Owner: Dawis & Lofquist (TWA). The MIQ measures work values and needs.

- **Pair to learn:** Career indecision → Career Decision Scale (Osipow, not Crites)

28 — B

Areas of Clinical Focus · Outline 3.U · Chapter 14

- **B. Cognitive Information Processing — Owner: Peterson, Sampson, & Reardon (CASVE cycle).** The CASVE cycle is the decision-making core of Cognitive Information Processing.
- **A. Social Cognitive Career Theory** — Owner: Lent, Brown, & Hackett (SCCT). SCCT centers on self-efficacy, outcome expectations, and personal goals.
- **C. Williamson's trait-factor counseling** — Owner: Williamson (analysis, synthesis, diagnosis, prognosis). Williamson's six steps also begin with analysis and synthesis, but they are a directive counseling sequence, not CASVE.
- **D. Tiedeman and O'Hara's decision theory** — Owner: Tiedeman & O'Hara (anticipation and implementation). Tiedeman and O'Hara described anticipation and implementation phases, not CASVE.
- **Pair to learn:** CASVE cycle → Peterson, Sampson, & Reardon (not Tiedeman & O'Hara or Williamson)

29 — A

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **A. Adler's individual psychology — Owner: Adler (individual psychology).** All three are listed among Adler's signature techniques, alongside early recollections and encouragement.
- **B. Frankl's logotherapy** — Owner: Frankl (existential/logotherapy). Frankl's techniques are paradoxical intention, dereflection, and Socratic dialogue about meaning.
- **C. Ellis's rational emotive behavior therapy** — Owner: Ellis (REBT). Ellis's techniques are disputing

irrational beliefs, shame-attacking exercises, and rational-emotive imagery.

- **D. Perls's Gestalt therapy** — Owner: Perls (Gestalt). Gestalt's signature techniques are the empty chair, two-chair dialogue, and exaggeration.
- **Pair to learn:** Spitting in the soup; acting as if; push-button → Adler

30 — C

Areas of Clinical Focus · Outline 3.AQ · Chapter 12

- **C. Part C early intervention — Owner: IDEA Part C (birth to 3).** IDEA Part C provides early intervention services for children from birth to age 3.
- **A. FERPA records rights** — Owner: FERPA (privacy of education records). FERPA protects the privacy of education records and does not provide services.
- **B. A Section 504 plan** — Owner: Rehabilitation Act of 1973. A 504 plan gives accommodations to students whose disability limits a major life activity; it is not the birth-to-3 program.
- **D. An Individualized Education Program** — Owner: IDEA special education (ages 3 to 21). An IEP sets special education goals and services for eligible students ages 3 to 21.
- **Pair to learn:** Birth to 3 services → IDEA Part C (not IEP or 504)

31 — D

Treatment Planning · Outline 4.D · Chapter 21

- **D. What helped you get through hard times before? — Owner: Strengths-based practice (SFBT; Saleebey).** Asking about past coping and successes surfaces strengths the plan can build on.
- **A. Which symptom bothers you most right now?** — Owner: Problem-focused symptom assessment. This clarifies the problem statement but doesn't identify strengths.

- **B. What do you think is wrong with you?** — Owner: Deficit-focused framing (opposite of the strengths perspective). This invites a deficit view rather than what's already working.
- **C. How did your childhood shape this pattern?** — Owner: Insight-oriented exploration (psychodynamic). This seeks insight into origins, not resources to use in the plan.
- **Pair to learn:** Past coping and successes → Strengths (4.D)

32 — B

Areas of Clinical Focus · Outline 3.AI · Chapter 11

- **B. Claudia Black — Owner: Black (don't talk, don't trust, don't feel).** Claudia Black described the unspoken rules of alcoholic families: don't talk, don't trust, don't feel.
- **A. Gerald Patterson** — Owner: Patterson (coercion theory). Patterson's coercion theory explains escalating parent-child behavior, not alcoholic family rules.
- **C. E. M. Jellinek** — Owner: Jellinek (disease concept; phases of alcoholism). Jellinek described the phases of alcoholism in the drinker, not rules in the family.
- **D. Sharon Wegscheider-Cruse** — Owner: Wegscheider-Cruse (family roles in addiction). Wegscheider-Cruse described family roles such as hero, scapegoat, lost child, mascot, and enabler, not family rules.
- **Pair to learn:** Don't talk, don't trust, don't feel → Claudia Black (not Wegscheider-Cruse)

33 — D

Areas of Clinical Focus · Outline 3.G · Chapter 12

- **D. Palliative care — Owner: Comfort-focused care at any stage of serious illness.** Palliative care focuses on comfort and quality of life for anyone with serious illness, at any stage, alongside curative treatment.

- **A. A health care proxy** — Owner: Health care power of attorney. A proxy names someone to make medical decisions if she can't; it doesn't provide comfort-focused care.
- **B. Hospice care** — Owner: Cicely Saunders (modern hospice movement). Hospice serves people expected to live about 6 months or less who are no longer seeking curative treatment; she is still in active treatment.
- **C. A living will** — Owner: Advance directive. A living will states which treatments a person wants or refuses; it is a document, not a care team.
- **Pair to learn:** Comfort care alongside curative treatment → palliative care (not hospice)

34 — B

Areas of Clinical Focus · Outline 3.N · Chapter 16

- **B. Depersonalization — Owner: Dissociation (detached from oneself).** Feeling detached from oneself, as if watching from outside, is depersonalization.
- **A. Hyperfocus** — Owner: ADHD attention pattern. Hyperfocus is intense absorption in highly interesting tasks, not detachment from oneself.
- **C. Derealization** — Owner: Dissociation (world feels unreal). Derealization is the sense that the world feels unreal or dreamlike, not detachment from oneself.
- **D. Hypnopompic hallucination** — Owner: Hallucination on waking (common, nonpsychotic). Hypnopompic hallucinations occur while waking, not as daytime detachment after trauma.
- **Pair to learn:** Detached from oneself → depersonalization (not derealization)

35 — B

Treatment Planning · Outline 4.P · Chapter 21

- **B. Respect her choice and explore her reasons and the risks — Owner: Right to refuse treatment (ACA A.2.b); adherence (4.P).** Clients may refuse

treatment and be advised of the consequences; exploring her reasons without judgment supports collaboration.

- **A. Avoid the topic to protect the alliance** — Owner: Avoidance (misses barriers to adherence). Exploring barriers openly is part of supporting adherence.
- **C. Insist she take it as a condition of counseling** — Owner: Compliance as obedience (older framing of 4.P). Adherence emphasizes collaboration, not making services conditional on obedience.
- **D. Suggest a different medication she might tolerate better** — Owner: Prescribing advice (outside counselor scope, Ch 9). Counselors don't prescribe; medication choices belong with the prescriber.
- **Pair to learn:** Client declines medication → Respect refusal, explore reasons and consequences (4.P)

36 — C

Intake, Assessment & Diagnosis · Outline 2.L · Chapter 19

- **C. Discuss the fit and refer to a suitable setting — Owner: Screening for fit (2.L); least restrictive appropriate level of care.** The first session screens whether these are the right services at the right level of care. A client who needs a structured environment should be referred to a setting that provides it.
- **A. Begin weekly sessions and reassess in a month** — Owner: Under-matching the level of care. Weekly outpatient care doesn't meet her need for a structured setting, so it keeps her at a level below what she needs.
- **B. Arrange inpatient hospitalization today** — Owner: Inpatient hospitalization (imminent danger, medical instability). Inpatient care is for imminent danger, severe inability to care for oneself, or medical instability; none is described, so this is more restrictive than needed.
- **D. Offer a weekly support group as a substitute** — Owner: Group modality (requires screening; not a level

of care). Group is a modality chosen for social learning and support after screening; it doesn't supply the structured setting she needs.

- **Pair to learn:** Needs exceed what the service offers → Screen for fit and refer (2.L)

37 — B

Professional Practice & Ethics · Outline 1.E · Chapter 17

- **B. Respect her choice and explain any consequences — Owner: ACA A.2.b (right to refuse services or modality changes).** Clients have the right to refuse any service or change in modality and to be advised of the consequences of refusing.
- **A. Make group attendance a condition of counseling** — Owner: Coercion (violates autonomy and A.2.b). Conditioning services on a modality she refused overrides her right to decline.
- **C. Enroll her in group and revisit consent later** — Owner: Consent after the fact (informed consent precedes services, A.2.a). Informed consent comes before a service begins, not after.
- **D. End counseling since she rejected the plan** — Owner: Abandonment (ACA A.12). Ending services because a client declines one recommendation abandons her.
- **Pair to learn:** Client declines a modality change → Right to refuse (ACA A.2.b)

38 — B

Areas of Clinical Focus · Outline 3.Y · Chapter 11

- **B. ICD-11 — Owner: World Health Organization.** ICD-11 includes compulsive sexual behaviour disorder as an impulse control disorder; it is not a DSM-5-TR diagnosis.
- **A. DSM-5-TR Section III** — Owner: DSM-5-TR conditions for further study (e.g., internet gaming disorder). Section III holds conditions for further study

such as internet gaming disorder; compulsive sexual behavior isn't listed as a DSM-5-TR diagnosis.

- **C. The DSM-IV substance abuse category** — Owner: DSM-IV (abuse vs. dependence). DSM-IV's abuse category covered substances, and the DSM-5 merged it into substance use disorder.
- **D. The DSM-5-TR addictive disorders chapter** — Owner: DSM-5-TR Substance-Related and Addictive Disorders. Gambling disorder is the only behavioral addiction in that chapter.
- **Pair to learn:** Compulsive sexual behaviour disorder → ICD-11 (not DSM-5-TR)

39 — A

Areas of Clinical Focus · Outline 3.AW · Chapter 13

- **A. Coercion theory — Owner: Patterson.** Patterson described how a child's escalation is rewarded when the parent gives in, and the parent's giving in is rewarded when the child stops.
- **B. Systematic Training for Effective Parenting** — Owner: Dinkmeyer & McKay (STEP). STEP is an Adlerian parenting program, not the theory behind parent management training.
- **C. Goals of misbehavior** — Owner: Dreikurs (Adlerian parenting). Dreikurs' goals (attention, power, revenge, inadequacy) explain the purpose of a child's misbehavior, not mutual reinforcement.
- **D. Parenting styles** — Owner: Baumrind. Baumrind classified parenting by warmth and firmness, not by reinforcement cycles.
- **Pair to learn:** Parent and child reinforce each other's escalation → coercion theory (Patterson)

40 — C

Counseling Skills & Interventions · Outline 5.AV · Chapter 8

- **C. Ask the questioner to share the statement behind her questions — Owner: Challenging**

questioning/interrogating. Asking for the statement behind the questions turns interrogation into owned, direct sharing.

- **A. Tell the targeted member what the leader thinks she should do** — Owner: Advice-giving (not a counseling skill). Advice shifts focus to the leader and models the same fix-it behavior.
- **B. Let it continue, since members are interacting** — Owner: Ignoring harmful member behavior. Interrogation pressures a member and isn't productive interaction.
- **D. Ask the other member to answer each question fully** — Owner: Pressuring disclosure. This adds pressure on the targeted member.
- **Pair to learn:** Questioning/interrogating → ask for the statement behind the question

41 — A

Intake, Assessment & Diagnosis · Outline 2.J · Chapter 19

- **A. A SUDS rating — Owner: Subjective units of distress (SUDS).** A SUDS rating is the client's own 0 to 100 rating of current distress, a quick way to assess the level of distress (2.J).
- **B. A Likert attitude scale** — Owner: Rensis Likert. A Likert scale rates agreement with a set of statements; it does not ask for a single 0 to 100 rating of current distress.
- **C. The WHODAS 2.0** — Owner: WHO Disability Assessment Schedule 2.0 (functioning). The WHODAS 2.0 measures functioning across daily life; it is the DSM-5-TR's suggested functioning measure, not a single in-the-moment distress rating.
- **D. The OQ-45** — Owner: Lambert (Outcome Questionnaire). The OQ-45 is a multi-item questionnaire for tracking progress over the course of counseling, not a one-question rating of distress right now.

- **Pair to learn:** 0 to 100 rating of current distress → SUDS (not WHODAS 2.0)

42 — C

Treatment Planning · Outline 4.C · Chapter 21

- **C. Barriers — Owner: Treatment plan component: barriers (4.C).** Work schedules and lack of childcare are barriers, named early so the plan can address them.
- **A. Strengths and resources** — Owner: Treatment plan component: strengths (4.D). Strengths are what help, such as supports and past successes; these factors get in the way.
- **B. Problem statement** — Owner: Treatment plan component: problem statement. The problem statement describes the clinical concern, not obstacles to treatment.
- **D. Responsible parties** — Owner: Treatment plan component: responsible parties. Responsible parties name who does what, not what may interfere.
- **Pair to learn:** Work schedule, no childcare → Barriers (4.C)

43 — B

Areas of Clinical Focus · Outline 3.AB · Chapter 12

- **B. Reorientation — Owner: Atchley (retirement phases).** Reorientation is developing a more realistic, satisfying routine after the letdown of disenchantment.
- **A. Recovery** — Owner: Oberg (culture shock). Recovery is Oberg's culture shock phase of learning a new culture's rules, not a phase of retirement.
- **C. Disenchantment** — Owner: Atchley (retirement phases). Disenchantment is the letdown he has already moved through; he is now building a new routine.
- **D. Reinvest** — Owner: Rando (six R processes of grief). Reinvesting is Rando's final grief process of putting energy into new life after a death, not a retirement phase.

- **Pair to learn:** Building a realistic, satisfying routine → reorientation (Atchley)

44 — D

Counseling Skills & Interventions · Outline 5.Q · Chapter 6

- **D. Anomic — Owner: Durkheim (anomic suicide).** Anomic suicide follows sudden disruption of social norms or regulation, such as economic collapse.
- **A. Egoistic** — Owner: Durkheim (egoistic suicide). Egoistic suicide reflects too little social integration.
- **B. Altruistic** — Owner: Durkheim (altruistic suicide). Altruistic suicide reflects too much integration and self-sacrifice for the group.
- **C. Fatalistic** — Owner: Durkheim (fatalistic suicide). Fatalistic suicide reflects excessive regulation and oppression.
- **Pair to learn:** Sudden disruption of norms (economic collapse) → anomic (Durkheim)

45 — C

Areas of Clinical Focus · Outline 3.AR · Chapter 13

- **C. Hazan and Shaver — Owner: Hazan & Shaver (adult attachment).** Hazan and Shaver applied attachment theory to adult romantic relationships.
- **A. Caryl Rusbult** — Owner: Rusbult (investment model). Rusbult explained commitment through satisfaction, alternatives, and investments.
- **B. Robert Sternberg** — Owner: Sternberg (triangular theory of love). Sternberg described love through intimacy, passion, and commitment.
- **D. Mary Ainsworth** — Owner: Ainsworth (Strange Situation, infant attachment). Ainsworth classified infant attachment in the Strange Situation, not adult romantic attachment.
- **Pair to learn:** Adult romantic attachment styles → Hazan & Shaver (not Ainsworth)

46 — D

Core Counseling Attributes · Outline 6.K · Chapter 22

- **D. "You needed out. How are you feeling about it now?" — Owner: Unconditional positive regard (values the person, not every act).** UPR values the client as a person without conditions; it stays with his experience without having to approve or condemn the choice.
- **A. "Quitting without notice wasn't very professional."** — Owner: Moralizing (judgment). Moral evaluation of the choice signals conditional acceptance.
- **B. "Yes, you did exactly the right thing by leaving."** — Owner: Approval (a misreading of UPR). UPR doesn't mean approving everything a client does, and saying so here would also be incongruent.
- **C. "Let's set that aside and talk about your budget."** — Owner: Avoidance (changing the subject). Changing the subject dodges the client's question and his feelings.
- **Pair to learn:** Valuing the person without approving the act → Unconditional positive regard (not approval)

47 — D

Core Counseling Attributes · Outline 6.G · Chapter 22

- **D. Empathic attunement — Owner: Empathic attunement (internal sensing, 6.G).** Empathic attunement is the internal process of sensing and resonating with the client's experience, including what's unspoken.
- **A. Countertransference** — Owner: Countertransference (counselor's reactions from own history). Countertransference is the counselor's own emotional reaction, often from the counselor's history, not sensing the client's experience.
- **B. Empathic responding** — Owner: Empathic responding (communicating understanding, 6.H).

Empathic responding communicates understanding back to the client; the counselor hasn't communicated anything yet.

- **C. Immediacy** — Owner: Immediacy (Ch 4 influencing skill). Immediacy is a spoken skill addressing what's happening between counselor and client right now.
- **Pair to learn:** Sensing the client's experience internally → Empathic attunement (not responding)

48 — B

Intake, Assessment & Diagnosis · Outline 2.M · Chapter 20

- **B. The condition's low base rate — Owner: Base rate.** Low base rates produce more false positives even with good tests, because true cases are so few compared with the many people who don't have the condition.
- **A. A floor effect on the screen** — Owner: Floor effect (test too hard). A floor effect means the test is too hard and many people score at the bottom; it doesn't explain excess false positives.
- **C. Poor interrater reliability** — Owner: Interrater reliability (scorer agreement). Interrater reliability concerns whether scorers agree; nothing suggests scoring disagreements.
- **D. Weak incremental validity** — Owner: Incremental validity. Incremental validity asks whether a test adds information beyond what's known; it doesn't explain why positives are mostly wrong.
- **Pair to learn:** Rare condition → many false positives → Low base rate (not floor effect)

49 — C

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **C. Adler's individual psychology — Owner: Adler ("the Question").** Adler's "the Question" asks what would be different if the client were well.

- **A. Solution-focused brief therapy** — Owner: de Shazer & Berg (SFBT). The SFBT miracle question asks the client to imagine waking up with the problem solved overnight; it is a different future-oriented question from a different approach.
- **B. Narrative therapy** — Owner: White & Epston (narrative). Narrative questions externalize the problem and search for unique outcomes.
- **D. Reality therapy** — Owner: Glasser/Wubbolding (WDEP). Reality therapy asks what the client wants and whether current behavior is getting it.
- **Pair to learn:** "What would be different if you were well?" → Adler (the Question), not the miracle question

50 — B

Treatment Planning · Outline 4.H · Chapter 21

- **B. Explore her feelings about ending and earlier losses — Owner: Termination task: process feelings about ending.** Endings can stir up past losses; processing these feelings is a core task of the final phase.
- **A. Extend counseling indefinitely until she feels ready** — Owner: Fostering dependence (avoids termination work). Postponing the ending avoids the work that helps her end well.
- **C. Shift the session to building her relapse prevention plan** — Owner: Relapse prevention (a later termination task). Relapse prevention matters, but skipping her feelings misses what's happening now.
- **D. Remind her the end date was set together at the start** — Owner: Defending the plan (misses the feeling). Restating the plan answers the logistics but not the hurt she just expressed.
- **Pair to learn:** Ending stirs past losses → Process feelings about termination (4.H)

51 — D

Core Counseling Attributes · Outline 6.H · Chapter 4

- **D. "You miss him and maybe wonder where you fit in his life." — Owner: Carkhuff level 4 (additive empathy).** Level 4 adds a feeling or meaning just below the surface, here the unspoken worry about her place in his life.
- **A. "Have you thought about texting him instead of waiting?"** — Owner: Carkhuff level 1 (advice; ignores feeling). Jumping to advice ignores the feeling entirely.
- **B. "You really miss him since he moved so far away."** — Owner: Carkhuff level 3 (interchangeable). This captures the expressed content and feeling without adding to them, which is interchangeable, level 3.
- **C. "Young adults often get busy and forget to call home."** — Owner: Carkhuff level 2 (subtractive generalization). A generalization loses the client's specific feeling, which subtracts from the message.
- **Pair to learn:** Adds meaning just below the surface → Carkhuff level 4 (not level 3)

52 — C

Counseling Skills & Interventions · Outline 5.AS · Chapter 8

- **C. Provide structure and help members set goals — Owner: Corey: initial stage (Tuckman: forming).** The initial stage calls for structure, trust building, goal setting, and linking.
- **A. Address members' feelings about the group ending** — Owner: Corey: final stage. Feelings about ending belong to the final stage.
- **B. Push members to share their deepest secrets now** — Owner: Premature deep disclosure. Pushing deep disclosure in session one, before trust is built, is a common leader error.

- **D. Step back and let members run the session** — Owner: Corey: working stage. Stepping back fits the working stage, once cohesion exists.
- **Pair to learn:** Polite, tentative, leader-focused → initial stage: provide structure

53 — C

Professional Practice & Ethics · Outline 1.B · Chapter 18

- **C. Transferability — Owner: Lincoln & Guba: transferability (parallels external validity).** Transferability asks whether findings can apply to other people and settings, the qualitative parallel to external validity.
- **A. Credibility** — Owner: Lincoln & Guba: credibility (parallels internal validity). Credibility parallels internal validity, not external validity.
- **B. Dependability** — Owner: Lincoln & Guba: dependability (parallels reliability). Dependability parallels reliability, the consistency of findings.
- **D. Confirmability** — Owner: Lincoln & Guba: confirmability (parallels objectivity). Confirmability parallels objectivity, not generalizability.
- **Pair to learn:** Qualitative parallel to external validity → Transferability (Lincoln & Guba)

54 — A

Areas of Clinical Focus · Outline 3.AA · Chapter 15

- **A. Positive religious coping — Owner: Pargament (religious coping).** Seeking spiritual support and benevolent religious reappraisal are forms of positive religious coping.
- **B. Negative religious coping** — Owner: Pargament (religious coping). Negative coping involves feeling punished or abandoned by God, which she doesn't describe.
- **C. Terror management** — Owner: Greenberg, Solomon, & Pyszczynski. Terror management theory

explains holding onto worldviews to buffer death awareness; it isn't Pargament's coping framework.

- **D. Spiritual bypassing** — Owner: Welwood. She still faces her fear openly, so she isn't using spirituality to avoid emotional pain.
- **Pair to learn:** Spiritual support and benevolent reappraisal → positive religious coping (Pargament)

55 — D

Core Counseling Attributes · Outline 6.H · Chapter 22

- **D. "You got in, and still you wonder if you really belong." — Owner: Empathic responding (client's frame of reference).** Empathy reflects the client's experience from the client's frame of reference and keeps the focus on the client.
- **A. "I was first-gen too, so I know exactly how you feel."** — Owner: Identification ("I know exactly how you feel"). Identification shifts focus to the counselor and assumes the experiences are the same.
- **B. "That must be so hard. I feel really bad for you."** — Owner: Sympathy (not empathy). Feeling sorry for the client comes from the counselor's frame of reference.
- **C. "You earned your place there, so don't doubt yourself."** — Owner: Reassurance. Reassurance dismisses the feeling instead of understanding it.
- **Pair to learn:** "I know exactly how you feel" → Identification, not empathy

56 — C

Counseling Skills & Interventions · Outline 5.Q · Chapter 6

- **C. Walker's cycle of violence — Owner: Lenore Walker.** Walker described this three-phase cycle in intimate partner violence.
- **A. The Power and Control Wheel** — Owner: Domestic Abuse Intervention Programs (Duluth). The

Duluth wheel maps tactics of power and control, not a three-phase cycle.

- **B. Coercive control** — Owner: Evan Stark. Stark's concept describes ongoing domination such as monitoring and isolation, not a cycle.
- **D. The Danger Assessment** — Owner: Jacquelyn Campbell. Campbell's instrument assesses lethality risk in IPV.
- **Pair to learn:** Tension → battering → honeymoon → Walker (not Duluth)

57 — A

Counseling Skills & Interventions · Outline 5.AR · Chapter 8

- **A. Leave time for members to personalize the material — Owner: Psychoeducation in group (process still matters).** Even in psychoeducational groups, members need time to react, personalize, and link material to their lives.
- **B. Convert the group into a psychotherapy group** — Owner: ASGW: psychotherapy group. Psychotherapy groups treat more serious problems; the group's purpose is teaching skills.
- **C. Add more content so members get full value** — Owner: Content without process. More content deepens the problem of no interaction.
- **D. Have members read the slides at home instead** — Owner: Information-giving without process. Moving content home still leaves no group interaction.
- **Pair to learn:** Psychoeducational group → teach, then let members process and personalize

58 — D

Areas of Clinical Focus · Outline 3.AF · Chapter 16

- **D. Sleep restriction — Owner: Spielman.** Limiting time in bed to actual sleep time to build sleep drive, then extending it, is Spielman's sleep restriction.

- **A. Stimulus control** — Owner: Bootzin. Stimulus control uses the bed only for sleep and getting up if awake; it doesn't set a time-in-bed limit.
- **B. Sleep hygiene** — Owner: General sleep habits (usually not enough alone). Sleep hygiene covers schedule, caffeine, screens, and environment, not restricting time in bed.
- **C. Progressive muscle relaxation** — Owner: Jacobson. PMR reduces physical arousal; it doesn't change time in bed.
- **Pair to learn:** Limiting time in bed to build sleep drive → sleep restriction (Spielman, not Bootzin)

59 — A

Areas of Clinical Focus · Outline 3.AI · Chapter 11

- **A. Community Reinforcement and Family Training — Owner: Robert Meyers (CRAFT).** CRAFT teaches concerned family members to reinforce sobriety and engage a treatment-refusing loved one.
- **B. Brief Strategic Family Therapy** — Owner: Szapocznik (adolescent substance use). BSFT is a family therapy for adolescent substance use, and her son is an adult who won't attend.
- **C. Multisystemic therapy** — Owner: Henggeler (serious antisocial behavior in youth). MST is intensive home- and community-based treatment for youth with serious antisocial behavior.
- **D. Multidimensional Family Therapy** — Owner: Liddle (adolescent substance use). MDFT treats adolescent substance use with the teen involved, not a family member working alone with an adult.
- **Pair to learn:** Family member engaging a treatment-refusing loved one → CRAFT (Meyers)

60 — A

Counseling Skills & Interventions · Outline 5.W · Chapter 7

- **A. Renegotiating the couple as a dyad — Owner: Carter & McGoldrick (launching stage).** Launching children requires the couple to renegotiate their relationship as a dyad.
- **B. Forming a new couple system** — Owner: Carter & McGoldrick (joining through marriage). That is the task of joining families through marriage or union.
- **C. Loosening boundaries for a teen's independence** — Owner: Carter & McGoldrick (adolescent stage). That is the task of families with adolescents.
- **D. Accepting shifting generational roles** — Owner: Carter & McGoldrick (later life). That is the task of families in later life.
- **Pair to learn:** Empty nest → launching stage: renegotiate the couple dyad

61 — D

Core Counseling Attributes · Outline 6.B · Chapter 22

- **D. Briefly own the mistake and correct it — Owner: Genuineness; Corey: effective counselors admit mistakes.** Genuineness means being real rather than hiding behind a role, and effective counselors make mistakes and are willing to admit them.
- **A. Continue as though nothing happened** — Owner: Professional façade. Pretending not to notice hides behind the role, the opposite of genuineness.
- **B. Apologize at length until the client reassures her** — Owner: Excessive apology (shifts focus to the counselor). A long apology makes the client care for the counselor's feelings.
- **C. Explain that a heavy caseload makes names hard** — Owner: Defensiveness (justifying). Justifying

the error defends the counselor instead of attending to the client's hurt.

- **Pair to learn:** Admitting mistakes, no façade → Genuineness (Corey's effective counselor)

62 — D

Areas of Clinical Focus · Outline 3.AW · Chapter 13

- **D. Display of inadequacy — Owner: Dreikurs (goals of misbehavior).** A child who believes he can't do anything right and wants to be left alone, with an adult who feels hopeless, fits display of inadequacy.
- **A. Learned helplessness** — Owner: Seligman. Learned helplessness is Seligman's theory of depression after uncontrollable events, not one of Dreikurs' goals.
- **B. Industry vs. inferiority** — Owner: Erikson (psychosocial stage 4). Erikson's school-age crisis is a developmental stage, not a goal of misbehavior in Dreikurs' framework.
- **C. Attention** — Owner: Dreikurs (goals of misbehavior). Attention-seeking typically leaves the adult annoyed, and this child wants to be left alone.
- **Pair to learn:** Adult feels hopeless → display of inadequacy (Dreikurs)

63 — A

Areas of Clinical Focus · Outline 3.AH · Chapter 14

- **A. Charles Figley — Owner: Figley (compassion fatigue).** Figley described compassion fatigue as exhaustion from caring for those who suffer, including secondary traumatic stress.
- **B. Hans Selye** — Owner: Selye (General Adaptation Syndrome). Selye described alarm, resistance, and exhaustion as a general stress response.
- **C. Christina Maslach** — Owner: Maslach (burnout). Maslach described burnout: exhaustion, depersonalization, and reduced accomplishment from chronic workplace stress.

- **D. McCann and Pearlman** — Owner: McCann & Pearlman (vicarious trauma). Vicarious trauma refers to lasting changes in the helper's beliefs and worldview.
- **Pair to learn:** Compassion fatigue → Figley (not Maslach)

64 — D

Areas of Clinical Focus · Outline 3.AK · Chapter 12

- **D. Bargaining — Owner: Kübler-Ross (stages of dying).** Trying to trade a promise for more time is bargaining, one of the common reactions Kübler-Ross described in terminally ill patients, though not a required sequence.
- **A. Yearning and searching** — Owner: Bowlby (phases of mourning). Yearning and searching is Bowlby's mourning phase after a loss, not a dying person's attempt to negotiate for time.
- **B. Accepting the reality of the loss** — Owner: Worden (first task of mourning). This is Worden's first task for mourners after a death, not a reaction of the person who is dying.
- **C. Relinquishing old attachments** — Owner: Rando (six R processes). Relinquishing is one of Rando's grief processes for the bereaved, not a bargain for more time.
- **Pair to learn:** Trading promises for more time → bargaining (Kübler-Ross)

65 — D

Counseling Skills & Interventions · Outline 5.AP · Chapter 8

- **D. Imitative behavior — Owner: Yalom (therapeutic factors).** Learning by watching the leader or other members is imitative behavior.
- **A. Interpersonal learning** — Owner: Yalom (therapeutic factors). Interpersonal learning is learning how one affects others and how others affect oneself.

- **B. Universality** — Owner: Yalom (therapeutic factors). Universality is the relief of "I'm not the only one."
- **C. Catharsis** — Owner: Yalom (therapeutic factors). Catharsis is emotional release.
- **Pair to learn:** Learning by watching others → imitative behavior (Yalom)

66 — C

Professional Practice & Ethics · Outline 1.I · Chapter 17

- **C. Professional will — Owner: Professional will (ACA C.2.h, B.6.i: plan for incapacity or death).** A professional will names a records custodian and arranges transfer of records and care if the counselor can't continue.
- **A. Release of information** — Owner: Release of information (client authorization to share). A release authorizes sharing specific information; it doesn't name a records custodian.
- **B. Business associate agreement** — Owner: HIPAA business associate agreement (vendor contract). A BAA is a contract with a vendor who handles protected health information, not a continuity plan.
- **D. Discharge summary** — Owner: Discharge summary (end-of-services document, 4.J). A discharge summary documents one client's treatment at discharge; it doesn't plan for the counselor's death.
- **Pair to learn:** Plan for counselor's death or incapacity → Professional will / records custodian

67 — C

Treatment Planning · Outline 4.N · Chapter 21

- **C. DAP — Owner: DAP (Data, Assessment, Plan).** In DAP, the Data section combines subjective and objective information.
- **A. GIRP** — Owner: GIRP (Goal, Intervention, Response, Plan). GIRP opens with the treatment goal, not combined client and counselor data.

- **B. BIRP** — Owner: BIRP (Behavior, Intervention, Response, Plan). BIRP begins with the client's behavior and presentation, then separates intervention and response.
- **D. SOAP** — Owner: SOAP (Subjective, Objective, Assessment, Plan). SOAP separates the client's report (S) from the counselor's observations (O).
- **Pair to learn:** Subjective and objective together in Data → DAP

68 — B

Areas of Clinical Focus · Outline 3.L · Chapter 10

- **B. Bipolar II disorder — Owner: DSM-5-TR (hypomania of 4+ days plus major depression).** At least 4 consecutive days of hypomanic symptoms without marked impairment, hospitalization, or psychosis, plus a major depressive episode, fits bipolar II.
- **A. Persistent depressive disorder** — Owner: DSM-5-TR (2 years in adults). PDD ignores the hypomanic history, which changes the diagnosis.
- **C. Cyclothymic disorder** — Owner: DSM-5-TR (2 years of subthreshold symptoms in adults). Cyclothymia involves symptoms that never meet full episode criteria, but she has had full major depressive episodes.
- **D. Bipolar I disorder** — Owner: DSM-5-TR (manic episode: 1 week or hospitalization). Bipolar I requires a manic episode lasting 1 week or involving marked impairment, hospitalization, or psychosis; none occurred.
- **Pair to learn:** 4+ days hypomania + major depression → bipolar II

69 — B

Professional Practice & Ethics · Outline 1.R · Chapter 9

- **B. Provide a qualified ASL interpreter — Owner: ADA reasonable accommodation (1.R).** A qualified

ASL interpreter is a reasonable accommodation that gives the client full access in her own language.

- **A. Communicate through written notes only** — Owner: Written English (ASL is a full, separate language). ASL is a full language, not signed or written English; notes don't give equal access.
- **C. Refer her elsewhere to avoid the cost** — Owner: Denial of accommodation (ADA; ACA C.5 nondiscrimination). Turning her away because of her disability-related needs is discrimination, not accommodation.
- **D. Have her hearing daughter interpret** — Owner: Family member as interpreter (compromises accuracy, confidentiality, roles). Family interpreters compromise accuracy, confidentiality, and family roles.
- **Pair to learn:** Deaf client → Qualified ASL interpreter (ADA accommodation)

70 — C

Counseling Skills & Interventions · Outline 5.V · Chapter 6

- **C. Build a written safety plan with her — Owner: Stanley & Brown (Safety Planning Intervention).** A collaborative safety plan, plus means restriction, is the supported approach; contracts lack evidence.
- **A. Rely on the SAD PERSONS score alone** — Owner: Patterson et al. (SAD PERSONS). SAD PERSONS is a teaching tool, not validated to predict risk on its own.
- **B. Arrange involuntary hospitalization today** — Owner: Overly restrictive care. Involuntary holds are for imminent risk; moderate risk calls for the least restrictive appropriate care.
- **D. Have her sign the contract and file a copy** — Owner: No-suicide contract (no supporting evidence). Contracts haven't been shown to prevent suicide and can give false reassurance.

- **Pair to learn:** Safety plan (Stanley & Brown) over no-suicide contract

71 — A

Counseling Skills & Interventions · Outline 5.X · Chapter 9

- **A. Reassess fit and consider in-person sessions — Owner: ACA H.4.c–d (appropriateness and effectiveness of distance counseling).** Counselors decide whether distance counseling is appropriate; severe dissociation and no privacy may call for face-to-face services.
- **B. Move sessions to unencrypted text for privacy** — Owner: ACA H.2 (security). Unencrypted text weakens security and doesn't fix the clinical concern.
- **C. Verify her identity with a code word each session** — Owner: ACA H.3 (client verification). Verification matters but doesn't address safety or privacy.
- **D. Continue unchanged, since she consented to telehealth** — Owner: Consent is not the same as fit. Consent doesn't settle whether distance counseling remains appropriate.
- **Pair to learn:** Telehealth not working for this client → H.4.d: consider face-to-face

72 — D

Treatment Planning · Outline 4.H · Chapter 21

- **D. Premature termination — Owner: Premature termination (dropout; Swift & Greenberg).** Stopping before goals are met is premature termination, estimated to occur in about one in five clients.
- **A. Counselor-initiated termination** — Owner: Counselor-initiated termination. Here the client, not the counselor, ends services.
- **B. Forced termination** — Owner: Forced termination (external limits). Forced termination comes from

outside limits such as insurance, agency policy, or a training placement ending.

- **C. Planned termination** — Owner: Planned (natural) termination. Planned termination happens when goals are met and the ending is discussed in advance.
- **Pair to learn:** Stops before goals are met → Premature termination (dropout)

73 — D

Intake, Assessment & Diagnosis · Outline 2.M · Chapter 20

- **D. 65 — Owner: T-score (mean 50, SD 10).** z = (52 − 40) ÷ 8 = 1.5. T = 50 + 10(1.5) = 65.
- **A. 62** — Owner: Error: adding the raw difference to 50. Adding the 12-point raw difference straight to 50 skips the conversion to z; the raw scale's SD is 8, not 10.
- **B. 70** — Owner: T-score for z = +2. A T of 70 corresponds to z = +2, which would require a raw score of 56.
- **C. 60** — Owner: T-score for z = +1. A T of 60 corresponds to z = +1, but this client is 1.5 SDs above the mean.
- **Pair to learn:** z = (score − mean) ÷ SD; T = 50 + 10z → raw 52 (mean 40, SD 8) = T 65

74 — A

Counseling Skills & Interventions · Outline 5.AG · Chapter 7

- **A. Physiological self-soothing — Owner: Gottman (antidote to stonewalling).** Self-soothing is the antidote to stonewalling when a partner is flooded.
- **B. Gentle startup** — Owner: Gottman (antidote to criticism). Gentle startup replaces criticism, not shutting down.

- **C. Building a culture of appreciation** — Owner: Gottman (antidote to contempt). Appreciation is the antidote to contempt.
- **D. Taking responsibility** — Owner: Gottman (antidote to defensiveness). Taking responsibility is the antidote to defensiveness.
- **Pair to learn:** Stonewalling → physiological self-soothing (Gottman)

75 — D

Intake, Assessment & Diagnosis · Outline 2.M · Chapter 20

- **D. Debra P. v. Turlington — Owner: Debra P. v. Turlington (1981): curricular/instructional validity.** Debra P. held that a graduation test must cover what students were actually taught, known as curricular or instructional validity.
- **A. Griggs v. Duke Power Co.** — Owner: Griggs v. Duke Power Co. (1971). Griggs held that employment tests must be job-related.
- **B. Hobson v. Hansen** — Owner: Hobson v. Hansen (1967). Hobson found ability grouping, or tracking, based on biased group tests unconstitutional.
- **C. Larry P. v. Riles** — Owner: Larry P. v. Riles (1979). Larry P. barred IQ tests for placing Black children in California classes for students labeled educable mentally retarded, because of cultural bias.
- **Pair to learn:** Test must cover what was taught → Debra P. v. Turlington (not Griggs)

76 — D

Areas of Clinical Focus · Outline 3.U · Chapter 14

- **D. Congruence — Owner: Holland (person-environment fit).** Congruence is the fit between a person's type and the work environment's type; a Social person in a solitary data job has low congruence.

- **A. Satisfactoriness** — Owner: Dawis & Lofquist (Theory of Work Adjustment). Satisfactoriness is the employer's view of the worker, and her employer is pleased.
- **B. Differentiation** — Owner: Holland (one type clearly stands out). Her profile is clearly Social, so differentiation isn't the problem.
- **C. Consistency** — Owner: Holland (closeness of top types on the hexagon). Consistency concerns how close a person's top codes sit on the hexagon, not fit with the job.
- **Pair to learn:** Person-environment mismatch → low congruence (Holland)

77 — C

Areas of Clinical Focus · Outline 3.U · Chapter 14

- **C. Strong Interest Inventory — Owner: E. K. Strong.** The Strong compares a client's interests with those of people satisfied in occupations, and also reports Holland's themes.
- **A. Career Thoughts Inventory** — Owner: Sampson et al. (CIP). The Career Thoughts Inventory measures negative career thoughts.
- **B. Self-Directed Search** — Owner: Holland (self-administered, self-scored). The SDS yields a RIASEC code that the client scores; it doesn't compare to satisfied workers.
- **D. Myers-Briggs Type Indicator** — Owner: Myers & Briggs (based on Jung). The MBTI identifies personality type for self-understanding, not interests compared with workers.
- **Pair to learn:** Compares interests with satisfied workers → Strong (not SDS)

78 — B

Intake, Assessment & Diagnosis · Outline 2.M · Chapter 20

- **B. Achievement — Owner: Achievement tests (WIAT, WRAT, Woodcock-Johnson Achievement).** Achievement tests measure what has been learned.
- **A. Neuropsychological** — Owner: Neuropsychological tests (Halstead-Reitan, Bender-Gestalt II). Neuropsychological tests assess brain–behavior functioning, not learned academic content.
- **C. Aptitude** — Owner: Aptitude tests (ASVAB, GATB). Aptitude tests measure potential to learn or perform, not what has already been learned.
- **D. Intelligence** — Owner: Intelligence tests (WAIS, WISC, Stanford-Binet). Intelligence tests measure general cognitive ability, not mastery of taught material.
- **Pair to learn:** What has been learned → Achievement (not aptitude)

79 — C

Areas of Clinical Focus · Outline 3.AT · Chapter 13

- **C. Sibling abuse — Owner: Family abuse (power imbalance, repeated victimization, fear).** A power imbalance, repeated victimization of the same child, and fear are the markers that separate sibling abuse from ordinary fighting.
- **A. Situational couple violence** — Owner: Michael P. Johnson (IPV typology). This is Johnson's category for partner violence, not behavior between siblings.
- **B. Power-seeking misbehavior** — Owner: Dreikurs (goals of misbehavior). Dreikurs' goals explain a child's misbehavior toward adults; they don't capture repeated victimization of a younger sibling.
- **D. Sibling rivalry** — Owner: Normal sibling conflict. Ordinary rivalry lacks the power imbalance, repeated targeting of one child, and fear described here.

- **Pair to learn:** Power imbalance + repeated victimization + fear → sibling abuse (not rivalry)

80 — C

Areas of Clinical Focus · Outline 3.F · Chapter 15

- **C. Khyâl cap — Owner: DSM-5-TR cultural concepts of distress (Cambodian).** Khyâl cap describes Cambodian wind attacks, which can resemble panic.
- **A. Susto** — Owner: DSM-5-TR cultural concepts of distress (fright). Susto is illness attributed to a frightening event that causes the soul to leave the body.
- **B. Kufungisisa** — Owner: DSM-5-TR cultural concepts of distress (Zimbabwe). Kufungisisa means thinking too much.
- **D. Taijin kyofusho** — Owner: DSM-5-TR cultural concepts of distress (Japan). Taijin kyofusho is the fear that one's body or behavior offends others.
- **Pair to learn:** Wind attacks → khyâl cap (Cambodian)

81 — C

Counseling Skills & Interventions · Outline 5.P · Chapter 7

- **C. Emotionally focused therapy — Owner: Sue Johnson (EFT).** EFT accesses primary emotions beneath secondary ones to de-escalate the cycle and restructure the bond.
- **A. The Gottman Method** — Owner: Gottman. Gottman targets the Four Horsemen and friendship through coaching, not accessing primary attachment emotion.
- **B. Bowen family systems therapy** — Owner: Bowen. Bowen coaches calm differentiation and detriangulation, not access to attachment emotions.
- **D. Strategic family therapy** — Owner: Haley (strategic). Strategic therapy uses directives to solve the presenting problem.

- **Pair to learn:** Primary emotion beneath secondary anger → EFT (Johnson), not Gottman

82 — B

Areas of Clinical Focus · Outline 3.H · Chapter 10

- **B. O. Hobart Mowrer — Owner: Mowrer (two-factor theory).** Mowrer's two-factor theory holds that fear is learned by classical conditioning and kept going because avoidance is negatively reinforced.
- **A. David Clark** — Owner: Clark (cognitive model of panic). Clark explained panic as catastrophic misinterpretation of body sensations.
- **C. Yerkes and Dodson** — Owner: Yerkes-Dodson law. The Yerkes-Dodson law relates arousal to performance; it doesn't explain how fear is acquired or maintained.
- **D. Martin Seligman** — Owner: Seligman (preparedness). Seligman's preparedness explains why some fears, such as snakes and heights, are learned more easily, not how avoidance maintains fear.
- **Pair to learn:** Classical acquisition + operant maintenance → Mowrer's two-factor theory

83 — B

Professional Practice & Ethics · Outline 1.F · Chapter 6

- **B. Ewing v. Goldstein — Owner: Ewing v. Goldstein (2004, California).** Ewing extended the duty to protect to credible threats relayed by a client's family member.
- **A. Jablonski v. United States** — Owner: Jablonski (duty from history of violence, no explicit threat). Jablonski based the duty on a client's history of violence, not a family member's report.
- **C. Tarasoff v. Regents (1976)** — Owner: Tarasoff II (duty to protect an identifiable victim). Tarasoff II broadened the duty to warn into a duty to protect; it involved the client's own threat.
- **D. Jaffee v. Redmond** — Owner: Jaffee v. Redmond (psychotherapist privilege in federal courts). Jaffee

recognized psychotherapist-patient privilege; it isn't a duty-to-protect case.

- **Pair to learn:** Threat reported by family member → Ewing v. Goldstein

84 — B

Areas of Clinical Focus · Outline 3.AI · Chapter 11

- **B. Opioid withdrawal — Owner: Opioids (flu-like withdrawal; rarely life-threatening).** Runny nose, tearing, yawning, goosebumps, dilated pupils, aches, and diarrhea form the flu-like opioid withdrawal picture.
- **A. Alcohol withdrawal** — Owner: Alcohol (tremor, sweating, seizures). Alcohol withdrawal brings tremor, sweating, rapid pulse, and possible seizures, not yawning, tearing, and goosebumps.
- **C. Cannabis withdrawal** — Owner: Cannabis (irritability, sleep difficulty). Cannabis withdrawal involves irritability, anxiety, sleep difficulty, and decreased appetite.
- **D. Stimulant intoxication** — Owner: Stimulants (euphoria, dilated pupils, agitation). Stimulant intoxication also dilates pupils but brings euphoria, rapid heart rate, and agitation rather than flu-like symptoms.
- **Pair to learn:** Yawning, tearing, goosebumps, dilated pupils → opioid withdrawal

85 — C

Areas of Clinical Focus · Outline 3.V · Chapter 10

- **C. Applied tension — Owner: Öst (blood-injection-injury phobia).** Applied tension counters the vasovagal drop in heart rate and blood pressure that can cause fainting in this phobia.
- **A. Relaxation response** — Owner: Benson. Benson's relaxation response lowers arousal, the opposite of what a client prone to fainting needs.

- **B. Progressive muscle relaxation** — Owner: Jacobson. PMR tenses and then releases muscles to lower arousal; relaxing would worsen a vasovagal blood pressure drop.
- **D. Interoceptive exposure** — Owner: Barlow (panic control treatment). Interoceptive exposure deliberately brings on feared body sensations in panic treatment; it isn't designed to prevent fainting.
- **Pair to learn:** Tensing muscles to prevent fainting → applied tension (Öst)

86 — B

Professional Practice & Ethics · Outline 1.L · Chapter 17

- **B. Assent — Owner: Assent (minors; ACA A.2.d, B. 5.b).** Parents or guardians generally consent; the minor gives assent.
- **A. Informed consent** — Owner: Informed consent (ACA A.2; given by those legally able). Legal consent here comes from the parents, so the child's agreement isn't informed consent.
- **C. Authorization** — Owner: HIPAA authorization (permission to release information). An authorization permits release of information, not participation in counseling.
- **D. Privileged communication** — Owner: Privileged communication (legal protection in court). Privilege protects communications from disclosure in court; it isn't agreement to services.
- **Pair to learn:** Minor's agreement alongside parental consent → Assent

87 — B

Areas of Clinical Focus · Outline 3.R · Chapter 13

- **B. Understanding the context — Owner: Baucom, Snyder, & Gordon (phase 2 of affair recovery).** The second phase examines how the relationship and each partner contributed, without excusing the affair.

- **A. Awareness** — Owner: Papernow (Stepfamily Cycle). Awareness is an early stage of Papernow's Stepfamily Cycle, not affair recovery.
- **C. Emotional station** — Owner: Bohannan (six stations of divorce). Bohannan's emotional station describes a deteriorating relationship in divorce, not recovery after an affair.
- **D. Dealing with the impact** — Owner: Baucom, Snyder, & Gordon (phase 1). Phase 1 is crisis, disclosure, safety, and stabilizing, which this couple has completed.
- **Pair to learn:** Exploring how each partner contributed → understanding the context (Baucom, Snyder, & Gordon)

88 — C

Treatment Planning · Outline 4.B · Chapter 21

- **C. Client reports 4 panic attacks a week and missed 3 workdays this month — Owner: Problem statement (behavioral terms).** A problem statement describes the concern in concrete, behavioral terms that can serve as a baseline.
- **A. Client has a great deal of anxiety and simply wants to feel calmer overall** — Owner: Vague goal language (not behavioral). This is vague and doesn't describe observable behavior or frequency.
- **B. Client will reduce panic attacks to 1 or fewer a week within 8 weeks** — Owner: Short-term objective (measurable step). This is a measurable short-term objective, not a statement of the problem.
- **D. Counselor will teach the panic cycle using CBT in weekly sessions** — Owner: Intervention (what the counselor will do). This describes an intervention, not the client's problem.
- **Pair to learn:** Concern in behavioral terms → Problem statement

89 — D

Counseling Skills & Interventions · Outline 5.H · Chapter 9

- **D. An emic approach — Owner: Emic (culture-specific).** Emic means culture-specific understanding from inside the client's culture.
- **A. Low-context communication** — Owner: Hall. Low-context communication carries meaning in explicit words.
- **B. An etic approach** — Owner: Etic (universal). Etic applies the same concepts across cultures.
- **C. Individualism** — Owner: Hofstede (cultural dimensions). Individualism prioritizes individual goals and independence.
- **Pair to learn:** Culture-specific → emic; universal → etic

90 — C

Areas of Clinical Focus · Outline 3.AD · Chapter 12

- **C. Disinhibited social engagement disorder — Owner: DSM-5-TR (overly familiar behavior after severe neglect).** Overly familiar behavior with unfamiliar adults and little hesitation to go off with strangers after severe neglect defines disinhibited social engagement disorder.
- **A. Reactive attachment disorder** — Owner: DSM-5-TR (withdrawn presentation after severe neglect). Reactive attachment disorder is emotionally withdrawn, with a child who rarely seeks or responds to comfort, the opposite of indiscriminate friendliness.
- **B. Anxious-resistant attachment** — Owner: Ainsworth (Strange Situation). Anxious-resistant is Ainsworth's infant classification of ambivalent behavior toward the caregiver, not friendliness toward strangers.
- **D. Separation anxiety disorder** — Owner: DSM-5-TR anxiety disorder (4 weeks in children). Separation

anxiety is excessive fear about separating from attachment figures; he shows no such fear.

- **Pair to learn:** Overly familiar with strangers after neglect → DSED (not RAD)

91 — B

Counseling Skills & Interventions · Outline 5.AV · Chapter 8

- **B. Cutting off — Owner: Group leader skill: cutting off.** Cutting off interrupts a member who goes on too long to manage time and focus.
- **A. Blocking** — Owner: Group leader skill: blocking. Blocking stops harmful behavior, such as an attack, to protect a member.
- **C. Linking** — Owner: Group leader skill: linking. Linking connects one member's concern to another's.
- **D. Process observation** — Owner: Group leader skill: process observation. Process observation comments on how the group is interacting.
- **Pair to learn:** Monopolizing → cutting off (not blocking)

92 — B

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **B. Prescribing the symptom — Owner: Haley (strategic).** Directing a family to perform its problem behavior on purpose is a paradoxical directive from strategic therapy.
- **A. Paradoxical intention** — Owner: Frankl (logotherapy). Paradoxical intention is used with an individual who intends the feared symptom, not as a family directive.
- **C. Enactment** — Owner: Minuchin (structural). Enactment has members interact in session so the counselor can observe and restructure.

- **D. Circular questioning** — Owner: Milan systemic. Circular questions ask how members respond to one another; they don't direct behavior.
- **Pair to learn:** Prescribing the symptom in a family → Haley (not Frankl's paradoxical intention)

93 — D

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **D. Carl Rogers — Owner: Rogers (person-centered).** The actualizing tendency is a core term in Rogers' person-centered therapy.
- **A. Viktor Frankl** — Owner: Frankl (logotherapy). Frankl centers on meaning and responsibility, not an actualizing tendency.
- **B. Abraham Maslow** — Owner: Maslow (hierarchy of needs). Maslow's self-actualization sits at the top of a motivational hierarchy of needs; he wrote a theory, not a therapy.
- **C. Alfred Adler** — Owner: Adler (individual psychology). Adler wrote of striving for superiority and social interest.
- **Pair to learn:** Actualizing tendency → Rogers (not Maslow's self-actualization)

94 — D

Professional Practice & Ethics · Outline 1.C · Chapter 17

- **D. Determine the nature of the dilemma — Owner: Forester-Miller & Davis: step 3.** After identifying the problem and applying the Code, the counselor examines the principles involved, relevant literature, and consultation.
- **A. Apply the publicity test to the choice** — Owner: Stadler's three tests (justice, publicity, universality). The publicity test belongs to Stadler and is used to check a chosen action, not as step 3.

- **B. Evaluate the selected course of action** — Owner: Forester-Miller & Davis: step 6. Evaluation comes after a course of action has been chosen.
- **C. Implement the course of action** — Owner: Forester-Miller & Davis: step 7. Implementing is the final step, after options are generated, weighed, and evaluated.
- **Pair to learn:** Step 3 → Determine nature and dimensions of the dilemma (Forester-Miller & Davis)

95 — B

Areas of Clinical Focus · Outline 3.AV · Chapter 13

- **B. Perpetual problem — Owner: Gottman (lasting differences).** Most couple conflicts are perpetual, rooted in lasting differences; the counseling goal is dialogue and acceptance, not a final solution.
- **A. Pursue-withdraw cycle** — Owner: Emotionally focused therapy. EFT's pursue-withdraw cycle describes one partner pressing and the other retreating in conflict; the stem describes a lasting difference in preferences.
- **C. Solvable problem** — Owner: Gottman (problems that can be settled). A difference rooted in personality that has lasted 15 years isn't the kind of problem that gets settled.
- **D. Fatuous love** — Owner: Sternberg (passion + commitment). Fatuous love is one of Sternberg's combinations of love components, not a type of conflict.
- **Pair to learn:** Lasting differences that won't be resolved → perpetual problem (Gottman)

96 — C

Counseling Skills & Interventions · Outline 5.K · Chapter 7

- **C. The interaction stage — Owner: Haley (first interview).** After the problem stage, members talk to

each other about the problem while the counselor observes.

- **A. The task-setting stage** — Owner: Haley (first interview). Task setting, giving a directive, is the final stage.
- **B. The goal-setting stage** — Owner: Haley (first interview). Goal setting follows the interaction stage.
- **D. Exploring feelings and emotions** — Owner: Roberts (crisis stage 4). This is a stage of Roberts' crisis model, not Haley's interview.
- **Pair to learn:** Social → problem → interaction → goal → task → Haley's first interview

97 — A

Counseling Skills & Interventions · Outline 5.AU · Chapter 8

- **A. Invite feelings about ending and review learning — Owner: Corey: final stage.** The final stage calls for addressing feelings about ending and consolidating learning.
- **B. Encourage members to challenge the leader's style** — Owner: Corey: transition stage. Challenges to the leader belong to the transition stage, not the ending.
- **C. Introduce an intense new disclosure exercise** — Owner: Opening new material at the end. Opening deep new material in the final sessions leaves no time to process it.
- **D. Add sessions so no one has to say goodbye** — Owner: Avoiding termination. Extending the group avoids the work of ending.
- **Pair to learn:** Sadness about ending → final stage: process ending, consolidate learning

98 — C

Areas of Clinical Focus · Outline 3.L · Chapter 10

- **C. Hopelessness theory — Owner: Abramson, Metalsky, & Alloy.** Abramson, Metalsky, and Alloy proposed that expecting negative outcomes with no way to change them leads to a hopelessness subtype of depression.
- **A. Reformulated helplessness theory** — Owner: Abramson, Seligman, & Teasdale (attributional theory). This theory emphasizes internal, stable, global explanations for bad events, not a hopelessness subtype.
- **B. Cognitive triad** — Owner: Beck (cognitive theory). Beck's triad describes negative views of self, world, and future; it does not propose a hopelessness subtype of depression.
- **D. Response styles theory** — Owner: Nolen-Hoeksema. Response styles theory explains how rumination prolongs and deepens depression.
- **Pair to learn:** Hopelessness subtype of depression → Abramson, Metalsky, & Alloy

99 — B

Areas of Clinical Focus · Outline 3.Z · Chapter 15

- **B. Stereotype threat — Owner: Steele & Aronson.** Concern about confirming a negative stereotype about one's group lowers performance when the stereotype is made relevant.
- **A. John Henryism** — Owner: Sherman James. John Henryism is high-effort coping linked to health problems.
- **C. Impostor phenomenon** — Owner: Clance & Imes. The impostor phenomenon is high achievers feeling like frauds, not a performance drop tied to how a test is framed.

- **D. Race-based traumatic stress** — Owner: Robert Carter. Race-based traumatic stress refers to trauma-like reactions to racism, not test performance shifts.
- **Pair to learn:** Fear of confirming a stereotype lowers performance → stereotype threat (Steele & Aronson)

100 — B

Intake, Assessment & Diagnosis · Outline 2.A · Chapter 19

- **B. Precipitating factor** — **Owner: 4 Ps: precipitating.** Precipitating factors answer the question of what triggered the current problem. The layoff came right before the panic began.
- **A. Predisposing factor** — Owner: 4 Ps: predisposing. Predisposing factors are vulnerabilities such as genetics, early experiences, and temperament, not the recent trigger.
- **C. Protective factor** — Owner: 4 Ps: protective. Protective factors are strengths and supports that help; a layoff is not one.
- **D. Perpetuating factor** — Owner: 4 Ps: perpetuating. Perpetuating factors keep the problem going, such as avoidance, substance use, or negative thinking.
- **Pair to learn:** What triggered the current problem → Precipitating factor (4 Ps)

101 — C

Counseling Skills & Interventions · Outline 5.I · Chapter 7

- **C. Joining** — **Owner: Minuchin (joining; used by all systemic models).** Joining connects with each member and respects the family's hierarchy and culture before any change is attempted.
- **A. Detriangulation** — Owner: Bowen. Detriangulation helps a pair deal directly with each other instead of drawing in a third person.
- **B. Enactment** — Owner: Minuchin (structural). Enactment has members interact in session around a problem.

- **D. Circular questioning** — Owner: Milan systemic. Circular questions ask how members respond to each other's behavior.
- **Pair to learn:** Engage every member, respect hierarchy and culture → joining

102 — C

Areas of Clinical Focus · Outline 3.AU · Chapter 13

- **C. Intimate terrorism — Owner: Michael P. Johnson (coercive controlling violence).** Violence used within a broader pattern of coercive control, usually frequent and escalating, is intimate terrorism.
- **A. Coercive family process** — Owner: Patterson (coercion theory). Patterson's coercion theory describes parents and children reinforcing escalating behavior, not partner violence.
- **B. Violent resistance** — Owner: Michael P. Johnson (IPV typology). Violent resistance is violence used in response to a partner's intimate terrorism; she is the one being controlled.
- **D. Situational couple violence** — Owner: Michael P. Johnson (IPV typology). Situational couple violence arises from specific conflicts without a broader pattern of control; here control is pervasive.
- **Pair to learn:** Violence within a pattern of coercive control → intimate terrorism (Johnson)

103 — C

Areas of Clinical Focus · Outline 3.AX · Chapter 10

- **C. Disruptive mood dysregulation disorder — Owner: DSM-5-TR (12 months; diagnosed ages 6 to 18; onset before 10).** Severe outbursts 3 or more times a week with persistently irritable mood between them for at least 12 months, with onset before age 10, defines DMDD.
- **A. Intermittent explosive disorder** — Owner: DSM-5-TR (outbursts out of proportion, age 6+). IED

involves outbursts, but not the persistently irritable mood between them that defines DMDD.

- **B. Bipolar I disorder** — Owner: DSM-5-TR (manic episode). No distinct manic episode is described; chronic irritability is not mania.
- **D. Oppositional defiant disorder** — Owner: DSM-5-TR (4 symptoms for 6 months). When criteria for both are met, only DMDD is diagnosed; severe recurrent outbursts with chronic irritability point to DMDD.
- **Pair to learn:** Outbursts 3+/week + chronic irritability for 12 months → DMDD

104 — A

Counseling Skills & Interventions · Outline 5.Q · Chapter 6

- **A. A batterer intervention program — Owner: Batterer intervention (IPV standard).** Batterer intervention programs, not anger management alone, are the standard referral for those who use violence.
- **B. Conjoint couples counseling** — Owner: Couples counseling (contraindicated with ongoing violence). Couples work is contraindicated while violence or coercive control is ongoing.
- **C. An anger management class alone** — Owner: Anger management (insufficient alone). Anger management alone is not the standard referral for partner violence.
- **D. An Al-Anon meeting** — Owner: Al-Anon (mutual support for families). Al-Anon is for family and friends of people with alcohol problems.
- **Pair to learn:** Person using violence → batterer intervention program (not anger management alone)

105 — D

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **D. Acceptance and commitment therapy — Owner: Hayes (ACT).** Defusion from thoughts and committed, values-based action are ACT's signature moves.
- **A. Rational emotive behavior therapy** — Owner: Ellis (REBT). REBT would dispute the belief directly, which is arguing with it.
- **B. Reality therapy** — Owner: Glasser/Wubbolding. Reality therapy evaluates whether current behavior meets the client's wants and needs.
- **C. Cognitive therapy** — Owner: Beck (CT). Beck would test the evidence for the thought rather than step back from it.
- **Pair to learn:** Cognitive defusion; values; committed action → Hayes (ACT)

106 — D

Counseling Skills & Interventions · Outline 5.R · Chapter 2

- **D. Displacement — Owner: Defense mechanism (Freud; A. Freud).** Displacement redirects feelings toward a safer target, here from the supervisor to the son.
- **A. Projection** — Owner: Defense mechanism (Freud; A. Freud). Projection attributes one's own unacceptable feelings to someone else; he isn't claiming his son is angry.
- **B. Reaction formation** — Owner: Defense mechanism (Freud; A. Freud). Reaction formation expresses the opposite of what one feels, such as excessive warmth toward the supervisor.
- **C. Sublimation** — Owner: Defense mechanism (Freud; A. Freud). Sublimation channels impulses into acceptable outlets; yelling at his son is not an acceptable channel.

- **Pair to learn:** Redirecting feelings to a safer target → displacement

107 — A

Counseling Skills & Interventions · Outline 5.AC · Chapter 4

- **A. "Help me understand what 'over it' means for you." — Owner: Clarification.** It asks about an unclear message rather than stating a meaning the counselor assumes.
- **B. "You're done with your job."** — Owner: Paraphrase. It states a meaning the counselor already assumes instead of checking it.
- **C. "You should update your résumé."** — Owner: Advice (not a listening skill). It tells the client what to do and skips exploration.
- **D. "You sound burned out and fed up."** — Owner: Reflection of feeling. It names emotions rather than checking what the client meant.
- **Pair to learn:** Asks because meaning is unclear → clarification (not paraphrase)

108 — A

Counseling Skills & Interventions · Outline 5.C · Chapter 9

- **A. Help her prepare to advocate for herself — Owner: ACA Advocacy Competencies (client level, acting with).** Empowering clients to self-advocate comes before acting on their behalf.
- **B. Call her employer and negotiate for her** — Owner: Client advocacy (acting on behalf of). Acting on her behalf is appropriate after empowerment is tried or when she can't act herself.
- **C. Launch a public campaign about workplace access** — Owner: Social/political advocacy (public arena). Public-arena advocacy is a broader level and not the first step for one client.

- **D. Refer her to an attorney and end counseling** — Owner: Abandonment. Ending services abandons the client and the counseling relationship.
- **Pair to learn:** Advocacy → empower the client first (acting with, then on behalf of)

109 — A

Areas of Clinical Focus · Outline 3.AM · Chapter 10

- **A. Not yet; the first month of school doesn't count — Owner: DSM-5-TR selective mutism (1 month, not the first month of school).** Selective mutism requires at least 1 month of not speaking in specific situations, and the first month of school doesn't count.
- **B. This fits social anxiety disorder** — Owner: DSM-5-TR (typically 6 months). Social anxiety typically requires 6 months, and nothing yet shows fear of judgment beyond a new-school adjustment.
- **C. This fits separation anxiety disorder** — Owner: DSM-5-TR (4 weeks in children). Separation anxiety is fear about separating from attachment figures, not consistently not speaking in one setting.
- **D. She meets criteria for selective mutism now** — Owner: Misapplied selective mutism time frame. Three weeks at the start of school doesn't satisfy the 1-month rule, which excludes the first month of school.
- **Pair to learn:** Selective mutism → 1 month, not counting the first month of school

110 — D

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **D. Jung's analytical psychology — Owner: Jung (analytical psychology).** These are Jung's terms; his goal is individuation, integrating conscious and unconscious.
- **A. Freud's psychoanalysis** — Owner: Freud (psychoanalysis). Freud's terms are id, ego, superego, psychosexual stages, and the unconscious.

- **B. Adler's individual psychology** — Owner: Adler (individual psychology). Adler's terms are social interest, lifestyle, birth order, and inferiority feelings.
- **C. Berne's transactional analysis** — Owner: Berne (TA). Berne's terms are Parent, Adult, and Child ego states, strokes, games, and life scripts.
- **Pair to learn:** Archetypes; collective unconscious; persona → Jung

111 — C

Intake, Assessment & Diagnosis · Outline 2.M · Chapter 20

- **C. Normal curve equivalent — Owner: Normal curve equivalent (NCE).** The NCE has a mean of 50 and an SD of 21.06 and is an equal-interval scale from 1 to 99 used in education.
- **A. Stanine** — Owner: Stanine (mean 5, SD about 2). Stanines have a mean of 5, an SD of about 2, and a range of 1 to 9.
- **B. Percentile rank** — Owner: Percentile rank (ordinal). Percentile ranks also run 1 to 99 but are ordinal, not equal-interval, and have no SD of 21.06.
- **D. T-score** — Owner: T-score (mean 50, SD 10). T-scores share the mean of 50 but have an SD of 10.
- **Pair to learn:** Mean 50, SD 21.06, equal-interval 1 to 99 → Normal curve equivalent (not T-score)

112 — A

Professional Practice & Ethics · Outline 1.U · Chapter 17

- **A. Get the client's consent — Owner: ACA A.7.b (consent before advocating for a specific client).** Counselors obtain client consent before advocating on a specific client's behalf.
- **B. Post about the office's practices online** — Owner: Public-arena advocacy without consent (risks H.6.d disclosure). Public posting about a client's situation risks confidentiality and still lacks her consent.

- **C. Stay out of it, since housing isn't counseling** — Owner: Ignoring client barriers (contrary to A.7.a). Counselors advocate to address barriers clients face; housing is one of them.
- **D. Call the housing office right away** — Owner: Advocacy without consent (contrary to A.7.b). Acting before the client agrees discloses her situation and skips her voice.
- **Pair to learn:** Advocating for a specific client → Client consent first (ACA A.7.b)

113 — B

Intake, Assessment & Diagnosis · Outline 2.H · Chapter 19

- **B. CALOCUS/CASII — Owner: CALOCUS/CASII (children and adolescents).** The CALOCUS/CASII is the level-of-care tool for children and adolescents.
- **A. The ASAM Criteria** — Owner: ASAM Criteria (substance use, six dimensions). The ASAM Criteria guide level of care for substance use, using six dimensions.
- **C. The WHODAS 2.0** — Owner: WHODAS 2.0 (functioning). The WHODAS 2.0 measures functioning; it is not a level-of-care placement tool.
- **D. LOCUS** — Owner: LOCUS (Level of Care Utilization System, adults). LOCUS is the level-of-care tool for adults.
- **Pair to learn:** Level of care for youth → CALOCUS/CASII (LOCUS is for adults)

114 — D

Intake, Assessment & Diagnosis · Outline 2.Q · Chapter 19

- **D. Weigh all sources and explore the discrepancy with him — Owner: Multiple sources of information (self-report plus collateral).** Self-report is efficient but subject to bias, insight, and

honesty. Using several sources, and exploring where they disagree, increases confidence in the assessment.

- **A. Accept the checklist, since self-report is most direct** — Owner: Single-source assessment (self-report only). Relying on self-report alone ignores conflicting collateral information; self-report is vulnerable to bias and limited insight.
- **B. Confront him with his wife's account right away** — Owner: Premature confrontation. Confrontation belongs after the alliance is established; confronting at intake invites defensiveness before the discrepancy has been explored.
- **C. Diagnose alcohol use disorder from the collateral reports** — Owner: Diagnosing from collateral alone (skipping integration). Collateral reports inform the assessment but don't replace it; diagnosing from them alone skips his own account and direct assessment.
- **Pair to learn:** Self-report conflicts with collateral → Integrate multiple sources (2.Q)

115 — A

Areas of Clinical Focus · Outline 3.AH · Chapter 16

- **A. Hardiness — Owner: Kobasa (commitment, control, challenge).** Kobasa's hardiness is made up of commitment, control, and challenge, which buffer stress.
- **B. Career adaptability** — Owner: Savickas (concern, control, curiosity, confidence). Savickas' four Cs are career construction resources; commitment, control, and challenge as a stress buffer are Kobasa's.
- **C. Resistance stage** — Owner: Selye (General Adaptation Syndrome). Resistance is a phase of the body's stress response, not a personality style that buffers stress.
- **D. Type A behavior** — Owner: Friedman & Rosenman. Type A is a competitive, impatient, hostile pattern, and hostility is the harmful part, not a buffer.

- **Pair to learn:** Commitment, control, challenge → hardiness (Kobasa, not Savickas)

116 — D

Core Counseling Attributes · Outline 6.J · Chapter 22

- **D. "Thanks for telling me. How safe are those drives home?" — Owner: Nonjudgmental stance that still addresses harm (6.J).** A nonjudgmental stance accepts the person without moral evaluation while still addressing harmful behavior and safety.
- **A. "I can't keep seeing you unless you stop this now."** — Owner: Ultimatum (conditional regard). Making counseling conditional on stopping sets conditions on acceptance and damages the alliance.
- **B. "Let's stay with what you came in for today."** — Owner: Avoiding harmful behavior. Nonjudgment doesn't mean ignoring a safety risk.
- **C. "That's illegal and could kill someone. You need to stop."** — Owner: Lecturing (moralizing). A lecture shows disapproval of the person, which the exam punishes even when the behavior is risky.
- **Pair to learn:** Accept the person, still address the risk → Nonjudgmental stance (not lecturing or ignoring)

117 — B

Intake, Assessment & Diagnosis · Outline 2.E · Chapter 19

- **B. E.5.d, Refraining from diagnosis — Owner: ACA Code E.5.d.** E.5.d allows counselors to refrain from making or reporting a diagnosis if they believe it would cause harm to the client.
- **A. E.5.b, Cultural sensitivity** — Owner: ACA Code E. 5.b. E.5.b concerns how culture affects the way problems are defined and expressed in diagnosis, not withholding a diagnosis.
- **C. E.4, Release of data to qualified personnel** — Owner: ACA Code E.4. E.4 governs releasing raw test data only to qualified persons with consent; it doesn't address whether to diagnose.

- **D. E.5.c, Historical and social prejudices** — Owner: ACA Code E.5.c. E.5.c concerns awareness of how some groups have been misdiagnosed and pathologized, not refraining from diagnosis.
- **Pair to learn:** Diagnosis would cause harm → Refraining from diagnosis (ACA E.5.d)

118 — A

Treatment Planning · Outline 4.L · Chapter 21

- **A. Discuss the alliance and adjust tasks with her — Owner: Checking the alliance when progress stalls (4.L).** A stalled plan is often an alliance problem; agreement on tasks is part of the alliance and can be revised together.
- **B. Continue the plan, since six sessions is early** — Owner: Repeating an ineffective plan. Flat scores plus alliance feedback call for review, not more of the same.
- **C. Refer her to a higher level of care** — Owner: Step up in level of care (4.E). Nothing suggests worsening symptoms; the feedback points to a task mismatch.
- **D. Switch to a new approach without discussing it** — Owner: Unilateral change (switching without collaboration). Changing approaches without discussion leaves her out of decisions about her care.
- **Pair to learn:** Stalled progress plus low task agreement → Check and repair the alliance (4.L)

119 — C

Professional Practice & Ethics · Outline 1.H · Chapter 17

- **C. Justice — Owner: Justice (ACA principle; Kitchener).** Fairness and equal access to services, such as a sliding scale, is justice.
- **A. Beneficence** — Owner: Beneficence (ACA principle; Kitchener). Beneficence is working for the client's good; the defining feature here is equitable access.

- **B. Fidelity** — Owner: Fidelity (ACA principle; Kitchener). Fidelity is keeping promises and commitments; no promise is at issue.
- **D. Autonomy** — Owner: Autonomy (ACA principle; Kitchener). Autonomy is the client's right to direct their own life, not fair distribution of services.
- **Pair to learn:** Sliding scale, equal access → Justice

120 — C

Counseling Skills & Interventions · Outline 5.AI · Chapter 8

- **C. Meet to process the conflict and plan ahead — Owner: Co-leadership (meet before and after sessions).** Co-leaders must meet to plan and process; open disagreement without repair can harm the group.
- **A. Ask members to vote on which leader was right** — Owner: Triangulating members into leader conflict. This pulls members into the leaders' conflict.
- **B. Never mention it, so members aren't upset** — Owner: Avoiding repair. Leaving a visible conflict unaddressed models avoidance.
- **D. Let the senior leader make all decisions from now on** — Owner: Abandoning shared leadership. This removes the benefits of co-leadership instead of repairing it.
- **Pair to learn:** Co-leader disagreement → meet before and after sessions to process

121 — B

Areas of Clinical Focus · Outline 3.X · Chapter 10

- **B. Posttraumatic growth — Owner: Tedeschi & Calhoun.** Positive change that follows a struggle with trauma is posttraumatic growth.
- **A. Window of tolerance** — Owner: Siegel. The window of tolerance is the arousal zone in which a person can function, not a description of growth.

- **C. Resilience** — Owner: Bonanno. Bonanno's resilience describes the common pattern of steady functioning after potential trauma; she describes positive change beyond where she started.
- **D. Shattered assumptions** — Owner: Janoff-Bulman. Shattered assumptions are broken beliefs about a benevolent world and a worthy self, not positive change.
- **Pair to learn:** Positive change after struggling with trauma → posttraumatic growth (Tedeschi & Calhoun, not Bonanno)

122 — D

Intake, Assessment & Diagnosis · Outline 2.R · Chapter 19

- **D. Interactional dynamics — Owner: Interactional dynamics (2.R).** In couple and family sessions, counselors observe who speaks, who stays silent, alliances, conflict patterns, and nonverbal responses.
- **A. The cultural definition of the problem** — Owner: CFI domain 1. The CFI's first domain asks how the client describes the problem; it is gathered by asking, not by observing who speaks for whom.
- **B. The daughter's thought process** — Owner: MSE thought process (how an individual thinks). Thought process describes how one person's thinking is organized; the observation here is about the pattern among family members.
- **C. The daughter's affect** — Owner: MSE affect (one person's observed emotional expression). Affect is one person's emotional expression in the moment; this observation concerns how members interact.
- **Pair to learn:** Who speaks, who's silent, alliances → Interactional dynamics (2.R)

123 — C

Areas of Clinical Focus · Outline 3.AE · Chapter 16

- **C. Rosemary Basson — Owner: Basson (circular model; responsive desire).** Basson's circular model

describes responsive rather than spontaneous desire, with emotional intimacy central.

- **A. Helen Singer Kaplan** — Owner: Kaplan (desire, excitement, orgasm). Kaplan's triphasic model places desire first, before excitement, rather than responsive to arousal.
- **B. Masters and Johnson** — Owner: Masters & Johnson (four-phase model). Masters and Johnson described excitement, plateau, orgasm, and resolution.
- **D. Jack Annon** — Owner: Annon (PLISSIT). Annon developed the PLISSIT model of sex counseling, not a model of sexual response.
- **Pair to learn:** Responsive desire, circular model → Basson (not Kaplan)

124 — B

Areas of Clinical Focus · Outline 3.O · Chapter 12

- **B. Vineland Adaptive Behavior Scales — Owner: Adaptive functioning measure.** The Vineland measures adaptive functioning, which is required for the diagnosis and is the basis for its severity level.
- **A. Wechsler Intelligence Scale for Children** — Owner: Wechsler (intellectual functioning, ages 6 to 16). The WISC measures intellectual functioning, the other half of the diagnosis, not adaptive skills.
- **C. Autism Diagnostic Observation Schedule** — Owner: ADOS-2 (autism assessment). The ADOS-2 is an observation schedule for assessing autism, not adaptive functioning.
- **D. Conners Rating Scales** — Owner: ADHD rating scales (parent and teacher forms). Conners scales rate ADHD symptoms, not adaptive functioning.
- **Pair to learn:** Adaptive functioning → Vineland (not WISC)

125 — C

Counseling Skills & Interventions · Outline 5.Q · Chapter 6

- **C. Examine alternatives — Owner: James & Gilliland (step 4).** Steps 1 to 3 (define the problem, ensure safety, provide support) are listening steps; examining alternatives begins the acting steps.
- **A. Provide support** — Owner: James & Gilliland (step 3). Providing support is the last of the listening steps.
- **B. Obtain commitment** — Owner: James & Gilliland (step 6). Obtaining commitment is the last acting step.
- **D. Make psychological contact** — Owner: Roberts (stage 2). Making psychological contact is a stage in Roberts' seven-stage model.
- **Pair to learn:** Listening steps then acting steps → James & Gilliland (not Roberts)

126 — A

Professional Practice & Ethics · Outline 1.D · Chapter 17

- **A. Decline and refer him to another counselor — Owner: ACA A.5.b (previous romantic partners).** Counselors may not become counselors to their previous romantic or sexual partners, no matter how much time has passed.
- **B. Accept after documenting the risks first** — Owner: Boundary extension (A.6), not applicable here. Documented boundary extensions apply to limited nonprofessional contact, not to a standard that prohibits the relationship.
- **C. Accept if he signs a written consent** — Owner: Client consent (doesn't override A.5.b). Consent doesn't make a prohibited relationship permissible.
- **D. Accept, since more than 5 years have passed** — Owner: ACA A.5.c 5-year rule (applies to former clients). The 5-year rule covers relationships with former clients, not counseling a former partner.

- **Pair to learn:** Former romantic partner seeks counseling → Prohibited (ACA A.5.b)

127 — A

Areas of Clinical Focus · Outline 3.U · Chapter 14

- **A. Tentative — Owner: Ginzberg et al. (about ages 11 to 17).** In the tentative period, choices are based increasingly on interests and abilities but remain provisional.
- **B. Social valuation** — Owner: Gottfredson (circumscription and compromise). Social valuation is Gottfredson's stage of ruling out options by prestige, not a Ginzberg period.
- **C. Fantasy** — Owner: Ginzberg et al. (to about age 11). The fantasy period comes before age 11, when choices aren't yet based on interests and abilities.
- **D. Growth** — Owner: Super (ages 0 to 14). Growth is Super's stage, not one of Ginzberg's periods.
- **Pair to learn:** Provisional choices from interests and abilities, ages 11 to 17 → Ginzberg's tentative period

128 — B

Professional Practice & Ethics · Outline 1.J · Chapter 9

- **B. Create a separate professional account — Owner: ACA H.6.a (virtual professional presence).** Counselors keep professional and personal social media presences separate.
- **A. Share brief client success stories** — Owner: Testimonials and confidentiality (C.3.b, H.6.d). Testimonials from current clients aren't allowed, and posting client stories risks disclosing confidential information.
- **C. Post the tips from her personal account** — Owner: Mixing personal and professional presence (contrary to H.6.a). Using her personal account blends her private life with her professional role.
- **D. Accept clients' friend requests to grow reach** — Owner: Personal virtual relationships with clients

(prohibited, A.5.e). Personal online relationships with current clients are prohibited.

- **Pair to learn:** Professional online presence → Separate accounts (ACA H.6.a)

129 — A

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **A. Gestalt therapy — Owner: Perls (Gestalt).** Contact-boundary disturbances are Gestalt vocabulary for ways awareness and contact are blocked.
- **B. Cognitive therapy** — Owner: Beck (CT). Beck's vocabulary is automatic thoughts, cognitive distortions, and schemas.
- **C. Structural family therapy** — Owner: Minuchin (structural). Minuchin's boundaries are rigid, clear, or diffuse lines between family subsystems, not contact-boundary disturbances.
- **D. Psychoanalysis** — Owner: Freud (defense mechanisms). Freud's defenses include repression, displacement, and reaction formation; the contact-boundary list belongs to Gestalt.
- **Pair to learn:** Contact-boundary disturbances → Perls (Gestalt)

130 — A

Areas of Clinical Focus · Outline 3.B · Chapter 12

- **A. Continuity theory — Owner: Atchley (continuity theory).** Continuity theory holds that people adapt to aging by keeping patterns, roles, and identity consistent with earlier life, which is exactly what she describes.
- **B. Activity theory** — Owner: Havighurst and colleagues (activity theory). Activity theory links satisfaction to staying active in general; her emphasis is on sameness with her earlier self, which is continuity.

- **C. Disengagement theory** — Owner: Cumming & Henry. Disengagement describes mutual withdrawal of older adults and society, the opposite of her ongoing involvement.
- **D. Socioemotional selectivity** — Owner: Carstensen. Socioemotional selectivity describes pruning peripheral ties to focus on emotionally meaningful ones, not preserving lifelong routines and roles.
- **Pair to learn:** Keeping lifelong patterns and identity → continuity theory (Atchley)

131 — A

Treatment Planning · Outline 4.K · Chapter 21

- **A. Review the scores with her and decide together — Owner: Using assessment results to facilitate client decisions (4.K).** Assessment results are a tool for the client's decision; reviewing them together supports an informed, collaborative choice.
- **B. Refer her for a medication evaluation** — Owner: Concurrent referral (4.F). Improving scores don't point to a new medication need; the question is session frequency.
- **C. Tell her the scores mean she must stay weekly** — Owner: Counselor-directed decision (undercuts collaboration, 4.A). Her scores have improved, and deciding for her bypasses her role in the plan.
- **D. End counseling now, since her score is under 10** — Owner: Termination without process (A.11.c pretermination counseling). She asked about reducing frequency, and ending abruptly skips termination work.
- **Pair to learn:** Client decision about frequency → Use assessment results together (4.K)

132 — C

Core Counseling Attributes · Outline 6.F · Chapter 22

- **C. Slow things down so each partner can speak and be heard — Owner: Conflict tolerance; modeling respectful conflict (6.F).** Effective

counselors stay present with conflict, name it non-defensively, and model respectful conflict in couples and families.

- **A. End the session until both partners have calmed down** — Owner: Conflict avoidance. Ending the session avoids the conflict instead of staying present with it.
- **B. Side with the partner who seems more reasonable** — Owner: Taking sides (appeasing one partner). Siding with one partner damages the alliance with the other and doesn't resolve the conflict.
- **D. Let them argue freely so pent-up anger is released** — Owner: Catharsis (emotional release, Yalom). Emotional release alone doesn't model respectful conflict and lets the escalation continue.
- **Pair to learn:** Staying present with escalating conflict → Conflict tolerance; model respectful conflict (6.F)

133 — B

Counseling Skills & Interventions · Outline 5.Q · Chapter 6

- **B. Erich Lindemann — Owner: Lindemann.** Lindemann's Cocoanut Grove work founded the field of crisis intervention.
- **A. Gerald Caplan** — Owner: Caplan (preventive psychiatry). Caplan built on crisis work with preventive psychiatry and the time-limited view of crisis.
- **C. Kenneth Doka** — Owner: Doka. Doka described disenfranchised grief, loss that isn't socially recognized.
- **D. Albert Roberts** — Owner: Roberts. Roberts developed the seven-stage crisis intervention model.
- **Pair to learn:** Cocoanut Grove fire; acute grief → Lindemann (not Caplan)

134 — D

Counseling Skills & Interventions · Outline 5.AT · Chapter 8

- **D. Universalizing — Owner: Group leader skill: universalizing.** Universalizing highlights a theme that many members share.
- **A. Linking** — Owner: Group leader skill: linking. Linking connects two specific members' concerns.
- **B. Drawing out** — Owner: Group leader skill: drawing out. Drawing out invites a quieter member in.
- **C. Process observation** — Owner: Group leader skill: process observation. Process observation comments on how members are interacting, not what they share.
- **Pair to learn:** Leader names a shared theme for the group → universalizing (not linking)

135 — B

Counseling Skills & Interventions · Outline 5.H · Chapter 9

- **B. Cultural encapsulation — Owner: Wrenn (1962).** Wrenn's term describes counselors who substitute their own cultural assumptions for reality.
- **A. Cultural humility** — Owner: Hook and colleagues. Cultural humility is the opposite stance: curious and aware of one's limits.
- **C. Acculturative stress** — Owner: Acculturation (Berry). Acculturative stress is the client's stress from adapting to a new culture.
- **D. Microinvalidation** — Owner: Sue (microaggressions). A microinvalidation is a specific remark that negates a person's experience.
- **Pair to learn:** Imposing one's own culture as reality → cultural encapsulation (Wrenn)

136 — A

Counseling Skills & Interventions · Outline 5.F · Chapter 5

- **A. Implosive therapy — Owner: Stampfl (implosive therapy).** Stampfl's implosive therapy is intense, imagined, often exaggerated exposure.
- **B. Systematic desensitization** — Owner: Wolpe. Wolpe pairs relaxation with imagined scenes climbed gradually up a hierarchy.
- **C. Interoceptive exposure** — Owner: CBT for panic. Interoceptive exposure brings on feared body sensations.
- **D. Flooding** — Owner: Behavioral (flooding). Flooding is typically prolonged, intense exposure to the actual feared situation; the imagined, deliberately exaggerated version is Stampfl's implosive therapy.
- **Pair to learn:** Intense imagined exposure → implosive therapy (Stampfl), not flooding

137 — C

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **C. Glasser's choice theory — Owner: Glasser (reality therapy/choice theory).** Choice theory names these five needs; reality therapy helps clients meet them through more effective choices.
- **A. Maslow's hierarchy of needs** — Owner: Maslow. Maslow's levels are physiological, safety, love/belonging, esteem, and self-actualization.
- **B. Lazarus's BASIC I.D.** — Owner: Lazarus (multimodal therapy). BASIC I.D. is seven assessment modalities, from Behavior to Drugs/biology, not a list of needs.
- **D. Yalom's four givens** — Owner: Yalom (existential). Yalom's givens are death, freedom, isolation, and meaninglessness.

- **Pair to learn:** Five basic needs; quality world → Glasser

138 — C

Areas of Clinical Focus · Outline 3.K · Chapter 12

- **C. He may meet criteria for prolonged grief disorder — Owner: DSM-5-TR prolonged grief disorder (6 months for children and adolescents).** For children and adolescents the threshold is 6 months since the death; he has daily yearning plus at least 3 of the 8 additional symptoms, beyond cultural norms.
- **A. This is uncomplicated bereavement** — Owner: Uncomplicated bereavement (Z code, not a disorder). Uncomplicated bereavement doesn't fit grief this intense, impairing, and clearly beyond cultural norms.
- **B. It is too early, since 12 months must pass** — Owner: DSM-5-TR adult threshold (12 months). The 12-month threshold applies to adults; children and adolescents need at least 6 months.
- **D. This is persistent depressive disorder** — Owner: DSM-5-TR (1 year in children and adolescents). PDD requires 1 year of depressed or irritable mood in youth, and his symptoms center on yearning for the deceased.
- **Pair to learn:** Prolonged grief disorder in youth → 6 months (adults 12)

139 — D

Treatment Planning · Outline 4.F · Chapter 21

- **D. Offer options, get a release, and follow up — Owner: Referral process (4.F).** Good referrals involve the client, offer options, use a signed release before sharing, and confirm the client connected.
- **A. Fax her records to the nearest psychiatrist today** — Owner: Sharing without a release (ACA B. 6.g). Ethically, information isn't shared without the client's signed release.
- **B. Give her one name and close her counseling case** — Owner: Abandonment (ACA A.12). This is a

concurrent referral; closing her case abandons her mid-treatment.

- **C. Tell her to find a prescriber through her insurer** — Owner: Passive referral without follow-up. Leaving the search to her skips options, coordination, and follow-up.
- **Pair to learn:** Concurrent medication referral → Options, release, follow-up (4.F)

140 — B

Intake, Assessment & Diagnosis · Outline 2.M · Chapter 20

- **B. E.11, Obsolete assessment and outdated results — Owner: ACA Code E.11.** E.11 directs counselors not to use outdated instruments or results for current decisions.
- **A. E.7, Conditions of administration** — Owner: ACA Code E.7. E.7 concerns standard administration conditions, noting irregularities, and validated technological administration.
- **C. E.10, Assessment security** — Owner: ACA Code E. 10. E.10 concerns keeping tests secure and not reproducing or modifying them without permission.
- **D. E.8, Multicultural issues in assessment** — Owner: ACA Code E.8. E.8 urges caution with instruments normed on populations other than the client's; age of the results is a different issue.
- **Pair to learn:** Outdated tests or results for current decisions → ACA E.11 (not E.10)

141 — B

Core Counseling Attributes · Outline 6.D · Chapter 22

- **B. Examine her gender-role assumptions in supervision — Owner: Awareness of one's own gender-role assumptions (6.D).** Sensitivity to gender issues includes awareness of one's own gender-role assumptions and their effect on clients; supervision supports that reflection.

- **A. Disclose the pattern to each client and apologize** — Owner: Self-disclosure that shifts focus to the counselor. This centers the counselor's process and burdens clients instead of changing her practice.
- **C. Continue, since these reflect most clients' interests** — Owner: Gender-role stereotyping. The pattern overrides clients' stated interests, which is the assumption she needs to examine.
- **D. Refer her male clients to a male colleague** — Owner: Premature referral (avoiding self-work). Referral sidesteps the counselor's own bias rather than addressing it.
- **Pair to learn:** Counselor's gender-role bias steering clients → Examine own assumptions (6.D)

142 — D

Counseling Skills & Interventions · Outline 5.F · Chapter 5

- **D. Exposure and response prevention — Owner: Behavioral (ERP for OCD).** ERP pairs facing the obsession with blocking the compulsion, here the repeated checking.
- **A. A thought record** — Owner: Beck (CT). A thought record examines automatic thoughts; alone it doesn't stop the checking cycle.
- **B. Systematic desensitization** — Owner: Wolpe. Systematic desensitization uses relaxation and imagined scenes; it doesn't block a compulsion.
- **C. Interoceptive exposure** — Owner: CBT for panic. Interoceptive exposure targets feared body sensations in panic, not compulsions.
- **Pair to learn:** Face the obsession, block the compulsion → ERP

143 — B

Treatment Planning · Outline 4.G · Chapter 21

- **B. His substance use — Owner: Sequencing: problems that block other work.** With safety and

basic needs secure, a problem that undermines counseling itself comes before his other goals.

- **A. Career exploration** — Owner: Sequencing: client's most pressing concern (comes later). His career goal matters, but substance use blocks progress on it.
- **C. Family-of-origin exploration** — Owner: Insight work (longer-term growth). Insight work can't take hold while he can't retain session content.
- **D. Long-term personal growth** — Owner: Sequencing: longer-term growth and prevention (last). Growth work comes after problems that block counseling are addressed.
- **Pair to learn:** Substance use undermining therapy → Problems that block other work first (4.G)

144 — C

Intake, Assessment & Diagnosis · Outline 2.M · Chapter 20

- **C. 4.5** — **Owner: SEM = SD × √(1 − reliability).** SEM = 15 × √(1 − .91) = 15 × √.09 = 15 × .3 = 4.5.
- **A. 1.35** — Owner: Error: SD × (1 − r) without the square root. 15 × .09 = 1.35 forgets to take the square root of (1 − reliability).
- **B. 13.65** — Owner: Error: SD × r. 15 × .91 = 13.65 multiplies by the reliability itself instead of by √(1 − reliability).
- **D. 14.3** — Owner: Error: SD × √r. 15 × √.91 is about 14.3, which uses reliability instead of (1 − reliability); higher reliability should shrink the SEM.
- **Pair to learn:** SEM = SD × √(1 − r); higher reliability → smaller SEM

145 — A

Counseling Skills & Interventions · Outline 5.E · Chapter 4

- **A. Gerard Egan — Owner: Egan (The Skilled Helper).** Egan's model, from The Skilled Helper, also gives us SOLER.
- **B. Allen Ivey** — Owner: Ivey (intentional interviewing). Ivey owns the microskills hierarchy and the five-stage interview.
- **C. Albert Roberts** — Owner: Roberts (crisis intervention). Roberts owns a seven-stage crisis intervention model.
- **D. Clara Hill** — Owner: Hill (helping skills model). Hill's three stages are exploration, insight, and action.
- **Pair to learn:** Current picture → preferred picture → way forward → Egan (not Hill)

146 — D

Intake, Assessment & Diagnosis · Outline 2.M · Chapter 20

- **D. Rotter Incomplete Sentences Blank — Owner: Julian Rotter.** Rotter's Incomplete Sentences Blank is a projective test built on completing sentence stems.
- **A. Thematic Apperception Test** — Owner: Henry Murray and Christiana Morgan. The TAT asks clients to tell stories about ambiguous pictures; it is scored for needs and press.
- **B. Kinetic Family Drawing** — Owner: Burns and Kaufman. The Kinetic Family Drawing asks the client to draw family members doing something.
- **C. House-Tree-Person** — Owner: John Buck. The House-Tree-Person is a projective drawing task, not sentence completion.
- **Pair to learn:** Completing sentence stems → Rotter Incomplete Sentences Blank (not TAT)

147 — B

Counseling Skills & Interventions · Outline 5.AP · Chapter 8

- **B. "Ana, I noticed you leaned in. Want to add anything?" — Owner: Drawing out.** It gently invites a quiet member in without forcing her to speak.
- **A. "Ana, everyone else has shared, so now it's your turn."** — Owner: Forcing a silent member to speak. Pressuring participation is punished and ignores cultural norms about speaking.
- **C. "Ana, your silence feels like resistance to the group."** — Owner: Labeling silence as resistance (ASGW caution). A quiet member is not automatically resistant.
- **D. "Ana's situation sounds just like Ben's, I'm guessing."** — Owner: Linking (premature). Linking connects members, but here it speaks for her before she has shared.
- **Pair to learn:** Inviting a quiet member in gently → drawing out

148 — C

Professional Practice & Ethics · Outline 1.W · Chapter 17

- **C. Psychotherapy notes — Owner: HIPAA psychotherapy notes (extra protection; specific authorization).** Psychotherapy notes are kept separate from the clinical record and generally require specific authorization to disclose.
- **A. The designated record set** — Owner: HIPAA designated record set (clinical record). The designated record set is the clinical record itself: dates, diagnosis, plan, progress summaries.
- **B. 42 CFR Part 2 records** — Owner: 42 CFR Part 2 (federally assisted SUD program records). Part 2 protects substance use disorder program records, not a counselor's separate process notes.

- **D. SOAP progress notes** — Owner: SOAP format (progress notes in the clinical record). SOAP notes are standard progress notes that belong in the clinical record.
- **Pair to learn:** Separate process notes with extra protection → HIPAA psychotherapy notes

149 — A

Intake, Assessment & Diagnosis · Outline 2.F · Chapter 19

- **A. A delusion, under thought content — Owner: Delusion (thought content).** A delusion is a fixed false belief held despite evidence. Delusions are recorded under thought content, which covers what the client thinks.
- **B. An obsession, under thought content** — Owner: Obsession. An obsession is an intrusive thought the person usually recognizes as excessive; this client is fully convinced.
- **C. A hallucination, under perception** — Owner: Hallucination (perception). A hallucination is a sensory experience with no stimulus present; this is a belief, not a sensory experience.
- **D. Loose associations, under thought process** — Owner: Loose associations (thought process). Loose associations describe how ideas connect, with no logical link between them; the problem here is what he believes, not how his thoughts flow.
- **Pair to learn:** Fixed false belief despite evidence → Delusion (not obsession)

150 — C

Counseling Skills & Interventions · Outline 5.AD · Chapter 4

- **C. “I moved far from family once too, and it took time to feel at home.” — Owner: Self-disclosure.** It shares something from the counselor’s own life; it should be brief and return focus to the client.

- **A. “You’ve said you’re fine, yet you’ve teared up twice today.”** — Owner: Confrontation. It points out a discrepancy between words and nonverbals.
- **B. “Right now I feel a bit shut out as you keep checking your phone.”** — Owner: Immediacy. It shares the counselor’s reaction to this client in this moment.
- **D. “It sounds like you feel lonely in this new city.”** — Owner: Reflection of feeling. It names the client’s emotion and shares nothing about the counselor.
- **Pair to learn:** Counselor’s own life → self-disclosure; reaction to client now → immediacy

151 — B

Professional Practice & Ethics · Outline 1.G · Chapter 17

- **B. Agency policy allows it and she knows other options — Owner: ACA A.10.a (self-referral).** Agency counselors may refer clients to their own practices only when the agency allows it and clients are informed of other options.
- **A. Five years pass after agency services end** — Owner: ACA A.5.c 5-year rule (former-client romantic relationships). The 5-year waiting period applies to romantic relationships with former clients, not self-referral.
- **C. Her supervisor gives informal verbal approval** — Owner: Consultation (useful, but not the A.10.a condition). Consulting is wise, but the standard turns on agency policy and the client’s knowledge of alternatives.
- **D. The client pays the same fee charged at the agency** — Owner: Establishing fees (A.10.c), not the self-referral test. Fee setting is a separate standard; matching the fee doesn’t satisfy the self-referral conditions.
- **Pair to learn:** Agency client to counselor’s private practice → Self-referral rules (ACA A.10.a)

152 — D

Treatment Planning · Outline 4.M · Chapter 18

- **D. Goal Attainment Scaling — Owner: Goal Attainment Scaling (Kiresuk & Sherman).** Goal Attainment Scaling rates progress on goals tailored to each client.
- **A. Reliable change index** — Owner: Reliable change index (Jacobson & Truax). The reliable change index tests whether change exceeds measurement error.
- **B. Measurement-based care** — Owner: Measurement-based care (routine brief measures guide treatment). Measurement-based care is a practice of routine measurement, not Kiresuk and Sherman's goal-rating method.
- **C. Outcome Rating Scale** — Owner: ORS (Miller & Duncan; feedback-informed treatment). The ORS tracks general functioning session by session; it belongs to Miller and Duncan.
- **Pair to learn:** Individualized goal ratings → Goal Attainment Scaling (Kiresuk & Sherman)

153 — A

Areas of Clinical Focus · Outline 3.Z · Chapter 15

- **A. Iris Marion Young — Owner: Young (five faces of oppression).** Iris Marion Young described these five faces of oppression.
- **B. Ignacio Martín-Baró** — Owner: Martín-Baró (liberation psychology). Martín-Baró is the owner of liberation psychology.
- **C. Kimberlé Crenshaw** — Owner: Crenshaw (intersectionality). Crenshaw is the owner of intersectionality.
- **D. Paulo Freire** — Owner: Freire (critical consciousness). Freire described critical consciousness, recognizing and acting against oppressive conditions.

- **Pair to learn:** Five faces of oppression → Iris Marion Young (not Freire)

154 — B

Counseling Skills & Interventions · Outline 5.T · Chapter 7

- **B. PREP (Markman, Stanley, & Blumberg) — Owner: PREP (speaker-listener technique).** The speaker-listener technique is PREP's structured communication tool.
- **A. Satir's human validation model** — Owner: Satir (experiential). Satir used family sculpting and congruent communication.
- **C. The Gottman Method** — Owner: Gottman. Gottman's tools include softened startup, repair attempts, and love maps.
- **D. Thomas Gordon's Parent Effectiveness Training** — Owner: Gordon (I-messages). Gordon taught I-messages and active listening to parents.
- **Pair to learn:** Speaker-listener technique → PREP (not Gottman)

155 — B

Areas of Clinical Focus · Outline 3.X · Chapter 10

- **B. Remembrance and mourning — Owner: Herman (stage 2 of trauma recovery).** After safety and stabilization, Herman's second stage is remembrance and mourning: telling the trauma story and grieving its losses.
- **A. Safety and stabilization** — Owner: Herman (stage 1 of trauma recovery). She has already completed stage 1; telling and grieving the story marks stage 2.
- **C. Disorganization and despair** — Owner: Bowlby (phases of mourning). This is Bowlby's phase of mourning after a loss, not a stage of trauma recovery.
- **D. Processing the pain of grief** — Owner: Worden (second task of mourning). This is Worden's task for

mourning a death, not a stage of Herman's trauma recovery model.

- **Pair to learn:** Telling and grieving the trauma story → remembrance and mourning (Herman)

156 — A

Areas of Clinical Focus · Outline 3.U · Chapter 14

- **A. Establishment — Owner: Super (ages 25 to 44).** Super's establishment stage involves stabilizing, consolidating, and advancing.
- **B. Implementation** — Owner: Tiedeman & O'Hara (induction, reformation, integration). Implementation is Tiedeman and O'Hara's phase, not a Super stage.
- **C. Maintenance** — Owner: Super (ages 45 to 64). Maintenance involves holding, updating, and innovating.
- **D. Realistic period** — Owner: Ginzberg et al. (17+). The realistic period is Ginzberg's, with exploration, crystallization, and specification.
- **Pair to learn:** Stabilizing, consolidating, advancing → Super's establishment stage

157 — D

Counseling Skills & Interventions · Outline 5.J · Chapter 2

- **D. Milan systemic therapy — Owner: Selvini Palazzoli, Boscolo, Cecchin, Prata (Milan).** These are the Milan team's guiding principles, along with circular questioning and positive connotation.
- **A. Satir's human validation model** — Owner: Satir (experiential). Satir uses sculpting, communication stances, and work on self-esteem.
- **B. Bowen family systems therapy** — Owner: Bowen. Bowen's terms are differentiation, triangles, and multigenerational transmission.
- **C. MRI brief strategic therapy** — Owner: Watzlawick, Weakland, Jackson, Fisch (MRI). MRI

focuses on attempted solutions that maintain the problem and on second-order change.

- **Pair to learn:** Hypothesizing, circularity, neutrality → Milan (not MRI)

158 — B

Counseling Skills & Interventions · Outline 5.AC · Chapter 4

- **B. "The house feels empty, and you may wonder what your role is now." — Owner: Carkhuff level 4 (additive).** It captures the feeling and adds a meaning just below the surface.
- **A. "Have you thought about turning her room into an office?"** — Owner: Carkhuff level 1 (irrelevant/advice). It ignores the feeling and jumps to problem-solving.
- **C. "The house feels empty since she left, and you really miss her."** — Owner: Carkhuff level 3 (interchangeable). It captures the same content and feeling without adding to them.
- **D. "Kids grow up so fast these days, don't they?"** — Owner: Carkhuff level 2 (subtractive). It generalizes and loses the client's specific feeling.
- **Pair to learn:** Adds meaning just below the surface → Carkhuff level 4

159 — B

Areas of Clinical Focus · Outline 3.AG · Chapter 15

- **B. Spiritual emergency — Owner: Stanislav & Christina Grof.** The Grofs described spiritual emergency: intense spiritual experiences that can resemble psychosis.
- **A. Existential isolation** — Owner: Yalom (ultimate concerns). Existential isolation is loneliness that persists even among people, one of Yalom's ultimate concerns.

- **C. Spiritual bypassing** — Owner: Welwood. Spiritual bypassing is using spiritual ideas to avoid emotional pain, not an overwhelming spiritual experience.
- **D. Terror management** — Owner: Greenberg, Solomon, & Pyszczynski. Terror management concerns death awareness driving worldview defense.
- **Pair to learn:** Intense spiritual experience resembling psychosis → spiritual emergency (Grof)

160 — D

Counseling Skills & Interventions · Outline 5.AC · Chapter 4

- **D. Attend and listen before trying to influence — Owner: Ivey (microskills hierarchy).** Ivey's sequence is attending, then listening, then influencing; you earn the right to influence.
- **A. Share a similar personal story to bond** — Owner: Self-disclosure (influencing skill). Self-disclosure is also an influencing skill and can shift focus to the counselor.
- **B. Give direct advice to seem helpful** — Owner: Advice-giving (not a counseling skill). Advice skips exploration and shifts responsibility to the counselor.
- **C. Offer an interpretation to show expertise** — Owner: Interpretation (influencing skill). Interpretation is an influencing skill and is premature early in counseling.
- **Pair to learn:** Attend → listen → influence → Ivey

161 — B

Counseling Skills & Interventions · Outline 5.AC · Chapter 4

- **B. "So the rent went up, and now you're working weekends." — Owner: Paraphrase (basic listening sequence, Ivey).** It restates the facts in fresh words without naming an emotion or value.

- **A. “Having a stable home really matters to you.”** — Owner: Reflection of meaning. It names the value underneath the statement.
- **C. “You sound stretched thin and anxious about money.”** — Owner: Reflection of feeling. It names an emotion under the content.
- **D. “How many weekend shifts are you working now?”** — Owner: Closed question. It gathers a specific fact instead of restating the message.
- **Pair to learn:** Restates facts → paraphrase (not reflection of feeling)

162 — C

Areas of Clinical Focus · Outline 3.H · Chapter 10

- **C. Agoraphobia — Owner: DSM-5-TR (2 of 5 situations, typically 6 months).** Fear or avoidance of at least 2 of the 5 situations (here public transportation and crowds) because escape or help might be hard defines agoraphobia.
- **A. Specific phobia, situational type** — Owner: DSM-5-TR specific phobia. A specific phobia targets one object or situation, not escape-focused fear across several situations.
- **B. Social anxiety disorder** — Owner: DSM-5-TR (fear of scrutiny or judgment). Social anxiety centers on fear of being judged, which she denies.
- **D. Panic disorder** — Owner: DSM-5-TR (recurrent unexpected panic attacks). Panic disorder requires recurrent unexpected panic attacks, which aren’t described.
- **Pair to learn:** Fear of 2+ situations where escape is hard → agoraphobia

163 — D

Core Counseling Attributes · Outline 6.A · Chapter 22

- **D. Address her self-care and raise it in supervision — Owner: Self-awareness and self-care (6.A).** An exhausted counselor’s presence suffers.

Noticing it, attending to self-care, and using supervision are the counselor's responsibility.

- **A. Refer all of her current clients to colleagues** — Owner: Premature referral. Referring everyone out overreacts before reflection, self-care, and supervision have been tried.
- **B. Tell her clients she is burned out and on edge** — Owner: Self-disclosure that burdens the client. This meets the counselor's needs and shifts the focus and burden onto clients.
- **C. Push through, since her skills are still adequate** — Owner: Ignoring impairment. Her presence is already suffering, which affects clients regardless of her skill level.
- **Pair to learn:** Counselor fatigue affecting presence → Self-care and supervision (6.A)

164 — D

Areas of Clinical Focus · Outline 3.M · Chapter 16

- **D. Emotional loneliness — Owner: Robert Weiss.** Missing a close, intimate attachment figure is Weiss's emotional loneliness.
- **A. Ambiguous loss** — Owner: Pauline Boss. Ambiguous loss is a loss without clarity or closure, not the absence of an intimate partner after divorce.
- **B. Social loneliness** — Owner: Robert Weiss. Social loneliness is missing a broader network, which he has.
- **C. Social isolation** — Owner: Objective lack of social contacts. Isolation is objective, and he has frequent contact with others.
- **Pair to learn:** Missing an intimate attachment figure → emotional loneliness (Weiss)

165 — B

Treatment Planning · Outline 4.E · Chapter 9

- **B. ASAM Criteria — Owner: ASAM Criteria (American Society of Addiction Medicine).** The

ASAM Criteria's six dimensions guide placement at the least restrictive level of care that's safe.

- **A. SBIRT** — Owner: SBIRT (screening, brief intervention, referral to treatment). SBIRT is a public health process for identifying and responding to risky use, not a six-dimension placement framework.
- **C. CRAFFT** — Owner: CRAFFT (adolescent substance use screen). CRAFFT is a brief adolescent screen, not a placement framework.
- **D. Addiction Severity Index** — Owner: ASI (measures addiction severity). The ASI assesses severity; it isn't the level-of-care placement framework.
- **Pair to learn:** Six dimensions for level of care → ASAM Criteria

166 — A

Professional Practice & Ethics · Outline 1.S · Chapter 17

- **A. Tell the client about the team and what is shared — Owner: ACA B.3.b (interdisciplinary teams).** Counselors inform clients of the team's existence and composition and what information will be shared.
- **B. Wait until the client asks about the team** — Owner: Passive disclosure (informed consent is proactive, A.2.a). Clients are informed up front; the counselor doesn't wait to be asked.
- **C. Share the full record so care is coordinated** — Owner: Minimal disclosure ignored (ACA B.2.e; B.3.b teams). Only the information needed should be shared, and the client should know about it first.
- **D. Keep all client information from the team** — Owner: Absolute confidentiality (ignores the B.3 team model). Team care allows appropriate sharing once the client is informed.
- **Pair to learn:** Interdisciplinary team → Inform client of team and what's shared (ACA B.3.b)

167 — D

Counseling Skills & Interventions · Outline 5.AB · Chapter 4

- **D. Interpretation — Owner: Interpretation (influencing skill).** It tentatively proposes a connection to the past that the client hasn't made.
- **A. Confrontation** — Owner: Confrontation (influencing skill). Confrontation points out a discrepancy between words, actions, or nonverbals.
- **B. Reframe** — Owner: Reframe (influencing skill). A reframe offers a new angle on the same facts; it doesn't link the present to the past.
- **C. Reflection of meaning** — Owner: Reflection of meaning (listening skill). Reflection of meaning names the value underneath; it doesn't propose a new connection.
- **Pair to learn:** Proposes a connection outside awareness → interpretation (not reframe)

168 — A

Core Counseling Attributes · Outline 6.E · Chapter 22

- **A. "I'd like to understand what this fast means for you." — Owner: Cultural humility (Hook et al.).** Cultural humility shows curiosity, openness, and awareness of the limits of one's knowledge, inviting the client's own meaning.
- **B. "Let's leave religion aside and focus on your stress."** — Owner: Ignoring culture. Setting the topic aside ignores a part of her identity she chose to bring in.
- **C. "I've read about that fast. It's mainly about discipline."** — Owner: Assuming (cultural encapsulation). Telling the client what her practice means imposes the counselor's assumptions.
- **D. "A counselor who shares your faith might suit you better."** — Owner: Premature referral. Difference

alone isn't a reason to refer; it skips the counselor's responsibility to learn.

- **Pair to learn:** Invites the client's cultural meaning → Cultural humility (not assuming)

169 — D

Intake, Assessment & Diagnosis · Outline 2.F · Chapter 19

- **D. Flat — Owner: Flat affect (virtually no expression).** Flat affect means virtually no emotional expression, the far end of the range of affect.
- **A. Incongruent** — Owner: Incongruent affect (mismatch with mood or content). Incongruent affect doesn't match the client's stated mood or the content of what is said; it describes fit, not the amount of expression.
- **B. Labile** — Owner: Labile affect (rapid shifts). Labile affect shifts rapidly between emotions, the opposite of showing almost none.
- **C. Blunted** — Owner: Blunted affect (significantly reduced expression). Blunted affect is significantly reduced expression, but some expression remains.
- **Pair to learn:** Virtually no expression → Flat affect (not blunted)

170 — B

Intake, Assessment & Diagnosis · Outline 2.E · Chapter 19

- **B. Unspecified depressive disorder — Owner: Unspecified disorder (common in emergency settings).** Unspecified is used when the clinician chooses not to, or can't, specify why full criteria aren't met. It is common in emergency settings.
- **A. Provisional major depressive disorder** — Owner: Provisional diagnosis. Provisional requires a strong presumption that full criteria for a specific disorder will be met; this counselor has no such basis yet.
- **C. A Z code for a problem in living** — Owner: Z codes (not mental disorders). Z codes mark conditions that are a focus of attention but are not mental

disorders; these symptoms are clinically significant and impairing.

- **D. Other specified depressive disorder** — Owner: Other specified disorder. Other specified requires the clinician to state why criteria aren't fully met, which there is no time to do here.
- **Pair to learn:** Can't or won't specify why criteria aren't met → Unspecified (not other specified)

171 — A

Professional Practice & Ethics · Outline 1.B · Chapter 18

- **A. Chi-square — Owner: Chi-square (frequencies in nominal categories).** Completed versus dropped out, by referral source, is a set of frequency counts in categories, which is what chi-square analyzes.
- **B. Kruskal-Wallis** — Owner: Kruskal-Wallis (nonparametric alternative to one-way ANOVA). Kruskal-Wallis compares ranked scores across groups; it isn't built for frequency counts in categories.
- **C. Pearson r** — Owner: Pearson r (relationship between two interval/ratio variables). Pearson r measures a relationship between two continuous variables, not frequencies in categories.
- **D. One-way ANOVA** — Owner: One-way ANOVA (three or more group means, one IV). ANOVA compares group means on an interval or ratio score; these data are counts, not means.
- **Pair to learn:** Nominal frequencies in categories → Chi-square

172 — D

Areas of Clinical Focus · Outline 3.AC · Chapter 10

- **D. Stanley Rachman — Owner: Rachman (cognitive theory of OCD).** Rachman's cognitive theory locates the problem in how intrusive thoughts are interpreted, not in having them.
- **A. Paul Salkovskis** — Owner: Salkovskis (inflated responsibility). Salkovskis emphasized an inflated

sense of responsibility for preventing harm as the driver of neutralizing and compulsions.

- **B. Adrian Wells** — Owner: Wells (metacognitive model of GAD). Wells' metacognitive model concerns beliefs about worry in GAD.
- **C. Edna Foa** — Owner: Foa (exposure and response prevention). Foa developed and studied ERP, a treatment, rather than this cognitive account of how OCD develops.
- **Pair to learn:** Intrusions interpreted as meaningful or dangerous → Rachman (not Salkovskis)

173 — C

Counseling Skills & Interventions · Outline 5.J · Chapter 7

- **C. Multifinality — Owner: Systems concept (multifinality).** The same starting point leading to different outcomes is multifinality.
- **A. Circular causality** — Owner: Systems concept (circular causality). Circular causality is behaviors causing each other in loops.
- **B. Homeostasis** — Owner: Don Jackson (homeostasis). Homeostasis is the family's push to restore its usual balance.
- **D. Equifinality** — Owner: Systems concept (equifinality). Equifinality is different starting points leading to the same outcome.
- **Pair to learn:** Same start, different outcomes → multifinality (not equifinality)

174 — B

Core Counseling Attributes · Outline 6.C · Chapter 22

- **B. "Starting late throws me a bit. What's getting in the way?" — Owner: Congruence expressed through immediacy.** Congruence means not pretending, and sharing a reaction when it serves the client; immediacy turns it into an invitation to explore.

- **A. “Honestly, I’m really annoyed with you right now.”** — Owner: Venting (not congruence). Congruence doesn’t mean saying everything you feel; this unloads the counselor’s feeling without serving the client.
- **C. “No problem at all! I’m just so glad you made it.”** — Owner: Incongruence (pretending). Claiming to feel fine while frustrated is a mismatch between inner experience and outward expression.
- **D. “Being late again breaks the agreement we made together.”** — Owner: Defending the rules. Leading with the rule defends the counselor and is likely to escalate rather than explore what’s happening.
- **Pair to learn:** Real reaction shared for the client’s benefit → Congruence (not venting or pretending)

175 — D

Core Counseling Attributes · Outline 6.I · Chapter 22

- **D. Instillation of hope — Owner: Yalom: instillation of hope.** Seeing someone further along improve gives newer members hope that change is possible; leaders foster this factor deliberately.
- **A. Universality** — Owner: Yalom: universality. Universality is the relief of learning one isn’t alone in a problem; the focus here is evidence of improvement.
- **B. Altruism** — Owner: Yalom: altruism. Altruism benefits the member who helps others, here the speaker, not the newer members listening.
- **C. Catharsis** — Owner: Yalom: catharsis. Catharsis is emotional release, not hope drawn from another member’s progress.
- **Pair to learn:** Seeing others improve → Instillation of hope (Yalom), not universality

176 — A

Intake, Assessment & Diagnosis · Outline 2.S · Chapter 6

- **A. The Danger Assessment — Owner: Jacquelyn Campbell.** Campbell's Danger Assessment is the lethality risk instrument for intimate partner violence.
- **B. The Power and Control Wheel** — Owner: Domestic Abuse Intervention Programs (Duluth). The Power and Control Wheel describes abusive tactics; it is a model, not a lethality instrument.
- **C. The Beck Hopelessness Scale** — Owner: Aaron Beck. Beck's scale measures hopelessness, a suicide risk factor, not danger from a partner.
- **D. The C-SSRS** — Owner: Posner and colleagues (Columbia–Suicide Severity Rating Scale). The C-SSRS rates suicide severity, not lethality risk from a violent partner.
- **Pair to learn:** IPV lethality instrument → Danger Assessment (Campbell), not C-SSRS (Posner)

177 — A

Treatment Planning · Outline 4.J · Chapter 21

- **A. Discharge summary — Owner: Discharge summary (4.J).** The discharge summary documents why services began, what was done, how the client progressed, and what comes next.
- **B. Psychotherapy notes** — Owner: HIPAA psychotherapy notes (separate process notes). Psychotherapy notes hold the counselor's analysis of sessions, kept separate from the record.
- **C. Treatment plan** — Owner: Treatment plan (goals, objectives, interventions). A treatment plan guides future care; it isn't the end-of-services record.
- **D. BIRP note** — Owner: BIRP progress note (behavior, intervention, response, plan). A BIRP note documents a single session, not the whole course of treatment.
- **Pair to learn:** Reason, treatment, progress, status, recommendations → Discharge summary

178 — B

Treatment Planning · Outline 4.A · Chapter 21

- **B. Involve his parents while honoring his goals — Owner: Collaborative goal-setting with minors (4.A).** With minors, counselors involve parents while respecting the young client's voice.
- **A. Adopt the parents' goal, since they consented** — Owner: Parental consent treated as goal ownership. Consent doesn't make the parents the only voice in goal-setting; ignoring his goal weakens collaboration.
- **C. Adopt only his goal and leave the parents out** — Owner: Excluding parents (ignores their legal rights, ACA B.5.b). Parents have legal rights the counselor respects, even while protecting the minor's voice.
- **D. Set the goals herself as the expert** — Owner: Counselor-set goals (conflict with autonomy). Goals the counselor sets alone are less likely to be met and conflict with autonomy.
- **Pair to learn:** Minor and parents disagree on goals → Involve parents, respect the youth's voice (4.A)

179 — A

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **A. Donald Meichenbaum — Owner: Meichenbaum (cognitive behavior modification).** Stress inoculation and self-instructional training are Meichenbaum's methods for changing self-talk and building coping skills.
- **B. Marsha Linehan** — Owner: Linehan (DBT). Linehan teaches skills modules such as distress tolerance and emotion regulation.
- **C. Aaron Beck** — Owner: Beck (CT). Beck uses Socratic questioning, thought records, and behavioral experiments.

- **D. Joseph Wolpe** — Owner: Wolpe (systematic desensitization). Wolpe pairs relaxation with an imagined anxiety hierarchy.
- **Pair to learn:** Stress inoculation → Meichenbaum (not Beck)

180 — A

Core Counseling Attributes · Outline 6.L · Chapter 22

- **A. Use his own language for his identity — Owner: Respect for diversity: the client's language for identity (6.L).** Respect and acceptance for diversity includes using the client's name, pronouns, and language for his identity.
- **B. Avoid mentioning deafness to stay neutral** — Owner: Ignoring culture. Steering around the identity he named ignores an important part of who he is.
- **C. Treat his hearing loss as the presenting problem** — Owner: Pathologizing identity (ACA E.5.c concerns). He described a community and identity, not a complaint; framing it as the problem pathologizes it.
- **D. Use the referral's clinical term for accuracy** — Owner: Imposing outside terminology. Choosing the referral's term over the client's own description puts the counselor's frame ahead of his identity.
- **Pair to learn:** Client names his identity → Use the client's own language (6.L)

181 — D

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **D. Self-efficacy — Owner: Bandura (social cognitive).** Modeling, rehearsal, and mastery experiences build Bandura's self-efficacy.
- **A. Unconditional self-acceptance** — Owner: Ellis (REBT). Ellis targets irrational beliefs and self-acceptance, not confidence from modeling and rehearsal.

- **B. Social interest** — Owner: Adler (individual psychology). Social interest is Adler's sense of community feeling, not belief in one's ability to perform a task.
- **C. Reciprocal inhibition** — Owner: Wolpe (systematic desensitization). Reciprocal inhibition pairs relaxation with anxiety cues; no relaxation hierarchy is used here.
- **Pair to learn:** Modeling + rehearsal → self-efficacy (Bandura)

182 — A

Counseling Skills & Interventions · Outline 5.G · Chapter 9

- **A. Harm reduction — Owner: Harm reduction (public health approach).** Harm reduction lowers the harms of use whether or not the person is ready to stop.
- **B. 12-step facilitation** — Owner: 12-step facilitation. 12-step facilitation encourages AA/NA involvement.
- **C. The disease model** — Owner: Jellinek; AA (disease model). The disease model frames addiction as a chronic, progressive disease.
- **D. The moral model** — Owner: Moral model (historical, rejected). The moral model sees addiction as a personal failing.
- **Pair to learn:** Reduce harms without requiring abstinence → harm reduction

183 — C

Areas of Clinical Focus · Outline 3.B · Chapter 12

- **C. Lars Tornstam — Owner: Tornstam (gerotranscendence).** Tornstam proposed gerotranscendence, in which late life brings a more cosmic, less materialistic outlook.
- **A. Robert Atchley** — Owner: Atchley (continuity theory; retirement phases). Atchley owns continuity theory and the phases of retirement, not gerotranscendence.

- **B. Robert Butler** — Owner: Butler (coined ageism; life review). Butler coined the term ageism and is linked to life review, not gerotranscendence.
- **D. Laura Carstensen** — Owner: Carstensen (socioemotional selectivity theory). Carstensen described focusing on emotionally meaningful relationships as time horizons shorten, not a cosmic shift in perspective.
- **Pair to learn:** Gerotranscendence → Tornstam (not Carstensen)

184 — B

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **B. A coping question — Owner: de Shazer & Berg (SFBT).** Coping questions are an SFBT technique that draws out the client's existing strengths and resources.
- **A. A unique outcome question** — Owner: White & Epston (narrative). Unique outcomes are times the externalized problem didn't win; no problem is being externalized here.
- **C. A scaling question** — Owner: de Shazer & Berg (SFBT). A scaling question asks for a 0 to 10 rating; this question asks how she copes.
- **D. Developing discrepancy** — Owner: Miller & Rollnick (MI). Developing discrepancy highlights the gap between behavior and values to build motivation to change.
- **Pair to learn:** Coping question → SFBT (de Shazer & Berg)

185 — B

Professional Practice & Ethics · Outline 1.M · Chapter 9

- **B. Its uses, reply times, and no emergency use — Owner: Electronic communication in informed consent (ACA H.2.a; Ch 9).** Clients need to know how texts will and won't be used, how fast to expect replies, and that texting isn't an emergency channel.

- **A. That texting is fine for any clinical topic** — Owner: Clinical detail by unencrypted text (contrary to H.2.b-d). Clinical details shouldn't be discussed by unencrypted text.
- **C. That texts are fully secure and private** — Owner: Guaranteed security (no system is completely secure). Counselors inform clients of the limits of confidentiality with technology rather than promising full security.
- **D. That she should text if she feels suicidal at night** — Owner: Texting as an emergency channel (contrary to H.2.a emergency procedures). Emergency procedures must not depend on a counselor seeing a text.
- **Pair to learn:** Texting between sessions → Consent covers uses, response time, not for emergencies

186 — D

Areas of Clinical Focus · Outline 3.I · Chapter 16

- **D. Explore her money stress, and refer her to a credit counselor — Owner: Scope of practice (refer for specific financial advice).** Counselors address the emotional and relational side of money and refer to financial professionals for specific financial advice.
- **A. Compare the loan offers with her and recommend the best one** — Owner: Specific financial advice (outside counselor scope). Choosing a loan is specific financial advice that belongs to a financial professional.
- **B. Teach only emotion-focused coping, since debt can't change** — Owner: Lazarus & Folkman (mismatched coping). Debt is a changeable stressor, so problem-focused steps belong in the plan too.
- **C. Set the money topic aside until her anxiety symptoms resolve** — Owner: Maslow (basic needs first, ignored). Financial stress is a basic-needs concern to address, not postpone.

- **Pair to learn:** Specific financial advice → refer; counselor addresses the emotional side

187 — A

Intake, Assessment & Diagnosis · Outline 2.M · Chapter 20

- **A. Construct validity — Owner: Construct validity (convergent and discriminant; Campbell & Fiske).** Construct validity shows a test measures the theoretical trait: convergent evidence (correlates with similar measures) plus discriminant evidence (doesn't correlate with unrelated ones).
- **B. Face validity** — Owner: Face validity (not a technical form). Face validity is whether a test looks like it measures the trait; it isn't a technical form of validity and uses no correlations.
- **C. Content validity** — Owner: Content validity (expert review of domain coverage). Content validity asks whether items sample the whole domain, judged by expert review against a blueprint, not by correlations.
- **D. Predictive validity** — Owner: Criterion validity: predictive. Predictive validity correlates scores with a future criterion; nothing here is measured later.
- **Pair to learn:** Correlates with similar, not with unrelated → Construct validity (convergent + discriminant)

188 — A

Counseling Skills & Interventions · Outline 5.E · Chapter 4

- **A. The bond — Owner: Bordin (working alliance: bond).** Distrust or distance from the counselor is a bond problem, repaired through empathy, reliability, and openness about ruptures.
- **B. Agreement on tasks** — Owner: Bordin (working alliance: tasks). The client says the exercises make sense.

- **C. Unconditional positive regard** — Owner: Rogers (core conditions). This is a Rogerian core condition, not one of Bordin's three alliance parts.
- **D. Agreement on goals** — Owner: Bordin (working alliance: goals). The client agrees with what they're working toward.
- **Pair to learn:** Distrust of the counselor → Bordin: bond

189 — A

Treatment Planning · Outline 4.I · Chapter 21

- **A. Prepare both the new member and the group — Owner: Group membership transitions (4.I; Ch 8).** Transitions go best when the new member is prepared and current members know and can talk about the change.
- **B. Let her join without telling the group** — Owner: Unprocessed transition. Surprising the group skips preparation that protects cohesion.
- **C. Address her arrival only if conflict arises** — Owner: Reactive processing (transitions are prepared for, 4.I). Waiting for conflict skips the preparation that helps the group absorb a new member.
- **D. Postpone her entry until the group ends** — Owner: Avoiding the transition (not preparing for it). An ongoing group can take new members with preparation; delaying avoids the task.
- **Pair to learn:** New member joining → Prepare the member and the group (4.I)

190 — D

Treatment Planning · Outline 4.E · Chapter 6

- **D. Stay with her and arrange an immediate hospital evaluation — Owner: High-risk response (Ch 6: matching level of care to risk).** A specific plan, intent, and access to means signal high risk: don't leave her alone, and arrange immediate evaluation, voluntary if possible.

- **A. Increase sessions to twice weekly and involve her partner** — Owner: Moderate-risk response (Ch 6). More frequent sessions fit moderate risk, not a client with plan, intent, and means.
- **B. Have her sign a no-suicide contract before she leaves** — Owner: No-suicide contract (no evidence; false reassurance). No-suicide contracts don't prevent suicide and don't address high risk.
- **C. Write a safety plan together and see her again next week** — Owner: Low-risk response (under-responds to high risk). A safety plan alone with a week's wait doesn't match a specific plan, intent, and means.
- **Pair to learn:** Plan, intent, means → Stay with client; immediate evaluation for hospitalization

191 — C

Intake, Assessment & Diagnosis · Outline 2.D · Chapter 19

- **C. A structured interview such as the SCID-5 — Owner: Structured interview (SCID-5, First et al.).** Structured interviews ask fixed questions in a fixed order, which gives them high reliability and makes them a good fit for research and diagnosis.
- **A. A semi-structured interview with open follow-up** — Owner: Semi-structured interview. Semi-structured interviews use core questions plus flexible follow-up, balancing reliability and flexibility rather than maximizing reliability.
- **B. An unstructured interview that follows the client** — Owner: Unstructured interview. Unstructured interviews build rapport and flexibility but have lower reliability and may miss information.
- **D. The Cultural Formulation Interview** — Owner: DSM-5-TR Cultural Formulation Interview (CFI). The CFI gathers the client's cultural view of the problem; it is not the fixed-order diagnostic interview built for reliability.

- **Pair to learn:** Fixed questions in fixed order → Structured interview (SCID-5), not unstructured

192 — C

Counseling Skills & Interventions · Outline 5.Q · Chapter 6

- **C. "Are you having thoughts of killing yourself?" — Owner: Direct closed question (standard of care).** Asking directly and in plain language is correct and doesn't plant the idea.
- **A. "You're not thinking of hurting yourself, are you?"** — Owner: Leading question with a euphemism. It signals the expected answer, and "hurting yourself" may not be heard as suicide.
- **B. "Why would you want everything to stop?"** — Owner: "Why" question. It invites justification and doesn't ask about suicide.
- **D. "What do you mean by 'everything'?"** — Owner: Clarification. Clarifying is useful, but it doesn't directly assess suicide.
- **Pair to learn:** Ask directly about suicide; avoid euphemisms

193 — C

Professional Practice & Ethics · Outline 1.C · Chapter 17

- **C. Assess her suicide risk directly — Owner: ACA AI Work Group: AI is never the sole basis for risk decisions.** The counselor is responsible for clinical judgment; goodbye letters are preparatory behavior that calls for the counselor's own direct risk assessment, whatever the app says.
- **A. Ask her to sign a no-suicide contract** — Owner: No-suicide contract (no evidence of preventing suicide; Ch 6). No-suicide contracts lack evidence and give false reassurance; they don't replace an assessment.
- **B. Rely on the app's low-risk rating** — Owner: AI as sole basis for a risk decision (contrary to ACA AI Work Group). AI output can't replace the counselor's

judgment, especially when the client reports a clear warning sign.

- **D. Hospitalize her involuntarily at once** — Owner: Over-response without assessment (least restrictive care, Ch 6). Level of care should follow an assessment of risk, which hasn't happened yet.
- **Pair to learn:** AI risk rating vs. clinical warning sign → Counselor's own assessment (AI not sole basis)

194 — B

Areas of Clinical Focus · Outline 3.W · Chapter 10

- **B. Make sure she gets a medical evaluation — Owner: Medical rule-out (e.g., hypothyroidism, anemia).** Medical conditions such as hypothyroidism, anemia, and vitamin deficiencies can cause or mimic depression, so new symptoms call for a recent medical evaluation.
- **A. Begin behavioral activation right away** — Owner: Lewinsohn (behavioral theory). Behavioral activation is well supported, but starting it now skips a possible medical cause.
- **C. Suggest she ask about an antidepressant** — Owner: Prescriber decision (outside counselor scope). Medication decisions belong to prescribers, and a medical cause hasn't been ruled out.
- **D. Diagnose major depression from the PHQ-9** — Owner: Kroenke, Spitzer, & Williams (screening tool). The PHQ-9 is a screen; diagnosing before ruling out a medical cause is premature.
- **Pair to learn:** New depressive symptoms → medical rule-out first

195 — A

Counseling Skills & Interventions · Outline 5.Q · Chapter 6

- **A. Stay with her and arrange a hospital evaluation — Owner: High risk → immediate evaluation, involuntary if she refuses.** A specific plan, intent,

and access to means is high risk: she shouldn't be left alone, and refusal of voluntary care calls for an immediate evaluation for involuntary hospitalization.

- **B. Ask her to sign a no-suicide contract** — Owner: No-suicide contract (no supporting evidence). Contracts lack evidence and give false reassurance.
- **C. Complete a safety plan and see her next week** — Owner: Low/moderate-risk response (under-responding). A safety plan alone under-responds to plan, intent, and means.
- **D. Explore the childhood roots of her hopelessness** — Owner: Exploring history before safety. Exploring history waits until safety is addressed.
- **Pair to learn:** Plan + intent + means → high risk: stay with client, immediate evaluation

196 — D

Counseling Skills & Interventions · Outline 5.K · Chapter 7

- **D. Super-reasonable stance — Owner: Satir (communication stances).** Satir's super-reasonable (computer) stance hides feeling behind detached logic.
- **A. Emotional cutoff** — Owner: Bowen. Emotional cutoff is distancing from family of origin to manage unresolved attachment, not a conversational style.
- **B. Stonewalling** — Owner: Gottman (Four Horsemen). Stonewalling is shutting down and withdrawing; he is still engaging, just without feeling.
- **C. Placater stance** — Owner: Satir (communication stances). A placater gives in to please others; he isn't appeasing her.
- **Pair to learn:** Detached logic, no feeling → super-reasonable (Satir)

197 — A

Counseling Skills & Interventions · Outline 5.G · Chapter 9

- **A. Alateen — Owner: Alateen (mutual support).** Alateen is for teens affected by someone else's drinking.
- **B. SMART Recovery** — Owner: SMART Recovery. SMART Recovery is CBT-based support for people with their own addiction.
- **C. Adult Children of Alcoholics** — Owner: ACA/ACoA. ACoA is for adults who grew up with alcoholic or dysfunctional caregivers.
- **D. Narcotics Anonymous** — Owner: NA (12-step). NA is a 12-step group for people with their own addiction.
- **Pair to learn:** Teen affected by a parent's drinking → Alateen

198 — D

Areas of Clinical Focus · Outline 3.J · Chapter 15

- **D. Caitlin Ryan's Family Acceptance Project — Owner: Ryan (Family Acceptance Project).** Caitlin Ryan's Family Acceptance Project documented the risk of family rejection and the protective effect of acceptance.
- **A. Ilan Meyer's minority stress model** — Owner: Meyer (distal and proximal stressors). Meyer explains how external and internal minority stressors raise risk, not the specific effect of family acceptance.
- **B. Anthony D'Augelli's life-span model** — Owner: D'Augelli (LGB identity processes). D'Augelli described identity development through six interrelated processes.
- **C. Richard Troiden's identity model** — Owner: Troiden (sensitization to commitment). Troiden described stages of gay and lesbian identity, not family acceptance research.

- **Pair to learn:** Family acceptance protects LGBTQ youth → Caitlin Ryan (not Meyer)

199 — C

Treatment Planning · Outline 4.N · Chapter 21

- **C. Use plain language and note the data's limits — Owner: Report writing (ACA E.9.a; 4.N).** Reports should be clear, free of jargon, limited to what's needed, and note limitations of the data.
- **A. Present clinical opinions as established fact** — Owner: Blurring fact and opinion. Good reports distinguish facts from clinical opinions.
- **B. Include every test score and session detail** — Owner: Excess disclosure (ACA B.2.e minimal disclosure). Reports share only what is necessary for the reader's purpose.
- **D. Use technical terms to show expertise** — Owner: Clinical jargon (contrary to clear report writing). Jargon makes a report less useful to its readers.
- **Pair to learn:** Report for another party → Clear, necessary, limits noted (ACA E.9.a)

200 — D

Professional Practice & Ethics · Outline 1.X · Chapter 17

- **D. Consult with and support the colleague — Owner: ACA C.2.g, I.2 (assisting an impaired colleague).** Colleagues who notice signs of impairment consult with and assist the colleague, who may need to limit services until it's safe.
- **A. Warn the colleague's clients about her** — Owner: Improper disclosure (bypasses colleague process, harms confidentiality). Contacting her clients is outside proper process and could cause harm.
- **B. File a complaint with the licensing board** — Owner: Formal report (I.2.b; for substantial harm or failed informal resolution). A formal report fits substantial harm or failed informal efforts; neither applies yet.

- **C. Say nothing, since grief is private** — Owner: Ignoring impairment (contrary to C.2.g). Ignoring signs of impairment leaves clients at risk and the colleague unsupported.
- **Pair to learn:** Colleague showing impairment, no harm yet → Consult with and assist (C.2.g)

Score by Domain

Domain	Items	Your correct	%
Counseling Skills & Interventions	60		
Areas of Clinical Focus	58		
Professional Practice & Ethics	24		
Intake, Assessment & Diagnosis	24		
Treatment Planning	18		
Core Counseling Attributes	16		
Total	**200**		

There's no official NCE passing percentage. Use this table to find your two weakest domains and give them extra study time next week.

Item numbers by domain

- **Counseling Skills & Interventions:** 3, 5, 6, 22, 25, 26, 29, 40, 44, 49, 52, 56, 57, 60, 65, 70, 71, 74, 81, 89, 91, 92, 93, 96, 97, 101, 104, 105, 106, 107, 108, 110, 120, 125, 129, 133, 134, 135, 136, 137, 142, 145, 147, 150, 154, 157, 158, 160, 161, 167, 173, 179, 181, 182, 184, 188, 192, 195, 196, 197
- **Areas of Clinical Focus:** 4, 11, 15, 16, 19, 20, 21, 24, 27, 28, 30, 32, 33, 34, 38, 39, 43, 45, 54, 58, 59, 62, 63, 64, 68, 76, 77, 79, 80, 82, 84, 85, 87, 90, 95, 98, 99, 102, 103, 109, 115, 121, 123, 124, 127, 130, 138, 153, 155, 156, 159, 162, 164, 172, 183, 186, 194, 198
- **Professional Practice & Ethics:** 9, 12, 13, 17, 18, 23, 37, 53, 66, 69, 83, 86, 94, 112, 119, 126, 128, 148, 151, 166, 171, 185, 193, 200

- **Intake, Assessment & Diagnosis:** 1, 2, 8, 36, 41, 48, 73, 75, 78, 100, 111, 113, 114, 117, 122, 140, 144, 146, 149, 169, 170, 176, 187, 191
- **Treatment Planning:** 31, 35, 42, 50, 67, 72, 88, 118, 131, 139, 143, 152, 165, 177, 178, 189, 190, 199
- **Core Counseling Attributes:** 7, 10, 14, 46, 47, 51, 55, 61, 116, 132, 141, 163, 168, 174, 175, 180

Pair Review

Cover the right column. Say the owner out loud. Add every pair you missed to your error log.

#	Pair
1	Compared with oneself → Ipsative (not norm-referenced)
2	Noticed in session without a structured method → Informal observation (not formal)
3	Ending stage → summarize progress and process feelings about ending
4	Confusion, hallucinations, autonomic instability 2 to 4 days after drinking stops → delirium tremens
5	Acknowledge doubt, don’t argue → rolling with resistance (MI)
6	Ego states (Parent, Adult, Child) → Berne, not Freud
7	Demonstrably effective relationship elements, including client feedback → Norcross & Lambert
8	Criteria met for two disorders → Diagnose both, integrated treatment (2.G)
9	Peer consultation → Share only what’s needed, no identifiers (ACA B.7)
10	Staying on the client’s topic → Verbal tracking (Ivey attending)

#	Pair
11	12+ months without criteria except craving → sustained remission
12	Individual change beyond measurement error → Reliable change index (Jacobson & Truax)
13	Client asks to see records → Reasonable access (ACA B.6.e)
14	Client vulnerable or anxious → Rogers' condition 2 (incongruence)
15	What one could learn to do → aptitude (not interest or achievement)
16	Ego-syntonic perfectionism without obsessions → OCPD (not OCD)
17	Intoxicated at intake → Capacity to consent (1.O)
18	Control group tries harder → John Henry effect
19	Feeling in-between, ages 18 to 29 → emerging adulthood (Arnett, not Levinson)
20	Instant love myth → Visher & Visher (not Papernow)
21	Rewriting and rehearsing nightmares → imagery rehearsal therapy
22	Remove something pleasant to decrease behavior → negative punishment (Skinner)
23	Changes for whom an effect holds → Moderator
24	Caregiver burden → Zarit Burden Interview (not MBI)
25	Structured review of one's whole life → life review (Butler), not reminiscence

#	Pair
26	Spiritual practices only when they fit the client and with consent → ASERVIC
27	Career indecision → Career Decision Scale (Osipow, not Crites)
28	CASVE cycle → Peterson, Sampson, & Reardon (not Tiedeman & O'Hara or Williamson)
29	Spitting in the soup; acting as if; push-button → Adler
30	Birth to 3 services → IDEA Part C (not IEP or 504)
31	Past coping and successes → Strengths (4.D)
32	Don't talk, don't trust, don't feel → Claudia Black (not Wegscheider-Cruse)
33	Comfort care alongside curative treatment → palliative care (not hospice)
34	Detached from oneself → depersonalization (not derealization)
35	Client declines medication → Respect refusal, explore reasons and consequences (4.P)
36	Needs exceed what the service offers → Screen for fit and refer (2.L)
37	Client declines a modality change → Right to refuse (ACA A. 2.b)
38	Compulsive sexual behaviour disorder → ICD-11 (not DSM-5-TR)
39	Parent and child reinforce each other's escalation → coercion theory (Patterson)

#	Pair
40	Questioning/interrogating → ask for the statement behind the question
41	0 to 100 rating of current distress → SUDS (not WHODAS 2.0)
42	Work schedule, no childcare → Barriers (4.C)
43	Building a realistic, satisfying routine → reorientation (Atchley)
44	Sudden disruption of norms (economic collapse) → anomic (Durkheim)
45	Adult romantic attachment styles → Hazan & Shaver (not Ainsworth)
46	Valuing the person without approving the act → Unconditional positive regard (not approval)
47	Sensing the client's experience internally → Empathic attunement (not responding)
48	Rare condition → many false positives → Low base rate (not floor effect)
49	"What would be different if you were well?" → Adler (the Question), not the miracle question
50	Ending stirs past losses → Process feelings about termination (4.H)
51	Adds meaning just below the surface → Carkhuff level 4 (not level 3)
52	Polite, tentative, leader-focused → initial stage: provide structure
53	Qualitative parallel to external validity → Transferability (Lincoln & Guba)

#	Pair
54	Spiritual support and benevolent reappraisal → positive religious coping (Pargament)
55	“I know exactly how you feel” → Identification, not empathy
56	Tension → battering → honeymoon → Walker (not Duluth)
57	Psychoeducational group → teach, then let members process and personalize
58	Limiting time in bed to build sleep drive → sleep restriction (Spielman, not Bootzin)
59	Family member engaging a treatment-refusing loved one → CRAFT (Meyers)
60	Empty nest → launching stage: renegotiate the couple dyad
61	Admitting mistakes, no façade → Genuineness (Corey’s effective counselor)
62	Adult feels hopeless → display of inadequacy (Dreikurs)
63	Compassion fatigue → Figley (not Maslach)
64	Trading promises for more time → bargaining (Kübler-Ross)
65	Learning by watching others → imitative behavior (Yalom)
66	Plan for counselor’s death or incapacity → Professional will / records custodian
67	Subjective and objective together in Data → DAP
68	4+ days hypomania + major depression → bipolar II
69	Deaf client → Qualified ASL interpreter (ADA accommodation)

#	Pair
70	Safety plan (Stanley & Brown) over no-suicide contract
71	Telehealth not working for this client → H.4.d: consider face-to-face
72	Stops before goals are met → Premature termination (dropout)
73	z = (score − mean) ÷ SD; T = 50 + 10z → raw 52 (mean 40, SD 8) = T 65
74	Stonewalling → physiological self-soothing (Gottman)
75	Test must cover what was taught → Debra P. v. Turlington (not Griggs)
76	Person-environment mismatch → low congruence (Holland)
77	Compares interests with satisfied workers → Strong (not SDS)
78	What has been learned → Achievement (not aptitude)
79	Power imbalance + repeated victimization + fear → sibling abuse (not rivalry)
80	Wind attacks → khyâl cap (Cambodian)
81	Primary emotion beneath secondary anger → EFT (Johnson), not Gottman
82	Classical acquisition + operant maintenance → Mowrer’s two-factor theory
83	Threat reported by family member → Ewing v. Goldstein
84	Yawning, tearing, goosebumps, dilated pupils → opioid withdrawal
85	Tensing muscles to prevent fainting → applied tension (Öst)

#	Pair
86	Minor's agreement alongside parental consent → Assent
87	Exploring how each partner contributed → understanding the context (Baucom, Snyder, & Gordon)
88	Concern in behavioral terms → Problem statement
89	Culture-specific → emic; universal → etic
90	Overly familiar with strangers after neglect → DSED (not RAD)
91	Monopolizing → cutting off (not blocking)
92	Prescribing the symptom in a family → Haley (not Frankl's paradoxical intention)
93	Actualizing tendency → Rogers (not Maslow's self-actualization)
94	Step 3 → Determine nature and dimensions of the dilemma (Forester-Miller & Davis)
95	Lasting differences that won't be resolved → perpetual problem (Gottman)
96	Social → problem → interaction → goal → task → Haley's first interview
97	Sadness about ending → final stage: process ending, consolidate learning
98	Hopelessness subtype of depression → Abramson, Metalsky, & Alloy
99	Fear of confirming a stereotype lowers performance → stereotype threat (Steele & Aronson)
100	What triggered the current problem → Precipitating factor (4 Ps)

#	Pair
101	Engage every member, respect hierarchy and culture → joining
102	Violence within a pattern of coercive control → intimate terrorism (Johnson)
103	Outbursts 3+/week + chronic irritability for 12 months → DMDD
104	Person using violence → batterer intervention program (not anger management alone)
105	Cognitive defusion; values; committed action → Hayes (ACT)
106	Redirecting feelings to a safer target → displacement
107	Asks because meaning is unclear → clarification (not paraphrase)
108	Advocacy → empower the client first (acting with, then on behalf of)
109	Selective mutism → 1 month, not counting the first month of school
110	Archetypes; collective unconscious; persona → Jung
111	Mean 50, SD 21.06, equal-interval 1 to 99 → Normal curve equivalent (not T-score)
112	Advocating for a specific client → Client consent first (ACA A.7.b)
113	Level of care for youth → CALOCUS/CASII (LOCUS is for adults)
114	Self-report conflicts with collateral → Integrate multiple sources (2.Q)
115	Commitment, control, challenge → hardiness (Kobasa, not Savickas)

#	Pair
116	Accept the person, still address the risk → Nonjudgmental stance (not lecturing or ignoring)
117	Diagnosis would cause harm → Refraining from diagnosis (ACA E.5.d)
118	Stalled progress plus low task agreement → Check and repair the alliance (4.L)
119	Sliding scale, equal access → Justice
120	Co-leader disagreement → meet before and after sessions to process
121	Positive change after struggling with trauma → posttraumatic growth (Tedeschi & Calhoun, not Bonanno)
122	Who speaks, who's silent, alliances → Interactional dynamics (2.R)
123	Responsive desire, circular model → Basson (not Kaplan)
124	Adaptive functioning → Vineland (not WISC)
125	Listening steps then acting steps → James & Gilliland (not Roberts)
126	Former romantic partner seeks counseling → Prohibited (ACA A. 5.b)
127	Provisional choices from interests and abilities, ages 11 to 17 → Ginzberg's tentative period
128	Professional online presence → Separate accounts (ACA H.6.a)
129	Contact-boundary disturbances → Perls (Gestalt)
130	Keeping lifelong patterns and identity → continuity theory (Atchley)

#	Pair
131	Client decision about frequency → Use assessment results together (4.K)
132	Staying present with escalating conflict → Conflict tolerance; model respectful conflict (6.F)
133	Cocoanut Grove fire; acute grief → Lindemann (not Caplan)
134	Leader names a shared theme for the group → universalizing (not linking)
135	Imposing one's own culture as reality → cultural encapsulation (Wrenn)
136	Intense imagined exposure → implosive therapy (Stampfl), not flooding
137	Five basic needs; quality world → Glasser
138	Prolonged grief disorder in youth → 6 months (adults 12)
139	Concurrent medication referral → Options, release, follow-up (4.F)
140	Outdated tests or results for current decisions → ACA E.11 (not E.10)
141	Counselor's gender-role bias steering clients → Examine own assumptions (6.D)
142	Face the obsession, block the compulsion → ERP
143	Substance use undermining therapy → Problems that block other work first (4.G)
144	SEM = SD × √(1 − r); higher reliability → smaller SEM
145	Current picture → preferred picture → way forward → Egan (not Hill)

#	Pair
146	Completing sentence stems → Rotter Incomplete Sentences Blank (not TAT)
147	Inviting a quiet member in gently → drawing out
148	Separate process notes with extra protection → HIPAA psychotherapy notes
149	Fixed false belief despite evidence → Delusion (not obsession)
150	Counselor's own life → self-disclosure; reaction to client now → immediacy
151	Agency client to counselor's private practice → Self-referral rules (ACA A.10.a)
152	Individualized goal ratings → Goal Attainment Scaling (Kiresuk & Sherman)
153	Five faces of oppression → Iris Marion Young (not Freire)
154	Speaker-listener technique → PREP (not Gottman)
155	Telling and grieving the trauma story → remembrance and mourning (Herman)
156	Stabilizing, consolidating, advancing → Super's establishment stage
157	Hypothesizing, circularity, neutrality → Milan (not MRI)
158	Adds meaning just below the surface → Carkhuff level 4
159	Intense spiritual experience resembling psychosis → spiritual emergency (Grof)
160	Attend → listen → influence → Ivey
161	Restates facts → paraphrase (not reflection of feeling)

#	Pair
162	Fear of 2+ situations where escape is hard → agoraphobia
163	Counselor fatigue affecting presence → Self-care and supervision (6.A)
164	Missing an intimate attachment figure → emotional loneliness (Weiss)
165	Six dimensions for level of care → ASAM Criteria
166	Interdisciplinary team → Inform client of team and what's shared (ACA B.3.b)
167	Proposes a connection outside awareness → interpretation (not reframe)
168	Invites the client's cultural meaning → Cultural humility (not assuming)
169	Virtually no expression → Flat affect (not blunted)
170	Can't or won't specify why criteria aren't met → Unspecified (not other specified)
171	Nominal frequencies in categories → Chi-square
172	Intrusions interpreted as meaningful or dangerous → Rachman (not Salkovskis)
173	Same start, different outcomes → multifinality (not equifinality)
174	Real reaction shared for the client's benefit → Congruence (not venting or pretending)
175	Seeing others improve → Instillation of hope (Yalom), not universality
176	IPV lethality instrument → Danger Assessment (Campbell), not C-SSRS (Posner)

#	Pair
177	Reason, treatment, progress, status, recommendations → Discharge summary
178	Minor and parents disagree on goals → Involve parents, respect the youth's voice (4.A)
179	Stress inoculation → Meichenbaum (not Beck)
180	Client names his identity → Use the client's own language (6.L)
181	Modeling + rehearsal → self-efficacy (Bandura)
182	Reduce harms without requiring abstinence → harm reduction
183	Gerotranscendence → Tornstam (not Carstensen)
184	Coping question → SFBT (de Shazer & Berg)
185	Texting between sessions → Consent covers uses, response time, not for emergencies
186	Specific financial advice → refer; counselor addresses the emotional side
187	Correlates with similar, not with unrelated → Construct validity (convergent + discriminant)
188	Distrust of the counselor → Bordin: bond
189	New member joining → Prepare the member and the group (4.I)
190	Plan, intent, means → Stay with client; immediate evaluation for hospitalization
191	Fixed questions in fixed order → Structured interview (SCID-5), not unstructured
192	Ask directly about suicide; avoid euphemisms

#	Pair
193	AI risk rating vs. clinical warning sign → Counselor's own assessment (AI not sole basis)
194	New depressive symptoms → medical rule-out first
195	Plan + intent + means → high risk: stay with client, immediate evaluation
196	Detached logic, no feeling → super-reasonable (Satir)
197	Teen affected by a parent's drinking → Alateen
198	Family acceptance protects LGBTQ youth → Caitlin Ryan (not Meyer)
199	Report for another party → Clear, necessary, limits noted (ACA E. 9.a)
200	Colleague showing impairment, no harm yet → Consult with and assist (C.2.g)

NCE Practice Exam 3

200 items · 3 hours 45 minutes · Answer every item.

Take this exam under timed conditions before checking the key. Use the four-step routine on every item: read the last line first, name the owner, predict, then eliminate by owner.

Pacing checkpoints: item 50 by 0:50 · item 100 by 1:40 · item 150 by 2:30 · item 200 by 3:20.

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1. At session four, a client's Session Rating Scale shows a sharp drop in her alliance rating, though she says, "Everything's fine." The counselor's best response is to:

A. Offer to refer her to a colleague she might prefer
B. Mention the drop and ask what hasn't felt right to her
C. Accept her statement and continue with the planned work
D. Stop using the scale, since it seems to upset her

2. A school counselor wants a substance use screen for a 15-year-old who was caught with alcohol at a dance. Which screen is recommended for adolescents?

A. The PCL-5
B. The CRAFFT
C. The OQ-45
D. The CBCL

3. A client's treatment goal is to reduce panic attacks. After a weekend workshop, her counselor is eager to try a new sand tray technique with her. The best course is to:

A. Choose interventions tied to her panic goal
B. Switch approaches now and explain the change later

C. Try the technique and see how she responds to it
D. Help her set a new goal that suits the technique

4. A study of 6,000 college students finds that a stress workshop lowered scores on a 50-point stress scale by less than one point ($p < .001$). The best interpretation is that the result:

A. Shows a large effect because the p-value is so small
B. Proves the null hypothesis true for this population
C. Likely reflects a Type II error caused by too little power
D. Is statistically but perhaps not clinically significant

5. After months of racial slurs from a neighbor, a Black client has nightmares, startles easily, and avoids his own street. Which concept most directly frames racism as the source of these trauma-like reactions?

A. Stereotype threat
B. Internalized racism
C. Race-based traumatic stress
D. John Henryism

6. A child scores 88 on the WISC-V. The score's SEM is 3. Within what range does the child's true score fall about 95% of the time?

A. 73 to 103
B. 85 to 91
C. 82 to 94
D. 79 to 97

7. Differentiation, practicing, rapprochement, and object constancy are subphases described by which theorist?

A. Erik Erikson
B. Mary D. S. Ainsworth
C. John Bowlby
D. Margaret Mahler

8. A leader comments, "I notice that whenever Leo shares, most of you look at me instead of responding to him." This leader skill is:

A. Universalizing
B. Linking
C. Summarizing
D. Process observation

9. In a skills group, a client role-plays talking to a neighbor whose dog barks all night. She smiles and says, “Must be nice to sleep so soundly while the rest of us listen to your dog.” Her response is best labeled:

A. Passive
B. Aggressive
C. Assertive
D. Passive-aggressive

10. A 9-year-old has had frequent eye blinking, shoulder shrugging, and throat-clearing sounds for about 18 months. Which presentation best fits?

A. ADHD, combined presentation
B. Tourette’s disorder
C. Autism spectrum disorder
D. OCD with tic-related specifier

11. A client who recently lost her job asks whether she could do the counselor’s bookkeeping in exchange for sessions. Under the ACA Code, this barter is acceptable only if:

A. It isn’t exploitative, fits local norms, and is put in writing
B. The value of her work is greater than the session fees
C. The agency allows it and she is told about her other options
D. The counselor proposes the terms as a fair way to help

12. A licensed counselor holds a master’s degree in counseling and a PhD in medieval history. She wants to advertise herself as “Dr.” in her counseling practice. Under the ACA Code, she:

A. May, since any earned doctorate allows the title in advertising
B. May not, since her doctorate isn’t in a counseling-re-

lated field
C. May, if no current clients appear in her advertising
D. May, as long as her NCC certification is listed next to it

13. A male client tears up, then quickly wipes his eyes and says, “Sorry. Men aren’t supposed to cry.” The response that best shows sensitivity to gender issues is:

A. “There’s no need to apologize. Real men cry too, and it’s healthy.”
B. “You learned men shouldn’t cry. What has that rule been like for you?”
C. “Let’s move on to what you could do to solve the problem itself.”
D. “I know exactly how you feel. I was raised with that rule myself.”

14. With five minutes left, a calm client who denies any safety concerns says, “Next I want to get into my childhood, the really painful parts.” The counselor’s best response is to:

A. Affirm her readiness and begin there next session
B. Invite her to start now while she feels ready to talk
C. Ask her why she waited so long to bring this up
D. Change the subject to her plans for the week ahead

15. Since converting to a new faith, a client has felt unsettled and is questioning some spiritual values. She has no other symptoms and functions well. The best DSM-5-TR classification is:

A. Adjustment disorder with depressed mood
B. OCD with religious scrupulosity
C. Moral, religious, or spiritual problem
D. A cultural concept of distress

16. Which theorist is associated with the finding that hopelessness predicts suicide more strongly than depression itself?

A. Kelly Posner
B. Thomas Joiner

C. David Jobes
D. Aaron Beck

17. A young refugee says he no longer feels connected to his parents' culture, yet doesn't feel accepted by or part of the host culture either. This best fits:

A. Reverse culture shock
B. Marginalization
C. Separation
D. Assimilation

18. After years of her husband monitoring her phone, controlling the money, and beating her, a woman struck him with a lamp during an attack. In Johnson's typology, her violence is best classified as:

A. Situational couple violence
B. Violent resistance
C. Intimate terrorism
D. Mutual violent control

19. In a psychotherapy group, two members mention they have started carpooling and talking about the group on the drive home. The leader's best response is to:

A. Invite them to discuss the carpool openly in group
B. Speak with each privately and keep it between them
C. Remove both members to protect the rest of the group
D. Ignore it, since the contact happens outside sessions

20. A client with schizophrenia has stopped eating, sleeps outside in freezing weather, and cannot say how he would get food. He denies suicidal or homicidal thoughts and refuses help. Involuntary hospitalization would most likely rest on:

A. His poor insight into the illness
B. Grave disability
C. Imminent danger to self through suicidal intent
D. Danger to others

21. Parents who used to enforce rules separately, each in their own way, begin meeting weekly to agree on household rules together. Systems theorists would call this:

A. Second-order change
B. Equifinality
C. Negative feedback
D. First-order change

22. Near the end of counseling, a counselor helps a client list early warning signs that depression is returning, coping steps to take, and whom to call. This termination task is:

A. Tapering
B. Relapse prevention
C. Pretermination counseling
D. Booster sessions

23. Retirement, the birth of a child, and entering adolescence can each set off which type of crisis?

A. Ecosystemic
B. Situational
C. Existential
D. Developmental

24. A woman has taken a prescribed benzodiazepine exactly as directed for a year. She now needs more for the same effect and feels shaky when a dose is late. She meets no other criteria. She has:

A. Mild sedative use disorder, with two criteria met
B. Sedative use disorder in early remission
C. No disorder; prescribed tolerance doesn’t count
D. Moderate sedative use disorder

25. A counselor wants to show whether ten sessions of counseling reduced a client’s depressive symptoms. Which plan best measures the outcome?

A. Rely on her own impression of how much the client has improved
B. Give the TAT at the end of treatment and review the

stories
C. Give the PHQ-9 at intake and again at the tenth session
D. Give the PHQ-9 once, at the tenth session, and compare it to norms

26. A counselor wants to see how a family new to town connects with school, church, work, and local agencies. The most useful tool is:

A. Family mapping
B. An ecomap
C. Family sculpting
D. A genogram

27. Two days after a tornado destroys several neighborhoods, a counselor is sent to a shelter. Which approach fits best?

A. Prolonged exposure to process each survivor's memories
B. A required debriefing where each survivor retells the event
C. Psychological first aid for safety and practical needs
D. Insight work on how past losses shape their reactions

28. A counselor wants a diagnostic interview that keeps a set of core questions for reliability but still lets her follow up when a client says something unexpected. Which format fits best?

A. A semi-structured interview
B. A mental status exam
C. A structured interview such as the SCID-5
D. An unstructured, conversational interview

29. Directive counseling built on analysis, synthesis, diagnosis, prognosis, counseling, and follow-up is associated with:

A. E. G. Williamson
B. Tiedeman and O'Hara
C. Peterson, Sampson, and Reardon
D. Frank Parsons

30. In motivational interviewing, asking a client to rate from 0 to 10 how important a change is and how confident she is that she could make it uses:

A. A decisional balance exercise
B. Importance and confidence rulers
C. SUDS ratings for a hierarchy
D. Solution-focused scaling questions

31. A memory test has a mean of 60 and a standard deviation of 12. A client's raw score is 36. What are the client's z-score and the equivalent score on the deviation IQ scale?

A. z = −2; deviation IQ score of 70
B. z = −2.4; deviation IQ score of 64
C. z = −2; deviation IQ score of 80
D. z = +2; deviation IQ score of 130

32. In a SOAP note, which entry belongs in the Objective section?

A. Review the safety plan at next session
B. Client says, "I can't get out of bed."
C. PHQ-9 score of 14; affect constricted
D. Progress toward Objective 2 is moderate

33. A client in counseling for anxiety learns that her application for disability benefits was denied, and she doesn't know how to appeal. Along with continuing counseling, the counselor should:

A. Set the issue aside until her anxiety is under control
B. Advise her on the legal steps to file her appeal
C. Refer her to legal or benefits resources, then follow up
D. Call the benefits office to argue her case on her behalf

34. Unconditional self-acceptance is a key term in the approach developed by:

A. Albert Ellis
B. Carl Rogers
C. Steven Hayes
D. Aaron Beck

35. A group of girls repeatedly leaves a less popular classmate out at lunch and spreads rumors about her. This is best classified as:

A. Ordinary peer conflict
B. Relational bullying
C. Verbal bullying
D. Cyberbullying

36. Which brief measures track a client's outcome and the alliance session by session?

A. The OQ-45 (Lambert)
B. The SCL-90-R and the BSI (Derogatis)
C. The ORS and SRS (Miller & Duncan)
D. The BASC-3 and the Conners

37. The APA task forces led by Norcross and Lambert judged several relationship elements demonstrably effective. Which of these is one of them?

A. Confrontation of discrepancies
B. Counselor self-disclosure
C. Interpretation of the client's defenses
D. Positive regard and affirmation

38. A counselor who has seen a 15-year-old individually begins family sessions with the teen and both parents. Which shift does the new modality call for?

A. Promising each member that private disclosures stay private
B. Treating the teen as the problem the family brought in
C. Keeping the focus on the teen's intrapersonal insight
D. Viewing the teen's symptoms as part of a family pattern

39. A client with panic disorder skips her exposure practice and says, "I don't see how doing the scary thing is supposed to help." The counselor's best response is to:

A. Use motivational interviewing to explore and resolve her ambivalence
B. Drop the exposure work, since she doesn't want to do

it
C. Explain the rationale for exposure and how it fits her goals
D. Tell her counseling will end if she doesn't comply

40. A client arrives agitated and suspicious, with dilated pupils, a racing heart, and heavy sweating. Which is most likely?

A. Stimulant intoxication
B. Sedative-hypnotic intoxication
C. Opioid intoxication
D. Cannabis withdrawal

41. A man with alcohol use disorder has met no criteria except craving during 5 months in a locked residential facility. Which specifiers apply?

A. Early remission, on maintenance therapy
B. Early remission, in a controlled environment
C. No specifier until he is discharged
D. Sustained remission, in a controlled environment

42. A member's voice breaks as she talks about her miscarriage. Before she can continue, another member says, "You'll have another baby, I just know it." The leader's best move is to:

A. Link the comment to other members' stories of hope
B. Move to another member to ease the tension
C. Thank the other member for being so supportive
D. Gently block the rescue and give her room to feel

43. A parent asks what decides whether her son's intellectual developmental disorder is rated mild, moderate, severe, or profound. The counselor should explain that severity is based on:

A. His full-scale IQ score by itself
B. How many settings his symptoms appear in
C. His results on a developmental screener
D. His adaptive functioning in daily life

44. During a couple session, a husband's heart is pounding and he can no longer take in what his wife is saying. What should the counselor recommend?

A. Using I-messages to express his anger
B. A break of about 20 minutes before resuming
C. Keep working on it until they reach a solution
D. A softened startup from his wife

45. Jungian sandplay, in which a child builds scenes in sand with miniatures, is most associated with:

A. Margaret Lowenfeld
B. Bernard and Louise Guerney
C. Virginia Axline
D. Dora Kalff

46. A client in a manic episode has not slept in four days, believes she can fly, and has been walking into traffic. Her family cannot keep her safe at home. Which level of care fits?

A. Weekly outpatient counseling with a safety plan
B. Inpatient hospitalization
C. Partial hospitalization
D. An intensive outpatient program

47. As a trauma survivor begins describing the assault, she goes numb, her voice flattens, and she says she feels "far away." Which concept best describes her state?

A. Learned helplessness from uncontrollable events
B. Hyperarousal outside her window of tolerance
C. Exhaustion stage of general adaptation
D. Hypoarousal outside her window of tolerance

48. A model of the working alliance built on agreement about goals, agreement about tasks, and the bond was proposed by:

A. Gerard Egan
B. Carl Rogers
C. Edward Bordin
D. Safran and Muran

49. A client mentions that she skipped her sister's wedding. Which counselor question is least likely to put her on the defensive?

A. "What was going on for you as the day got closer?"
B. "Were you invited? Did you want to go? Who else stayed?"
C. "Don't you think your sister was hurt by that?"
D. "Why did you decide not to go to the wedding?"

50. Which concept belongs to Albert Bandura's social cognitive theory?

A. Reciprocal determinism
B. Reciprocal inhibition
C. Self-instructional training
D. Schedules of reinforcement

51. Six weeks after her husband's death, a client feels sad and misses him, but she sleeps, works, and sees friends. She meets criteria for no disorder and wants support. How is the focus of counseling best documented?

A. Adjustment disorder with depressed mood
B. A Z code for uncomplicated bereavement
C. Provisional major depressive disorder
D. Unspecified depressive disorder

52. A depressed client says, "I'm a failure, people only put up with me, and next year will be just as bad." A cognitive therapist would recognize this as:

A. Awfulizing
B. The cognitive triad
C. Basic mistakes
D. A problem-saturated story

53. With her signed release, a client asks her counselor to coordinate with her primary care physician about her sleep problems. What should the counselor share?

A. Nothing, since physicians aren't part of counseling
B. Her psychotherapy notes, which explain session content

C. Her full counseling record, so the picture is complete
D. Only what the physician needs to help with her sleep

54. The NCE outline's phrase "treatment plan compliance" is increasingly called adherence because adherence:

A. Is the legal term for court-mandated treatment plans
B. Emphasizes collaboration and the client's active role
C. Applies only to taking prescribed psychiatric medication
D. Emphasizes the counselor's authority over the plan

55. A newly divorced mother asks what might help her two school-age children adjust. Which counselor response best provides an education resource?

A. "Here's information on a nearby children's divorce group."
B. "You should keep them out of counseling until things settle."
C. "Why do you think the children are struggling so much?"
D. "Kids are resilient, so they'll bounce back on their own."

56. An 81-year-old declines invitations to new clubs and spends her time with two lifelong friends and her grandchildren. She says, "I don't have time to waste on people who don't matter to me." Which theory best explains this?

A. Activity theory
B. Disengagement theory
C. Socioemotional selectivity theory
D. Selective optimization with compensation

57. CACREP's approval of a university's master's program in clinical mental health counseling is an example of:

A. Accreditation
B. Registry
C. Licensure
D. Certification

58. Two months after retiring, a client has taken three trips and started golf and painting. He says, “Every day feels like Saturday.” According to Atchley, he is most likely in which phase?

A. Disengagement
B. Disenchantment
C. Honeymoon
D. Stability

59. A client who has been severely depressed for months arrives smiling and says, “I finally feel at peace. Everything is settled.” She mentions she recently updated her will. The counselor should first:

A. Explore what helped her feel at peace this week
B. Begin planning for termination given her progress
C. Ask her directly about thoughts of killing herself
D. Note the improvement and space sessions further apart

60. A helping model that moves from exploration to insight to action belongs to:

A. Gerard Egan
B. Allen Ivey
C. Robert Carkhuff
D. Clara Hill

61. Stabilizing daily routines and sleep to help prevent mood episodes in bipolar disorder is the focus of:

A. Interpersonal therapy’s four problem areas
B. Behavioral activation
C. Interpersonal and social rhythm therapy
D. Stimulus control for insomnia

62. A couple reports that every disagreement ends with one partner calling her mother, who then phones the other partner to take sides. A Bowenian counselor would most likely:

A. Map the boundaries between their family’s subsystems
B. Sculpt the family to show who is close to whom

C. Help them step out of the triangle with her mother
D. Prescribe even more calls to the mother as a directive

63. During chemotherapy, a counselor finds that she loses track of what clients said earlier in sessions and is too exhausted to focus. The most ethical response is to:

A. Close her practice and end all services by the end of the week
B. Explain her illness to clients so they can make allowances
C. Seek help and limit or pause her caseload until she can resume
D. Keep her full caseload, since the illness isn't her fault

64. Client: "Since my best friend moved overseas, I keep reaching for my phone to text her and then remember the time difference." Counselor: "Friendships change as people get older." On Carkhuff's scale, this response is best rated:

A. Level 2, because it subtracts from the message
B. Level 5, because it points the client toward action
C. Level 4, because it adds meaning below the surface
D. Level 3, because it is interchangeable

65. The 1998 study linking childhood adversity to later heart disease, substance use, and depression was conducted by:

A. Felitti and Anda
B. Judith Herman
C. George Bonanno
D. Bessel van der Kolk

66. A 7-year-old who recently arrived from Guatemala and speaks mainly Spanish is referred for a special education evaluation. Which approach best follows the legal standard for testing her?

A. Make sure the test covers only what she was taught in school
B. Give the English WISC-V, since it's the most widely used test

C. Place her by ability group using last year's group test scores
D. Test her in Spanish and add nonverbal measures before placement

67. Define the problem, generate alternatives, decide, implement, and evaluate. This problem-solving sequence is credited to:

A. Fisher and Ury
B. Jay Haley
C. James and Gilliland
D. D'Zurilla and Goldfried

68. Which instrument, tied to the Cognitive Information Processing approach, assesses negative career thoughts?

A. Career Maturity Inventory
B. Career Thoughts Inventory
C. Career Decision Scale
D. Career Beliefs Inventory

69. A 2-year-old hospitalized for weeks without his parents first screamed and searched for them, then became listless and sad. Now he barely reacts when his mother visits. Which response is he showing now?

A. Anxious-avoidant attachment
B. Despair
C. Detachment
D. Acceptance

70. Which instrument is a screen for posttraumatic stress that a counselor might use when assessing trauma at intake?

A. The PC-PTSD-5
B. The CRAFFT
C. The MoCA
D. The GAD-7

71. A client hesitantly begins, "There's something I haven't told anyone…" and pauses. The counselor wants

to show attention and encourage her to continue without steering her. The best response is:

A. A soft “Mm-hmm… go on,” with an attentive posture
B. “So far you’ve told me about work stress and poor sleep.”
C. “I wonder if this connects to how your family kept secrets.”
D. “Is it about your marriage?”

72. A counselor assesses a client’s behavior, affect, sensation, imagery, cognition, interpersonal relationships, and drugs/biology. This assessment framework was developed by:

A. William Glasser
B. Arnold Lazarus
C. Donald Meichenbaum
D. Aaron Beck

73. Definitional ceremonies with outsider witnesses are a technique of:

A. Gestalt therapy
B. Solution-focused brief therapy
C. Structural family therapy
D. Narrative therapy

74. In family systems theory, feedback that reduces deviation and keeps the system the same is called:

A. Multifinality
B. Positive feedback
C. Morphogenesis
D. Negative feedback

75. In Papernow’s Stepfamily Cycle, which stage begins the middle phase?

A. Mobilization
B. Contact
C. Awareness
D. Storming

76. A family assessment model that rates families on cohesion, from disengaged to enmeshed, and on flexibility, from rigid to chaotic, was developed by:

A. Salvador Minuchin
B. W. Robert Beavers
C. David Olson
D. Nathan Epstein

77. A 50-year-old client describes his first-ever episode of sudden chest pain, a pounding heart, and fear of dying at work. What should the counselor do first?

A. Diagnose panic disorder
B. Begin interoceptive exposure for his panic
C. Make sure he gets a medical evaluation
D. Teach applied tension

78. Which type of program evaluation compares a program's costs with outcomes measured in nonmonetary units, such as the cost per client who improved?

A. Process evaluation
B. Cost-effectiveness analysis
C. Needs assessment
D. Cost-benefit analysis

79. A client with terminal cancer tells his counselor that he is thinking about ways to hasten his death. Under the ACA Code, the counselor:

A. Must break confidentiality, as she would for any other suicide risk
B. Should end counseling, since the decision is outside its scope
C. Should encourage him to proceed, out of respect for autonomy
D. May keep it confidential after consulting and reviewing the law

80. In a first session, a client in visible distress says she is "totally fine" about her recent breakup, then cries as she describes it. The counselor's best response is to:

A. Point out that her words and her tears don't match
B. Reflect the sadness she is showing as she talks
C. Suggest that the breakup may echo an earlier loss
D. Reassure her that breakups get easier with time

81. A counselor tells a Latino engineer, "It's impressive that someone like you made it this far." In Sue's framework, this remark is a:

A. Microinsult
B. Cultural encapsulation
C. Microinvalidation
D. Microassault

82. A researcher wants to examine how treatment type (CBT or interpersonal therapy) and client gender each affect depression scores, and whether the two interact. The appropriate test is:

A. Factorial ANOVA
B. One-way ANOVA
C. MANOVA
D. ANCOVA

83. Yalom describes here-and-now work as having two tiers. In order, they are:

A. Storming, then norming
B. Illuminating, then experiencing
C. Catharsis, then insight
D. Experiencing, then illuminating

84. When should a group leader first address expectations about members' contact with each other outside the group?

A. Only after a subgroup has already formed
B. During informed consent, before the group begins
C. Never, since members' private lives are their own
D. In the final stage, as members plan to stay in touch

85. A colleague confides that he has begun a sexual relationship with a current client and asks the counselor to keep it to herself. The counselor's best action is to:

A. Report it, since substantial harm to the client is likely
B. Talk with him privately and let him end it on his own terms
C. Contact his client directly to warn her about the risk
D. Keep his confidence, since he told her this as a friend

86. A client who has never been harmed by a snake developed an intense snake fear after one frightening film, yet has no fear of electrical outlets. Which concept best explains why some fears are learned so easily?

A. Two-factor theory
B. Catastrophic misinterpretation
C. Preparedness
D. Yerkes-Dodson law

87. Which statement best describes congruence as Rogers used the term?

A. Openly telling the client everything one feels, moment to moment
B. Valuing the client as a person without any conditions
C. Not pretending, and sharing reactions when that serves the client
D. Sensing the client's experience, including what is unspoken

88. A client says her husband shoves her "only when he's stressed about money," and adds that it shows how much pressure he's under. The counselor's best response is to:

A. Suggest couples sessions to improve their communication
B. Reflect her concern and ask directly about her safety
C. Reframe the shoving as a sign of how much he cares
D. Explore how money was handled in her family growing up

89. In Helms' White racial identity model, which status comes immediately after contact?

A. Reintegration
B. Dissonance

C. Disintegration
D. Encounter

90. A client washes her hands for hours each day and is completely convinced that skipping a wash would kill her children. She has no other unusual beliefs. The best formulation is:

A. Delusional disorder
B. Generalized anxiety disorder
C. OCD with absent insight/delusional beliefs
D. Obsessive-compulsive personality disorder

91. A counselor is considering telling a client's partner that the client has a contagious, life-threatening disease. Under the 2014 ACA Code, before disclosing, the counselor must:

A. Apply the Tarasoff duty to warn standard for violent threats
B. Assess whether the client intends to inform the partner
C. Confirm the diagnosis with the client's physician
D. Obtain a court order authorizing the disclosure

92. At intake, a counselor hands a new client a finished treatment plan with goals she wrote in advance and asks him to sign it. The main problem is that:

A. Goals can't be set until a full ASAM review is done
B. Goals he didn't help choose conflict with his autonomy
C. The plan lists no barriers that could block his progress
D. The plan should first be sent to his insurer for review

93. After a client meets her goals and ends regular counseling, the counselor schedules check-ins at one and three months to help her maintain her gains. These are:

A. Tapered sessions
B. Booster sessions
C. Step-down care
D. Relapse prevention

94. Sensitization, identity confusion, identity assumption, and commitment are stages in the model of:

A. Vivienne Cass
B. Anthony D'Augelli
C. Richard Troiden
D. Arlene Istar Lev

95. Which theorist is most closely associated with multicultural counseling competence?

A. Heinz Kohut
B. Robert Carkhuff
C. Derald Wing Sue
D. Edward Bordin

96. A counselor ended services with a client six years ago. They meet again at a community event and want to start dating. Under the ACA Code, the counselor:

A. Needs only the former client's written consent to proceed
B. Must still use forethought and document it isn't exploitative
C. Is permanently barred from dating this former client
D. Is free to proceed, since well over 2 years have passed

97. Over several sessions, members increasingly blame one member for the group's lack of progress, saying he "drags everyone down." The leader's priority is to:

A. Ask him what he could change to fit in with the group
B. Let members work it out, since conflict is normal in groups
C. Suggest he move to individual counseling instead
D. Block it, protect him, and explore what the group avoids

98. A counselor's HIPAA-compliant AI tool drafts her progress notes. One draft states that the client "denied suicidal ideation," but suicide never came up in that session. The counselor should:

A. Sign it as drafted, since the tool meets HIPAA requirements
B. Sign it now and add a dated correction to the note afterward

C. Fix the draft before signing; the note is her responsibility
D. Ask the client to read and approve the AI draft for accuracy

99. For five sessions a client has been talkative and engaged. Since the counselor suggested a journaling task, she agrees with everything, gives short answers, and changes the subject. The best response is to:

A. Remind her that homework is part of their agreement
B. Continue with the plan, since she has agreed to it
C. Refer her to a counselor who assigns less homework
D. Name the shift gently and ask how the task landed

100. Which of Rogers' six conditions states that the counselor experiences empathic understanding of the client's internal frame of reference and tries to communicate it?

A. The sixth condition
B. The fourth condition
C. The fifth condition
D. The third condition

101. Whenever his wife raises a concern, a husband shoots back, "That's not my fault, you're the one who forgot." In Gottman's terms, the antidote the counselor would coach is:

A. Taking responsibility
B. A gentle startup
C. A culture of appreciation
D. Physiological self-soothing

102. A Cultural Formulation Interview question asks whether anything about the counselor's background might make it harder for the client to get help here. Which CFI domain does this question belong to?

A. Cultural factors affecting current help-seeking
B. Cultural factors affecting self-coping and past help-seeking
C. Cultural definition of the problem
D. Cultural perceptions of cause, context, and support

103. The terms “good-enough mother” and “holding environment” belong to which object relations theorist?

A. Donald Winnicott
B. John Bowlby
C. Margaret Mahler
D. Melanie Klein

104. Which ACA Code standard calls for verifying a distance counseling client’s identity, for example with code words?

A. H.4.c, technology-assisted services
B. H.3, client verification
C. H.6.a, virtual professional presence
D. H.1.b, laws and statutes

105. A counselor receives a subpoena from an attorney in a client’s custody dispute requesting all counseling records. The client hasn’t signed a release. The counselor’s best first step is to:

A. Disregard the subpoena, since the client hasn’t given consent
B. Contact the client and assert privilege on the client’s behalf
C. Send only a brief treatment summary to limit what is disclosed
D. Send the records, since a subpoena is a legal order to comply

106. Research showing that loneliness makes people more alert to social threat, which can make reaching out harder, is most associated with:

A. Julianne Holt-Lunstad
B. Laura Carstensen
C. Robert Weiss
D. John Cacioppo

107. A client in recovery from opioid use looks down and says, “You must think I’m disgusting.” The response that best reflects a nonjudgmental stance is:

A. “You expect me to judge you. I’d rather understand what it’s been like.”
B. “Well, using does hurt the people who care about you.”
C. “I’m so sorry you feel that way. That must be awful.”
D. “Of course not! You’re a wonderful person, and you shouldn’t think that way.”

108. An older client’s daughter comes to the intake. With the client’s consent, the counselor wants the daughter’s view of how the family understands the problem and its cultural meaning. Which tool fits?

A. A 4 Ps formulation of the daughter’s report
B. The CFI informant version
C. The WHODAS 2.0, filled out by the daughter
D. The SCID-5, given to the daughter

109. After failing one exam, a student says, “It’s because I’m stupid, I always will be, and I mess up everything I try.” Which theory most directly explains this as a risk for depression?

A. Reformulated helplessness theory
B. Response styles theory
C. Hopelessness theory
D. Original learned helplessness theory

110. Being authentic and honest, open to feedback, willing to admit mistakes, and having a sense of humor appear on whose list of the personal characteristics of effective counselors?

A. Truax and Carkhuff
B. Jeffrey Hayes and Charles Gelso
C. Norcross and Lambert
D. Gerald Corey

111. Which cluster of physical signs most suggests depression rather than anxiety?

A. Pinpoint pupils, drowsiness, slowed breathing
B. Fatigue, psychomotor slowing, low libido, slowed thinking
C. Racing heart, shortness of breath, trembling, sweating

D. Dilated pupils, rapid heart rate, heavy sweating, paranoia

112. Since his divorce, a client spends hours replaying what went wrong and why he failed. The thoughts don't feel alien to him, and he does nothing to neutralize them. This is best described as:

A. Future-focused worry
B. Intrusive trauma memories
C. Depressive rumination
D. Ego-dystonic obsessions

113. In the first meeting of a counseling group, a member asks whether everything said in the group will stay private. The leader's most accurate response is to explain that:

A. A signed group agreement guarantees that nothing will leave
B. Privileged communication protects what members say in court
C. She will protect it and ask members to, but can't guarantee it
D. Confidentiality doesn't apply to group work, so share carefully

114. A man caring for his father, whose long addiction has left him at home but emotionally unreachable, says he grieves a father who hasn't died. Which concept fits best?

A. Caregiver burden
B. Ambiguous loss
C. Disenfranchised grief
D. Anticipatory grief

115. A couple married three weeks after meeting. They describe intense physical attraction and a firm vow to stay together, but admit they barely know each other. This pattern best fits:

A. Romantic love
B. Anxious adult attachment

C. Eros
D. Fatuous love

116. The Incredible Years parenting program was developed by:

A. Gerald Patterson
B. Dinkmeyer and McKay
C. Matthew Sanders
D. Carolyn Webster-Stratton

117. A family brings in their 12-year-old, Maya, for refusing to go to school. Which question best reflects a circular, systemic focus?

A. “Why do you think Maya keeps refusing to go to school lately?”
B. “When Maya stays home, who notices first? What does Dad do?”
C. “How many school days has Maya missed this month?”
D. “What is it about Maya that makes school so hard?”

118. A 9-year-old whose parents divorced last year keeps planning ways to get them to the same restaurant “so they’ll fall in love again.” This is best understood as:

A. Self-blame and regression typical of preschoolers
B. Bargaining, as in Kübler-Ross’s stages
C. Wallerstein’s sleeper effect
D. A reconciliation fantasy common at school age

119. Emotional vulnerability combined with an invalidating environment produces pervasive emotion dysregulation, according to:

A. Mowrer’s two-factor theory
B. Siegel’s window of tolerance
C. Gross’s process model
D. Linehan’s biosocial theory

120. A refugee family arrives for intake. The parents speak little English, and their 13-year-old daughter offers to interpret. The counselor’s best action is to:

A. Arrange a trained professional interpreter
B. Ask a bilingual receptionist to help out
C. Proceed with simple English and gestures
D. Let the daughter interpret, since she knows the family

121. A veteran says he can't forgive himself for an order he carried out overseas that went against everything he believed was right. This is best described as:

A. Traumatic brain injury
B. Military sexual trauma
C. Disenfranchised grief
D. Moral injury

122. Which addiction treatment approach gives clients tangible rewards, such as vouchers, for verified abstinence?

A. Contingency management
B. CRAFT
C. Community Reinforcement Approach
D. 12-step facilitation

123. In an inpatient setting, when a client has more than one DSM-5-TR diagnosis, what is the condition chiefly responsible for the admission called?

A. The Axis I diagnosis
B. The provisional diagnosis
C. The diagnostic specifier
D. The principal diagnosis

124. The term psychache, meaning intolerable psychological pain, was introduced by:

A. Émile Durkheim
B. Edwin Shneidman
C. Aaron Beck
D. Thomas Joiner

125. A client scored 30 on a symptom measure at intake and 28 after eight sessions. The measure's SEM is 3. What is the most accurate interpretation?

A. The change is within one SEM, so it may reflect measurement error
B. The measure's test-retest reliability must be near zero
C. A ceiling effect on the measure is hiding the client's real change
D. The two-point drop confirms that counseling worked

126. A client wants help being more assertive at work. She mentions that her partner choked her last weekend and that she is afraid to go home tonight. The first treatment priority is:

A. Exploring how her family history shapes her choices
B. Inviting her partner to join couples sessions
C. Assertiveness training for her concerns at work
D. Assessing lethality and safety planning with her

127. At intake, a counselor wants to evaluate a client's level of mental health functioning. Which question does this most directly?

A. "How are these symptoms affecting your work and daily life?"
B. "Can you tell me today's date and where we are right now?"
C. "How would you describe this problem to your family and friends?"
D. "From 0 to 100, how upsetting does this feel right now?"

128. A client with major depressive disorder has the long-term goal of returning to full functioning. Which entry is a short-term objective?

A. Client reports low mood and misses work two days a week
B. Client will experience reduced depression overall
C. Behavioral activation and cognitive restructuring weekly
D. Client will score below 10 on the PHQ-9 within 12 weeks

129. A manager assumes that an employee who uses a wheelchair must also struggle with complex projects and leading meetings. This error is best described as:

A. Cultural imperialism
B. Impostor phenomenon
C. The spread effect
D. Stereotype threat

130. A client from a family that makes major decisions together says she wants her parents involved in shaping her counseling goals. The counselor's best approach is to:

A. Encourage her to set her own goals independently first
B. Let her parents set her goals for her instead
C. Respect that value and explore how to involve them
D. List her family's role as a barrier in the plan

131. A client is being discharged after a week of inpatient care following a suicide attempt and will continue in outpatient counseling. The counselor should prioritize:

A. Completing the discharge summary before any contact
B. A first outpatient appointment sometime within the next month
C. A follow-up appointment within days, with safety planning
D. Waiting for the client to call once she feels ready

132. A counselor asks, "As you talk about your father right now, what are you noticing in your chest and hands?" The counselor makes no reference to the counseling relationship. This response is best described as:

A. Immediacy
B. Here-and-now awareness
C. An interpretation
D. A reflection of meaning

133. A laid-off engineer received a generous severance, so money isn't a worry. Still, he says his days feel shapeless and he misses having somewhere to be and people to talk to. Which model best explains his distress?

A. Holland’s congruence
B. Maslach’s burnout model
C. Jahoda’s latent deprivation model
D. Atchley’s retirement phases

134. For 8 months, a 10-year-old has often lost his temper, argued with his mother and teachers, refused rules, and deliberately annoyed others. He hasn’t hurt anyone, destroyed property, or stolen. This best fits:

A. Disruptive mood dysregulation disorder
B. Conduct disorder
C. Oppositional defiant disorder
D. Intermittent explosive disorder

135. A supervisor is held legally responsible for harm caused by her supervisee’s actions, even though she was not negligent herself. This legal doctrine is:

A. Direct liability
B. Dereliction of duty
C. Vicarious liability
D. Direct causation

136. Family reconstruction, a method for exploring family-of-origin patterns, is associated with:

A. Murray Bowen
B. Virginia Satir
C. Carl Whitaker
D. Salvador Minuchin

137. An adult survivor of childhood sexual abuse says, “No matter what I said or did, it kept happening.” Finkelhor and Browne would link this to which traumagenic dynamic?

A. Stigmatization
B. Powerlessness
C. Betrayal
D. Shattered assumptions

138. A client says, “My car broke down again, and I don’t know how I’ll get to work this week.” Which response is interchangeable, at about level 3 on Carkhuff’s scale?

A. “With your car out again, you’re really worried about getting to work.”
B. “You’re worried about getting to work, and maybe tired of always being one breakdown from falling behind.”
C. “Cars always seem to break down at the worst time.”
D. “Have you looked into a bus pass or asking a coworker for a ride?”

139. A client has repeated panic-like episodes, but each involves fewer symptoms than the criteria require. The counselor wants the record to state that reason. Which designation fits?

A. Panic disorder (provisional)
B. Unspecified anxiety disorder
C. Malingering, recorded as a Z code
D. Other specified anxiety disorder

140. A counseling group leader introduces a role-play exercise. One member says she doesn’t want to take part. The leader should:

A. Require her to join so the exercise works as planned
B. Drop structured exercises for the rest of the group
C. Respect her choice; she may observe and share reactions
D. Ask her to wait outside the room while the others role-play

141. A 48-year-old accountant laid off after 20 years is again exploring interests, taking classes, and trying new fields, much as he did at 20. This best illustrates:

A. Updating in the maintenance stage
B. Chronic career indecisiveness
C. Disenchantment after a role ends
D. Recycling into the exploration stage

142. Before a first session, a counselor reads about her new client’s ethnic group and starts planning interven-

tions based on what she read. The best use of that reading is to:

A. Build the treatment plan around the values typical of that group
B. Treat it as a hypothesis and ask the client how it fits her
C. Refer the client to a counselor who shares her background
D. Set it aside and avoid raising culture so she doesn't seem biased

143. After session five, a member stays behind and tells the leader privately that she feels the leader cuts her off more than others. The leader's best response is to:

A. Listen non-defensively and invite her to raise it in group
B. Explain the reasons for each interruption so she understands
C. Agree to keep the complaint between the two of them
D. Promise her extra time next session to make up for it

144. The modern hospice movement is credited to:

A. Dame Cicely Saunders
B. Elisabeth Kübler-Ross
C. Robert Butler
D. Erich Lindemann

145. After two months of weekly outpatient sessions, a client's depression has worsened. She misses most work and is struggling to care for herself, but she has no suicidal intent. The most appropriate option to discuss with her is:

A. Stepping up to intensive outpatient or partial hospitalization
B. Reducing sessions to monthly to build her independence
C. Continuing weekly sessions under the current treatment plan

D. Inpatient hospitalization, given how poorly she is functioning

146. A 24-year-old client says she scratches her arms until they bleed when she feels overwhelmed, and that she has no wish to die. The counselor's best response is to:

A. Make continued counseling conditional on her stopping now
B. Arrange an inpatient admission today to keep her safe
C. Skip suicide assessment, since she has no wish to die
D. Assess suicide and medical risk, then build coping skills

147. A client makes a credible threat to harm his counselor after she declines to write a letter he requested. Under the ACA Code, the counselor:

A. May end services, with safety steps and referrals
B. Must continue, since ending would be abandonment
C. May end services only once his balance is paid
D. Must warn his family under the Tarasoff duty to protect

148. A client says calmly, "I think you've got my mother all wrong. She's not the problem." The response that best shows conflict tolerance is:

A. "I may have missed something. Help me see her the way you do."
B. "It sounds like you're in denial about your relationship with her."
C. "I've been doing this a long time, and the pattern is clear."
D. "You're right, I'm so sorry. I won't bring her up again."

149. Six weeks into a plan to cut back on gambling, a client who had been making steady changes says, "I'm not sure I even want to stop anymore." The counselor's best step is to:

A. Reassess his readiness and revise the plan with him
B. Switch to a new approach without discussing it
C. Keep the current plan and remind him of his original

goal
D. Confront his denial so that he will recommit

150. Which progress note format records the client's presentation, what the counselor did, how the client reacted, and next steps?

A. DAP
B. GIRP
C. SOAP
D. BIRP

151. In a working-stage session, a member tells the leader, "When Ray talks over me, I feel dismissed." The leader's best response is:

A. "Ray, please let her finish from now on."
B. "It sounds like you feel dismissed by him."
C. "Has anyone else here felt dismissed?"
D. "Could you say that directly to Ray?"

152. The idea that awareness of death drives people to cling to cultural worldviews and self-esteem as buffers is called:

A. Neurotic anxiety
B. Terror management theory
C. Spiritual bypassing
D. Viktor Frankl's will to meaning

153. Holland's construct of vocational identity is measured by:

A. Self-Directed Search
B. My Vocational Situation
C. Career Maturity Inventory
D. Minnesota Importance Questionnaire

154. Recognize, react, recollect and reexperience, relinquish, readjust, and reinvest are grief processes described by:

A. Therese Rando
B. J. William Worden

C. Robert Neimeyer
D. John Bowlby

155. In Stanley and Brown's Safety Planning Intervention, what does the client identify first?

A. Professionals and agencies to contact for help
B. Internal coping strategies to use when alone
C. Warning signs that a crisis may be starting
D. Ways to make the environment safer

156. The view that a well-handled alliance rupture often strengthens the counseling relationship comes from:

A. Robert Carkhuff
B. Edward Bordin
C. Gerard Egan
D. Safran and Muran

157. A terminally ill client tells her counselor she is considering hastening her own death, which may be legal in their state. Which action best follows ACA B.2.b?

A. Break confidentiality now, as with any suicidal client
B. Redirect her to life review and legacy work
C. Encourage her to proceed, since it is legal
D. Seek consultation, then weigh keeping it confidential

158. In a quiet office, a client says the hum of the air vent briefly sounded like someone whispering her name. When she listened closely, she could tell it was only the vent. In the MSE, this is recorded as:

A. Thought blocking
B. An auditory hallucination
C. A delusion
D. An illusion

159. A capable student says, "I know I could finish a nursing degree, but I don't believe it would lead to a good job for someone like me." Which construct is most directly involved?

A. Outcome expectations
B. Task-approach skills

C. Self-efficacy
D. Satisfactoriness

160. In Cass's model of homosexual identity formation, which stage comes immediately after identity comparison?

A. Moratorium
B. Immersion-emersion
C. Identity tolerance
D. Identity acceptance

161. A counselor's reflections accurately repeat each client statement, yet the client says, "You sound like you're reading from a script." Which element is most likely missing?

A. Verbal tracking
B. Empathic attunement
C. Unconditional positive regard
D. Psychological contact

162. A client who has drunk heavily every day for years says, "I'm quitting cold turkey tomorrow, on my own." The counselor's most important response is to:

A. Arrange medical evaluation; withdrawal could be life-threatening
B. Encourage him to attend AA meetings every day during the first week
C. Explore his ambivalence with a decisional balance exercise
D. Affirm his decision and schedule a relapse prevention session

163. A parent asks whether her son's excessive online gaming counts as an official diagnosis. Which statement is accurate?

A. ICD-11 lists it as an impulse control disorder
B. Neither the DSM-5-TR nor ICD-11 addresses it
C. DSM-5-TR lists it for further study; ICD-11 lists it
D. It is an official DSM-5-TR addictive disorder

164. A counselor notices she now refers to clients as "the 2 o'clock" and feels cynical and detached from them. This best reflects:

A. Depersonalization as a dissociative symptom
B. The depersonalization dimension of burnout
C. Reduced personal accomplishment
D. Vicarious trauma

165. The 1974 Tarasoff decision, often called Tarasoff I, held that therapists have a duty to:

A. Act for a foreseeable victim based on violent history
B. Act on a threat reported by a client's family member
C. Warn an identifiable victim of a serious threat
D. Take reasonable steps to protect an intended victim

166. A client describes a harsh inner voice that demands perfection and another part of her that whines and makes excuses to avoid the work. A Gestalt counselor would most likely frame this split as:

A. Persona and shadow
B. Parent and Child ego states
C. Superego and id
D. Top dog and underdog

167. When an ACA ethical standard conflicts with a law, the ACA Code directs counselors first to:

A. Follow the Code, since ethics always outrank the law
B. State their commitment to the Code and try to resolve it
C. Apply Stadler's universality test to settle the conflict
D. Follow the law without raising the conflict with anyone

168. A client who uses a wheelchair comes in for depression and says it's about losing his job. The counselor suspects the disability is the real issue. The best approach is to:

A. Tell him how hard life with a disability must be
B. Refer him to a counselor who specializes in disability
C. Start with the client's own account of what's wrong

D. Gently suggest that his disability may be the deeper issue

169. A counselor wants to screen for adversity in a client's childhood, a known risk factor for later substance use. Which tool fits this purpose?

A. The ACEs questionnaire
B. The Child Behavior Checklist
C. The Social Readjustment Rating Scale
D. The Columbia-Suicide Severity Rating Scale

170. A counselor coaches an escalated teen to put words to what he feels ("I'm furious and embarrassed") because labeling emotions helps regulate them. This reflects:

A. Siegel's "name it to tame it"
B. Wells' metacognitive therapy
C. Jacobson's progressive relaxation
D. Gross's response modulation

171. Two business partners keep arguing over whose plan to adopt. The counselor helps them set their plans aside and name what each of them actually needs from the decision. This conflict resolution approach comes from:

A. The speaker-listener technique (PREP)
B. Softened startup (Gottman)
C. Interests, not positions (Fisher & Ury)
D. I-messages and active listening (Gordon)

172. Which federal law strengthened HIPAA enforcement and added breach notification requirements for electronic health records?

A. The ADA
B. HITECH Act
C. 42 CFR Part 2
D. FERPA

173. Before a first session, a counselor wants to look through a new client's public Instagram profile to learn

more about her. Under the ACA Code, the counselor should:

A. View it, since the profile is public
B. Follow it from her professional account
C. View it and note findings at intake
D. Not view it unless the client consents

174. A 40-year-old client whose late father drank heavily throughout her childhood asks about peer support for people with her history. The best-fitting group is:

A. Al-Anon
B. Alateen
C. Adult Children of Alcoholics
D. SMART Recovery

175. In First's differential diagnosis sequence, which question is asked at the final step?

A. Is there clinically significant distress or impairment?
B. Could a substance or medication explain the symptoms?
C. Could a general medical condition account for the symptoms?
D. Are the symptoms being intentionally produced?

176. A client who has described years of resentment toward his older brother now speaks of him with lavish, almost gushing praise. If this is a defense, it is most likely:

A. Rationalization
B. Sublimation
C. Intellectualization
D. Reaction formation

177. A client whose drinking has led to two DUIs and abnormal liver tests says, "I don't have a drinking problem. Everyone else is overreacting." In the MSE, this statement speaks most directly to his:

A. Insight
B. Thought process

C. Orientation
D. Judgment

178. Before acting on a hard ethical decision, a counselor asks herself, “Would I be comfortable seeing this choice reported in the newspaper?” This question is Stadler’s:

A. Justice test
B. Publicity test
C. Universality test
D. Step 6 evaluation

179. When should a counselor first raise the topic of termination with a client?

A. Only when the client brings up wanting to stop
B. Once sessions start tapering to every other week
C. In the final session, once the goals are met
D. In informed consent, revisiting it as goals near

180. A client who takes high daily doses of nonprescribed benzodiazepines announces she will quit “cold turkey” this weekend. The counselor’s best response is to:

A. Explain that withdrawal is unpleasant but safe
B. Refer her for medically managed withdrawal
C. Wait until she is abstinent to treat her anxiety
D. Affirm her plan and check in on Monday

181. A client with generalized anxiety says, “My worrying is out of control, and if I can’t stop it I’ll lose my mind.” Treatment that targets these beliefs about worry follows which model?

A. Clark’s cognitive model of panic
B. Dugas’ intolerance of uncertainty model
C. Borkovec’s avoidance theory of worry
D. Wells’ metacognitive model

182. A client grieving her father asks, “Have you ever lost a parent?” The counselor has. Which response is the most appropriate self-disclosure?

A. “Yes, and honestly, I’m still struggling with it most days.”

B. “Another client of mine lost her dad and found a group helpful.”
C. “Yes. Let me tell you about my father’s illness and how we coped.”
D. “I have, and it was disorienting. What has it been like for you?”

183. In a grief group, the leader asks, “Who else has felt guilty for laughing since the loss?” Several members nod in surprise. Which therapeutic factor is the leader fostering?

A. Catharsis
B. Instillation of hope
C. Universality
D. Altruism

184. After a house fire, a woman suddenly developed delusions and disorganized speech. Two weeks later the symptoms are gone and she is back to her usual functioning. Which presentation best fits?

A. Brief psychotic disorder
B. Schizoaffective disorder
C. Delusional disorder
D. Schizophreniform disorder

185. A counselor promises to hold a client’s Tuesday evening slot while he travels for work. When another client asks for that time, she declines. Which ethical principle does this most directly reflect?

A. Veracity
B. Beneficence
C. Justice
D. Fidelity

186. Midway through a study of a campus stress-reduction program, a hurricane closes the university for two weeks, and stress scores rise in both the treatment and control groups. This threat to internal validity is:

A. Maturation
B. Instrumentation

C. Experimental mortality
D. History

187. While teaching stress management, a counselor explains the relaxation response. This concept is owned by:

A. Joseph Wolpe
B. Herbert Benson
C. Jon Kabat-Zinn
D. Edmund Jacobson

188. Brief Strategic Family Therapy for adolescent substance use was developed by:

A. Robert Meyers
B. Howard Liddle
C. José Szapocznik
D. Scott Henggeler

189. A client takes a benzodiazepine exactly as prescribed by her physician. She has developed tolerance and feels shaky when a dose is late. She meets no other substance use criteria. The counselor should conclude that she:

A. Has a sedative-induced anxiety disorder
B. Has mild sedative use disorder, with two criteria met
C. Does not meet criteria for a sedative use disorder
D. Has sedative use disorder in early remission

190. A parent tells a 9-year-old that he can play outside as soon as he finishes his reading. This arrangement illustrates:

A. The Premack principle
B. A token economy
C. Shaping
D. Negative reinforcement

191. A 12-year-old who once wanted to be a mail carrier now rejects it because “smart people don’t do that kind of job.” This shift best reflects:

A. Orientation to social valuation
B. The growth stage of Super’s model

C. Ginzberg’s tentative period
D. Orientation to sex roles

192. After normalizing a couple’s concerns and giving basic information about arousal, a counselor assigns structured, non-demand touching exercises for home. In the PLISSIT model, this step is:

A. Limited information
B. Specific suggestions
C. Permission
D. Intensive therapy

193. A counselor is clearing out closed client files that are past the retention period her state requires. The appropriate step is to:

A. Dispose of them in a way that protects confidentiality
B. Mail the original files back to each former client
C. Place them in the office’s regular paper recycling bin
D. Give them to the records custodian named in her will

194. For 5 months, a client has had weekly episodes of eating large amounts with a sense of lost control, followed by intense shame. She does not vomit, fast, or overexercise. This best fits:

A. Bulimia nervosa
B. Anorexia nervosa, binge-eating/purging type
C. Avoidant/restrictive food intake disorder
D. Binge-eating disorder

195. A researcher sorts a state’s licensed counselors into urban, suburban, and rural groups, then randomly selects counselors from each group in proportion to its size. This sampling method is:

A. Systematic sampling
B. Stratified sampling
C. Cluster sampling
D. Convenience sampling

196. A counselor reflects on how her professional role and her identity carry power in her relationship with each

client. This awareness belongs most directly to which core attribute?

A. Self-awareness and impact on clients
B. Nonpossessive warmth toward the client
C. Empathic attunement to the client
D. Conflict tolerance and resolution

197. A client who uses a wheelchair says his biggest problem is that his workplace has no ramp and his manager won't move meetings to the ground floor. His view of disability reflects the:

A. Biopsychosocial model
B. Moral model
C. Social model
D. Medical model

198. Adoptive parents are hurt that their 15-year-old has started asking about her birth mother and fear she is rejecting them. The counselor should help them see that:

A. Her interest shows she was told about adoption too early
B. She should wait until 18 to explore birth family
C. Teen identity questions are a normal adoption issue
D. Her questions suggest reactive attachment disorder

199. A client says her goal is "to stop being so lonely." Using de Shazer's criteria for well-formed goals, the best rewrite is:

A. Feel more connected to people in general
B. Never feel lonely on a weekend again
C. Understand why she keeps pulling away from others
D. Call or meet up with one friend twice a week

200. A client hasn't paid for four sessions, and the counselor plans to use a collection agency. Under the ACA Code, the counselor must first:

A. Give the agency the client's diagnosis to support the claim
B. End services at once and then file with the agency

C. Have told the client in advance and offer a chance to pay
D. Raise the client’s fee to cover the expected collection costs

NCE Practice Exam 3 — Answer Key with Ownership Map

Every rationale names who or what **owns** each option. Read all four, not just the correct one: each item teaches four pairings.

How to review: for every item you missed *or guessed*, write in your error log (Workbook, Part 4) the owner you picked and the owner of the correct answer.

Quick Key

1. B	2. B	3. A	4. D	5. C	6. C	7. D	8. D	9. D	10. B
11. A	12. B	13. B	14. A	15. C	16. D	17. B	18. B	19. A	20. B
21. A	22. B	23. D	24. C	25. C	26. B	27. C	28. A	29. A	30. B
31. A	32. C	33. C	34. A	35. B	36. C	37. D	38. D	39. C	40. A
41. B	42. D	43. D	44. B	45. D	46. B	47. D	48. C	49. A	50. A
51. B	52. B	53. D	54. B	55. A	56. C	57. A	58. C	59. C	60. D
61. C	62. C	63. C	64. A	65. A	66. D	67. D	68. B	69. C	70. A
71. A	72. B	73. D	74. D	75. A	76. C	77. C	78. B	79. D	80. B
81. A	82. A	83. D	84. B	85. A	86. C	87. C	88. B	89. C	90. C
91. B	92. B	93. B	94. C	95. C	96. B	97. D	98. C	99. D	100. C
101. A	102. A	103. A	104. B	105. B	106. D	107. A	108. B	109. A	110. D
111. B	112. C	113. C	114. B	115. D	116. D	117. B	118. D	119. D	120. A

121. D	122. A	123. D	124. B	125. A	126. D	127. A	128. D	129. C	130. C
131. C	132. B	133. C	134. C	135. C	136. B	137. B	138. A	139. D	140. C
141. D	142. B	143. A	144. A	145. A	146. D	147. A	148. A	149. A	150. D
151. D	152. B	153. B	154. A	155. C	156. D	157. D	158. D	159. A	160. C
161. B	162. A	163. C	164. B	165. C	166. D	167. B	168. C	169. A	170. A
171. C	172. B	173. D	174. C	175. A	176. D	177. A	178. B	179. D	180. B
181. D	182. D	183. C	184. A	185. D	186. D	187. B	188. C	189. C	190. A
191. A	192. B	193. A	194. D	195. B	196. A	197. C	198. C	199. D	200. C

Rationales

1 — B

Intake, Assessment & Diagnosis · Outline 2.U · Chapter 4

- **B. Mention the drop and ask what hasn't felt right to her — Owner: Rupture repair: name and explore (Safran & Muran).** Feedback measures are meant to be used; naming the drop non-defensively and exploring it repairs a possible withdrawal rupture.
- **A. Offer to refer her to a colleague she might prefer** — Owner: Premature referral. Referral abandons the alliance instead of repairing it.
- **C. Accept her statement and continue with the planned work** — Owner: Ignoring a withdrawal rupture. Agreeing too easily can itself signal a withdrawal rupture; pressing on leaves it unrepaired.
- **D. Stop using the scale, since it seems to upset her** — Owner: Discarding client feedback (contrary to Norcross & Lambert). Collecting client feedback was judged demonstrably effective; the useful step is to act on it.

- **Pair to learn:** Feedback shows an alliance drop → Name and explore the rupture (Safran & Muran)

2 — B

Intake, Assessment & Diagnosis · Outline 2.P · Chapter 11

- **B. The CRAFFT — Owner: Adolescent substance use screen.** The CRAFFT is the screen recommended for adolescents.
- **A. The PCL-5** — Owner: PTSD symptom measure. The PCL-5 measures posttraumatic stress symptoms.
- **C. The OQ-45** — Owner: Lambert (Outcome Questionnaire). The OQ-45 tracks counseling progress; it is not a substance screen.
- **D. The CBCL** — Owner: Achenbach (CBCL). The CBCL is a behavior rating checklist, not a substance use screen.
- **Pair to learn:** Adolescent substance use screen → CRAFFT (not CBCL)

3 — A

Counseling Skills & Interventions · Outline 5.D · Chapter 5

- **A. Choose interventions tied to her panic goal — Owner: Implementing the treatment plan (5.D).** Every intervention should trace back to a treatment goal; a valid technique unrelated to the goal is the wrong choice.
- **B. Switch approaches now and explain the change later** — Owner: Changing approach without discussing it with the client. Changes to the approach should be discussed with the client, not explained after the fact.
- **C. Try the technique and see how she responds to it** — Owner: Technique unrelated to stated goals. A technique chosen for the counselor's interest rather than the client's goal breaks the link to the plan.
- **D. Help her set a new goal that suits the technique** — Owner: Fitting the client to the

technique. Goals come from the client's concern; they are not reshaped to fit a technique.

- **Pair to learn:** Every intervention traces back to a treatment goal (5.D)

4 — D

Professional Practice & Ethics · Outline 1.B · Chapter 18

- **D. Is statistically but perhaps not clinically significant — Owner: Statistical vs. clinical significance (Jacobson & Truax).** Very large samples can make tiny differences statistically significant; clinical significance asks whether the change matters in people's lives.
- **A. Shows a large effect because the p-value is so small** — Owner: Effect size (Cohen's d), not the p-value, gauges size. A small p-value shows the result is unlikely under the null, not that the effect is large; a one-point change on a 50-point scale is tiny.
- **B. Proves the null hypothesis true for this population** — Owner: Null hypothesis (never proved; rejected or not rejected). Researchers never prove the null; here the null was actually rejected.
- **C. Likely reflects a Type II error caused by too little power** — Owner: Type II error and power. The null was rejected, so a Type II error (a missed real effect) didn't occur, and 6,000 participants means power is very high.
- **Pair to learn:** Significant p, trivial change → Statistical, not clinical, significance

5 — C

Areas of Clinical Focus · Outline 3.Z · Chapter 15

- **C. Race-based traumatic stress — Owner: Robert T. Carter.** Carter's race-based traumatic stress holds that racism can produce trauma-like reactions.
- **A. Stereotype threat** — Owner: Steele & Aronson. Stereotype threat is lowered performance from fear of confirming a stereotype.

- **B. Internalized racism** — Owner: Levels of racism (internalized). Internalized racism is accepting negative messages about one's own group, not trauma symptoms.
- **D. John Henryism** — Owner: Sherman James. John Henryism is high-effort coping linked to health problems, not trauma reactions to racism.
- **Pair to learn:** Racism producing trauma-like reactions → race-based traumatic stress (Carter, not Sue)

6 — C

Intake, Assessment & Diagnosis · Outline 2.M · Chapter 20

- **C. 82 to 94 — Owner: ±2 SEM = 95% confidence band.** Plus or minus 2 SEM is 88 ± 6, giving 82 to 94.
- **A. 73 to 103** — Owner: ±1 SD (15), not the SEM. 88 ± 15 uses the test's standard deviation instead of its SEM.
- **B. 85 to 91** — Owner: ±1 SEM = 68% confidence band. 88 ± 3 is the 68% band, not the 95% band.
- **D. 79 to 97** — Owner: ±3 SEM. 88 ± 9 uses three SEMs, wider than the 95% band.
- **Pair to learn:** ±2 SEM → 95% band (88 with SEM 3 → 82 to 94)

7 — D

Areas of Clinical Focus · Outline 3.S · Chapter 3

- **D. Margaret Mahler — Owner: Mahler (separation-individuation).** Mahler described the infant psychologically "hatching" from the caregiver through these separation-individuation subphases.
- **A. Erik Erikson** — Owner: Erikson (psychosocial development). Erikson named life-span crises such as autonomy vs. shame and doubt, not these subphases.
- **B. Mary D. S. Ainsworth** — Owner: Ainsworth (Strange Situation). Ainsworth classified attachment as

secure, anxious-avoidant, or anxious-resistant; she did not name separation-individuation subphases.

- **C. John Bowlby** — Owner: Bowlby (attachment theory). Bowlby owns the secure base, internal working models, and separation distress, not these subphases.
- **Pair to learn:** Rapprochement → Mahler (not Bowlby)

8 — D

Counseling Skills & Interventions · Outline 5.AT · Chapter 8

- **D. Process observation — Owner: Group leader skill: process observation.** Commenting on how the group is interacting, rather than what it is discussing, is process observation.
- **A. Universalizing** — Owner: Group leader skill: universalizing. Universalizing names a shared experience across members.
- **B. Linking** — Owner: Group leader skill: linking. Linking connects one member's concern to another's.
- **C. Summarizing** — Owner: Group leader skill: summarizing. Summarizing ties together themes at the end of a segment or session.
- **Pair to learn:** Comment on how the group interacts → process observation (not summarizing)

9 — D

Counseling Skills & Interventions · Outline 5.T · Chapter 5

- **D. Passive-aggressive — Owner: Assertiveness training (behavioral tradition: Wolpe, Salter, Lazarus).** An indirect, sarcastic jab instead of a direct request is passive-aggressive.
- **A. Passive** — Owner: Assertiveness training. A passive response says nothing and resents it privately.
- **B. Aggressive** — Owner: Assertiveness training. An aggressive reply is an open attack or insult.

- **C. Assertive** — Owner: Assertiveness training. An assertive reply would name the behavior, the feeling, the impact, and a request.
- **Pair to learn:** Sarcastic indirect jab → passive-aggressive (not assertive)

10 — B

Areas of Clinical Focus · Outline 3.AQ · Chapter 12

- **B. Tourette's disorder — Owner: DSM-5-TR Tourette's disorder.** Tourette's requires multiple motor tics and at least one vocal tic for more than 1 year, with onset before age 18.
- **A. ADHD, combined presentation** — Owner: DSM-5-TR ADHD. ADHD involves inattention and hyperactivity-impulsivity, not motor and vocal tics.
- **C. Autism spectrum disorder** — Owner: DSM-5-TR autism spectrum disorder. Autism requires social communication deficits plus restricted, repetitive behaviors; tics alone don't meet it.
- **D. OCD with tic-related specifier** — Owner: DSM-5-TR OCD (tic-related specifier). This specifier requires obsessions or compulsions, which are not described here.
- **Pair to learn:** Motor + vocal tics, over 1 year, onset before 18 → Tourette's disorder

11 — A

Professional Practice & Ethics · Outline 1.H · Chapter 17

- **A. It isn't exploitative, fits local norms, and is put in writing — Owner: Bartering conditions (ACA A. 10.e).** A.10.e allows bartering only when the client requests it, it isn't exploitative or harmful, it's accepted in the community, and it's documented in a written contract.
- **B. The value of her work is greater than the session fees** — Owner: Unequal exchange (exploitation risk). An unequal exchange risks

exploitation and unfair advantage; it doesn't make bartering acceptable.

- **C. The agency allows it and she is told about her other options** — Owner: Self-referral rule (ACA A.10.a). Agency permission plus informing clients of other options is the rule for referring agency clients to a private practice, not for bartering.
- **D. The counselor proposes the terms as a fair way to help** — Owner: Counselor-initiated bartering (punished). Bartering must be requested by the client; counselor-initiated bartering is what the exam punishes.
- **Pair to learn:** Client-requested barter → ACA A.10.e conditions and written contract

12 — B

Professional Practice & Ethics · Outline 1.U · Chapter 17

- **B. May not, since her doctorate isn't in a counseling-related field — Owner: Use of "Dr." (ACA C.4.d).** C.4.d permits counselors to use "Dr." only with a doctorate in counseling or a closely related field.
- **A. May, since any earned doctorate allows the title in advertising** — Owner: Misreading of ACA C.4.d. C.4.d limits the title to doctorates in counseling or a closely related field, not any doctorate.
- **C. May, if no current clients appear in her advertising** — Owner: Testimonials rule (ACA C.3.b). C.3.b bars soliciting testimonials from current or former clients or others vulnerable to undue influence; it has nothing to do with claiming a title.
- **D. May, as long as her NCC certification is listed next to it** — Owner: Certification (NCC from NBCC). A national certification doesn't change which degree qualifies a counselor to use "Dr."
- **Pair to learn:** "Dr." in counseling practice → Doctorate in counseling or closely related field (C.4.d)

13 — B

Core Counseling Attributes · Outline 6.D · Chapter 22

- **B. "You learned men shouldn't cry. What has that rule been like for you?" — Owner: Gender sensitivity: exploring gender-role messages (6.D).** It names the gender-role message without endorsing it and invites him to explore its effect on him.
- **A. "There's no need to apologize. Real men cry too, and it's healthy."** — Owner: Reassurance. Reassurance argues with his belief instead of understanding where it came from.
- **C. "Let's move on to what you could do to solve the problem itself."** — Owner: Premature problem-solving (avoiding feeling). Moving away from his tears quietly reinforces the rule that men shouldn't show feeling.
- **D. "I know exactly how you feel. I was raised with that rule myself."** — Owner: Identification (not empathy). "I know exactly how you feel" shifts the focus to the counselor.
- **Pair to learn:** Exploring gender-role messages → Gender sensitivity (6.D), not reassurance

14 — A

Counseling Skills & Interventions · Outline 5.AL · Chapter 4

- **A. Affirm her readiness and begin there next session — Owner: Containment: don't open painful material at session end.** New painful material shouldn't be opened in the last minutes; leave time to contain and plan.
- **B. Invite her to start now while she feels ready to talk** — Owner: Opening new painful material at session end. Starting now risks ending the session with the client unstabilized.

- **C. Ask her why she waited so long to bring this up** — Owner: "Why" question. A why question invites defensiveness and misses her readiness.
- **D. Change the subject to her plans for the week ahead** — Owner: Changing the subject. Changing the subject avoids what she raised instead of acknowledging it.
- **Pair to learn:** Last minutes of session → don't open new painful material; plan for next time

15 — C

Areas of Clinical Focus · Outline 3.AA · Chapter 15

- **C. Moral, religious, or spiritual problem — Owner: DSM-5-TR Z65.8 (focus of clinical attention).** Z65.8 covers problems with conversion and questioning spiritual values; it is a condition that may be a focus of attention, not a mental disorder.
- **A. Adjustment disorder with depressed mood** — Owner: DSM-5-TR adjustment disorder. She has no depressive symptoms or impairment that would support a mental disorder.
- **B. OCD with religious scrupulosity** — Owner: OCD (scrupulosity). Scrupulosity is obsessive fear of sin or offending God, with compulsions, which she doesn't describe.
- **D. A cultural concept of distress** — Owner: DSM-5-TR cultural concepts of distress. Cultural concepts are culture-specific idioms of suffering, such as susto, not faith questioning after conversion.
- **Pair to learn:** Questioning faith after conversion → Z65.8 moral, religious, or spiritual problem (not a disorder)

16 — D

Counseling Skills & Interventions · Outline 5.Q · Chapter 6

- **D. Aaron Beck — Owner: Beck (hopelessness; Beck Hopelessness Scale).** Beck identified

hopelessness as a stronger predictor of suicide than depression.

- **A. Kelly Posner** — Owner: Posner and colleagues (C-SSRS). Posner's team developed the Columbia-Suicide Severity Rating Scale.
- **B. Thomas Joiner** — Owner: Joiner (interpersonal theory of suicide). Joiner explains suicidal desire through belongingness and burdensomeness.
- **C. David Jobes** — Owner: Jobes (CAMS). Jobes developed the Collaborative Assessment and Management of Suicidality.
- **Pair to learn:** Hopelessness predicts suicide → Beck (not Joiner)

17 — B

Areas of Clinical Focus · Outline 3.F · Chapter 15

- **B. Marginalization — Owner: Berry (acculturation strategies).** Berry's marginalization means connecting with neither the heritage nor the host culture, and it is linked to the most distress.
- **A. Reverse culture shock** — Owner: Gullahorn & Gullahorn (W-curve). Reverse culture shock is the dip on returning home, not a lack of connection to both cultures.
- **C. Separation** — Owner: Berry (acculturation strategies). Separation keeps the heritage culture while avoiding the host; he has lost his heritage connection.
- **D. Assimilation** — Owner: Berry (acculturation strategies). Assimilation drops the heritage culture but joins the host; he doesn't feel part of the host culture.
- **Pair to learn:** Connected to neither culture → marginalization (Berry)

18 — B

Areas of Clinical Focus · Outline 3.AU · Chapter 13

- **B. Violent resistance — Owner: Michael P. Johnson (IPV typology).** Violent resistance is violence used in response to a partner's intimate terrorism, often in self-defense.
- **A. Situational couple violence** — Owner: Michael P. Johnson (IPV typology). Situational couple violence arises from escalating conflicts without a broader pattern of control; here he exerts coercive control.
- **C. Intimate terrorism** — Owner: Michael P. Johnson (IPV typology). Intimate terrorism describes his pattern of coercive control, not her response to it.
- **D. Mutual violent control** — Owner: Michael P. Johnson (IPV typology). Mutual violent control requires both partners to be controlling and violent; she is not controlling him.
- **Pair to learn:** Violence in response to a partner's coercive control → violent resistance (Johnson)

19 — A

Counseling Skills & Interventions · Outline 5.AW · Chapter 8

- **A. Invite them to discuss the carpool openly in group — Owner: Bringing outside contact back into the group (5.AW).** The expected norm is that outside contact is brought into the group to discuss, rather than kept secret.
- **B. Speak with each privately and keep it between them** — Owner: Creating more secrets. Private handling adds secrets, which is the problem itself.
- **C. Remove both members to protect the rest of the group** — Owner: Punitive removal without exploration. Removal is punitive and skips exploring what happened.

- **D. Ignore it, since the contact happens outside sessions** — Owner: Ignoring subgrouping. Subgroups can pull energy from the group and threaten cohesion.
- **Pair to learn:** Outside contact → bring it back into group (5.AW)

20 — B

Intake, Assessment & Diagnosis · Outline 2.K · Chapter 19

- **B. Grave disability — Owner: Involuntary criteria: grave disability (inability to meet basic needs).** His inability to meet basic needs for food and shelter is grave disability, one of the three general grounds for involuntary care.
- **A. His poor insight into the illness** — Owner: MSE: insight. Poor insight alone is not a basis for involuntary care; the grounds are danger to self, danger to others, or grave disability.
- **C. Imminent danger to self through suicidal intent** — Owner: Involuntary criteria: danger to self. He denies suicidal thoughts; the risk comes from his inability to meet basic needs.
- **D. Danger to others** — Owner: Involuntary criteria: danger to others. Nothing suggests threats or risk toward another person.
- **Pair to learn:** Cannot meet basic needs → Grave disability (involuntary criterion)

21 — A

Counseling Skills & Interventions · Outline 5.W · Chapter 7

- **A. Second-order change — Owner: MRI / systems theory (second-order change).** A change to the system's rules themselves is second-order change.
- **B. Equifinality** — Owner: Systems theory. Equifinality means different starting points lead to the same outcome.

- **C. Negative feedback** — Owner: Systems theory (morphostasis). Negative feedback keeps the system the same.
- **D. First-order change** — Owner: MRI / systems theory (first-order change). First-order change works within the existing rules, such as a longer grounding.
- **Pair to learn:** Change in the rules themselves → second-order change

22 — B

Treatment Planning · Outline 4.O · Chapter 21

- **B. Relapse prevention** — **Owner: Relapse prevention (termination task).** Identifying warning signs, a coping plan, supports, and what to do if problems return is relapse prevention.
- **A. Tapering** — Owner: Tapering (reducing session frequency). Tapering eases the transition by spacing sessions out, not by planning for warning signs.
- **C. Pretermination counseling** — Owner: Pretermination counseling (ACA A.11.c). Pretermination counseling is the broader ethical process of preparing for ending, not this specific plan for warning signs.
- **D. Booster sessions** — Owner: Booster sessions (follow-up after ending, 4.J). Booster sessions are follow-up contacts after ending; the stem describes building a plan for warning signs.
- **Pair to learn:** Warning signs and coping plan → Relapse prevention

23 — D

Counseling Skills & Interventions · Outline 5.Q · Chapter 6

- **D. Developmental** — **Owner: Crisis types: developmental.** Developmental crises arise from predictable life transitions.

- **A. Ecosystemic** — Owner: Crisis types: ecosystemic. Ecosystemic crises are disasters affecting whole communities.
- **B. Situational** — Owner: Crisis types: situational. Situational crises follow sudden, unexpected events such as an accident or job loss.
- **C. Existential** — Owner: Crisis types: existential. Existential crises involve inner conflicts about meaning, purpose, or identity.
- **Pair to learn:** Predictable life transitions → developmental crisis (not situational)

24 — C

Areas of Clinical Focus · Outline 3.AI · Chapter 11

- **C. No disorder; prescribed tolerance doesn't count — Owner: DSM-5-TR exception for supervised prescribed use.** Tolerance and withdrawal from medication taken as prescribed under medical supervision don't count toward a substance use disorder.
- **A. Mild sedative use disorder, with two criteria met** — Owner: SUD severity (mild = 2-3). This counts tolerance and withdrawal from prescribed use, which the DSM-5-TR excludes.
- **B. Sedative use disorder in early remission** — Owner: Early remission specifier. Remission requires a prior use disorder, which she never met.
- **D. Moderate sedative use disorder** — Owner: SUD severity (moderate = 4-5). Even counting both criteria would not reach moderate severity, and they shouldn't be counted.
- **Pair to learn:** Tolerance/withdrawal from prescribed use → not counted toward SUD

25 — C

Intake, Assessment & Diagnosis · Outline 2.T · Chapter 20

- **C. Give the PHQ-9 at intake and again at the tenth session — Owner: Pre-test/post-test**

measurement (2.T). Using the same measure before and after counseling provides a baseline and shows change over time.

- **A. Rely on her own impression of how much the client has improved** — Owner: Informal observation alone. A single informal source is subject to bias; outcomes call for a standardized measure.
- **B. Give the TAT at the end of treatment and review the stories** — Owner: Projective test (Murray & Morgan, TAT). A projective test with lower reliability, given once, cannot show symptom change.
- **D. Give the PHQ-9 once, at the tenth session, and compare it to norms** — Owner: Posttest only (no baseline). Without a pretest there is no baseline, so change cannot be shown.
- **Pair to learn:** Same measure before and after → Pre-test/post-test outcome measurement (2.T)

26 — B

Counseling Skills & Interventions · Outline 5.AN · Chapter 7

- **B. An ecomap — Owner: Hartman (ecomap).** The ecomap looks outward at the family's connections to community and support systems.
- **A. Family mapping** — Owner: Minuchin (structural). Family mapping diagrams boundaries, subsystems, and coalitions.
- **C. Family sculpting** — Owner: Satir (sculpting). Sculpting shows closeness, distance, and power through physical positions.
- **D. A genogram** — Owner: Bowen; McGoldrick & Gerson (genogram). The genogram maps patterns across generations inside the family.
- **Pair to learn:** Links to school, church, agencies → ecomap (Hartman), not genogram

27 — C

Counseling Skills & Interventions · Outline 5.Q · Chapter 6

- **C. Psychological first aid for safety and practical needs — Owner: Psychological first aid (NCTSN & National Center for PTSD).** PFA is the evidence-informed approach for the hours and days after a disaster: safety, comfort, practical help, and connection.
- **A. Prolonged exposure to process each survivor's memories** — Owner: Foa (prolonged exposure). Prolonged exposure is an adult trauma treatment, not a first response days after a disaster.
- **B. A required debriefing where each survivor retells the event** — Owner: Single-session psychological debriefing. Forced detailed retelling soon after trauma is not recommended and may increase distress.
- **D. Insight work on how past losses shape their reactions** — Owner: Psychodynamic insight work. Exploring the past is not a first-response approach in the immediate aftermath.
- **Pair to learn:** Days after a disaster → psychological first aid (not debriefing)

28 — A

Intake, Assessment & Diagnosis · Outline 2.B · Chapter 19

- **A. A semi-structured interview — Owner: Semi-structured interview.** Semi-structured interviews use core questions with room for follow-up, balancing reliability and flexibility, though they require training.
- **B. A mental status exam** — Owner: Mental Status Exam (MSE). The MSE is an observation-based snapshot of current mental functioning, not an interview format built on core diagnostic questions.
- **C. A structured interview such as the SCID-5** — Owner: Structured interview (SCID-5). Fixed questions

in a fixed order give high reliability but leave little room for unplanned follow-up.

- **D. An unstructured, conversational interview** — Owner: Unstructured interview. It is flexible and builds rapport, but its lower reliability does not give her the consistent core she wants.
- **Pair to learn:** Core questions plus room for follow-up → Semi-structured interview (not structured SCID-5)

29 — A

Areas of Clinical Focus · Outline 3.U · Chapter 3

- **A. E. G. Williamson — Owner: Williamson (trait-factor, Minnesota point of view).** Williamson systematized trait-factor matching into these six directive steps.
- **B. Tiedeman and O'Hara** — Owner: Tiedeman & O'Hara (decision-making). Tiedeman and O'Hara described anticipation and implementation phases of decision-making.
- **C. Peterson, Sampson, and Reardon** — Owner: Peterson, Sampson, & Reardon (CIP). They developed the CASVE cycle within Cognitive Information Processing.
- **D. Frank Parsons** — Owner: Parsons (trait-factor). Parsons proposed knowing yourself, knowing the world of work, and true reasoning, which Williamson later systematized.
- **Pair to learn:** Six directive steps → Williamson (not Parsons)

30 — B

Counseling Skills & Interventions · Outline 5.O · Chapter 5

- **B. Importance and confidence rulers — Owner: Miller & Rollnick (MI).** Rulers for importance and confidence are an MI tool.
- **A. A decisional balance exercise** — Owner: Prochaska & DiClemente (stage-matched intervention).

Decisional balance weighs pros and cons of changing; it is not a 0 to 10 rating.

- **C. SUDS ratings for a hierarchy** — Owner: Wolpe (systematic desensitization). SUDS rate distress to build an anxiety hierarchy.
- **D. Solution-focused scaling questions** — Owner: de Shazer & Berg (SFBT). SFBT scaling questions also use 0 to 10, but the importance/confidence rulers are MI's.
- **Pair to learn:** Importance and confidence rulers → MI (not SFBT scaling)

31 — A

Intake, Assessment & Diagnosis · Outline 2.M · Chapter 20

- **A. z = −2; deviation IQ score of 70 — Owner: z = (x − mean) ÷ SD; deviation IQ = 100 + 15z.** (36 − 60) ÷ 12 = −2, and 100 + 15(−2) = 70, about the 2nd percentile.
- **B. z = −2.4; deviation IQ score of 64** — Owner: Divided by 10 instead of the test's SD of 12. −24 ÷ 10 = −2.4 uses the wrong SD; the test's SD is 12.
- **C. z = −2; deviation IQ score of 80** — Owner: T-score SD of 10 used instead of 15. The z is right, but 100 + 10(−2) uses the T-score's SD; deviation IQ uses SD 15.
- **D. z = +2; deviation IQ score of 130** — Owner: Sign error. The raw score is below the mean, so z is negative.
- **Pair to learn:** Raw 36 (mean 60, SD 12) → z = −2 → deviation IQ 70

32 — C

Treatment Planning · Outline 4.N · Chapter 21

- **C. PHQ-9 score of 14; affect constricted — Owner: SOAP: Objective (counselor observations, test scores).** What the counselor observes or measures, such as affect and test scores, goes in Objective.

- **A. Review the safety plan at next session** — Owner: SOAP: Plan (next steps). Next steps belong in Plan.
- **B. Client says, "I can't get out of bed."** — Owner: SOAP: Subjective (client's report). The client's own words belong in Subjective.
- **D. Progress toward Objective 2 is moderate** — Owner: SOAP: Assessment (clinical impression). The counselor's impression of progress belongs in Assessment.
- **Pair to learn:** Counselor-observed affect and test score → SOAP: Objective

33 — C

Treatment Planning · Outline 4.F · Chapter 21

- **C. Refer her to legal or benefits resources, then follow up — Owner: Concurrent referral to community resources (4.F).** Legal and financial needs are referred to appropriate community resources, with follow-up to confirm she connected, while counseling continues.
- **A. Set the issue aside until her anxiety is under control** — Owner: Ignoring basic needs (sequencing). Practical and financial needs come before longer-term work, not after.
- **B. Advise her on the legal steps to file her appeal** — Owner: Practicing outside competence (ACA C.2.a). Giving legal advice falls outside a counselor's competence.
- **D. Call the benefits office to argue her case on her behalf** — Owner: Advocacy without client consent (ACA A.7.b). Advocating for a specific client requires her consent first, and empowering her to advocate is preferred where possible.
- **Pair to learn:** Legal or benefits problem → Concurrent referral to community resources (4.F)

34 — A

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **A. Albert Ellis — Owner: Ellis (REBT).** REBT lists unconditional self-acceptance among its key terms and goals.
- **B. Carl Rogers** — Owner: Rogers (person-centered). Rogers owns unconditional positive regard, a core condition the counselor offers, not unconditional self-acceptance.
- **C. Steven Hayes** — Owner: Hayes (ACT). Hayes's terms are defusion, experiential avoidance, values, and committed action.
- **D. Aaron Beck** — Owner: Beck (cognitive therapy). Beck's terms are automatic thoughts, distortions, schemas, and collaborative empiricism.
- **Pair to learn:** Unconditional self-acceptance → Ellis (not Rogers' unconditional positive regard)

35 — B

Areas of Clinical Focus · Outline 3.D · Chapter 16

- **B. Relational bullying — Owner: Olweus (bullying types).** Relational (social) bullying uses exclusion and rumors, and the repetition and power imbalance meet Olweus' definition.
- **A. Ordinary peer conflict** — Owner: Peer conflict (not bullying). Peer conflict lacks the repetition and power imbalance present here.
- **C. Verbal bullying** — Owner: Olweus (bullying types). Verbal bullying is name-calling, threats, or teasing aimed at the target.
- **D. Cyberbullying** — Owner: Olweus (bullying types). Cyberbullying happens through texts, social media, or online platforms.
- **Pair to learn:** Exclusion and rumors → relational bullying

36 — C

Intake, Assessment & Diagnosis · Outline 2.U · Chapter 20

- **C. The ORS and SRS (Miller & Duncan) — Owner: Miller & Duncan (Outcome and Session Rating Scales).** The ORS and SRS are brief session-by-session outcome and alliance measures.
- **A. The OQ-45 (Lambert)** — Owner: Lambert (Outcome Questionnaire). The OQ-45 tracks progress but does not rate the alliance.
- **B. The SCL-90-R and the BSI (Derogatis)** — Owner: Derogatis (broad symptom checklists). These are broad symptom checklists, not session-by-session alliance measures.
- **D. The BASC-3 and the Conners** — Owner: Behavior rating scales for children. These rate children's behavior, not counseling outcome and alliance.
- **Pair to learn:** Session-by-session outcome and alliance → ORS/SRS (Miller & Duncan), not OQ-45

37 — D

Core Counseling Attributes · Outline 6.K · Chapter 22

- **D. Positive regard and affirmation — Owner: Norcross & Lambert: demonstrably effective element.** Positive regard and affirmation were judged demonstrably effective, along with the alliance, empathy, and collecting client feedback.
- **A. Confrontation of discrepancies** — Owner: Ivey: influencing skill. Confrontation is an influencing skill, not on the task forces' list.
- **B. Counselor self-disclosure** — Owner: Influencing skill. Self-disclosure was not among the demonstrably effective elements.
- **C. Interpretation of the client's defenses** — Owner: Psychodynamic technique. Interpretation is a technique, not one of the demonstrably effective relationship elements.

- **Pair to learn:** Demonstrably effective element → Positive regard and affirmation (Norcross & Lambert)

38 — D

Counseling Skills & Interventions · Outline 5.B · Chapter 5

- **D. Viewing the teen's symptoms as part of a family pattern — Owner: Family modality: the system is the client.** In family work the system is the client; symptoms are seen as serving a function, with circular rather than linear causality.
- **A. Promising each member that private disclosures stay private** — Owner: Unplanned secrets promise (ACA B.4.b calls for confidentiality terms agreed at the start). A secrets policy must be set and explained at the start, not promised ad hoc.
- **B. Treating the teen as the problem the family brought in** — Owner: Identified patient as 'the problem' (linear view). Blaming the IP is a wrong answer in family work.
- **C. Keeping the focus on the teen's intrapersonal insight** — Owner: Individual modality focus. An intrapersonal focus belongs to individual work, not the family modality.
- **Pair to learn:** Individual → family modality: the system is the client (circular, not linear)

39 — C

Treatment Planning · Outline 4.P · Chapter 21

- **C. Explain the rationale for exposure and how it fits her goals — Owner: Adherence: explain the rationale (4.P).** Her barrier is not understanding why the task matters, so explaining the rationale is the first step toward adherence.
- **A. Use motivational interviewing to explore and resolve her ambivalence** — Owner: Motivational interviewing (when ambivalence is the barrier). MI fits

when mixed feelings about change are the barrier; she is asking how the task works.

- **B. Drop the exposure work, since she doesn't want to do it** — Owner: Abandoning the plan without discussion. She hasn't refused; she is confused about why it helps, so dropping it skips a needed explanation.
- **D. Tell her counseling will end if she doesn't comply** — Owner: Compliance as obedience (older framing). Ultimatums reflect an obedience model, not the collaborative adherence the exam rewards.
- **Pair to learn:** Doesn't understand why the task helps → Explain the rationale (4.P)

40 — A

Areas of Clinical Focus · Outline 3.AI · Chapter 11

- **A. Stimulant intoxication — Owner: Stimulant intoxication (DSM-5-TR).** Stimulant intoxication brings dilated pupils, rapid heart rate, sweating, agitation, and paranoia.
- **B. Sedative-hypnotic intoxication** — Owner: Sedative, hypnotic, or anxiolytic intoxication. Sedative intoxication resembles alcohol intoxication, with slurred speech, poor coordination, and memory problems, not dilated pupils and paranoia.
- **C. Opioid intoxication** — Owner: Opioid intoxication. Opioid intoxication causes pinpoint pupils, drowsiness, and slowed breathing.
- **D. Cannabis withdrawal** — Owner: Cannabis withdrawal. Cannabis withdrawal involves irritability, anxiety, sleep difficulty, and decreased appetite, not dilated pupils and paranoia.
- **Pair to learn:** Dilated pupils, racing heart, paranoia → stimulant intoxication (not opioids)

41 — B

Areas of Clinical Focus · Outline 3.AI · Chapter 11

- **B. Early remission, in a controlled environment — Owner: DSM-5-TR remission specifiers.** Three to

under 12 months with no criteria except craving is early remission, and a locked facility restricting access adds “in a controlled environment.”

- **A. Early remission, on maintenance therapy** — Owner: On maintenance therapy specifier. Maintenance therapy applies to opioid and tobacco use with agonist or antagonist medication, not described here.
- **C. No specifier until he is discharged** — Owner: Misapplied specifier rule. Specifiers apply now; the controlled-environment specifier exists for exactly this situation.
- **D. Sustained remission, in a controlled environment** — Owner: Sustained remission specifier. Sustained remission requires 12 months or more.
- **Pair to learn:** Locked facility + 3 to 12 months → early remission, in a controlled environment

42 — D

Counseling Skills & Interventions · Outline 5.AH · Chapter 8

- **D. Gently block the rescue and give her room to feel — Owner: Blocking rescuing / band-aiding.** Smoothing over pain too quickly is rescuing; the leader blocks it gently so the member can stay with the feeling.
- **A. Link the comment to other members’ stories of hope** — Owner: Linking (misapplied). Linking here would build on the rescue instead of protecting the grieving member.
- **B. Move to another member to ease the tension** — Owner: Avoiding the feeling. Moving on avoids the emotion the member was beginning to express.
- **C. Thank the other member for being so supportive** — Owner: Reinforcing a rescue (confused with supporting). Praising the rescue reinforces band-aiding.

- **Pair to learn:** Smoothing over pain too fast → rescuing: block gently

43 — D

Areas of Clinical Focus · Outline 3.O · Chapter 12

- **D. His adaptive functioning in daily life — Owner: DSM-5-TR intellectual developmental disorder (severity by adaptive functioning).** The DSM-5-TR bases intellectual disability severity on adaptive functioning (conceptual, social, practical), not on IQ score alone.
- **A. His full-scale IQ score by itself** — Owner: IQ-only approach (WISC score). IQ helps establish deficits in intellectual functioning, but severity is not set by IQ alone.
- **B. How many settings his symptoms appear in** — Owner: ADHD criterion (two or more settings). Symptoms across two or more settings is an ADHD requirement, not how intellectual disability severity is rated.
- **C. His results on a developmental screener** — Owner: Ages and Stages Questionnaires (screening). Developmental screens flag possible delays for referral; they do not assign intellectual disability severity.
- **Pair to learn:** Severity based on adaptive functioning → intellectual developmental disorder (not IQ alone)

44 — B

Areas of Clinical Focus · Outline 3.AV · Chapter 13

- **B. A break of about 20 minutes before resuming — Owner: Gottman (flooding).** Gottman recommends recognizing flooding and taking a break of at least about 20 minutes before returning to the conversation.
- **A. Using I-messages to express his anger** — Owner: Gordon (PET I-messages). I-messages help express feelings, but a flooded partner first needs to calm down physiologically.

- **C. Keep working on it until they reach a solution** — Owner: Pushing through conflict (not recommended). A flooded partner can't process information; pushing on usually escalates conflict.
- **D. A softened startup from his wife** — Owner: Gottman (softened startup). Softened startup replaces criticism at the start of a conversation; it doesn't address flooding already underway.
- **Pair to learn:** Partner flooded → take a break of about 20 minutes (Gottman)

45 — D

Counseling Skills & Interventions · Outline 5.A · Chapter 5

- **D. Dora Kalff — Owner: Kalff (Jungian sandplay).** Dora Kalff developed Jungian sandplay.
- **A. Margaret Lowenfeld** — Owner: Lowenfeld (world technique). Lowenfeld owns the world technique, the earlier sand tray method, not the Jungian version.
- **B. Bernard and Louise Guerney** — Owner: Guerney (filial therapy). The Guerneys train parents to run child-centered play sessions at home.
- **C. Virginia Axline** — Owner: Axline (child-centered play therapy). Axline applied Rogers' core conditions to play.
- **Pair to learn:** Jungian sandplay → Kalff (world technique → Lowenfeld)

46 — B

Intake, Assessment & Diagnosis · Outline 2.H · Chapter 19

- **B. Inpatient hospitalization — Owner: Inpatient care: imminent danger, acute mania or psychosis.** Imminent danger plus acute mania calls for 24-hour medically supervised care; that is the least restrictive level that is safe here.
- **A. Weekly outpatient counseling with a safety plan** — Owner: Outpatient care. Weekly sessions fit

mild to moderate symptoms in someone who is safe, which she is not.

- **C. Partial hospitalization** — Owner: Partial hospitalization (day program, home at night). She would return home at night, where her family cannot keep her safe.
- **D. An intensive outpatient program** — Owner: Intensive outpatient (IOP). IOP offers several hours a day while living at home, far less than her imminent danger requires.
- **Pair to learn:** Acute mania with imminent danger → Inpatient hospitalization (not PHP)

47 — D

Areas of Clinical Focus · Outline 3.X · Chapter 10

- **D. Hypoarousal outside her window of tolerance — Owner: Siegel (window of tolerance).** Siegel describes trauma narrowing the window of tolerance, so arousal swings into hypoarousal: numbness, shutdown, and dissociation.
- **A. Learned helplessness from uncontrollable events** — Owner: Seligman (learned helplessness). Learned helplessness is a theory of depression, not an in-session dissociative shutdown.
- **B. Hyperarousal outside her window of tolerance** — Owner: Siegel (window of tolerance). Hyperarousal shows as anxiety, panic, or anger, not numbness and shutdown.
- **C. Exhaustion stage of general adaptation** — Owner: Selye (general adaptation syndrome). Selye's exhaustion stage follows prolonged stress, not a moment-to-moment shutdown when recalling trauma.
- **Pair to learn:** Numbness, shutdown, dissociation → hypoarousal (Siegel), not Selye's exhaustion

48 — C

Counseling Skills & Interventions · Outline 5.E · Chapter 4

- **C. Edward Bordin — Owner: Bordin (working alliance).** Goals, tasks, and bond are Bordin's three parts of the working alliance.
- **A. Gerard Egan** — Owner: Egan (The Skilled Helper). Egan owns SOLER and the three-stage skilled helper model.
- **B. Carl Rogers** — Owner: Rogers (core conditions). Rogers owns congruence, unconditional positive regard, and empathy.
- **D. Safran and Muran** — Owner: Safran & Muran (alliance ruptures). Safran and Muran describe withdrawal and confrontation ruptures and their repair.
- **Pair to learn:** Goals, tasks, bond → Bordin (not Safran & Muran)

49 — A

Counseling Skills & Interventions · Outline 5.S · Chapter 4

- **A. "What was going on for you as the day got closer?" — Owner: Open "what" question.** A what/how question invites exploration without asking her to justify herself.
- **B. "Were you invited? Did you want to go? Who else stayed?"** — Owner: Question string. Several questions in a row turn the session into an interrogation.
- **C. "Don't you think your sister was hurt by that?"** — Owner: Leading question (advice in disguise). A leading question carries the counselor's judgment.
- **D. "Why did you decide not to go to the wedding?"** — Owner: "Why" question. Why questions ask the client to justify and invite defensiveness.

- **Pair to learn:** Convert “why” to “what” or “how” to avoid defensiveness

50 — A

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **A. Reciprocal determinism — Owner: Bandura (social cognitive).** Reciprocal determinism is a Bandura key term, alongside modeling and self-efficacy.
- **B. Reciprocal inhibition** — Owner: Wolpe (systematic desensitization). Reciprocal inhibition is Wolpe’s principle of pairing relaxation with anxiety.
- **C. Self-instructional training** — Owner: Meichenbaum (cognitive behavior modification). Self-instructional training is Meichenbaum’s technique.
- **D. Schedules of reinforcement** — Owner: Skinner (operant conditioning). Schedules of reinforcement belong to Skinner’s operant model.
- **Pair to learn:** Reciprocal determinism → Bandura; reciprocal inhibition → Wolpe

51 — B

Intake, Assessment & Diagnosis · Outline 2.E · Chapter 19

- **B. A Z code for uncomplicated bereavement — Owner: Z codes (conditions that may be a focus of clinical attention).** Z codes record conditions such as uncomplicated bereavement that are a focus of attention but are not mental disorders.
- **A. Adjustment disorder with depressed mood** — Owner: Adjustment disorder (First’s step 5). Any disorder, adjustment disorder included, requires clinically significant distress or impairment, which she does not show.
- **C. Provisional major depressive disorder** — Owner: Provisional diagnosis. Nothing suggests she will go on to meet criteria, so a provisional label would pathologize normal grief.

- **D. Unspecified depressive disorder** — Owner: Unspecified disorder. An unspecified diagnosis still names a mental disorder; expected grief with intact functioning is not one.
- **Pair to learn:** Uncomplicated bereavement → Z code (not a mental disorder)

52 — B

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **B. The cognitive triad — Owner: Beck (cognitive therapy).** A negative view of self, world, and future is Beck's cognitive triad of depression.
- **A. Awfulizing** — Owner: Ellis (REBT). Awfulizing is Ellis's term for an irrational belief, not the self-world-future pattern of depression.
- **C. Basic mistakes** — Owner: Adler (individual psychology). Basic mistakes are Adler's mistaken beliefs about how to belong.
- **D. A problem-saturated story** — Owner: White & Epston (narrative). A problem-saturated story is narrative therapy's term for an oppressive dominant story.
- **Pair to learn:** Negative view of self, world, future → cognitive triad (Beck)

53 — D

Treatment Planning · Outline 4.N · Chapter 21

- **D. Only what the physician needs to help with her sleep — Owner: Minimal disclosure (ACA B.2.e).** Even with a release, counselors share only the information essential to the purpose, here her sleep problems.
- **A. Nothing, since physicians aren't part of counseling** — Owner: Ignores collaboration and integrated care (4.N). Coordinating with physicians, with a release, is part of good care.

- **B. Her psychotherapy notes, which explain session content** — Owner: HIPAA psychotherapy notes (extra protection). Psychotherapy notes are kept separate and generally require specific authorization; they aren't needed here.
- **C. Her full counseling record, so the picture is complete** — Owner: Exceeds minimal disclosure (ACA B.2.e). Sharing the whole record goes beyond what the physician needs.
- **Pair to learn:** Coordination with a release → Share the minimum necessary

54 — B

Treatment Planning · Outline 4.P · Chapter 21

- **B. Emphasizes collaboration and the client's active role — Owner: Adherence (collaborative follow-through).** Adherence frames follow-through as collaboration, with the client as an active participant.
- **A. Is the legal term for court-mandated treatment plans** — Owner: Mandated clients (ACA A.2.e). Mandated treatment has its own rules about sharing information; adherence isn't a legal term for it.
- **C. Applies only to taking prescribed psychiatric medication** — Owner: Medication referral (4.F). Adherence covers homework and attendance too, not just medication.
- **D. Emphasizes the counselor's authority over the plan** — Owner: Compliance (older, obedience framing). Emphasizing counselor authority is the obedience framing that adherence moves away from.
- **Pair to learn:** Collaborative follow-through → Adherence (not compliance)

55 — A

Counseling Skills & Interventions · Outline 5.Y · Chapter 5

- **A. "Here's information on a nearby children's divorce group." — Owner: Providing education**

resources (psychoeducational group). Divorce adjustment for children is a classic psychoeducational group; offering it is information-giving, not advice.

- **B. "You should keep them out of counseling until things settle."** — Owner: Advice-giving. Telling the client what to do shifts responsibility and skips exploration.
- **C. "Why do you think the children are struggling so much?"** — Owner: "Why" question. A why question invites defensiveness and offers no resource.
- **D. "Kids are resilient, so they'll bounce back on their own."** — Owner: Reassurance. Reassurance dismisses the concern and offers nothing the client can use.
- **Pair to learn:** Information-giving and resources (not advice) → 5.Y

56 — C

Areas of Clinical Focus · Outline 3.B · Chapter 12

- **C. Socioemotional selectivity theory — Owner: Carstensen (socioemotional selectivity).** Carstensen proposed that as time horizons shorten, people focus on emotionally meaningful relationships and prune peripheral ones.
- **A. Activity theory** — Owner: Havighurst and colleagues (activity theory). Activity theory links satisfaction to staying active and involved; she is deliberately narrowing her involvement.
- **B. Disengagement theory** — Owner: Cumming & Henry (disengagement theory). Disengagement describes mutual withdrawal of older adults and society; she is deepening close ties, not withdrawing overall.
- **D. Selective optimization with compensation** — Owner: Baltes & Baltes (SOC). SOC concerns selecting goals and compensating for losses in ability, not pruning relationships to the most meaningful ones.

- **Pair to learn:** Focus on meaningful relationships as time shortens → Carstensen (not disengagement)

57 — A

Professional Practice & Ethics · Outline 1.C · Chapter 17

- **A. Accreditation — Owner: Accreditation (programs, e.g., CACREP).** Accreditation approves programs, not individuals; CACREP accredits counseling programs.
- **B. Registry** — Owner: Registry (listing of practitioners). A registry lists practitioners who meet basic criteria; it doesn't approve programs.
- **C. Licensure** — Owner: Licensure (state permission to practice). Licensure is a state's permission for an individual to practice.
- **D. Certification** — Owner: Certification (e.g., NCC from NBCC). Certification is a credential given to individuals by a professional body.
- **Pair to learn:** Approves programs → Accreditation (CACREP)

58 — C

Areas of Clinical Focus · Outline 3.AB · Chapter 12

- **C. Honeymoon — Owner: Atchley (phases of retirement).** Atchley's honeymoon phase is the early period of enjoying freedom, travel, and new activities.
- **A. Disengagement** — Owner: Super (career stage, 65+). Disengagement is Super's career stage of decelerating and retirement planning, not one of Atchley's retirement phases.
- **B. Disenchantment** — Owner: Atchley (phases of retirement). Disenchantment is the letdown that follows when retirement doesn't match expectations; he is still enjoying it.
- **D. Stability** — Owner: Atchley (phases of retirement). Stability is a later, settled and predictable lifestyle, not the early burst of excitement.

- **Pair to learn:** Enjoying new freedom early in retirement → honeymoon phase (Atchley)

59 — C

Counseling Skills & Interventions · Outline 5.Q · Chapter 6

- **C. Ask her directly about thoughts of killing herself — Owner: Sudden calm as a warning sign; ask directly.** A sudden calm can follow a decision to die, and putting affairs in order is preparatory behavior; reassess directly.
- **A. Explore what helped her feel at peace this week** — Owner: Exploring before assessing safety. Safety assessment comes before exploration when warning signs appear.
- **B. Begin planning for termination given her progress** — Owner: Premature termination planning. Termination planning ignores the warning signs.
- **D. Note the improvement and space sessions further apart** — Owner: Myth: sudden improvement means danger has passed. Treating a warning sign as progress under-responds to risk.
- **Pair to learn:** Sudden calm + putting affairs in order → reassess suicide risk directly

60 — D

Counseling Skills & Interventions · Outline 5.E · Chapter 4

- **D. Clara Hill — Owner: Hill (helping skills model).** Exploration, insight, action are Hill's three stages.
- **A. Gerard Egan** — Owner: Egan (The Skilled Helper). Egan's stages are current picture, preferred picture, and the way forward.
- **B. Allen Ivey** — Owner: Ivey (microskills hierarchy). Ivey's hierarchy runs attending, listening, influencing.
- **C. Robert Carkhuff** — Owner: Carkhuff (empathy scale). Carkhuff owns the five-level empathy rating scale.

- **Pair to learn:** Exploration → insight → action → Hill (not Egan)

61 — C

Areas of Clinical Focus · Outline 3.L · Chapter 10

- **C. Interpersonal and social rhythm therapy — Owner: Ellen Frank (IPSRT).** Frank's IPSRT stabilizes daily routines and sleep for people with bipolar disorder.
- **A. Interpersonal therapy's four problem areas** — Owner: Klerman & Weissman (IPT). IPT targets grief, role disputes, role transitions, and interpersonal deficits in depression, not social rhythms.
- **B. Behavioral activation** — Owner: Lewinsohn (behavioral theory). Behavioral activation increases response-contingent positive reinforcement for depression.
- **D. Stimulus control for insomnia** — Owner: Bootzin (CBT-I stimulus control). Stimulus control reassociates the bed with sleep for insomnia; it isn't a bipolar mood-stabilizing therapy.
- **Pair to learn:** Stabilizing routines and sleep in bipolar disorder → IPSRT (Frank), not IPT

62 — C

Counseling Skills & Interventions · Outline 5.AK · Chapter 7

- **C. Help them step out of the triangle with her mother — Owner: Bowen (triangles; detriangulation).** Drawing in a third person to reduce tension is a triangle; the Bowenian move is to help the original pair deal directly.
- **A. Map the boundaries between their family's subsystems** — Owner: Minuchin (family mapping). Mapping boundaries and subsystems is structural, not Bowenian.

- **B. Sculpt the family to show who is close to whom** — Owner: Satir (family sculpting). Sculpting is Satir's experiential technique.
- **D. Prescribe even more calls to the mother as a directive** — Owner: Haley (prescribing the symptom). Prescribing the symptom is a strategic directive.
- **Pair to learn:** In-law pulled into every fight → triangle; detriangulate (Bowen)

63 — C

Professional Practice & Ethics · Outline 1.X · Chapter 17

- **C. Seek help and limit or pause her caseload until she can resume — Owner: Impairment (ACA C. 2.g).** C.2.g directs counselors to seek help and limit, suspend, or terminate professional responsibilities until they can practice safely.
- **A. Close her practice and end all services by the end of the week** — Owner: Abandonment (ACA A.12). Abrupt closure without arranging continuation of care risks abandonment; she can limit services and return when ready.
- **B. Explain her illness to clients so they can make allowances** — Owner: Counselor self-disclosure that burdens clients. This shifts the burden onto clients and doesn't address the impairment.
- **D. Keep her full caseload, since the illness isn't her fault** — Owner: Practicing while impaired (punished). Fault is irrelevant; continuing while impaired puts clients at risk, which C.2.g prohibits.
- **Pair to learn:** Counselor's own impairment → Limit or suspend services (C.2.g)

64 — A

Counseling Skills & Interventions · Outline 5.AC · Chapter 4

- **A. Level 2, because it subtracts from the message — Owner: Carkhuff level 2 (subtractive).** A

generalization that misses the client's feeling subtracts from her message.

- **B. Level 5, because it points the client toward action** — Owner: Carkhuff level 5. Level 5 adds significantly and invites action or deeper understanding.
- **C. Level 4, because it adds meaning below the surface** — Owner: Carkhuff level 4 (additive). Level 4 reaches a feeling or meaning just below the surface; this response reaches neither.
- **D. Level 3, because it is interchangeable** — Owner: Carkhuff level 3 (interchangeable). Level 3 would capture both the content and the feeling, such as missing her friend.
- **Pair to learn:** Generalization that misses the feeling → Carkhuff level 2 (subtracts)

65 — A

Areas of Clinical Focus · Outline 3.X · Chapter 10

- **A. Felitti and Anda — Owner: Felitti & Anda (ACE Study).** Felitti and Anda's Adverse Childhood Experiences Study linked more childhood adversity to higher risk of later physical and mental health problems.
- **B. Judith Herman** — Owner: Herman (Trauma and Recovery). Herman described complex trauma and three stages of recovery.
- **C. George Bonanno** — Owner: Bonanno (resilience). Bonanno showed that resilience is the most common response to potential trauma.
- **D. Bessel van der Kolk** — Owner: van der Kolk (The Body Keeps the Score). Van der Kolk wrote about trauma's effects on the body and brain, but did not conduct the ACE Study.
- **Pair to learn:** ACE Study → Felitti & Anda (not van der Kolk)

66 — D

Intake, Assessment & Diagnosis · Outline 2.M · Chapter 20

- **D. Test her in Spanish and add nonverbal measures before placement — Owner: Diana v. State Board of Education (1970).** Diana held that children must be tested in their primary language, with nonverbal measures, before special education placement.
- **A. Make sure the test covers only what she was taught in school** — Owner: Curricular validity (Debra P. v. Turlington). Debra P. concerned graduation tests covering what was taught, not the language of special education testing.
- **B. Give the English WISC-V, since it's the most widely used test** — Owner: Ignoring primary language (contrary to Diana; ACA E.8). Testing in English would measure her English, not her ability.
- **C. Place her by ability group using last year's group test scores** — Owner: Tracking by biased group tests (Hobson v. Hansen). Hobson found ability tracking based on biased group tests unconstitutional.
- **Pair to learn:** Test in the primary language before special education placement → Diana v. State Board of Education

67 — D

Counseling Skills & Interventions · Outline 5.M · Chapter 5

- **D. D'Zurilla and Goldfried — Owner: D'Zurilla & Goldfried (problem-solving training).** This five-step problem-solving sequence belongs to D'Zurilla and Goldfried.
- **A. Fisher and Ury** — Owner: Fisher & Ury (interest-based negotiation). Fisher and Ury focus on interests, not positions, in conflict resolution.

- **B. Jay Haley** — Owner: Haley (strategic first interview). Haley's sequence is social, problem, interaction, goal-setting, task-setting.
- **C. James and Gilliland** — Owner: James & Gilliland (six-step crisis model). Their model has listening steps then acting steps for crisis intervention.
- **Pair to learn:** Define → alternatives → decide → implement → evaluate → D'Zurilla & Goldfried

68 — B

Areas of Clinical Focus · Outline 3.U · Chapter 14

- **B. Career Thoughts Inventory — Owner: Sampson et al. (CIP).** The Career Thoughts Inventory, from Sampson and colleagues, measures negative career thoughts within CIP.
- **A. Career Maturity Inventory** — Owner: Crites (CMI). The CMI measures career readiness attitudes and competencies.
- **C. Career Decision Scale** — Owner: Osipow (Career Decision Scale). The Career Decision Scale measures career indecision.
- **D. Career Beliefs Inventory** — Owner: Krumboltz (Career Beliefs Inventory). Krumboltz's Career Beliefs Inventory assesses beliefs that block career progress, outside the CIP framework.
- **Pair to learn:** Negative career thoughts (CIP) → Career Thoughts Inventory (not Krumboltz's Career Beliefs Inventory)

69 — C

Areas of Clinical Focus · Outline 3.AD · Chapter 12

- **C. Detachment — Owner: Bowlby (with Robertson): protest, despair, detachment.** Bowlby and Robertson described separated young children moving from protest to despair to detachment; seeming indifference to the returning parent is detachment.

- **A. Anxious-avoidant attachment** — Owner: Ainsworth (Strange Situation). Anxious-avoidant is an Ainsworth attachment classification, not a phase of response to prolonged separation.
- **B. Despair** — Owner: Bowlby (with Robertson): protest, despair, detachment. Despair is the earlier listless, sad phase; his current indifference marks the next phase.
- **D. Acceptance** — Owner: Kübler-Ross (stages of dying). Acceptance is one of Kübler-Ross's reactions to dying, not a child's separation response.
- **Pair to learn:** Protest → despair → detachment → Bowlby & Robertson (not Kübler-Ross)

70 — A

Intake, Assessment & Diagnosis · Outline 2.O · Chapter 19

- **A. The PC-PTSD-5 — Owner: Trauma screen (2.O).** The PC-PTSD-5 is one of the trauma screens counselors use, along with the LEC-5, PCL-5, and ACEs.
- **B. The CRAFFT** — Owner: Adolescent substance use screen. The CRAFFT screens adolescents for substance use, not trauma.
- **C. The MoCA** — Owner: Cognitive screen (MoCA). The MoCA screens cognition, for example in older adults.
- **D. The GAD-7** — Owner: Anxiety symptom screen. The GAD-7 screens for generalized anxiety, not posttraumatic stress.
- **Pair to learn:** Trauma screen at intake → PC-PTSD-5 (not GAD-7)

71 — A

Core Counseling Attributes · Outline 6.M · Chapter 4

- **A. A soft "Mm-hmm… go on," with an attentive posture — Owner: Minimal encourager (Ivey's basic listening sequence).** Minimal encouragers

signal attention and keep the client talking without directing the content.

- **B. "So far you've told me about work stress and poor sleep."** — Owner: Summary. Summarizing earlier themes pulls her away from what she is about to say.
- **C. "I wonder if this connects to how your family kept secrets."** — Owner: Interpretation. Interpreting before she has spoken is premature and steers the content.
- **D. "Is it about your marriage?"** — Owner: Closed question (leading). A guess steers her toward the counselor's topic.
- **Pair to learn:** Keep the client talking without directing → Minimal encourager

72 — B

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **B. Arnold Lazarus — Owner: Lazarus (multimodal therapy, BASIC I.D.).** BASIC I.D. is Lazarus's seven-modality assessment in multimodal therapy.
- **A. William Glasser** — Owner: Glasser (reality therapy). Glasser's total behavior has four parts: acting, thinking, feeling, and physiology.
- **C. Donald Meichenbaum** — Owner: Meichenbaum (cognitive behavior modification). Meichenbaum focuses on self-talk and stress inoculation.
- **D. Aaron Beck** — Owner: Beck (cognitive therapy). Beck assesses automatic thoughts and schemas, not seven modalities.
- **Pair to learn:** BASIC I.D. → Lazarus (not Glasser's total behavior)

73 — D

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **D. Narrative therapy — Owner: White & Epston (narrative).** Definitional ceremonies and outsider witnesses are listed among narrative therapy's signature techniques.
- **A. Gestalt therapy** — Owner: Perls (Gestalt). Gestalt's group technique is making the rounds, not outsider witnesses.
- **B. Solution-focused brief therapy** — Owner: de Shazer & Berg (SFBT). SFBT's techniques are the miracle, exception, scaling, and coping questions, compliments, and the formula task.
- **C. Structural family therapy** — Owner: Minuchin (structural). Structural techniques are joining, enactment, boundary making, and unbalancing.
- **Pair to learn:** Outsider witnesses; definitional ceremonies → White & Epston (narrative)

74 — D

Counseling Skills & Interventions · Outline 5.J · Chapter 7

- **D. Negative feedback — Owner: Systems theory (morphostasis).** Negative feedback reduces deviation and maintains stability; 'negative' describes direction, not value.
- **A. Multifinality** — Owner: Systems theory. Multifinality means the same start can lead to different outcomes.
- **B. Positive feedback** — Owner: Systems theory (morphogenesis). Positive feedback amplifies deviation and pushes the system toward change.
- **C. Morphogenesis** — Owner: Systems theory. Morphogenesis is change in the system's structure, the opposite of staying the same.
- **Pair to learn:** Deviation-reducing feedback → negative feedback (morphostasis)

75 — A

Areas of Clinical Focus · Outline 3.AO · Chapter 13

- **A. Mobilization — Owner: Papernow (Stepfamily Cycle).** Papernow's middle phase consists of mobilization and then action, following the early stages of fantasy, immersion, and awareness.
- **B. Contact** — Owner: Papernow (Stepfamily Cycle). Contact begins the later phase, followed by resolution.
- **C. Awareness** — Owner: Papernow (Stepfamily Cycle). Awareness is the last stage of the early phase, not the first of the middle phase.
- **D. Storming** — Owner: Tuckman (group development). Storming is a Tuckman group stage, not part of the Stepfamily Cycle.
- **Pair to learn:** Middle phase begins with mobilization → Papernow (not Tuckman)

76 — C

Counseling Skills & Interventions · Outline 5.J · Chapter 7

- **C. David Olson — Owner: Olson (Circumplex Model; FACES).** Cohesion and flexibility are the two dimensions of Olson's Circumplex Model.
- **A. Salvador Minuchin** — Owner: Minuchin (structural). Minuchin speaks of boundaries and enmeshment, but the two-dimension model is Olson's.
- **B. W. Robert Beavers** — Owner: Beavers (Beavers Systems Model). Beavers assesses family competence and style.
- **D. Nathan Epstein** — Owner: Epstein (McMaster Model). The McMaster Model uses six dimensions, including roles and behavior control.
- **Pair to learn:** Cohesion + flexibility → Olson's Circumplex (not McMaster)

77 — C

Areas of Clinical Focus · Outline 3.V · Chapter 10

- **C. Make sure he gets a medical evaluation — Owner: Medical rule-out for anxiety symptoms (3.V).** A first panic-like attack, especially with chest pain, warrants medical evaluation because cardiac and other conditions can cause these symptoms.
- **A. Diagnose panic disorder** — Owner: DSM-5-TR panic disorder. Panic disorder requires recurrent unexpected attacks and at least 1 month of worry or behavior change.
- **B. Begin interoceptive exposure for his panic** — Owner: Barlow (panic control treatment). Interoceptive exposure treats panic disorder; starting it before ruling out a cardiac cause is premature.
- **D. Teach applied tension** — Owner: Öst (applied tension). Applied tension prevents fainting in blood-injection-injury phobia; it does not address possible chest pain.
- **Pair to learn:** First panic-like attack with chest pain → medical evaluation first

78 — B

Professional Practice & Ethics · Outline 1.B · Chapter 18

- **B. Cost-effectiveness analysis — Owner: Cost-effectiveness analysis (program evaluation).** Cost-effectiveness analysis relates costs to nonmonetary outcomes, like cost per client improved.
- **A. Process evaluation** — Owner: Process evaluation (fidelity). Process evaluation asks whether the program was delivered as planned, not what it costs per outcome.
- **C. Needs assessment** — Owner: Needs assessment (before program design). A needs assessment identifies needs and gaps before a program is designed.
- **D. Cost-benefit analysis** — Owner: Cost-benefit analysis (program evaluation). Cost-benefit analysis

measures the benefits in dollars, not in units such as clients improved.

- **Pair to learn:** Cost per nonmonetary outcome → Cost-effectiveness analysis (not cost-benefit)

79 — D

Professional Practice & Ethics · Outline 1.F · Chapter 17

- **D. May keep it confidential after consulting and reviewing the law — Owner: End-of-life decisions (ACA B.2.b).** B.2.b gives counselors the option to maintain confidentiality for terminally ill clients considering hastening death, after consultation and depending on law and circumstances.
- **A. Must break confidentiality, as she would for any other suicide risk** — Owner: Serious and foreseeable harm (ACA B.2.a). B.2.a permits disclosure to prevent serious and foreseeable harm, but B.2.b specifically gives counselors an option with terminally ill clients.
- **B. Should end counseling, since the decision is outside its scope** — Owner: Abandonment (ACA A. 12). Ending services at such a moment abandons the client when support matters most.
- **C. Should encourage him to proceed, out of respect for autonomy** — Owner: Autonomy (misapplied as imposing a view). Autonomy means respecting the client's own choices, not steering him toward one; encouraging the decision imposes the counselor's view.
- **Pair to learn:** Terminally ill client considering hastening death → Option to keep confidentiality after consultation (B.2.b)

80 — B

Counseling Skills & Interventions · Outline 5.AE · Chapter 4

- **B. Reflect the sadness she is showing as she talks — Owner: Reflection of feeling (validate before**

challenging). First session plus acute distress calls for reflecting the feeling; confrontation waits for an established alliance.

- **A. Point out that her words and her tears don't match** — Owner: Confrontation (premature). Confrontation is wrong in a first session and when the client is in acute distress.
- **C. Suggest that the breakup may echo an earlier loss** — Owner: Interpretation (premature). Interpretation is wrong early in counseling, before there is evidence and readiness.
- **D. Reassure her that breakups get easier with time** — Owner: Reassurance. Reassurance dismisses the feeling.
- **Pair to learn:** First session + distress → reflect, don't confront yet

81 — A

Counseling Skills & Interventions · Outline 5.H · Chapter 9

- **A. Microinsult — Owner: Sue (racial microaggressions).** A subtle remark that conveys surprise at a person's achievement based on identity is a microinsult.
- **B. Cultural encapsulation** — Owner: Wrenn (1962). Encapsulation is a counselor substituting their own cultural assumptions for reality, not a single remark.
- **C. Microinvalidation** — Owner: Sue (racial microaggressions). A microinvalidation negates a person's experience, such as "I don't see color."
- **D. Microassault** — Owner: Sue (racial microaggressions). A microassault is explicit and often conscious, such as a slur.
- **Pair to learn:** Surprise at achievement based on identity → microinsult (Sue)

82 — A

Professional Practice & Ethics · Outline 1.B · Chapter 18

- **A. Factorial ANOVA — Owner: Factorial ANOVA (two or more IVs; interaction effects).** Two independent variables with a question about their interaction call for a factorial ANOVA, such as a 2 × 2 design.
- **B. One-way ANOVA** — Owner: One-way ANOVA (one IV, three or more groups). A one-way ANOVA handles only one independent variable and can't test an interaction.
- **C. MANOVA** — Owner: MANOVA (two or more dependent variables). MANOVA is for two or more dependent variables; this study has one outcome, depression scores.
- **D. ANCOVA** — Owner: ANCOVA (means compared while controlling a covariate). ANCOVA adjusts for a covariate; gender here is a second independent variable whose interaction is of interest.
- **Pair to learn:** Two or more IVs and their interaction → Factorial ANOVA

83 — D

Counseling Skills & Interventions · Outline 5.AU · Chapter 8

- **D. Experiencing, then illuminating — Owner: Yalom (here-and-now).** The group first experiences the here-and-now, then illuminates it by reflecting on what just happened.
- **A. Storming, then norming** — Owner: Tuckman (group stages). Storming and norming are Tuckman's group stages.
- **B. Illuminating, then experiencing** — Owner: Yalom (order reversed). This reverses Yalom's sequence.
- **C. Catharsis, then insight** — Owner: Yalom: catharsis (therapeutic factor). Catharsis is a therapeutic factor, not a tier of here-and-now work.

- **Pair to learn:** Here-and-now: experience, then illuminate (Yalom)

84 — B

Counseling Skills & Interventions · Outline 5.AW · Chapter 8

- **B. During informed consent, before the group begins — Owner: Group rules set at the start (1.N, 5.AW).** Expectations about outside contact belong in informed consent, set before they are needed.
- **A. Only after a subgroup has already formed** — Owner: Reactive rule-setting. Waiting until a subgroup forms leaves members without a norm when it matters.
- **C. Never, since members' private lives are their own** — Owner: Ignoring outside contact. Outside contact can create subgroups and secrets, so it must be addressed.
- **D. In the final stage, as members plan to stay in touch** — Owner: Final stage tasks. The final stage consolidates learning; outside-contact norms belong at the start.
- **Pair to learn:** Outside-contact expectations → set during informed consent

85 — A

Professional Practice & Ethics · Outline 1.C · Chapter 17

- **A. Report it, since substantial harm to the client is likely — Owner: Reporting colleague violations (ACA I.2.b).** When a violation has caused or is likely to cause substantial harm, counselors take further action, such as reporting to the licensing board.
- **B. Talk with him privately and let him end it on his own terms** — Owner: Informal resolution (ACA I. 2.a). Informal resolution fits minor violations; a sexual relationship with a current client is likely to cause substantial harm.
- **C. Contact his client directly to warn her about the risk** — Owner: Breach of confidentiality and

proper process. Reaching out to the colleague's client bypasses proper channels and intrudes on that client's care.

- **D. Keep his confidence, since he told her this as a friend** — Owner: Ignoring a harmful violation (punished). Friendship doesn't outweigh the duty to protect clients; ignoring a harmful violation is what the exam punishes.
- **Pair to learn:** Colleague violation with substantial harm → Report (I.2.b), not informal resolution

86 — C

Areas of Clinical Focus · Outline 3.H · Chapter 10

- **C. Preparedness — Owner: Seligman (preparedness).** Seligman proposed that people are biologically prepared to fear some things, such as snakes and heights, more easily than others.
- **A. Two-factor theory** — Owner: Mowrer (two-factor theory). Two-factor theory explains how fears are acquired and maintained, not why some objects are feared more readily.
- **B. Catastrophic misinterpretation** — Owner: Clark (cognitive model of panic). Catastrophic misinterpretation is misreading body sensations in panic, not object fears.
- **D. Yerkes-Dodson law** — Owner: Yerkes & Dodson. Yerkes-Dodson describes performance rising with arousal up to a point, then declining.
- **Pair to learn:** Biologically easy-to-learn fears → preparedness (Seligman), not Mowrer

87 — C

Core Counseling Attributes · Outline 6.C · Chapter 22

- **C. Not pretending, and sharing reactions when that serves the client — Owner: Congruence (Rogers).** Congruence means inner experience matches outward communication; it does not mean saying everything one feels.

- **A. Openly telling the client everything one feels, moment to moment** — Owner: Venting the counselor's feelings (not congruence). Congruence does not require disclosing every reaction; sharing is guided by the client's benefit.
- **B. Valuing the client as a person without any conditions** — Owner: Unconditional positive regard (Rogers). Valuing the person without conditions is unconditional positive regard.
- **D. Sensing the client's experience, including what is unspoken** — Owner: Empathic attunement. Sensing the client's unspoken experience is empathic attunement.
- **Pair to learn:** Congruence is not saying everything you feel → Share reactions when it serves the client (Rogers)

88 — B

Counseling Skills & Interventions · Outline 5.AB · Chapter 4

- **B. Reflect her concern and ask directly about her safety — Owner: Safety first (never reframe abuse as care).** When harm is described, safety comes first; a reframe must never minimize real harm.
- **A. Suggest couples sessions to improve their communication** — Owner: Conjoint work when violence is present (contraindicated). Couples counseling is contraindicated with ongoing violence.
- **C. Reframe the shoving as a sign of how much he cares** — Owner: Reframe that minimizes harm. Reframing abuse as care is the misuse of reframing the exam punishes.
- **D. Explore how money was handled in her family growing up** — Owner: Exploring history before safety. Exploring the past before assessing safety reverses the priority.
- **Pair to learn:** Never reframe abuse as care; assess safety first

89 — C

Areas of Clinical Focus · Outline 3.Z · Chapter 3

- **C. Disintegration — Owner: Helms (White racial identity).** Helms' order is contact, disintegration, reintegration, pseudo-independence, immersion/emersion, autonomy.
- **A. Reintegration** — Owner: Helms (White racial identity). Reintegration follows disintegration, so it is two steps after contact.
- **B. Dissonance** — Owner: Atkinson, Morten, & Sue (R/CID). Dissonance is the second stage of the Racial/Cultural Identity Development model.
- **D. Encounter** — Owner: Cross (nigrescence). Encounter is the second stage of Cross's model of Black racial identity.
- **Pair to learn:** Contact → disintegration → reintegration → Helms (not Cross)

90 — C

Areas of Clinical Focus · Outline 3.T · Chapter 10

- **C. OCD with absent insight/delusional beliefs — Owner: DSM-5-TR OCD insight specifier.** She has compulsions driven by an obsession; full conviction is captured by OCD's absent insight/delusional beliefs specifier.
- **A. Delusional disorder** — Owner: DSM-5-TR delusional disorder. Delusional disorder involves delusions without compulsions; her washing to prevent harm is an OCD pattern.
- **B. Generalized anxiety disorder** — Owner: DSM-5-TR GAD. GAD worry concerns real-life matters and doesn't drive compulsions like hours of washing.
- **D. Obsessive-compulsive personality disorder** — Owner: OCPD (ego-syntonic perfectionism). OCPD is a pervasive pattern of perfectionism and control without true obsessions or compulsions.

- **Pair to learn:** Full conviction plus compulsions → OCD with absent insight (not delusional disorder)

91 — B

Professional Practice & Ethics · Outline 1.S · Chapter 17

- **B. Assess whether the client intends to inform the partner — Owner: Contagious, life-threatening diseases (ACA B.2.c, 2014).** Before disclosing, counselors assess the client's intent to inform the third party or to engage in harmful behavior, and follow state law.
- **A. Apply the Tarasoff duty to warn standard for violent threats** — Owner: Tarasoff I (duty to warn of violence). Tarasoff concerns serious threats of physical violence; disease transmission falls under B.2.c and state law.
- **C. Confirm the diagnosis with the client's physician** — Owner: 2005 ACA Code requirement (dropped in 2014). Confirming the diagnosis was a step in the 2005 Code; the 2014 Code removed it.
- **D. Obtain a court order authorizing the disclosure** — Owner: Court-ordered disclosure (ACA B.2.d). B.2.d governs what to do when a court orders disclosure; B. 2.c doesn't require a court order.
- **Pair to learn:** Before disclosing contagious disease → Assess client's intent to inform (B.2.c, 2014)

92 — B

Treatment Planning · Outline 4.A · Chapter 21

- **B. Goals he didn't help choose conflict with his autonomy — Owner: Collaborative goal setting (4.A); autonomy.** Goals the counselor sets alone are less likely to be met and conflict with the client's autonomy.
- **A. Goals can't be set until a full ASAM review is done** — Owner: ASAM Criteria (substance use level of care). ASAM guides level-of-care placement for

substance use; it isn't required before every treatment plan.

- **C. The plan lists no barriers that could block his progress** — Owner: Barriers (4.C). Barriers do belong in a plan, but the central flaw is that the client had no voice in his own goals.
- **D. The plan should first be sent to his insurer for review** — Owner: Third-party payers (ACA B.3.d; disclose only with authorization). Sharing with an insurer requires the client's authorization and doesn't address the missing collaboration.
- **Pair to learn:** Counselor-written goals → Collaboration and client autonomy (4.A)

93 — B

Treatment Planning · Outline 4.J · Chapter 21

- **B. Booster sessions — Owner: Booster sessions (follow-up after discharge, 4.J).** Follow-up contacts after ending that support maintenance are booster sessions.
- **A. Tapered sessions** — Owner: Tapering (reducing frequency before ending). Tapering spaces out sessions before termination; these contacts happen after counseling has ended.
- **C. Step-down care** — Owner: Step down (level-of-care change, 4.E). Stepping down moves a client to a less intensive level of care during treatment, not after goals are met.
- **D. Relapse prevention** — Owner: Relapse prevention (termination task). Relapse prevention is the plan for warning signs and coping, built before ending; the stem describes scheduled follow-up contacts.
- **Pair to learn:** Scheduled check-ins after ending → Booster sessions (4.J)

94 — C

Areas of Clinical Focus · Outline 3.J · Chapter 15

- **C. Richard Troiden — Owner: Troiden (sexual identity model).** Troiden's model moves from sensitization through identity confusion and identity assumption to commitment.
- **A. Vivienne Cass** — Owner: Cass (homosexual identity formation). Cass's stages run from identity confusion to synthesis, with comparison, tolerance, acceptance, and pride between.
- **B. Anthony D'Augelli** — Owner: D'Augelli (life-span LGB model). D'Augelli described six interrelated processes rather than fixed stages.
- **D. Arlene Istar Lev** — Owner: Lev (transgender emergence). Lev's model describes transgender emergence, from awareness to integration and acceptance.
- **Pair to learn:** Sensitization → commitment → Troiden (not Cass)

95 — C

Core Counseling Attributes · Outline 6.E · Chapter 22

- **C. Derald Wing Sue — Owner: Sue: multicultural competence.** Derald Wing Sue is the owner most associated with multicultural competence.
- **A. Heinz Kohut** — Owner: Kohut: self psychology. Kohut described empathy as vicarious introspection.
- **B. Robert Carkhuff** — Owner: Carkhuff: five-level empathy scale. Carkhuff built the five-level empathy scale.
- **D. Edward Bordin** — Owner: Bordin: working alliance. Bordin defined the alliance as goals, tasks, and bond.
- **Pair to learn:** Multicultural competence → Derald Wing Sue (not Kohut)

96 — B

Professional Practice & Ethics · Outline 1.D · Chapter 17

- **B. Must still use forethought and document it isn't exploitative — Owner: Former clients after 5 years (ACA A.5.c).** After 5 years, A.5.c still requires forethought and documentation that the relationship isn't exploitative, and avoiding it if exploitation is possible.
- **A. Needs only the former client's written consent to proceed** — Owner: Client consent (doesn't override A.5.c). Consent doesn't satisfy A.5.c; the counselor must still weigh and document the risk of exploitation.
- **C. Is permanently barred from dating this former client** — Owner: Previous partners rule (ACA A.5.b), misapplied. The permanent bar in A.5.b concerns counseling a former romantic partner; A.5.c sets a 5-year limit, not a lifetime ban, on dating former clients.
- **D. Is free to proceed, since well over 2 years have passed** — Owner: 2-year time frame (not the ACA standard). The ACA standard is 5 years, not 2, and even after 5 years conditions still apply.
- **Pair to learn:** Former client after 5 years → Forethought and documentation still required (A.5.c)

97 — D

Counseling Skills & Interventions · Outline 5.AV · Chapter 8

- **D. Block it, protect him, and explore what the group avoids — Owner: Scapegoating response (ACA A.9.b protecting members).** Scapegoating calls for blocking and protecting the target, then exploring what the group is avoiding.
- **A. Ask him what he could change to fit in with the group** — Owner: Blaming the target. This joins the group in blaming the scapegoated member.
- **B. Let members work it out, since conflict is normal in groups** — Owner: Leaving the member

unprotected. Normal conflict is not scapegoating; leaving it alone fails to protect the target.

- **C. Suggest he move to individual counseling instead** — Owner: Removing the target. Removing him punishes the target and leaves the group's pattern untouched.
- **Pair to learn:** Group targets one member → scapegoating: block and protect

98 — C

Professional Practice & Ethics · Outline 1.W · Chapter 17

- **C. Fix the draft before signing; the note is her responsibility — Owner: Counselor responsibility for AI output (ACA AI Work Group; Ch 17 Part 11).** The counselor is responsible for every note, including AI-drafted ones, so she must review and fix inaccurate content before it becomes part of the record.
- **A. Sign it as drafted, since the tool meets HIPAA requirements** — Owner: HIPAA compliance (privacy and security, not accuracy). HIPAA compliance addresses privacy and security; it doesn't make the tool's clinical content accurate or shift responsibility from the counselor.
- **B. Sign it now and add a dated correction to the note afterward** — Owner: Error correction rule for finalized records (dated correction). Dated corrections fix errors discovered in finalized entries; knowingly signing an inaccurate note is not the way to use that rule.
- **D. Ask the client to read and approve the AI draft for accuracy** — Owner: Client access to records (ACA B.6.e). Clients have reasonable access to records, but approving notes is the counselor's job, not a substitute for her own review.
- **Pair to learn:** AI-drafted note → Counselor reviews and is responsible for it

99 — D

Counseling Skills & Interventions · Outline 5.E · Chapter 4

- **D. Name the shift gently and ask how the task landed — Owner: Repairing a withdrawal rupture (Safran & Muran).** Agreeing too easily, short answers, and subject changes signal a withdrawal rupture; name it non-defensively and explore it.
- **A. Remind her that homework is part of their agreement** — Owner: Defending the counselor's plan. Defending the plan escalates a rupture instead of repairing it.
- **B. Continue with the plan, since she has agreed to it** — Owner: Ignoring the rupture. Pressing on ignores the signs that the alliance has strained.
- **C. Refer her to a counselor who assigns less homework** — Owner: Referral instead of repair. Referral abandons the alliance; a repaired rupture often strengthens it.
- **Pair to learn:** Agrees too easily, goes quiet → withdrawal rupture: name and explore (Safran & Muran)

100 — C

Core Counseling Attributes · Outline 6.H · Chapter 22

- **C. The fifth condition — Owner: Rogers (1957): condition 5.** The fifth condition is the counselor's empathic understanding and the attempt to communicate it.
- **A. The sixth condition** — Owner: Rogers (1957): condition 6. The sixth condition is that the client perceives the empathy and regard.
- **B. The fourth condition** — Owner: Rogers (1957): condition 4. The fourth condition is unconditional positive regard.

- **D. The third condition** — Owner: Rogers (1957): condition 3. The third condition is counselor congruence.
- **Pair to learn:** Counselor experiences and communicates empathic understanding → Rogers' condition 5

101 — A

Counseling Skills & Interventions · Outline 5.AG · Chapter 7

- **A. Taking responsibility — Owner: Gottman (antidote to defensiveness).** Defensiveness is one of the Four Horsemen; its antidote is taking responsibility.
- **B. A gentle startup** — Owner: Gottman (antidote to criticism). Gentle startup is the antidote to criticism.
- **C. A culture of appreciation** — Owner: Gottman (antidote to contempt). Building appreciation is the antidote to contempt.
- **D. Physiological self-soothing** — Owner: Gottman (antidote to stonewalling). Self-soothing is the antidote to stonewalling.
- **Pair to learn:** Defensiveness → antidote: taking responsibility (Gottman)

102 — A

Intake, Assessment & Diagnosis · Outline 2.C · Chapter 19

- **A. Cultural factors affecting current help-seeking — Owner: CFI domain 4.** The fourth domain covers current help-seeking, including concerns about the counselor-client relationship.
- **B. Cultural factors affecting self-coping and past help-seeking** — Owner: CFI domain 3. The third domain looks back at coping and earlier help-seeking, not concerns about this counselor.
- **C. Cultural definition of the problem** — Owner: CFI domain 1. The first domain asks how the client

describes the problem, for example to family and friends.

- **D. Cultural perceptions of cause, context, and support** — Owner: CFI domain 2. The second domain asks why the client thinks this is happening and what makes it better or worse.
- **Pair to learn:** Concerns about the counselor-client relationship → CFI domain 4 (current help-seeking)

103 — A

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **A. Donald Winnicott — Owner: Winnicott (object relations).** The good-enough mother and the holding environment are Winnicott's concepts.
- **B. John Bowlby** — Owner: Bowlby (attachment). Bowlby owns the secure base and internal working models.
- **C. Margaret Mahler** — Owner: Mahler (object relations). Mahler owns separation-individuation.
- **D. Melanie Klein** — Owner: Klein (object relations). Klein is an object relations founder, but these two terms are Winnicott's.
- **Pair to learn:** Good-enough mother; holding environment → Winnicott (not Bowlby)

104 — B

Counseling Skills & Interventions · Outline 5.X · Chapter 9

- **B. H.3, client verification — Owner: ACA Code H.3.** H.3 requires verifying the client's identity at the start and throughout distance counseling.
- **A. H.4.c, technology-assisted services** — Owner: ACA Code H.4.c. H.4.c covers whether the client can use the technology and whether it fits the client's needs, not identity verification.

- **C. H.6.a, virtual professional presence** — Owner: ACA Code H.6.a. H.6.a covers separate professional and personal social media accounts.
- **D. H.1.b, laws and statutes** — Owner: ACA Code H. 1.b. H.1.b covers following the laws of the counselor's and client's locations.
- **Pair to learn:** Code words to confirm identity → ACA H.3 (client verification)

105 — B

Professional Practice & Ethics · Outline 1.K · Chapter 17

- **B. Contact the client and assert privilege on the client's behalf — Owner: Subpoena response (assert privilege; Jaffee v. Redmond, privileged communication).** A subpoena isn't a court order; the counselor contacts the client (and/or the client's attorney), seeks legal guidance, and asserts the client's privilege if it hasn't been waived.
- **A. Disregard the subpoena, since the client hasn't given consent** — Owner: Ignoring a subpoena (legal risk). Ignoring a subpoena can bring legal consequences; the counselor responds by asserting privilege, not by doing nothing.
- **C. Send only a brief treatment summary to limit what is disclosed** — Owner: Minimal disclosure (ACA B.2.e). Minimal disclosure applies once disclosure is actually required; nothing has yet authorized any release.
- **D. Send the records, since a subpoena is a legal order to comply** — Owner: Court order (judge-signed), confused with a subpoena. Compliance with minimal disclosure applies to a judge-signed court order; releasing records on a subpoena without consent is what the exam punishes.
- **Pair to learn:** Attorney-issued subpoena → Contact client, assert privilege

106 — D

Areas of Clinical Focus · Outline 3.M · Chapter 16

- **D. John Cacioppo — Owner: Cacioppo (loneliness and social cognition).** Cacioppo showed that loneliness changes social thinking, heightening alertness to social threat.
- **A. Julianne Holt-Lunstad** — Owner: Holt-Lunstad (isolation and mortality). Holt-Lunstad's research links isolation and loneliness to higher mortality risk.
- **B. Laura Carstensen** — Owner: Carstensen (socioemotional selectivity). Carstensen described pruning relationships as time horizons shorten.
- **C. Robert Weiss** — Owner: Weiss (emotional vs. social loneliness). Weiss distinguished emotional loneliness from social loneliness.
- **Pair to learn:** Loneliness heightens alertness to social threat → Cacioppo (not Holt-Lunstad)

107 — A

Core Counseling Attributes · Outline 6.J · Chapter 22

- **A. "You expect me to judge you. I'd rather understand what it's been like." — Owner: Nonjudgmental stance (6.J).** It names the client's fear and shows acceptance by turning toward understanding, without evaluation.
- **B. "Well, using does hurt the people who care about you."** — Owner: Moralizing. Lecturing confirms the judgment the client feared.
- **C. "I'm so sorry you feel that way. That must be awful."** — Owner: Sympathy (not empathy). Feeling sorry for the client comes from the counselor's frame.
- **D. "Of course not! You're a wonderful person, and you shouldn't think that way."** — Owner: Reassurance. Reassurance tells the client how to feel and skips past the shame.
- **Pair to learn:** Client expects judgment → Nonjudgmental stance (not reassurance or moralizing)

108 — B

Intake, Assessment & Diagnosis · Outline 2.C · Chapter 19

- **B. The CFI informant version — Owner: DSM-5-TR Cultural Formulation Interview, informant version.** The CFI has an informant version for family members, gathering their cultural view of the problem.
- **A. A 4 Ps formulation of the daughter's report** — Owner: 4 Ps case formulation. The 4 Ps organize intake information into predisposing, precipitating, perpetuating, and protective factors; they are not a cultural interview.
- **C. The WHODAS 2.0, filled out by the daughter** — Owner: WHODAS 2.0 (functioning). The WHODAS 2.0 measures functioning, not how the family culturally understands the problem.
- **D. The SCID-5, given to the daughter** — Owner: Structured interview (SCID-5). The SCID-5 is a structured diagnostic interview about symptoms, not a family member's cultural perspective.
- **Pair to learn:** Family member's cultural view of the problem → CFI informant version

109 — A

Areas of Clinical Focus · Outline 3.L · Chapter 10

- **A. Reformulated helplessness theory — Owner: Abramson, Seligman, & Teasdale (attributional theory).** The reformulated (attributional) theory says depression follows attributing bad events to internal, stable, and global causes, exactly what he does.
- **B. Response styles theory** — Owner: Nolen-Hoeksema (response styles). Response styles theory concerns rumination prolonging depression, not the internal, stable, global explanation he gives.
- **C. Hopelessness theory** — Owner: Abramson, Metalsky, & Alloy (hopelessness theory). Hopelessness theory centers on expecting bad outcomes with no way to change them, a related but different subtype model.

- **D. Original learned helplessness theory** — Owner: Seligman (learned helplessness). Seligman's original theory focused on uncontrollable events teaching that action is useless, not on attributional style.
- **Pair to learn:** Internal, stable, global attributions → Abramson, Seligman, & Teasdale (not Lewinsohn or Nolen-Hoeksema)

110 — D

Core Counseling Attributes · Outline 6.B · Chapter 22

- **D. Gerald Corey — Owner: Corey: personal characteristics of effective counselors.** Corey describes effective counselors as authentic, open to feedback, willing to admit mistakes, and having a sense of humor, among other traits.
- **A. Truax and Carkhuff** — Owner: Truax & Carkhuff: accurate empathy, nonpossessive warmth, genuineness. They operationalized three core conditions, not a list of personal characteristics.
- **B. Jeffrey Hayes and Charles Gelso** — Owner: Hayes & Gelso: managing countertransference. Their factors (self-insight, self-integration, anxiety management) concern managing countertransference.
- **C. Norcross and Lambert** — Owner: APA task forces on evidence-based therapy relationships. They evaluated relationship elements such as the alliance and empathy, not counselor personality traits.
- **Pair to learn:** Personal characteristics of effective counselors → Corey (not Truax & Carkhuff)

111 — B

Areas of Clinical Focus · Outline 3.W · Chapter 10

- **B. Fatigue, psychomotor slowing, low libido, slowed thinking — Owner: Physical signs of depression (3.W).** Fatigue, psychomotor slowing, low libido, and slowed thinking are listed physical signs of depression.

- **A. Pinpoint pupils, drowsiness, slowed breathing** — Owner: Opioid intoxication. Pinpoint pupils and slowed breathing signal opioid intoxication.
- **C. Racing heart, shortness of breath, trembling, sweating** — Owner: Physical signs of anxiety (3.V). These are the classic physical signs of anxiety and panic.
- **D. Dilated pupils, rapid heart rate, heavy sweating, paranoia** — Owner: Stimulant intoxication. This pattern points to stimulant intoxication, not depression.
- **Pair to learn:** Fatigue, psychomotor slowing, low libido, slowed thinking → physical signs of depression (not anxiety)

112 — C

Areas of Clinical Focus · Outline 3.AC · Chapter 10

- **C. Depressive rumination — Owner: Nolen-Hoeksema (rumination).** Rumination dwells on past losses and one's own distress; it isn't usually experienced as intrusive and doesn't drive compulsions.
- **A. Future-focused worry** — Owner: GAD worry. Worry is "what if" thinking about possible future outcomes, not replaying a past loss.
- **B. Intrusive trauma memories** — Owner: PTSD (intrusion cluster). Intrusive memories are tied to a specific traumatic event, not to dwelling on why a marriage ended.
- **D. Ego-dystonic obsessions** — Owner: OCD (obsessions). Obsessions feel intrusive and alien and usually prompt neutralizing; his thoughts do neither.
- **Pair to learn:** Dwelling on past loss, not alien, no neutralizing → depressive rumination (not obsession)

113 — C

Professional Practice & Ethics · Outline 1.F · Chapter 17

- **C. She will protect it and ask members to, but can't guarantee it — Owner: Confidentiality in groups (ACA B.4).** Counselors explain that confidentiality can't be guaranteed in groups, while clarifying their own commitment and asking members to respect it.
- **A. A signed group agreement guarantees that nothing will leave** — Owner: Overpromising confidentiality. A signed agreement encourages privacy but can't guarantee it; promising otherwise misinforms members.
- **B. Privileged communication protects what members say in court** — Owner: Privileged communication (legal, client-held). Privilege is a legal right that varies by state and doesn't guarantee that group members keep what they hear private.
- **D. Confidentiality doesn't apply to group work, so share carefully** — Owner: Understating the leader's duty. The leader still has a duty of confidentiality; the limit is that she can't control other members.
- **Pair to learn:** Group privacy → Can't be guaranteed (ACA B.4)

114 — B

Areas of Clinical Focus · Outline 3.E · Chapter 16

- **B. Ambiguous loss — Owner: Pauline Boss.** Boss's ambiguous loss includes a person who is physically present but psychologically absent, as with a parent with addiction.
- **A. Caregiver burden** — Owner: Zarit Burden Interview. Caregiver burden is the broader strain of caregiving, not the specific grief for someone present but absent.

- **C. Disenfranchised grief** — Owner: Doka. Disenfranchised grief is loss that isn't socially recognized, not grief for someone still present.
- **D. Anticipatory grief** — Owner: Lindemann. Anticipatory grief prepares for an expected death; his grief is for who his father has already become.
- **Pair to learn:** Physically present, psychologically absent → ambiguous loss (Boss) in caregiving

115 — D

Areas of Clinical Focus · Outline 3.AR · Chapter 13

- **D. Fatuous love — Owner: Sternberg (triangular theory: passion + commitment).** In Sternberg's triangle, passion plus commitment without intimacy is fatuous love.
- **A. Romantic love** — Owner: Sternberg (intimacy + passion). Romantic love combines intimacy and passion; this couple lacks intimacy but has commitment.
- **B. Anxious adult attachment** — Owner: Hazan & Shaver (adult attachment). Adult attachment style describes relational security, not the components present in this relationship.
- **C. Eros** — Owner: Lee (love styles). Eros is Lee's passionate love style and doesn't account for the commitment-without-intimacy pattern.
- **Pair to learn:** Passion + commitment, no intimacy → fatuous love (Sternberg, not Lee)

116 — D

Areas of Clinical Focus · Outline 3.AW · Chapter 13

- **D. Carolyn Webster-Stratton — Owner: Webster-Stratton (Incredible Years).** Webster-Stratton developed the Incredible Years parenting program.
- **A. Gerald Patterson** — Owner: Patterson (coercion theory). Patterson developed coercion theory, the basis for parent management training.

- **B. Dinkmeyer and McKay** — Owner: Dinkmeyer & McKay (STEP). Dinkmeyer and McKay developed STEP, based on Adlerian principles.
- **C. Matthew Sanders** — Owner: Sanders (Triple P). Sanders developed Triple P, the Positive Parenting Program.
- **Pair to learn:** Incredible Years → Webster-Stratton (not Sanders' Triple P)

117 — B

Counseling Skills & Interventions · Outline 5.K · Chapter 7

- **B. "When Maya stays home, who notices first? What does Dad do?" — Owner: Milan (circular questioning).** Circular questions put the pattern, not one member, at the center.
- **A. "Why do you think Maya keeps refusing to go to school lately?"** — Owner: Linear "why" question. A linear question locates the cause inside one member.
- **C. "How many school days has Maya missed this month?"** — Owner: Closed question. A closed question gathers a fact but not the family pattern.
- **D. "What is it about Maya that makes school so hard?"** — Owner: Identified patient as 'the problem'. This treats the IP as the problem rather than looking at the system.
- **Pair to learn:** Who notices, what happens next → circular question (Milan)

118 — D

Areas of Clinical Focus · Outline 3.AS · Chapter 13

- **D. A reconciliation fantasy common at school age — Owner: Children's reactions to divorce by age.** Reconciliation fantasies, along with loyalty conflicts and sadness, are common reactions in school-age children of divorce.
- **A. Self-blame and regression typical of preschoolers** — Owner: Preschool reactions to

divorce. Self-blame and regression are typical of preschoolers, and neither is described here.

- **B. Bargaining, as in Kübler-Ross's stages** — Owner: Kübler-Ross (stages of dying). Bargaining is a reaction described in terminally ill patients, not a divorce reaction model.
- **C. Wallerstein's sleeper effect** — Owner: Wallerstein (sleeper effect). The sleeper effect describes effects appearing in adulthood, not a child's current wish for reunion.
- **Pair to learn:** Plans to reunite parents → school-age reconciliation fantasy

119 — D

Areas of Clinical Focus · Outline 3.AX · Chapter 10

- **D. Linehan's biosocial theory — Owner: Linehan (biosocial theory; DBT).** Linehan's biosocial theory pairs biological emotional sensitivity with an invalidating environment as the source of dysregulation.
- **A. Mowrer's two-factor theory** — Owner: Mowrer (two-factor theory). Mowrer explains fear acquisition and maintenance through conditioning.
- **B. Siegel's window of tolerance** — Owner: Siegel (window of tolerance). Siegel describes the arousal zone for functioning, not an invalidating environment as a cause.
- **C. Gross's process model** — Owner: Gross (process model of emotion regulation). Gross describes points where emotion can be regulated, not how dysregulation develops.
- **Pair to learn:** Biosocial theory of dysregulation → Linehan (not Gross)

120 — A

Counseling Skills & Interventions · Outline 5.H · Chapter 9

- **A. Arrange a trained professional interpreter — Owner: Professional interpreter (best practice).** A trained professional interpreter protects accuracy, confidentiality, and family roles.
- **B. Ask a bilingual receptionist to help out** — Owner: Untrained interpreter. An untrained staff member raises accuracy and confidentiality concerns.
- **C. Proceed with simple English and gestures** — Owner: Inadequate communication. Simplified English and gestures don't provide adequate communication.
- **D. Let the daughter interpret, since she knows the family** — Owner: Child as interpreter. Using a child compromises accuracy, confidentiality, and family roles.
- **Pair to learn:** Interpretation → trained professional, never a child

121 — D

Counseling Skills & Interventions · Outline 5.C · Chapter 9

- **D. Moral injury — Owner: Veterans: moral injury.** Distress after acts that violate one's own moral code is moral injury.
- **A. Traumatic brain injury** — Owner: Veterans: TBI. TBI is a physical brain injury.
- **B. Military sexual trauma** — Owner: Veterans: MST. MST involves sexual assault or harassment during service.
- **C. Disenfranchised grief** — Owner: Doka. Disenfranchised grief is loss that isn't socially recognized.
- **Pair to learn:** Violated one's own moral code → moral injury

122 — A

Counseling Skills & Interventions · Outline 5.G · Chapter 9

- **A. Contingency management — Owner: Contingency management.** Contingency management provides tangible rewards for verified abstinence.
- **B. CRAFT** — Owner: Meyers (CRAFT). CRAFT trains family members to encourage a loved one into treatment.
- **C. Community Reinforcement Approach** — Owner: Community Reinforcement Approach (CRA). CRA works to make a sober lifestyle more rewarding than use.
- **D. 12-step facilitation** — Owner: 12-step facilitation. 12-step facilitation encourages active involvement in AA or NA.
- **Pair to learn:** Rewards for verified abstinence → contingency management (not CRA)

123 — D

Intake, Assessment & Diagnosis · Outline 2.B · Chapter 19

- **D. The principal diagnosis — Owner: Principal diagnosis (DSM-5-TR).** The principal diagnosis is the condition chiefly responsible for the admission, and it is listed first.
- **A. The Axis I diagnosis** — Owner: DSM-IV multiaxial system (eliminated in DSM-5). The DSM-5 eliminated the multiaxial system, so Axis I is no longer current practice.
- **B. The provisional diagnosis** — Owner: Provisional diagnosis. Provisional is used when criteria are expected to be met but information is incomplete; it does not mark the reason for the admission.
- **C. The diagnostic specifier** — Owner: Specifier (e.g., severity, “with anxious distress”). Specifiers add detail

to a diagnosis; they do not identify which condition prompted the admission.

- **Pair to learn:** Chiefly responsible for the admission, listed first → Principal diagnosis (not Axis I)

124 — B

Counseling Skills & Interventions · Outline 5.Q · Chapter 6

- **B. Edwin Shneidman — Owner: Shneidman (suicidology).** Shneidman, the founder of suicidology, introduced psychache.
- **A. Émile Durkheim** — Owner: Durkheim (sociological types of suicide). Durkheim described egoistic, altruistic, anomic, and fatalistic suicide.
- **C. Aaron Beck** — Owner: Beck (hopelessness). Beck linked hopelessness to suicide risk.
- **D. Thomas Joiner** — Owner: Joiner (interpersonal theory of suicide). Joiner owns thwarted belongingness, perceived burdensomeness, and acquired capability.
- **Pair to learn:** Psychache → Shneidman (not Joiner)

125 — A

Intake, Assessment & Diagnosis · Outline 2.T · Chapter 20

- **A. The change is within one SEM, so it may reflect measurement error — Owner: SEM confidence band (interpret scores as ranges).** The 68% band around 30 is 27 to 33, and 28 falls inside it, so the two-point drop may be error rather than real change.
- **B. The measure's test-retest reliability must be near zero** — Owner: Test-retest reliability. A small change says nothing by itself about stability; the issue is measurement error around each score.
- **C. A ceiling effect on the measure is hiding the client's real change** — Owner: Ceiling effect (test too easy; many score at the top). Nothing suggests scores are bunched at the top of the scale.

- **D. The two-point drop confirms that counseling worked** — Owner: Treating scores as exact points (ignores SEM). Every observed score has error; a change smaller than one SEM does not confirm improvement.
- **Pair to learn:** Change smaller than the SEM → May be measurement error; interpret scores as ranges

126 — D

Treatment Planning · Outline 4.G · Chapter 21

- **D. Assessing lethality and safety planning with her — Owner: Safety first (sequencing priorities; Campbell's lethality indicators).** Safety comes first in sequencing; prior strangulation is a high lethality indicator, so assessment and safety planning come before other goals.
- **A. Exploring how her family history shapes her choices** — Owner: Longer-term growth (sequencing step 5). Insight work is the last priority, well after safety.
- **B. Inviting her partner to join couples sessions** — Owner: Couples counseling (contraindicated with ongoing IPV). Conjoint work is contraindicated while violence is ongoing and could increase her danger.
- **C. Assertiveness training for her concerns at work** — Owner: Client's most pressing stated concern (sequencing step 4). Her stated concern matters, but it comes after safety.
- **Pair to learn:** Danger at home → Safety first in sequencing

127 — A

Intake, Assessment & Diagnosis · Outline 2.K · Chapter 19

- **A. "How are these symptoms affecting your work and daily life?" — Owner: Functioning (2.K): impact on work, relationships, and self-care.** Functioning is judged by how symptoms affect daily

activities such as work, school, relationships, and self-care.

- **B. "Can you tell me today's date and where we are right now?"** — Owner: MSE cognition: orientation. Orientation questions check cognition in the MSE, not daily functioning.
- **C. "How would you describe this problem to your family and friends?"** — Owner: CFI domain 1 (cultural definition of the problem). This CFI question explores the client's cultural definition of the problem, not functioning.
- **D. "From 0 to 100, how upsetting does this feel right now?"** — Owner: SUDS (distress, 2.J). A SUDS rating measures subjective distress, which can differ sharply from actual functioning.
- **Pair to learn:** Impact on work and daily life → Functioning (2.K), not distress (SUDS)

128 — D

Treatment Planning · Outline 4.B · Chapter 21

- **D. Client will score below 10 on the PHQ-9 within 12 weeks — Owner: Short-term objective (measurable step).** It is a measurable, time-bound step toward the long-term goal.
- **A. Client reports low mood and misses work two days a week** — Owner: Problem statement (concern in behavioral terms). This describes the presenting concern in behavioral terms, which is the problem statement.
- **B. Client will experience reduced depression overall** — Owner: Long-term goal (broad outcome). This is a broad outcome, which describes a long-term goal rather than a measurable step.
- **C. Behavioral activation and cognitive restructuring weekly** — Owner: Interventions (what the counselor will do). This names what the counselor will do, which belongs under interventions.

- **Pair to learn:** Measurable, time-bound step → Short-term objective

129 — C

Areas of Clinical Focus · Outline 3.A · Chapter 16

- **C. The spread effect — Owner: Beatrice Wright (disability psychology).** Wright's spread effect is assuming that one limitation means global limitation.
- **A. Cultural imperialism** — Owner: Iris Marion Young (faces of oppression). Cultural imperialism is one of Young's faces of oppression, not the specific error of generalizing one limitation.
- **B. Impostor phenomenon** — Owner: Clance & Imes. The impostor phenomenon is a high achiever feeling like a fraud.
- **D. Stereotype threat** — Owner: Steele & Aronson. Stereotype threat affects the target's own performance; the error here is the manager's assumption.
- **Pair to learn:** Assuming one limitation means global limitation → spread (Beatrice Wright)

130 — C

Treatment Planning · Outline 4.A · Chapter 21

- **C. Respect that value and explore how to involve them — Owner: Respecting cultural values in goal setting (4.A).** Collaboration includes respecting cultural values about family involvement in goals and help-seeking.
- **A. Encourage her to set her own goals independently first** — Owner: Cultural encapsulation (Wrenn). Pushing an individualistic standard substitutes the counselor's cultural assumptions for the client's.
- **B. Let her parents set her goals for her instead** — Owner: Overriding client autonomy. Involving her family doesn't mean handing them the decision; she still chooses her goals.

- **D. List her family's role as a barrier in the plan** — Owner: Barriers (4.C), misapplied. Family resistance can be a barrier, but here her family is a valued resource she wants included.
- **Pair to learn:** Collectivist family involvement → Respect cultural values in goals (4.A)

131 — C

Treatment Planning · Outline 4.J · Chapter 21

- **C. A follow-up appointment within days, with safety planning — Owner: Follow-up after discharge (4.J; elevated post-discharge risk).** The days and weeks after discharge carry elevated suicide risk, so prompt follow-up and safety planning matter most.
- **A. Completing the discharge summary before any contact** — Owner: Discharge summary (documentation). A discharge summary documents care; it isn't the clinical priority for keeping the client safe.
- **B. A first outpatient appointment sometime within the next month** — Owner: Delayed follow-up during a high-risk period. Waiting weeks leaves the client unsupported during the period of highest risk.
- **D. Waiting for the client to call once she feels ready** — Owner: Passive follow-up (no plan). Leaving contact up to the client provides no plan during an elevated-risk period.
- **Pair to learn:** High-risk period after hospital discharge → Prompt follow-up

132 — B

Counseling Skills & Interventions · Outline 5.AF · Chapter 4

- **B. Here-and-now awareness — Owner: Perls (Gestalt here-and-now).** Directing attention to present experience is the Gestalt here-and-now focus; it says nothing about the counselor-client relationship.

- **A. Immediacy** — Owner: Immediacy (microskill). Immediacy addresses what is happening between counselor and client right now.
- **C. An interpretation** — Owner: Interpretation (microskill). Interpretation proposes a connection outside the client's awareness.
- **D. A reflection of meaning** — Owner: Reflection of meaning (microskill). Reflection of meaning names a value or belief underneath.
- **Pair to learn:** Present-moment awareness, no relationship comment → here-and-now (Gestalt), not immediacy

133 — C

Areas of Clinical Focus · Outline 3.U · Chapter 14

- **C. Jahoda's latent deprivation model — Owner: Jahoda (latent deprivation).** Jahoda argued that work provides latent benefits such as time structure, social contact, and purpose, whose loss harms well-being even when income is secure.
- **A. Holland's congruence** — Owner: Holland (congruence). Congruence is fit between a person's type and the work environment.
- **B. Maslach's burnout model** — Owner: Maslach (burnout). Burnout comes from chronic workplace stress, not from losing the structure work provided.
- **D. Atchley's retirement phases** — Owner: Atchley (retirement). Atchley's phases describe retirement, not distress after an involuntary job loss.
- **Pair to learn:** Lost time structure, contact, purpose → Jahoda's latent deprivation (not Maslach)

134 — C

Areas of Clinical Focus · Outline 3.C · Chapter 16

- **C. Oppositional defiant disorder — Owner: DSM-5-TR ODD.** ODD involves angry or irritable mood, argumentative or defiant behavior, or vindictiveness, with at least 4 symptoms for 6 months.

- **A. Disruptive mood dysregulation disorder** — Owner: DSM-5-TR DMDD. DMDD requires severe outbursts 3 or more times a week with persistent irritability for 12 months.
- **B. Conduct disorder** — Owner: DSM-5-TR conduct disorder. Conduct disorder involves violating others' rights, such as aggression, destruction, or theft, which are absent.
- **D. Intermittent explosive disorder** — Owner: DSM-5-TR IED. IED centers on recurrent aggressive outbursts out of proportion to triggers, not ongoing defiance.
- **Pair to learn:** Angry, defiant, 4+ symptoms, 6 months, no rights violations → ODD (not conduct disorder)

135 — C

Professional Practice & Ethics · Outline 1.V · Chapter 17

- **C. Vicarious liability** — **Owner: Vicarious liability (respondeat superior).** Under vicarious liability, supervisors can be held legally responsible for their supervisees' actions.
- **A. Direct liability** — Owner: Direct liability (supervisor's own negligent supervision). Direct liability applies when the supervisor's own supervision was negligent, which the stem rules out.
- **B. Dereliction of duty** — Owner: Malpractice element: dereliction (breach). Dereliction is the breach element of a malpractice claim, not the doctrine that assigns responsibility to a supervisor.
- **D. Direct causation** — Owner: Malpractice element: direct causation. Direct causation is the malpractice element linking a breach to the harm, not a doctrine of supervisor responsibility.
- **Pair to learn:** Supervisor liable for supervisee's acts → Vicarious liability

136 — B

Counseling Skills & Interventions · Outline 5.AM · Chapter 7

- **B. Virginia Satir — Owner: Satir (human validation / experiential).** Family reconstruction is one of Satir's signature techniques for family-of-origin work.
- **A. Murray Bowen** — Owner: Bowen (multigenerational). Bowen's family-of-origin tools are the genogram, process questions, and coaching differentiation.
- **C. Carl Whitaker** — Owner: Whitaker (symbolic-experiential). Whitaker is known for co-therapy, playfulness, and absurdity.
- **D. Salvador Minuchin** — Owner: Minuchin (structural). Minuchin works on present structure: boundaries, subsystems, hierarchy.
- **Pair to learn:** Family reconstruction → Satir (not Bowen)

137 — B

Areas of Clinical Focus · Outline 3.AP · Chapter 13

- **B. Powerlessness — Owner: Finkelhor & Browne (traumagenic dynamics).** Powerlessness is the dynamic in which the child's will and sense of control are repeatedly overridden.
- **A. Stigmatization** — Owner: Finkelhor & Browne (traumagenic dynamics). Stigmatization involves shame and feeling "damaged," not the inability to stop the abuse.
- **C. Betrayal** — Owner: Finkelhor & Browne (traumagenic dynamics). Betrayal concerns broken trust in someone the child depended on, not overridden control.
- **D. Shattered assumptions** — Owner: Janoff-Bulman (shattered assumptions). Shattered assumptions is

Janoff-Bulman's concept of broken beliefs about the world, not one of the traumagenic dynamics.

- **Pair to learn:** Will and control repeatedly overridden → powerlessness (Finkelhor & Browne)

138 — A

Core Counseling Attributes · Outline 6.H · Chapter 4

- **A. "With your car out again, you're really worried about getting to work." — Owner: Carkhuff level 3 (interchangeable).** It captures the same content (the breakdown) and feeling (worry about getting to work) the client expressed, without adding or subtracting.
- **B. "You're worried about getting to work, and maybe tired of always being one breakdown from falling behind."** — Owner: Carkhuff level 4 (additive). It adds a meaning just below the surface (being tired of living one setback from falling behind), which makes it level 4, not interchangeable.
- **C. "Cars always seem to break down at the worst time."** — Owner: Carkhuff level 2 (subtracts). A generalization about cars loses the client's specific worry about getting to work.
- **D. "Have you looked into a bus pass or asking a coworker for a ride?"** — Owner: Carkhuff level 1 (advice; ignores the feeling). Jumping to transportation solutions ignores the client's feeling.
- **Pair to learn:** Same content and feeling → Carkhuff level 3 (interchangeable), not level 4

139 — D

Intake, Assessment & Diagnosis · Outline 2.E · Chapter 19

- **D. Other specified anxiety disorder — Owner: Other specified disorder (DSM-5-TR).** "Other specified" is used when the clinician records why full criteria are not met.
- **A. Panic disorder (provisional)** — Owner: Provisional diagnosis. Provisional assumes full criteria

will likely be met once more information arrives; here the symptom count itself falls short.

- **B. Unspecified anxiety disorder** — Owner: Unspecified disorder (DSM-5-TR). “Unspecified” is chosen when the clinician cannot, or chooses not to, state why criteria are not met; this counselor wants to state it.
- **C. Malingering, recorded as a Z code** — Owner: Malingering (Z code). Nothing suggests intentional production of symptoms for an external reward.
- **Pair to learn:** Clinician states why criteria aren’t met → Other specified (not unspecified)

140 — C

Counseling Skills & Interventions · Outline 5.AO · Chapter 8

- **C. Respect her choice; she may observe and share reactions — Owner: Voluntary participation in structured exercises.** Members can decline to participate in an exercise.
- **A. Require her to join so the exercise works as planned** — Owner: Coerced participation. Coercion violates voluntary participation.
- **B. Drop structured exercises for the rest of the group** — Owner: Overcorrection. One member declining doesn’t mean exercises lack purpose for the group.
- **D. Ask her to wait outside the room while the others role-play** — Owner: Excluding a member. Excluding her harms safety and cohesion.
- **Pair to learn:** Members can decline structured exercises

141 — D

Areas of Clinical Focus · Outline 3.U · Chapter 14

- **D. Recycling into the exploration stage — Owner: Super (recycling).** Super noted that people can re-

enter earlier stages, such as re-exploring after a midlife layoff.

- **A. Updating in the maintenance stage** — Owner: Super (maintenance: holding, updating, innovating). Updating keeps skills current within a held position; he has left that position and is exploring anew.
- **B. Chronic career indecisiveness** — Owner: Indecisiveness (chronic, generalized). Indecisiveness is a long-standing difficulty deciding across life areas, not a response to a layoff.
- **C. Disenchantment after a role ends** — Owner: Atchley (retirement phases). Disenchantment is Atchley's post-honeymoon letdown in retirement, not career re-exploration.
- **Pair to learn:** Re-entering exploration after midlife layoff → recycling (Super)

142 — B

Core Counseling Attributes · Outline 6.E · Chapter 22

- **B. Treat it as a hypothesis and ask the client how it fits her — Owner: Cultural humility (6.E).** Cultural humility pairs knowledge with curiosity and awareness of its limits, so the client confirms what fits.
- **A. Build the treatment plan around the values typical of that group** — Owner: Stereotyping (assumes the group describes the individual). Planning from group generalizations imposes assumptions on this client.
- **C. Refer the client to a counselor who shares her background** — Owner: Premature referral. Cultural difference alone is not a reason to refer.
- **D. Set it aside and avoid raising culture so she doesn't seem biased** — Owner: Ignoring culture. Avoiding culture misses its influence; the attribute calls for broaching it respectfully.
- **Pair to learn:** Group knowledge as a hypothesis, checked with the client → Cultural humility (not stereotyping)

143 — A

Counseling Skills & Interventions · Outline 5.AQ · Chapter 8

- **A. Listen non-defensively and invite her to raise it in group — Owner: Encouraging direct member-leader interaction (transition stage).** Leaders respond to challenges non-defensively and encourage members to express reactions directly in the group.
- **B. Explain the reasons for each interruption so she understands** — Owner: Defending the leader. Explaining or justifying is a defensive response to a challenge.
- **C. Agree to keep the complaint between the two of them** — Owner: Creating a secret with one member. A private side agreement keeps the reaction out of the group's process.
- **D. Promise her extra time next session to make up for it** — Owner: Special arrangement outside the group. A private bargain sidesteps exploring the reaction in the group.
- **Pair to learn:** Member's reaction to the leader → receive non-defensively, bring it into group

144 — A

Areas of Clinical Focus · Outline 3.G · Chapter 12

- **A. Dame Cicely Saunders — Owner: Saunders (modern hospice movement).** Dame Cicely Saunders founded the modern hospice movement, which provides comfort care to people no longer seeking curative treatment.
- **B. Elisabeth Kübler-Ross** — Owner: Kübler-Ross (stages of dying). Kübler-Ross described reactions of terminally ill patients; she did not found the hospice movement.
- **C. Robert Butler** — Owner: Butler (ageism; life review). Butler coined "ageism" and developed life review, not the hospice movement.

- **D. Erich Lindemann** — Owner: Lindemann (acute and anticipatory grief). Lindemann described acute grief reactions and anticipatory grief, not hospice care.
- **Pair to learn:** Modern hospice movement → Cicely Saunders (not Kübler-Ross)

145 — A

Treatment Planning · Outline 4.E · Chapter 21

- **A. Stepping up to intensive outpatient or partial hospitalization — Owner: Step up to least restrictive higher level of care (4.E).** Worsening symptoms that outpatient care isn't handling call for stepping up to the least restrictive level that's safe and effective, chosen with the client.
- **B. Reducing sessions to monthly to build her independence** — Owner: Step down (for clients who are stabilizing). Stepping down fits a client who is improving, not one who is getting worse.
- **C. Continuing weekly sessions under the current treatment plan** — Owner: Continuing an ineffective plan unchanged (punished). Repeating a plan that isn't working is what the exam punishes.
- **D. Inpatient hospitalization, given how poorly she is functioning** — Owner: Inpatient care (over-restrictive without imminent risk). Without imminent risk, inpatient care is more restrictive than needed.
- **Pair to learn:** Worsening, no imminent risk → Step up to IOP or PHP

146 — D

Counseling Skills & Interventions · Outline 5.V · Chapter 6

- **D. Assess suicide and medical risk, then build coping skills — Owner: NSSI response (assessment plus DBT-type skills).** NSSI still requires suicide and medical assessment, followed by work on alternative coping.

- **A. Make continued counseling conditional on her stopping now** — Owner: Ultimatum (not recommended for NSSI). Making counseling conditional on stopping harms the alliance and builds no skills.
- **B. Arrange an inpatient admission today to keep her safe** — Owner: Over-response without assessment. Hospitalizing before assessing risk is not matched to the level of risk.
- **C. Skip suicide assessment, since she has no wish to die** — Owner: Assuming NSSI rules out suicide risk. NSSI and suicide risk can coexist; NSSI is linked to higher later risk.
- **Pair to learn:** NSSI → assess suicide and medical risk, build coping; no ultimatums

147 — A

Treatment Planning · Outline 4.H · Chapter 21

- **A. May end services, with safety steps and referrals — Owner: Termination when the counselor is in danger (ACA A.11.c).** A.11.c permits termination when the counselor is in danger from the client, with appropriate safety steps and referrals to other providers.
- **B. Must continue, since ending would be abandonment** — Owner: Abandonment (ACA A.12), misapplied. Ending services for a legitimate A.11.c reason, with referrals, is not abandonment.
- **C. May end services only once his balance is paid** — Owner: Termination for nonpayment rule. Payment has nothing to do with this situation; the counselor's safety is the basis for ending.
- **D. Must warn his family under the Tarasoff duty to protect** — Owner: Duty to protect (Tarasoff II). The duty to protect concerns a third party at risk; it doesn't require warning the client's family here.
- **Pair to learn:** Counselor in danger from client → Termination permitted (A.11.c)

148 — A

Core Counseling Attributes · Outline 6.F · Chapter 22

- **A. "I may have missed something. Help me see her the way you do." — Owner: Conflict tolerance: explore non-defensively (6.F).** It stays present with the disagreement and explores the client's view without defending or retreating.
- **B. "It sounds like you're in denial about your relationship with her."** — Owner: Interpretation used as a counterattack. Labeling the disagreement as denial is defensive and premature.
- **C. "I've been doing this a long time, and the pattern is clear."** — Owner: Defending the counselor. Defending credentials escalates the conflict and dismisses the client's view.
- **D. "You're right, I'm so sorry. I won't bring her up again."** — Owner: Appeasing (avoiding conflict). Appeasing avoids the conflict instead of exploring it, and closes off a possibly important topic.
- **Pair to learn:** Client disagrees with the counselor → Explore non-defensively (not defend or appease)

149 — A

Treatment Planning · Outline 4.L · Chapter 21

- **A. Reassess his readiness and revise the plan with him — Owner: Check readiness when progress stalls (4.L; stages of change).** When progress stalls, the counselor checks whether the client's stage of change has shifted and adjusts the plan collaboratively.
- **B. Switch to a new approach without discussing it** — Owner: Switching approaches without collaboration (punished). Changing approaches without discussion is what the exam punishes.
- **C. Keep the current plan and remind him of his original goal** — Owner: Continuing an ineffective plan unchanged. Continuing unchanged ignores what the client just said about his readiness.

- **D. Confront his denial so that he will recommit** — Owner: Confrontational approach in addiction work (punished). Confrontation tends to increase resistance; the exam rewards exploring readiness instead.
- **Pair to learn:** Readiness has shifted → Reassess stage of change, revise plan

150 — D

Treatment Planning · Outline 4.N · Chapter 21

- **D. BIRP — Owner: BIRP (Behavior, Intervention, Response, Plan).** BIRP covers the client's behavior (presentation), the counselor's intervention, the client's response, and the plan.
- **A. DAP** — Owner: DAP (Data, Assessment, Plan). DAP has no separate intervention or response section.
- **B. GIRP** — Owner: GIRP (Goal, Intervention, Response, Plan). GIRP opens with the treatment goal, not the client's presentation.
- **C. SOAP** — Owner: SOAP (Subjective, Objective, Assessment, Plan). SOAP separates the client's report from the counselor's observations; it has no intervention or response section.
- **Pair to learn:** Behavior, intervention, response, plan → BIRP

151 — D

Counseling Skills & Interventions · Outline 5.AP · Chapter 8

- **D. "Could you say that directly to Ray?" — Owner: Facilitating member-to-member interaction.** Facilitating encourages members to talk to each other, not just to the leader.
- **A. "Ray, please let her finish from now on."** — Owner: Leader-centered rule-setting. The leader settles it for them instead of promoting direct interaction.
- **B. "It sounds like you feel dismissed by him."** — Owner: Reflection of feeling (individual counseling in

group). A reflection keeps the exchange between member and leader.

- **C. "Has anyone else here felt dismissed?"** — Owner: Universalizing. Universalizing shifts to a shared theme and away from the direct exchange.
- **Pair to learn:** Tell me → tell Ray directly: facilitating member interaction

152 — B

Areas of Clinical Focus · Outline 3.AG · Chapter 15

- **B. Terror management theory — Owner: Greenberg, Solomon, & Pyszczynski.** Terror management theory proposes that death awareness motivates people to hold onto worldviews and self-esteem as buffers.
- **A. Neurotic anxiety** — Owner: Rollo May (normal vs. neurotic anxiety). May distinguished normal from neurotic anxiety; this isn't a theory of worldview defense.
- **C. Spiritual bypassing** — Owner: Welwood. Spiritual bypassing uses spiritual ideas to avoid emotional pain.
- **D. Viktor Frankl's will to meaning** — Owner: Frankl (logotherapy). Frankl's will to meaning is the drive to find meaning, including in suffering, not worldview defense.
- **Pair to learn:** Fear of death drives worldview defense → terror management theory (not Yalom or Frankl)

153 — B

Areas of Clinical Focus · Outline 3.U · Chapter 14

- **B. My Vocational Situation — Owner: Holland (My Vocational Situation).** My Vocational Situation measures vocational identity, need for information, and barriers.
- **A. Self-Directed Search** — Owner: Holland (SDS). The SDS yields a self-scored RIASEC code, not a measure of vocational identity.

- **C. Career Maturity Inventory** — Owner: Crites (CMI). The CMI measures career readiness attitudes and competencies.
- **D. Minnesota Importance Questionnaire** — Owner: Dawis & Lofquist (MIQ). The MIQ measures work values and needs.
- **Pair to learn:** Vocational identity → My Vocational Situation (Holland)

154 — A

Areas of Clinical Focus · Outline 3.K · Chapter 12

- **A. Therese Rando — Owner: Rando (six R processes).** Rando described six "R" processes of mourning, each beginning with the letter R.
- **B. J. William Worden** — Owner: Worden (tasks of mourning). Worden described four tasks of mourning, from accepting the reality of the loss to finding an enduring connection.
- **C. Robert Neimeyer** — Owner: Neimeyer (meaning reconstruction). Neimeyer is known for meaning reconstruction after loss, not a set of R processes.
- **D. John Bowlby** — Owner: Bowlby (phases of mourning). Bowlby described phases of numbness, yearning and searching, disorganization and despair, and reorganization.
- **Pair to learn:** Six R processes → Rando (not Worden)

155 — C

Counseling Skills & Interventions · Outline 5.V · Chapter 6

- **C. Warning signs that a crisis may be starting — Owner: Stanley & Brown: step 1.** Step 1 is warning signs: thoughts, moods, situations, or behaviors that signal a crisis is starting.
- **A. Professionals and agencies to contact for help** — Owner: Stanley & Brown: step 5. Professionals, crisis lines, and 988 are step 5.

- **B. Internal coping strategies to use when alone** — Owner: Stanley & Brown: step 2. Internal coping strategies are step 2.
- **D. Ways to make the environment safer** — Owner: Stanley & Brown: step 6. Reducing access to lethal means is the final step.
- **Pair to learn:** Safety plan step 1 → warning signs (Stanley & Brown)

156 — D

Core Counseling Attributes · Outline 6.F · Chapter 22

- **D. Safran and Muran** — **Owner: Safran & Muran: alliance ruptures and repair.** Safran and Muran described withdrawal and confrontation ruptures and showed that repair often strengthens the alliance.
- **A. Robert Carkhuff** — Owner: Carkhuff: five-level empathy scale. Carkhuff rated empathic responses, not ruptures.
- **B. Edward Bordin** — Owner: Bordin: working alliance (goals, tasks, bond). Bordin defined the parts of the alliance, not the rupture-and-repair model.
- **C. Gerard Egan** — Owner: Egan: SOLER and The Skilled Helper. Egan owns SOLER and a three-stage helping model.
- **Pair to learn:** Repaired rupture strengthens the alliance → Safran & Muran (not Bordin)

157 — D

Areas of Clinical Focus · Outline 3.AK · Chapter 12

- **D. Seek consultation, then weigh keeping it confidential** — **Owner: ACA Code B.2.b.** B.2.b gives counselors of terminally ill clients considering hastening death the option to maintain confidentiality, depending on law and circumstances, after consulting professional and legal parties.
- **A. Break confidentiality now, as with any suicidal client** — Owner: Standard duty to protect (Chapter 6).

B.2.b sets a different standard for terminally ill clients; automatic disclosure ignores it and the legal context.

- **B. Redirect her to life review and legacy work** — Owner: Life review (Butler). Life review can help later, but steering away from her disclosure avoids the decision the code addresses.
- **C. Encourage her to proceed, since it is legal** — Owner: Imposing counselor values (ACA A.4.b). Encouraging either choice imposes the counselor's view; she still needs careful assessment for depression, pain, and coercion.
- **Pair to learn:** Terminally ill client considering hastening death → consult, option to keep confidentiality (ACA B.2.b)

158 — D

Intake, Assessment & Diagnosis · Outline 2.F · Chapter 19

- **D. An illusion — Owner: MSE perception: illusion (misperceived real stimulus).** An illusion is a misperception of a real stimulus, here the sound of the vent.
- **A. Thought blocking** — Owner: MSE thought process: thought blocking. Thought blocking is a disruption in the flow of thinking, not a sensory experience.
- **B. An auditory hallucination** — Owner: MSE perception: hallucination (no stimulus). A hallucination occurs with no stimulus present; this experience began with a real sound.
- **C. A delusion** — Owner: MSE thought content: delusion. A delusion is a fixed false belief; she corrected her perception as soon as she listened closely.
- **Pair to learn:** Misperceived real sound → Illusion (not hallucination)

159 — A

Areas of Clinical Focus · Outline 3.U · Chapter 3

- **A. Outcome expectations — Owner: Lent, Brown, & Hackett (SCCT).** In SCCT, outcome expectations are beliefs about what will result from an action; she doubts the payoff, not her ability.
- **B. Task-approach skills** — Owner: Krumboltz (social learning theory). Task-approach skills are Krumboltz's decision-making skills, not beliefs about results.
- **C. Self-efficacy** — Owner: Lent, Brown, & Hackett (SCCT); Bandura. Self-efficacy is belief in one's ability, which she expresses confidently.
- **D. Satisfactoriness** — Owner: Dawis & Lofquist (TWA). Satisfactoriness is the employer's view of the worker's fit, not the student's expectations.
- **Pair to learn:** Belief about what an action will lead to → outcome expectations (SCCT, not Krumboltz)

160 — C

Areas of Clinical Focus · Outline 3.J · Chapter 3

- **C. Identity tolerance — Owner: Cass (homosexual identity formation).** Cass's order is confusion, comparison, tolerance, acceptance, pride, synthesis, so tolerance follows comparison.
- **A. Moratorium** — Owner: Marcia (identity statuses). Moratorium is Marcia's status of exploration without commitment, not a stage of sexual identity formation.
- **B. Immersion-emersion** — Owner: Cross (nigrescence). Immersion-emersion belongs to Cross's model of Black racial identity.
- **D. Identity acceptance** — Owner: Cass (homosexual identity formation). Acceptance comes after tolerance in Cass's sequence, not directly after comparison.
- **Pair to learn:** Confusion → comparison → tolerance → acceptance → pride → synthesis → Cass (not Marcia)

161 — B

Core Counseling Attributes · Outline 6.G · Chapter 22

- **B. Empathic attunement — Owner: 6.G: empathic attunement (internal resonance).** Responding without attunement sounds formulaic; attunement is the internal sensing that gives reflections life.
- **A. Verbal tracking** — Owner: Ivey: attending (verbal tracking). The counselor is staying on the client's topic, so tracking is present.
- **C. Unconditional positive regard** — Owner: Rogers' condition 4. Nothing suggests the counselor values the client conditionally.
- **D. Psychological contact** — Owner: Rogers' condition 1. The two are clearly in contact; the problem is how the responses feel.
- **Pair to learn:** Accurate but formulaic reflections → Missing empathic attunement (6.G)

162 — A

Intake, Assessment & Diagnosis · Outline 2.P · Chapter 11

- **A. Arrange medical evaluation; withdrawal could be life-threatening — Owner: Alcohol withdrawal is potentially life-threatening (assess withdrawal risk, 2.P).** Alcohol withdrawal can cause seizures and delirium tremens, so stopping heavy daily drinking calls for medical evaluation and managed withdrawal.
- **B. Encourage him to attend AA meetings every day during the first week** — Owner: Alcoholics Anonymous (Bill W. and Dr. Bob). Mutual support can help, but it does not address the medical risk of stopping abruptly.
- **C. Explore his ambivalence with a decisional balance exercise** — Owner: Motivational interviewing / stages of change (Prochaska & DiClemente). He has already decided; the priority is making withdrawal medically safe, not building motivation.

- **D. Affirm his decision and schedule a relapse prevention session** — Owner: Relapse prevention (Marlatt). Relapse prevention matters later, but it skips the immediate medical danger of withdrawal.
- **Pair to learn:** Heavy daily drinker stopping abruptly → Medical evaluation (alcohol withdrawal can be dangerous)

163 — C

Areas of Clinical Focus · Outline 3.Y · Chapter 11

- **C. DSM-5-TR lists it for further study; ICD-11 lists it — Owner: DSM-5-TR Section III; ICD-11 gaming disorder.** Internet gaming disorder is a Section III condition for further study in the DSM-5-TR, while ICD-11 includes gaming disorder as a disorder due to addictive behaviors.
- **A. ICD-11 lists it as an impulse control disorder** — Owner: ICD-11 compulsive sexual behaviour disorder. ICD-11 places compulsive sexual behaviour disorder among impulse control disorders; gaming disorder is a disorder due to addictive behaviors.
- **B. Neither the DSM-5-TR nor ICD-11 addresses it** — Owner: Misstatement. Both systems address it: one as a condition for further study, the other as a disorder.
- **D. It is an official DSM-5-TR addictive disorder** — Owner: Gambling disorder (only DSM behavioral addiction). Gambling disorder is the only behavioral addiction in the DSM-5-TR addictions chapter.
- **Pair to learn:** Internet gaming disorder → DSM-5-TR condition for further study; ICD-11 gaming disorder

164 — B

Areas of Clinical Focus · Outline 3.AH · Chapter 14

- **B. The depersonalization dimension of burnout — Owner: Maslach (burnout dimensions).** In Maslach's model, depersonalization means cynicism and detachment from clients or work.

- **A. Depersonalization as a dissociative symptom** — Owner: Dissociative symptom (3.N). Dissociative depersonalization is feeling detached from oneself, not cynicism toward clients.
- **C. Reduced personal accomplishment** — Owner: Maslach (burnout dimensions). Reduced accomplishment is feeling ineffective, a different burnout dimension than detachment.
- **D. Vicarious trauma** — Owner: McCann & Pearlman (vicarious trauma). Vicarious trauma involves lasting changes in beliefs from repeated exposure to clients' trauma.
- **Pair to learn:** Cynicism and detachment from clients → depersonalization (Maslach burnout), not dissociation

165 — C

Counseling Skills & Interventions · Outline 5.Q · Chapter 6

- **C. Warn an identifiable victim of a serious threat — Owner: Tarasoff I (1974): duty to warn.** Tarasoff I established the duty to warn an identifiable victim.
- **A. Act for a foreseeable victim based on violent history** — Owner: Jablonski v. United States (1983). Jablonski extended the duty to foreseeable victims without an explicit threat.
- **B. Act on a threat reported by a client's family member** — Owner: Ewing v. Goldstein (2004). Ewing held that a family member's report of a credible threat can trigger the duty.
- **D. Take reasonable steps to protect an intended victim** — Owner: Tarasoff II (1976): duty to protect. The broader duty to protect came from the 1976 decision.
- **Pair to learn:** Duty to warn (1974) → Tarasoff I; duty to protect (1976) → Tarasoff II

166 — D

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **D. Top dog and underdog — Owner: Perls (Gestalt).** Top dog/underdog is a Gestalt key term (Perls), and Gestalt works with such splits through techniques like two-chair dialogue.
- **A. Persona and shadow** — Owner: Jung (analytical psychology). Persona and shadow are Jung's archetypal terms, not Gestalt's.
- **B. Parent and Child ego states** — Owner: Berne (transactional analysis). Ego states are Berne's TA vocabulary, not Gestalt's.
- **C. Superego and id** — Owner: Freud (psychoanalysis). Id, ego, and superego are Freud's structures of personality, not a Gestalt framing.
- **Pair to learn:** Top dog / underdog → Perls (Gestalt), not Berne's ego states

167 — B

Professional Practice & Ethics · Outline 1.K · Chapter 17

- **B. State their commitment to the Code and try to resolve it — Owner: Conflicts between ethics and laws (ACA I.1.c).** I.1.c directs counselors to make their commitment to the Code known and take steps to resolve the conflict; if that fails, they may follow the law.
- **A. Follow the Code, since ethics always outrank the law** — Owner: Misreading of ACA I.1.c. I.1.c allows counselors to follow the law if the conflict can't be resolved; ethics don't automatically outrank law.
- **C. Apply Stadler's universality test to settle the conflict** — Owner: Stadler: universality test. Stadler's tests check a chosen course of action; they aren't the Code's procedure for ethics-law conflicts.
- **D. Follow the law without raising the conflict with anyone** — Owner: Mandatory ethics (minimum rules

only). Following the law may be the end point, but I.1.c first asks counselors to state their commitment and try to resolve the conflict.

- **Pair to learn:** Ethics-law conflict → State commitment to the Code, seek resolution (I.1.c)

168 — C

Core Counseling Attributes · Outline 6.L · Chapter 22

- **C. Start with the client's own account of what's wrong — Owner: Respect and acceptance for diversity (6.L).** Respect means letting the client define his concern rather than assuming his disability explains it.
- **A. Tell him how hard life with a disability must be** — Owner: Sympathy. Sympathy assumes hardship and comes from the counselor's frame.
- **B. Refer him to a counselor who specializes in disability** — Owner: Premature referral. He came for job-related depression; disability alone is not a reason to refer.
- **D. Gently suggest that his disability may be the deeper issue** — Owner: Premature interpretation (counselor assumption). It imposes the counselor's assumption before exploring the client's own account.
- **Pair to learn:** Client defines his own problem → Respect for diversity (not assuming the disability is the issue)

169 — A

Intake, Assessment & Diagnosis · Outline 2.O · Chapter 19

- **A. The ACEs questionnaire — Owner: Adverse Childhood Experiences (ACEs) screen.** The ACEs screen targets childhood adversity, which raises risk for later substance use and other problems.
- **B. The Child Behavior Checklist** — Owner: Achenbach (CBCL). The CBCL gathers parent, teacher,

and youth ratings of a child's behavior, not an adult's childhood history.

- **C. The Social Readjustment Rating Scale** — Owner: Holmes & Rahe (life change units). It totals life change units from stressful life events; it is not a childhood adversity screen.
- **D. The Columbia-Suicide Severity Rating Scale** — Owner: Posner et al. (C-SSRS). The C-SSRS assesses suicide risk, not childhood adversity.
- **Pair to learn:** Childhood adversity → ACEs (not Holmes & Rahe)

170 — A

Areas of Clinical Focus · Outline 3.AX · Chapter 10

- **A. Siegel's "name it to tame it" — Owner: Siegel (name it to tame it).** Siegel's phrase captures the idea that naming an emotion helps regulate it.
- **B. Wells' metacognitive therapy** — Owner: Wells (metacognitive model). Wells targets beliefs about worry, not labeling emotions in the moment.
- **C. Jacobson's progressive relaxation** — Owner: Jacobson (progressive muscle relaxation). Progressive muscle relaxation lowers physical tension; it doesn't involve naming emotions.
- **D. Gross's response modulation** — Owner: Gross (process model: response modulation). Response modulation acts on the emotional response after it arises, such as suppression, rather than labeling it.
- **Pair to learn:** Labeling emotions to regulate them → Siegel (name it to tame it)

171 — C

Counseling Skills & Interventions · Outline 5.U · Chapter 5

- **C. Interests, not positions (Fisher & Ury) — Owner: Fisher & Ury (interest-based negotiation).** Focusing on interests rather than positions, and

separating people from problems, is Fisher and Ury's approach.

- **A. The speaker-listener technique (PREP)** — Owner: Markman, Stanley, & Blumberg (PREP). Speaker-listener has one partner speak briefly and the other paraphrase before replying.
- **B. Softened startup (Gottman)** — Owner: Gottman Method. Softened startup is Gottman's antidote to harsh, critical openings in couples.
- **D. I-messages and active listening (Gordon)** — Owner: Gordon (Parent Effectiveness Training). I-messages replace blaming 'you' messages; they don't shift from positions to interests.
- **Pair to learn:** Interests, not positions → Fisher & Ury

172 — B

Professional Practice & Ethics · Outline 1.M · Chapter 17

- **B. HITECH Act — Owner: HITECH Act (2009).** The HITECH Act strengthened HIPAA enforcement and added breach notification requirements for electronic records.
- **A. The ADA** — Owner: Americans with Disabilities Act. The ADA requires reasonable accommodations for people with disabilities; it isn't a records privacy law.
- **C. 42 CFR Part 2** — Owner: 42 CFR Part 2 (SUD program records). 42 CFR Part 2 gives extra protection to records of federally assisted substance use disorder programs.
- **D. FERPA** — Owner: FERPA (student education records). FERPA protects the privacy of student education records and parents' access to them.
- **Pair to learn:** Breach notification, stronger HIPAA enforcement → HITECH Act

173 — D

Professional Practice & Ethics · Outline 1.J · Chapter 9

- **D. Not view it unless the client consents — Owner: Client privacy on social media (ACA H.6.c).** H.6.c directs counselors to respect clients' privacy and not view their social media without consent.
- **A. View it, since the profile is public** — Owner: Public posting (not consent under H.6.c). A public profile is not the client's consent; H.6.c still applies.
- **B. Follow it from her professional account** — Owner: Personal virtual relationships with clients (ACA A.5.e). Following a client creates a virtual relationship and still involves viewing without consent.
- **C. View it and note findings at intake** — Owner: Documentation (doesn't cure lack of consent). Documenting what she saw doesn't make viewing it without consent appropriate.
- **Pair to learn:** Viewing a client's social media → Only with consent (H.6.c)

174 — C

Counseling Skills & Interventions · Outline 5.G · Chapter 9

- **C. Adult Children of Alcoholics — Owner: Adult Children of Alcoholics (ACA/ACoA).** ACoA is for adults who grew up with alcoholic or dysfunctional caregivers.
- **A. Al-Anon** — Owner: Al-Anon. Al-Anon is open to anyone affected by someone else's drinking, but ACoA is the group built specifically for adults who grew up with alcoholic caregivers.
- **B. Alateen** — Owner: Alateen. Alateen is for teens affected by someone's drinking.
- **D. SMART Recovery** — Owner: SMART Recovery. SMART Recovery is for people with addiction themselves.

- **Pair to learn:** Adult who grew up with an alcoholic parent → ACoA

175 — A

Intake, Assessment & Diagnosis · Outline 2.E · Chapter 19

- **A. Is there clinically significant distress or impairment? — Owner: First: step 6 (boundary with no mental disorder).** The last step establishes the boundary with no mental disorder by asking whether distress or impairment is clinically significant.
- **B. Could a substance or medication explain the symptoms?** — Owner: First: step 2 (substance or medication cause). Ruling out a substance or medication cause is the second step.
- **C. Could a general medical condition account for the symptoms?** — Owner: First: step 3 (medical condition). Ruling out a medical condition is the third step.
- **D. Are the symptoms being intentionally produced?** — Owner: First: step 1 (rule out malingering and factitious disorder). Ruling out intentional symptom production is the first step, not the last.
- **Pair to learn:** Final step: clinically significant distress or impairment → Boundary with no mental disorder (First)

176 — D

Counseling Skills & Interventions · Outline 5.R · Chapter 2

- **D. Reaction formation — Owner: Freud / Anna Freud (defense mechanisms).** Expressing the opposite of what one feels is reaction formation.
- **A. Rationalization** — Owner: Freud / Anna Freud (defense mechanisms). Rationalization creates a logical-sounding excuse.

- **B. Sublimation** — Owner: Freud / Anna Freud (defense mechanisms). Sublimation channels impulses into acceptable outlets.
- **C. Intellectualization** — Owner: Freud / Anna Freud (defense mechanisms). Intellectualization retreats into abstract thinking to avoid feeling.
- **Pair to learn:** Expressing the opposite of what one feels → reaction formation

177 — A

Intake, Assessment & Diagnosis · Outline 2.F · Chapter 19

- **A. Insight — Owner: MSE: insight (understanding of one's own problem).** Insight is the client's understanding of his problem; denying a clear problem reflects poor insight.
- **B. Thought process** — Owner: MSE: thought process (how the client thinks). Thought process describes the organization and flow of thinking, which is not described here.
- **C. Orientation** — Owner: MSE cognition: orientation (person, place, time). Orientation concerns knowing who and where one is and the time, not understanding one's problem.
- **D. Judgment** — Owner: MSE: judgment (ability to make sound decisions). Judgment concerns decision-making; this statement is about whether he understands that a problem exists.
- **Pair to learn:** Understanding of one's own problem → Insight (not judgment)

178 — B

Professional Practice & Ethics · Outline 1.C · Chapter 17

- **B. Publicity test — Owner: Stadler: publicity test.** The publicity test asks whether you would want your behavior reported in the press.
- **A. Justice test** — Owner: Stadler: justice test. The justice test asks whether you would treat others in the same situation the same way.

- **C. Universality test** — Owner: Stadler: universality test. Universality asks whether you would recommend the same action to another counselor in the same situation.
- **D. Step 6 evaluation** — Owner: Forester-Miller & Davis: step 6 (evaluate the selected course). Step 6 of Forester-Miller and Davis's model is evaluating the chosen action; the newspaper question belongs to Stadler's tests.
- **Pair to learn:** Would I want this in the news? → Publicity test (Stadler)

179 — D

Treatment Planning · Outline 4.H · Chapter 21

- **D. In informed consent, revisiting it as goals near — Owner: Raise termination early (planned termination).** Termination is introduced early, starting with informed consent, and revisited as goals are approached.
- **A. Only when the client brings up wanting to stop** — Owner: Client-initiated termination. Waiting for the client to raise it leaves ending unplanned and often abrupt.
- **B. Once sessions start tapering to every other week** — Owner: Tapering. Tapering comes late in the process; termination should already be part of the conversation.
- **C. In the final session, once the goals are met** — Owner: Last-session announcement (punished). Raising termination for the first time in the last session is what the exam punishes.
- **Pair to learn:** Raise termination early → Starting with informed consent

180 — B

Areas of Clinical Focus · Outline 3.AI · Chapter 11

- **B. Refer her for medically managed withdrawal — Owner: Sedative withdrawal is potentially life-**

threatening. Sedative withdrawal, like alcohol withdrawal, can cause seizures and delirium, so it needs medical care and a supervised taper.

- **A. Explain that withdrawal is unpleasant but safe** — Owner: Opioid withdrawal profile. Rarely life-threatening describes opioid withdrawal in healthy adults, not sedative withdrawal.
- **C. Wait until she is abstinent to treat her anxiety** — Owner: Sequential treatment (not recommended). Requiring sobriety before addressing co-occurring problems is discouraged, and it misses the medical risk.
- **D. Affirm her plan and check in on Monday** — Owner: MI affirmation (misapplied). Affirming motivation is useful, but supporting an unsupervised stop ignores a dangerous withdrawal.
- **Pair to learn:** Sedative withdrawal → potentially life-threatening; medically managed withdrawal

181 — D

Areas of Clinical Focus · Outline 3.AM · Chapter 10

- **D. Wells' metacognitive model — Owner: Wells (metacognitive model of GAD).** Wells holds that beliefs about worry, such as "my worry is uncontrollable," drive GAD.
- **A. Clark's cognitive model of panic** — Owner: Clark (catastrophic misinterpretation). Clark's model concerns catastrophic misreading of body sensations in panic.
- **B. Dugas' intolerance of uncertainty model** — Owner: Dugas (intolerance of uncertainty). Dugas focuses on difficulty tolerating uncertainty, not beliefs about the worry process itself.
- **C. Borkovec's avoidance theory of worry** — Owner: Borkovec (avoidance theory). Borkovec explains worry as verbal activity that dampens arousal, not as driven by beliefs about worry.

- **Pair to learn:** Beliefs about worry → Wells (metacognitive), not Dugas

182 — D

Counseling Skills & Interventions · Outline 5.AD · Chapter 4

- **D. "I have, and it was disorienting. What has it been like for you?" — Owner: Appropriate self-disclosure (brief, relevant, returns focus).** Brief, relevant, and immediately returns the focus to the client.
- **A. "Yes, and honestly, I'm still struggling with it most days."** — Owner: Self-disclosure that meets the counselor's needs. Sharing current struggle burdens the client with the counselor's needs.
- **B. "Another client of mine lost her dad and found a group helpful."** — Owner: Confidentiality breach (not self-disclosure). Talking about another client is a confidentiality violation, not self-disclosure.
- **C. "Yes. Let me tell you about my father's illness and how we coped."** — Owner: Counselor-centered self-disclosure. A lengthy account shifts the session to the counselor's experience.
- **Pair to learn:** Brief, relevant, back to the client → appropriate self-disclosure

183 — C

Core Counseling Attributes · Outline 6.I · Chapter 22

- **C. Universality — Owner: Yalom: universality.** Discovering that others share the same feeling, so one is not alone, is universality.
- **A. Catharsis** — Owner: Yalom: catharsis. Catharsis is emotional release, not the discovery of shared experience.
- **B. Instillation of hope** — Owner: Yalom: instillation of hope. Hope comes from seeing others improve, not from finding shared guilt.

- **D. Altruism** — Owner: Yalom: altruism. Altruism is benefiting from helping others.
- **Pair to learn:** Others feel it too → Universality (Yalom)

184 — A

Areas of Clinical Focus · Outline 3.AL · Chapter 16

- **A. Brief psychotic disorder — Owner: DSM-5-TR brief psychotic disorder.** Brief psychotic disorder involves sudden psychotic symptoms lasting at least 1 day but less than 1 month, with full return to functioning.
- **B. Schizoaffective disorder** — Owner: DSM-5-TR schizoaffective disorder. Schizoaffective disorder requires a major mood episode alongside schizophrenia symptoms.
- **C. Delusional disorder** — Owner: DSM-5-TR delusional disorder. Delusional disorder requires delusions for at least 1 month without other prominent psychotic symptoms.
- **D. Schizophreniform disorder** — Owner: DSM-5-TR schizophreniform disorder. Schizophreniform requires symptoms for at least 1 month but less than 6.
- **Pair to learn:** 1 day to under 1 month, full recovery → brief psychotic disorder (not schizophreniform)

185 — D

Professional Practice & Ethics · Outline 1.C · Chapter 17

- **D. Fidelity — Owner: Fidelity (Kitchener; ACA principles).** Honoring commitments and keeping promises is fidelity.
- **A. Veracity** — Owner: Veracity (ACA principles). Veracity is dealing truthfully with people; the core of this scenario is keeping a promise.
- **B. Beneficence** — Owner: Beneficence (Kitchener; ACA principles). Beneficence is working for the client's good; the specific act here is honoring a commitment.

- **C. Justice** — Owner: Justice (Kitchener; ACA principles). Justice concerns fairness and equal access across people, not keeping a promise to one client.
- **Pair to learn:** Keeping a promise → Fidelity

186 — D

Professional Practice & Ethics · Outline 1.B · Chapter 18

- **D. History — Owner: History (Campbell & Stanley).** An outside event occurring during the study that affects results is the history threat.
- **A. Maturation** — Owner: Maturation (Campbell & Stanley). Maturation refers to natural changes within participants over time, such as growing older or recovering on their own, not an outside event.
- **B. Instrumentation** — Owner: Instrumentation (Campbell & Stanley). Instrumentation is a change in the measure or raters between testings; the stress scale didn't change.
- **C. Experimental mortality** — Owner: Experimental mortality / attrition (Campbell & Stanley). Mortality is uneven dropout across groups; nothing here indicates participants left the study.
- **Pair to learn:** Outside event during the study → History

187 — B

Counseling Skills & Interventions · Outline 5.Z · Chapter 5

- **B. Herbert Benson — Owner: Benson (relaxation response).** The relaxation response is Herbert Benson's concept.
- **A. Joseph Wolpe** — Owner: Wolpe (systematic desensitization). Wolpe pairs relaxation with an imagined hierarchy; the relaxation response is Benson's.
- **C. Jon Kabat-Zinn** — Owner: Kabat-Zinn (MBSR). Kabat-Zinn developed Mindfulness-Based Stress Reduction.

- **D. Edmund Jacobson** — Owner: Jacobson (progressive relaxation). Jacobson owns progressive muscle relaxation.
- **Pair to learn:** Relaxation response → Benson (not Jacobson's progressive relaxation)

188 — C

Areas of Clinical Focus · Outline 3.AI · Chapter 11

- **C. José Szapocznik — Owner: Szapocznik (BSFT).** Szapocznik developed BSFT, a family-based approach for adolescent substance use.
- **A. Robert Meyers** — Owner: Meyers (CRAFT). Meyers developed CRAFT, a family-focused approach built on the community reinforcement approach.
- **B. Howard Liddle** — Owner: Liddle (MDFT). Liddle developed Multidimensional Family Therapy, a different family approach for adolescent substance use.
- **D. Scott Henggeler** — Owner: Henggeler (MST). Henggeler developed Multisystemic Therapy for serious antisocial behavior.
- **Pair to learn:** BSFT → Szapocznik (not Liddle's MDFT)

189 — C

Intake, Assessment & Diagnosis · Outline 2.P · Chapter 11

- **C. Does not meet criteria for a sedative use disorder — Owner: DSM-5-TR exception: tolerance and withdrawal from prescribed use don't count.** Tolerance and withdrawal that occur only with medication taken as prescribed under medical supervision don't count toward the diagnosis.
- **A. Has a sedative-induced anxiety disorder** — Owner: Substance-induced disorder. Brief shakiness when a dose is late is not described as an independent anxiety syndrome.
- **B. Has mild sedative use disorder, with two criteria met** — Owner: Severity: mild = 2 to 3 criteria (miscounted here). Counting tolerance and withdrawal

from prescribed use is the error; with them excluded, no criteria are met.

- **D. Has sedative use disorder in early remission** — Owner: Early remission specifier. Remission applies to someone who previously met criteria; she never did.
- **Pair to learn:** Tolerance/withdrawal from prescribed use → Not counted toward a substance use disorder

190 — A

Counseling Skills & Interventions · Outline 5.F · Chapter 2

- **A. The Premack principle — Owner: David Premack.** Using a preferred activity (playing outside) to reinforce a less-preferred one (reading) is the Premack principle.
- **B. A token economy** — Owner: Ayllon & Azrin. A token economy uses tokens exchanged for rewards; no tokens are involved here.
- **C. Shaping** — Owner: Skinner (operant). Shaping reinforces successive approximations of a target behavior.
- **D. Negative reinforcement** — Owner: Skinner (operant). Negative reinforcement removes something unpleasant; here a pleasant activity is offered.
- **Pair to learn:** Preferred activity reinforces a less-preferred one → Premack principle (not token economy)

191 — A

Areas of Clinical Focus · Outline 3.U · Chapter 3

- **A. Orientation to social valuation — Owner: Gottfredson (circumscription and compromise).** Gottfredson's social valuation stage is when children rule out options based on prestige.
- **B. The growth stage of Super's model** — Owner: Super (life-span, life-space). Super's growth stage concerns developing self-concept and learning about work, not ruling out jobs by status.

- **C. Ginzberg's tentative period** — Owner: Ginzberg et al. (developmental). Ginzberg's tentative period involves provisional choices from interests and abilities, not prestige-based elimination.
- **D. Orientation to sex roles** — Owner: Gottfredson (circumscription and compromise). This earlier stage rules out options by gender, not by prestige.
- **Pair to learn:** Ruling out options by prestige → social valuation (Gottfredson)

192 — B

Areas of Clinical Focus · Outline 3.AE · Chapter 16

- **B. Specific suggestions — Owner: Annon (PLISSIT).** Offering concrete strategies such as sensate focus exercises is the specific suggestions level of PLISSIT.
- **A. Limited information** — Owner: Annon (PLISSIT). Limited information is providing basic, accurate facts, which the counselor already did.
- **C. Permission** — Owner: Annon (PLISSIT). Permission normalizes talking about sex, the first step already completed.
- **D. Intensive therapy** — Owner: Annon (PLISSIT). Intensive therapy is specialized sex therapy requiring advanced training, beyond assigning a structured exercise.
- **Pair to learn:** Offering a concrete strategy like sensate focus → PLISSIT specific suggestions (Annon)

193 — A

Professional Practice & Ethics · Outline 1.W · Chapter 17

- **A. Dispose of them in a way that protects confidentiality — Owner: Records retention and disposal (ACA B.6.h).** After the required retention period, records are disposed of in a manner that protects client confidentiality.
- **B. Mail the original files back to each former client** — Owner: Client access to records (ACA B.6.e).

B.6.e concerns reasonable access to records, not disposal; mailing originals risks confidentiality.

- **C. Place them in the office's regular paper recycling bin** — Owner: Unsecured disposal (confidentiality breach). Ordinary recycling exposes client information; disposal must protect confidentiality.
- **D. Give them to the records custodian named in her will** — Owner: Professional will / records custodian (ACA C.2.h, B.6.i). A records custodian handles records if the counselor dies or becomes incapacitated; that isn't the situation here.
- **Pair to learn:** Past retention period → Confidential disposal (B.6.h)

194 — D

Areas of Clinical Focus · Outline 3.Q · Chapter 16

- **D. Binge-eating disorder — Owner: DSM-5-TR binge-eating disorder.** Binge-eating disorder involves recurrent binges with loss of control and distress at least weekly for 3 months, without regular compensatory behaviors.
- **A. Bulimia nervosa** — Owner: DSM-5-TR bulimia nervosa. Bulimia requires compensatory behaviors such as vomiting, fasting, or excessive exercise.
- **B. Anorexia nervosa, binge-eating/purging type** — Owner: DSM-5-TR anorexia nervosa. Anorexia requires significantly low body weight, which isn't described.
- **C. Avoidant/restrictive food intake disorder** — Owner: DSM-5-TR ARFID. ARFID involves restricted eating, not binge episodes.
- **Pair to learn:** Binges without compensatory behavior → binge-eating disorder (not bulimia)

195 — B

Professional Practice & Ethics · Outline 1.B · Chapter 18

- **B. Stratified sampling — Owner: Stratified random sampling.** The population is divided into subgroups

(strata) and then randomly sampled from each, often in proportion to size.

- **A. Systematic sampling** — Owner: Systematic sampling. Systematic sampling takes every nth person from a list; it doesn't divide the population into strata first.
- **C. Cluster sampling** — Owner: Cluster sampling. Cluster sampling randomly selects whole groups and studies everyone in them, rather than sampling individuals within every group.
- **D. Convenience sampling** — Owner: Convenience sampling. Convenience sampling uses whoever is available, with no random selection at all.
- **Pair to learn:** Random selection within proportional subgroups → Stratified random sampling

196 — A

Core Counseling Attributes · Outline 6.A · Chapter 22

- **A. Self-awareness and impact on clients — Owner: 6.A: power and privilege (MSJCC).** Awareness of the power the counselor's role and identity carry is part of self-awareness and ethical practice.
- **B. Nonpossessive warmth toward the client** — Owner: Truax & Carkhuff: nonpossessive warmth. Warmth is caring without creating dependence, not awareness of power.
- **C. Empathic attunement to the client** — Owner: 6.G: empathic attunement. Attunement is sensing the client's experience, not examining the counselor's own power.
- **D. Conflict tolerance and resolution** — Owner: 6.F: conflict tolerance. Conflict tolerance is staying present with disagreement or anger, not reflecting on power.
- **Pair to learn:** Awareness of the counselor's power and privilege → Self-awareness (6.A)

197 — C

Counseling Skills & Interventions · Outline 5.C · Chapter 9

- **C. Social model — Owner: Social model of disability.** The social model sees disability as created by environmental and social barriers.
- **A. Biopsychosocial model** — Owner: Biopsychosocial model (addiction). The biopsychosocial model (the mainstream addiction model) describes interacting biological, psychological, and social factors; the client points specifically to environmental barriers.
- **B. Moral model** — Owner: Moral model (addiction). The moral model, now rejected, frames addiction as a personal failing.
- **D. Medical model** — Owner: Medical model of disability. The medical model treats disability as an individual deficit.
- **Pair to learn:** Disability from environmental barriers → social model (not medical)

198 — C

Areas of Clinical Focus · Outline 3.AN · Chapter 13

- **C. Teen identity questions are a normal adoption issue — Owner: Silverstein & Kaplan (identity as a core issue).** Identity is one of Silverstein and Kaplan's core adoption issues, and adolescence often brings interest in birth family without rejecting the adoptive family.
- **A. Her interest shows she was told about adoption too early** — Owner: Late-disclosure belief (contradicts guidance). Early, age-appropriate disclosure is recommended; it doesn't cause adolescent identity questions.
- **B. She should wait until 18 to explore birth family** — Owner: Secrecy and delay (discouraged). Delay and secrecy tend to harm trust and identity; her questions deserve ongoing, honest conversation.

- **D. Her questions suggest reactive attachment disorder** — Owner: DSM-5-TR reactive attachment disorder. RAD involves emotional withdrawal after severe neglect, evident before age 5; curiosity about birth family is not a symptom.
- **Pair to learn:** Adolescent adoptee asks about birth family → normal identity issue (Silverstein & Kaplan)

199 — D

Treatment Planning · Outline 4.A · Chapter 21

- **D. Call or meet up with one friend twice a week — Owner: Well-formed goal (de Shazer, SFBT).** It is small, concrete, behavioral, within her control, and stated as the presence of something.
- **A. Feel more connected to people in general** — Owner: Vague goal (fails SMART: Specific). It is too vague to tell when it has been reached.
- **B. Never feel lonely on a weekend again** — Owner: Absence-based, unachievable goal (fails SMART: Achievable). It is stated as an absence and is unrealistic, so it is neither well-formed nor achievable.
- **C. Understand why she keeps pulling away from others** — Owner: Insight goal (not behavioral). Understanding is an insight aim, not a concrete behavior she can do and observe.
- **Pair to learn:** Presence of a concrete behavior → Well-formed goal (de Shazer), applied rewrite

200 — C

Professional Practice & Ethics · Outline 1.H · Chapter 17

- **C. Have told the client in advance and offer a chance to pay — Owner: Nonpayment and collections (ACA A.10.d).** A.10.d requires informing clients in advance that collection agencies may be used and giving them an opportunity to pay first.
- **A. Give the agency the client's diagnosis to support the claim** — Owner: Minimal disclosure (ACA B.2.e). A collection agency doesn't need clinical

information; disclosing a diagnosis goes far beyond what is essential.

- **B. End services at once and then file with the agency** — Owner: Abandonment (ACA A.12). Ending services abruptly without pretermination counseling or referrals risks abandonment, and it skips the required notice.
- **D. Raise the client's fee to cover the expected collection costs** — Owner: Establishing fees (ACA A. 10.c). A.10.c asks counselors to consider clients' financial status when setting fees; raising the fee doesn't meet the notice requirement.
- **Pair to learn:** Collections → Advance notice and chance to pay (A.10.d)

Score by Domain

Domain	Items	Your correct	%
Counseling Skills & Interventions	60		
Areas of Clinical Focus	58		
Professional Practice & Ethics	24		
Intake, Assessment & Diagnosis	24		
Treatment Planning	18		
Core Counseling Attributes	16		
Total	**200**		

There's no official NCE passing percentage. Use this table to find your two weakest domains and give them extra study time next week.

Item numbers by domain

- **Counseling Skills & Interventions:** 3, 8, 9, 14, 16, 19, 21, 23, 26, 27, 30, 34, 38, 42, 45, 48, 49, 50, 52, 55, 59, 60, 62, 64, 67, 72, 73, 74, 76, 80, 81, 83, 84, 88, 97, 99, 101, 103, 104, 117, 120, 121, 122, 124, 132, 136, 140, 143, 146, 151, 155, 165, 166, 171, 174, 176, 182, 187, 190, 197

- **Areas of Clinical Focus:** 5, 7, 10, 15, 17, 18, 24, 29, 35, 40, 41, 43, 44, 47, 56, 58, 61, 65, 68, 69, 75, 77, 86, 89, 90, 94, 106, 109, 111, 112, 114, 115, 116, 118, 119, 129, 133, 134, 137, 141, 144, 152, 153, 154, 157, 159, 160, 163, 164, 170, 180, 181, 184, 188, 191, 192, 194, 198
- **Professional Practice & Ethics:** 4, 11, 12, 57, 63, 78, 79, 82, 85, 91, 96, 98, 105, 113, 135, 167, 172, 173, 178, 185, 186, 193, 195, 200
- **Intake, Assessment & Diagnosis:** 1, 2, 6, 20, 25, 28, 31, 36, 46, 51, 66, 70, 102, 108, 123, 125, 127, 139, 158, 162, 169, 175, 177, 189
- **Treatment Planning:** 22, 32, 33, 39, 53, 54, 92, 93, 126, 128, 130, 131, 145, 147, 149, 150, 179, 199
- **Core Counseling Attributes:** 13, 37, 71, 87, 95, 100, 107, 110, 138, 142, 148, 156, 161, 168, 183, 196

Pair Review

Cover the right column. Say the owner out loud. Add every pair you missed to your error log.

#	Pair
1	Feedback shows an alliance drop → Name and explore the rupture (Safran & Muran)
2	Adolescent substance use screen → CRAFFT (not CBCL)
3	Every intervention traces back to a treatment goal (5.D)
4	Significant p, trivial change → Statistical, not clinical, significance
5	Racism producing trauma-like reactions → race-based traumatic stress (Carter, not Sue)
6	±2 SEM → 95% band (88 with SEM 3 → 82 to 94)
7	Rapprochement → Mahler (not Bowlby)

#	Pair
8	Comment on how the group interacts → process observation (not summarizing)
9	Sarcastic indirect jab → passive-aggressive (not assertive)
10	Motor + vocal tics, over 1 year, onset before 18 → Tourette's disorder
11	Client-requested barter → ACA A.10.e conditions and written contract
12	"Dr." in counseling practice → Doctorate in counseling or closely related field (C.4.d)
13	Exploring gender-role messages → Gender sensitivity (6.D), not reassurance
14	Last minutes of session → don't open new painful material; plan for next time
15	Questioning faith after conversion → Z65.8 moral, religious, or spiritual problem (not a disorder)
16	Hopelessness predicts suicide → Beck (not Joiner)
17	Connected to neither culture → marginalization (Berry)
18	Violence in response to a partner's coercive control → violent resistance (Johnson)
19	Outside contact → bring it back into group (5.AW)
20	Cannot meet basic needs → Grave disability (involuntary criterion)
21	Change in the rules themselves → second-order change
22	Warning signs and coping plan → Relapse prevention

#	Pair
23	Predictable life transitions → developmental crisis (not situational)
24	Tolerance/withdrawal from prescribed use → not counted toward SUD
25	Same measure before and after → Pre-test/post-test outcome measurement (2.T)
26	Links to school, church, agencies → ecomap (Hartman), not genogram
27	Days after a disaster → psychological first aid (not debriefing)
28	Core questions plus room for follow-up → Semi-structured interview (not structured SCID-5)
29	Six directive steps → Williamson (not Parsons)
30	Importance and confidence rulers → MI (not SFBT scaling)
31	Raw 36 (mean 60, SD 12) → z = −2 → deviation IQ 70
32	Counselor-observed affect and test score → SOAP: Objective
33	Legal or benefits problem → Concurrent referral to community resources (4.F)
34	Unconditional self-acceptance → Ellis (not Rogers' unconditional positive regard)
35	Exclusion and rumors → relational bullying
36	Session-by-session outcome and alliance → ORS/SRS (Miller & Duncan), not OQ-45
37	Demonstrably effective element → Positive regard and affirmation (Norcross & Lambert)

#	Pair
38	Individual → family modality: the system is the client (circular, not linear)
39	Doesn't understand why the task helps → Explain the rationale (4.P)
40	Dilated pupils, racing heart, paranoia → stimulant intoxication (not opioids)
41	Locked facility + 3 to 12 months → early remission, in a controlled environment
42	Smoothing over pain too fast → rescuing: block gently
43	Severity based on adaptive functioning → intellectual developmental disorder (not IQ alone)
44	Partner flooded → take a break of about 20 minutes (Gottman)
45	Jungian sandplay → Kalff (world technique → Lowenfeld)
46	Acute mania with imminent danger → Inpatient hospitalization (not PHP)
47	Numbness, shutdown, dissociation → hypoarousal (Siegel), not Selye's exhaustion
48	Goals, tasks, bond → Bordin (not Safran & Muran)
49	Convert "why" to "what" or "how" to avoid defensiveness
50	Reciprocal determinism → Bandura; reciprocal inhibition → Wolpe
51	Uncomplicated bereavement → Z code (not a mental disorder)
52	Negative view of self, world, future → cognitive triad (Beck)
53	Coordination with a release → Share the minimum necessary

#	Pair
54	Collaborative follow-through → Adherence (not compliance)
55	Information-giving and resources (not advice) → 5.Y
56	Focus on meaningful relationships as time shortens → Carstensen (not disengagement)
57	Approves programs → Accreditation (CACREP)
58	Enjoying new freedom early in retirement → honeymoon phase (Atchley)
59	Sudden calm + putting affairs in order → reassess suicide risk directly
60	Exploration → insight → action → Hill (not Egan)
61	Stabilizing routines and sleep in bipolar disorder → IPSRT (Frank), not IPT
62	In-law pulled into every fight → triangle; detriangulate (Bowen)
63	Counselor's own impairment → Limit or suspend services (C.2.g)
64	Generalization that misses the feeling → Carkhuff level 2 (subtracts)
65	ACE Study → Felitti & Anda (not van der Kolk)
66	Test in the primary language before special education placement → Diana v. State Board of Education
67	Define → alternatives → decide → implement → evaluate → D'Zurilla & Goldfried
68	Negative career thoughts (CIP) → Career Thoughts Inventory (not Krumboltz's Career Beliefs Inventory)

#	Pair
69	Protest → despair → detachment → Bowlby & Robertson (not Kübler-Ross)
70	Trauma screen at intake → PC-PTSD-5 (not GAD-7)
71	Keep the client talking without directing → Minimal encourager
72	BASIC I.D. → Lazarus (not Glasser's total behavior)
73	Outsider witnesses; definitional ceremonies → White & Epston (narrative)
74	Deviation-reducing feedback → negative feedback (morphostasis)
75	Middle phase begins with mobilization → Papernow (not Tuckman)
76	Cohesion + flexibility → Olson's Circumplex (not McMaster)
77	First panic-like attack with chest pain → medical evaluation first
78	Cost per nonmonetary outcome → Cost-effectiveness analysis (not cost-benefit)
79	Terminally ill client considering hastening death → Option to keep confidentiality after consultation (B.2.b)
80	First session + distress → reflect, don't confront yet
81	Surprise at achievement based on identity → microinsult (Sue)
82	Two or more IVs and their interaction → Factorial ANOVA
83	Here-and-now: experience, then illuminate (Yalom)
84	Outside-contact expectations → set during informed consent

#	Pair
85	Colleague violation with substantial harm → Report (I. 2.b), not informal resolution
86	Biologically easy-to-learn fears → preparedness (Seligman), not Mowrer
87	Congruence is not saying everything you feel → Share reactions when it serves the client (Rogers)
88	Never reframe abuse as care; assess safety first
89	Contact → disintegration → reintegration → Helms (not Cross)
90	Full conviction plus compulsions → OCD with absent insight (not delusional disorder)
91	Before disclosing contagious disease → Assess client's intent to inform (B.2.c, 2014)
92	Counselor-written goals → Collaboration and client autonomy (4.A)
93	Scheduled check-ins after ending → Booster sessions (4.J)
94	Sensitization → commitment → Troiden (not Cass)
95	Multicultural competence → Derald Wing Sue (not Kohut)
96	Former client after 5 years → Forethought and documentation still required (A.5.c)
97	Group targets one member → scapegoating: block and protect
98	AI-drafted note → Counselor reviews and is responsible for it
99	Agrees too easily, goes quiet → withdrawal rupture: name and explore (Safran & Muran)

#	Pair
100	Counselor experiences and communicates empathic understanding → Rogers' condition 5
101	Defensiveness → antidote: taking responsibility (Gottman)
102	Concerns about the counselor-client relationship → CFI domain 4 (current help-seeking)
103	Good-enough mother; holding environment → Winnicott (not Bowlby)
104	Code words to confirm identity → ACA H.3 (client verification)
105	Attorney-issued subpoena → Contact client, assert privilege
106	Loneliness heightens alertness to social threat → Cacioppo (not Holt-Lunstad)
107	Client expects judgment → Nonjudgmental stance (not reassurance or moralizing)
108	Family member's cultural view of the problem → CFI informant version
109	Internal, stable, global attributions → Abramson, Seligman, & Teasdale (not Lewinsohn or Nolen-Hoeksema)
110	Personal characteristics of effective counselors → Corey (not Truax & Carkhuff)
111	Fatigue, psychomotor slowing, low libido, slowed thinking → physical signs of depression (not anxiety)
112	Dwelling on past loss, not alien, no neutralizing → depressive rumination (not obsession)
113	Group privacy → Can't be guaranteed (ACA B.4)

#	Pair
114	Physically present, psychologically absent → ambiguous loss (Boss) in caregiving
115	Passion + commitment, no intimacy → fatuous love (Sternberg, not Lee)
116	Incredible Years → Webster-Stratton (not Sanders' Triple P)
117	Who notices, what happens next → circular question (Milan)
118	Plans to reunite parents → school-age reconciliation fantasy
119	Biosocial theory of dysregulation → Linehan (not Gross)
120	Interpretation → trained professional, never a child
121	Violated one's own moral code → moral injury
122	Rewards for verified abstinence → contingency management (not CRA)
123	Chiefly responsible for the admission, listed first → Principal diagnosis (not Axis I)
124	Psychache → Shneidman (not Joiner)
125	Change smaller than the SEM → May be measurement error; interpret scores as ranges
126	Danger at home → Safety first in sequencing
127	Impact on work and daily life → Functioning (2.K), not distress (SUDS)
128	Measurable, time-bound step → Short-term objective
129	Assuming one limitation means global limitation → spread (Beatrice Wright)

#	Pair
130	Collectivist family involvement → Respect cultural values in goals (4.A)
131	High-risk period after hospital discharge → Prompt follow-up
132	Present-moment awareness, no relationship comment → here-and-now (Gestalt), not immediacy
133	Lost time structure, contact, purpose → Jahoda's latent deprivation (not Maslach)
134	Angry, defiant, 4+ symptoms, 6 months, no rights violations → ODD (not conduct disorder)
135	Supervisor liable for supervisee's acts → Vicarious liability
136	Family reconstruction → Satir (not Bowen)
137	Will and control repeatedly overridden → powerlessness (Finkelhor & Browne)
138	Same content and feeling → Carkhuff level 3 (interchangeable), not level 4
139	Clinician states why criteria aren't met → Other specified (not unspecified)
140	Members can decline structured exercises
141	Re-entering exploration after midlife layoff → recycling (Super)
142	Group knowledge as a hypothesis, checked with the client → Cultural humility (not stereotyping)
143	Member's reaction to the leader → receive non-defensively, bring it into group

#	Pair
144	Modern hospice movement → Cicely Saunders (not Kübler-Ross)
145	Worsening, no imminent risk → Step up to IOP or PHP
146	NSSI → assess suicide and medical risk, build coping; no ultimatums
147	Counselor in danger from client → Termination permitted (A.11.c)
148	Client disagrees with the counselor → Explore non-defensively (not defend or appease)
149	Readiness has shifted → Reassess stage of change, revise plan
150	Behavior, intervention, response, plan → BIRP
151	Tell me → tell Ray directly: facilitating member interaction
152	Fear of death drives worldview defense → terror management theory (not Yalom or Frankl)
153	Vocational identity → My Vocational Situation (Holland)
154	Six R processes → Rando (not Worden)
155	Safety plan step 1 → warning signs (Stanley & Brown)
156	Repaired rupture strengthens the alliance → Safran & Muran (not Bordin)
157	Terminally ill client considering hastening death → consult, option to keep confidentiality (ACA B.2.b)
158	Misperceived real sound → Illusion (not hallucination)
159	Belief about what an action will lead to → outcome expectations (SCCT, not Krumboltz)

#	Pair
160	Confusion → comparison → tolerance → acceptance → pride → synthesis → Cass (not Marcia)
161	Accurate but formulaic reflections → Missing empathic attunement (6.G)
162	Heavy daily drinker stopping abruptly → Medical evaluation (alcohol withdrawal can be dangerous)
163	Internet gaming disorder → DSM-5-TR condition for further study; ICD-11 gaming disorder
164	Cynicism and detachment from clients → depersonalization (Maslach burnout), not dissociation
165	Duty to warn (1974) → Tarasoff I; duty to protect (1976) → Tarasoff II
166	Top dog / underdog → Perls (Gestalt), not Berne's ego states
167	Ethics-law conflict → State commitment to the Code, seek resolution (I.1.c)
168	Client defines his own problem → Respect for diversity (not assuming the disability is the issue)
169	Childhood adversity → ACEs (not Holmes & Rahe)
170	Labeling emotions to regulate them → Siegel (name it to tame it)
171	Interests, not positions → Fisher & Ury
172	Breach notification, stronger HIPAA enforcement → HITECH Act
173	Viewing a client's social media → Only with consent (H.6.c)

#	Pair
174	Adult who grew up with an alcoholic parent → ACoA
175	Final step: clinically significant distress or impairment → Boundary with no mental disorder (First)
176	Expressing the opposite of what one feels → reaction formation
177	Understanding of one's own problem → Insight (not judgment)
178	Would I want this in the news? → Publicity test (Stadler)
179	Raise termination early → Starting with informed consent
180	Sedative withdrawal → potentially life-threatening; medically managed withdrawal
181	Beliefs about worry → Wells (metacognitive), not Dugas
182	Brief, relevant, back to the client → appropriate self-disclosure
183	Others feel it too → Universality (Yalom)
184	1 day to under 1 month, full recovery → brief psychotic disorder (not schizophreniform)
185	Keeping a promise → Fidelity
186	Outside event during the study → History
187	Relaxation response → Benson (not Jacobson's progressive relaxation)
188	BSFT → Szapocznik (not Liddle's MDFT)
189	Tolerance/withdrawal from prescribed use → Not counted toward a substance use disorder

#	Pair
190	Preferred activity reinforces a less-preferred one → Premack principle (not token economy)
191	Ruling out options by prestige → social valuation (Gottfredson)
192	Offering a concrete strategy like sensate focus → PLISSIT specific suggestions (Annon)
193	Past retention period → Confidential disposal (B.6.h)
194	Binges without compensatory behavior → binge-eating disorder (not bulimia)
195	Random selection within proportional subgroups → Stratified random sampling
196	Awareness of the counselor's power and privilege → Self-awareness (6.A)
197	Disability from environmental barriers → social model (not medical)
198	Adolescent adoptee asks about birth family → normal identity issue (Silverstein & Kaplan)
199	Presence of a concrete behavior → Well-formed goal (de Shazer), applied rewrite
200	Collections → Advance notice and chance to pay (A.10.d)

WORKBOOK

Study Plan, Drills, and Trackers

Workbook — Study Plan, Drills, and Trackers

This workbook turns the book into a routine. Use it alongside the chapters and the audio companion.

Part 1 — The 8-Week Plan

Total time: about 80 hours (10 hours a week), allocated by scored items (Chapter 1). Adjust the weeks to your exam date; the proportions matter more than the calendar.

Domain	Scored items	Hours
Counseling Skills & Interventions	48	24
Areas of Clinical Focus	47	23
Professional Practice & Ethics	19	10
Intake, Assessment & Diagnosis	19	10
Treatment Planning	14	7
Core Counseling Attributes	13	6

Every Day

- **15 minutes** of ownership drills (Chapter 2 three-pass drill, or the flash pairs below)
- **One audio track** during a drive, walk, workout, or chore. Repeat a track until you can answer every item before the pause ends.
- **5 minutes** logging any misses in the error log (Part 4)

Week by Week

Week	Read	Audio	Practice
1	Ch 1, 2	2.1, 2.2	Ch 1–2 quick checks as a baseline; log every miss by domain
2	Ch 3, 4	2.3, 2.4, 3.1, 4.1	Ch 3–4 quick checks; three-pass drill daily
3	Ch 5, 6, 7	5.1, 6.1, 7.1	Quick checks; review error log patterns
4	Ch 8, 9, 10	3.3, 8.1, 9.1, 10.1	Quick checks; time yourself at 60 seconds per item
5	Ch 11, 12, 13	11.1, 12.1, 13.1	**Practice Exam 1** under timed conditions (3 hours 45 minutes); review with the Ownership Map
6	Ch 14, 15, 16	3.2, 14.1, 15.1, 16.1	Quick checks; drill every pair you missed on Exam 1
7	Ch 17, 18, 19, 20	3.4, 17.1, 18.1, 19.1, 20.1	**Practice Exam 2**; compare domain scores with Exam 1
8	Ch 21, 22; review weakest domains	21.1, 22.1; replay Ch 2 tracks	**Practice Exam 3** at least 3 days before the test; final error-log review; rest the day before

If you have less time: keep the daily audio and ownership drills, read the chapters for your weakest domains first, and take at least one full timed practice exam.

Part 2 — Flash Pairs: 83 Most-Swapped Owners

Cut these into cards or cover the right column. Say the answer out loud.

Theories (Chapter 2)

Term	Owner
Social interest; birth order	Adler
Collective unconscious; archetypes	Jung
Free association; defense mechanisms	Freud
Unconditional positive regard	Rogers
Empty chair; unfinished business	Perls (Gestalt)
Logotherapy; paradoxical intention	Frankl
Four givens: death, freedom, isolation, meaninglessness	Yalom
Reciprocal inhibition; SUDS	Wolpe
Shaping; schedules of reinforcement	Skinner
Modeling; self-efficacy	Bandura
Disputing irrational beliefs; A-B-C	Ellis (REBT)
Cognitive triad; automatic thoughts	Beck
WDEP; quality world	Glasser / Wubbolding
Stress inoculation	Meichenbaum
BASIC I.D.	Lazarus
Parent, Adult, Child ego states	Berne (TA)
OARS; change talk	Miller & Rollnick (MI)
Precontemplation → maintenance	Prochaska & DiClemente
Distress tolerance; wise mind	Linehan (DBT)
Defusion; psychological flexibility	Hayes (ACT)
Miracle question; exceptions; scaling	de Shazer & Berg (SFBT)
Externalizing; unique outcomes	White & Epston (narrative)
Differentiation; triangles; genogram	Bowen

Term	Owner
Boundaries; enmeshment; enactment	Minuchin
Directives; ordeal	Haley
Circular questioning	Milan group
Double bind	Bateson
Placater, blamer, super-reasonable	Satir
Four Horsemen	Gottman
Pursue–withdraw; softening	Sue Johnson (EFT)

Development, Career, Group, Testing, Research (Chapter 3)

Term	Owner
Industry vs. inferiority	Erikson
Conservation; object permanence	Piaget
Zone of proximal development	Vygotsky
Law and order; social contract	Kohlberg
Ethic of care	Gilligan
Foreclosure; moratorium	Marcia
Strange Situation	Ainsworth
Imaginary audience; personal fable	Elkind
Tasks of mourning	Worden
Alarm, resistance, exhaustion	Selye
Encounter; immersion–emersion	Cross
Contact; disintegration; reintegration	Helms
Integration vs. assimilation (acculturation)	Berry
RIASEC	Holland
Life-Career Rainbow	Super
Fantasy, tentative, realistic	Ginzberg
Planned happenstance	Krumboltz
Circumscription and compromise	Gottfredson
Self-efficacy in career (SCCT)	Lent, Brown, & Hackett
Forming, storming, norming	Tuckman
Transition and working stages	Corey

Term	Owner
Universality; instillation of hope	Yalom
Psychodrama; role reversal	Moreno
Fight–flight; pairing	Bion
Deviation IQ	Wechsler
g factor	Spearman
Fluid vs. crystallized	Cattell
Threats to internal validity	Campbell & Stanley
Grounded theory	Glaser & Strauss

Clinical, Ethics, and Beyond (Chapters 4–22)

Term	Owner / answer
SOLER	Egan
Five levels of empathy	Carkhuff
Goals, tasks, bond	Bordin
Seven-stage crisis model	Roberts
Interpersonal theory of suicide	Joiner
Safety Planning Intervention	Stanley & Brown
Duty to protect (1976)	Tarasoff II
Cycle of violence	Walker
Learned helplessness	Seligman
Rumination prolongs depression	Nolen-Hoeksema
Two-factor theory of fear	Mowrer
Safety, remembrance and mourning, reconnection	Herman
Window of tolerance	Siegel
Biosocial theory	Linehan
Ambiguous loss	Boss
Dual process model of grief	Stroebe & Schut
Socioemotional selectivity	Carstensen
Goals of misbehavior	Dreikurs
Coercion theory	Patterson
Minority stress	Meyer

Term	Owner / answer
PLISSIT	Annon
Transactional model of stress	Lazarus & Folkman
Six necessary and sufficient conditions	Rogers
Seven-step ethical decision model	Forester-Miller & Davis

Part 3 — The Four-Step Item Routine (Pocket Card)

1. **Read the last line first.** What's being asked?
2. **Name the owner.** Which theory, domain, or framework is this item living in?
3. **Predict, then look.**
4. **Eliminate by owner.** Who owns each wrong option?

For applied items: safety → relationship → assessment → code and law → scope → client autonomy.

For statistics items: how many groups, what kind of data, difference or relationship?

Part 4 — Error Log

Copy this table into a notebook or spreadsheet. Fill it in for every missed or guessed item.

#	Domain	Topic	My answer	Who/ what owns my answer	Correct answer	Who/ what owns the correct answer	Why I missed it (didn't know / confused pair / misread / rushed)
1							
2							
3							

Review weekly: count misses by domain and by reason. If “confused pair” dominates, spend more time on the swap-pair tables and audio. If “misread” dominates, slow down on step 1. If “rushed” dominates, practice pacing.

Part 5 — Practice Exam Score Worksheet

Domain	Items on practice exam	Correct	%	Target
Counseling Skills & Interventions	60			
Areas of Clinical Focus	58			
Professional Practice & Ethics	24			
Intake, Assessment & Diagnosis	24			
Treatment Planning	18			
Core Counseling Attributes	16			
Total	**200**			

The practice exams scale NBCC’s scored-item weights up to 200 items. **There’s no official passing percentage** (Chapter 1), so use these results to find weak domains, not to predict a pass. Your two weakest domains get the next week’s extra study time.

Part 6 — Pacing Checkpoints

Write these on your scratch materials at the start of the exam.

Item	Target time elapsed	Hard limit
50	0:50	0:56
100	1:40	1:52
150	2:30	2:49
200	3:20	3:45

Part 7 — Test-Day Checklist

The week before - [ ] Confirm the test center location, time, and ID requirements in your registration materials - [ ] Take your last timed practice exam no later than 3 days before - [ ] Review your error log, not new material

The day before - [ ] Light review only: one or two audio tracks - [ ] Prepare ID and directions - [ ] Sleep

Exam day - [ ] Eat, hydrate, arrive early - [ ] Write pacing checkpoints on scratch materials - [ ] Use the four-step routine on every item - [ ] Flag an item only when you can name a reason to revisit it - [ ] Answer every item. Strange items may be unscored; don’t let them shake you - [ ] Check that nothing is blank before submitting

BACK MATTER

“Who Owns It?” Master Index

Every swap pair in the book, alphabetized by the term you’ll see in a question stem. Find the term, name the owner, and note the look-alike it gets confused with. The chapter column points to the full teaching. If the owner surprises you, that pair goes in your error log.

How to drill this index: cover the “Owner” column, read the term, and say the owner out loud. Then cover the “Term” column, read an owner, and name one term it owns. The “Not…” column is the distractor the exam will offer you instead.

381 pairs from Chapters 2–22. Chapter 4’s microskills drill uses sample counselor responses rather than terms, so it lives in Chapter 4 only.

Numbers

Term	Owner	Not…	Ch
3 of 15 criteria, 12 months, violating rights	Conduct disorder	ODD	16
4 S’s of transition	Schlossberg	Super (career stages)	3

A

Term	Owner	Not…	Ch
Acceptance plus change in couples	IBCT (Christensen & Jacobson)	EFT	7
Accommodations without special education	Section 504	IDEA	12

Term	Owner	Not…	Ch
Accurate empathy, nonpossessive warmth, genuineness	Truax & Carkhuff	Rogers' six conditions	22
ACE Study	Felitti & Anda	van der Kolk	10
Adjacent top codes	Consistency	Differentiation	14
Adolescent family therapy for substance use	BSFT (Szapocznik), MDFT (Liddle)	Filial therapy	11
Adult romantic attachment styles	Hazan & Shaver	Ainsworth (infants)	13
Adults' level of care tool	LOCUS	ASAM	19
Alarm, resistance, exhaustion	Selye	Kübler-Ross	3
Anomic suicide	Durkheim	Shneidman	6
Antecedent, behavior, consequence	Functional behavior assessment	Conflict resolution	16
Approves programs	Accreditation (CACREP)	Certification	17
Archetypes; collective unconscious; persona	Jung	Adler or Freud	2
Associations, affect, ambivalence, autism	Bleuler	Kraepelin	16
At least 4 symptoms, 6 months, angry and defiant	ODD	Conduct disorder	16
Ataque de nervios, susto	Cultural concepts of distress	Panic disorder	15

Term	Owner	Not…	Ch
Attention, power, revenge, inadequacy	Dreikurs	Baumrind	13
Attorney-issued document requesting records	Subpoena	Court order	17
Authoritative vs. authoritarian	Baumrind	Thomas & Chess (temperament, not parenting)	3
Autocratic, democratic, laissez-faire	Lewin, Lippitt, & White	Yalom	3
Avoiding harm	Nonmaleficence	Beneficence	17
Awareness, knowledge, skills	Sue, Arredondo, & McDavis	MSJCC (Ratts et al.)	9

B

Term	Owner	Not…	Ch
BASIC I.D.	Lazarus	Beck	2
Bed only for sleep; get up if awake	Stimulus control (Bootzin)	Sleep restriction (Spielman)	16
Behavior, intervention, response, plan	BIRP	DAP	21
Being real, no façade	Genuineness	Empathy	22
Beliefs about worry	Wells (metacognitive)	Dugas	10
Binge + compensatory behavior	Bulimia nervosa	Binge-eating disorder	16
Biopsychosocial model	Engel	Rosenstock	16
Biosocial theory of dysregulation	Linehan	Gross	10

Term	Owner	Not…	Ch
Blind and hidden areas	Luft & Ingham (Johari Window)	Yalom	3
Blocking career beliefs	Career Beliefs Inventory (Krumboltz)	Career Thoughts Inventory	14
Boundaries; enmeshment; enactment; joining	Minuchin	Bowen or Haley	2
Building theory from data	Grounded theory (Glaser & Strauss)	Phenomenology	18

C

Term	Owner	Not…	Ch
Career adaptability; career stories	Savickas	Super (who used “career maturity”)	3
Career decision-making styles: rational, intuitive, dependent	Harren	Tiedeman	14
Career indecision	Career Decision Scale (Osipow)	Career Maturity Inventory (Crites)	14
CASVE cycle	Peterson, Sampson, & Reardon	Tiedeman & O’Hara	3
Catastrophic misinterpretation of body sensations	Clark	Borkovec	10
CBT-based, non-12-step mutual support	SMART Recovery	AA	9
Change in the rules themselves	Second-order change	First-order change	7

Term	Owner	Not…	Ch
Changes because they know they're observed	Hawthorne effect	Placebo effect	18
Changes when or for whom an effect holds	Moderator	Mediator	18
Chasing losses	Gambling disorder	Substance use disorder	11
Chronic difficulty deciding anything	Indecisiveness	Career indecision	14
Circular questioning; positive connotation; invariant prescription	Milan	MRI	2
Circumscription and compromise	Gottfredson	Ginzberg (who also used "compromise")	3
Client must perceive empathy and UPR	Rogers' sixth condition	Rogers' first condition	22
Client says "I feel down"	Mood	Affect	19
Client's collaborative follow-through with treatment	Adherence	Compliance (older term)	21
Client's own words in the note	S in SOAP	O	21
Client's right to choose	Autonomy	Beneficence	17
Clinician explains why criteria aren't met	Other specified	Unspecified	19
Coaching parents live through an earpiece	PCIT (Eyberg)	Filial therapy	5

Term	Owner	Not…	Ch
Cocoanut Grove fire; acute grief	Lindemann	Caplan	6
Coercion theory	Patterson	Baumrind	13
Coercive control	Stark	Walker	6
Cognitive triad; collaborative empiricism; Socratic questioning	Beck (CT)	Ellis	2
Cohesion and flexibility	Olson’s Circumplex Model	McMaster Model	7
Coined “ageism”	Robert Butler	Rowe & Kahn	12
Commenting on how the group interacts	Process observation	Summarizing content	8
Commitment, control, challenge	Kobasa (hardiness)	Lazarus	16
Communicating understanding	Empathic responding	Empathic attunement	22
Compare group means on two or more IVs	Factorial ANOVA	One-way ANOVA	18
Compare groups on two or more DVs	MANOVA	ANCOVA	18
Compare two group means	t-test	ANOVA	18
Compares interests with satisfied workers	Strong Interest Inventory	SDS	14
Conflict matters more than divorce	Amato	Wallerstein	13
Conformity → integrative awareness	Atkinson, Morten, & Sue	Cross	3

Term	Owner	Not…	Ch
Connecting two members' concerns	Linking	Universalizing (the leader names a shared theme for the whole group)	8
Conservation; object permanence	Piaget	Erikson or Vygotsky	3
Contact; disintegration; reintegration	Helms	Cross	3
Contemplation	Prochaska & DiClemente (stages of change)	Corey's group stages (contemplation is never a group stage)	3
Contempt predicts divorce	Gottman	Sue Johnson (EFT)	7
Control group tries harder	John Henry effect	Hawthorne effect	18
Correlates with similar measures	Convergent validity	Discriminant validity	3
Counselor can't evaluate own client for court	ACA E.13.c	ACA A.8	13
Counselor imposes own culture as reality	Cultural encapsulation (Wrenn)	Microaggression	9
Counselor observes tearful, restricted expression	Affect	Mood	19
Counselor-observed affect; test score	O in SOAP	S	21
Crash: fatigue, hypersomnia, hunger, low mood	Stimulant withdrawal	Cannabis withdrawal	11
Critical consciousness	Freire	Martín-Baró	15

Term	Owner	Not…	Ch
Crystallizing, specifying, implementing	Super's exploration stage	Ginzberg	14

D

Term	Owner	Not…	Ch
Danger Assessment	Campbell	Posner (C-SSRS, suicide)	6
Day program, home at night	Partial hospitalization	Residential	19
Demonstrably effective relationship elements	Norcross & Lambert	Wampold	22
Desire, Ability, Reasons, Need	Change talk (MI)	Stages of change	5
Desire, excitement, orgasm	Kaplan	Masters & Johnson	16
Deviation IQ	Wechsler	Terman (ratio IQ)	3
Differentiation; triangles; genogram; cutoff	Bowen	Minuchin	2
Directives; ordeal; prescribing the symptom	Haley (strategic)	Minuchin	2
Disability from environmental barriers	Social model	Medical model	9
Disputing irrational beliefs; "musts"	Ellis (REBT)	Beck: Beck *tests* automatic thoughts collaboratively; Ellis *disputes* beliefs directly	2
Distress about gender incongruence	Gender dysphoria	Being transgender	15

Term	Owner	Not...	Ch
Doing good	Beneficence	Nonmaleficence	17
Double bind	Bateson / MRI	Milan	2
Duty to warn (1974)	Tarasoff I	Tarasoff II (duty to protect, 1976)	6
Duty, dereliction, direct causation, damages	Malpractice	Ethical complaint	17

E

Term	Owner	Not...	Ch
Ego states (Parent, Adult, Child); strokes; games	Berne (TA)	Freud's id/ego/ superego	2
Ego-dystonic obsessions	OCD	OCPD (ego-syntonic)	10
Emerging adulthood	Arnett	Levinson	12
Employment tests must be job-related	*Griggs v. Duke Power*	*Larry P. v. Riles*	20
Empty chair; unfinished business; "what" and "how"	Perls (Gestalt)	Rogers: Rogers is nondirective and reflective	2
Encounter; immersion–emersion	Cross	Helms	3
Eros, ludus, storge, pragma, mania, agape	Lee	Sternberg	13
Essence of lived experience	Phenomenology	Ethnography	18
Ethic of care	Gilligan	Kohlberg	3
Ethical duty not to disclose	Confidentiality	Privilege	17

Term	Owner	Not…	Ch
Evaluative, ongoing, shared responsibility	Supervision	Consultation	17
Excessive thoughts about real physical symptoms	Somatic symptom disorder	Illness anxiety disorder	16
Excitement, plateau, orgasm, resolution	Masters & Johnson	Kaplan	16
Exhaustion, depersonalization, reduced accomplishment	Maslach (burnout)	Figley (compassion fatigue)	14
Expected rejection, concealment, internalized stigma	Proximal minority stressors (Meyer)	Distal stressors	15
Experiencing, then reflecting on, what just happened in the room	Here-and-now (Yalom)	Catharsis	8
Explains how the IV affects the DV	Mediator	Moderator	18
Externalizing; unique outcomes; re-authoring	White & Epston (narrative)	SFBT	2
Extra protection for process notes	HIPAA psychotherapy notes	42 CFR Part 2	17
Extreme scorers drift toward the mean on retest	Statistical regression	Maturation	3
Extreme scorers move toward the mean	Statistical regression	Testing	18

F

Term	Owner	Not…	Ch
Face the obsession, block the compulsion	ERP	Systematic desensitization	5
Fairness and equal access	Justice	Autonomy	17
Faith develops through six stages	Fowler	Kohlberg	9
Fakes symptoms for money or to avoid prosecution	Malingering	Factitious disorder	19
Fakes symptoms to be seen as sick	Factitious disorder	Malingering	19
False positive	Type I	Type II	18
Family acceptance protects LGBTQ youth	Caitlin Ryan	Meyer	15
Family criticism and over-involvement predict relapse	Expressed emotion (Brown; Vaughn & Leff)	Double bind	16
Family's links to school, church, agencies	Ecomap (Hartman)	Genogram	7
Fantasy, immersion, awareness…	Papernow	Visher & Visher	13
Fantasy, tentative, realistic	Ginzberg et al.	Super	3
Fear acquired by classical, maintained by operant	Mowrer's two-factor theory	Wolpe	10

Term	Owner	Not…	Ch
Fear of confirming a stereotype lowers performance	Stereotype threat (Steele & Aronson)	Impostor phenomenon	15
Fear of death drives worldview defense	Terror management theory	Yalom	15
Fear of illness with few or no symptoms	Illness anxiety disorder	Somatic symptom disorder	16
Feedback that keeps things the same	Negative feedback (morphostasis)	Positive feedback	7
Feeling like a fraud	Impostor phenomenon (Clance & Imes)	Stereotype threat	15
Feeling sorry for the client	Sympathy	Empathy	22
Feeling with, from the client's frame	Empathy	Sympathy	22
Fight–flight; pairing	Bion	Tuckman	3
Five faces of oppression	Iris Marion Young	Freire	15
Fixed questions in fixed order	Structured interview (SCID-5)	Unstructured	19
Fluid vs. crystallized	Cattell (and Horn)	Gardner	3
Focus on meaningful relationships as time shortens	Socioemotional selectivity (Carstensen)	Continuity theory	12
Foreclosure; moratorium	Marcia	Erikson (Marcia built on Erikson's identity stage)	3

Term	Owner	Not…	Ch
Former client romantic relationship	Prohibited for 5 years (A.5.c)	2 years	17
Four givens (death, freedom, isolation, meaninglessness)	Yalom	Frankl, who focuses on meaning	2
Four Horsemen	Gottman	Sue Johnson (EFT)	2

G

Term	Owner	Not…	Ch
g factor	Spearman	Thurstone (primary mental abilities)	3
Gambling disorder severity: 6–7 criteria	Moderate	Mild	11
Gaming addiction: DSM condition for further study	Internet gaming disorder	Gambling disorder	11
Gender identity vs. sexual orientation	Distinct constructs	The same thing	15
General systems theory	von Bertalanffy	Wiener (cybernetics)	7
Good-enough mother; holding environment	Winnicott	Bowlby	2
Grief not socially recognized	Doka (disenfranchised grief)	Worden	6
Group equivalent of the alliance	Cohesiveness	Universality	8

H

Term	Owner	Not…	Ch
Hearing voices with no source	Hallucination	Illusion	19
Hero, scapegoat, lost child, mascot	Wegscheider-Cruse	Satir’s stances	9
Hierarchy of needs	Maslow	Rogers	2
High-effort coping and health	John Henryism (Sherman James)	Stereotype threat	15
High-risk period after hospital discharge	Prompt follow-up	Discharge without plan	21
Home- and community-based treatment for antisocial youth	MST (Henggeler)	FFT	16
Honeymoon, crisis, recovery, adaptation	Oberg’s culture shock	Kübler-Ross	15
Honeymoon, disenchantment, reorientation	Atchley’s retirement phases	Kübler-Ross	12
Hopelessness predicts suicide	Beck	Joiner	6

I

Term	Owner	Not…	Ch
“I don’t see color”	Microinvalidation	Microassault	9
“I’ve blown it, so I might as well keep going”	Abstinence violation effect (Marlatt)	Contemplation	5
I-messages	Thomas Gordon	Ellis	5

Term	Owner	Not…	Ch
Identify gaps before designing	Needs assessment	Outcome evaluation	18
Identity confusion → synthesis	Cass	Marcia (identity statuses, not sexual identity)	3
Imaginary audience; personal fable	Elkind	Piaget	3
Improve a program in progress	Formative evaluation	Summative evaluation	3
Improve a program while running	Formative	Summative	18
Incredible Years	Webster-Stratton	Sanders (Triple P)	13
Industry vs. inferiority	Erikson	Piaget (stages named by cognition, not crisis)	3
Inflated responsibility in OCD	Salkovskis	Rachman	10
Inner experience matches outward expression	Congruence	Positive regard	22
Insomnia: 3 nights a week for 3 months	Insomnia disorder	Hypersomnolence	16
Instant love myth; wicked stepmother	Visher & Visher	Papernow	13
Intact groups, no random assignment	Quasi-experiment	True experiment	18
Integration vs. assimilation	Berry	Piaget’s assimilation (a cognitive process)	3

Term	Owner	Not...	Ch
Intense imagined exposure	Implosive therapy (Stampfl)	Flooding (real life)	5
Intentional, repeated, power imbalance	Olweus' definition of bullying	Conflict	16
Internal, stable, global attributions	Abramson, Seligman, & Teasdale	Lewinsohn	10
Intimacy, passion, commitment	Sternberg	Lee	13
Intimate terrorism vs. situational couple violence	Michael P. Johnson	Walker's cycle	13
Intolerance of uncertainty	Dugas	Wells	10
Intrinsic vs. extrinsic religiosity	Allport	Pargament	15
Inviting a quiet member in	Drawing out	Linking	8
Items hang together	Internal consistency	Test-retest	3

J

Term	Owner	Not...	Ch
Judge-signed order	Court order	Subpoena	17

K

Term	Owner	Not...	Ch
Keeping promises	Fidelity	Veracity	17
Know yourself + know the work world + match	Parsons	Williamson (who systematized it)	3

L

Term	Owner	Not…	Ch
Law and order; social contract	Kohlberg	Gilligan	3
Learned helplessness	Seligman	Beck	10
Learning how one affects others	Interpersonal learning	Imitative behavior	8
Legal right in court; client holds it	Privileged communication	Confidentiality	17
Licensure for telehealth	Where the client is located	Where the counselor is located only	9
Life-Career Rainbow; self-concept; maintenance stage	Super	Holland or Ginzberg	3
Locus of control and locus of responsibility	Sue's worldview model	Rotter's locus of control alone	9
Loss, rejection, guilt, grief, identity, intimacy, mastery	Silverstein & Kaplan	Worden	13

M

Term	Owner	Not…	Ch
Mean 50, SD 10	T-score	Stanine or z	3
Mean 50, SD 10	T-score	Normal curve equivalent (SD 21.06)	20
Meaning; logotherapy; paradoxical intention	Frankl	Haley's paradoxical *directives*	2

Term	Owner	Not…	Ch
Members can join anytime	Open group	Closed group	8
Members test the leader and conflict appears	Transition (Corey) / storming (Tuckman)	Initial / forming	8
Miracle question; exceptions; scaling	de Shazer & Berg (SFBT)	Narrative	2
Missing an intimate attachment	Emotional loneliness (Weiss)	Social loneliness	16
Modeling; self-efficacy	Bandura	Skinner	2
Modern hospice movement	Cicely Saunders	Kübler-Ross	12
Moral, religious, or spiritual problem	Z65.8 (focus of attention, not a disorder)	A mental disorder	15
Most children of divorce adjust well	Hetherington	Wallerstein	13
Motor and vocal tics, over 1 year, onset before 18	Tourette’s disorder	Persistent tic disorder	12
Mutual withdrawal of elders and society	Disengagement theory (Cumming & Henry)	Activity theory	12

N

Term	Owner	Not…	Ch
Needs and press	Murray (TAT)	Maslow	3
No logical connection between ideas	Loose associations	Flight of ideas	19

Term	Owner	Not…	Ch
Nominal frequencies	Chi-square	Pearson r	3
Nominal frequencies	Chi-square	t-test	18
Nonverbal reasoning	Raven’s	WAIS verbal index	20
NREM, early night, little memory	Sleep terrors	Nightmares	16

O

Term	Owner	Not…	Ch
OARS; change talk; rolling with resistance	Miller & Rollnick (MI)	Prochaska & DiClemente	2
Observer is part of the system	Second-order cybernetics (von Foerster)	First-order	7
OCEAN	NEO-PI (Costa & McCrae)	16PF	20
One type clearly highest	Differentiation	Congruence	14
Ongoing connection to the deceased is healthy	Continuing bonds (Klass et al.)	Disenfranchised grief	12
Opposite on the hexagon	R–S, I–E, A–C	Adjacent types	14
Oscillating between loss and restoration	Dual process model (Stroebe & Schut)	Worden’s tasks	12
Outside event during the study	History	Maturation	18

P

Term	Owner	Not…	Ch
Parent/teacher behavior ratings	CBCL (Achenbach)	MMPI-A	20
Parents run play sessions at home	Filial therapy (Guerney)	Child-centered play therapy (Axline/Landreth)	5
Parents take charge of eating	Family-based treatment (Maudsley)	Structural family therapy	5
Parent–child relationship shapes career	Roe	Super	3
Participants drop out unevenly	Mortality/ attrition	Selection	3
Participants improve because they’re observed	Hawthorne effect	Placebo effect	3
Percentage scoring at or below	Percentile rank	Percentage correct	20
Permission, limited information, specific suggestions, intensive therapy	PLISSIT (Annon)	Sensate focus	16
Person, place, time	Orientation ×3	Insight	19
Person–environment fit	Congruence	Consistency	14
Physically present, psychologically absent	Ambiguous loss (Boss)	Disenfranchised grief (Doka)	12
Pinpoint pupils, slow breathing	Opioid intoxication	Stimulant intoxication	11
Placater, blamer, super-reasonable, irrelevant; sculpting	Satir	Bowen	2

Term	Owner	Not…	Ch
Planned happenstance; learning experiences	Krumboltz	Savickas	3
Positive vs. negative religious coping	Pargament	Allport	15
Posttraumatic growth	Tedeschi & Calhoun	Bonanno	10
Prealcoholic, prodromal, crucial, chronic	Jellinek	Prochaska	11
Precontemplation → maintenance	Prochaska & DiClemente	MI or group stages	2
Predicts later performance	Predictive validity	Concurrent validity	3
Predisposing, precipitating, perpetuating, protective	4 Ps case formulation	Biopsychosocial model	19
Preventive psychiatry; crisis lasts about 4–6 weeks	Caplan	Lindemann	6
Primary and secondary appraisal	Lazarus & Folkman	Selye	16
Privileged/ marginalized counselor and client quadrants	MSJCC (Ratts et al.)	Sue et al. (1992)	9
Process model of emotion regulation	Gross	Linehan	10
Program evaluation: outcomes in dollars	Cost-benefit	Cost-effectiveness	18
Prolonged grief disorder in adults	12 months	6 months (children and adolescents)	12

Term	Owner	Not…	Ch
Protagonist; auxiliary ego; role reversal	Moreno (psychodrama)	Perls (Gestalt also uses enactment, but these terms are Moreno's)	3
Protest → despair → detachment	Bowlby (with Robertson)	Kübler-Ross	12
Psychache	Shneidman	Joiner	6
Psychotic symptoms 1 day to under 1 month, full recovery	Brief psychotic disorder	Schizophreniform	16
Pursue–withdraw cycle; softening; attachment in couples	Sue Johnson (EFT)	Gottman	2

R

Term	Owner	Not…	Ch
Race-based traumatic stress	Robert Carter	Sue	15
Raising culture directly in session	Broaching (Day-Vines)	Cultural humility	9
Random assignment	True experiment	Quasi-experiment	18
Random assignment present	True experiment	Quasi-experiment	3
Rapid jumps between related topics	Flight of ideas	Loose associations	19
Rapprochement	Mahler	Bowlby	3
Ratio IQ (MA/CA × 100)	Terman / Stanford-Binet	Wechsler	3
Reducing frequency before ending	Tapering	Abrupt termination	21

Term	Owner	Not…	Ch
Referring a client because of the counselor's beliefs	Values-based referral (prohibited, ACA A.11.b)	Competence-based referral	9
Rejecting a true null	Type I	Type II	3
Relaxation + imagined hierarchy	Systematic desensitization (Wolpe)	Flooding	5
Relaxation response	Herbert Benson	Edmund Jacobson (progressive relaxation)	5
REM, later night, remembered	Nightmares	Sleep terrors	16
Replaced the DOT	O*NET	OOH	14
Replaced the GAF	WHODAS 2.0	Multiaxial Axis V	19
Respect for persons, beneficence, justice	Belmont Report	ACA principles	18
Responsive desire	Basson	Kaplan	16
Restriction without body-image concern	ARFID	Anorexia	16
RIASEC; hexagon; congruence	Holland	Super	3
Rumination prolongs depression	Nolen-Hoeksema	Beck	10
Runny nose, yawning, goosebumps, diarrhea	Opioid withdrawal	Alcohol withdrawal	11

S

Term	Owner	Not…	Ch
Safety Planning Intervention	Stanley & Brown	Jobes (CAMS)	6
Safety, remembrance and mourning, reconnection	Herman	Kübler-Ross	10
Same start, different outcomes	Multifinality	Equifinality	7
Satisfaction, alternatives, investments	Rusbult	Gottman	13
Schizophrenia symptoms 1 to under 6 months	Schizophreniform	Schizophrenia	16
Second dip on returning home	W-curve; reverse culture shock	U-curve	15
Selection, optimization, compensation	Baltes & Baltes	Carstensen	12
Self-administered, self-scored RIASEC	SDS (Holland)	Strong Interest Inventory	14
Self-efficacy + outcome expectations in career	Lent, Brown, & Hackett (SCCT)	Krumboltz	3
Self-insight, self-integration, anxiety management	Managing countertransference (Hayes & Gelso)	Burnout prevention	22
Sensing the client’s experience internally	Empathic attunement	Empathic responding	22
Separation–individuation	Mahler	Erikson	2

Term	Owner	Not…	Ch
Seven-stage crisis intervention model	Roberts	James & Gilliland	6
Seven-step ethical decision-making model	Forester-Miller & Davis	Kitchener	17
Severity based on adaptive functioning	Intellectual developmental disorder	ADHD	12
Severity rated by levels of support needed	Autism spectrum disorder	Intellectual disability	12
Shadow mistaken for a person	Illusion	Hallucination	19
Shaping; token economy; schedules of reinforcement	Skinner / operant	Wolpe	2
Shattered assumptions	Janoff-Bulman	Tedeschi & Calhoun	10
“She nags because he withdraws because she nags”	Circular causality	Linear causality	7
Six dimensions incl. roles and behavior control	McMaster Model (Epstein)	Circumplex	7
Six necessary and sufficient conditions	Rogers (1957)	Truax & Carkhuff	22
Six R processes	Rando	Worden	12
Six stations of divorce	Bohannan	Kübler-Ross	13
Six-step crisis model: listening, then acting	James & Gilliland	Roberts	6

Term	Owner	Not…	Ch
Sleeper effect	Wallerstein	Hetherington	13
Small, concrete, presence of something	Well-formed goal (de Shazer)	SMART	21
Smoothing over pain too fast	Rescuing / band-aiding	Supporting	8
Social interest; birth order; lifestyle	Adler	Freud: Freud is drives and psychosexual stages	2
Social, problem, interaction, goal-setting, task-setting	Haley's first interview	Roberts' crisis model	7
Speaker–listener technique	PREP (Markman et al.)	Gottman	7
Special education with an IEP	IDEA	Section 504	12
Speech wanders and never gets to the point	Tangential	Circumstantial	19
Speech wanders but gets to the point	Circumstantial	Tangential	19
Staggered intervention across behaviors	Multiple baseline	ABAB	18
State permission to practice	Licensure	Certification	17
State vs. trait anxiety	Spielberger (STAI)	Beck (BAI)	10
STEP	Dinkmeyer & McKay	Gordon (PET)	13

Term	Owner	Not…	Ch
Stopping harmful behavior to protect a member	Blocking	Cutting off (managing time or focus)	8
Stories about ambiguous pictures	TAT (Murray)	Rorschach	20
Storming; norming	Tuckman	Corey	3
Strange Situation	Ainsworth	Bowlby (theory) or Harlow (monkeys)	3
Strengths perspective	Saleebey	Maslow	21
Stress inoculation; self-instructional training	Meichenbaum	Beck	2
Structured review of one's whole life	Life review (Butler)	Reminiscence therapy	5
Substances with no DSM withdrawal diagnosis	Hallucinogens, PCP, inhalants	Cannabis (which has one)	11
SUD remission: 12+ months without criteria (except craving)	Sustained remission	Early remission	11
SUD remission: 3–12 months without criteria (except craving)	Early remission	Sustained remission	11
SUD severity: 2–3 criteria	Mild	Moderate	11
SUD severity: 4–5 criteria	Moderate	Severe	11

Term	Owner	Not…	Ch
SUD severity: 6 or more criteria	Severe	Moderate	11
Sudden, fluctuating confusion with poor attention	Delirium	Dementia	12
Supervisor liable for supervisee	Vicarious liability	Direct liability	17
Susceptibility, severity, benefits, barriers, cues	Health belief model	Biopsychosocial model	16
Symptoms before age 12, two or more settings	ADHD	Specific learning disorder	12
Systematic desensitization; reciprocal inhibition; SUDS	Wolpe	Skinner: Skinner is operant (consequences)	2

T

Term	Owner	Not…	Ch
Tail to the right; mean > median	Positive skew	Negative skew	18
Tasks of mourning	Worden	Kübler-Ross (stages of dying)	3
Teaching skills, prevention focus	Psychoeducational group	Counseling group	8
Team consensus on themes	CQR (Hill)	Grounded theory	18
Telling the truth	Veracity	Fidelity	17
Tension → battering → honeymoon	Walker	Duluth (Power and Control Wheel)	6

Term	Owner	Not…	Ch
Termination: raise the ending early	Planned termination	Last-session announcement	21
Test identifies those who don’t have the condition	Specificity	Sensitivity	20
Test identifies those who have the condition	Sensitivity	Specificity	20
Test in the primary language	*Diana v. State Board of Education*	*Hobson v. Hansen*	20
Test must cover what was taught	*Debra P. v. Turlington*	*Griggs*	20
Test of best possible performance	Maximal performance	Typical performance	20
Test of potential to learn	Aptitude	Achievement	20
Test of usual (typical) behavior	Typical performance	Maximal performance	20
Test of what’s been learned	Achievement	Aptitude	20
Test too hard; everyone scores low	Floor effect	Ceiling effect	20
Test with easy items, strict time	Speed test	Power test	20
Test with harder items, generous time	Power test	Speed test	20
Thwarted belongingness; perceived burdensomeness	Joiner	Durkheim	6
Time structure, social contact, purpose, status, activity	Jahoda	Maslach	14

Term	Owner	Not…	Ch
Too little response-contingent reinforcement	Lewinsohn	Seligman	10
Training family to encourage a loved one into treatment	CRAFT (Meyers)	Johnson intervention	9
Transgender emergence	Lev	Cass	15
Transition stage; working stage	Corey	Tuckman	3
Traumagenic dynamics	Finkelhor & Browne	Walker	13
Treatment plan: broad outcome	Long-term goal	Short-term objective	21
Treatment plan: measurable step	Short-term objective	Long-term goal	21
Treatment progress stalls	Review plan, check alliance, reassess	Repeat the same intervention	21
Tremor, sweating, seizures, hallucinations	Alcohol or sedative withdrawal	Opioid withdrawal	11
True score range around observed	SEM confidence band	Standard deviation	20
Two groups, compare means	t-test	ANOVA	3

U

Term	Owner	Not…	Ch
Unconditional positive regard; nondirective	Rogers	Gestalt	2

Term	Owner	Not…	Ch
Universal (culture-general) approach	Etic	Emic	9
Universality; instillation of hope	Yalom	Lewin	3
Using spirituality to avoid pain	Spiritual bypassing (Welwood)	Spiritual emergency (Grof)	15

V

Term	Owner	Not…	Ch
Valuing the person without conditions	Unconditional positive regard	Approval of all behavior	22
Vicarious introspection	Kohut	Rogers	22

W

Term	Owner	Not…	Ch
WDEP; quality world; five needs	Glasser / Wubbolding	Ellis	2
Who owns the problem; I-messages	Gordon (PET)	Dreikurs	13
Window of tolerance	Siegel	Porges	10
Work values	MIQ (Dawis & Lofquist)	Strong	14
Worry as avoidance of emotional arousal	Borkovec	Clark	10
Would I want this in the news?	Publicity test	Universality test	17

Z

Term	Owner	Not…	Ch
Zone of proximal development; scaffolding	Vygotsky	Piaget	3

Glossary

Definitions are short by design. The chapter number after each entry points to the full explanation.

A

Abstinence violation effect — The all-or-nothing reaction to one slip: “I’ve blown it, so I might as well keep going.” Counselors reframe a lapse as a learning event (Alan Marlatt). Compare: Sustain talk. *Ch 5*

Acculturation strategies — Four ways of handling contact with a new culture: integration (keep heritage and join the host culture), assimilation (drop heritage), separation (keep heritage, avoid host), and marginalization (neither) (John Berry). *Ch 3*

Actualizing tendency — Rogers’ term for a person’s inherent drive toward growth; it belongs to a therapy, not a motivational theory (Carl Rogers). Compare: Self-actualization. *Ch 2*

Acute stress disorder — Trauma symptoms right after exposure, in the window from 3 days to 1 month after the trauma. Compare: Posttraumatic stress disorder. *Ch 10*

Adjustment disorder — Emotional or behavioral symptoms in response to an identifiable stressor that don’t meet criteria for another disorder. Symptoms begin within 3 months and resolve within 6 months after the stressor ends. *Ch 10*

Affect — The counselor’s observation of the client’s emotional expression in the moment, described by range, congruence, and lability. Think of it as the “weather.” Compare: Mood. *Ch 19*

Alliance rupture — A strain in the counseling relationship, either a withdrawal rupture (going quiet, agreeing

too easily) or a confrontation rupture (open complaints). Repair by naming and exploring it non-defensively (Safran & Muran). *Ch 4*

Ambiguous loss — A loss without clarity or closure: physically absent but psychologically present, or physically present but psychologically absent, as with a parent who has dementia (Pauline Boss). Compare: Disenfranchised grief. *Ch 12*

ANOVA (one-way) — Tests whether three or more group means differ on one independent variable, using the F statistic (Ronald Fisher). Compare: t-test. *Ch 18*

Assimilation — A word with two owners: fitting new information into an existing schema (Piaget), or giving up one's heritage culture to join the host culture (Berry). The stem's context decides. Compare: Acculturation strategies. *Ch 3*

Autonomy — The ethical principle of fostering the client's right to control the direction of their own life. Compare: Beneficence. *Ch 17*

B

BASIC I.D. — The seven modalities assessed in multimodal therapy: Behavior, Affect, Sensation, Imagery, Cognition, Interpersonal, and Drugs/biology (Arnold Lazarus). *Ch 2*

Beneficence — The ethical principle of working for the good of the individual and society by promoting mental health and well-being. Compare: Nonmaleficence. *Ch 17*

Biopsychosocial model — The view that illness involves biological, psychological, and social factors; the basis for the whole-person intake history (George Engel). Compare: Health belief model. *Ch 16*

Bipolar II disorder — At least one hypomanic episode and at least one major depressive episode, with never a full manic episode. Compare: Bipolar I disorder. *Ch 10*

Blocking — A group leader skill that stops harmful or unproductive behavior while protecting the member. Compare: Cutting off. *Ch 8*

Boundary crossing — A departure from the usual counselor role that may be harmless or even helpful, like a brief condolence card. It calls for careful, documented judgment. Compare: Boundary violation. *Ch 17*

Broaching — The counselor openly raising race, ethnicity, and culture in counseling rather than waiting for the client to bring them up (Norma Day-Vines). Compare: Cultural humility. *Ch 9*

Bulimia nervosa — Recurrent binge eating plus compensatory behaviors such as vomiting, fasting, or excessive exercise, both at least once a week for 3 months. Weight is often in the normal range. Compare: Binge-eating disorder. *Ch 16*

Burnout — Exhaustion and cynicism from chronic workplace stress, with three dimensions: emotional exhaustion, depersonalization, and reduced personal accomplishment (Christina Maslach). Compare: Compassion fatigue. *Ch 14*

C

Certification — A credential from a professional body, such as the NCC from NBCC; voluntary at the national level. Compare: Licensure. *Ch 17*

Change talk — Client statements in favor of change: Desire, Ability, Reasons, Need (preparatory) and Commitment, Activation, Taking steps (mobilizing), remembered as DARN-CAT (Miller & Rollnick). Compare: Sustain talk. *Ch 5*

Chi-square — Tests whether observed frequencies in categories differ from expected frequencies, using nominal data (Karl Pearson). Compare: Pearson r. *Ch 3*

Circular causality — Behaviors cause and are caused by each other in loops: “She nags because he withdraws; he

withdraws because she nags." Compare: Linear causality. *Ch 7*

Circumstantial thought — Speech that wanders through extra detail but eventually reaches the point. Compare: Tangential thought. *Ch 19*

Cognitive triad — Beck's description of depression as negative views of the self, the world, and the future (Aaron Beck). *Ch 2*

Collaborative empiricism — Beck's stance in which client and counselor test thoughts together like scientists (Aaron Beck). Compare: Disputing. *Ch 2*

Compassion fatigue — Emotional and physical exhaustion from caring for people who are suffering; includes secondary traumatic stress (Charles Figley). Compare: Burnout. *Ch 14*

Confidentiality — The counselor's ethical, and often legal, duty not to disclose client information. Compare: Privileged communication. *Ch 17*

Confrontation — Gently pointing out a discrepancy (words vs. words, words vs. actions, words vs. nonverbals), best used once the alliance is established. Compare: Immediacy. *Ch 4*

Congruence (counselor) — The counselor's inner experience matches what the counselor outwardly communicates. It doesn't mean saying everything one feels (Carl Rogers). Compare: Genuineness. *Ch 22*

Congruence (Holland) — The fit between a person's personality type and the work environment's type (John Holland). Compare: Consistency (Holland). *Ch 14*

Consistency (Holland) — How close a person's top types are on the hexagon; adjacent types are consistent and opposite types are inconsistent (John Holland). Compare: Congruence (Holland). *Ch 14*

Consultation — A voluntary, nonhierarchical request for advice; the consulting counselor keeps full responsibility for the client. Compare: Supervision. *Ch 17*

Contemplation — A stage of change in which the client acknowledges a problem but feels ambivalent. It is never a group stage (Prochaska & DiClemente). Compare: Transition stage. *Ch 5*

Continuing bonds — Maintaining a connection to the deceased can be healthy, not a failure to "let go" (Klass, Silverman, & Nickman). Compare: Prolonged grief disorder. *Ch 12*

Countertransference — The counselor's own emotional reactions to the client, often rooted in the counselor's history. It is managed through self-reflection and supervision or consultation. Compare: Transference. *Ch 5*

Court order — A disclosure order signed by a judge. Comply, but disclose only what is required and ask the court to limit disclosure if needed. Compare: Subpoena. *Ch 17*

Crisis — A person's response when an event overwhelms their usual coping. Crises are time-limited (often about 4 to 6 weeks; Gerald Caplan) and carry both danger and opportunity (James & Gilliland). *Ch 6*

Cultural Formulation Interview — A 16-question DSM-5-TR interview covering the cultural definition of the problem, perceived causes and supports, self-coping and past help-seeking, and current help-seeking. It can be used with any client. *Ch 19*

Cultural humility — An other-oriented, curious stance of ongoing self-reflection that recognizes the limits of one's cultural knowledge (Joshua Hook and colleagues). Compare: Broaching. *Ch 9*

Cutting off — A group leader skill that interrupts a member who goes on too long or off topic, to manage time or focus. Compare: Blocking. *Ch 8*

D

Defense mechanism — An unconscious strategy that protects against anxiety, such as repression, projection, or displacement (Sigmund Freud; later systematized by Anna Freud). *Ch 2*

Delirium — Sudden (hours to days), fluctuating confusion with markedly impaired attention, usually from a medical cause. It is a medical emergency and often reversible. Compare: Major neurocognitive disorder. *Ch 12*

Delusion — A fixed false belief held with full conviction despite evidence. Compare: Obsession. *Ch 19*

Deviation IQ — A standard score with a mean of 100 and an SD of 15 (David Wechsler). Compare: Ratio IQ. *Ch 3*

Dialectical behavior therapy — A therapy that balances acceptance and change, teaching mindfulness, distress tolerance, emotion regulation, and interpersonal effectiveness skills (Marsha Linehan). *Ch 2*

Diana v. State Board of Education (1970) — Court case holding that children must be tested in their primary language, with nonverbal measures, before special education placement. Compare: Larry P. v. Riles. *Ch 20*

Differentiation (Holland) — How clearly one personality type stands out from the others; a flat profile is undifferentiated (John Holland). Compare: Differentiation of self. *Ch 14*

Differentiation of self — Bowen's central concept. Low differentiation lets anxiety pass across generations, so raising it is the goal of therapy (Murray Bowen). Compare: Enmeshment. *Ch 2*

Disenfranchised grief — Grief over a loss that isn't socially recognized, such as the death of an ex-partner, a pet, or a pregnancy loss (Kenneth Doka). Compare: Ambiguous loss. *Ch 12*

Disputing — Ellis's direct, often forceful challenge to irrational beliefs such as "musts" (Albert Ellis). Compare: Collaborative empiricism. *Ch 2*

Disruptive mood dysregulation disorder — Severe temper outbursts 3 or more times a week with irritable mood between them, for at least 12 months; diagnosed ages 6 to 18, with onset before 10. Compare: Oppositional defiant disorder. *Ch 10*

Duty to protect — Tarasoff II (1976): take reasonable steps to protect the intended victim of a client's serious threat, such as warning, notifying police, or arranging hospitalization. Compare: Duty to warn. *Ch 6*

Duty to warn — Tarasoff I (1974): therapists must warn an identifiable victim of a client's serious threat. Compare: Duty to protect. *Ch 6*

E

Ecomap — A diagram of a family's connections to outside systems such as school, work, church, and agencies (Ann Hartman). Compare: Genogram. *Ch 7*

Effect size — How large a difference is, apart from statistical significance. For Cohen's d, about .2 is small, .5 medium, and .8 large. *Ch 18*

Empathy — Understanding the client's experience from the client's frame of reference. Compare: Sympathy. *Ch 22*

Empty chair — A signature Gestalt technique, alongside two-chair dialogue, tied to here-and-now awareness and unfinished business (Fritz Perls). Compare: Psychodrama. *Ch 2*

Enactment — Having family members interact in session so the counselor can see the pattern live (Salvador Minuchin). *Ch 7*

Enmeshment — Diffuse boundaries between family members (Salvador Minuchin). Compare: Triangulation. *Ch 2*

Ethic of care — Moral reasoning grounded in care and relationships, not only justice; a critique of Kohlberg (Carol Gilligan). Compare: Moral development stages. *Ch 3*

Exception questions — SFBT questions about times the problem was absent or weaker (de Shazer & Berg). Compare: Unique outcomes. *Ch 2*

Exposure and response prevention — Facing the obsession without doing the compulsion; the treatment with the strongest support for OCD. Compare: Systematic desensitization. *Ch 5*

Expressed emotion — Family criticism, hostility, and emotional over-involvement linked to higher relapse in schizophrenia; family psychoeducation lowers it (George Brown; Vaughn & Leff). Compare: Double bind. *Ch 16*

External validity — Whether results generalize to other people, settings, and times; supported by random sampling. Compare: Internal validity. *Ch 18*

Externalizing — Talking about the problem as a separate entity acting on the client: “The person is not the problem; the problem is the problem” (White & Epston). Compare: Exception questions. *Ch 2*

F

Factitious disorder — Intentionally falsifying symptoms without an obvious external reward, in order to take on the sick role; imposed on self or on another. Compare: Malingering. *Ch 19*

Family-based treatment — The first-line treatment for adolescent anorexia, in which parents temporarily take charge of refeeding; also called the Maudsley approach (Lock & Le Grange). *Ch 5*

Fidelity — The ethical principle of honoring commitments and keeping promises. Compare: Veracity. *Ch 17*

Filial therapy — Parents are trained to run child-centered play sessions at home (Bernard & Louise Guerney). Compare: PCIT. *Ch 5*

First-order change — Change within the system's rules, so the pattern stays, like grounding a teen for a week instead of a day (MRI). Compare: Second-order change. *Ch 7*

Five stages of dying — Denial, anger, bargaining, depression, and acceptance; described for terminally ill patients and not a required sequence (Elisabeth Kübler-Ross). Compare: Tasks of mourning. *Ch 12*

Flight of ideas — Rapid jumps between loosely connected topics, common in mania. Compare: Loose associations. *Ch 19*

Flooding — Immediate, intense, prolonged real-life exposure to a feared situation. Compare: Implosive therapy. *Ch 5*

Four Horsemen — Criticism, contempt, defensiveness, and stonewalling; contempt is the strongest predictor of divorce (John & Julie Gottman). Compare: Pursue–withdraw cycle. *Ch 7*

G

Gambling disorder — The only behavioral addiction in the DSM-5-TR addictions chapter: at least 4 of 9 criteria in 12 months, with "chasing" losses as the classic sign. Suicide risk is elevated. *Ch 11*

Gender dysphoria — Clinically significant distress related to incongruence between experienced and assigned gender, lasting at least 6 months. Being transgender is not itself a mental disorder. *Ch 15*

General Adaptation Syndrome — The stress response stages of alarm, resistance, and exhaustion; also distin-

guishes eustress from distress (Hans Selye). Compare: Transactional model of stress. *Ch 3*

Generalized anxiety disorder — Excessive, hard-to-control worry about many things plus at least 3 of 6 symptoms (1 in children), for at least 6 months. Compare: Panic disorder. *Ch 10*

Genogram — A diagram of three or more generations showing relationships, patterns, cutoffs, illnesses, and conflict (Bowen's concept; standardized by McGoldrick & Gerson). Compare: Ecomap. *Ch 7*

Genuineness — Being real and authentic, not hiding behind a professional role or façade. Compare: Congruence (counselor). *Ch 22*

Griggs v. Duke Power Co. (1971) — Court case holding that employment tests must be job-related; tests that screen out protected groups without job relevance are discriminatory. *Ch 20*

Grounded theory — A qualitative tradition that develops a theory from the data (Glaser & Strauss). Compare: Phenomenology. *Ch 18*

Group cohesiveness — Belonging and acceptance; the group equivalent of the therapeutic alliance and a precondition for the other factors (Irvin Yalom). Compare: Universality. *Ch 8*

H

Hallucination — A sensory experience with no stimulus present; auditory hallucinations are most common in psychotic disorders. Compare: Illusion. *Ch 19*

Hawthorne effect — Participants change because they know they're being studied (Elton Mayo). Compare: Placebo effect. *Ch 18*

Here-and-now focus — Attention to present experience. It is a central focus of Gestalt therapy (Fritz Perls); in groups, members first experience reactions in the room,

then reflect on what just happened (Irvin Yalom). Compare: Immediacy. *Ch 8*

Hierarchy of needs — Maslow's motivational theory: physiological, safety, love/belonging, esteem, then self-actualization. Unmet lower needs block growth (Abraham Maslow). Compare: Actualizing tendency. *Ch 2*

HIPAA — Federal law protecting the privacy and security of health information. Clients can access their records, and psychotherapy notes get extra protection. *Ch 17*

Hypomanic episode — A manic-like mood pattern lasting at least 4 consecutive days, without marked impairment, hospitalization, or psychotic features. Compare: Manic episode. *Ch 10*

I

Identified patient — The family member whose symptoms bring the family to counseling; the symptom often serves a function for the family. *Ch 7*

Identity statuses — Four statuses: diffusion (no exploration, no commitment), foreclosure (commitment without exploration), moratorium (exploration without commitment), and achievement (commitment after exploration) (James Marcia). *Ch 3*

Illusion — Misperceiving a real stimulus, like a coat on a chair that looks like a person. Compare: Hallucination. *Ch 19*

Immediacy — Addressing what is happening between counselor and client right now. Compare: Self-disclosure. *Ch 4*

Impairment — Physical, mental, or emotional problems that compromise a counselor's work. Counselors refrain from practicing when impaired and seek help (ACA C.2.g). *Ch 17*

Implosive therapy — Intense imagined exposure, often exaggerated (Thomas Stampfl). Compare: Flooding. *Ch 5*

Impostor phenomenon — High achievers feeling like frauds, often heightened for people underrepresented in their fields (Clance & Imes). Compare: Stereotype threat. *Ch 15*

Independent variable — What the researcher manipulates or compares, such as type of treatment. Compare: Dependent variable. *Ch 18*

Industry vs. inferiority — Erikson's school-age stage, when a child compares her work to classmates' and may feel incompetent (Erik Erikson). Compare: Concrete operational stage. *Ch 3*

Informed consent — An ongoing process, not a one-time form, covering services, risks, fees, confidentiality and its limits, and the right to refuse (ACA A.2). *Ch 17*

Intellectual developmental disorder — Deficits in both intellectual and adaptive functioning, beginning in the developmental period. Severity is based on adaptive functioning, not IQ alone. *Ch 12*

Internal validity — Whether the independent variable really caused the change in the dependent variable; supported by random assignment. Compare: External validity. *Ch 18*

Interpersonal theory of suicide — Thwarted belongingness plus perceived burdensomeness create the desire for death, and acquired capability makes an attempt possible (Thomas Joiner). Compare: Psychache. *Ch 6*

Interpretation — Proposing a connection outside the client's current awareness, often to the past, offered tentatively. Compare: Reframe. *Ch 4*

Intimate terrorism — Violence used as part of a pattern of coercive control, usually more frequent, severe, and escalating (Michael P. Johnson). Compare: Situational couple violence. *Ch 13*

J

Jaffee v. Redmond (1996) — U.S. Supreme Court case recognizing psychotherapist–patient privilege in federal courts, including for licensed clinical social workers. *Ch 17*

John Henry effect — The control group works harder to compete with the treatment group. Compare: John Henryism. *Ch 18*

John Henryism — High-effort coping with chronic stressors, including discrimination, linked to health problems in Black Americans with limited resources (Sherman James). Compare: John Henry effect. *Ch 15*

Joining — Connecting with every family member, accommodating the family's style, and earning trust before trying to change anything (Salvador Minuchin). *Ch 7*

Justice — The ethical principle of treating individuals equitably and fostering fairness and equality. Compare: Autonomy. *Ch 17*

K

Kruskal–Wallis test — The nonparametric alternative to one-way ANOVA. Compare: Mann–Whitney U. *Ch 18*

L

Larry P. v. Riles (1979) — Court case holding that IQ tests could not be used to place Black children in California classes for "educable mentally retarded" students because of cultural bias. Compare: Diana v. State Board of Education. *Ch 20*

Learned helplessness — Repeated uncontrollable bad events teach a person that action is useless (Martin Seligman). Compare: Reformulated helplessness theory. *Ch 10*

Licensure — State permission to practice, such as the LPC; the strongest credential, protecting both title and practice. Compare: Certification. *Ch 17*

Life review — A structured review of one's whole life to resolve conflicts and find meaning, suited to late life (Robert Butler). Compare: Reminiscence therapy. *Ch 5*

Life-span, life-space stages — Growth (0–14), exploration (15–24), establishment (25–44), maintenance (45–64), and disengagement (65+), with recycling to earlier stages possible (Donald Super). Compare: Ginzberg's developmental periods. *Ch 14*

Linking — A group leader skill that connects one member's concern to another's, building universality and member-to-member interaction. Compare: Universalizing. *Ch 8*

Loose associations — Ideas with no logical connection, common in psychosis. Compare: Flight of ideas. *Ch 19*

M

Major depressive episode — At least 5 of 9 symptoms, including depressed mood or loss of interest, for at least 2 weeks, with significant distress or impairment. Compare: Persistent depressive disorder. *Ch 10*

Malingering — Intentionally producing or exaggerating symptoms for an external reward, such as money or avoiding prosecution; a Z code, not a mental disorder. Compare: Factitious disorder. *Ch 19*

Malpractice — A legal claim that generally requires four Ds: duty, dereliction (breach of the standard of care), direct causation, and damages. *Ch 17*

Mandated reporting — Counselors must report suspected child abuse on reasonable suspicion, not proof, promptly and without investigating. Reporters acting in good faith are protected. *Ch 6*

Manic episode — Abnormally elevated, expansive, or irritable mood with increased energy for at least 1 week (any duration if hospitalized), with marked impairment. Compare: Hypomanic episode. *Ch 10*

Means restriction counseling — Helping a client and family limit access to firearms, stockpiled medications, and other lethal means during periods of suicide risk. *Ch 6*

Mediator — A variable that explains how or why the independent variable affects the dependent variable. Compare: Moderator. *Ch 18*

Mental status exam — A snapshot of current mental functioning, mostly from observation: appearance, behavior, speech, mood, affect, thought process and content, perception, cognition, insight, and judgment. *Ch 19*

Microinsult — Subtle rudeness that demeans a person's identity, like telling a Black client "You're so articulate" (Derald Wing Sue). Compare: Microinvalidation. *Ch 9*

Microinvalidation — A remark that negates a person's thoughts, feelings, or experiences, like "I don't see color" (Derald Wing Sue). Compare: Microinsult. *Ch 9*

Minority stress model — Distal stressors (discrimination, harassment) and proximal stressors (expected rejection, concealment, internalized stigma) explain higher distress among sexual and gender minority people. The stress, not the identity, is the problem (Ilan Meyer). *Ch 15*

Miracle question — "Suppose you went to sleep tonight and the problem was solved. What would be different tomorrow?" (de Shazer & Berg). Compare: Exception questions. *Ch 2*

Moderator — A variable that changes the strength or direction of a relationship, showing for whom or when an effect holds. Compare: Mediator. *Ch 18*

Modified Angoff method — How NBCC sets each exam form's passing score: experts estimate how a minimally qualified candidate would perform on each item. *Ch 1*

Mood — The client's self-reported, sustained emotional state, often in their own words. Think of it as the "climate." Compare: Affect. *Ch 19*

Moral development stages — Preconventional (obedience and punishment; self-interest), conventional (interpersonal conformity; law and order), and postconventional (social contract; universal ethical principles), judged by justice reasoning (Lawrence Kohlberg). Compare: Ethic of care. *Ch 3*

Motivational interviewing — A method for resolving ambivalence, built on a spirit of partnership, acceptance, compassion, and evocation and the OARS skills (Miller & Rollnick). Compare: Transtheoretical model. *Ch 5*

N

Necessary and sufficient conditions — Rogers' six conditions for change: psychological contact, client incongruence, counselor congruence, unconditional positive regard, empathic understanding, and the client perceiving that empathy and regard (Carl Rogers). *Ch 22*

Negative feedback — Feedback that reduces deviation and keeps the system the same (morphostasis). "Negative" describes direction, not value. Compare: Positive feedback. *Ch 7*

Negative reinforcement — Removing something unpleasant to increase a behavior, like a seatbelt chime that stops when you buckle. Compare: Punishment. *Ch 2*

Nigrescence model — Black racial identity stages: preencounter, encounter, immersion–emersion, internalization, and internalization–commitment (William Cross). Compare: White racial identity model. *Ch 3*

Nonmaleficence — The ethical principle of avoiding actions that cause harm. Compare: Beneficence. *Ch 17*

Normal curve — A symmetrical distribution where mean, median, and mode are equal; about 68%, 95%, and 99.7% of scores fall within 1, 2, and 3 SDs. *Ch 18*

Norm-referenced — Scoring that compares a person to other people. Compare: Criterion-referenced. *Ch 3*

No-secrets policy — In couple or family work, information shared individually may be brought into joint sessions. Explain it at the start, before anyone shares a secret. *Ch 7*

No-suicide contract — A promise not to harm oneself. It has no evidence of preventing suicide and can give the counselor false reassurance. Compare: Safety plan. *Ch 6*

Null hypothesis — The prediction of no difference or relationship. Researchers reject or fail to reject it; they never prove it. *Ch 18*

O

OARS — Motivational interviewing's core skills: Open questions, Affirmations, Reflections, and Summaries (Miller & Rollnick). *Ch 5*

Obsession — An intrusive, unwanted thought, urge, or image that causes anxiety and is usually experienced as alien (ego-dystonic). Compare: Delusion. *Ch 10*

O*NET — The Department of Labor's occupational database, which replaced the Dictionary of Occupational Titles. Compare: Occupational Outlook Handbook. *Ch 14*

Open question — A question that invites elaboration, usually starting with "what" or "how"; the default choice. Compare: Closed question. *Ch 4*

Ordinal scale — Rank order, like class rank; Likert ratings are usually treated as ordinal. Compare: Interval scale. *Ch 3*

Other specified disorder — A diagnosis in which the clinician states why full criteria aren't met. Compare: Unspecified disorder. *Ch 19*

P

Panic disorder — Recurrent unexpected panic attacks followed by at least 1 month of worry about more attacks or changed behavior to avoid them. Compare: Generalized anxiety disorder. *Ch 10*

Paradoxical intention — Asking an individual to intend the feared symptom, like trying hard to stay awake for insomnia (Viktor Frankl). Compare: Prescribing the symptom. *Ch 2*

Paraphrase — Restating the content of one client statement in fresh words. Compare: Reflection of feeling. *Ch 4*

Parenting styles — Authoritative (warm and firm, best outcomes), authoritarian (firm, not warm), and permissive (warm, not firm); Maccoby & Martin added neglectful/uninvolved (Diana Baumrind). *Ch 3*

Part 2 (42 CFR Part 2) — Federal rules giving extra confidentiality protection to records from federally assisted substance use disorder programs; disclosure generally requires specific written consent. Compare: HIPAA. *Ch 9*

PCIT — Parent–Child Interaction Therapy, which coaches parents in real time, often through an earpiece, with a behavioral focus (Sheila Eyberg). Compare: Filial therapy. *Ch 5*

Pearson r — Measures the relationship between two interval/ratio variables, from −1.00 to +1.00; the absolute value shows strength. Compare: Spearman rho. *Ch 18*

Persistent depressive disorder — Depressed mood most days plus at least 2 other symptoms for at least 2 years in adults (1 year in children and adolescents). Compare: Major depressive episode. *Ch 10*

Planned happenstance — Treating unplanned events, like a layoff, as career opportunities (John Krumboltz). Compare: Career adaptability. *Ch 3*

Positive feedback — Feedback that amplifies deviation and pushes the system toward change (morphogenesis), like an argument that escalates. “Positive” describes direction, not value. Compare: Negative feedback. *Ch 7*

Positive skew — A distribution with its tail to the right and most scores low, as on a hard test; mean > median > mode. Compare: Negative skew. *Ch 18*

Posttraumatic stress disorder — After actual or threatened death, serious injury, or sexual violence: intrusion, avoidance, negative changes in thinking and mood, and arousal symptoms for more than 1 month. Compare: Acute stress disorder. *Ch 10*

Prescribing the symptom — A paradoxical directive used with a family to interrupt a stuck pattern (Jay Haley; MRI). Compare: Paradoxical intention. *Ch 2*

Privileged communication — A legal right that protects client communications from disclosure in court; the client holds it and can waive it. Compare: Confidentiality. *Ch 17*

Projection — A defense that attributes one’s own unacceptable feelings to someone else (Sigmund Freud). Compare: Displacement. *Ch 2*

Prolonged grief disorder — Intense yearning for or preoccupation with the deceased, plus other symptoms, at least 12 months after the death for adults (6 months for youth), beyond cultural norms. Compare: Continuing bonds. *Ch 12*

Psychache — Intolerable psychological pain, the core of suicide in Shneidman’s view (Edwin Shneidman). Compare: Interpersonal theory of suicide. *Ch 6*

Psychodrama — A group method with a protagonist, director, auxiliary egos, audience, and stage, using doubling, role reversal, and the mirror technique (J. L. Moreno). Compare: Empty chair. *Ch 3*

Psychological first aid — An evidence-informed early response after a disaster: contact, safety, stabilization,

practical help, and connection to supports, without pushing survivors to retell the trauma. Compare: Psychological debriefing. *Ch 6*

Punishment — A consequence that decreases behavior: positive punishment adds something unpleasant, and negative punishment removes something pleasant. Compare: Negative reinforcement. *Ch 2*

p-value — The probability of results at least this extreme if the null hypothesis were true; the result is significant when p is below alpha. *Ch 18*

Q

Quasi-experiment — A design that manipulates the independent variable without random assignment, often using intact groups such as two classrooms. Compare: True experiment. *Ch 18*

R

Random assignment — Placing participants into groups by chance; it supports internal validity and cause-and-effect conclusions. Compare: Random sampling. *Ch 18*

Reaction formation — A defense that expresses the opposite of what one feels (Sigmund Freud). *Ch 2*

Reflection of feeling — Naming the emotion under the client's content. Compare: Paraphrase. *Ch 4*

Reflection of meaning — Naming the value or belief underneath, or why something matters to the client. Compare: Reflection of feeling. *Ch 4*

Reframe — Offering a new meaning for the same facts. Compare: Interpretation. *Ch 4*

Repression — A defense that pushes threatening material out of awareness (Sigmund Freud). Compare: Denial. *Ch 2*

RIASEC — Six personality and work types: Realistic, Investigative, Artistic, Social, Enterprising, Conventional,

arranged in a hexagon. Opposite pairs are R–S, I–E, and A–C (John Holland). *Ch 14*

Romantic relationships with former clients — Prohibited for 5 years after the last professional contact; after that, counselors must still show the relationship isn't exploitative (ACA A.5.c). *Ch 17*

S

Safety plan — A written plan made with the client: warning signs, internal coping, distracting people and places, people to ask for help, professionals and 988, and making the environment safe (Stanley & Brown). Compare: No-suicide contract. *Ch 6*

Schizophrenia — At least 2 of 5 psychotic symptoms with a decline in functioning; active symptoms for at least 1 month and continuous signs for at least 6 months. Compare: Schizophreniform disorder. *Ch 16*

Schizophreniform disorder — The same symptoms as schizophrenia, lasting at least 1 month but less than 6 months. Compare: Schizophrenia. *Ch 16*

Second-order change — Change to the system's rules themselves, like parents starting to agree on rules together. Compare: First-order change. *Ch 7*

Section 504 plan — Accommodations for a student whose disability substantially limits a major life activity, even without a need for special education. Compare: Individualized Education Program. *Ch 12*

Self-actualization — The top of Maslow's hierarchy of needs; a motivational theory, not a therapy (Abraham Maslow). Compare: Actualizing tendency. *Ch 2*

Self-disclosure — Briefly sharing something from the counselor's own life for the client's benefit, then returning focus to the client. Compare: Immediacy. *Ch 4*

Self-efficacy — Confidence in one's ability, built through mastery experiences, modeling, and encouragement. Low

self-efficacy, not low ability, can keep capable clients from a field (Albert Bandura; applied to careers by Betz & Hackett). *Ch 14*

Sensitivity — A test's ability to correctly identify people who have the condition; screening tools prioritize it. Compare: Specificity. *Ch 20*

Situational couple violence — Violence arising from specific conflicts that escalate, without a broader pattern of control; the most common type in community samples (Michael P. Johnson). Compare: Intimate terrorism. *Ch 13*

SMART objectives — Treatment objectives that are Specific, Measurable, Achievable, Relevant, and Time-bound. Compare: Well-formed goals. *Ch 21*

SOAP note — A progress note: Subjective (the client's report, in their words), Objective (counselor observations and scores), Assessment, and Plan. Compare: BIRP note. *Ch 21*

Social interest — Adler's central concept (Gemeinschaftsgefühl). Adlerian counseling aims to increase it and to correct basic mistakes (Alfred Adler). *Ch 2*

SOLER — An attending posture: face the client Squarely, Open posture, Lean slightly in, Eye contact, Relaxed. Adapt it to the client's culture (Gerard Egan). *Ch 4*

Specificity — A test's ability to correctly identify people who don't have the condition (true negative rate). Compare: Sensitivity. *Ch 20*

Standard error of measurement — Builds a confidence band around one person's score: about 68% within ±1 SEM and 95% within ±2 SEM. Higher reliability means a smaller SEM. Compare: Standard deviation. *Ch 20*

Statistical regression — Participants selected for extreme scores drift toward the mean on retest. Compare: Maturation. *Ch 18*

Stereotype threat — Concern about confirming a negative stereotype about one's group lowers performance

(Steele & Aronson). Compare: Impostor phenomenon. *Ch 15*

Strange Situation — A procedure identifying secure, anxious-avoidant, and anxious-resistant infant attachment; Main & Solomon later added disorganized (Mary Ainsworth). *Ch 3*

Subpoena — A request for records, often issued by an attorney. Don't ignore it or simply hand over records; contact the client and assert privilege. Compare: Court order. *Ch 17*

Substance use disorder severity — Based on the number of 11 criteria met: mild 2–3, moderate 4–5, severe 6 or more. Early remission is 3 to under 12 months and sustained remission 12+ months with no criteria met except craving. Compare: Gambling disorder. *Ch 11*

Summary — A response that ties together several statements or themes. Compare: Paraphrase. *Ch 4*

Supervision — An ongoing, evaluative, hierarchical relationship in which the supervisor shares responsibility for client care. Compare: Consultation. *Ch 17*

Sustain talk — The client's arguments for staying the same, met with reflection rather than argument (Miller & Rollnick). Compare: Change talk. *Ch 5*

Sympathy — Feeling sorry for the client from one's own frame of reference. Compare: Empathy. *Ch 22*

Systematic desensitization — Relaxation paired with imagined, gradual steps up an anxiety hierarchy, based on reciprocal inhibition (Joseph Wolpe). Compare: Flooding. *Ch 5*

T

Tangential thought — Speech that wanders off and never returns to the point. Compare: Circumstantial thought. *Ch 19*

Tasks of mourning — Accept the reality of the loss, process the pain, adjust to a world without the deceased, and find an enduring connection while embarking on a new life (J. William Worden). Compare: Five stages of dying. *Ch 12*

Telehealth licensure — Counselors generally must be licensed or otherwise authorized in the state where the client is physically located during the session. Under ACA H. 1.b, distance counselors may be subject to the laws of both the counselor's location and the client's place of residence. *Ch 9*

Test-retest reliability — Stability over time, checked by giving the same test to the same people twice. Compare: Internal consistency. *Ch 3*

Therapeutic factors — Yalom's 11 curative elements of group work, including instillation of hope, universality, altruism, group cohesiveness, catharsis, and interpersonal learning (Irvin Yalom). *Ch 8*

Tolerance — Needing more of a substance for the same effect, or getting less effect from the same amount. It doesn't count toward diagnosis with medications taken as prescribed. Compare: Withdrawal. *Ch 11*

Transference — The client redirects feelings and expectations from important past relationships onto the counselor. Compare: Countertransference. *Ch 5*

Transition stage — The group stage of anxiety, resistance, conflict, and challenges to the leader (Tuckman's storming); the leader responds non-defensively (Gerald Corey). Compare: Contemplation. *Ch 8*

Transtheoretical model — The stages of change: precontemplation, contemplation, preparation, action, and maintenance. It describes readiness and is not a therapy (Prochaska & DiClemente). Compare: Motivational interviewing. *Ch 5*

Triangular theory of love — Love's three components are intimacy, passion, and commitment. All three make

consummate love; intimacy plus commitment is companionate love (Robert Sternberg). *Ch 13*

Triangulation — Two people in conflict draw in a third to reduce tension, like a parent confiding in a child about the other parent (Murray Bowen). Compare: Enmeshment. *Ch 7*

True experiment — A design with random assignment, manipulation of the independent variable, and a control group; the strongest for cause and effect. Compare: Quasi-experiment. *Ch 18*

T-score — A standard score with a mean of 50 and an SD of 10. On the MMPI-3, 65 or above is generally clinically elevated. Compare: z-score. *Ch 20*

t-test — Compares two group means: independent for different groups, paired for the same people measured twice (William Gosset). Compare: ANOVA (one-way). *Ch 18*

Tuckman's stages — Forming, storming, norming, performing, and adjourning, with adjourning added in 1977 (Bruce Tuckman, with Mary Ann Jensen). Compare: Transition stage. *Ch 3*

Type I error — Rejecting a true null hypothesis: a false positive. Compare: Type II error. *Ch 18*

Type II error — Failing to reject a false null hypothesis: a false negative. Compare: Type I error. *Ch 18*

U

Unconditional positive regard — Valuing the client as a person without conditions; it doesn't mean approving of everything the client does (Carl Rogers). *Ch 22*

Unique outcomes — Narrative therapy's counterpart to the SFBT exception; the word, not the meaning, tells you the owner (White & Epston). Compare: Exception questions. *Ch 2*

Universality — Yalom's therapeutic factor of realizing "I'm not the only one" (Irvin Yalom). Compare: Group cohesiveness. *Ch 8*

Universalizing — A group leader skill that names a shared theme for the whole group. Compare: Linking. *Ch 8*

Unspecified disorder — A diagnosis in which the clinician chooses not to, or can't, specify why full criteria aren't met; common in emergency settings. Compare: Other specified disorder. *Ch 19*

V

Values-based referral — Referring a client solely because of the counselor's personal values, which is prohibited (ACA A.11.b). Referral is appropriate for competence reasons, not values. *Ch 9*

Veracity — The ethical principle of dealing truthfully with individuals. Compare: Fidelity. *Ch 17*

Vicarious liability — Supervisors can be held legally responsible for supervisees' actions (respondeat superior). Compare: Direct liability. *Ch 17*

Vicarious trauma — Lasting changes in the helper's beliefs and worldview from repeated exposure to clients' trauma (McCann & Pearlman). Compare: Burnout. *Ch 14*

W

WDEP — Wants, Doing, Evaluation, Planning, as in "Is what you're doing getting you what you want?" (Robert Wubbolding, building on Glasser's reality therapy). Compare: Disputing. *Ch 2*

Well-formed goals — Goals that are small, concrete, behavioral, stated as the presence of something, important to the client, and within the client's control (Steve de Shazer). Compare: SMART objectives. *Ch 21*

WHODAS 2.0 — The disability assessment schedule the DSM-5-TR suggests for measuring functioning, in place of the dropped GAF. *Ch 19*

Window of tolerance — The zone of arousal where a person can function; trauma narrows it, causing swings into hyperarousal or hypoarousal (Daniel Siegel). *Ch 10*

Withdrawal — A substance-specific syndrome that appears when use stops or decreases. Compare: Tolerance. *Ch 11*

Working alliance — Agreement on goals, agreement on tasks, and the bond between client and counselor (Edward Bordin). *Ch 4*

Working stage — The group stage of cohesion, genuine self-disclosure, risk-taking, and direct feedback (Tuckman's norming/performing) (Gerald Corey). Compare: Transition stage. *Ch 8*

Y

Yerkes–Dodson law — Performance improves with arousal up to a point, then declines (Yerkes & Dodson). *Ch 10*

Z

Z codes — Other conditions that may be a focus of clinical attention, such as uncomplicated bereavement or relationship distress; not mental disorders. *Ch 19*

Zone of proximal development — Learning that happens socially, just beyond current ability, supported by scaffolding (Lev Vygotsky). *Ch 3*

z-score — The basic standard score, with a mean of 0 and an SD of 1: z = (score − mean) ÷ SD. Compare: T-score. *Ch 20*

Audio Companion Track List

The audio companion is sold separately on PassReady Prep. Every track is call-and-response: you hear a term, the narrator pauses, you say the owner out loud, then the answer plays. Play tracks on repeat during drives, walks, and chores.

Track numbers match chapter numbers (Track 10.1 goes with Chapter 10). Chapter 1 covers exam format and strategy and has no track.

Track	Title	Chapter	Covers	Target length
2.1	Theory Ownership: Psychodynamic, Humanistic, and Existential	2. The Theory Ownership Matrix	Psychodynamic + Humanistic/ Existential	~12 min
2.2	Theory Ownership: Behavioral, Cognitive-Behavioral, and Change Models	2. The Theory Ownership Matrix	Behavioral + Cognitive-Behavioral + change models	~10 min
2.3	Theory Ownership: Postmodern and Family Systems	2. The Theory Ownership Matrix	Postmodern + Family Systems	~10 min
2.4	Swap Pairs Rapid Fire	2. The Theory Ownership Matrix	Swap Pairs Rapid Fire (all families)	~8 min
3.1	Ownership Beyond Theory: Development and Identity	3. Ownership Beyond Theory	Development + identity models	~5 min
3.2	Ownership Beyond Theory: Career	3. Ownership Beyond Theory	Career theorists	~3 min

Track	Title	Chapter	Covers	Target length
3.3	Ownership Beyond Theory: Group	3. Ownership Beyond Theory	Group stages and group owners	~2 min
3.4	Ownership Beyond Theory: Assessment and Research	3. Ownership Beyond Theory	Assessment and research rapid fire	~4 min
4.1	Name That Skill	4. The Alliance and the Microskills	label the counselor's response	~10 min
5.1	Match It	5. Theory into Intervention	problem, age, and stage → intervention	~10 min
6.1	Safety First	6. Crisis, Safety, and Risk	crisis models, suicide assessment, duty to protect, reporting	~8 min
7.1	Think in Systems	7. Couples and Family Systems in Session	concepts, tools, life cycle, couples	~7 min
8.1	Lead the Group	8. Group Work	stages, skills, harmful behaviors, ethics	~7 min
9.1	Fit the Client	9. Delivery Formats and Special Populations	culture, faith, addiction, telehealth, populations	~7 min
10.1	Time Frames and Owners	10. Mood, Anxiety, Obsessive, and Trauma Presentations	mood, anxiety, OCD, trauma	~8 min
11.1	Count It and Spot It	11. Substance Use and Process Addictions	criteria, severity, intoxication, withdrawal, gambling	~5 min

Track	Title	Chapter	Covers	Target length
12.1	Across the Life Span	12. Life Span and Development	neurodevelopment, separation, aging, grief, end of life	~6 min
13.1	Families in Transition	13. Relationships and Family Life	love, divorce, stepfamilies, adoption, parenting, violence	~6 min
14.1	Work It	14. Career and Occupation	Holland, Super, instruments, burnout, job loss	~4 min
15.1	Identity and Meaning	15. Culture, Identity, and Meaning	culture shock, gender identity, minority stress, racism, faith, existential concerns	~4 min
16.1	Body and Mind	16. Body and Mind	illness, behavior, bullying, sleep, eating, sex, stress, psychosis	~7 min
17.1	Code and Law	17. Ethics and Law	principles, consent, confidentiality, boundaries, records, decision-making	~6 min
18.1	Pick the Test	18. Research, Statistics, and Program Evaluation	designs, validity threats, statistics, evaluation	~5 min
19.1	Intake and MSE	19. Intake, Interview, Mental Status Exam, and Diagnosis	interview, MSE terms, diagnosis, level of care	~5 min
20.1	Score It	20. Assessment and Testing	test types, scores, SEM, instruments, ethics	~5 min

Track	Title	Chapter	Covers	Target length
21.1	Plan It	21. Treatment Planning, Referral, and Termination	goals, priorities, referral, notes, revision, termination	~4 min
22.1	Be the Instrument	22. The Counselor as Instrument: Core Counseling Attributes	Rogers' conditions, empathy, congruence, regard, conflict, self-awareness	~4 min

27 tracks.

References

All sources cited in Chapters 1–22, in APA 7 style. Each chapter also ends with its own short reference list.

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